PHD IN CHILDREN AND ADOLESCENT COUNSELING

CAC.902

Theories of Counseling and Psychotherapy

A complete course volume with original counseling lectures, real-life applications, vector diagrams, doctoral seminar tasks, and attributed open educational readings.

Book 2 of 32 • PDF edition

Educational coursebook. Not an official university publication and not a substitute for supervised clinical training, diagnosis, or emergency care.

How to Use This Book

Purpose and scope

This volume covers CAC.902 Theories of Counseling and Psychotherapy within the supplied PhD curriculum. It compares major theoretical traditions and applies them to children, adolescents, families, schools, and community settings. It is a complete course volume, not a claim to contain the entire discipline.

Two clearly identified layers

Each chapter begins with a newly written counseling lecture, a real-life application, and a doctoral seminar task. An original vector diagram makes a central relationship visible. The chapter then provides attributed foundational readings adapted from CC BY open educational texts. The readings supply broader theoretical background; the new lectures provide the counseling interpretation and critical cautions.

How to study

Read the lecture first, then the diagram, then the adapted reading. Use the workbook to turn theory into a testable professional task. In supervision, compare at least two formulations for the same case before choosing an intervention.

Simple English and real-life application

Technical terms are introduced gradually and defined in the glossary. Fictional cases show how theory changes the questions a counselor asks. The cases are teaching examples, not treatment protocols.

Doctoral level and safety

The book assumes graduate study in counseling or a related field. It does not authorize unsupervised practice. Crisis, abuse, suicide risk, psychosis, and severe impairment require qualified assessment and local emergency procedures.

Source transparency

The adapted readings are used under Creative Commons Attribution 4.0. They have been selected, reorganized, and typeset for this course, with new lectures and diagrams added. They are not presented as wholly original text. Module-level attribution and pinned source snapshots appear in the attribution register. Inherited references are identified separately from the supplemental sources checked for this edition.

Integrative areas

Diversity, ethics, research, and supervision are integrated throughout. Chapter 17 gives focused attention to culture, African perspectives, and feminist critique; Chapter 20 consolidates ethics, supervision, and research.

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CHAPTER 01 / CAC.902

What a Theory Does: Maps, Models, and Evidence

A theory is a map, not the territory. When a counselor meets a child who refuses school, a theory helps the counselor ask useful questions. A behavioral map may ask what happens before and after the refusal. A cognitive map may ask what the child expects will happen at school. A family map may ask who is worried, who is angry, and what changes at home when the child stays home. An attachment map may ask whether the child trusts that adults will respond when distress is high. None of these maps is the whole child. Each map makes some features easier to see and other features easier to miss.

This chapter begins the book with a simple claim: theories are tools for organized curiosity. They are not labels to pin on a young person, and they are not permission to stop listening. A good theory helps a counselor notice patterns, form a tentative explanation, choose an intervention, and check whether the explanation was useful. A bad use of theory happens when the counselor decides in advance what the client must mean. The client then becomes evidence for the theory instead of a person whose experience can correct the theory.

Learning goals

By the end of this chapter, you should be able to explain the difference between a theory, a model, and a technique; describe why evidence-based practice includes more than a manual; and write a small case formulation that separates observation from interpretation. You should also be able to name one strength and one limit of at least two theoretical orientations. These goals matter because doctoral counseling work requires more than knowing many names. It requires the ability to justify a choice, notice when the choice is not helping, and change course without losing respect for the client.

Three words that are often confused

A theory is a broad set of ideas about how people change. Psychoanalytic theory, for example, proposes that unconscious conflict and early relationships shape current experience. A model is a more specific picture of a process. A cognitive model of panic may show how a bodily sensation, a catastrophic thought, and a safety behavior keep fear alive. A technique is an action used in session or between sessions. A thought record is a technique. A reflective statement is a technique. A technique can be used well or badly, and the same technique can appear in more than one theory.

Consider a school counselor who asks a student to rate anxiety from zero to ten. The rating itself is not a theory. If the counselor uses the rating to test whether gradual exposure reduces fear, the technique is being guided by a behavioral model. If the counselor uses the rating to help the student notice a felt sense in the body, the same tool may serve an experiential purpose. The meaning of the technique depends on the case formulation and the goal.

Evidence is a question, not a badge

Students sometimes ask which theory is true. A more practical question is: true for what, for whom, under what conditions, and compared with what? A treatment may have strong evidence for adult panic disorder and weaker evidence for a seven-year-old with separation anxiety and a recent foster placement. A relationship practice may be supported across many studies, while a specific technique may be supported only for a narrow problem. Evidence-based practice asks the counselor to combine research, clinical skill, and the client's culture, preferences, and circumstances.

Imagine two adolescents with depression. One wants a structured plan and likes homework. Another feels controlled by adults and shuts down when given tasks. The same CBT manual may fit the first youth well and damage engagement with the second if used rigidly. The counselor does not abandon evidence. The counselor adapts the pace, language, and goals, and monitors whether the adaptation is working. Flexibility without measurement can become drift. Measurement without flexibility can become a bureaucratic exercise. Doctoral practice holds both.

A child-adolescent example

Nine-year-old Amina cries when her mother leaves the classroom. The teacher says Amina is manipulative. The mother says Amina is shy. Amina says, "I don't know why, my stomach hurts." A counselor using only a behavioral label might miss grief after a grandmother's death. A counselor using only an attachment label might miss bullying at recess. A counselor using only a medical label might miss the fact that symptoms disappear on weekends. A good formulation begins with observation: when does the stomach pain happen, who responds, what has changed recently, and what does Amina fear will happen?

The counselor can write two columns. In the first column: "Amina cries at classroom door on Monday and Tuesday; mother stays for twenty minutes; pain reported before entry; no pain on Saturday." In the second column: "Possible separation fear; possible avoidance maintained by extra time at home; possible peer problem; possible grief reaction." The second column is hypothesis, not fact. The next step is to test the hypotheses with respectful questions, collateral information, and small changes.

Common factors and specific ingredients

Research on psychotherapy often distinguishes common factors from specific ingredients. Common factors include alliance, empathy, positive regard, goal agreement, and a credible rationale for treatment. Specific ingredients include exposure, cognitive restructuring, interpretation of transference, or chair work. The debate is not settled by saying that only relationships matter or only techniques matter. A warm relationship without a useful task may not change a phobia. A powerful technique delivered without trust may not be accepted. In child work, the alliance also includes caregivers and systems. A child may trust the counselor while the parent does not trust the plan.

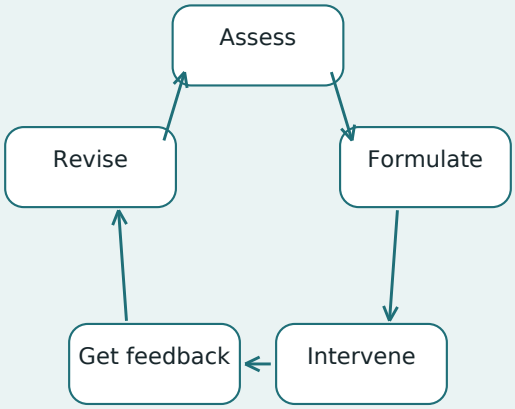
A practical rule is to build the relationship and the rationale together. The counselor might say, "We will try to understand the fear, and we will also practice small steps so your body can learn that school is safe." This sentence offers empathy, a goal, and a method. It also invites correction. If the child says, "It's not fear, it's the noise," the counselor has new information. The theory must move.

Diagram and reflection

The diagram for this chapter shows assessment feeding formulation, formulation feeding intervention, and feedback returning to assessment. The loop is more important than any single box. A doctoral counselor is not the person who never changes a formulation. A doctoral counselor is the person who changes it for good reasons and documents those reasons.

For reflection, choose one theory you already like. Write what it helps you notice. Then write what it may cause you to overlook. Finally, write one question you could ask a client that would allow the client to disagree with your favorite theory. That question is the beginning of ethical integration.

The theory-to-practice loop



Theory guides action, and feedback corrects theory. The loop protects against rigid labels.

Attributed Foundational Reading

Attribution and critical-reading note

Adapted from Psychology 2e by Rose M. Spielman, William J. Jenkins, and Marilyn D. Lovett, OpenStax, Rice University (2020), CC BY 4.0. Selection, organization, and PDF layout modified for this course; new lectures and course diagrams are separate additions.

Read historical and theoretical claims critically. A theory is a lens for inquiry, not a verdict about a child or family. Where source claims are dated or culture-bound, the new lecture supplies cautions and contemporary framing.

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Before we explore the various approaches to therapy used today, let’s begin our study of therapy by looking at how many people experience mental illness and how many receive treatment. According to the U.S. Department of Health and Human Services (2017), 18.9% of U.S. adults experienced mental illness in 2017. For teens (ages 13–18), the rate is similar to that of adults, and for children ages 8–15, current estimates suggest that approximately 13% experience mental illness in a given year (National Institute of Mental Health [NIMH], 2017).

With many different treatment options available, approximately how many people receive mental health treatment per year? According to the Substance Abuse and Mental Health Services Administration (SAMHSA), in 2017, 14.8% of adults received treatment for a mental health issue (NIMH, 2017). These percentages, shown in , reflect the number of adults who received care in inpatient and outpatient settings and/or used prescription medication for psychological disorders.

Source figure caption: The percentage of adults who received mental health treatment in 2004–2008 is shown. Adults seeking treatment increased slightly from 2004 to 2008.

Children and adolescents also receive mental health services. The Centers for Disease Control and Prevention's National Health and Nutrition Examination Survey (NHANES) found that approximately half (50.6%) of children with mental disorders had received treatment for their disorder within the past year (NIMH, n.d.). However, there were some differences between treatment rates by category of disorder (). For example, children with anxiety disorders were least likely to have received treatment in the past year, while children with ADHD or a conduct disorder were more likely to receive treatment. Can you think of some possible reasons for these differences in receiving treatment?

Source figure caption: About one-third to one-half of U.S. adolescents (ages 8–15) with mental disorders receive treatment, with behavior-related disorders more likely to be treated.

Considering the many forms of treatment for mental health disorders available today, how did these forms of treatment emerge? Let’s take a look at the history of mental health treatment from the past (with some questionable approaches in light of modern understanding of mental illness) to where we are today.

Treatment in the Past

For much of history, people with mental illness have been treated very poorly. It was believed that mental illness was caused by demonic possession, witchcraft, or an angry god (Szasz, 1960). For example, in medieval times, abnormal behaviors were viewed as a sign that a person was possessed by demons. If someone was considered to be possessed, there were several forms of treatment to release spirits from the individual. The most common treatment was exorcism, often conducted by priests or other religious figures: Incantations and prayers were said over the person’s body, and they may have been given some medicinal drinks. Another form of treatment for extreme cases of mental illness was trephining: A small hole was made in the afflicted individual’s skull to release spirits from the body. Most people treated in this manner died. In addition to exorcism and trephining, other practices involved execution or imprisonment of people with psychological disorders. Still others were left to be homeless beggars. Generally speaking, most people who exhibited strange behaviors were greatly misunderstood and treated cruelly. The prevailing theory of psychopathology in earlier history was the idea that mental illness was the result of demonic possession by either an evil spirit or an evil god because early beliefs incorrectly attributed all unexplainable phenomena to deities deemed either good or evil.

From the late 1400s to the late 1600s, a common belief perpetuated by some religious organizations was that some people made pacts with the devil and committed horrible acts, such as eating babies (Blumberg, 2007). These people were considered to be witches and were tried and condemned by courts—they were often burned at the stake. Worldwide, it is

estimated that tens of thousands of mentally ill people were killed after being accused of being witches or under the influence of witchcraft (Hemphill, 1966)

By the 18th century, people who were considered odd and unusual were placed in asylums (). **Asylums** were the first institutions created for the specific purpose of housing people with psychological disorders, but the focus was ostracizing them from society rather than treating their disorders. Often these people were kept in windowless dungeons, beaten, chained to their beds, and had little to no contact with caregivers.

Source figure caption: This painting by Francisco Goya, called The Madhouse, depicts a mental asylum and its inhabitants in the early 1800s. It portrays those with psychological disorders as victims.

In the late 1700s, a French physician, Philippe **Pinel**, argued for more humane treatment of people with mental illness. He suggested that they be unchained and talked to, and that's just what he did for patients at La Salpêtrière in Paris in 1795 (). Patients benefited from this more humane treatment, and many were able to leave the hospital.

Source figure caption: This painting by Tony Robert-Fleury depicts Dr. Philippe Pinel ordering the removal of chains from patients at the Salpêtrière asylum in Paris.

In the 19th century, Dorothea **Dix** led reform efforts for mental health care in the United States (). She investigated how those who are mentally ill and poor were cared for, and she discovered an underfunded and unregulated system that perpetuated abuse of this population (Tiffany, 1891). Horrified by her findings, Dix began lobbying various state legislatures and the U.S. Congress for change (Tiffany, 1891). Her efforts led to the creation of the first mental asylums in the United States.

Source figure caption: Dorothea Dix was a social reformer who became an advocate for the indigent insane and was instrumental in creating the first American mental asylum. She did this by relentlessly lobbying state legislatures and Congress to set up and fund such institutions.

Despite reformers' efforts, however, a typical asylum was filthy, offered very little treatment, and often kept people for decades. At Willard Psychiatric Center in upstate New York, for example, one treatment was to submerge patients in cold baths for long periods of time. Electroshock treatment was also used, and the way the treatment was administered often broke patients' backs; in 1943, doctors at Willard administered 1,443 shock treatments (Willard Psychiatric Center, 2009). (Electroshock is now called electroconvulsive treatment, and the therapy is still used, but with safeguards and under anesthesia. A brief application of electric stimulus is used to produce a generalized seizure. Controversy continues over its effectiveness versus the side effects.) Many of the wards and rooms were so cold that a glass of water would be frozen by morning (Willard Psychiatric Center, 2009). Willard's doors were not closed until 1995. Conditions like these remained commonplace until well into the 20th century.

Starting in 1954 and gaining popularity in the 1960s, antipsychotic medications were introduced. These proved a tremendous help in controlling the symptoms of certain psychological disorders, such as psychosis. Psychosis was a common diagnosis of individuals in mental hospitals, and it was often evidenced by symptoms like hallucinations and delusions, indicating a loss of contact with reality. Then in 1963, Congress passed and John

F. Kennedy signed the Mental Retardation Facilities and Community Mental Health Centers Construction Act, which provided federal support and funding for community mental health centers (National Institutes of Health, 2013). This legislation changed how mental health services were delivered in the United States. It started the process of **deinstitutionalization**, the closing of large asylums, by providing for people to stay in their communities and be treated locally. In 1955, there were 558,239 severely mentally ill patients institutionalized at public hospitals (Torrey, 1997). By 1994, by percentage of the population, there were 92% fewer hospitalized individuals (Torrey, 1997).

Mental Health Treatment Today

Today, there are community mental health centers across the nation. They are located in neighborhoods near the homes of clients, and they provide large numbers of people with mental health services of various kinds and for many kinds of problems. Unfortunately, part of what occurred with deinstitutionalization was that those released from institutions were supposed to go to newly created centers, but the system was not set up effectively. Centers were underfunded, staff was not trained to handle severe illnesses such as schizophrenia, there was high staff burnout, and no provision was made for the other services people needed, such as housing, food, and job training. Without these supports, those people released under deinstitutionalization often ended up homeless. Even today, a large portion of the homeless population is considered to be mentally ill (). Statistics show that 26% of homeless adults living in shelters experience mental illness (U.S. Department of Housing and Urban Development [HUD], 2011).

Source figure caption: (a) Of the homeless individuals in U.S. shelters, about one-quarter have a severe mental illness (HUD, 2011). (b) Correctional institutions also report a high number of individuals living with mental illness. (credit a: modification of work by "Carl Campbell"/Flickr; credit b: modification of work by Bart Everson)

Mental health issues are also significant concerns regarding those involved in the corrections system. According to a 2006 special report by the Bureau of Justice Statistics (BJS), approximately 705,600 adults with mental health issues were incarcerated in the state prison system, and another 78,800 were incarcerated in the federal prison system. A further 479,000 were in local jails. According to the study, “people with mental illnesses are overrepresented in probation and parole populations at estimated rates ranging from two to four times the general population” (Prins & Draper, 2009, p. 23). The correctional system faces increasing burdens in managing and supporting people with mental health issues (Torrey et al., 2014). The incarceration experience itself exacerbates existing issues, triggers disturbances, or leads to long term effects such as PTSD, depression, or other disorders. While incarcerated, people are often subjected to or witness violence or sexual assault, face isolation from family, and endure other physical or emotional traumas (Edgemon & Clay-Warner, 2019).

Today, instead of asylums, there are psychiatric hospitals run by state governments and local community hospitals focused on short-term care. In all types of hospitals, the emphasis is on short-term stays, with the average length of stay being less than two weeks and often only several days. This is partly due to the very high cost of psychiatric hospitalization, which can be about $800 to $1000 per night (Stensland, Watson, & Grazier, 2012). Therefore, insurance coverage often limits the length of time a person can be hospitalized

for treatment. Usually individuals are hospitalized only if they are an imminent threat to themselves or others.

Note: View this timeline that shows the history of mental institutions in the United States to learn more.

Most people who have mental illnesses are not hospitalized. If someone is feeling very depressed, complains of hearing voices, or feels anxious all the time, they might seek psychological treatment. A friend, spouse, or parent might refer someone for treatment. The individual might go see their primary care physician first and then be referred to a mental health practitioner.

Some people seek treatment because they are involved with the state's child protective services—that is, their children have been removed from their care due to abuse or neglect. The parents might be referred to psychiatric or substance abuse facilities and the children would likely receive treatment for trauma. If the parents are interested in and capable of becoming better parents, the goal of treatment might be family reunification. For other children whose parents are unable to change—for example, the parent or parents who are heavily addicted to drugs and refuse to enter treatment—the goal of therapy might be to help the children adjust to foster care and/or adoption ().

Source figure caption: Therapy with children may involve play. (credit: modification of work by UNHCR Ukraine/Flickr)

Some people seek therapy because the criminal justice system referred them or required them to go. For some individuals, for example, attending weekly counseling sessions might be a condition of parole. If an individual is mandated to attend therapy, they are seeking services involuntarily. **Involuntary treatment** refers to therapy that is not the individual's choice. Other individuals might voluntarily seek treatment. **Voluntary treatment** means the person chooses to attend therapy to obtain relief from symptoms.

Psychological treatment can occur in a variety of places. An individual might go to a community mental health center or a practitioner in private or community practice. A child might see a school counselor, school psychologist, or school social worker. An incarcerated person might receive group therapy in prison. There are many different types of treatment providers, and licensing requirements vary from state to state. Besides psychologists and psychiatrists, there are clinical social workers, marriage and family therapists, and trained religious personnel who also perform counseling and therapy.

A range of funding sources pay for mental health treatment: health insurance, government, and private pay. In the past, even when people had health insurance, the coverage would not always pay for mental health services. This changed with the Mental Health Parity and Addiction Equity Act of 2008, which requires group health plans and insurers to make sure there is parity of mental health services (U.S. Department of Labor, n.d.). This means that co-pays, total number of visits, and deductibles for mental health and substance use treatment need to be equal to and cannot be more restrictive or harsher than those for physical illnesses and medical/surgical problems.

Finding treatment sources is also not always easy: there may be limited options, especially in rural areas and low-income urban areas; waiting lists; poor quality of care available for indigent patients; and financial obstacles such as co-pays, deductibles, and time off from

work. Over 85% of the l,669 federally designated mental health professional shortage areas are rural; often primary care physicians and law enforcement are the first-line mental health providers (Ivey, Scheffler, & Zazzali, 1998), although they do not have the specialized training of a mental health professional, who often would be better equipped to provide care. Availability, accessibility, and acceptability (the stigma attached to mental illness) are all problems in rural areas. Approximately two-thirds of those with symptoms receive no care at all (U.S. Department of Health and Human Services, 2005; Wagenfeld, Murray, Mohatt, & DeBruiynb, 1994). At the end of 2013, the U.S. Department of Agriculture announced an investment of $50 million to help improve access and treatment for mental health problems as part of the Obama administration's effort to strengthen rural communities.

Summary

It was once believed that people with psychological disorders, or those exhibiting strange behavior, were possessed by demons. These people were forced to take part in exorcisms, were imprisoned, or executed. Later, asylums were built to house the mentally ill, but the patients received little to no treatment, and many of the methods used were cruel. Philippe Pinel and Dorothea Dix argued for more humane treatment of people with psychological disorders. In the mid-1960s, the deinstitutionalization movement gained support and asylums were closed, enabling people with mental illness to return home and receive treatment in their own communities. Some did go to their family homes, but many became homeless due to a lack of resources and support mechanisms.

Today, instead of asylums, there are psychiatric hospitals run by state governments and local community hospitals, with the emphasis on short-term stays. However, most people who have mental illness are not hospitalized. A person suffering symptoms could speak with a primary care physician, who most likely would refer him to someone who specializes in therapy. The person can receive outpatient mental health services from a variety of sources, including psychologists, psychiatrists, marriage and family therapists, school counselors, clinical social workers, and religious personnel. These therapy sessions would be covered through insurance, government funds, or private (self) pay.

Review Questions

Who of the following does not support the humane and improved treatment of people with mental illness?

1. Philippe Pinel

1. medieval priests

1. Dorothea Dix

1. All of the above

The process of closing large asylums and providing for people to stay in the community to be treated locally is known as _____.

1. deinstitutionalization

1. exorcism

1. deactivation

1. decentralization

Joey was convicted of domestic violence. As part of his sentence, the judge has ordered that he attend therapy for anger management. This is considered _____ treatment.

1. involuntary

1. voluntary

1. forced

1. mandatory

Today, most people with mental health issues are not hospitalized. Typically they are only hospitalized if they _____.

1. have schizophrenia

1. have insurance

1. are an imminent threat to themselves or others

1. require therapy

Critical Thinking Questions

People with psychological disorders have been treated poorly throughout history. Describe some efforts to improve treatment, include explanations for the success or lack thereof.

Usually someone is hospitalized only if they are an imminent threat to themselves or others. Describe a situation that might meet these criteria.

Personal Application Questions

Do you think there is a stigma associated with mentally ill persons today? Why or why not?

What are some places in your community that offer mental health services? Would you feel comfortable seeking assistance at one of these facilities? Why or why not?

CHAPTER 02 / CAC.902

The Helping Relationship and Common Factors

The helping relationship is the working climate in which theory becomes usable. A child does not experience a theory. A child experiences a person who listens, a room that feels safe or unsafe, a parent who is included or excluded, and a plan that makes sense or feels imposed. Common factors are the elements that appear across many therapies: alliance, empathy, collaboration, positive regard, hope, and agreement about goals and tasks. They are not vague kindness. They are observable practices that can be strengthened, repaired, and measured.

Alliance has three parts

Alliance is often described as bond, goals, and tasks. Bond is the felt sense of trust and respect. Goals are the shared aims of therapy. Tasks are the agreed activities used to reach those aims. A counselor may have a warm bond with a teenager but no agreement about tasks. The teen enjoys talking but refuses to practice a skill. Another counselor may have clear tasks but a weak bond. The teen completes worksheets to please the adult but does not use the skill when distressed. Strong alliance means the three parts are aligned enough for work to continue.

With children, alliance is multiply layered. The child must feel understood. The caregiver must feel respected and see a credible plan. The school may need a role. The counselor must not confuse compliance with alliance. A quiet child who nods may be frightened. A parent who agrees in session may disagree at home because childcare, work, or cultural beliefs make the plan impossible. Alliance work includes asking, "What would make this plan hard to carry out?"

Empathy is accurate, not merely nice

Empathy means sensing the client's private world as if it were your own, without losing the as-if quality. It is not sympathy, advice, or agreement. A counselor can empathize with a child's anger without agreeing that hitting is acceptable. A counselor can understand a parent's fear without endorsing harsh punishment. Empathy is tested by the client's response. If the counselor says, "You felt left out," and the child says, "No, I felt embarrassed," the correction is data. The counselor should thank the child and adjust.

A simple practice is to reflect content and feeling, then check. "You wanted to sit with your friends, and when they moved away you felt humiliated. Is that close?" The check matters. It gives the client authority over their own experience. In doctoral work, empathy also includes cultural humility. The counselor does not assume that eye contact, emotional expression, or family roles mean the same thing in every community.

Positive regard and congruence

Unconditional positive regard means valuing the person even when behavior is harmful or symptoms are severe. It does not mean approving of every action. A counselor can say, "I care about what happens to you, and I will not let you hurt yourself or others in this room." Congruence means the counselor is genuine and aware of their own experience, not performing a role so rigidly that the client senses falseness. Congruence does not mean saying every thought. It means that what the counselor says and does is not misleading.

Adolescents are often skilled at detecting incongruence. If a counselor says, "I respect your privacy," but immediately reports every minor statement to a parent without explanation, trust breaks. The counselor should explain confidentiality limits at the start and revisit them when needed. Transparency is a common-factor intervention.

Rupture and repair

A rupture is a strain or break in the working relationship. It may be obvious, such as a client yelling, or subtle, such as shorter answers and missed appointments. Ruptures are not failures by themselves. They are opportunities to practice repair. Repair begins with noticing, then naming, then listening, then adjusting. "I think I moved too fast last week. You looked away when I suggested the exposure plan. Did I miss something important?" The counselor does not defend the theory first. The counselor protects the relationship enough to think together.

In child therapy, rupture may appear in play. A child who previously built cities may suddenly destroy them when the counselor announces a vacation. The counselor can wonder aloud whether the break feels like abandonment. The interpretation should be tentative and tied to the child's response. Repair may also require caregiver work. If a parent feels blamed, the child's therapy may stall even when the child likes the counselor.

Measuring the relationship

Feedback tools can help, but they are not a substitute for attention. A short session rating may reveal that a teen felt unheard. A goal-tracking sheet may show that the parent and child want different outcomes. The counselor should ask what the score means. A low score is not an attack. A high score is not proof that therapy is working. The number opens a conversation.

Consider a group for anxious adolescents. Cohesion, safety, and shared goals are common factors in group work. A teen may benefit from hearing that others also fear panic. But if the group becomes a place to compete over who is most impaired, the factor has turned harmful. The counselor must structure the group so that mutual aid is possible. Theory tells the counselor why group work may help; common-factor skills tell the counselor whether this group is helping these members now.

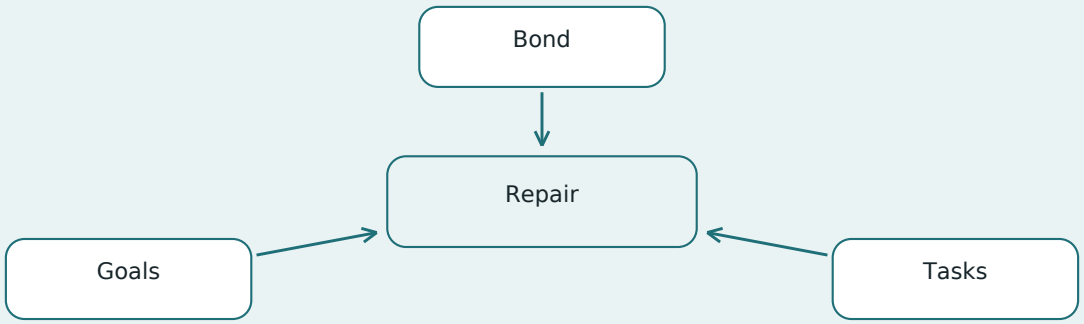
Real-life application

A school counselor meets with a twelve-year-old who stole a phone. The administrator wants an apology. The parent wants punishment. The child says, "Everyone already thinks I'm bad." A common-factor response does not ignore the theft. It begins with positive regard and accurate empathy: "You expect me to see you as a thief, and that makes it hard to talk." Then the counselor works on goals: repair harm, understand the context, and build a plan to avoid repetition. The tasks may include a behavioral chain analysis, a family conversation, and a restitution plan. The relationship makes the tasks possible.

Reflection

Think of a time when someone tried to help you but did not understand your goal. What did you do? Did you argue, comply, or withdraw? Clients do the same. The helping relationship is not a soft preface to real therapy. It is the condition in which real therapy becomes possible.

Three parts of alliance



A working alliance needs trust, shared aims, agreed activities, and repair when strain appears.

Attributed Foundational Reading

Attribution and critical-reading note

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Individual Therapy

In **individual therapy**, also known as individual psychotherapy or individual counseling, the client and clinician meet one-on-one (usually from 45 minutes to 1 hour). These meetings typically occur weekly or every other week, and sessions are conducted in a confidential and caring environment (). The clinician will work with clients to help them explore their feelings, work through life challenges, identify aspects of themselves and their lives that they wish to change, and set goals to help them work towards these changes. A client might see a clinician for only a few sessions, or the client may attend individual therapy sessions for a year or longer. The amount of time spent in therapy depends on the needs of the client as well as her personal goals.

Source figure caption: In an individual therapy session, a client works one-on-one with a trained therapist. (credit: Alan Cleaver)

Group Therapy

In **group therapy**, a clinician meets together with several clients with similar problems (). When children are placed in group therapy, it is particularly important to match clients for age and problems. One benefit of group therapy is that it can help decrease a client's shame and isolation about a problem while offering needed support, both from the therapist and other members of the group (American Psychological Association, 2014). A nine-year-old sexual abuse victim, for example, may feel very embarrassed and ashamed. If he is placed in a group with other sexually abused boys, he will realize that he is not alone. A child struggling with poor social skills would likely benefit from a group with a specific curriculum to foster special skills. A woman suffering from post-partum depression could feel less guilty and more supported by being in a group with similar women.

Group therapy also has some specific limitations. Members of the group may be afraid to speak in front of other people because sharing secrets and problems with complete strangers can be stressful and overwhelming. There may be personality clashes and arguments among group members. There could also be concerns about confidentiality: Someone from the group might share what another participant said to people outside of the group.

Source figure caption: In group therapy, usually 5–10 people meet with a trained therapist to discuss a common issue such as divorce, grief, an eating disorder, substance abuse, or anger management. (credit: Cory Zanker)

Another benefit of group therapy is that members can confront each other about their patterns. For those with some types of problems, such as sexual abusers, group therapy is the recommended treatment. Group treatment for this population is considered to have several benefits:

Group treatment is more economical than individual, couples, or family therapy. Sexual abusers often feel more comfortable admitting and discussing their offenses in a treatment group where others are modeling openness. Clients often accept feedback about their behavior more willingly from other group members than from therapists. Finally, clients can practice social skills in group treatment settings. (McGrath, Cumming, Burchard, Zeoli, & Ellerby, 2009)

Groups that have a strong educational component are called psycho-educational groups. For example, a group for children whose parents have cancer might discuss in depth what cancer is, types of treatment for cancer, and the side effects of treatments, such as hair loss. Often, group therapy sessions with children take place in school. They are led by a school counselor, a school psychologist, or a school social worker. Groups might focus on test anxiety, social isolation, self-esteem, bullying, or school failure (Shechtman, 2002). Whether the group is held in school or in a clinician's office, group therapy has been found to be effective with children facing numerous kinds of challenges (Shechtman, 2002).

During a group session, the entire group could reflect on an individual's problem or difficulties, and others might disclose what they have done in that situation. When a clinician is facilitating a group, the focus is always on making sure that everyone benefits and participates in the group and that no one person is the focus of the entire session. Groups can be organized in various ways: some have an overarching theme or purpose, some are time-limited, some have open membership that allows people to come and go, and some are closed. Some groups are structured with planned activities and goals, while others are unstructured: There is no specific plan, and group members themselves decide how the group will spend its time and on what goals it will focus. This can become a complex and emotionally charged process, but it is also an opportunity for personal growth (Page & Berkow, 1994).

Couples Therapy

Couples therapy involves two people in an intimate relationship who are having difficulties and are trying to resolve them (). The couple may be dating, partnered, engaged, or married. The primary therapeutic orientation used in couples counseling is cognitive-behavioral therapy (Rathus & Sanderson, 1999). Couples meet with a therapist to discuss conflicts and/or aspects of their relationship that they want to change. The therapist helps them see how their individual backgrounds, beliefs, and actions are affecting their relationship. Often, a therapist tries to help the couple resolve these problems, as well as implement strategies that will lead to a healthier and happier relationship, such as how to listen, how to argue, and how to express feelings. However, sometimes, after working with a therapist, a couple will realize that they are too incompatible and will decide to separate. Some couples seek therapy to work out their problems, while others attend therapy to determine whether staying together is the best solution. Counseling couples in a high-conflict and volatile relationship can be difficult. In fact, psychologists Peter Pearson and Ellyn Bader, who founded the Couples Institute in Palo Alto, California, have compared the experience of the clinician in couples' therapy to be like "piloting a helicopter in a hurricane" (Weil, 2012, para. 7).

Source figure caption: In couples counseling, a therapist helps people work on their relationship. (credit: Cory Zanker)

CHAPTER 03 / CAC.902

Psychoanalysis: Freud and the Unconscious

Sigmund Freud's psychoanalysis is one of the most influential and most criticized traditions in counseling. Its influence is not only in technique. It shaped how many people think about childhood, dreams, defense, desire, and the hidden meanings of behavior. A doctoral counselor should know psychoanalysis well enough to use its insights and well enough to recognize its limits. Some Freudian claims are historically important but not supported as universal science. Some clinical ideas, such as defense and transference, remain useful when held as hypotheses rather than dogma.

The unconscious is not a single thing

Freud used the term unconscious in more than one way. Sometimes it refers to mental processes outside awareness. Sometimes it refers to wishes or conflicts that are kept out of awareness because they are threatening. Modern cognitive science also studies processes outside awareness, but that does not prove every Freudian claim. A counselor can accept that people do not always know why they act without accepting a fixed psychosexual stage theory. Precision matters.

A child who says, "I hate my brother," after a new baby arrives may not be expressing a simple wish. The child may fear losing love, feel guilty for anger, and then act more aggressively to prove that love is still available. A psychodynamic formulation asks what the behavior means in the child's emotional world. It does not stop at the label sibling rivalry.

Structure and defense

Freud's structural model describes id, ego, and superego. The id represents drive and impulse, the ego mediates reality, and the superego represents internalized standards. These are not brain regions. They are theoretical constructs. Defense mechanisms are ways the ego manages anxiety and conflict. Repression pushes material out of awareness. Denial refuses a painful reality. Projection attributes one's own feeling to another. Reaction formation turns a feeling into its opposite. Sublimation channels impulse into socially valued activity.

In practice, defense language should be used gently. A teenager who jokes about a parent's illness may be using humor to manage terror. The counselor does not say, "You are in denial." The counselor might say, "Jokes seem to help you get through the moment. I wonder if there are moments when the fear comes through anyway." The goal is not to strip defenses away. The goal is to widen the client's capacity to feel and think.

Psychosexual stages and their limits

Freud proposed that development moves through oral, anal, phallic, latency, and genital stages. Each stage centers on a zone of pleasure and a developmental conflict. Fixation was thought to shape adult personality. This model is historically important, but it should not be used as a literal map of all children. It is culture-bound, gender-bound, and difficult to test in its original form. Contemporary child counseling uses developmental science, attachment research, and trauma research more often than literal psychosexual stages.

Still, the stage theory asks a useful question: how do early experiences of dependency, control, and bodily shame become part of a person's expectations? A child who was shamed harshly during toilet training may not have an anal fixation, but the child may carry shame about bodily control. The counselor can study the meaning without accepting the whole stage theory.

Free association, dreams, and interpretation

Classical psychoanalysis uses free association: the client says what comes to mind without censoring. The analyst listens for patterns, slips, and connections. Dreams are explored as possible expressions of wish, fear, and conflict. Interpretation offers a hypothesis about unconscious meaning. Transference is the redirection of feelings and expectations from earlier relationships onto the analyst. Countertransference is the therapist's emotional response to the client.

With children, free association may appear in play. A child moves from a dinosaur to a baby to a locked door. The counselor does not force a symbolic dictionary. The counselor follows the affect and the sequence. "The baby keeps getting left outside. I wonder what happens next." If the child corrects the counselor, the correction is part of the work. Interpretation is not a clever statement. It is a shared discovery that the client can use.

A clinical example

Thirteen-year-old Daniel is referred for oppositional behavior. He mocks the counselor, calls therapy stupid, and arrives late. A purely behavioral plan may miss the meaning of the mockery. Daniel's father left suddenly. Daniel learned that needing someone leads to humiliation. In session, he attacks the counselor before the counselor can disappoint him. The counselor feels useless and angry. That countertransference is information, not a verdict.

A psychodynamic response might be: "It seems important to make sure I don't get too important to you. If I matter, then I could let you down." Daniel may reject the statement. The counselor waits. Over time, Daniel tests whether the counselor survives his anger without retaliation. The therapy does not ignore behavior limits. It links the limit to the relationship: "I won't let you break the lamp, and I'm still here to understand what just happened."

Evidence and criticism

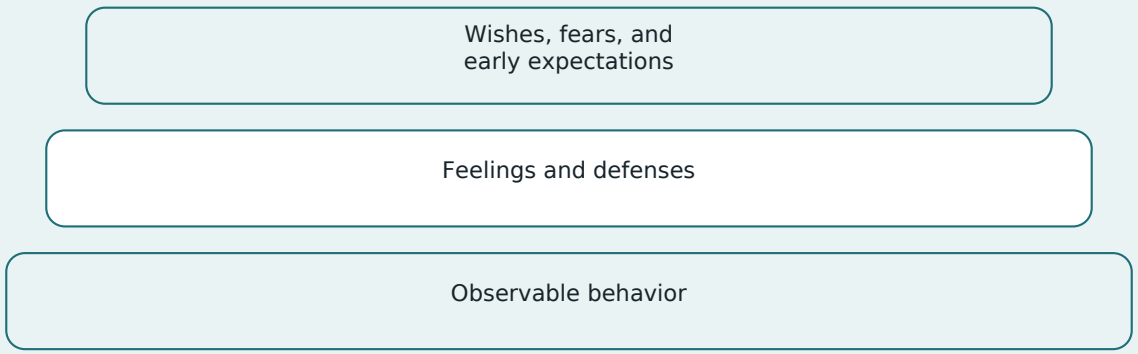
Psychoanalytic therapy has a research base, especially for certain adult conditions and for short-term psychodynamic approaches. The evidence is not uniform across all problems or all versions of the theory. Critics note that classical concepts can be unfalsifiable, that treatment can be long and costly, and that early formulations often reflected the biases of their time. A counselor should not present psychoanalysis as the only deep therapy. Cognitive, behavioral, systemic, and humanistic therapies can also address meaning and pattern.

For children and adolescents, psychodynamic work is often adapted. Play therapy, time-limited psychodynamic therapy, and mentalization-based approaches may be more practical than classical analysis. The counselor must consider development, family context, school demands, and safety. A long interpretive silence may be experienced by a child as abandonment rather than neutrality.

Reflection

Choose a behavior that puzzles you in a young client. Write one behavioral hypothesis, one cognitive hypothesis, and one psychodynamic hypothesis. Do not choose a winner yet. Ask what information would help you tell them apart. Psychoanalysis earns its place when it generates questions that can be checked in the relationship and in the client's life.

From surface behavior to hidden meaning



Psychodynamic work moves from what is seen to what may be meant, always as a hypothesis.

Attributed Foundational Reading

Attribution and critical-reading note

Adapted from Personality Theory in a Cultural Context by Mark D. Kelland, OpenStax CNX / Lansing Community College (2015), CC BY 4.0. Selection, organization, and PDF layout modified for this course; new lectures and course diagrams are separate additions.

Read historical and theoretical claims critically. A theory is a lens for inquiry, not a verdict about a child or family. Where source claims are dated or culture-bound, the new lecture supplies cautions and contemporary framing.

Source module m58019 • Personality Theory in a Cultural Context • pinned GitHub snapshot a52283699509. Full repository and license details appear in the attribution register.

Sigmund Freud is unquestionably the most famous person in the fields of psychiatry and psychology, and one of the most famous individuals in modern history. He is of particular importance for this subject because he was probably the first person to address psychological problems by examining the individual’s personal development in detail. As he developed his psychodynamic theory, and the treatment known as psychoanalysis, he attempted to carefully observe and listen to his patients in order to determine not only how and why they had become the person they were, but also whether those developmental processes might be common to all people. This careful approach to studying psychological conditions was likely the result of Freud’s substantial scientific research in anatomy and physiology earlier in his career.

But why is Freud so famous? Much of his theory may not seem relevant today, and it’s hard to imagine how anyone could ever have come up with the theory of penis envy. And yet

Freud remains extraordinarily influential. There are at least three good reasons for Freud's enduring influence and popularity. First, Freud was first! No one before him had established a cohesive theory of the development of personality, especially a theory that attempted to explain both normal and abnormal development. Thus, most theories developed since then have been viewed as extending, modifying, or opposing Freud's psychodynamic theory. Second, key elements of Freud's theory are generally accepted in psychology and psychiatry, such as the existence of unconscious elements of our mind that can affect our thoughts and behaviors and both the normal and abnormal roles of psychological defense mechanisms. The final factor contributing to Freud's lasting influence is somewhat more complicated. Psychodynamic theory was not well received at first. In fact, the emphasis on childhood sexuality was ridiculed and scorned by many in the medical profession. However, Freud was determined, and he did not let the rejection of others deter him from continuing his studies. In addition, there were several very famous and influential individuals who supported his efforts. Thus, Freud found the motivation to persevere, and the rest, as they say, is history.

A Brief Biography of Sigmund Freud, M.D.

Sigismund Schlomo Freud was born on May 6th, 1856, in the small, industrial town of Freiberg in Moravia (today it is known as Pribor in the Czech Republic). Freud never used the name Schlomo, his paternal grandfather's name, and he shortened his first name while at the University of Vienna. His family life was unusual, and somewhat complicated. His father, Jakob Freud, was 40 years old when he married Freud's mother, Amalia Nathanson. She was 20 years younger than Jakob Freud, and several years younger than Jakob's son, Emanuel, from an earlier marriage. One of Freud's first friends was a nephew who was a year older than Freud!

Jakob Freud was never particularly successful in business. The industrial importance of Freiberg was declining, so the young family left and eventually settled in Vienna, Austria (Jakob's sons from his first marriage, Emanuel and Philipp, emigrated to England). At this point Jakob and Amalia Freud had two children, Sigmund and his sister Anna (a brother born between them, Julius, died at 7 or 8 months of age). Shortly after arriving in Vienna, however, they had five more children during the years 1860-1866: Rosa, Marie, Adolfine, Pauline, and Alexander. This resulted in continued financial difficulties, which appears to have been painful for the young Freud (Gay, 1998). There were also personal difficulties that made it difficult for Freud to enjoy a close relationship with his father. Jakob Freud once told his son a story about being abused by an Austrian Christian, a man who knocked Jakob Freud's hat into the muddy street and then ordered the "Jew" to get off the sidewalk. When Freud asked his father how he had responded, his father said he simply stepped off the sidewalk and picked up his hat. Freud was very disappointed by what he apparently perceived as weakness in his father (Gay, 1998). There was also an embarrassing episode involving his father's brother, Josef. Josef Freud was convicted and sent to jail for trading in counterfeit money. This caused a great deal of concern for Jakob Freud, who might have been involved in the illegal scheme along with his sons, Emanuel and Philipp (Gay, 1998; Jones, 1953).

Still, Jakob Freud did try to be a good father. His children were generally successful, and he remained active and supportive in the lives of his children and grandchildren. The story mentioned above, when Jakob Freud tried to impart some "fatherly" wisdom to his son, may

not have had the intended effect, but it demonstrates that he cared about teaching his son some of life's lessons. On Freud's thirty-fifth birthday his father sent his "dear son" a copy of the family's Philippson Bible (this Bible contains the Old Testament, which is the only testament in the Jewish faith), which Freud had often studied as a young child (Gay, 1998; Jones, 1953; Nicholi, 2002). The inscription written by Jakob Freud in the Bible ended with a description of the gift "as a token of love from your old father" (see Jones, 1953; Nicholi, 2002). When Jakob Freud died, Freud wrote to a friend that his father's death had profoundly affected him, leaving him feeling uprooted. He described the death of one's father as "the most important event, the most poignant loss, in a man's life" (see Nicholi, 2002). The death of his father appears to have stimulated Freud's self-analysis, the writing of The Interpretation of Dreams (Freud, 1900/1995) and the formulation of his theory of the Oedipus complex (Nicholi, 2002). Jakob Freud was also remembered quite fondly by his grandson Martin, the eldest son of Freud (M. Freud, 1983).

Freud's relationship with his mother was also complex. Amalia Freud is described as young (which she was, compared to Jakob), attractive, and energetic. She always took great pride in her son, and was a strong and positive influence throughout his life. Later in life he wrote that "A man who has been the indisputable favorite of his mother keeps for life the feeling of a conqueror, that confidence of success that often induces real success" (see Jones, 1953). During Freud's self-analysis, around the year 1897, he uncovered profound memories from his earliest years. Sometime between the ages of 2 ½ and 4 years old, Freud accidentally saw his mother naked. This event awakened a powerful desire in Freud. Shortly after recovering this memory, he remembered the deep jealousy he had felt when his brother Julius was born, shortly before Freud was 2 years old. So jealous was Freud, that he remembered welcoming the death of his infant brother (see Gay, 1998; Jones, 1953). Each of these incidents certainly had an impact of Freud's theory of the Oedipus complex. Surprisingly, however, during the first 2 ½ years of Freud's life he actually spent very little time with his mother, since he was being raised by a nursemaid. Keep in mind that his mother became pregnant again, then his brother Julius became ill and died, and then his mother became pregnant again, finally giving birth to his sister Anna, all by the time Freud was 2 ½ years old.

Freud's nursemaid has been described as an old and ugly woman, but Freud loved her and dreamed about her later in life (see Gay, 1998; Jones, 1953; Nicholi, 2002). The nursemaid was a devout Roman Catholic, and she regularly took Freud to church with her. Despite his young age (less than 2 ½ years old), Freud would come home from church and preach to his family about God. Even though his family was Jewish, they did not practice their faith with much devotion, and it must have been quite interesting to listen to the sermons of their little boy. Why then, as we will see, did Freud come to reject religion and spirituality? It turns out that this relationship ended abruptly. Freud's half brother Philipp accused the nursemaid of petty theft, and she was sent to prison. At this time Freud's mother was confined with his recently born sister, so Freud was suddenly denied access to both his mother and his nursemaid. It has been suggested that because he was abandoned so suddenly, and at such a critical time (Freud was 2 ½ years old at this time), by his Roman Catholic nursemaid, that his anger and disappointment led to his ultimate rejection of the spiritual worldview and his antagonism toward the Catholic church (Gay, 1998; Jones, 1953; Nicholi, 2002).

Freud's Early Career in Basic Research (Pre-Psychiatry)

Freud was very successful in school from an early age. At the Gymnasium, which is the term for a preparatory school in countries such as Germany and Austria, he was first in his class for 7 years. This led to a variety of special privileges, including seldom being required to take any examinations (Freud, 1952). It also led to privileges at home. According to his sister Anna, Freud always had his own room to study in, no matter how difficult the family's financial situation (Gay, 1998). As he prepared for college, Freud initially wanted to study law. However, after learning about Darwin's theory of evolution and hearing Goethe's essay on nature, he decided to become a medical student (Freud, 1952).

In 1873, Freud entered the University of Vienna. Initially he suffered greatly from prejudice and discrimination against him because he was Jewish. Believing that he was expected to feel inferior and alien because he was Jewish, he nonetheless persevered. As a result of these experiences, later in life he was prepared for dealing with the considerable resistance that occurred in response to his theories (Freud, 1952). His first research project in medical school came at the suggestion of Professor Carl Claus. Prof. Claus was interested in a report that the Polish scientist Simone de Syrski had identified structures that might represent the testes of the male eel. This was a question that had been studied for centuries without success. After dissecting some 400 eels, Freud appeared to have confirmed Syrski's findings. The research was not definitive, however, and Freud found little satisfaction in the publication of his work (Gay, 1998; Jones, 1953). He was, however, about to find satisfaction, in the physiological laboratory of Ernst Brucke.

Brucke was a renowned physiologist, anatomist, histologist, and more. Freud had great respect for his newfound mentor, referring to him as Master Brucke and describing him as "the greatest authority I ever met." In Brucke's laboratory Freud "found rest and full satisfaction at last" (Gay, 1998; Jones, 1953). The research he conducted under Brucke's guidance was impressive. Brucke put Freud to work studying the anatomy of the spinal cord and its neurons. At that time, the structure of neurons was not understood. Freud modified the histological staining methods being used in Brucke's laboratory, and eventually developed a gold chloride method of staining nervous system tissue around the year 1880 (Jones, 1953). This was one of the first uses of a heavy metal stain on nervous system tissue. The silver nitrate method of staining neurons had been developed by Camillo Golgi a few years earlier, in 1873, but it was not until 1888 that Santiago Ramon y Cajal first reported on the structure of the brain using Golgi's technique. For this research, Golgi and Ramon y Cajal shared the Nobel Prize for Medicine in 1906 (Finger, 1994). If Freud had not left basic research for a career in medicine, he might have ended up famous just the same.

Freud did eventually leave the university, however, and began a career in medicine at the General Hospital in Vienna. Part of the reason for leaving and beginning his medical career was that he had met Martha Bernays, the woman who would become his wife, and he needed to begin earning enough money to support a wife and family. First, however, he needed to establish himself in his career. At the General Hospital he met and worked with the eminent Theodor Meynert, who, among other accomplishments, was the first to correctly suggest that Parkinson's disease resulted from abnormal functioning of the basal ganglia (Finger, 1994). This stimulated Freud's continued interest in anatomy and brain function, and in 1891 Freud published a book entitled On Aphasia. You may remember from introductory psychology that the two primary speech centers in the human brain are Broca's area (speech production) and Wernicke's area (speech reception), and that damage to these areas results in Broca's aphasia or Wernicke's aphasia. Carl Wernicke had also been a student of Meynert,

but Freud's book on aphasia was especially critical of Wernicke (Finger, 1994). This put both men firmly in the middle of the debate on structuralism vs. functionalism as it pertains to the activities of the human brain (see Finger, 1994). Although Meynert suggested that Freud should devote himself to studying the anatomy of the brain, Freud had had enough of this sort of work in Brucke's laboratory. Instead, Freud's interest turned toward the diseases of the brain (Freud, 1952). With the help of a recommendation by Brucke, Freud was awarded a Traveling Fellowship, which allowed him to afford a trip to Paris to study at the prestigious Salpetriere. He intended to study under Jean-Martin Charcot, one of the world's foremost neurologists of his day, and the man who named Parkinson's disease after the physician James Parkinson (Finger, 1994).

Freud was largely ignored when he arrived at the Salpetriere, since he was just one of a crowd of foreign visitors. As luck would have it, one day he heard Charcot expressing regret that Charcot had not heard from his German translator in some time, and he wished someone could be found to translate his latest lectures into German. Freud wrote to Charcot, offered to do the job, and was accepted. From that point on he became a member of Charcot's inner circle, and was active in all aspects of the work at the clinic (Freud, 1952). One of the main topics Freud studied with Charcot was the use of hypnosis in the study of hysteria. Freud discussed the earliest conceptions of his psychodynamic theory with Charcot. Charcot was supportive and agreed with Freud's fledgling ideas, but Charcot's interests remained firmly in the field of neurology, not in psychology or psychiatry (Freud, 1952).

Upon returning to Vienna and settling down as a practicing physician, Freud was finally able to marry Martha in 1886 (he was 30 years old, and she was 25). They had six children: Matilde, Martin, Oliver, Ernst, Sophie, and Anna. According to his son Martin, Freud was a loving and generous father (M. Freud, 1983). He was also very supportive of his children. As Martin became disillusioned with the study of law, he turned to his father for advice:

…It had always been his hope that one of his sons would become a lawyer. Thus he watched, and I think guided, my first faltering steps in my law studies with the greatest concern.

He agreed that my first studies were dull and boring, but he assured me that one day I would find a teacher with an impressive personality, perhaps a man of genius, and that I would become deeply interested and carried away by his lectures…

Father always expressed himself with great clarity and, when advising me at so critical a time in my life, he added to his normal clarity of expression a natural tenderness and concern… (M. Freud, 1983; pg. 161)

Martin did become a lawyer and, after Martin served as an officer in the Austrian army during World War I, his father helped him to establish his practice.

Finally, any discussion of Freud's early research career would not be complete without mentioning what Ernest Jones, Freud's official biographer, called "the cocaine episode" (Jones, 1953). In his last autobiographical book (Freud, 1952; originally published in 1925 in a collection of medical autobiographies) he makes only passing reference to studying cocaine, reporting another near miss in his research career. He had begun studying cocaine while he was away from Vienna, and an opportunity arose to return home for vacation and an opportunity to see his fiancé Martha. As he prepared to leave, he suggested to a couple of colleagues that they examine the effectiveness of cocaine as an anesthetic for use in eye

diseases. While Freud was visiting Martha, one of his colleagues, Carl Koller, confirmed the local anesthetic properties of cocaine and became famous for it. Afterward, Freud noted that “it was the fault of my fiancé that I was not already famous at that early age,” but he insists that “I bore my fiancé no grudge for her interruption of my work” (Freud, 1952). Something quite fascinating is that Freud’s interest in cocaine was initially based on the possibility that its euphoric properties might be used to alleviate the problems associated with withdrawal from morphine. A close and long-time friend, Ernst von Fleischl-Marxow had become addicted to morphine because of the extreme pain of an infection, and Freud hoped that cocaine would help. Freud himself began using cocaine to boost his own mood. He sent some to Martha with the recommendation that she try it (there is no evidence that she ever did), and he even began sending cocaine to friends, colleagues, and his sisters. Eventually, however, Freud realized that cocaine was not helping his friend; indeed von Fleischl-Marxow became addicted to cocaine instead of morphine. Freud eventually deeply regretted his research on cocaine, especially since the one positive result of that research had garnered fame for a colleague while Freud was on vacation (Gay, 1998; Jones, 1953).

Freud’s Psychiatric Career

Many people believe that psychoanalysis was developed by Freud during the early years of his medical practice in Vienna. Freud, however, would disagree. He insisted that psychoanalysis was begun by the Viennese physician Josef Breuer (Freud, 1914/1995), a close friend and mentor of Freud. The basis of psychoanalysis lay in a patient that Breuer had seen as early as 1880, and had treated with hypnosis. This case, and the use of hypnosis as part of the “cathartic procedure” developed by Breuer, was the original inspiration for Freud’s interest in hypnosis and his trip to Paris to study the technique with Charcot. When Freud returned to Vienna, he asked Breuer to tell him all of the details of this case, which involved a young woman. This famous patient, known as Anna O., was described by Breuer in the book coauthored by the two men (Freud & Breuer, 1895/2004). As Freud used Breuer’s techniques with his own patients, however, he began to realize that something was lacking. Hypnosis did little to reveal the underlying causes of the hysteria that their patients were experiencing. Since Freud was every bit the scientist, he needed to know more about why he was able to help some patients. He eventually replaced hypnosis with his own techniques of free association (early 1890s) and, eventually, dream analysis (essentially done in 1896, but not published until 1900). This was the point at which psychoanalysis, in the sense that we think of it today, was born (Freud, 1914/1995, 1952).

As Freud’s ideas diverged from those of Breuer, the two parted ways. Freud then developed the aspect of psychodynamic theory that led to his near total rejection by the German and Austrian medical communities: the primacy of childhood sexuality. This theory was so difficult for others to accept that Freud spent nearly 10 years working on psychoanalysis in isolation. However, Freud claims that the concept of a sexual etiology for the neuroses was not really his idea, it had been superficially suggested by Breuer, Charcot, and a highly respected Viennese gynecologist named Chrobak (Freud, 1914/1995). During those years of isolation Freud began to define other major aspects of psychodynamic theory, such as: resistance, repression, conflict, and unconscious impulses.

Around 1902, Freud began to find support for his theories among a select group of physicians. Shortly thereafter a group of psychiatrists in Zurich, Switzerland, which included Eugen Bleuler (the man credited with identifying both schizophrenia and autism as we

define them today) and his assistant Carl Jung, began “taking a lively interest in psychoanalysis” (Freud, 1952). In 1909 Freud and Jung were invited to America, where they were warmly received, and psychoanalysis became well-established in America and Canada. By the 1910s it was reported that psychoanalysis was being championed in Austria, Switzerland, the United States, Canada, England, India, Chile, Australasia (the region), France, Italy, Sweden, Russia, Hungary, Holland, and Norway (where the first textbook on psychiatry that included psychoanalysis was written) (Freud, 1914/1995). Germany proved quite resistant, although the renowned Karl Abraham practiced psychoanalysis in Berlin.

Perhaps it was inevitable that all of this success should eventually lead to conflict. Two major groups, whose members differed significantly in their views on psychodynamic theory and psychoanalysis, broke away from the main psychoanalytic groups. They were led by Alfred Adler (see Chapter 4) and Carl Jung (Chapter 3). In his first autobiography, Freud is not exactly kind to these two men. He goes to great length to dismiss Adler’s theories as mistaken, and he flatly rejects Jung’s perspective:

Of the two movements under consideration here, Adler’s is undoubtedly the more important. Though radically false, it is, nevertheless, characterized by consistency and coherence, and it is still founded on the theory of the instincts. On the other hand, Jung’s modification has slackened the connection between the phenomena and the instinctive-life; besides as its critics (Abraham, Firenze, and Jones) have already pointed out, it is so unintelligible, muddled and confused, that … it is impossible to know how one can arrive at a correct understanding of it... (Freud, 1914/1995; pg. 940).

It is curious to speculate whether Freud’s isolation for so many years may have led to the profound possessiveness he later expressed regarding psychoanalysis as his technique, and his alone, in The History of the Psychoanalytic Movement:

...For psychoanalysis is my creation; for ten years I was the only one occupied with it, and all the annoyance which this new subject caused among my contemporaries has been hurled upon my head in the form of criticism. Even today, when I am no longer the only psychoanalyst, I feel myself justified in assuming that nobody knows better than I what psychoanalysis is… (Freud, 1914/1995; pg. 901)

Freud’s Final Years

Freud’s final years were somewhat tumultuous. The Nazis had taken over Germany and Austria, and they were rapidly preparing for World War II. Being Jewish, Freud’s life was in danger; indeed, at least three of his sisters were murdered in the concentration camps, most likely in Auschwitz (M. Freud, 1983). Freud, however, had influential friends, including European royalty and wealthy individuals with ties to the British and American governments. The American secretary of state, Cordell Hull, took word of the situation to President Franklin Roosevelt, and following Roosevelt’s instructions, Hull had the American ambassador to Germany intervene on Freud’s behalf (Gay, 1998; Jones, 1957). Freud also received substantial help and comfort from Marie Bonaparte, H.R.H. the Princess George of Greece, including the payment of a ransom in order to secure permission for Freud to leave Austria (M. Freud, 1983). Finally, in May 1938, Freud, his wife Martha, and their daughter Anna left together for England, along with Freud’s dog. They were all received quite warmly in London, except for the dog. She was quarantined for six months (M. Freud, 1983).

Freud, however, had already been ill for many years, and was suffering a great deal of pain due to cancer. He was also in his eighties. Nonetheless, Freud continued to work, and he completed An Outline of Psychoanalysis (1938/1949) and Moses and Monotheism (1939/1967) while living in London. But the end was near, and the cancer was progressing rapidly. In September 1939, Freud asked his doctor, Max Schur, to remember an agreement the two had made not to prolong Freud's life unnecessarily. Freud asked Schur to discuss his condition with Anna Freud. Anna Freud at first resisted, but eventually submitted to the inevitable, and Schur administered a series of morphine injections that proved fatal. Sigmund Freud died on September 23, 1939 (Gay, 1998; Jones, 1957).

In the funeral oration delivered by Ernest Jones, Jones remembered that three qualities had particularly impressed him upon first meeting Freud: first, "his nobility of character;" second, "his direct and instinctive love of truth;" and third, "his courage and inflexible determination." Jones also said that a "great spirit has passed from the world…for Freud so inspired us with his personality, his character and his ideas that we can never truly part from him…" (Jones, 1957). Prior to the escape from Austria, Freud had expressed a sincere desire to "die in freedom." He loved England, where he was able to accomplish that goal. In reference to England and the funeral ceremony, Jones said:

He died surrounded by every loving care, in a land that had shown him more courtesy, more esteem and more honor than his own or any other land, a land which I think he himself esteemed beyond all others. (pg. 247; Jones, 1957).

Placing Freud in Context: Connecting Personality Theories

Sigmund Freud was one of the greatest minds of modern times. He was the first person to provide a comprehensive theory of personality and personality development, and he did so in what he considered to be a logical and scientific manner. Since he was first, however, how can we place him amongst the other great psychologists? As I contemplated the importance of Freud to the history of psychology, I looked back at my own graduate school training. The textbook assigned for my graduate history of psychology course was Theories and Systems of Psychologyby Robert Lundin (1979). In the chapter titles, Lundin mentions only three psychologists by name: Wilhelm Wundt, the founder of experimental psychology; William James, America's preeminent psychologist; and Sigmund Freud. Since Freud's name is also mentioned in the title of the chapter devoted to his followers, Freud actually has two chapters devoted to his influence. I also looked at A History of Psychological Theories by Ross Stagner (1988), who was an esteemed faculty member in the psychology department at Wayne State University in Detroit and author of one of the first personality textbooks (Stagner, 1937). Stagner mentions six individuals in the titles of his twenty-two chapters, and once again Freud is among them. In addition, the well-known psychological historian Ludy Benjamin includes a chapter on the correspondence between Freud and Jung in A History of Psychology in Letters(1993). There are actually numerous books published on the correspondence between Freud and a variety of other people, and thousands of those letters have been published. These are just a few examples of how deeply Freud is recognized as a major figure in the history of psychology.

Another testament to the legacy of Freud is how enduring some of the issues he addressed have proven to be. In the early years of the twenty-first century there has been a growing conflict between religion and society. In the United States the concept of separation of church and state has been challenged perhaps most aggressively in our schools, with issues

such as praying at school sporting events and the teaching of creationism in science classes. In other countries, religious fundamentalists often stand in opposition to the establishment of democratic governments. Increasing globalization does not seem to be bringing people together, but rather bringing people into competition and conflict. Freud used the knowledge he had learned in his studies on psychoanalysis to address such major societal issues. He presented his ideas in books such as The Future of an Illusion (1927/1961) and Civilization and its Discontents (1930/1961), and he hoped that by advancing our knowledge of the human psyche we could help to continue the development of the human species and civilization. The recognition that problems like these still plague humanity suggests that we have a long way to go. But brilliant men like Sigmund Freud have helped to provide us with a basis for moving forward.

Basic Concepts

It is not easy to read the earliest writings of Freud on psychoanalysis. Following his years of working in isolation, Freud published four books in a span of 5 years: The Interpretation of Dreams (1900/1995), Psychopathology of Everyday Life (1904/1995), Three Contributions to the Theory of Sex (1905/1995), and Wit and Its Relation to the Unconscious(1905/1995). Each of these books clearly reflects their author: a genius, educated in Europe, and writing in a style well suited to the late 1800s/early 1900s. Not only are these books intellectually challenging, but even the English translations are sprinkled with lines in German, French, and Latin. In 1917, however, Freud published a series of lectures he had given at the University of Vienna during the years 1915-1917. His Introductory Lectures on Psycho-Analysis (1917/1966) describes the essential aspects of his theory in neatly organized lectures that are much easier to grasp than his earlier work. Shortly before he died, Freud presented a very brief outline of his theories in the aptly named An Outline of Psycho-Analysis (1938/1949). In what seems to be a logical approach to the study of Freud’s work, we will begin with the general theory and then address the psychoanalytic method. Keep in mind, however, that Freud actually worked the other way around: first he developed his modifications of Breuer’s cathartic method and began treating patients (actually, treating patients contributed to his development of the methods), and then he developed his theoretical perspectives in order to explain what had already proved successful.

Hysteria and Psychic Determinism

The term **hysteria** generally refers to a condition in which psychological trauma or stress is converted into physical symptoms and/or excessive emotional behavior. Today, this condition is typically referred to as a **conversion disorder** (DSM-IV-TR; American Psychiatric Society, 2000). However, Freud meant to use the term in a rather broad sense, and he applied it to a collection of disorders that are not officially recognized today: the **neuroses** (relatively mild mental illnesses, often associated with stress, but which do not result in a loss of contact with reality).

Freud and Breuer (1895/2004) believed that their clinical observations revealed a number of key elements that provided the early framework for **psychodynamic theory** and **psychoanalysis**. In each case, the symptoms exhibited by their patients were connected to some earlier psychological trauma. This connection was not always obvious, however, and often could not be remembered by the patient. When the patient was helped to remember the traumatic event, the symptoms were typically relieved, a process known as **catharsis**. In order to help patients remember, Breuer and Freud (as well as Charcot and a few others)

relied primarily on hypnosis. What intrigued Freud and Breuer was the observation that these traumatic memories seemed to last for a very long time without getting weaker, even though they were not conscious memories. What seemed to matter most was whether there had been an energetic reaction to the emotional event when the memory was formed. In order for the trauma to be released, there needed to be a cathartic event strong enough to adequately dissipate the energy associated with the formation of the traumatic memory.

Both Freud and Breuer recognized that this was only the beginning of this new field of clinical research. Although they were somewhat satisfied that they had described the nature of hysterical symptoms, and that they had moved further than Charcot, they recognized that they were no closer to understanding the internal causes of hysteria and the neuroses. This would become the work of Freud alone, at least for a number of years.

The concept of **psychic determinism** arises naturally from these early observations. Freud believed that all behavior and thought is the result of psychological connections created during previous experiences, nothing happens by accident or chance. The fact that we might find it difficult to recognize the connections between some emotion or behavior and a previous incident does nothing to minimize the reality of those connections, it just presents a challenge for the psychoanalyst. In Psychopathology of Everyday Life, Freud (1904/1995) described how psychic determinism results in many common problems, certainly the most famous of which is the "**Freudian slip**." A Freudian slip is an instance where someone says something wrong, but it actually reflects the persons true feelings. Freud attributed the following example to Dr. Brill:

While writing a prescription for a woman who was especially weighed down by the financial burden of the treatment, I was interested to hear her say suddenly: "Please do not give me big bills, because I cannot swallow them." Of course, she meant to say pills. (italics in the original, pg. 50; Freud, 1904/1995)

Discussion Question: Consider psychic determinism and what it means for your own life. Do you believe that everything you think and do is predetermined by earlier experiences? And what would that mean for your ability to change and grow?

Freud's Theory of Instincts

Freud used the term instinct in a way that does not fit with the technical term instinct as defined by Tinbergen (see Beck, 1978). It has been suggested that the German word trieb should not have been translated as instinct, and actually referred to something more like a drive or impulse. Freud was not concerned with specific behaviors, but rather with general categories of behavior. As a former scientist, Freud never left his interests in biology behind. When Freud referred to the psyche, or mind, he considered both its physical elements, the brain and the rest of the nervous system, and its mental elements, primarily our consciousness (which is made possible by the structure and function of the brain). Given our basic biological nature, and our genetic make-up, we inherit basic instincts essential to our survival: both our individual survival and the survival of our species. In recognition of the general rule in nature that all systems are comprised of opposing forces (attraction and repulsion) Freud hypothesized a life instinct and a death instinct.

Freud gave the life instinct the name **Eros**. Each organism has available to it energy to act within its environment. The energy associated with Eros is called **libido**. Libido has been mistakenly associated with the concept of a sexual impulse. What Freud was really referring

to was a general survival impulse, both individual and species survival. While it is true that the survival of our species depends on sexual reproduction, there are many aspects of our behavior that are not directly related to sex. For example, we might have many friends, but our sexual interests are typically limited to only a few (it is our culture that encourages us to limit our interests to only one person). From an evolutionary perspective, of course, friends and others within our social group helped to protect us from predators and enemies. Similarly, the love and care we provide for our children are essential to the survival of our species, but are not usually associated with sexual acts. Incest appears to be one of the most common cultural taboos, and Freud found this to be a fascinating observation amongst primitive societies, which could not be expected to know anything of Western ideas of morality (Freud, 1913/1995). So it becomes apparent that the impulse to survive, Eros and its associated libido, involves many types of behavior, of which sexual intimacy is just one.

Libido is limited. We have only so much energy to devote to the many aspects and responsibilities of our lives. **Cathexis**refers to the attachment of libidinal energy to some psychical phenomenon. This is what Freud and Breuer meant by an energetic reaction to some experience. When we are attracted to someone, we connect some of our libidinal energy to that relationship. That energy is no longer available to us for other relationships, or to deal with the daily stress of our lives. If we have previously connected libidinal energy to some traumatic event, which might require a great deal of libidinal energy, it may prove difficult to maintain the level of energy we desire for our new relationship. As a result, that relationship, indeed all of our relationships, may suffer.

Although the libido is limited, it has the important characteristic of mobility (Freud, 1938/1949). In other words, it can switch from one task to another as necessary. At least, that is how it is supposed to work under normal conditions. Sometimes, however, problems arise, such as the failure to satisfy the needs that occur during a particular **psychosexual stage of development** (see below). When this occurs, the libido can become **fixated**on particular psychological objects. These fixations can last a lifetime, interfering with continued normal development and the individual’s ability to live a healthy adult life.

Freud also proposed a destructive instinct, which is sometimes referred to as the death instinct. The energy associated with the death instinct is aggressive, but Freud never gave names to either the death instinct or its associated aggressive energy. This was never an important aspect of Freud’s theories, but he did address it in some detail in the book Beyond the Pleasure Principle (Freud, 1920/1961). In this book, Freud makes one thing very clear: the life instinct is far more influential than the death instinct. The primary role of the death instinct is protective. This may sound strange, but he considered the developing organism, even well before birth, as a fragile being assailed on all sides by threatening stimuli (both external stimuli and internal psychical stimuli). The death instinct creates a shell of inert tissue (figuratively, if not also literally), which protects the developing organism from harm.

Although Freud did not include the death instinct among his major concepts, other psychologists have. The **neo-Freudian** theorist Melanie Klein found evidence of the death instinct in the aggressive fantasies of children, and Wilhelm Reich’s concept of armoring is reminiscent of Freud’s description of the theoretical shell protecting the developing organism. Regarding aggression itself, there are many different forms, including predatory aggression, self-defense, defense of one’s young, learned aggression, etc. The noted animal behaviorist and Nobel Laureate Konrad Lorenz wrote extensively on aggression, and he

proposed a very Freudian perspective in which instinctive aggressive energy builds up and lashes out, unless an opportunity for catharsis arises first (see Beck, 1978).

At first it might seem strange that Freud suggested the role of the death instinct is to create a protective shell around the core of the developing nervous system, but the important question is whether we can find any evidence of it. Daniel Goleman, in Emotional Intelligence (1995), suggests a similar theory. The primitive role of emotion is evident in the brain regions devoted to emotion, which are common to many species other than humans. As the mammalian brain evolved, structures were added to the reptilian brain, culminating in the neocortex of the cerebral hemispheres. The cerebral hemispheres are necessary for the cognitive functions that are characteristic of humans. Still, we retain the emotional structures that developed first, and our rational thoughts can easily be hijacked by emotional reactions (Goleman, 1995, 1998). Perhaps the most important brain region involved in the processing of emotional information is the **amygdala**. Jerome Kagan has suggested that if the amygdala is overly sensitive a child will avoid external stimuli, leading to a life of shyness, and vice versa (cited in Goleman, 1995). The development of brain structures that process emotion and allow for cognitive processing well beyond the primitive and most basic emotions sounds very much like what Freud had proposed regarding the role of the death instinct. This is not to suggest that either the amygdala or some portion of the neocortex is the anatomical location of the death instinct, but the evidence that such functions exist within the brain lends support to Freud's concept. According to Goleman, the ability to work with emotional intelligence is essential to one's well-being in life, and fortunately emotional intelligence can be trained and strengthened (Goleman, 1995, 1998).

Discussion Question: Compare Freud's concept of a life instinct and a death instinct, and consider the choices you make in life. Do you make choices that provide an opportunity to grow and change, or do you get caught up in pointless, even self-defeating, activities? If you make bad choices, where do you think those choices come from?

The Development of Libido and Psychosexual Function

Freud's most controversial theories related to sexual function and its role in personality development. Even more controversial than that initial statement was his suggestion that the sexual life of every person begins at birth. It is important, of course, to remember that Freud did not mean intimate sexual behavior when he talked about sexual impulses, but rather a general life impulse. He made an important distinction between "sexual" and "genital." By sexual he was referring to a wider concept of obtaining pleasure from different regions of the body, whereas genital refers to the act of reproduction, which comes into play following puberty.

Freud was well aware of this controversy during the early days of psychoanalysis, and many of his books make a special point of defending the theory of infantile sexuality. As mentioned in the biography, he actually attributed the initial observations of the role of sexuality in the development of neuroses to Breuer, Charcot, and Chrobak (Freud, 1914/1995). As he reflected on the history of psychoanalysis, Freud described how he and others before him had not intended to address infantile sexuality, but it proved unavoidable after extensive experience with psychoanalysis. In other words, Freud kept encountering infantile sexuality, and eventually he concluded that it was both universal and far too important to ignore. Therefore, he felt he could not allow old prejudices against recognizing or discussing the

relevance of sexuality to interfere with the development of psychoanalysis (see Freud, 1938/1949).

Freud also defended his theory of sexuality in logical ways. In his initial work on this topic, Three Contributions to the Theory of Sex (1905/1995), Freud specifically argued against the prevailing views that sexuality develops at puberty for the purpose of attracting a man and a woman to each other for the ultimate purpose of reproduction. He noted that there are individuals who are attracted to members of their own sex, that there are those who engage in sexual acts that disregard the genitalia (e.g., fetishes), and there are undeniable examples of children who become interested in their genitalia and obtain some excitation from them. Finally, in his Introductory Lectures… (Freud, 1917/1966) he declared his position quite clearly:

To suppose that children have no sexual life – sexual excitations and needs and a kind of satisfaction – but suddenly acquire it between the ages of twelve and fourteen, would (quite apart from any observations) be as improbable, and indeed senseless, biologically as to suppose that they brought no genitals with them into the world and only grew them at the time of puberty. (pg. 385).

Levels of Consciousness

From the very beginning of psychoanalysis, Freud and Breuer (1895/2004) recognized that their patients were often unaware of the connections between their symptoms and earlier traumatic events, and they might not even recall the events themselves. And yet, as described above, the memory of those events remained strong. How can a memory be strong but not remembered? The answer lies in the theory that there are different levels of consciousness. Freud described three levels of consciousness: the **conscious**, the **preconscious**, and the **unconscious**.

The conscious mind is our awareness, the knowledge that we exist and are alive. As you read this book you are conscious of it, when you talk to a friend you are aware of what they are saying and how you will respond (unless, of course, you respond with a Freudian slip!). Although the conscious mind is usually identified with our personality, and Freud recognized that people viewed consciousness as nothing more or less than the defining characteristic of the mind, his clinical experience with psychoanalysis made it impossible for him to accept the identification of the conscious mind with the mental mind (Freud, 1917/1966).

The unconscious mind, according to Freud, is the true psychic reality, and all conscious thought has a preliminary unconscious stage. And yet, the unconscious mind is truly inaccessible. In The Interpretation of Dreams (1900/1995), Freud wrote about the unconscious mind that:

…in its inner nature it is just as much unknown to us as the reality of the external world, and it is just as imperfectly communicated to us by the data of consciousness as is the external world by the reports of our sense-organs. (pg. 510)

How then does the unconscious mind affect our personality? Between the unconscious and conscious minds there is an intermediary: the preconscious. Technically, the preconscious mind is part of the unconscious, but only through the preconscious mind can the impulses arising in the unconscious enter into our conscious awareness. Freud distinguished between the two by theorizing that the unconscious cannot enter into consciousness, but if certain

rules are followed, the preconscious can enter into consciousness (but perhaps only after being censored; Freud, 1900/1995).

Freud also made two important points regarding these levels of the mind. First, the unconscious, preconscious, and conscious minds are not located in different regions of the brain. Instead, the level of consciousness of any particular psychical phenomenon depends on the cathexis of libidinal energy (or perhaps energy related to the death instinct) and repression (see Anxiety and Defense Mechanisms below). If the memory of a traumatic event is significantly repressed, it will remain in the unconscious, if not, it may enter into consciousness through the preconscious. Yet it has remained the same memory within the same memory structure of the brain. Freud also distinguished between the mind and reality, particularly between the unconscious mind and reality. He did, however, remind his readers that they would do well to remember that psychic reality is a special form of existence, though not to be confused with material reality (Freud, 1900/1995).

The levels of consciousness and the structures of the mind, as proposed by Freud. The iceberg analogy should actually be attributed to Theodor Lipps, whose work on the unconscious mind and humor was cited extensively in Freud's early books.

Structure of Personality

It is no accident that our discussion of the **id**, **ego**, and **superego** follow immediately after our discussion of the levels of consciousness. In The Ego and the Id (which also discuss the superego, despite not including it in the title; Freud, 1923/1960), Freud begins with a chapter on consciousness and what is unconscious, then follows with a chapter on the ego and the id, and then a chapter on the ego and the superego. It is difficult to discuss the two concepts, levels of consciousness and the **psychical apparatus** (a term Freud used for the id, ego, and superego), without intertwining them. In addition, these three structures begin as one, the ego develops from the id, and later the superego develops from the ego. As with levels of consciousness, it is inappropriate to think of the id, ego, and superego as actual structures within the brain, rather they are **constructs**to help us understand the psychodynamic functioning of the mind. Freud acknowledged this lack of understanding, and went so far as to say that even if we could localize them within the brain we wouldn't necessarily be any closer to understanding how they function (Freud, 1938/1949).

Id, Ego, Superego

The oldest aspect of the psyche is the id, which includes all that we inherit at birth, including our temperament and our instincts. The only goal of the id is to satisfy instinctual needs and desires; therefore, it acts according to the **pleasure principle**. It knows nothing of value judgments, no good, no evil, and no morality at all. It does not change or mature over time. According to Freud, there is nothing in the id except instinctual cathexes seeking discharge (Freud, 1933/1965). The energy associated with these impulses, however, is different from other regions of the mind. It is highly mobile and capable of discharge, and the quality of the discharge seems to be disregarded. This is a very important point, because it means that the id does not need to satisfy its desires in reality. Instead, they can be satisfied through dreams and fantasy.

Because the id demands satisfaction, and knows nothing of restraint, it is said to operate as a **primary process**. Since it can be satisfied in unreal ways, if we examine phenomena such as fantasies and dreams we can uncover the nature of the id. It was during his studies on

dream-work that Freud developed his understanding of the primary process of the id (Freud, 1923/1960). Actually, we can only know the id through psychoanalysis, since it exists entirely within the unconscious mind. Therefore, we need a **secondary process** structure in order for the mind to interact with the external world. This structure is found in the ego.

The ego arises from the id as an intermediary between the id and the external world. The ego functions according to the **reality principle**, and tries to bring the external world to bear on the impulses of the id. In other words, as the id demands satisfaction it is hindered by the reality of our environment, our societal and cultural norms. The ego postpones satisfaction until the time or the circumstances are appropriate, or it may suppress the id impulses altogether (Freud, 1938/1949). Freud believed that the ego is associated with perception (of reality), in the same way that the id is associated with instinct. The id is passionate, whereas the ego represents reason and common sense. But the id has the energy, the libido, to demand its satisfaction in some way, and the ego can only derive its energy from the id. Freud likened the ego to a horseback rider on a horse named id. The rider cannot always control the far more powerful horse, so the rider attempts to transform the will of the horse as if it were the rider's own will (Freud, 1923/1960).

The ego develops in part because it is that portion of the mind impacted by sensory input from the external world. Therefore, it resides partially in the conscious mind, and must serve three tyrannical masters: the id, the external world, and the superego (which we will discuss below). The goals of these three masters are typically at odds with one another, and so the ego's task is not an easy one (Freud, 1933/1965). The ego approaches this task by monitoring the **tension** that exists within the mind. This tension arises from internal and external stimuli making demands upon the mind, lowering this tension is felt as pleasurable, and increasing the tension is unpleasant. The id demands immediate reduction of tension, in accordance with its pleasure principle, whereas the ego seeks an appropriate reduction of tension, in accordance with its reality principle. A key point, of course, is that the ego also seeks pleasure. It does not try to deny the impulses of the id, only to transform or delay them. But why does the ego even bother to do that? There are times when pursuing pleasure can get us in serious trouble, but there are also times when we make choices because they seem right to us. These decisions, based on justice, morality, humanism, whatever term you choose, are mediated by the superego.

According to Freud, the superego is heir to the **Oedipus complex** (which we will discuss below), and arises as the child abandons their intense attachment to their parents. As a replacement for that attachment, the child begins to identify with their parents, and so incorporates the ideals and moral values of their parents and, later, teachers and other societal role models (Freud, 1933/1965). According to this view, the superego cannot fully develop if the child does not resolve the Oedipus complex, which, as we will discuss below, cannot happen for girls (Note: In addition to further discussion below, the issue of a more balanced female psychology will be discussed again in later chapters). The superego functions across all levels of the conscious and unconscious mind.

The superego takes two forms: an **ego-ideal** and a **conscience**. Freud considered the term ego-ideal as an alternative to the term superego, and it is not until we incorporate the development of conscience that we can recognize ego-ideal and conscience as different aspects of the superego. Indeed, it might be more appropriate, if one reads The Ego and the Id carefully (Freud, 1923/1960), to consider the ego-ideal and conscience as consecutive

transformations of that portion of the ego that becomes known in general as the superego. The development of the superego is a complicated process, and seems to derive from the development of the ego itself. For an infant, the attachment to the parents and identification with them is not recognized as something different. The ego is weak, and can do little to restrain the id. As the child grows, the erotic nature of the love for the mother is slowly transformed into identification; the ego grows stronger, and begins to become associated with being a love-object itself. When the ego is capable of presenting itself to the id as an object worthy of love, narcissistic libido is generated and the ego becomes fully formed (Freud, 1923/1960). In other words, the child becomes an individual, aware that they are separate from their parents. There is still an intense attachment to the mother, however, which stems from the early days of breast feeding. The child must eventually lose this intense attachment to the mother, and begin to more fully identify with either the father (for boys) or the mother (for girls). As noted above, this final transformation from attachment to identification should occur during the Oedipus complex, and the ego-ideal arises within the context of the child knowing “I should act like my father” (for boys) or “I should act like my mother” (for girls).

Although the ego-ideal could represent the culmination of development, Freud believed that one more step came into play. Because of the difficulty the child encounters during the loss of the intense, erotic desires of the Oedipus complex, Freud felt there was more than simply a residue of those love-objects in the mind. He proposed an energetic **reaction-formation**against the earlier choices. Now, the child incorporates concepts of “I must not act like my father or mother.” Under the influences of authority, schooling, religion, etc., the superego develops an ever stronger conscience against inappropriate behavior. This conscience has a compulsive character and takes the form of a categorical imperative (Freud, 1923/1960). This conscience is our knowledge of right and wrong, and early on it is quite simplistic. There is right and there is wrong (as with Kohlberg’s earliest stages of moral development; Kohlberg, 1963).

Discussion Question: Do you feel that your behavior is being driven by the unconscious impulses of the id? Do you believe that your moral development (your superego) is the result of internalizing your parent’s views of what is right or wrong? How close are your values to those of your parents?

Anxiety

We have already taken a look at the challenge faced by the ego in trying to balance the demands of the id, the superego, and the external world. What happens when the demands of these conflicting elements become too much for the ego to deal with? Simply put, we get scared, we experience fear and **anxiety** as a signal that there is some impending danger. Only the ego can experience anxiety, even if the underlying cause begins with the id or superego. Anxiety arises primarily from libido that has not been utilized. For example, if we are frustrated from fulfilling some id impulse, such as needing to go to the bathroom in the middle of a great movie, the libido cathexed to that impulse grows. This creates tension and the corresponding unpleasant feelings. As the id demands satisfaction, but the ego cannot figure out how to satisfy the id (and you really don’t want to miss the good part of the movie), the fear arises that the id will satisfy itself. Most of us would consider the possibility of going to the bathroom in our pants while at a movie a real danger to our self-esteem, and we could be arrested if we simply went to the bathroom right there in the movie theater. As

the ego is reduced to helplessness in its inability to find a reasonable outlet for the impulse of needing to go to the bathroom, anxiety serves the useful and important purpose of warning the ego that the impulse must be satisfied in order to avoid the danger (Freud, 1926/1959). And in support of Freud's view regarding our sexual nature, who would deny the great pleasure felt upon finally getting to the bathroom?

Freud described three general types of anxiety. Realistic anxiety involves actual threats to our physical safety. It is similar to fear, in that there is a real and external object that could harm us, but it differs from fear in that we may not be aware of a specific danger. For example, after the famous book Jaws (Benchley, 1974) was made into a movie (the kind of movie that you don't want to miss the good scenes) many people became anxious about swimming in the ocean, even though there were no specific sharks for them to fear. Still, there are sharks in the ocean, so it might be reasonable to experience some anxiety. Sometimes we are anxious about a real danger, but the anxiety we experience is completely out of proportion in relation to the threat. This suggests that there is an element of neurotic anxiety accompanying the realistic anxiety (Freud, 1926/1959).

Neurotic anxiety generally arises from an internal danger, the threat that unacceptable id impulses will break through and be acted on by the individual. The ultimate danger that exists is that we really will be harmed as a result of our actions. Therefore, Freud considered there to be a close association between neurotic and realistic anxiety (Freud, 1926/1959). For example, if we are being harassed by a bully, our aggressive id impulse might be to respond by killing this bully. Of course, that could result in going to prison or having the bully's friends kill us. So the anxiety that our violent id impulse might break out and influence our behavior is associated with the real danger posed by the consequences of that behavior, if it should happen to occur. Therefore, our neurotic anxiety is composed, in part, of our internalized realistic anxiety.

In a similar way, moral anxiety arises from conflict between our ego and the constraints imposed on it by the superego. Since the superego arises from the internalization of our parent's teaching us what is or is not appropriate behavior, we again have an association between the internal threat of the superego and the real, external threat of being punished by our parents. Therefore, as with neurotic anxiety, the precursor to our moral anxiety is realistic anxiety, even if our fears are based on our psychological impressions of a situation as opposed to an actual danger (e.g., the fear of castration; Freud, 1926/1959, 1933/1965). Freud (1933/1965) described the relationships this way:

Thus the ego, driven by the id, confined by the super-ego, repulsed by reality, struggles to master its economic task of bringing about harmony among the forces and influences working in and upon it; and we can understand how it is that so often we cannot suppress a cry: 'Life is not easy!' If the ego is obliged to admit its weakness, it breaks out in anxiety – realistic anxiety regarding the external world, moral anxiety regarding the super-ego and neurotic anxiety regarding the strength of the passions in the id. (pgs. 97-98)

Freud also described an overall pattern to the development and expression of anxiety and its useful role in life. In early childhood we experience traumatic situations in which we are helpless. Remember that Freud believed that psychic reality is every bit as significant as actual reality (Freud, 1900/1995), so the nature of these traumatic events is subject to individual perception. As the child's capacity for self-preservation develops, the child learns to recognize dangerous situations. Rather than waiting passively to be threatened or

harmed, an older child or an adult will respond actively. The initial response is anxiety, but anxiety is a warning of danger in anticipation of experiencing helplessness once again. In a sense, the ego is recreating to the helplessness of infancy, but it does so in the hope that now the ego will have at its command some means of dealing with the situation. Therefore, anxiety has hopefully transformed from a passive response in infancy to an active and protective response in later childhood and/or adulthood (Freud, 1926/1959).

Discussion Question: What makes you anxious, and how do you respond to those feelings?

Defense Mechanisms

We will cover **defense mechanisms** only briefly in this chapter. Although Freud talked about a wide variety of defense mechanisms during his career, he left it to his daughter Anna to literally write the book on The Ego and the Mechanisms of Defense (Anna Freud, 1936/1966). Freud himself discussed primarily two defense mechanisms: **repression**and **regression**. The recognition of these defense mechanisms was essential to the development of psychoanalysis, and they are the only two defenses mentioned by Freud in The History of the Psychoanalytic Movement (1914/1995). The purpose of these defense mechanisms is to protect the ego during the early years of life, when the ego has not adequately developed in its ability to control the libidinal impulses of the id. Thus, defense mechanisms serve a useful function at first, but later prove inadequate when the re-animation of the sexual life is reinforced following puberty (Freud, 1938/1949). Similarly, in adult life, defense mechanisms are useful in the short-term, but since they do not deal with problems directly they must eventually prove inadequate.

Freud identified repression as one of the key elements establishing psychoanalysis as unique from the cathartic method he had been working on thanks to the contributions of Josef Breuer (Freud, 1914/1995). Indeed, according to Freud, his own contributions that transformed Breuer’s cathartic method into psychoanalysis were repression, **resistance**, infantile sexuality, and dream analysis for the understanding of the unconscious mind. The value of repression cannot be underestimated:

The theory of repression is the pillar upon which the edifice of psychoanalysis rests. It is really the most essential part of it, and yet, it is nothing but the theoretical expression of an experience which can be repeatedly observed whenever one analyses a neurotic without the aid of hypnosis. One is then confronted with a resistance which opposes and blocks the analytic work by causing failures of memory. This resistance was always covered by the use of hypnosis; the history of psychoanalysis proper, therefore, starts with the technical innovation of the rejections of hypnosis. (pg. 907; Freud, 1914/1995)

The resistance Freud is referring to here is the defense mechanism of repression, which is the means by which the ego refuses to associate itself with an unacceptable instinctual impulse generated by the id. The ego is able to keep the “reprehensible” impulse from entering into the conscious mind (Freud, 1926/1959). But an important question arises: What then happens to this impulse seeking satisfaction? There are several possibilities, and Freud himself considered the answer to be rather complex. One thing that might happen is that the ego attempts to shift the libido cathexed to the impulse toward release as anxiety (Freud, 1926/1959). However, anxiety is unpleasant, and the id demands satisfaction in accordance with its pleasure principle. Therefore, this procedure is doomed to failure (and, therefore, the development of neurosis). There are, of course, alternatives that can occur

prior to the failure of this initial defense. The ego may find some acceptable alternative to the impulse through the other defense mechanisms, such as **sublimation**or reaction-formation.

Regression can be seen when an individual engages in behavior typical of an earlier stage of development. As Freud and Breuer tried to work out the causes of their patient's neuroses by using the cathartic method, they repeatedly found that they could not help their patients by focusing on the actual event that had led to a crisis. Instead, their patients inevitably made associations between the traumatic event and earlier experiences. Initially, these earlier experiences went back to puberty, and ultimately they went back to early childhood. Although Breuer favored some physiological explanation of this phenomenon, Freud insisted that it was psychological, and he termed the process regression (Freud, 1914/1995). According to Freud:

This regressive direction became an important characteristic of the analysis. It was proved that psychoanalysis could not clear up anything actual, except by going back to something in the past. (pg. 903)

Psychosexual Stages of Development

Above, we examined the basic development of libido and psychosexual function. Freud also believed that psychosexual function developed in a series of stages that occur in two waves. The first three stages occur during early childhood, from infancy to about the age of 5. Freud referred to this early period as the **pregenital phase** (Freud, 1905/1995). There is then a **latency period**, which lasts until puberty, after which the final stage, the **genital stage**, is realized and the individual is capable of physically mature reproductive functioning. These stages are typically presented as if they are exclusive and sequential. Although it is true that they are sequential, they are not entirely exclusive. Therefore, it is possible for the stages to overlap (Freud, 1938/1949). However, it remains true that during a particular stage one region of the body will be dominant, and most of the libido will be focused on that region (see Jarvis, 2004). For the sake of simplicity, we will treat the stages as if they occur one after another.

The Psychosexual Stages

The first phase of psychosexual development begins at infancy with the **oral stage**. According to Freud, the mouth is the first region of the body to become an **erotogenic zone**, and this lasts for approximately the first year of life. During this time the mouth makes libidinal demands on the mind. In other words, the region of the mouth demands that the mind direct adequate libidinal energy to satisfy the desires of the oral region. Although this serves the purpose of sustaining the infant by satisfying its nutritional needs, Freud believed that the infant's persistent sucking belied a need for satisfaction that was far greater than simply taking care of physiological needs. He believed that the infant needed to satisfy its desire for psychological pleasure independent of nourishment, and this was the basis for arguing that the behavior was sexual (in the larger, life-oriented perspective on sexuality).

Evidence for this stage is easy to see, and such commonplace observances contributed to Freud's thinking. Infants suck almost continuously, even when not being fed. They suck their thumbs, they can be comforted with pacifiers, as they become older they put everything they can get their hands on into their mouths. If they cannot satisfy this need, they may become fixated in the oral stage. As a result of this oral fixation, when an older child or adult

becomes frustrated or overwhelmed, they may regress and engage in oral behavior. This occurs because the fixation of libido on the oral region during infancy results in a deficiency of the libidinal energy needed to cope with some stressful period of adulthood. This oral behavior can take many forms, such as: overeating, smoking, drinking too much (of course, this usually refers to drinking alcoholic beverages), or just talking excessively.

During the second stage of psychosexual development, the **anal stage**, the anus becomes the focus of the libido, and the child derives pleasure from the ability to both retain and expel feces. Initially, the pleasure associated with evacuating the bowels is felt within the child, something Freud referred to as the **autoerotic** nature of sexual development (Freud, 1917/1966). Soon, however, the child learns that the world can be an inhibiting place, that greater pleasure can be derived only if the child defecates when and where others consider it to be appropriate. This realization occurs as a result of the most significant parent-child interaction during this stage of development: toilet training. If the parents are either too strict or begin too early the child may develop the traits of an anal character (Jarvis, 2004). These traits include excessive orderliness, stubbornness, and parsimony, each of which results from denial of the child's anal pleasure during toilet training. Orderliness, or tidiness, serves as a denial of taking pleasure in defecating, and stubbornness carries over from the child's assertion of their right to defecate at will (Jarvis, 2004). Parsimony, or being stingy, stems from the child's association of the feces with money. According to Freud (1917/1966):

He feels no disgust at his feces, values them as a portion of his own body with which he will not readily part, and makes use of them as his first 'gift,' to distinguish people whom he values especially highly. (pg. 390)

Freud was by no means unaware of how strange this part of his theory seemed. Immediately following the above quote, he went on to say the following, and remember that his Introductory Lectures on Psycho-analysis were actual lectures delivered to students at the University of Vienna:

I know you have been wanting for a long time to interrupt me and exclaim: 'Enough of these atrocities! You tell us that defecating is a source of sexual satisfaction, and already exploited in infancy! That feces are a valuable substance and that the anus is a kind of genital! We don't believe all that…' No, Gentlemen. You have merely forgotten that I have been trying to introduce the facts of infantile sexual life to you in connection with the facts … that for a large number of adults, homosexual and heterosexual alike, the anus does really take over the role of the vagina in sexual intercourse? (pg. 391)

So Freud was simply trying to address very real issues that he, and others, had observed in their clinical practices. His years of working in isolation had prepared him for dealing with disapproval, and he was not going to be shy about studying things that might make others uncomfortable.

The **phallic stage**, in which the libido focuses on the genitalia, represents the culmination of infantile sexuality. Although it typically occurs between the ages of 3 to 5 years old, it sets the stage for adult sexuality. Therefore, it is a very important period. According to Freud (1905/1995), this stage will "leave behind the profoundest (unconscious) impressions in the person's memory; if the individual remains healthy they determine his character and if he becomes sick after puberty, they determine the symptomatology of his neurosis." There are

two critical aspects of this stage: the first involves the **castration complex** and **penis envy**, and the second is the Oedipus complex. Technically, the Oedipus complex subsumes castration anxiety, but it is important to consider these aspects separately. In Three Contributions to the Theory of Sex (Freud, 1905/1995), the book that obviously focuses on the sexual aspects of psychoanalysis, Freud discussed the castration complex and penis envy, but not the Oedipus complex. Although he discussed the Oedipus legend in his earlier books, he did not use the term Oedipus complex until 1910 (see Jarvis, 2004). The other reason for considering the major aspects of this stage separately is the difference between boys and girls. Penis envy is obviously something that only girls can experience and, according to Freud, the Oedipus complex is something that only boys can experience. Girls experience something similar to the Oedipus complex, but their efforts are in vain, and this has a permanent, negative effect on their character (Freud, 1938/1949). For more discussion on the latter point, see the section below on Freud's perspective on the female psyche.

As we begin to address the castration complex and penis envy, it is important to realize that Freud's phallic stage is entirely focused on the penis, for both boys and girls. As the libido becomes focused on the genitals, a boy begins to manipulate his penis. He experiences no shame, and even enjoys displaying his body (Freud, 1905/1995). As a counterpart to this joy in his own sexuality, children are often curious about seeing other children's bodies. This creates a very different experience for boys and girls. When boys see a girl's body, they believe the absence of a penis means that the girl has been castrated. This leads to a fear of the same thing happening to them, and creates castration anxiety, which in adulthood can lead to a castration complex. When girls see a boy's body they are fully prepared to recognize the penis, and they become envious of it. Indeed, this envy can become strong enough that the girl wishes she was a boy (Freud, 1905/1995). Since she cannot be a boy, she begins to seek an alternative, either her father or a child of her own.

As Freud continued to develop his theory of the phallic stage, he included a broader perspective beyond the castration complex: the Oedipus complex. A boy's first love object is his mother, the one who feeds him and attends to all of his needs. This is only natural. As his libido becomes focused on his genitalia, however, this takes on the sexual desire that became so controversial. Keeping in mind that Freud did not believe that children understand the adult reality of their desires (much of it is unconscious), he described their behaviors as obvious evidence of their desire: boys talk about loving and marrying their mother, they persistently pursue being in her presence when she is dressing, they want to be in bed with her at night, etc. (Freud, 1917/1966). This eventually leads to conflict with the father, and with it the castration anxiety described above. Since there is no satisfactory resolution, given that the father is much more powerful than the child, the boy is compelled to give up the Oedipus complex, to repress it. In the healthiest condition, the Oedipus complex is completely destroyed in the id, and gives rise to a severe superego (Freud, 1933/1965). This repression, or destruction, of the Oedipus complex allows the boy to transition into the latency period.

For girls, the Oedipus complex takes a very different course. A girl's first love object is also her mother, since again it is the mother who provides most of the childcare. Once the girl realizes that she does not have a penis she develops penis envy, and she must undertake a change in her love object to her father, the one who can provide her with the desired penis. The mother is then seen as a rival for the father's penis, and indeed as someone who has already received all that the girl wants from her father. This can intensify feelings of hostility

toward the mother. Consequently, she may enter into the Oedipus complex feeling that it is a refuge or safe haven. However, the girl does not fear castration, since she has no penis to begin with, and without the fear of castration leading to repression of the Oedipus complex there is no motivation for the girl to move into the latency period (Freud, 1933/1965). Freud believed that girls remain in the Oedipus complex, and thus the phallic stage, for an indeterminate period of time, and that they cannot ever completely resolve it. Adding one more point of controversy, the only way in which girls can shift from an initially masculine situation (the mother as love object) to a feminine situation (the father as love object) is if they replace their penis envy with the desire for a child, given the ancient symbolic equivalence between the penis and giving birth (Freud, 1933/1965).

Following the oral, anal, and phallic stages there is a period of latency, during which progress is at a standstill. There are, however, some interesting things that happen during this period. The child knows that they are still incapable of procreation (even though this knowledge may be unconscious), so they begin to turn away from their sexual desires. They begin to view sexual impulses with disgust and shame, and to consider them immoral (Freud, 1905/1995). Although their education has much to do with this, Freud believed that it is also a natural occurrence. To compensate, the child (or the child's mind, as this again may be entirely unconscious) engages the defense mechanism of sublimation: the conversion of the unacceptable sexual impulses into activities that are socially acceptable (such as school work or sports). Another important consequence of this mental activity is that we forget our infantile sexual impulses, something Freud called **infantile amnesia**. Infantile amnesia is critical to the whole theory of the development of neuroses and the technique of psychoanalysis (Freud, 1938/1949), and it is one of the main reasons that many adults insist upon denying the possibility of infantile sexuality in the first place.

With the onset of puberty, the individual enters the final stage of psychosexual development: the genital stage. If the challenges of the earlier stages have been resolved in a satisfactory way, the individual is finally capable of appropriate and mature intimacy and sexual behavior. All of the psychodynamic processing that has taken place is not gone, however. According to Freud (1938/1949), even normal people have some of the following factors included in their final psychosexual organization: some libidinal cathexes are retained, others are taken into sexual activity as preliminary acts (such as foreplay), and still others are excluded from the organization either by repression or sublimation.

Freud's Perspective on the Female Psyche

I would like to begin this section by being fair to Freud. First and foremost, there were no other theories on the development of personality for Freud to consider as he developed his own theory. Second, most of the patients Freud saw were women, and apparently he needed to explain how it was that so many of his patients were women and not men. There were certainly other possible explanations than those offered by Freud, but it has been easy for others to look back and criticize him with the benefit of new and different ideas. One must also keep in mind that Freud was a basic scientist for many years, and he put a lot of emphasis on details. It is an undeniable, biological fact that men are male and women are female! Freud believed that psychology could never truly understand sex and gender differences unless we could understand why so many species exists as two different sexes in the first place (Freud, 1933/1965). Nonetheless, having acknowledged this, Freud's theory

does, unfortunately, describe women as the products of an incomplete and frustrated male development.

Freud believed that the development of the feminine psyche was more difficult and complicated than that of the masculine psyche for two main reasons, neither of which is faced by boys during their development. Initially, there are basic biological differences, both anatomically and in terms of temperament. Girls are typically less aggressive, defiant, and self-sufficient; they also seem to have a greater need for affection, and as a result become more dependent and pliant. However, Freud disregarded these differences, feeling that they were insignificant compared to individual differences between boys and girls (Freud, 1933/1965). Through much of the first wave of infantile sexuality, particularly the oral and anal stages, there is no difference between boys and girls. Even during the early portion of the phallic stage there is no difference between the sensations boys experience via the penis and the sensations girls experience via the clitoris. The first difference, however, arises from the need for girls to shift the focus of their libido from the clitoris to the vagina, particularly with regard to the importance of the latter for sexual activity leading to procreation. The second difference is the need for the girl to shift the focus of her libido from the mother to the father as she enters the Oedipus complex. This shift in the love object is not easy, and the outcome is not pleasant.

Freud proposed that the young girl’s attraction to her father at the beginning of the Oedipus complex is not simply a shift in the libido, but involves a rejection of the mother who had been the previous love object. The girl’s rejection of her mother is accompanied by hostility, and can end in hate. The hate can be profound and last the girl’s entire life, even if it is carefully compensated for later in life (Freud, 1933/1965). The reason for this hostility toward the mother arises from the castration complex and penis envy. Whereas a boy fears being castrated, the girl believes that she already has been, and the mother is responsible. The girl then develops an overwhelming desire to have a penis, the so-called penis envy, but it simply cannot be. Even when a girl is old enough to understand the basic biology of sex differences, she still retains an unconscious feeling of having been wronged and a considerable cathexis of libido remains (Freud, 1933/1965). It is interesting to note that Freud acknowledges that it is difficult to see these processes in action if one simply observes young girls. However, in his clinical practice he saw patients whose neuroses amplified these processes, and following psychoanalysis the underlying basis of this developmental stage seemed evident to Freud.

Table 3.1: A Summary of the Psychosexual Stages		
Stage	**Approximate Age Range**	**Essential Elements**
Oral Stage	Birth through the first year	The mouth is the source of erotic pleasure; this helps to obtain nourishment, but babies also put everything else they get their hand on into their mouths

Anal Stage	From age 1 to 3 years old	The anus is the source of erotic pleasure; toilet training is the major task of this stage (and as any parent can tell you, a most interesting experience!)
Phallic Stage	From age 3 to 5 years old	The genitals are the source of erotic pleasure; boys may fear castration, girls develop penis envy; boys resolve the Oedipus complex due to the motivation provided by their castration anxiety; since girls are not motivated by castration anxiety, they lack the motivation to completely resolve the Oedipus complex
Latency Period	From age 6 to puberty	At the beginning of this period infantile amnesia hides our earlier experiences from us; recovering them is the major task of psychoanalysis
Genital Stage	From puberty throughout adulthood	If the earlier stages were resolved successfully, then normal adult life proceeds; if not, a neurosis may develop; according to Freud, women cannot resolve the Oedipus complex, so they must have some psychological deficiencies relative to men

How then is a girl to resolve the Oedipus complex and achieve a healthy, adult personality? As described above, she can't! In Freud's own words:

The girl, after vainly attempting to do the same as the boy, comes to recognize her lack of a penis or rather the inferiority of her clitoris, with permanent effects on the development of her character; as a result of this first disappointment in rivalry, she often begins by turning away altogether from sexual life. (pg. 26; Freud, 1938/1949)

In these circumstances the formation of the superego must suffer; it cannot attain the strength and independence which give it its cultural significance, and feminists are not pleased when we point out to them the effects of this factor upon the average feminine character. (pgs. 160-161; Freud, 1933/1965).

Another important observation that came to Freud in his early psychoanalytic practice was the recognition that almost all of his female patients reported having been seduced by their fathers (which, if true, is an inaccurate way of saying they were sexually assaulted by a child molester). This led Freud to propose a **seduction theory**, which was the basis for first proposing the role of sexual trauma in the development of neuroses. This sexual trauma did not have to be as severe as rape, but it was considered significant nonetheless (see Jarvis, 2004). Later, however, Freud came to the conclusion that these seductions occurred only in the fantasies of the young girls. Indeed, Freud described the fantasy of a young girl being seduced by her father as the typical expression of the Oedipus complex in women (Freud, 1933/1965). Freud was criticized for theorizing that young girls fantasized about being

seduced by their fathers, and Breuer ended his pursuit of understanding sexual trauma when faced with the same conclusion (Freud, 1914/1995). Today, however, Freud is criticized more for having turned away from what may have been his most startling discovery, the prevalence of sexual abuse (see Jarvis, 2004).

Connections Across Cultures: Male/Female Differences

We don't normally think of men and women as being different cultures, but this has become a more popular approach to understanding their differences (e.g., see Brislin, 2000; Ferraro, 2006a; Haviland, 2005; Matlin, 2004). As we consider gender differences it is, of course, important to avoid stereotyping individuals. Nonetheless, the reality of cultural expectations related to sex and gender, some of which have a basis in the development of the human species, has led to some interesting research in both professional and popular psychology. As mentioned in Chapter 1, psychology has often been portrayed as a discipline focusing on White, European males. However, research on the psychology of women continues to expand, it has begun to address the specific differences of women of color, and it has led to the establishment of specific men's studies as well (Matlin, 2004). The latter point is an important one, since the earlier emphasis on White males was more circumstantial than intentional. For our purposes, I would like us to consider some of the interesting popular work on male/female differences.

In 1992, John Gray first published a very popular book entitled Men are From Mars, Women are From Venus. This book openly addresses the different ways in which men and women typically communicate and express their emotions. He offers practical advice on how men and women should react, or perhaps not react, to one another in everyday interactions. In his introduction to the paperback edition, Gray (2004) emphasizes how important it is notto try changing your partner. To expect members of the opposite sex to become more like yourself sends a message that they are not good enough as they are. He also notes that not all men or women will fall into the typical gender roles, so it is important not to stereotype. What is important, according to Gray, is that we recognize the general differences that exist between men and women and keep them in mind when we communicate with each other. If we can, then hopefully we can avoid conflicts that need not be inevitable. It might sound simple, but Gray clearly struck a chord in couples and individuals across America. He wrote numerous follow-up books, including Mars and Venus on a Date (1997) and the parenting guide Children are From Heaven (1999), each of which became #1 New York Times Bestsellers like the original. There is an interesting website (www.marsvenus.com), there are workshops and counseling centers based on Gray's work, and workplace seminars based on Mars and Venus in the Workplace (2002). Clearly, relationship problems based on the differences between men and women are of great interest to people in our society today. As harshly as Freud has been criticized for his views on the differences between men and women, he was certainly on to something. Perhaps that's part of the reason why even those who criticize Freud continue to talk about him.

More recently, Norah Vincent (2006) spent 18 months masquerading as a man, and then wrote about her experiences in Self-Made Man. Ms. Vincent spent a substantial amount of time working to look and act like a man. She consulted a professional make-up artist (especially to help with her "beard"), a voice coach, and a personal trainer to help her build muscle mass in her arms and shoulders. She even went to a sex shop and purchased a prosthetic penis, just to help make sure that her appearance was as convincing as possible.

Using the name Ned, she then traveled around the country, to five different states, and attempted to pass for a man in a variety of settings. She played on a men's bowling team, went to strip clubs with "other" men, spent time in a monastery, got a job as a salesman, and joined a men's group. One of the most interesting aspects of her masquerade involved dating. Curiously, it was not really difficult for her to consider dating women, because she happens to be a lesbian. But she was not prepared for how often she would be rejected when trying to meet women! Maybe that's why she wrote: "It was hard being a guy. Really hard." In fact, she found the entire experience quite disturbing. As a woman, she had always been viewed as very masculine. But when pretending to be a man, she was seen as very effeminate. She felt that her masculinity was constantly being judged, both by men and women. And the constant pressure to be a "real man" was overwhelmingly stressful. As she concluded her book, talking about the camaraderie of the men on the bowling team, she observed that:

Making this removed comforting contact with men and feeling the relief it gave me as my life as a man went on was not a sign of having joined the overclass, for whom superiority is assumed and bucking up unnecessary. It was more like joining a union. It was the counterpart to and the refuge from my excruciating dates, which were often alienating and grating enough to make me wonder whether getting men and women together amicably on a permanent basis wasn't at times like brokering Middle East peace.

I believe we are that different in agenda, in expression, in outlook, in nature, so much so that I can't help almost believing, after having been Ned, that we live in parallel worlds, that there is at bottom really no such thing as that mystical unifying creature we call a human being, but only male human beings and female human beings, as separate as sects. (pgs. 281-282; Vincent, 2006)

As an indication of how popular this topic continues to be, shortly after Ms. Vincent published her book she was interviewed on the popular news/comedy show The Colbert Report on Comedy Central. I guess it's important to keep smiling as we struggle through the many challenges of male/female relationships.

Freud's perspectives on women have created a great deal of negativity toward him and his theory, particularly among women. But should we judge Freud so harshly? It was common in the society in which he lived to consider women as the "weaker sex." In his practice, most of the patients he saw were female, so he needed his theory to explain why most of the people with psychological disorders were women (at least, that's what he thought). Granted he had made fundamental errors by not realizing that men might be avoiding help for psychological problems because of the culture and by not recognizing that women might be suffering from oppression caused by men, but since there were no other theories to compare his own ideas to, it is easy to condemn him. However, those theorists who began to address the cultural issues, like Adler and Horney, had Freud's theory for comparison. Horney in particular also had a growing body of research on anthropology and sociology to draw on. So as much as Adler and Horney may have disagreed with Freud, he still laid the foundation for their work and the work of many others.

Psychoanalysis

Most psychologists today make a distinction that Freud seldom, if ever, made. We refer to all theoretical perspectives related to the views of Sigmund Freud as psychodynamic theories, reserving the term psychoanalysis for the therapeutic method developed by Freud. Freud simply referred to both his theories and his therapy as psychoanalysis. This may well have resulted from the fact that Freud began as a therapist, and only developed his theories in order to explain why certain approaches worked and others did not. It may also have something to do with the fact that Freud's personality theories came first, and so there was no need in his mind to distinguish his views from the work of others. Whatever the reason, for our purposes we will use the term psychoanalysis to refer to the therapeutic method developed by Freud, which was uniquely different from the techniques already in use by people such as Breuer and Charcot. I would also like to note that many of the references in this section cite the book co-authored by Freud and Breuer (1895/2004). However, the citations come from a portion of the book written by Freud alone, and in which he takes personal credit for the work. Therefore, we need to acknowledge that although the book is published as the work of both men, it contains mostly each man's individual work, and only the "Preliminary Statement" is co-authored by Freud and Breuer. This point is by no means a small one, because it was at this point in their careers that the two men went their separate ways.

As mentioned above, Freud came to believe that the use of hypnosis, which had been championed by Breuer and Charcot, was unable to get at the root causes of patient's neuroses. He also learned through experience that psychoanalysis could only be effective if he was able to go back into the early childhood of his patients and uncover the unconscious conflicts and repressions that led to their neurotic behaviors. In order to accomplish this goal, Freud relied primarily on **free association** and **dream analysis**. In the history of psychoanalysis, there have been those who believed that psychoanalysis officially began when Freud rejected hypnosis and introduced free association (Freud, 1914/1995). Free association is often used, of course, during the interpretation of dreams, so the two techniques are not mutually exclusive. With regard to the value of interpreting dreams, Freud wrote perhaps his most famous line: "…the interpretation of dreams is the via regia [royal road] to a knowledge of the unconscious element in our psychic life." (pg. 508; Freud, 1900/1995).

Free Association – Freud's Therapeutic Breakthrough

Free association grew out of a need that resulted from problems implementing Breuer's cathartic method. The first problem was that many patients could not be hypnotized. With the patients that Freud could not hypnotize, especially those who would not even allow him to try hypnotizing them, Freud tried a technique of pressing them to remember. This technique also came up short, and Freud recognized a need to work around the patient's resistance (which we will examine in more detail below). The first technique that Freud developed involved pressing his hand against his patient's forehead and asking them to say whatever thought, no matter how seemingly irrelevant, came first into their consciousness (Freud & Breuer, 1895/2004). Freud himself described this technique as a trick, one that disconnects the patient's attention from his conscious searching and reflecting. However, trick or not, Freud found the technique to be indispensable. The thoughts that came to the forefront of consciousness, those believed to be easily accessible via the preconscious, were likely to be connected to the underlying associations responsible for the neurotic patient's

symptoms. Freud used this technique of free association quite successfully. As early as 1892, he treated a patient known as Fräulein Elisabeth von R. by relying entirely on free association (Freud & Breuer, 1895/2004).

Still, Freud did make some modifications in the technique. Two other methods used to begin the free association were to have the patient think of a number or a name at random. Of course, Freud did not believe that it was possible for anything in the mind to occur at random, and by continuing the association brought up by that first name or number, Freud could help his patient to arrive at the true unconscious associations that were the root of their problems (Freud, 1917/1966). He also used free association during the interpretation of dreams, and often found it helpful to examine which part of the dream the patient chose to begin making free associations (Freud, 1933/1965). Freud also considered psychoanalysis to be effective with children, but cautioned that a child's lack of psychological development limited their ability for free association. Other psychodynamic theorists worked more extensively with children, however, including Adler, Anna Freud, and Klein.

The Elements of Dream Analysis

For someone who considered dreams to be the royal road to the unconscious mind, it is no surprise that Freud's first book of his own was The Interpretation of Dreams (Freud, 1900/1995). The value of this work never diminished, and Freud devoted a chapter to dream interpretation in one of his last books: An Outline of Psycho-Analysis (Freud, 1938/1949). In the latter book, published only a year before Freud died, he wrote:

The only thing that can help us are states of conflict and uproar, when the contents of the unconscious id have a prospect of forcing their way into the ego and into consciousness and the ego puts itself once more on the defensive against this invasion. …Now, our nightly sleep is precisely a state of this sort, and for that reason psychical activity during sleep, which we perceive as dreams, is our most favourable object of study. In that way, too, we avoid the familiar reproach that we base our constructions of normal mental life on pathological findings; for dreams are regular events in the life of a normal person… (pg. 38)

Freud described our recollection of a dream as a façade, a covering that hides the underlying process of the dream. Thus, a dream has both **manifest content** and **latent content**. The manifest content (or the dream-content) of a dream is what we actually remember when we wake up. The latent content (or the dream-thoughts), however, is the true underlying meaning of the dream, the unconscious material from the id desiring satisfaction. Freud described the process by which the latent content is transformed into the manifest content as the **dream-work** (Freud, 1900/1995). Studying the nature of the dream-work, the way in which the unconscious material from the id forces its way into the ego but is transformed by the ego's opposition to the impulse, allows us to understand what is known as **dream-distortion** (Freud, 1938/1949). The importance of dream-distortion becomes clear when we consider the purpose of dreams. Freud believed that all dreams represent our true desires. Therefore, all dreams can be viewed as wish fulfillment. Although some dreams can be very anxiety-provoking, and certainly do not seem to represent our wishes and desires, this is the result of the distortion. If we successfully analyze the dream and identify its latent content, then Freud believed we would recognize the true wish-fulfillment nature of even anxiety-provoking or frightening dreams (Freud, 1900/1995).

When we sleep, the ability of the ego to repress or otherwise redirect the unacceptable impulses of the id is paralyzed. The id, then, is afforded “a harmless amount of liberty” (Freud, 1938/1949). But the ego is still the seat of consciousness, and still exerts some influence over the expression of the id impulses. And so the dream is distorted, transformed into something less threatening to the ego, particularly into something not threatening enough to wake the person up. To summarize this situation, when we are asleep the ego is less able to restrain the id. Consequently, the impulses of the id intrude in the preconscious and then into the conscious mind. This provokes anxiety and threatens to wake us up. However, the dream transforms the id impulse into the fulfillment of a wish, and we are able to continue sleeping. As Freud described it:

We shall be taking every experience into account if we say that a dream is invariably an attempt to get rid of a disturbance of sleep by means of a wish-fulfillment, so that the dream is a guardian of sleep. (pg. 46; Freud, 1938/1949)

Does it seem reasonable to say that all dreams are wish fulfillment? Certainly some dreams clearly fulfill our wishes and desires, at least through fantasy. Such dreams do not require any analysis. Other dreams, however, seem to make no sense at all. The id and the unconscious mind are not logical at all, contradictory ideas easily coexist side by side, and Freud even referred to the unconscious mind as the “Realm of the Illogical” (pg. 43; Freud 1938/1949). As these latent impulses are transformed into manifest content, it can be very difficult to separate them and make sense of a given dream. The dream-work itself, the very process of distorting or transforming the latent content into the manifest content in order to disguise the meaning of our dreams, involves a variety of factors, including: **condensation**, **displacement**, the use of **symbolic representation**, and **secondary elaboration** (Freud, 1900/1995).

According to Freud, condensation refers to the tendency to create unity out of a variety of dream elements that we would keep separate if we were awake. So, a single element of the manifest content of a dream might represent a number of latent thoughts. Thus, the analysis of a dream could be much longer than the dream itself. Displacement is not unrelated to condensation according to Freud, and refers to the switching of libidinal energy from one object to another, such that the important object of a dream might seem inconsequential, and vice versa. In other words, the apparent focus of the dream is probably not the actual focus of the dream. This does not simply suggest that we might substitute one person for another in a dream, it also happens that we might represent various elements through symbols. Once again, these symbols are employed by dreams to disguise the representation of latent content. As important as Freud considered symbols to be in a dream, he did not support the idea that dream dictionaries can identify universal meanings of dream symbols. It is only through the associations relevant to a specific dreamer that we can make sense of a dream’s symbolism (Freud, 1938/1949). Finally, as the dream is actually presented to the conscious mind, the ego ensures that the material is acceptable by performing what Freud termed the secondary elaboration. As with any perception, the ego fills in gaps and connections, but also misunderstands the true nature of the dream. As a result the secondary elaboration can offer little more than a smooth façade for the dream. Also, the secondary elaboration may only be partial, or even absent (Freud, 1933/1965). All of these processes together form the manifest content of the dream, resulting in something that might be difficult to understand, but which is within the reach of a determined psychoanalyst.

Discussion Question: Have you ever analyzed your own dreams? Were you able to discover any revelations after considering a dream in greater depth, even though it made no sense at first?

The Therapeutic Process

Initially, Freud began with a fundamental belief in the effectiveness of catharsis, the discharge of pent-up emotion that follows the recall and re-experiencing of traumatic memories (see Jarvis, 2004). If only a patient can recognize the unconscious association between an early traumatic event and their current symptoms, then the symptoms should be relieved. As Breuer and Freud noted in the introduction to their book:

For we found, at first to our great surprise, that the individual hysterical symptoms disappeared immediately and did not recur if we succeeded in wakening the memory of the precipitating event with complete clarity, arousing with it the accompanying affect, and if the patient then depicted the event in the greatest possible detail and put words to the affect. Remembering without affect almost always fails to be effective… (pg. 10; Freud and Breuer, 1895/2004).

One can see from this description, however, that the process of psychoanalysis is not easy. There must be a clear recognition of the initial traumatic event, in detail, with all of its original emotional impact, and the patient must then be willing to talk about the event in relation to their current problems.

As we have already seen, the first obstacle is resistance, the patient's reluctance to experience the anxiety associated with recovering repressed material. The more severe the symptoms, the more severe the resistance is likely to be. Even when a little trick is successful, such as pressing on the forehead to break the patient's concentration and allow free association, in serious cases the self remembers its intentions (which are often unconscious motives) and resumes its resistance (Freud & Breuer, 1895/2004). Because of this challenge, Freud believed that the therapist must be patient. Resistance that has been constituted over a long period of time can only be resolved slowly, step by step. In addition to the intellectual role of the therapist, there is an important emotional role as well. In some cases, Freud found that only the personal influence of the doctor could successfully break down the patient's defense mechanisms.

A large part of the reason that psychoanalysis can be so difficult has to do with how the unconscious mind exists. Freud believed that memory of a traumatic event exists as a pathogenic nucleus within multiple layers of pathogenic psychical material of varying resistance. The outer layers may be easy to uncover, but as one progresses into the deeper layers, resistance grows steadily. Adding to the challenge, the associations between layers do not simply go deeper, they can travel at odd angles, in something of a zigzag fashion, or branch out in multiple ways (Freud & Breuer, 1895/2004). Because multiple associations may exist between a patient's neurosis and the underlying traumatic event, it is critical to address all of the psychical material that comes to bear on the current condition of the patient. Even if the therapist rightly knows the basis for the patient's problems:

…there is no point at all in advancing directly to the nucleus of the pathogenic organization. Even if it were possible for us to guess this, the patient would not know what to do with the elucidation given to him and would not be altered by it psychically. (pg. 293; Freud & Breuer, 1895/2004)

As suggested above, a different kind of obstacle arises when the relationship between the doctor and the patient has been damaged somehow. Freud considered this to be a likely occurrence in serious cases of analysis. Freud described three ways in which the doctor/patient relationship can suffer. The first case involves the patient feeling estranged, neglected, undervalued, insulted, or if they have heard negative things about the doctor. Freud considered this problem to be fairly easily handled through good communication, although he noted that good communication can be difficult with hysterical patients. The second situation involves patients who fear that they will become too dependent on the therapist and that they will lose their independence. This can lead to new resistances. As an example, Freud described patients who complained of headaches when he pressed on their forehead, but really they were just creating a new hysterical symptom to mask their aversion to the belief that they were being manipulated or controlled. The final problem that commonly disturbs the relationship between the therapist and the patient is known as **transference** (Freud & Breuer, 1895/2004). Transference occurs when the patient reacts as if the therapist were an important figure from the patient's childhood or past, and transfers onto the therapist feelings and reactions appropriate to that person from the past. Although transference can interfere with the therapeutic process, it also offers advantages. The power conferred on the therapist by transference affords him an opportunity to re-educate the patient, correcting the mistakes of the parents, and it leads patients to reveal more about themselves than they might have if they had not developed such a connection to the therapist (Freud, 1938/1949).

Following transference, it is also possible for **countertransference** to occur. Countertransference refers first to an unconscious influence of the patient on the therapist, after which the therapist directs their own emotional states back onto the patient. In Freud's circle of analysts their own psychoanalysis was conducted in large part to eliminate the influence of this distorting effect. Today, there are some therapists who view countertransference as a useful means to gain a deeper perception about what is going on in their patient's mind (see Jarvis, 2004).

Is Psychoanalysis Effective?

The effectiveness of psychoanalysis as a treatment for psychological disorders has been a source of ongoing debate. In 2006, a select task force, established by the presidents of five major psychoanalytic organizations, published the Psychodynamic Diagnostic Manual (PDM Task Force, 2006). Included within the PDM is a section on research, including meta-analytic studies on the effectiveness of psychoanalysis on patient populations in the United States (Westen et al., 2006), the United Kingdom (Fonagy, 2006), and Germany (Leichsenring, 2006). Each of these chapters emphasize the difficulty in empirically evaluating the effectiveness of psychotherapy, and even more so comparing the effectiveness of different psychotherapeutic approaches. Nonetheless, for a variety of psychological disorders, there is evidence supporting the efficacy of psychoanalytic treatments. Both Fonagy (2006) and Leichsenring (2006) identify another area of research that needs to be continued: there is not just one type of psychoanalysis. Thus, continued research on the efficacy of psychoanalytic treatments should address the relative efficacy of different styles of psychoanalytically based therapies.

Religion and Spirituality

In the last section of this book, we will look at spiritual philosophies that provide positive guidelines for personal development and living one's life. Freud had rather strong feelings about religion, and simply put, he did not approve! Freud actually considered religion to be an obstacle to the further development of civilization, and of the "three powers" that oppose a scientific worldview (art, philosophy, and religion), "religion alone is to be taken seriously as an enemy" (Freud, 1933/1965). In his first and last books reviewing psychoanalysis Freud makes almost no mention of religion (Freud, 1917/1966, 1938/1949), a rather conspicuous absence. In between, however, he wrote two books thoroughly condemning religion and societies depending on it.

In The Future of an Illusion (Freud, 1927/1961), Freud describes the role that religion has played in establishing and maintaining inequitable civilizations. According to Freud, the primary purpose of civilization is the gathering of wealth and, then, its distribution. This distribution is almost always unfair, and leads to the establishment of a small group of wealthy elite and a much larger mass of lower classes. The challenge for the wealthy elite is to maintain this unfair distribution, without the unreasonable use of force. The use of force will ultimately fail, since the instinctual demands for pleasure by the masses (driven by their id impulses) will drive them to take whatever they want from each other and from the wealthy. And the masses have power in numbers! Religion has served civilization by providing a controlling force over these instinctual demands, incorporated directly into the psyche of each individual by means of the development of the superego. The problem, however, lies in the fact (according to Freud) that there is no God, it is all an illusion. Thus, in Civilization and Its Discontent (Freud, 1930/1961), Freud states that the "religions of mankind must be classed among the mass-delusions…" that are used to provide people with a certain degree of happiness and protection against suffering, in spite of the reality of an unfair and uncertain world.

In his last completed book, Moses and Monotheism (Freud, 1939/1967), Freud flatly rejects the entire basis for Judaism and Christianity. Based on archaeological evidence, Freud claims that Moses was not Jewish, but rather an Egyptian. Furthermore, he argues that the monotheistic religion that provides the basis for Judaism, Christianity, and Islam (the Abrahamic religions) was a discredited Egyptian religion established by the pharaoh Amenhotep IV. When Amenhotep IV died, and Egyptians who still followed the traditional religion of Egypt came back into power, Moses led the Jewish people out of Egypt so that he would have followers to whom he could teach the religion he hoped to maintain. Since Moses was a hero to the Jewish people, Freud claims that they rewrote the story to say that Moses was Jewish and that their one true God had been revealed to them long before they entered Egypt (Freud, 1939/1967).

If religion is nothing more than an illusion, where does it come from, and what is the danger of it? According to Freud, the development of religion is analogous to the development of each individual. Basically, God is symbolic of our relationship with our own father. In his New Introductory Lectures… (Freud, 1933/1965), Freud neatly lays out this relationship. Religion serves to provide us with an understanding of the origins of the universe and life, it offers us hope for protection and ultimate happiness, and it lays down moral guidelines for living our lives. Similarly, our fathers give each of us life, they protect us when we are young, and they teach the rules and morality of our culture. Although the establishment of religions by our primitive ancestors may be quite understandable, Freud used rather harsh

language when referring to religion. He wrote that "…our wretched, ignorant and downtrodden ancestors…" "…were far more ignorant than we are…" (Freud, 1927/1961). In his opinion, ancient religious books are the product of "fraud" from a time when "man's ignorance was very great…," and he includes the Bible and the Koran in that category (Freud, 1933/1965). He considered religion to be the antithesis of science and art, the two highest achievements of man (Freud, 1930/1961). Even when addressing more modern times, he compares the political oppression by the Russian Bolsheviks, with its prohibition of thought (punishable by death), as "just as ruthless as was that of religion in the past…doubts of its correctness are punished in the same way as heresy was once punished by the Catholic Church" (Freud, 1933/1965). And in anticipation of his views being challenged, Freud wrote that his considerations "will impress only that minority of readers familiar with analytical reasoning and able to appreciate its conclusions" (Freud, 1939/1967). The danger in all of this, according to Freud, is what might happen if people become aware of this illusion and fraud. If people become aware that there is no God, if they discover that no one else believes, then there may be a violent reaction as a result of the inherent hostility toward civilization by the masses (Freud, 1927/1961). Accordingly:

…either these dangerous masses must be held down most severely and kept most carefully away from any chance of intellectual awakening, or else the relationship between civilization and religion must undergo a fundamental revision. (pg. 39; Freud, 1927/1961)

And yet, in spite of such harsh condemnation of religion, Freud was at times fascinated by individuals who expressed deep spiritual experiences. While in college, Freud was profoundly impressed by the religious philosopher Franz Brentano, a former Catholic priest. Freud wrote to a friend that he could not refute any of Brentano's theistic arguments, and he referred to Brentano as a "remarkable man" (cited in Nicholi, 2002). Much later in life, Freud discussed an alternative to his earlier analysis of the basis for religion. He had sent a copy of The Future of an Illusion to a friend who was well versed in traditional Yoga. This friend, named Romain Rolland, described for Freud an "oceanic" feeling, a sense of eternity and limitlessness. Freud was unable to discover such feelings within himself, and expressed a general dissatisfaction with scientific investigations of such phenomena. Today, however, neuroscientists are using real-time brain imaging techniques, such as functional magnetic resonance imaging (fMRI), to study the alterations in brain activity unique to meditative states and, in particular, positive emotions (e.g., see Barinaga, 2003; Goleman, 1988, 2003; Mathew, 2001). Although Freud obviously had no knowledge that such studies would someday be possible, he did acknowledge that experiences like the "oceanic" feeling might form the basis for religious sentiments in the human species (Freud, 1930/1961). Yet another friend encouraged Freud to practice Yoga, particularly meditation, to experience these altered states of mind for himself. It is unclear whether Freud ever attempted to meditate, but he does make mention of his friend's belief that meditation may reveal a primordial state of mind, perhaps even deeper than that of the id and the unconscious with which Freud had occupied his career (Freud, 1930/1961). Freud even went so far as to suggest that if we could somehow achieve a complete reduction, an extinction, of the tension between our instinctual needs and the constraints imposed by reality and the superego that we might achieve "nirvana" (Freud, 1938/1949).

How might we reconcile the seeming contradiction between Freud's harsh attitude toward religion with his apparent fascination with mystical spirituality and deeply spiritual individuals? Freud believed that religion had failed society. Religion has ruled human

civilization for thousands of years, and yet, “We see that an appallingly large number of people are dissatisfied with civilization and unhappy in it…In every age immorality has found no less support in religion than morality has…” (Freud, 1927/1961). Also, it is undeniable that death awaits each of us, and Freud was definitely concerned with death. In 1909 Freud met William James (Freud, 1952). James asked Freud to carry the bag James had with him and to walk on; James said he would catch up after dealing with an attack of angina pectoris. James died of heart disease a year later. Freud wrote that “I have always wished that I might be as fearless as he was in the face of approaching death.” Freud was also concerned about how his own death might affect his mother, who lived to a ripe old age. Martin Freud noted that “Grandmother Amalia…looked for some time as if she would live forever, and my father was terrified by the thought that she might survive him and, in consequence, have to be told of his death.” (M. Freud, 1983). Perhaps we should not be surprised that someone who was so thoughtful regarding death, someone who proposed a death instinct, might be inclined to have some concern regarding what happens after one’s life comes to an end.

As gloomy as Freud’s perspective may seem at first, he remained hopeful regarding the future of humanity. He considered religion to have been just one step in the development of our species, and that science had reached a point where it could move us ahead another step. When addressing the belief of many people that we were created in the image and likeness of God, a god who must also have created evil and the Devil (other theologians have come to a different conclusion on this point; see, for example, Mere Christianity by C. S. Lewis [1952]), Freud suggested that we bow to the deeply moral nature of mankind, which has overcome this difficulty (Freud, 1930/1961). He acknowledged the positive role that religion has played in redirecting and transforming some of our sexual impulses into impulses experienced as love. Indeed, the purpose of civilization itself is to serve Eros, the life instinct, by combining individuals into “families, then races, peoples and nations, into one great unity, the unity of mankind” (Freud, 1930/1961). But the natural aggressive instinct, the death instinct manifested as the hostility of the individual for civilization, opposes the establishment of civilizations. Thus, the meaning of the evolution of civilizations becomes clear:

It must present the struggle between Eros and Death, between the instinct of life and the instinct of destruction, as it works itself out in the human species. This struggle is what all life essentially consists of, and the evolution of civilization may therefore be simply described as the struggle for life of the human species. And it is this battle of the giants that our nurse-maids try to appease with their lullaby about Heaven. (Freud, 1930/1961)

Discussion Question: Freud believed that religion has failed to resolve the difficulties that many people face, that it has outlived its usefulness, and that God is only an illusion anyway. Does this fit with your view of the world in which we live?

Personality Theory in Real Life:

The Things We Do Are Not Really Surprising!

It isn’t difficult to apply Freud’s theories to everyday life. Only a few years after publishing his landmark book on dream analysis, Freud published Psychopathology of Everyday Life (1904/1995). One year later, he published Wit and Its Relation to the Unconscious (1905/1995). So Freud clearly intended his theory to address all aspects of life, and he was

well aware that jokes and laughter are as much a part of life as any of the darker aspects of psychoanalysis (with concepts such as demanding id impulses and the death instinct).

As mentioned earlier in the chapter, the most famous example of a simple, everyday psychopathology is the famous Freudian slip. A very humorous example of a Freudian slip can be found on the Wikipedia website (http://en.wikipedia.org): “Excuse me, but I’m having doubts about your theories, Dr. Fraud.” There are, of course, a variety of other mistakes we commonly make when we talk. We often forget names, words, the order of phrases and sayings, and these errors can carry over into reading and writing as well as speaking. Of course, Freud believed that these mistakes are neither random nor just the result of forgetfulness. Rather, they represent psychological processes leading to the expression of one’s real feelings and beliefs. An interesting example that often comes to mind is a quote by former Vice-President of the United States Dan Quayle. The Honorable Mr. Quayle intended to quote the slogan of the United Negro College Fund: A mind is a terrible thing to waste. Instead, he said “What a waste it is to lose one’s mind.” Since “losing one’s mind” is slang for becoming mentally ill, it might be argued by some that the vice-president had revealed a negative attitude toward minority groups. We should always be very careful, of course, to avoid analyzing situations with only the bare minimum of information. Without the implication of the slang meaning of “losing one’s mind” and some alleged unconscious intentions, Vice-President Quayle’s statement seems like nothing more than a simple mistake. Haven’t we all made mistakes like this that were very embarrassing at the time? I don’t know if it’s good or bad, but Vice-President Quayle made a variety of other infamous comments that can easily be found on the Internet, suggesting that his only issue was a penchant for making mistakes in ordinary speech. However, people seem to find such mistakes quite interesting. An Internet search for embarrassing quotes will locate a wide variety of examples like the one above, but they are not always attributed to the same person. Some of the same embarrassing quotes have been attributed to whoever happens to be the current political target of the person posting the webpage. So obviously a lot of people enjoy the embarrassment of others, but be careful about attributing any quote you locate on a random webpage.

Of course, any time a famous person like Vice-President Quayle makes that big a mistake, the late-night comedians are all over it! And that brings us to Wit and Its Relation to the Unconscious (Freud, 1905/1995). In this early book, Freud discussed jokes and witticisms in great detail. He actually considered wit-work, the process of forming a joke or witty remark, as being essentially the same thing as dream-work. Thus, the examination of how an individual uses humor on a daily basis might reveal a great deal about their personality. The following is one of the jokes Freud included in his book for analysis. It takes a little thought, since it is in a category that Freud referred to as sophistic faulty thinking (Freud described over 20 types of joke):

A gentleman entered a shop and ordered a fancy cake, which, however, he soon returned, asking for some liqueur in its stead. He drank the liqueur, and was about to leave without paying for it. The shopkeeper held him back. “What do you want of me?” he asked. “Please pay for the liqueur,” said the shopkeeper. “But I have given you the fancy cake for it.” “Yes, but you have not paid for that either.” “Well, neither have I eaten it.” (pg. 634)

The next time you think of a really good joke or something really funny to say, or the next time you hear a joke that really makes you laugh, take a minute or two to consider what that laughter might be saying about who you really are.

A Final Note

An important aspect of Freud's theory is his belief that development occurs in a series of predictable stages. This belief is not common to all of the theories we will cover in this textbook. Furthermore, stage theories are likely to be influenced by **cultural relativism**, the perspective that the significance of an idea or concept is determined by how it is valued within a given culture. Noted psychologists who have offered such a developmental perspective include Vygotsky and Bronfenbrenner, and it has been suggested that an approach incorporating cultural relativism may be of particular importance when studying the development of African Americans and other minority groups (Belgrave & Allison, 2006; Howard-Hamilton & Frazier, 2005). For example, it has been noted that religion is a very important aspect of African American, Hispanic American, and Asian American culture (Axelson, 1999; Belgrave & Allison, 2006; Taylor, Chatters, & Levin, 2004), and we have just examined how little Freud cared for religious or spiritual matters. Other psychologists, however, gave significantly more consideration to cultural influences (e.g., Adler, Horney, and the Stone Center Group). We will examine their contributions in later chapters.

Review of Key Points

• Early in his career, Freud felt that hypnosis could not help to understand why hysteria developed or why some patients got better with treatment. Psychoanalysis grew out of his desire to understand psychological processes better.

• The terms hysteria and neurosis are no longer recognized as technical terms. Hysteria would commonly be called a conversion disorder today, and the neuroses refer to a variety of disorders.

• Catharsis refers to the release of pent-up emotion that occurs when a patient remembers a traumatic event

• Psychic determinism is the concept that all thoughts and behaviors have some basis in prior experience, nothing happens by accident or chance.

• Each person is driven by a life force known as Eros. The energy associated with this life force is called libido.

• Cathexis refers to the attachment of libido to some psychical phenomenon. Since libido is limited, experiencing numerous traumatic events can leave a person with limited resources to cope with normal life.

• Freud believed that individual development is driven by sexual impulses that begin at birth. However, when Freud referred to sexual impulses he was really referring to the life force Eros.

• Freud described three levels of consciousness: the unconscious, the preconscious, and the conscious minds.

• Personality has three basic components according to Freud: the id, the ego, and the superego. The id is the oldest and most basic component, and the source of all sexual impulses and libidinal energy. The id acts according to the pleasure principle.

• The ego arises from the id, and acts according to the reality principle. It tries to balance the desires of the id with the constraints of the superego and the real world.

• The superego arises from the ego as we internalize the rules and customs of society. As it develops it takes two forms: the ego-ideal and the conscience.

• When we are conscious of the ego being unable to restrain the impulses of the id we experience anxiety. The threat can be real, or primarily psychological. Freud emphasized that psychological reality can be every bit as important actual reality.

• When faced with anxiety, we resort to defense mechanisms. The defense mechanisms first proposed by Freud were repression and regression.

• There are four psychosexual stages, separated by a latency period. The oral, anal, and phallic stages occur during early childhood. Following the latency period and puberty, the genital stage represents physically mature reproductive functioning.

• During each psychosexual stage, a different region of the body becomes an erotogenic zone. The child’s recognition of these sexual feelings belies the autoerotic nature of sexual development.

• The most dramatic development occurs during the phallic stage. It is in this stage that the Oedipus complex occurs, with its potential for the castration complex or penis envy.

• Infantile amnesia occurs naturally, according to Freud, and is the reason that we do not remember these psychosexual processes from our childhood.

• Freud’s perspective on the development of girls has been problematic since its inception. He considered the female psyche to be the result of an incomplete and frustrated male development.

• Freud’s initial contributions to therapy were the development of free association and dream analysis, leading some to suggest that this was point when psychoanalysis was created.

• In analyzing dreams, Freud distinguished between the manifest and latent content. The difference results from distortion that occurs during the dream-work.

• The analysis of a dream can be quite complex, since information might be condensed, displaced, represented symbolically, and finally it all undergoes a secondary elaboration.

• The challenge faced by a psychoanalyst is the nature of the unconscious mind. It exists as layer upon layer with different degrees of resistance. Only through patience can the psychoanalyst overcome this resistance and help the patient.

• During the process of psychoanalysis itself the psychoanalyst may encounter transference and experience countertransference.

• Freud acknowledged that religion had served a useful role in the history of humanity, but he firmly believed that there is no God. He felt we have grown beyond the ignorance of our ancestors, and the observations of psychoanalysis could explain how religion had arisen. He believed that the future of humanity was being hindered by clinging to these ancient and meaningless beliefs.

CHAPTER 04 / CAC.902

Object Relations and Attachment-Informed Therapy

Object relations theory shifts attention from drive to relationship. The word object is historical and can sound cold. In this tradition, an object is usually a person, or an internal image of a person, toward whom feelings are directed. The central question is not only what drive seeks discharge, but how early relationships become internal patterns that shape later experience. A child does not only learn that a caregiver feeds them. The child learns whether the world is responsive, whether distress can be shared, and whether the self is worthy of care.

Internal working models

Attachment research, associated with John Bowlby and Mary Ainsworth, describes internal working models of self and other. A child who experiences consistent responsiveness may develop an expectation that help is available. A child who experiences unpredictable care may become vigilant and preoccupied. A child who experiences rejection of need may minimize attachment signals. These patterns are not destiny. They are adaptations to a caregiving environment. A counselor should not use attachment language to blame a child or a parent. The pattern made sense in the context in which it developed.

Object relations theorists such as Melanie Klein, Donald Winnicott, and Ronald Fairbairn described related ideas in different ways. Klein emphasized early fantasies and the integration of love and hate. Winnicott emphasized the holding environment, the good-enough mother, and the true self. Fairbairn emphasized that the child seeks relationship, not merely pleasure. These theories are not identical, and they should not be collapsed into one simple story.

Splitting and integration

Splitting is the separation of experience into all-good and all-bad parts. A young child may do this developmentally. Under stress, older clients may also experience people as either perfect or worthless. A teenager with intense relationship instability may idealize a counselor after one session and devalue the counselor after a boundary is set. The counselor should not take either position as the whole truth. The task is to help the client hold mixed feelings: "I am disappointed in you, and you have also helped me."

In child therapy, splitting may appear in play. The good queen is protected; the bad witch is destroyed. The counselor does not interpret too quickly. The play may be practicing integration. Over time, the story may allow a character to be both angry and loved. That narrative change can be a clinical marker.

The holding environment

Winnicott's holding environment is not only physical holding. It is the reliable provision of care that allows the infant to develop without constant impingement. In therapy, the frame can function as a holding environment: consistent time, clear boundaries, and a therapist who survives the client's affect without retaliation. For a child, the holding environment may include the room, the toys, the routine, and the caregiver's participation. A chaotic waiting room or frequent cancellations can undo the sense of safety.

A good-enough therapist is not perfect. The therapist makes small mistakes and repairs them. The repair teaches that relationship can survive imperfection. A child who has experienced unpredictable care may test the frame. The counselor responds with calm consistency rather than punishment. "We have ten minutes left. I know it is hard to stop. We will continue next week." The boundary is part of care.

Transitional phenomena

Winnicott described transitional objects, such as a blanket or teddy, as existing in an intermediate area between inner and outer reality. The object is not merely a thing. It supports the child's move from dependence toward separateness. In therapy, a drawing, a stone, or a small toy may serve a transitional function. The counselor should respect the meaning without turning the object into a magical cure. A child may keep a drawing from session to remember that the counselor exists between meetings.

Adolescents may use music, a hoodie, or a phone wallpaper as transitional supports. The counselor can ask what the object helps them remember. The question links the object to internalization. Over time, the client may no longer need the external object because the soothing function has become internal.

Attachment-informed assessment

An attachment-informed assessment asks how the child seeks comfort, how the caregiver responds, and what happens when distress is high. It does not assign a label after one observation. A child who does not seek comfort may have learned that seeking is unsafe. A caregiver who appears intrusive may be responding to a child's medical trauma. The counselor looks for cycles. Child distress leads to caregiver anxiety; caregiver anxiety leads to overcontrol; overcontrol leads to child protest; protest confirms caregiver fear. The cycle is the target, not the blame.

Clinical application

Seven-year-old Leo was adopted after several foster moves. He controls play, hides food, and becomes furious when the counselor ends a session. A behavioral plan alone may increase his sense of threat. An attachment-informed approach combines predictability, caregiver coaching, and play. The counselor uses the same opening ritual each week. The caregiver is helped to respond to Leo's fear rather than only his control. When Leo hides food, the caregiver might say, "You want to make sure there is enough. We keep food in the kitchen, and I will make sure you eat." The limit is clear; the meaning is recognized.

The counselor also watches their own countertransference. Leo may make the counselor feel incompetent or rejected. The counselor does not withdraw. The feeling is discussed in supervision and used to understand Leo's expectation that adults leave when he is difficult. Therapy becomes a place where difficulty does not end relationship.

Limits and integration

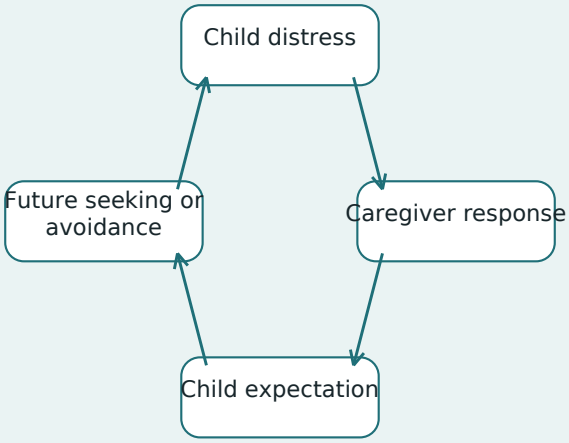
Attachment language can be misused. A counselor should not diagnose attachment disorders loosely or assume that all behavior problems are attachment problems. Autism, ADHD, trauma, language delay, and contextual stress can look similar in some settings. Assessment must be broad. Object relations concepts can also become vague if every behavior is interpreted as an internal object relation. The counselor should tie interpretations to observable patterns and client feedback.

Integration is common. A counselor may use attachment-informed formulation, behavioral caregiver coaching, and cognitive work with an adolescent. The theory provides a map of relational expectation; the techniques provide steps. The relationship provides the experience that can update the map.

Reflection

Recall a time when you expected someone to reject you. How did you protect yourself? Did you demand reassurance, withdraw, or test the person? Clients protect themselves in similar ways. The therapeutic task is not to remove protection by force. It is to make protection less necessary.

The attachment cycle



Repeated cycles shape internal working models. Therapy can offer a new cycle.

Attributed Foundational Reading

Attribution and critical-reading note

Adapted from Personality Theory in a Cultural Context by Mark D. Kelland, OpenStax CNX / Lansing Community College (2015), CC BY 4.0. Selection, organization, and PDF layout modified for this course; new lectures and course diagrams are separate additions.

Read historical and theoretical claims critically. A theory is a lens for inquiry, not a verdict about a child or family. Where source claims are dated or culture-bound, the new lecture supplies cautions and contemporary framing.

Source module m58036 • Personality Theory in a Cultural Context • pinned GitHub snapshot a52283699509. Full repository and license details appear in the attribution register.

Who were the neo-Freudians, and what exactly does this term mean? Many early psychoanalysts remained basically true to Freud and his theories. These individuals are collectively known as either neo-Freudians or as ego psychologists, for their emphasis on the ego. Shifting from Freud's emphasis on the id to an emphasis on the ego is a major change, but it does not require rejecting the basic elements of Freud's theory. The shift also encourages the study of children. After all, it is during childhood that most of this dramatic psychological development occurs. The neo-Freudians stand in contrast to Alfred Adler and Carl Jung, who intentionally distanced themselves from Freud, and Karen Horney, who initially brought a female perspective to psychodynamic theory (a perspective in which she directly challenged some of Freud's ideas on women) but later shifted to a cultural perspective on the psychology of women. For an interesting introduction to a number of important neo-Freudians, see Freud and Beyond (Mitchell & Black, 1995), and for an introduction to some of their papers see The First Freudians (Ruitenbeek, 1973).

In this chapter, we will only be able to take a brief look at a handful of these theorists. Among the many neo-Freudians, there are some notable individuals we will not be covering. Karl Abraham was an active psychoanalyst in Berlin when the rest of Germany was largely mocking Freud's theory. Sándor Ferenczi accompanied Freud and Jung on their landmark trip to America. A. A. Brill was influential in the development of psychoanalysis in America, and an early translator of Freud's works into English. As important as these theorists were in their own right, they have not had quite the influence of those whom we will cover in this chapter.

First, we will examine Anna Freud's contributions on the defense mechanisms. Although her father had described many of the defense mechanisms, he left it to his daughter to literally write the book on them (A. Freud, 1936/1966). Anna Freud also moved beyond her father's work in at least one significant way: she contributed to the study of applying psychoanalysis to children. Two other early contributors to the application of psychoanalysis to children were Melanie Klein and D. W. Winnicott. Although Klein and Anna Freud shared an interest in studying children, they often did not agree. This conflict led to a split in the English school of psychoanalysis (Mitchell & Black, 1995). Winnicott had been trained in part by colleagues of Klein, and he was supervised by Klein personally for a time, but eventually his independence led him to develop his own theories. Having been a pediatrician before he became a psychoanalyst, he was able to draw on a wealth of experience observing children interacting with their mothers (Mitchell & Black, 1995). Finally, we will take a very brief look at the work of Heinz Kohut and Margaret Mahler, and their perspectives on how an individual finally becomes just that.

Placing the Neo-Freudians in Context - 1: Connecting Personality Theories

In one sense, it is not possible to put the neo-Freudians in context, because to do so would be to suggest that these theorists have concluded their work. As we will see, the process of modifying Sigmund Freud's theories toward some final, comprehensive theory accepted by all psychoanalysts continues today. So we must keep in mind that we are really just putting those theorists mentioned in this chapter in context, and this task is one that will continue into the future.

If you had been able to predict in 1910 what future perspectives on Freud's theory would become, you might have easily done it. Some theorists stayed true to Freud's basic principles, others took radically different approaches, and the rest fell somewhere in

between. Not surprisingly, the strongest supporter of Freud's theory was his own daughter Anna. Although she shifted the focus of psychoanalysis from the id to the ego, and emphasized analyzing children, these were reasonable extensions of Freud's own work. And more importantly, she made these changes only within the constraints that her father's theory allowed.

In contrast, Melanie Klein made radical changes to psychoanalytic theory, and directly challenged the views of Anna Freud. This challenge led to a public battle, one in which Anna Freud seems to have been respectful toward Klein, but Klein did not return the courtesy. Subsequently a third group arose, a group of moderates who appreciated the direction Klein had taken, but who had their own differences of opinion with her. Following a series of open discussions during World War II, it was agreed by all to acknowledge the differences among those who followed Anna Freud, Klein, or their own independent paths.

Eventually, subsequent theorists began to recognize the value in each different approach to psychoanalysis. Today, theorists like Otto Kernberg have come a long way toward blending the different neo-Freudian approaches together. But there remained an important area of psychology that needed further study in its own right: the psychology of women. The object relations theorists had laid an interesting foundation for what became a relational-cultural approach to the study of women, and ultimately all people.

Anna Freud and Ego Psychology

Anna Freud (1895-1982) was the youngest of Sigmund and Martha Freud's six children, and the only one to pursue a career in psychoanalysis. However, this did not come about immediately, and Anna Freud never attended medical school as her father had. Therefore, she was one of the first lay psychoanalysts, which is an important consideration for all mental health practitioners today (since Freud and most other early psychoanalysts were actually psychiatrists who had attended medical school). In 1971, a survey conducted among psychiatrists and psychoanalysts identified Anna Freud as the most outstanding colleague among both groups (see Peters, 1985).

Anna Freud lived with her parents until Sigmund Freud's death in 1939. She was a lively child, with a reputation for being mischievous. Although she always enjoyed a good relationship with her father, it was her older sister Sophie who was her father's favorite daughter (Peters, 1985). She was quite intelligent, but never attended college. She did, however, attend private schools, eventually entering the Cottage Lyceum in Vienna during fifth grade. She soon entered the Cottage Lyceum's high school, graduating in 1912. Since she had not chosen a career, she traveled to England to improve her English, one of several languages she had learned. Upon returning to Vienna, she became a teacher at the Cottage Lyceum's elementary school. She was very popular among her students, one of whom described her as "such a marvelous and simple figure that I loved her deeply at that time" (cited in Peters, 1985). Her popularity likely resulted from her own love of teaching and for her students (Coles, 1992). Anna Freud considered this experience as a teacher to have been very valuable for her later career as a child psychoanalyst:

The people who follow this line of thought hold that those who analyze children should possess not only the correct analytical training and mental attitude but something further: something which is called for by the idiosyncrasies of childhood, namely, the training and the mental attitude of the pedagogue… (pg. 130; A. Freud, 1973).

Even before she graduated, Anna Freud had begun reading her father's works. But it was not until 1918 that she entered into psychoanalysis. Such a situation, a father psychoanalyzing his own daughter, would be considered inappropriate today, but at that time the entire field was still quite new and many aspects of it were still experimental. In any case, Anna Freud subsequently became one of her father's most unwavering supporters and an important psychoanalyst in her own right. In 1920 she attended the International Psychoanalytical Congress with her father, and 2 years later she was a member of the Vienna Psychoanalytical Society and began presenting her own papers. In 1923 she began her own practice treating children.

As Anna Freud was developing her theories regarding the psychoanalysis of children, Melanie Klein was developing her theories in England. There were significant disagreements between them, including a symposium in 1927 organized specifically to provide an opportunity for Klein to publicly attack Anna Freud's theories (Peters, 1985). After Anna Freud arrived to stay in England with her family in 1938, the conflict between them threatened to split the British Psychoanalytic Society. However, during World War II, a series of discussion forums resulted in the establishment of parallel training courses for the two groups.

After the war began, Anna Freud helped to set up the Hampstead War Nursery to provide foster care for over 80 children, a number that rose to a total of 190 children over several years (Peters, 1985). In addition to simply providing care for the children, she hoped to provide the children with continuity in their relationships with staff and family. With long-time friend Dorothy Burlingham she studied the effects of stress on young children in wartime. This work continued after the war when she had an opportunity to help provide care for six orphans who had survived the Theresienstadt concentration camp (Coles, 1992; Peters, 1985).

Throughout the rest of her life, she remained devoted to her work at the Hampstead War Nursery. She helped to establish the Hampstead Child Therapy Course (in 1947) and a children's clinic. Eventually the nursery became known more simply as the Hampstead Clinic, and Anna Freud became one of the major figures in psychoanalysis. She often traveled to lecture in the United States, and in the 1970s she was a co-author, with two professors from Yale University, of two books about governmental involvement in the lives of children who have been emotionally deprived and socially disadvantaged (see below). She received many honors, including honorary doctorates from Clark University (where her father had lectured during his trip to America), Harvard University, and Vienna University. The honorary medical degree she received from Vienna University was awarded in 1972, only 1 year after she returned to visit her native city for the first time since the Freud's had escaped the Nazis in 1938.

Shortly after her death, the Hampstead Clinic was renamed the Anna Freud Centre, in her honor, and in 1986 her former home became the Freud Museum. Although she has been considered by some to have done little more than continue her father's work, she was a pioneer in both ego psychology and the psychoanalysis of children. Accordingly, she deserves to be considered one of the most influential neo-Freudians. Much of the information in this biography can be found on The Anna Freud Centre website, as well as much more information on the center itself (http://www.annafreudcentre.org).

Ego Psychology and the Defense Mechanisms

In 1936, Anna Freud published perhaps her most influential book: The Ego and the Mechanisms of Defense(A. Freud, 1936/1966). She began by stating a redefinition of the field of psychoanalysis. There was a general bias, in her opinion, among many psychoanalysts to focus on the deep instinctual impulses of the id at the expense of considering the ego. However, since the id is always unconscious, its processes can never be observed directly. It may also be difficult to observe the processes of the ego as well, but at least the ego exists partially within the conscious mind. Since it is the ego that observes both the impulses of the id and the restraints of the superego, and since the ego is available to the psychoanalyst, she concluded “this means that the proper field for our observation is always the ego.” (A. Freud, 1936/1966)

Although the ego is observable, that doesn’t mean that a person’s thoughts and behaviors always make sense. As the id demands the satisfaction of its impulses, the ego attempts to restrain the id, in accordance with the external demands of society and the internal representation of those demands in the superego. When these factors come in conflict, and the ego cannot easily resolve the conflict, anxiety develops. In order to help alleviate that anxiety, and to continue restraining the impulses of the id, the ego resorts to defense mechanisms. In so doing, the ego transforms the conflict somewhat and attempts to keep both the conflict and the basis for the conflict unconscious. When an individual is suffering psychologically and has sought help from a therapist, according to Anna Freud, the psychoanalyst arrives on the scene as someone who disturbs this fragile peace. This is because “it is the task of the analyst to bring into consciousness that which is unconscious…” (A. Freud, 1936/1966).

In order to understand how the ego uses defense mechanisms, it is necessary to understand the defense mechanisms themselves and how they function. Some defense mechanisms are seen as protecting us from within, from the instinctual impulses of the id (e.g., repression); other defense mechanisms protect us from external threats (e.g., denial). When treating a patient, the goal of the psychoanalyst is to determine how much a given defense mechanism contributes to the symptoms and to the ego resistance of the patient (in other words, resistance to therapy). To help understand these issues, Anna Freud identified and discussed ten defense mechanisms as being commonly recognized in the field of psychoanalysis: **regression, repression, reaction-formation, isolation, undoing, projection, introjection, turning against the self, reversal**, and **sublimation.** See Table 6.1 for a description of some of the defense mechanisms most commonly discussed today.

The defense mechanisms are not all available to an individual at the same time. As originally proposed by her father, Anna Freud believed that the defense mechanisms develop with the structures of personality (the id, ego, and superego). For example, projection and introjection depend on the differentiation of the ego from the outside world, so they would not be available to the ego as defense mechanisms until the ego had sufficiently developed (and, perhaps, differentiated into the superego as well; A. Freud, 1936/1966). This became an important point of contention, however, with the English school of analysis that included Melanie Klein. Whereas Anna Freud and her colleagues believed that projection and introjection would not be available in early childhood, since the structures of personality have not adequately developed, members of the English school believed that projection and introjection were a necessary part of that development. Although this debate and others between Freudian and Kleinian theorists became rather bitter (Mitchell & Black, 1995), Anna Freud did acknowledge the following:

These differences of opinion bring home to us the fact that the chronology of psychic processes is still one of the most obscure fields of analytic theory…So a classification of the defense mechanisms according to position in time inevitably partakes of all the doubt and uncertainty which even today attach to chronological pronouncements in analysis. It will probably be best to abandon the attempt so to classify them and, instead, to study in detail the situations which call for the defensive reactions. (pg. 53; A. Freud, 1936/1966)

The final outcome of personality development, whether primarily normal or abnormal, depends on the overall process of these defense mechanisms throughout development. It is natural that defense mechanisms are called upon by the ego during both waves of psychosexual development. Whether or not the ego develops as a yielding and flexible structure depends on the strength of the instincts during development and the environment in which the individual develops. If instinctual demands are excessively urgent during development, the ego will redouble its defensive activities. This can stiffen the resistance of the ego to the instinctual impulses, leading to symptoms and inhibitions that remain throughout life (A. Freud, 1936/1966). At least, they might remain until the patient is successfully analyzed by a qualified psychoanalyst.

What is the status of defense mechanisms in psychology today? Clinical psychologists have always found the concept of defense mechanisms useful, but for a period of time defense mechanisms did not receive much attention from academic psychologists (Cramer, 2000). The primary reason for this was that early studies aimed at demonstrating the existence of the defense mechanisms and their processes were not promising. However, these are particularly difficult processes to study, since much of the processing occurs unconsciously (Cramer, 2000; Kernberg, 2004). According to Cramer (2000), as the various fields of psychology developed, they began to examine psychological processes that received new names within the particular field, even though the processes being studied were actually defense mechanisms that had already been discovered within psychoanalysis. Just to cite a few examples: what cognitive psychologists describe as selective attention may involve the defenses of splitting and dissociation, in social psychology scapegoating is a form of displacement, and in developmental psychology a child's verbal report of positive emotion while their facial expression clearly represents negative emotion is a classic case of denial (for a variety of examples and references see Cramer, 2000). Therefore, one can conclude that defense mechanisms, and defensive processes, have remained an important aspect of psychology and psychotherapy since they were first described by Sigmund Freud and Josef Breuer in 1895 (Freud & Breuer, 1895/2004).

Discussion Question: Anna Freud believed that even normal personality development involved the common use of defense mechanisms. Consider your own personality. Are there situations that make you anxious and, if so, can you recognize the defense mechanisms you rely on? Do you have a common defense mechanism that you use more than any other?

Psychoanalysis with Children

As mentioned above, Anna Freud began her career as a school teacher. This interest in children never diminished, and as she began to devote her career to psychoanalysis and psychoanalytic research her focus remained on the psychological lives of children. In 1946 she published The Psycho-Analytical Treatment of Children (A. Freud, 1946). This book is a collection of works she had written mostly in 1926 and 1927, with the final portion having been written in 1945. The book begins with an interesting preface, an apology that the book

had not been available in English at an earlier date, particularly in the country of England. The reason for this, according to Anna Freud (and many historians agree), is that in England the theories of Melanie Klein dominated the psychoanalytic community (see also Mitchell & Black, 1995). Klein and her colleagues believed that psychoanalysis could be conducted successfully with young children, and that the process of transference occurred in the same manner as it did with adult patients. Initially, Anna Freud believed that psychoanalysis could not be performed with young children. Later, she acknowledged that the efforts of her colleagues had helped to make that a possibility, but she steadfastly denied that she had ever seen the normal process of transference in anyone younger than adulthood (A. Freud, 1946).

Table 6.1: Common Defense Mechanisms	
Repression	Repression involves blocking an impulse from conscious expression. Examples include forgetting a traumatic event, such as sexual abuse, or being unaware of hostile feelings toward family members.
Regression	When faced with difficult situations that we cannot resolve, we may regress to behavior indicative of an earlier stage of development. For example, when we are very sick, we may act as helpless as if we were an infant and hope that someone will take care of us.
Denial	Denial refers to simply refusing to believe an unpleasant reality. For example, when someone is told they have a terminal illness, they may deny it and refuse to follow treatment recommendations.
Projection	Projection involves attributing our own negative impulses to another person. If, for example, we want to see another person fail, perhaps to make us feel superior, we may claim that they are trying to interfere with our success.
Reaction-Formation	A reaction-formation is the process of suppressing unacceptable impulses and adopting an opposite course of action. For example, a parent who resents having children may shower them with love.
Identification	We often model our behavior after people we admire, or adjust our behavior based on people we fear. Internalizing this process of identifying with others is primarily how the superego develops, how we adopt the rules and guidelines of our culture and make them our own.
Displacement	Sometimes we cannot respond directly to unpleasant situations, so we displace (or transfer) our impulses onto another object. For example, if your boss yells at you at work, you then go home and yell at people in your family.

Rationalization	Rationalization is the process of finding logical reasons for unacceptable behavior or thoughts. For example, a professor may constantly battle with administrators about policies, while claiming that he/she only has the best interests of their students in mind.
Isolation	Isolation involves separating the anxiety-provoking aspects of an event from one's other thoughts and behaviors. For example, following the death of a child, one parent may set aside their grief in order to be able to provide support for the other parent.
Sublimation	Sometimes referred to as the successful defense mechanism, sublimation is the process of channeling unacceptable impulses into socially acceptable forms. It is often said that great artists must suffer before they can find the inspiration to master their craft.

The importance of the analysis of children in terms of research is threefold, according to Anna Freud. It helps to confirm the theories develop by Sigmund Freud and others based on the analysis of adults, it leads to fresh conclusions and new conceptions (and she commends Melanie Klein on this point), and it serves as a point of transition to a field of applied analysis that she felt might become the most important of all: working with children as they develop (A. Freud, 1927/1973). What Anna Freud tried most to do in her writings was to point out the circumstances that make psychoanalysis with children different than psychoanalysis with adults. Fundamentally, there is no difference in the process. However, the results of the process must be viewed differently in order to understand what happens with children. Since Melanie Klein was actively pursuing the same goals, Anna Freud often wrote specifically about Klein, acknowledging her accomplishments, but also pointing out their differences.

The basic argument was aimed at the intellectual and psychological abilities of young children, and the extent to which the psychical apparatus (the id, ego, and superego) have developed. Since all three personality structures have developed in an adult, there is no need for the analyst to worry about that. All the analyst needs to do is to bring into consciousness the neurotic processes that have led to the patient's symptoms (granted, that may be easier said than done). In children, however, the ego and especially the superego are still developing. Therefore, the analyst must consider the role he/she may play in the development of the child. The analyst must consider the extent to which the superego has already developed, and the analyst may be able to take advantage of its continuing development and help to direct and/or instruct superego development. According to Anna Freud (1927/1973):

…In the analysis of the adult we are at a point where the superego has already established its independence - an independence which is unshakable by any influence from the outside world…But child analysis must include all those cases in which the superego has as yet not reached any true independence. Only too clearly it strives to please its taskmasters, the child's parents and others responsible for his training…we have to use our influence from without in an educational manner by changing the child's relation to those who are bringing him up… (pgs. 138-139)

As these processes are actually observed, Anna Freud believed that the child's symptoms could transform in the presence of the psychoanalyst, in a way that simply did not happen with adults. Thus, it was essential to observe children from a different point of view than one observes adults. She acknowledged that Melanie Klein had contributed to our understanding of how children might be observed, but she felt that Klein had attributed too much to what Klein and her colleagues observed. Klein allowed children the opportunity to play with toys in her office, a situation in which the child's imagination can run wild. Klein believed this was the same for a child as free association was for an adult. However, Anna Freud countered that an adult is aware of their goals in psychoanalysis, whereas a child at play is not aware of being in therapy. As a result, Anna Freud viewed the play of children as fundamentally different than an adult's free association:

The play technique worked out by Mrs. Melanie Klein is certainly valuable for observing the child. Instead of taking the time and trouble to pursue it into its domestic environment we establish at one stroke the whole of its known world in the analyst's room, and let it move about in it under the analyst's eye but at first without his interference…

Mrs. Klein however…assumes the same status for these play-actions of the child as for the free associations of the adult patient…if the child's play is not dominated by the same purposive attitude as the adult's free association, there is no justification for treating it as having the same significance. (pgs. 28-29; A. Freud, 1946)

Anna Freud's steadfast belief that children do not have the intellectual or psychological capacity for free association, in part because they simply can't relax and lie still for an hour on the psychoanalyst's couch, also raises questions for the use of the second most common psychoanalytic technique, dream analysis. Dream analysis depends on the patient's ability to freely associate about the manifest content of the dream. With children, it is necessary for the psychoanalyst to connect the manifest content with the latent content, and this process will always be somewhat suspect (A. Freud, 1946). These problems lead into another controversy, the age at which psychoanalysis can occur. Whereas Klein and her colleagues believed that psychoanalysis could occur at any age, since babies are involved in play as part of their earliest activities, Anna Freud believed that some amount of speech was necessary to form an adequate therapeutic relationship with a child (so the earliest reasonable age for psychoanalysis would be around two to three years of age; A. Freud, 1946).

In her conclusion to The Psycho-Analytical Treatment of Children, Anna Freud re-emphasizes the role that the developmental processes play as a child (and their id, ego, and superego) grows. As such, a child analyst needs to be "as intimately familiar with the normal sequence of child development as he is familiar with the neurotic or psychotic disturbances of it" (A. Freud, 1946). She specifically mentions academic psychology and the tests being created by psychologists for the measurement of personality as useful in this regard. She also mentions one test as being especially useful in examining libido development and its disturbances: the Rorschach test. Still, she acknowledges that our understanding of the developmental processes of the libido and of the ego is "very incomplete." Yet she cautions psychoanalytic investigators "not to confine examinations to short-cuts of any kind…" (A. Freud, 1946). Clearly her concern for children, and for a professional approach to psychoanalytic research and practice, were foremost in her mind.

So what does current research tell us about the effectiveness of psychoanalysis on children? As is often the case, the results are not clear. Although a number of studies have shown that

children benefit from psychoanalysis, the degree of that benefit has been disappointing (see Jarvis, 2004). When children of different age groups were studied, the results showed that psychoanalysis was more effective for younger children than it was for adolescents, something that Anna Freud would probably not have predicted. An important problem in many of the studies providing positive results, however, is that the results are based on reports by the therapists. Such reports are highly subjective and open to bias (Jarvis, 2004). Also, as we might expect, the effectiveness of psychoanalysis is dependent on the experience and skill of the analyst. Typically, analysts who have experience teaching and supervising psychoanalysis, as well as those who are prepared to try a variety of psychoanalytic techniques, achieve significantly better outcomes with their patients (Kernberg, 2004). So we must consider the question of the effectiveness of child psychoanalysis as one that remains unanswered.

Discussion Question: Is a child at play engaging in the same mental activity as an adult engaged in free association? Melanie Klein believed yes, but Anna Freud disagreed. Do you think that children are capable of the same participatory role in psychoanalysis as adults, and is play the best way to observe children?

Late in her life and career, Anna Freud extended her work beyond the psychoanalytic treatment of children to larger issues of child advocacy. In collaboration with Joseph Goldstein, a professor of law at Yale University, and Albert Solnit, a professor of pediatrics and psychiatry at Yale's medical school and Director of the Child Study Center at Yale, she co-authored two books: Beyond the Best Interests of the Child (Goldstein, A. Freud, & Solnit, 1973) and Before the Best Interests of the Child (Goldstein, A. Freud, & Solnit, 1979). These books focus on the importance of placing the interests of children first when the government intervenes in cases involving the custody and placement of children. These situations arise is many circumstances, such as in the case of orphans or following a difficult divorce, but also in more extreme cases of abuse or when certain parents do not believe in allowing medical care for very sick children. Since these situations can pit one parent against another, or the parents against the interests of society, the authors addressed very clearly reasons why the interests of the child should be placed first:

Some will assert that the views presented in this volume are so child-oriented as to neglect the needs and rights of the adults. In fact, this is not the case. There is nothing one-sided about our position, that the child's interests should be the paramount consideration once, but not before, a child's placement becomes the subject of official controversy. Its other side is that the law, to accord with the continuity guideline, must safeguard the rights of any adults, serving as parents, to raise their children as they see fit, free of intervention by the state, and free of law-aided and law-abetted harassment by disappointed adult claimants. To say that a child's ongoing relationship with a specific adult, the psychological parent, must not be interrupted, is also to say that this adult's rights are protected against intrusion by the state on behalf of other adults.

As set out in this volume, then, a child's placement should rest entirely on consideration for the child's own inner situation and developmental needs… (Goldstein, A. Freud, & Solnit, 1973)

So long as the child is part of a viable family, his own interests are merged with those of the other members. Only after the family fails in its function should the child's interests become a matter for state intrusion. (Goldstein, A. Freud, & Solnit, 1979)

Object Relations Theory

There are those who say that it is inappropriate to refer to **object relations theory** as if it were a single theory. It is more appropriate to refer to object relations theorists, a group of psychoanalysts who share a common interest in object relations, but whose theories tend to vary with each individual theorist. Sigmund Freud used the term object to refer to any target of instinctual impulses. In the current context, an object is a person, or some substitute for a person such as a blanket or a teddy bear, which is the aim of the relational needs of a developing child. Melanie Klein is generally recognized as the first object relations theorist, and her change in emphasis from Sigmund Freud's view was rather profound. Freud believed that a child is born more like an animal than a human, driven entirely by instinctual impulses. Only after the ego and the superego begin to develop is the child psychologically human. Klein, however, felt that a baby is born with drives that include human objects, and the corresponding need for relationships. In other words, the infant's instinctual impulses are designed to help the child adapt to the distinctly human world into which the child is born (Mitchell & Black, 1995).

Melanie Klein

Melanie Klein (1882-1960) was also born in Vienna, about 13 years before Anna Freud. However, she did not remain there. She moved first to Budapest, where Klein entered into psychoanalysis with Sándor Ferenczi. She then moved to Berlin, where she continued her psychoanalysis with Karl Abraham. Since the move to Berlin occurred in 1921, and since she credited this period with Abraham much more significantly than the time she spent with Ferenczi, the most significant portion of her psychoanalysis actually occurred shortly after that of Anna Freud (Mitchell, 1986). However, the time periods are so close that, despite the difference in age, they really should be considered contemporaries. But they certainly did not agree, as we have already seen.

There are two factors that contributed to the differences between Klein and Anna Freud. Since Klein underwent psychoanalysis with Ferenczi in Budapest, and then Abraham in Berlin, her exposure to multiple points of view likely gave her a unique perspective on psychoanalysis. Anna Freud, remember, never left her father's home while he was alive. Also, in 1925, just as the two women were embarking fully on their own careers, Klein moved to England following the death of her mentor Karl Abraham. This separation from the continent of Europe, in a country where analysts already shared ideas similar to Klein's, led to a freedom of thought that allowed Klein to develop her own theories without restraint (Mitchell, 1986).

As mentioned above, Klein believed that an infant is born with the capacity and drive to relate to others. An inherent problem with this reality, however, is that the infant must be prepared to deal with all types of people and relationships. Thus, Klein believed that the death-instinct and its aggressive energy are every bit as important as the life-instinct (Eros) and its libidinal energy:

…What then happens is that the libido enters upon a struggle with the destructive impulses and gradually consolidates its positions…the vicious circle dominated by the death-instinct, in which aggression gives rise to anxiety and anxiety reinforces aggression, can be broken through by the libidinal forces when these have gained in strength. As we know, in the early stages of development the life-instinct has to exert its power to the utmost in order to

maintain itself against the death-instinct. But this very necessity stimulates the growth of the sexual life of the individual. (pgs. 211-212; Klein, 1932/1963)

As the child continues to develop, love becomes the manifestation of the life-instinct, and hate becomes the manifestation of the death-instinct (Mitchell, 1986). As for people in the child's life, the child will begin to recognize both good and bad elements of their support for and relationship to the child. The child will also recognize good and bad aspects of its own thoughts and behaviors. As a result, the child will begin a process known as **splitting**, in which the bad parts of an object are split off and not allowed to contaminate the good parts of the object. In simpler terms, a child can continue to love its parents, even though there may be times that the parents do not satisfy the impulses of the child. Similarly, the child can continue to feel a positive sense of self-esteem, even though they sometimes fail or do bad things. Such split attitudes can continue into adulthood, and we sometimes hear people talk about "love-hate" relationships.

Since the child is born with the life-instincts and death-instincts necessary to establish and maintain object relations, Klein did not focus on development as going through a series of stages. Instead, she suggested two basic developmental orientations that help the child to reconcile its emotions and feelings regarding the inner and outer worlds in which the child exists: the **paranoid-schizoid position** and the **depressive position** (Jarvis, 2004; Kernberg, 2004; Mitchell, 1986; Mitchell & Black, 1995). The means by which the child processes these emotions and orientations is based largely on fantasy. Klein believed that the child is capable at birth of an active fantasy-life. This fantasy emanates from within, and imagines what is without, and it represents the child's primitive form of thinking about the world and about the child's relationships (Jarvis, 2004; Kernberg, 2004; Mitchell, 1986). With regard to the mother, the child's first object:

In the baby's mind, the 'internal' mother is bound up with the 'external' one, of whom she is a 'double', though one which at once undergoes alterations in his mind through the very process of internalization; that is to say, her image is influenced by his phantasies, and by internal stimuli and internal experiences of all kinds. (pgs. 148-149; Klein, 1940/1986)

Klein believed that object relations are present at birth, and the first object is the mother's breast (Klein, 1946/1986). Due, in part, to the trauma of birth, the child's destructive impulses are directed toward the mother's breast from the beginning of life. As the child fantasizes attacking and destroying its mother, it begins to fear retaliation. This leads to the paranoid position. Because of this fear, and in order to protect itself, the child begins the process of splitting the mother's breast and itself into good and bad parts (the schizoid position). The child then relies on two principle defense mechanisms to reduce this anxiety: introjection leads the child to incorporate the good parts of the object into itself, and projection involves focusing the bad parts of the object and the child onto the external object. This introjection and projection then provide the basis for the development of the ego and the superego (Klein, 1946/1986; Mitchell, 1986).

As the child continues to develop, it becomes intellectually capable of considering the mother, or any other object, as a whole. In other words, the mother can be both good and bad. With this realization, the child begins to feel guilt and sadness over the earlier fantasized destruction of the mother. This results in the depressive position, and it represents an advancement of the child's maturity (Jarvis, 2004; Kernberg, 2004; Klein, 1946/1986; Mitchell, 1986).

Discussion Question: Melanie Klein is unique in her emphasis on aggression and the death-instinct. Does it seem reasonable to consider aggression as important in human development as libido (and Eros)? Is it possible that aggression was an essential element in the development of the human species, but one that is no longer needed?

Another important contribution by Klein was the method of **play analysis**. She acknowledged that some psychoanalytic work had been done with children prior to 1920, particularly by Dr. Hug-Hellmuth (Klein, 1955/1986). Dr. Hug-Hellmuth used some drawings and play during psychoanalysis, but she did not develop a specific technique and she did not work with any children under the age of 6. Although Klein believed that even younger children could be psychoanalyzed in the same manner as adults, that doesn't mean they have the same ability to communicate as adults. Klein's interest in play analysis began with a 5 year-old boy known as 'Fritz.' Initially Klein worked with the child's mother, but when his symptoms were not sufficiently relieved, Klein decided to psychoanalyze him. During the course of psychoanalysis, she not only listened to the child's free associations, she observed his play and considered that to be an equally valuable expression of the child's unconscious mind (Klein, 1955/1986). In The Psycho-Analysis of Children (1932/1963), she described the basics of the technique:

On a low table in my analytic room there are laid out a number of small toys of a primitive kind - little wooden men and women, carts, carriages, motor-cars, trains, animals, bricks and houses, as well as paper, scissors and pencils. Even a child that is usually inhibited in its play will at least glance at the toys or touch them, and will soon give me a first glimpse into its complexive life by the way in which it begins to play with them or lays them aside, or by its general attitude toward them. (pg. 40)

Klein believed that by watching children at play an analyst can gain a deep understanding of the psychodynamic processes taking place in the child's mind.

It is interesting to note that although Anna Freud often commented on Klein's work, Klein seldom mentioned Anna Freud. It may be that Anna Freud felt compelled to address the work of a leading figure whom Anna Freud considered to be incorrect, whereas Klein felt no such need to address the work of the younger Anna Freud. Klein certainly cited Sigmund Freud's work extensively, but when she mentioned Anna Freud she typically failed to give credit where credit is due. For example, in The Psycho-Analysis of Children(Klein, 1932/1963), she mentions Anna Freud only once, in the introduction to the book:

Anna Freud has been led by her findings in regard to the ego of the child to modify the classical technique, and has worked out her method of analysing children in the latency period quite independently of my procedure…In her opinion children do not develop a transference-neurosis, so that a fundamental condition for analytical treatment is absent…My observations have taught me that children can quite well produce a transference-neurosis, and that a transference-situation arises just as in the case of grown-up persons…Moreover, in so far as it does so without having recourse to any educational influence, analysis not only does not weaken the child's ego, but actually strengthens it. (pg. 18-19)

This quote not only emphasizes a fundamental disagreement between Klein and Anna Freud, it also seems to dismiss the value Anna Freud placed on her educational background. Later

in her career, Klein even went so far as to suggest that she herself was closer to Sigmund Freud's perspective than Anna Freud was:

I do not know Anna Freud's view about this aspect of Freud's work. But, as regards the question of auto-eroticism and narcissism, she seems only to have taken into account Freud's conclusion that an auto-erotic and a narcissistic stage precede object relations, and not to have allowed for the other possibilities implied in some of Freud's statements such as the ones I referred to above. This is one of the reasons why the divergence between Anna Freud's conception and my conception of early infancy is far greater than that between Freud's views, taken as a whole, and my view. (pg. 206; Klein, 1952/1986)

Clearly, whereas Anna Freud felt that Klein was reading too much into her analysis of children, Klein felt that Anna Freud had failed to consider the wider perspectives allowed by the work of Sigmund Freud. Given the complexity of individual personality, it may be that the true answer to this question is different for each person undergoing psychoanalysis.

Placing the Neo-Freudians in Context - 2:

The Psychoanalysis of Children

Before continuing our examination of object relations theorists, it is important to stop and ask why the psychoanalysis of children received so much attention. Many people think of early childhood as a carefree time to run and play, a time when our parents take care of every need, and we have no responsibilities at all. However, for many children, life holds much more challenging problems than just the normal psychological processes of growing up. Abuse, neglect, being caught in the middle of a bitter divorce, these are just some of the things that occur in the lives of too many children. In considering situations where society is forced to intervene, Anna Freud and her colleagues believed that we should shift our focus from thinking about the "best interests" of the child and think instead about providing the "least detrimental available alternative for safeguarding the child's growth and development" (Goldstein, Freud, & Solnit, 1973). Their reasoning was that in cases of abuse, neglect, divorce, etc., the "best interests" of the child are no longer possible, and certainly cannot be restored by a judge. Therefore, the best that society can hope to do is to help the child as much as possible. Obviously, psychotherapy may play an important role in this process for those children who are emotionally disturbed.

The question remains, however: at how early an age can psychoanalysis be effective? The answer depends somewhat on your perspective. As we have already seen, Anna Freud did not consider children capable of fully participating in psychoanalysis as adults can; she did not consider their play behavior to be the same thing as free association. Melanie Klein, however, did consider children to be good subjects for psychoanalysis at very early ages. In fact, Klein took it one step further: she practically considered psychoanalysis necessary for normal development! Klein's childhood was not easy. Her father seemed to care only for her sister Emilie, and Emilie and their brother Emmanuel constantly harassed Klein. Her closest sister in age, Sidonie, took pity on Klein and taught her arithmetic and how to read. However, when Klein was only 4 years old, both she and Sidonie came down with tuberculosis. Sidonie died, and her death was very traumatic for Klein. Klein suffered from depression throughout her life, and even spent some time in a hospital being treated for it during her 20s (Sayers, 1991; Segal, 2004). This may have had a lot to do with Klein's focus

on the death instinct and aggression during early childhood development. Her own descriptions of childhood can seem quite frightening:

We get to look upon the child's fear of being devoured, or cut up, or torn to pieces, or its terror of being surrounded and pursued by menacing figures, as a regular component of its mental life; and we know that the man-eating wolf, the fire-spewing dragon, and all the evil monsters out of myths and fairy stories flourish and exert their unconscious influence in the fantasy of each individual child, and it feels itself persecuted and threatened by those evil shapes. (pgs. 254-255; Klein, 1930/1973)

Not only are such early childhood challenges frightening for individuals, Klein also believed that all attempts to improve humanity as a whole have failed because no one has understood "the full depth and vigor" of the aggressive instincts in each person. Klein believed that psychoanalysis could help both individuals and all humanity by alleviating the anxiety caused by the hatred and fear that she proposed all children experience during their psychodynamic development (Klein, 1930/1973). And so, Klein expressed the following desire for psychoanalysis:

…I hope, child analysis will become as much a part of every person's upbringing as school education is now. Then, perhaps, that hostile attitude, springing from fear and suspicion, which is latent more or less strongly in each human being, and which intensifies a hundredfold in him every impulse of destruction, will give way to kindlier and more trustful feelings toward his fellow men, and people may inhabit the world together in greater peace and goodwill than they do now. (pgs. 267-268; Klein, 1930/1973).

Donald W. Winnicott

Anna Freud and Melanie Klein represent two extremes in the debate over the development of personality in childhood and how psychoanalysis can help to understand that development and treat psychological disorders. Anna Freud strictly adhered to her father's theory, believing that young children lacked the psychological development necessary for participating fully in adult-like psychoanalysis. Klein, on the other hand, considered children quite advanced at birth, with the death-instinct and its aggressive impulses being every bit as important as Eros and the libido. In contrast to these extremes, an independent school of object relations theorists developed with more moderate views. Donald Winnicott was one of the most influential of these more moderate theorists, as were Margaret Mahler and Heinz Kohut. We will take a look at some of the ideas of Mahler and Kohut in the next section.

Winnicott (1896-1971) was a pediatrician before becoming an analyst, so he brought a wealth of experience in observing mother-infant interactions to psychoanalysis. Already well respected for his medical treatment of children, Winnicott became increasingly interested in their emotional disorders. So, he joined a group of psychoanalysts being formed in London under the guidance of Sigmund Freud (Winnicott, Shepherd, & Davis, 1986). His first analyst was James Strachey, the man responsible for translating much of Freud's work into English and who was also instrumental in bringing Klein to England. Winnicott continued his analysis with Joan Riviere, one of Klein's closest colleagues, and he was eventually supervised by Klein herself (Mitchell & Black, 1995). Due to his prior experience and independent spirit, however, he developed his own theories separately from those of Klein.

Winnicott saw the early years of life as being a time when the child must transition from a state of **subjective omnipotence** toward one of **objective reality**. When a newborn is

hungry, the breast appears. When a newborn is cold, it is wrapped in a blanket and warmed. The baby believes that it has created these conditions through its own wishing, and so it feels omnipotent. The mother's responsibility during this time is to cater to the baby's every wish, to anticipate the needs of the child. As a result, the baby does indeed have its wishes granted almost immediately. This subjective sense of self, as an empowered individual, is crucial to the core of personality as the child grows and represents the **true self** (Kernberg, 2004; Mitchell & Black, 1995; Winnicott, 1967/1986).

For this development to proceed in a healthy manner, the child must have what Winnicott called a **good enough mother** (Winnicott, 1945/1996, 1968a,b/2002, 1968c/1986). The good enough mother at first fulfills the child's wishes immediately and completely, but then withdraws when not needed. This creates an environment in which the child is protected without realizing it is being protected. Over time, the mother slowly withdraws even from the immediate satisfaction of the child's needs. This allows the child to develop a sense of objective reality, the reality that the world does not immediately and completely satisfy anyone's desires and needs, and that wishing does not lead to satisfaction. So the good enough mother is not a perfect mother in the sense that she provides forever anything that the child wants. Instead, she does what is best for the development of the child, offering fulfillment and protection when needed, and withdrawing when the child must pursue its own development. Winnicott considered the unique condition of the good enough mother as something quite fascinating:

A good enough mother starts off with a high degree of adaptation to the baby's needs. That is what "good-enough" means, this tremendous capacity that mothers ordinarily have to give themselves over to identification with the baby…The mother is laying down the basis for the mental health of the baby, and more than health - fulfillment and richness, with all the dangers and conflicts that these bring, with all the awkwardnesses that belong to growth and development. (pg. 234; Winnicott, 1968b/2002)

A good enough mother satisfies the needs of her child, but withdraws when the child does not need her, eventually no longer being available to the child in an instant. Over time, this allows the child to develop a realistic sense of the world.

Discussion Question: Donald Winnicott believed that healthy development required a child to have a good enough mother. Do you think you had a good enough mother (or father), and do you agree with this approach to raising an infant?

There is at least one big problem with discussing how extraordinary the good enough mother is: it seems to ignore the role of the father. However, this was not the case. Although Winnicott emphasized the biological reality that the father does not share the same physiological relationship that the mother and child share, he did acknowledge that in the course of development the father plays an important role (Winnicott, 1968b/2002, 1968c/1986). Thus, in considering the overall development of the child, he acknowledges the role of good enough parents:

I must be careful. So easily in describing what very young children need I can seem to be wanting parents to be selfless angels, and expecting the world to be ideal…Of children, even of babies, it can be said that they do not do well on mechanical perfection. They need human beings around them who both succeed and fail.

I like to use the words “good enough.” Good enough parents can be used by babies and young children, and good enough means you and me. In order to be consistent, and so to be predictable for our children, we must be ourselves. If we are ourselves our children can get to know us. Certainly if we are acting a part we shall be found out when we get caught without our make-up. (pg. 179; Winnicott, 1969/2002)

Objective reality is not, however, the goal of development. It is just as extreme as subjective omnipotence. While it is true that wishing does not lead to satisfaction, it is also true that loved ones will help to satisfy our needs and desires to the best of their ability. An individual living entirely in the realm of objective reality lacks the subjective core of their true self and cannot connect with others. Instead, they live in expectation of what others will do, influenced entirely by external stimuli (Mitchell & Black, 1995). Such individuals develop what is called a **false self disorder** (Winnicott, 1964/1986, 1967/1986, 1971). Although Winnicott described the false self as a successful defense, within the context of ongoing development, he did not consider it to be a condition of psychological good health (Winnicott, 1964/1986, 1967/1986). However, it is part of normal development in every person’s life. For example, children are taught to say “thank you” even when they may not be thankful for something. We actually teach them to lie, as part of the price for socialization. However, some children find it difficult because of the need to continually re-establish the importance of the true self relative to the false self (Winnicott, 1964).

For Winnicott, the process of transitioning from subjective omnipotence toward objective reality is crucial to development. The **transitional experience** is not just a concept, however, since it often involves **transitional objects**. A child’s blanket, or a teddy bear, is very important to the child. They do not exist merely as a substitute for the mother, they are also an extension of the child’s own self. This allows the child to experience a world that is neither entirely within its control nor entirely beyond its control (Kernberg, 2004; Mitchell & Black, 1995). Such a world is closer to the condition in which most of us actually live, and fits well with Winnicott’s definition of the good enough parent: one who is honest and real in dealing with their children. The relationship between the child and its mother, as well as the relationships between the child and its larger family, are actively involved in this transitional experience. There is an intimate connection between a mother and a child when they are playing, and that connection exists in a common ground: the transitional space that is neither child nor mother. Because this is a shared and secret place, it is a symbol of the trust and union between them. Taken further, this space becomes an opportunity for the child to see itself **mirrored**in the mother’s face. If the mother is loving and supportive, the child is able to develop a sense of feeling real (Winnicott, 1968a/2002). Although the relationship with the mother may be the most special, these phenomena do carry over to the father and the rest of the family as well (Winnicott, 1966/2002).

Discussion Question: Winnicott felt that transitional objects were important for helping children to develop without too much anxiety. Did you have a favorite transitional object, and do you still have it? Do you think it is healthy for children to have such objects, and what might you do with your own children if you have them? If you already have children, do they have transitional objects, and did you ever consciously expect them to have them?

Winnicott proposed that the transition that occurs during early development, from subjective omnipotence to objective reality, is facilitated by transitional objects. In the picture on the left, John is cuddling his blanket. To the right is John's other important transitional object, his gorilla HaHas, and the author's old Teddy bear. [Images © 2010 Mark Kelland]	

For Winnicott, the psychoanalytic process was an opportunity for the patient to re-experience the early subjective experiences of a relationship with the good enough mother. The therapist takes the role of the good enough mother, allowing the patient to spontaneously "be" in the relationship, while the analyst tries to anticipate and accommodate the patient's needs. The hope is that the analyst and the therapeutic environment will allow the patient's aborted development to be reanimated, with the patient's true self emerging as a result (Mitchell & Black, 1995). However, there can be no single technique in this process, as each case is different (Winnicott, 1971). More important than technique is the analyst's overall skill as an analyst, their ability to make use of various techniques within the psychoanalytic session. Perhaps the most important aspect of this overall view of what is necessary for effective psychoanalysis, according to Winnicott, is that the analyst needs to have been a good, healthy candidate in the first place. Winnicott believed that "it is not easy to turn a badly selected candidate into a good analyst…" (Winnicott, 1971).

Although Winnicott may have felt that technique was not some special trick to be used by anyone in performing psychoanalysis, he did have some favorite techniques. As described above, he watched the playful interaction between child and mother, in much the same way as Klein used her play technique. Winnicott also liked to use the **Squiggle Game**, a technique that makes use of drawings by the child and the analyst, including the opportunity for each to make changes in the other's drawings. Winnicott believed that this process provided a special opportunity to make contact with the child, in which it felt to him as if the child were alongside him helping to describe the case (Winnicott, 1971). In Therapeutic Consultations in Child Psychiatry, Winnicott (1971) offers many examples of such drawings along with brief descriptions and analyses of the corresponding cases.

In closing, Winnicott felt it was important to focus on psychological health, and he defined this as something much more than simply making it through each day, going to work, and raising a family. He believed that healthy individuals actually lived three different lives: 1) a life in the world, with interpersonal relationships being key; 2) a personal psychic reality, including creativity and dreams; and 3) their cultural experience. Winnicott admitted that it was difficult to incorporate the cultural experience into the life of an individual. However, he favored the transitional space between the child and its mother, and felt that it was dependent on the mother having been very supportive of the child during development (Winnicott, 1967/1986). In considering the overall purpose of life, in contrast to Freud's perspective, Winnicott wrote:

…What is life about? I do not need to know the answer, but we can agree that it is more nearly about BEING than about sex…Being and feeling real belong essentially to health, and it is only if we can take being for granted that we can get on to the more positive things…the vast majority of people take feeling real for granted, but at what cost? To what extent are they denying a fact, namely, that there could be a danger for them of feeling unreal, of

feeling possessed, of feeling they are not themselves, of falling for ever, of having no orientation, of being detached from their bodies, of being annihilated, of being nothing, nowhere? Health is not associated with denial of anything. (pgs. 34-35; Winnicott, 1967/1986)

The Final Development of Individuality: Margaret Mahler and Heinz Kohut

Margaret Mahler (1897-1985), was also a pediatrician before becoming a child analyst, and the early relationship between a child and its mother had a significant impact on her views of developmental ego psychology. At birth, according to Mahler, a child is focused entirely on itself, in a state of **primary narcissism**known as the **normal autistic phase**. In agreement with Sigmund Freud, Mahler believed that in the first few weeks of life there is very little cathexis of libido outside of the child itself. She borrowed Freud's analogy of a bird's egg to describe this period in which the child has minimal interaction with external stimuli. Through contact with the mother, however, the child slowly becomes aware that it cannot satisfy its needs by itself. As the child becomes dimly aware of the mother's activities, the child begins to think of itself and its mother as an inseparable system. This intimate connection between child and mother is called **normal symbiosis** (Kernberg, 2004; Mahler, Pine, & Bergman, 1975; Mitchell & Black, 1995). As important as this stage is for the development of the child, the child still needs to develop a sense of individuality. That process is known as **separation-individuation**:

We refer to the psychological birth of the individual as the separation-individuation process: the establishment of a sense of separateness from, and relation to, a world of reality, particularly with regard to the experiences of one's own body and to the principal representative of the world as the infant experiences it, the primary love object. Like any intrapsychic process, this one reverberates throughout the life cycle. It is never finished; it remains always active…(pg. 3; Mahler, Pine, & Bergman, 1975)

Separation-individuation, therefore, refers to the two main tasks that a young child must accomplish in order to grow up. First, they must separate from their mother (including the psychological understanding that they and their mother are two separate beings), and then they must fully develop their individuality. According to Mahler, this process involves a series of four subphases: **differentiation, practicing, rapprochement**, and **consolidation**. The earliest subphase, differentiation, is signaled by the child's increasing alertness around the age of 4 to 5 months. Following a "hatching process", the child directs much of its attention outward, but this alternates with the child often turning back to the mother as its point of orientation. Transitional objects, as described by Winnicott, are also important during this period. As the child becomes old enough to start crawling, it moves out into the world and begins practicing its ability to interact with the environment. The practicing subphase enters full force as the child begins to walk, and an important aspect of this is a full, physical understanding of the child's separateness from its mother. Accordingly, its interests can now spill over into the many toys and other objects the child discovers in the world (Kernberg, 2004; Mahler, Pine, & Bergman, 1975; Mitchell & Black, 1995).

During the rapprochement subphase (approximately 1 ½ to 2 years of age), the child's psychological development catches up with its physical development, and the child potentially enters a state of confusion and anxiety. The child becomes aware that the mobility it gained during the practicing subphase has had the unfortunate effect of truly, and

physically, separating the child from its mother. The distress this causes leads the child to regularly check in with its mother for security. This is a progressive stage, but is often seen as a regression by the parents (Mitchell & Black, 1995). Pushing the child away too early at this stage can lead to psychological problems later in life, and Mahler urged that one cannot emphasize too strongly the importance of the mother providing optimal emotional availability to the child (Mahler, Pine, & Bergman, 1975). If all goes well, the child will then enter the final subphase and consolidate a definite, and in some aspects lifelong, individuality. Mahler believed that this process indicated a far-reaching structuralization of the ego and definite signs that the child has internalized parental demands, an indication that the superego has developed as well (Mahler, Pine, & Bergman, 1975).

Louise Kaplan, who worked with Mahler for a time, was interested in applying the theory Mahler had developed to the full range of human life, both in terms of age and cultural differences. Although Kaplan agreed that the most profound development occurs during early childhood, she emphasized that the purpose of all this, from the point of view of society, is what sort of person will grow out of each child.

In the first three years of life every human being undergoes yet a second birth, in which he is born as a psychological being possessing selfhood and separate identity. The quality of self an infant achieves in those crucial three years will profoundly affect all of his subsequent existence. (pg. 15; Kaplan, 1978)

The conditions of these early years, however, are not always good. In many cultures women are oppressed, sometimes violently. This has an effect on the mothering these women are able to provide their children. Male children may be valued, but in a possessive way. Female children may be scorned, as they lack the male privileges the mother wishes she had herself (Kaplan, 1978). Of course, not all cultures are like this. Kaplan describes a wide variety of cultures, both primitive and modern, and considers some of the many factors that contribute to the nature of adulthood. She compared hunter/gatherer cultures such as the Zhun/twasi or the Ik, tribes found in southern Africa, as they are compelled to transition from old ways of life toward more modern ways. The increased aggressiveness and general life stress that Kaplan observed coinciding with these changes in culture suggests to her that our modern way of life has led to many of these psychological problems. She believed that in “every adult human there still lives a helpless child who is afraid of aloneness.” When social conditions are competitive and/or abusive, adults are as alone and helpless as children. According to Kaplan, this would be true even if there were perfect babies and perfect mothers (Kaplan, 1978).

Discussion Question: Mahler believed that children develop through three stages. First the child focuses on itself, then the child becomes aware of their intimate relationship with their mother, and finally a sense of individuality develops. If you look at your relationship with your parents, which stage seems more dominant: your narcissism, your symbiosis, or your separation-individuation?

Heinz Kohut (1913-1981) continued and expanded on this perspective of the important and revealing relationship between childhood development and the life and psychological health (or not) of adults. Kohut was born in Vienna, and studied medicine at the University of Vienna, as Sigmund Freud had. Also similar to Freud, he took some time to study medicine in Paris. In 1937, Kohut’s father died and he was deeply troubled. He first went to a psychologist for treatment, but later sought psychoanalysis from August Aichhorn. Aichhorn

was a highly respected analyst, and a close personal friend of both Sigmund and Anna Freud. The success of his analysis greatly interested Kohut himself, and led to his becoming an analyst as well. After fleeing Nazi controlled Austria in 1939, Kohut eventually settled in America. He continued his psychoanalytic training at the Chicago Institute for Psychoanalysis (where Karen Horney had been the first associate director), but not without difficulty. Initially, Kohut was soundly rejected by the institute. He then entered into therapy with Ruth Eissler, a training and supervising analyst at the institute, and the wife of a protégé of the well-respected Aichhorn. Making these connections was an intentional effort at good networking, and Kohut was later accepted into training (Strozier, 2001).

In his theory, Kohut focused on the **self**and **narcissism**. Most theorists express a negative view of narcissism, but Kohut felt it served an essential role in the development of individuality. Early childhood is a time of vitality, children are exuberant, expansive, and creative. Kohut was interested in the fate of this vitality, and how it can be preserved into adulthood (Mitchell & Black, 1995). The development of a healthy self depends on three kinds of **selfobject**experiences. Selfobjects are the adults who care for the child, and they need to provide for both physiological and psychological needs. First, a child needs selfobjects who confirm the child's vitality, who look on the child with joy and approval. In this first basic narcissistic process, known as **mirroring**, the child is able to see itself as wonderful through the eyes of others. An important aspect of mirroring is empathy, a state in which the mother and child actually share their feelings as if they were one (Strozier, 2001). The second type of selfobject satisfies the child's need to be involved with powerful others, people the child can look up to as images of calmness, control, and omnipotence. This second basic narcissistic process, known as **idealizing**, allows the child to experience the wonder of others, and to consider itself special due to its relationship with them. Finally, the child needs to experience others who are open and similar to the child, allowing the child to sense an essential likeness between the child and the selfobject. Although this was not described as a basic narcissistic process, its lack of development can be seen in the twinship transference described below. These various relationships will help the child to develop a healthy narcissism, a realistic sense of self-esteem. Although reality will begin to chip away at this narcissism, in a healthy environment the child will survive the occasional frustration and disappointment and develop a secure, resilient self that maintains some kernel of the vitality of early childhood into adulthood (Mitchell & Black, 1995).

An important question, however, is how are the selfobjects incorporated into the child's sense of self? As suggested above, mirroring is the first important step. As the child observes the mother's joy and approval of the child, the child comes to believe that it must be wonderful. Why else would the mother be so happy to see the child? Similarly, as the child observes selfobjects that are powerful and calm, those selfobjects the child has idealized, the child projects the best part of itself onto those selfobjects. Accordingly, the child sees those selfobjects as wonderful and, since the child is with them, the child must be wonderful too. In these instances the child strengthens its own sense of self, its own narcissism, in comparison to others. These processes can be seen in the psychoanalytic session with patients who have not developed a healthy sense of self. They will exhibit three types of selfobject transference toward the analyst: **mirroring transference, idealizing transference**, and **twinship transference**. In mirroring transference, the attention of the analyst allows the patient to feel more real and more internally substantial. In idealizing transference, the patient comes to believe that the analyst is an important and powerful

person, and the patient is to be valued by virtue of their association with the analyst. And finally, in twinship transference, the patient feels as if they are a companion to the analyst in the process of therapy (Mitchell & Black, 1995; Strozier, 2001).

Discussion Question: Heinz Kohut also considered a degree of narcissism to be necessary for a child to develop a sense of individuality. Are you more likely to choose friends who admire you (mirroring), or whom you admire (idealizing)? Or do you choose friends who are similar to you, and who help you to develop a realistic sense of self (twinship)? In each instance, is your choice an overwhelming desire, or just one aspect of choosing your friends?

Late in his career Kohut turned his attention to a topic that had also captured Sigmund Freud's attention late in his career: God and religion. Kohut felt that Freud had made a crucial error in evaluating religion. Freud believed that religion would be undone by the study of science, but Kohut felt that it was simply wrong to try evaluating religion in a scientific way. He did not consider God to be an internalized image of the frightening and all-powerful father, but rather an internalization of the earliest and most wonderful relationship in life: the love of a mother (Strozier, 2001). In keeping with his basic theory, he tried to outline the precise psychological needs that were being satisfied by religion. Most importantly, there is something uplifting about religion. The mirroring need is typically referred to as grace, the gifts freely given to us by God, something psychologically similar to the love shown by a mother holding and cuddling her beloved child. God is, of course, the ultimate in idealization, a perfect being, all-knowing and all-powerful. While an immense and ornate cathedral or temple may seem awesome to those who are religious, other spiritual people can be similarly impressed looking down from a mountaintop, walking along the ocean shore, or listening to beautiful music. As for the final selfobject need, twinship, one can easily relate the community of a religious congregation. Perhaps it is no coincidence that we often hear priests and ministers talking about a congregation as the children of God. Although it was never quite clear what Kohut's own religious or spiritual beliefs were, he did write:

There is something about this world in our experience that does lift us up beyond the simplicity of an individual existence, that lifts us into something higher, enduring, or, as I would rather say, timeless. (pg. 332; quoted in Strozier, 2001)

Connections Across Cultures: Cultural Perspectives

on Parent-Child Attachment

This is a true story. I was at our local gym while my older son was at gymnastics practice. There were some children attending a party at the gym, including a little boy about 2 years old who was running around on one of the gymnastics floors. He fell down and hurt himself, and he started crying. A couple of the coaches walked over to help him, but he just cried louder and roughly turned away from them. Then he heard his mother calling him. He ran over to his mother, crying all the way, and she scooped him up into her arms. Almost immediately he stopped crying, started squirming around, and when she put him down he raced back onto the floor and started running wildly in circles and yelling for joy! This is a marvelous example of what psychologists call a secure attachment.

Attachment theory was developed by John Bowlby and advanced by Mary Ainsworth (see Jarvis, 2004; Mitchell & Black, 1995; Rothbaum, Weisz, Pott, Miyake, & Morelli, 2000). Bowlby considered attachment theory to fit within an object relations approach to

psychodynamic theory, but it was largely rejected by the psychodynamic community. He proposed an evolutionary basis for attachment, a basis that serves the species by aiding in the survival of the infant. In other words, the attachment between an infant and its primary caregivers helps to ensure both that the infant stays close to the parents (the objects, if we consider object relations theory) and the parents respond quickly and appropriately to the needs of the infant. Ainsworth studied the attachment styles of children using a technique called the **strange situation**. In the strange situation, one of the caregivers (let's say the mother) takes a child into an unfamiliar playroom, and allows the child to explore. A stranger enters, interacts with the mother, and then tries to interact with the child. The mother leaves, then returns, the stranger leaves, and then the mother leaves again. The stranger then returns, then leaves, and finally the mother returns. Throughout all of these events, the child is observed for evidence of having a secure base (feeling comfortable enough to explore the unfamiliar room), separation anxiety (due to the absence of the mother), stranger anxiety (due to the presence of the stranger), and, finally, for its attachment to its mother (when the mother returns at the end of the experiment) (Jarvis, 2004). A securely attached child, as in the story above, will feel free to explore a new environment. When hurt or frightened, however, the child will seek its mother for protection and comfort. Having found that comfort, having affirmed its secure base, the child will then venture out again. But is this true for children in all cultures?

It has been suggested that attachment theory and interpretations of the strange situation are embedded in Western perspectives and ideals, particularly those of middle-class White Americans. In particular, a secure attachment seems to promote the independence of the child, and its ability to separate from the mother and move out into the world. One of the key measures of a secure attachment is that child is comforted by the presence of its mother, particularly after the child has been in the presence of strangers. However, numerous cultural problems arise from these perspectives. For example, in many African American households children are raised by different members of an extended family, possible including individuals who are not related to the family. Thus, African American children raised in such an environment may respond quite differently to the strange situation, it may not be novel to them (Belgrave & Allison, 2006). As mentioned briefly in Chapter 1, Kenneth and Mamie Clark were two very important individuals who studied the development of African American children. Respectively, they were the first African American man and African American woman to receive Ph.D. degrees in psychology. In addition to studying racial identification in African American children during the 1940s (Clark & Clark, 1947), they established what became the Northside Center for Child Development in Harlem, New York. Primarily under Mamie Clark's guidance, the center provided a broad range of psychological services including consultations for behavioral and emotional problems, vocational guidance for adolescents, and child-rearing education for African American parents. In addition, the center provided the same services for a smaller number of White and Puerto Rican children from working-class families in Harlem. Mamie Clark's goal was to give the children of Harlem the same sense of emotional security that she had enjoyed as a child, a sense of security that was elusive in the poor neighborhoods of Harlem (Lal, 2002).

Rothbaum et al. (2000) compared American perspectives on attachment to those in Japan, a country with similar socioeconomic conditions but a very different history and culture. Attachment theory has been considered to have three, universal core hypotheses: sensitivity, competence, and the secure base. In order for a child to feel secure, the mother must

respond quickly and appropriately when the child perceives a threat. In other words, she must be sensitive to the child's needs. When a child feels secure, and has a secure relationship with its primary caregivers, attachment theory predicts that the child will grow up socially and emotionally competent. And finally, the secure base is intimately linked with the child's exploration of the environment and the child's ability to respond appropriately to environmental stimuli.

If we compare Japan to the United States, and how we define each of the factors listed above, we come to very different conclusions. According to Rothbaum et al. (2000), so-called sensitive parents in the United States emphasize the child's autonomy. They expect their children to explore the environment, and they wait for their children to express their needs before responding. In Japan, however, mothers try to anticipate their children's needs, and they promote the child's dependence on its mother. In Japan, mothers emphasize emotion and social factors, as opposed to communication and physical objects. Similar differences are seen with regard to social competence. An American who grows up socially competent (assumed to be the result of secure attachments in childhood) is expected to be independent and self-sufficient, willing to express and defend their own opinions. In Japan, however, as in all typical collectivist cultures, a socially competent adult is expected to be dependent on the social in-group and emotionally restrained (Rothbaum et al., 2000). With regard to the secure base, in the United States it is expected to encourage the child's autonomy, exploration, and general orientation to the environment first. In contrast, Japanese children are encouraged to focus more on their mothers, in both distressing situations and in those involving positive emotions. Since the expectations of each aspect of attachment theory are so different in Japan and the United States, which are assumed to be representative of Western and Eastern societies, Rothbaum et al. (2000) question whether attachment theory itself is truly universal. They do not question that children and their parents form important and deeply meaningful attachments, but they do question whether attachment can be reasonably evaluated the same way in all cultures.

There are other researchers, however, who question whether the perspectives of Rothbaum et al. (2000) justify rejecting the universality of attachment theory. For example, Posada and Jacobs (2001) acknowledge differences in behavior among different cultures, but they emphasize that all children have the potential for developing secure base relations with their parents and the subsequent secure attachments. It is important to keep in mind that Bowlby's theory was originally proposed in an evolutionary context and humans are, after all, primates. Also, Ainsworth first coined the term secure base relationship after studying a rural, African community in Uganda, not in a Western culture (Posada and Jacobs, 2001). Rothbaum et al. (2000) also suggest that the relationship between Japanese mothers and their children is better expressed by **amae**, a dependence on and presumption of another's love. Amae has been described as what a child feels when seeking his or her mother (consider the child in the story at the beginning of this section, as he ran crying to his mother). However, when the question is asked in the right way, Japanese mothers would prefer their children to fit a definition of a secure child as opposed to one experiencing amae (van IJzendoorn and Sagi, 2001). Indeed, the very meaning of amae is not clearly understood, and may not be easily compared to behaviors recognized in Western cultures (Gjerde, 2001). It may also be true that insecure relationships may be more adaptive in some cultures than secure attachments, and our misunderstanding of these concepts does not

allow us to conclude which perspective on attachment theory, if any, should be preferred (Kondo-Ikemura, 2001).

Finally, since attachment problems do sometimes arise, and since attachment must be defined within a relational context, is an individual therapy such as psychoanalysis the best course? If family therapy might be a better option in some circumstances, is anything being done to address cultural issues there? Fortunately, the answer is yes. Psychologists have begun comparing and contrasting family therapy in such diverse cultures as Japan, Israel, and the U.S. Virgin Islands (Dudley-Grant, 2001; Halpern, 2001; Kameguchi & Murphy-Shigematsu, 2001; see also Kaslow, 2001). In keeping with the hopeful sentiments that Melanie Klein expressed regarding child psychoanalysis, Kaslow (2001) believes that family psychology has a role to play “in undertaking the challenges of working with and for families in creating a healthier, more peaceful, less violent world for all.”

A Contemporary Perspective: Otto Kernberg

Otto Kernberg (1928-present) is one of the leading figures in psychodynamic theory today. Kernberg has focused on two major paths: trying to integrate the various psychodynamic, ego psychology, and object relations theories into a unified perspective and trying to provide a research-based methodology for the treatment of patients, particularly patients with **borderline personality organization** (a pathological identity formation that includes all of the major personality disorders; Kernberg, 2004, Kernberg & Caligor, 2005).

Despite seemingly significant differences between Freud’s classical theory and the theories of the neo-Freudians we have examined above (as well as others we have not looked at), Kernberg has done an admirable job of bringing the theories into a cohesive framework. This was accomplished by setting up a hierarchical series of developmental levels at which failure to develop normally causes characteristic types of disorders, whereas successful development leads to a healthy individual. In contrast to Freud, Kernberg believes that an infant begins life as an emotional being unable to separate its own reality from others around it. As the child experiences object relations in this first stage of development, those emotions develop into the drives described by Freud: pleasant emotions lead to libidinal drives and unpleasant emotions lead to aggressive drives. During the second stage of development, the child’s continued development in relation to others leads to an understanding that objects can be both negative and positive (the process of splitting described by Klein), and this leads to a reduction in the intensity of love and hate toward those objects. In other words, the child can love flawed individuals, since the child does not need to completely love or completely hate the important objects in their life. In simple terms, according to Kernberg, individuals who fail to accomplish the first stage of development, an understanding that they are separate from others, develop psychotic disorders. Individuals who fail to accomplish the splitting necessary in the second stage of development will develop borderline disorders, characterized by an exaggerated fixation on “bad” self and object representations (Kernberg, 2004). Completing these first two stages does not end the process, however, because the third level is the one described by Freud himself: the developmental stage in which unconscious id (emotional) impulses threaten the individual’s sense of what is good and acceptable behavior. Thus, classic neurotic disorders still potentially face those who have moved beyond the more severe psychological pathologies of psychotic and borderline conditions (Kernberg, 2004; Mitchell & Black,

1995). We will examine Kernberg's theory in more detail at the end of the chapter, where we will examine his psychoanalytic theory of personality disorders.

In 2004, Kernberg published an excellent book entitled Contemporary Controversies in Psychoanalytic Theory, Techniques, and Their Applications. In this chapter we have seen that many disagreements arose between neo-Freudian theorists, and at first glance their theories seem to disagree more than they agree. Kernberg, however, has this to say:

Psychoanalytic object relations theories constitute so broad a spectrum of approaches that it might be said that psychoanalysis itself, by its very nature, is an object relations theory: all psychoanalytic theorizing deals, after all, with the impact of early object relations on the genesis of unconscious conflict, the development of psychic structure, and the re-actualization or enactments of past pathogenic internalized object relations in transference developments in the current psychoanalytic situation. (pg. 26; Kernberg, 2004)

He offers an excellent summary of the basic elements of theorists we have examined (Klein, Winnicott, Sullivan, Mahler), as well as some we haven't (Fairbairn, Jacobson), and how their theories can be blended with classical Freudian psychoanalytic theory. He then examines how psychoanalysts today are addressing a wide variety of unresolved topics, including: Freud's dual-drive theory (libido and aggression), homosexuality and bisexuality, mourning and depression, social violence, and the resistance among many in the field of psychoanalysis to improved research and changes in psychoanalytic education and training (Kernberg, 2004).

In the final chapter of his book on Contemporary Controversies…, Kernberg examines the historical progression of psychoanalytic thought in English speaking countries (the so-called English schools). The "controversial discussions" of the 1940s led to a mutual agreement to disagree among three major lines of thought: the ego psychologists following Anna Freud, the object relations theorists following Melanie Klein, and the independent school that included D. W. Winnicott. Although the result of these discussion was to delineate the differences among these approaches, over time practicing psychoanalysts recognized the limitations of each approach (Kernberg, 2004). So, many theorists and clinicians began bringing together those elements of each approach that were most valuable. Along the way came some very different perspectives, such as those of Kohut and his self psychology and the culturalist views of Sullivan, and the field was changed dramatically. Kernberg also contrasts these developments to those within the French school of psychoanalysis, a somewhat more traditional approach that emphasizes psychoanalytic method over technique (Kernberg, 2004). He concludes by suggesting that the future of psychoanalytic thought may be a blending of the English and French schools (Kernberg, 2004). One notable early French psychoanalyst was Princess Marie Bonaparte, a personal friend of Sigmund and Anna Freud. We will briefly look at her contributions to psychoanalytic theory in a later chapter.

Personality Theory in Real Life: Kernberg's Psychoanalytic

Theory of Personality Disorders

Otto Kernberg is one of the leading figures advocating a psychoanalytic theory of personality disorders, particularly within an object relations perspective (see Kernberg & Caligor, 2005). In important ways he has followed the model of Sigmund Freud, in that he has based much of this theory on experience psychoanalyzing patients. His theory has been developed in conjunction with the therapeutic approach that grew out of both that experience and his

developing theory. Thus, Kernberg's work represents an applied approach to the study of personality disorders.

Kernberg's model of personality disorder emphasizes personality structures, which are derived from the interaction of constitutional (i.e., temperamental) and environmental factors during early childhood. These structures are relatively stable mental functions or processes that serve to organize an individual's behavior and subjective experiences. Psychological structures that are conscious and observable are typically referred to as **"surface" structures**, whereas those that are primarily unconscious are called **"deep" structures**. The basic building blocks of these personality structures are internalized object relations. Internalized object relations are particular emotional states linked to a specific image of a particular relationship (e.g., anxiety linked with an image of a confused and unsure self and a critical, judgmental parent). These internal object relations are integrated and hierarchically organized into the higher-order structures that form the personality. At the core of this personality organization is the individual's **"identity."** According to Kernberg and Caligor (2005), a healthy, consolidated identity corresponds with a stable and realistic sense of self and others. In contrast, a pathological identity stems from an unstable, polarized, and unrealistic sense of self and others. This pathology arises because the emotional states of the internalized object relations are predominately negative; they are crude, intense, and poorly modulated. There is also a preponderance of aggression and defensive mechanisms based on primitive dissociation (splitting).

A normal personality, according to Kernberg, is characterized by an integrated concept of self and an integrated concept of significant others. Individuals with a normal personality can express a wide range of emotions, and even intense emotions do not lead to a loss of impulse control. Normal individuals have an integrated and mature system of internalized values, and they can appropriately manage their sexual, dependent, and aggressive motivations. The development of the normal personality depends to a large extent on the relationship the child has with its mother (the primary caregiver). If the mother is successful in helping the child transform highly emotional states into integrated experiences, then the child's internalized object relations will be primarily positive. Erik Erikson first proposed this concept, and he believed that ego identity was not complete until adolescence (see Kernberg & Caligor, 2005).

The abnormal personality, in contrast, results from early childhood interactions with caregivers who do not help to transform highly emotional states into integrated experiences. On the contrary, caregiver's failure to help the child integrate its emotional experiences can intensify the child's anger and anxiety, perhaps leading to an increase in aggression. Consequently, a child who cannot integrate the good and bad aspects of its emotional states and object relations will come to depend on defense mechanisms that enhance continued splitting. The child becomes fixated at a poorly integrated level of development. One of the most important implications of this approach to personality disorders is the role that primary caregivers play. What constitutes a parent, or other caregiver, who cannot help the child to integrate its emotional states in a healthy, normal manner? Numerous studies in different countries have identified a high rate of physical and sexual abuse in patients with borderline disorders.

The level at which integration fails to occur results in the nature of the pathological personality organization. A psychotic personality organization occurs when the individual

has not integrated a clear concept of self and significant others. They do not have a clear identity formation, they may not distinguish intrapsychic processes from external stimuli, and therefore there may be a lack of reality testing. According to Kernberg, psychotic personality organization represents an atypical form of psychosis (Kernberg & Caligor, 2005). Borderline personality organization is similar, but less severe. The individual has achieved a level of integration in which the self and others are seen as separate, but that integration is pathological. There is a great deal of defensive splitting, and the individual lacks a clearly developed set of internalized values (the superego). Emotion fluctuates from intense to superficial, emotion is generally negative, and there may be excessive aggression. There is also a lack of integration of the sense of others, making relationships particularly unstable and unrealistic. Curiously, achieving an integrated sense of self and others does not resolve all psychological problems associated with personality development. Even if an individual has moved beyond the stages of psychotic and borderline personality development, and developed an integrated sense of self and others, they may still be prone to neurotic personality organization. At this level, defenses are based primarily on repression and stable reality testing. This is the level at which Sigmund Freud studied personality and psychological disorders, according to Kernberg (2004).

With this perspective in mind, how then do we treat individuals with personality disorders? Kernberg's primary focus is an exploration of the patient's internal object relations. These pathological internal object relations play out in the patient's current interpersonal relationships, and through the process of transference they play out in the therapeutic relationship as well. In patients with neurotic personality disorganization these transferences are relatively stable and understandable within a psychoanalytic context. With more severe personality organizations, the transferences are poorly organized, unstable, unrealistic, and the activation of the internal object relations can be immediate and chaotic. There is also a rapid interchange between the roles played by the patient with regard to which object of an internal object relation they identify with, and, accordingly, which object they attribute as the analyst. With severe borderline patients special attention must always be paid to the strong tendency toward acting out, which can lead to suicide attempts, drug abuse, self-mutilation, and other aggressive behavior (Kernberg & Caligor, 2005).

Therapy with these patients is a long and difficult task. In order for psychoanalysis to be successful, it may require multiple sessions each week for 4 to 6 years. This is necessary because the goal is no less ambitious than modifying personality organization and the quality of the patient's internal object relations, all of which were laid down during the formative years of infancy and early childhood (Kernberg & Caligor, 2005). Within the context of this theory, the outcome for individuals with a neurotic personality organization is hopeful, whereas the outcome for those with a borderline personality organization remains challenging. Kernberg and Caligor (2005) do not propose an approach to the treatment of patients with a psychotic personality organization. Therefore, continued research and clinical application will be necessary if we hope to be able to treat all patients suffering from personality disorders.

Review of Key Points

• The neo-Freudians, also known as ego psychologists, remained true to much of Freud's original theory, but they shifted their focus from the id to the ego.

• Anna Freud began her career as a teacher. This concern for children continued throughout her career, much of which was spent caring for children at the Hampstead War Nursery (which later became the Hampstead Clinic, and is now called the Anna Freud Centre, as a tribute to the career of Anna Freud).

• Anna Freud established ego psychology, believing that it made more sense to focus on the conscious ego, which we can observe directly, than on the unconscious id.

• As her father had, Anna Freud described defense mechanisms as the means by which the ego avoids the anxiety associated with being unable to meet the demands of the id or the constraints of the superego.

• There are a wide variety of defense mechanisms, which are generally oriented toward protecting us from internal threats (id impulses) or external threats (the rules and expectations of society).

• Anna Freud was one of the first psychoanalysts to work with children, and she used the same basic approach that she did with adults.

• Later in her career, Anna Freud accepted that even very young children (under the age of 6) could benefit from psychoanalysis, but she insisted that they could not cooperate with the psychoanalyst in the same way as an adult.

• When working with children, Anna Freud felt it was important to fill in gaps that the child could not (such as connecting manifest and latent content in dreams). She also believed that the psychoanalyst should help to educate the child with regard to his/her relationships.

• Object relations theory emphasizes that children are born with the capacity and drive to relate to others.

• Since all aspects of relationships have importance, Melanie Klein proposed that the death-instinct and aggression are just as important as the life-instinct (Eros) and libido.

• Splitting is an important process that involves recognizing the good and bad aspects of objects.

• Klein proposed that an infant goes through two developmental orientations: the paranoid-schizoid position later develops into the depressive position.

• One of Klein’s major contributions was the method of play analysis. She felt that observing the play of children could reveal as much unconscious material as free association by adults.

• Anna Freud and Melanie Klein disagreed about how fully the child could be psychoanalyzed, and how young they could be during psychoanalysis.

• According to Winnicott, a child must transition from a state of subjective omnipotence toward one of objective reality.

• In order for a child to develop a healthy personality and realize their true self, according to Winnicott, the child must have a good enough mother (and good enough parents).

• When development does not follow a healthy path, children can develop a false self disorder.

• The transitional experience that children must go through may be facilitated by transitional objects (such as a blanket or teddy bear).

• For Winnicott, the primary purpose of therapy is to provide an opportunity for the patient to re-experience the relationship of a good enough mother.

• Winnicott was an advocate of the Squiggle Game, a therapeutic technique that allows children to draw pictures to represent their thoughts and feelings.

• Mahler believed that children begin life in a state of primary narcissism known as the normal autistic phase. As they become aware of their mother they enter into normal symbiosis.

• In order for the child to develop a sense of individuality, according to Mahler, the child must go through a process known as separation-individuation.

• Louise Kaplan, a student and colleague of Mahler, suggested that much of the stress experienced during development is the result of societal changes, and that modern cultures exacerbate this stress.

• Kohut believed that a certain measure of narcissism was necessary for the development of individuality.

• According to Kohut, children need several types of selfobjects. They will see themselves mirrored in the eyes of others, they will idealize others, and they will develop a realistic sense of self-esteem through relationships with others.

• During psychoanalysis, the analyst can provide each of these types of relationships through mirroring transference, idealizing transference, and twinship transference. This allows the patient to feel more real and more substantial.

• Kohut questioned Freud’s rejection of religion. He offered a point of view in which religion fulfills a variety of basic psychological needs for people.

• Otto Kernberg has offered a perspective that blends all of the neo-Freudian and object relations theories together, suggesting that they represent a continuum of stages in human development.

CHAPTER 05 / CAC.902

Adler and Sullivan: Belonging, Goals, and Interpersonal Life

Alfred Adler and Harry Stack Sullivan are often grouped with psychodynamic thinkers, but both moved theory toward social life. Adler emphasized belonging, inferiority, goals, and social interest. Sullivan emphasized interpersonal relationships and the self-system that develops to manage anxiety. For child and adolescent counseling, these traditions are especially useful because young people are constantly negotiating belonging in families, peer groups, schools, and online communities.

Adler: inferiority and striving

Adler proposed that feelings of inferiority are a normal part of human life. A child is small, dependent, and less skilled than adults. The question is not whether inferiority exists, but how the child responds. Some children strive toward competence and contribution. Others develop a superiority pose, avoidance, or excessive approval-seeking. The counselor asks what the child is moving toward, away from, or against. The behavior is purposeful, even when the purpose is outside awareness.

A ten-year-old who disrupts class may be seeking significance. If the child cannot feel useful through contribution, the child may feel visible through disruption. The counselor does not excuse harm. The counselor asks what goal the behavior serves and how the child could meet the need differently. Encouragement is central. Encouragement is not empty praise. It notices effort, contribution, and possibility.

Birth order and family constellation

Adler gave attention to birth order and family atmosphere. Firstborn, middle, youngest, and only children may experience different positions, but birth order is not destiny. A child who is oldest but has a chronic illness may not experience the usual role. A child who is youngest in a large extended family may have many caregivers. The counselor studies the subjective family constellation: how the child experiences their place. Questions include: Who is expected to succeed? Who is protected? Who is blamed? Who is compared with whom?

In doctoral assessment, family constellation can be mapped with a genogram and a simple question: "What is it like to be you in this family?" The answer may reveal a role such as the responsible one, the funny one, or the problem. Therapy can help the child expand the role.

Social interest and encouragement

Social interest, or Gemeinschaftsgefühl, refers to a sense of connection and contribution to the common good. Adler saw mental health not as isolated self-esteem but as the courage to participate. A child who feels discouraged may withdraw or dominate. Encouragement helps the child experience capability and belonging. The counselor might involve the child in a real contribution: helping set up the playroom, teaching the counselor a game, or planning a family task. The contribution must be genuine, not a trick to produce compliance.

Sullivan: anxiety and the self-system

Sullivan argued that personality is shaped in interpersonal situations. The self-system develops to manage anxiety, especially anxiety transmitted by important others. A child learns which feelings are acceptable and which lead to disapproval. Over time, the child may develop a persona that protects against anxiety but limits authenticity. Sullivan's concepts of prototaxic, parataxic, and syntaxic experience describe early undifferentiated experience, distorted interpersonal perception, and shared logical understanding. The counselor need not use the terms heavily, but the idea is useful: some interpersonal reactions are based on old, unexamined patterns.

Parataxic distortion is similar to transference but framed interpersonally. A teenager may assume a counselor will criticize them because a previous adult did. The counselor can notice the pattern and invite a new experience. "You expected me to be angry. What do you notice now?" The question is not a debate. It is an invitation to update the interpersonal map.

Clinical example

Fifteen-year-old Maya is a high achiever who panics before exams. Adlerian formulation might note that Maya's significance depends on being the successful child. A younger brother's illness has shifted family attention. Maya's striving protects against feeling forgotten. Sullivan's formulation might note that Maya learned anxiety when a parent's approval felt conditional. The counselor combines encouragement, family conversation, and cognitive-behavioral exam strategies. The goal is not only lower panic. The goal is a wider base of belonging that does not depend on perfect performance.

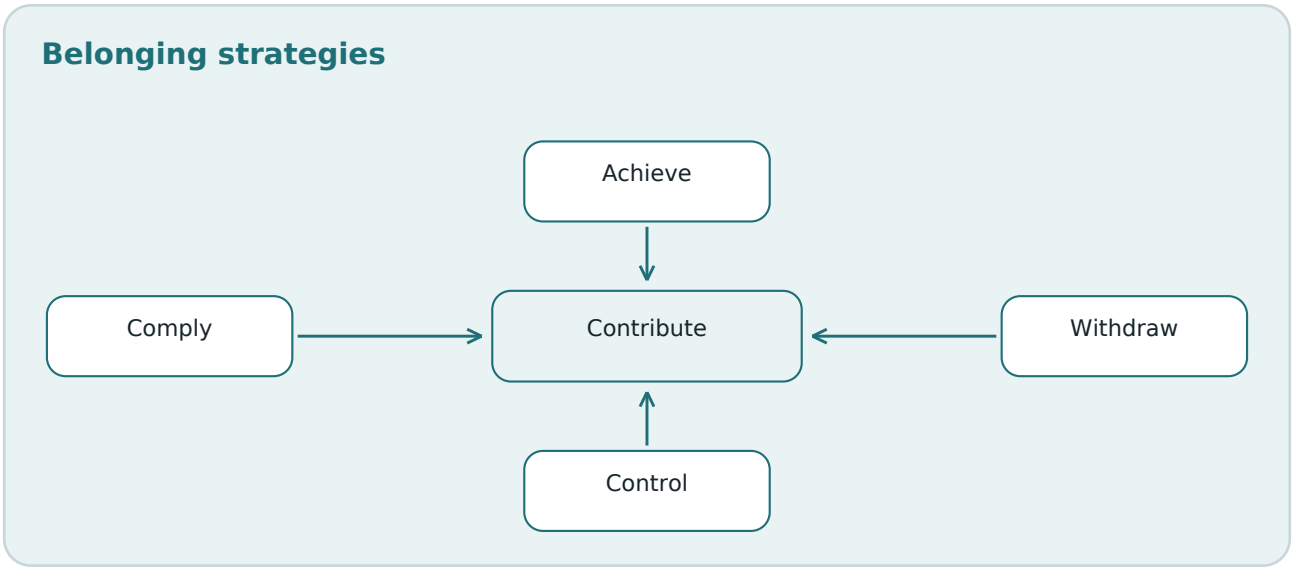
The counselor asks, "If you got a B, what would it mean about you?" Maya says, "That I'm not needed." The sentence reveals the interpersonal meaning of achievement. Therapy can test the belief through family dialogue and small experiments. Maya might share a struggle with a parent and observe the response. The experiment is behavioral, but the target is belonging.

Evidence and limits

Adlerian and interpersonal ideas influence many modern approaches, including family therapy, school consultation, and brief psychodynamic work. The research base is not as large as for CBT for some disorders, but the concepts are clinically generative. A limitation is that purpose can be overread. Not every behavior is a hidden goal. Sleep loss, hunger, neurodevelopmental differences, and unsafe environments matter. The counselor should not turn social context into a personal strategy.

Reflection

Write about a time when you felt you did not belong. What did you do to regain a place? Did you achieve, withdraw, rebel, or caretake? Young clients are often doing the same. Adler and Sullivan remind us that symptoms can be attempts to solve the problem of belonging.



Adlerian and interpersonal lenses ask what strategy a child uses to secure a place.

Attributed Foundational Reading

Attribution and critical-reading note

Adapted from Personality Theory in a Cultural Context by Mark D. Kelland, OpenStax CNX / Lansing Community College (2015), CC BY 4.0. Selection, organization, and PDF layout modified for this course; new lectures and course diagrams are separate additions.

Read historical and theoretical claims critically. A theory is a lens for inquiry, not a verdict about a child or family. Where source claims are dated or culture-bound, the new lecture supplies cautions and contemporary framing.

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Alfred Adler was an early member and president of the Vienna Psychoanalytic Society, but he never considered himself a follower of Sigmund Freud. He strongly disagreed with Freud's emphasis on sexual desire in the development of personality, focusing instead on children's striving for superiority and the importance of social relationships. He began to address the psychology of women as a cultural phenomenon, as opposed to Freud's view that women are fundamentally incapable of developing a complete and healthy personality. Adler also addressed issues of education, an individual's unique perspective on the world, and family therapy. Adler provided a perspective in which the striving of individuals to improve themselves is an essential characteristic of personality development. Most importantly, he believed that personal improvement and success are best achieved in cooperation with others, and that culture is an important factor in determining how that can be accomplished.

It has been suggested that Adler may have had an even greater influence on the overall development of psychiatry and psychology than Freud himself, and that theorists such as Sullivan, as well as Karen Horney and Erich Fromm, should be recognized as neo-Adlerians, not neo-Freudians (Ellis, 1973; Kaufmann, 1992; Mosak, 1995; Watts, 1999). Indeed, a reviewer of one of Karen Horney's books once wrote that Horney had just written a new book by Adler (see Mosak, 1995). Albert Ellis suggested that Adler set the stage for the cognitive/behavioral psychotherapies that are so popular today (Ellis, 1973). Late in life,

Adler encouraged the wife of a good friend to write his biography, and he gave Phyllis Bottome, who was herself a friend of Adler, a great deal of assistance (Bottome, 1957). He wanted to be understood. Perhaps, however, she came to understand him too well:

Adler was at once the easiest of men to know, and the most difficult; the frankest and the most subtle; the most conciliatory - and the most ruthless. As a colleague he was a model of generosity, accuracy and wholehearted integrity, but woe betide that colleague who dared to presume upon his generosity; or who was himself guilty of inaccuracy; or who failed in common honesty!

Adler never again worked with a person whom he distrusted; except when that person was a patient. (pg. 13; Bottome, 1957)

Adler also had the ability to make an impression on people who did not know him. When Raymond Corsini, a well-known psychologist in his own right, was 21 years old, he was invited by a friend to hear Adler speak at the City College of New York. During the question period that followed the lecture, an angry woman called Adler stupid, and berated one of the observations he had discussed. The young Corsini shared Adler's perspective, and Corsini looked forward to hearing Adler's "crushing reply." However, something quite different occurred:

He seemed interested in the question, waited a moment, then in the most natural and careful manner, completely unruffled by her evident antagonism, spoke to her very simply…

He seemed so calm, so reasonable, so precise, and so kind, I knew we were in the presence of a great man, a humble and kind person, one who repaid hostility with friendship. (pg. 86; Corsini cited in Manaster, et al., 1977)

Harry Stack Sullivan extended Adler's focus on the individual and social interest, believing that each of us can be understood only within the context of our interpersonal relationships. Like Freud, Sullivan focused intently on developmental stages, though he recognized seven of them, and believed that the primary purpose of development was to form better interpersonal relationships. In regard to his interest in relationships he can be closely associated with Adler, who believed that social interest, and its resulting social interaction, was the best way for an individual to overcome either real inferiority (such as in the case of a helpless newborn) or feelings of inferiority that might develop as part of one's personality. Unlike Freud and Adler, however, Sullivan was born in America. Thus, he should be considered one of the most important figures in American psychology, particularly within the field of psychodynamic theory.

Brief Biography of Alfred Adler

Alfred Adler was born on February 7, 1870, on the outskirts of Vienna. The second of six children, the family was fairly typical of the middle-class. His father was a corn-merchant, and did well in his business. While Adler while still quite young the family moved out into the country, where they kept cows, horses, chickens, goats, rabbits, and they had a very large garden. Adler was particularly fond of flowers when he was a toddler, and the move out of Vienna had the consequence of protecting him from his bad habit of stealing flowers from the garden of the Palace of Schonbrunn, which belonged to the Kaiser! Despite the seemingly idyllic setting, and the family's financial comfort, Adler did not have a happy childhood. The two main reasons for this were his sibling rivalry with his older brother and

the unfortunate fact that he seemed to be surrounded by illness and death (Bottome, 1957; Manaster, et al., 1977; Sperber, 1974).

His older brother seemed to be an ideal child, and Adler felt he could never match his brother's accomplishments. Even late in life, Adler told Phyllis Bottome with a sigh "My eldest brother…he was always ahead of me…he is still ahead of me!" (pg. 27; Bottome, 1957). As for his younger brothers, however, one felt the same sort of jealousy of Adler himself, whereas the youngest brother adored Adler. As for the illness and death, he suffered from rickets (a vitamin D deficiency) and spasms of his vocal cords, both of which made physical activity very difficult during his early childhood years. He was often forced to sit on a bench while watching his older brother run and jump. As he recovered, he joined his brother and the other local children in playing in a large field. Despite the fact that there were very few vehicles at the time, and those that were there moved very slowly, Adler was run over twice! Fortunately, he was not injured seriously. One of his younger brothers, however, had died suddenly when Adler was 4, an event that deeply affected him. And when Adler was 5, he came down with a serious case of pneumonia. After he had been examined by the doctor, Adler heard the doctor tell his father that there was no point in caring for Adler any more, as there was no hope for his survival. Adler was stricken with terror, and when he recovered he resolved to become a doctor so that he might have a better defense against death (Bottome, 1957; Manaster, et al., 1977; Sperber, 1974).

On the lighter side, most of the family was musically gifted. One of his brothers played and taught the violin, and one of his sisters was an excellent pianist. Despite the throat problems Adler had in early childhood, he developed a beautiful tenor voice. He was often encouraged to set aside his interest in science and pursue a career as an opera singer. Adler's parents encouraged the musical interests of their children, and took advantage of the marvelous musical culture available in Vienna at the time. Adler attended every opera and play that was running, and even by the age of 4 years old could sing entire operettas (Bottome, 1957; Manaster, et al., 1977; Sperber, 1974).

Although Adler spent a great deal of time reading, he was not a particularly good student. His worst subject was math, until he finally had a breakthrough one day. When the instructor and the best student in class failed to solve a problem, Adler raised his hand. Everyone in the room, including the instructor, laughed out loud at him. However, he was able to solve the problem. After that, he did quite well in math, and overall he did well enough to enter the University of Vienna. He studied medicine, as he had planned since being a young child, and graduated in 1895. Almost nothing is known of his time spent at the University of Vienna. Afterward, he briefly practiced ophthalmology, but then switched to general practice, a field in which he was very popular amongst his patients. He also became active in socialist politics, where he met his future wife: Raissa Timofeyewna Epstein (Bottome, 1957; Manaster, et al., 1977; Sperber, 1974).

Raissa and Alfred Adler had three daughters and one son between the years 1898 and 1909. The family lived rather simply, but they always had enough to meet their needs. Their daughter Alexandra and son Kurt both became psychiatrists. Alexandra Adler described her relationship with her father as close and positive, and she considered it a privilege to follow in his footsteps, whereas Kurt Adler said that everyone in the family felt respected as an individual and that no one had to search for their identity (see Manaster, et al., 1977).

In his general practice, Adler began to see psychiatric patients. The first was a distant cousin who complained of headaches. Adler suggested that no one ever has only a headache, and asked if her marriage was happy. She was deeply offended, and left in a huff, but 2 months later she filed for divorce. As he saw more psychiatric patients, Adler treated each case as unique, and followed whatever therapy seemed most appropriate for the particular patient. This was the beginning, of course, of Individual Psychology. Adler was so popular in this regard that his biographer had the following experience herself when leaving a message for Adler:

'Are you sure', she asked the clerk at the desk, 'that Professor Adler will get this message directly he comes in?' [sic] 'Adler?' the clerk replied. 'If it's for him you needn't worry. He always gets all his messages. You can hardly keep the bell-boys or the porter out of his room. They'll take any excuse to talk to him, and as far as that goes, I'm not much better myself!' (pg. 54; Bottome, 1957)

In 1900, Sigmund Freud gave a lecture to the Vienna Medical Association on his recently published book The Interpretation of Dreams (Freud, 1900/1995). The audience was openly hostile, and Freud was ridiculed. Adler was appalled, and he said so publicly, writing to a medical journal that Freud's theories should be given the consideration they deserved. Freud was deeply flattered, he sent his thanks to Adler, and the two men met. In 1902, Adler was one of four doctors asked to meet weekly at Freud's home to discuss work, philosophies, and the problem of neurosis. These meeting evolved into the Psychoanalytic Society. Adler and Freud maintained their cooperative relationship for eight years, and in 1910 Adler became the president of the International Psychoanalytic Association and co-editor of the newly established Zentrallblatt fur Psychoanalyse (with Freud as Editor-in-chief). During the preceding 8 years, however, the differences between Freud and Adler had become increasingly apparent. By 1911 he had resigned from the both the association and the journal's editorial board. Although Freud had threatened to resign from the journal if Adler's name was not removed, leading to Adler's own decision to resign from the journal, Freud urged Adler to reconsider leaving the discussion group. He invited Adler to dinner to discuss a resolution, but none was to be found. Adler is said to have asked Freud: "Why should I always do my work under your shadow?" (pg. 76; Bottome, 1957). The Psychoanalytic Society debated whether or not Adler's views were acceptable amongst the members of the society. The no votes counted fourteen, and the yes votes counted nine. Freud's supporters had won a small majority, and the nine other members left to join Adler in forming a new society, which in 1912 became the Society for Individual Psychology (Bottome, 1957; Manaster, et al., 1977; Sperber, 1974).

During the years in which Adler was still active in the Psychoanalytic Society he had begun his studies on organ inferiority and the inferiority complex, and after the split with Freud he focused his career on psychiatry (giving up his general medical practice). During World War I he served in the Army as a physician, and he continued his observations on psychiatric conditions as he helped injured servicemen. Following the war the Austrian Republic began to emphasize education and school reform. Adler established his first child guidance center in 1922, and by the late 1920s there were thirty-two clinics in Vienna alone (as well as some in Germany). The clinics were intended to help train teachers to work with special needs children, but Adler felt it was important to help the children themselves as well. In 1930, Adler brought together a number of his colleagues, including his daughter Alexandra, and published Guiding the Child: On the Principles of Individual Psychology. This volume

contains twenty-one chapters on the work being conducted in the Vienna child guidance clinics (including one chapter by Adler, and two by his daughter; Adler, 1930a). In addition, Adler taught at an adult education center and at a teacher training college. Adler continued to be so popular that after a long day of work he would settle in at the Cafe Siller and carry on friendly conversations until late at night.

In 1926, Adler made his first visit to America. Becoming a regular visitor, he lectured at Harvard and Brown Universities, in Chicago, Cincinnati, Milwaukee, and several schools in California. In 1929, he was appointed a visiting professor at Columbia University, and in 1932 he was appointed as the first chair of Medical Psychology in the United States, at Long Island Medical College. All of his child guidance clinics were closed when the fascists overthrew the Austrian Republic in 1934, and Alfred and Raissa Adler made New York their official home. In the spring of 1937 he began a tour of Europe, giving lectures and holding meetings. As he traveled to Aberdeen, Scotland on May 28th, he collapsed from a heart attack and died before he reached the hospital.

One of the enduring questions about Adler's career was the nature of his relationship with Freud. As mentioned above, it was the very popular Adler who defended Freud's early theories, and helped Freud to gain recognition in psychiatry (remember that Freud was well known as an anatomist and neurophysiologist). And yet, it is often suggested that Adler was a student or disciple of Freud. According to Abraham Maslow, Adler was deeply offended by these suggestions:

I asked some question that implied his disciplineship under Freud. He became very angry and flushed, and talked so loudly that other people's attention was attracted. He said that this was a lie and a swindle for which he blamed Freud entirely. He said that he had never been a student of Freud, or a disciple, or a follower…Freud, according to Adler, spread the version of the break which has since been accepted by all - namely, that Adler had been a disciple of Freud and then had broken away from him. It was this that made Adler bitter…I never heard him express personal opinions of Freud at any other time. This outburst must, therefore, be considered unusual. (pg. 93; Maslow cited in Manaster, et al., 1977; see also Kaufmann, 1992)

Placing Adler in Context: Perhaps the Most Influential Person

in the History of Psychology and Psychotherapy

To suggest that anyone may have been more influential than Freud, let alone a contemporary of Freud, is difficult to say. And yet, if we look honestly at the accomplishments of Alfred Adler, and the breadth of his areas of interest, we will see that the case can be made. When Freud first proposed his psychodynamic theory, with its emphasis on infantile sexuality, Freud was often mocked, or simply ignored. It was the popular Dr. Adler's defense of Freud, and Adler's favorable review of the interesting nature of Freud's theories, that helped Freud find an interested audience. As supportive as Adler was, he had his own theories from the very beginning of their association, and Adler's Individual Psychology has a certain logical appeal, without the corresponding controversy generated by Freud.

Infants are inferior, and we all try to gain control over our environments. Thus, the basic inferiority/striving for superiority concept seems self-evident. Likewise, the inferiority complex is one of the most widely recognized and intuitively understood concepts in the

history of psychology. Suggesting that each person adopts a style of life that helps them to pursue their goals also again makes perfect sense, and the suggestion that we have within us a creative power to form our style of life is a decidedly hopeful perspective on the human condition.

Adler's influence within the psychodynamic field has been widely recognized, if not adequately advertised. When he split with Freud, nearly half of the psychoanalytic society left with him. His emphasis on social interactions and culture provided a framework within which theorists such as Karen Horney and Erich Fromm flourished. Adler's emphasis on child guidance, and including school teachers as being just as important as parents, must have had an important influence on Anna Freud (though she would never have admitted it). Within the child guidance centers, Adler was one of the first (if not the first) to utilize family therapy and group psychotherapy, as well as school psychology. It was within such an environment, influenced also by Maria Montessori, that Erik Erikson evolved into the analyst and theorist he became.

Adler's influence also extended well beyond the psychodynamic realm. His scheme of apperception set the stage for the cognitive psychology and therapies that are so popular (and effective) today. He is recognized by many as the founder of humanistic psychology, though it was Rogers, Maslow, and the existentialists Viktor Frankl (Frankl worked closely with Adler for a time) and Rollo May who clearly split from psychodynamic theory into new schools of psychology.

Given this extraordinary influence, it is surprising that Adler is not widely recognized as belonging amongst the greatest theorists and clinical innovators in the history of psychology and psychiatry. The honor is certainly well deserved.

Adler's Individual Psychology

Adler developed the concept of **Individual Psychology** out of his observation that psychologists were beginning to ignore what he called the **unity of the individual**:

A survey of the views and theories of most psychologists indicates a peculiar limitation both in the nature of their field of investigation and in their methods of inquiry. They act as if experience and knowledge of mankind were, with conscious intent, to be excluded from our investigations and all value and importance denied to artistic and creative vision as well as to intuition itself. (pg. 1; Adler, 1914/1963)

To summarize Individual Psychology briefly, children begin life with feelings of **inferiority** toward their parents, as well as toward the whole world. The child's life becomes an ongoing effort to overcome this inferiority, and the child is continuously restless. As the child seeks **superiority** it creatively forms goals, even if the ultimate goal is a fictional representation of achieving superiority. Indeed, Adler believed that it is impossible to think, feel, will, or act without the perception of some goal, and that every psychological phenomenon can only be understood if it is regarded as preparation for some goal. Thus, the person's entire life becomes centered on a given plan for attaining the final goal (whatever that may be). Such a perspective must be uniquely individual, since each person's particular childhood feelings of inferiority, creative style of life, and ultimate goals would be unique to their own experiences (Adler, 1914/1963).

The suggestion that seeking to overcome one's inferiorities is the driving force underlying personality development is, of course, a significant departure from Freud's suggestion that development revolves around seeking psychosexual gratification. Another important difference is that Adler did not distinguish between the conscious and unconscious minds as Freud had:

The use of the terms "consciousness" and "unconsciousness" to designate distinctive factors is incorrect in the practice of Individual Psychology. Consciousness and unconsciousness move together in the same direction and are not contradictions, as is so often believed. What is more, there is no definite line of demarcation between them. It is merely a question of discovering the purpose of their joint movement. (pg. 56; Adler, 1929a)

Inferiority and Compensation

In 1907, Adler published his classic Study of Organ Inferiority and Its Psychical Compensation, which was translated into English 10 years later (Adler, 1917). This was primarily a medical article on the consequences of **organ inferiority**, in which Adler looked at how the nervous system helped the body to adapt to physical infirmities that resulted from, literally, inferior organ development. For example, it is often suggested that people who are blind develop better hearing. However, social psychologists have demonstrated that the social environment can profoundly affect our sensitivity to external stimuli. The reason for this is probably just what Adler described as the primary means through which the brain can compensate for any deficiency: by bringing attention to the processes necessary for compensation. Thus, if a person has difficulty seeing, they pay more careful attention to hearing, as well as to the other senses. However, this is not a perfect system, and it can also lead to over-compensation. As a result, a wide variety of physical symptoms can result from the psyche's efforts (including unconscious efforts) to compensate for some problem. As noted by Freud, hysterical symptoms are typically manifested as physical problems. According to Adler, underlying these physical symptoms, even when they are caused solely by the psyche, there must be some organ inferiority within the body (Adler, 1917).

Adler did not limit his theory of organ inferiority to medical problems or neurotic symptoms, but rather, he expanded the theory to incorporate all aspects of life. **Compensation** refers to the typical manner in which a person seeks to overcome challenges. For example, if one breaks their arm, they learn to function with a cast, or if one loses their eyesight, they learn to use a cane or work with a seeing-eye dog (Dreikurs, 1950; Mosak & Maniacci, 1999). If we examine compensation in a more psychosocial realm, examples might include a college student who cannot find a suitable boyfriend or girlfriend so they focus on becoming a straight A student, a student who does not do well academically focuses their efforts on becoming a star athlete, or an only child who wished to have brothers and sisters has many children of their own (Lundin, 1989). In such instances, compensation leads to balance in one's life. A weakness, or at least a perceived weakness, is compensated for in other ways (Manaster & Corsini, 1982). **Overcompensation** involves taking compensation to extremes. For example, a person born with a bad foot strives to become a professional dancer, or a person born in poverty strives to become a millionaire and then continues to work 80 hours a week or more striving to become a billionaire. Generally speaking, the mechanisms of personality inferiority are more complex than those of organ inferiority. Likewise, compensation and overcompensation are more complex when they pertain to one's personality than when they involve physical challenges (Manaster & Corsini, 1982).

When a person finds it difficult to overcome their challenges in life, they can develop what Adler called an **inferiority complex** (Adler, 1928, 1929a, 1931a). Although feelings of inferiority are universal, as is the striving for superiority, people are not created equal. We all have different strengths and weaknesses. However, when an individual cannot compensate for their weaknesses, and their feelings of inferiority overwhelm them, the inferiority complex arises. According to Adler, the term complex is not really accurate, because the so-called inferiority complex is complicated, and it permeates the entire personality. And yet, it may not always be obvious. An individual with an inferiority complex may feel comfortable in situations in which they have enough experience to feel self-assured, although they may create those situations by avoiding competition that might expose their weaknesses (Adler, 1929a). The inferiority complex will show itself, however, in tense or difficult situations, and often takes the form of excuses as to why the individual can't pursue a certain course of action. For psychologists, according to Adler, the presence of an inferiority complex can typically be recognized by contradictions, by certain emotions such as doubt, and by generally hesitant behavior. The proper treatment, therefore, is to encourage people, never to discourage them, and to help them understand that they are capable of solving problems and facing the difficulties of life (Adler, 1929a).

When the intense feelings of inferiority associated with the inferiority complex become too much to bear, they can be transformed into a unique delusion that Adler described as the **superiority complex** (Adler, 1928, 1929a, 1931a). The superiority complex should not be viewed as an extension of the normal process of striving for superiority. The superiority complex arises out of the inferiority complex, and is actually an extension of the intense feelings of inferiority. Interestingly, such people typically do not present themselves as superior individuals, instead they may be arrogant, snooty, domineering, or they may cling to prominent and important people. In contrast, people who truly are superior often have a sense of modesty (Lundin, 1989; Mosak & Maniacci, 1999). The complexity of the superiority complex, and its origin in feelings of inferiority and the continued striving for superiority that is universal can be seen not only in neurotic symptoms and other forms of mental illness, but also in criminal behavior:

We see children who start stealing suffering from the feeling of superiority. They believe they are deceiving others; that others do not know they are stealing. Thus they are richer with little effort. This same feeling is very pronounced among criminals who have the idea that they are superior heroes…he wants to arrange matters so that he escapes the solution of the problems of life. Criminality is thus, the result of a superiority complex and not the expression of fundamental and original viciousness. (pp. 80-81; Adler, 1929a)

Discussion Question: Adler believed that we all begin life with feelings of inferiority and then strive for superiority. What sort of things have you tried to be really good at in life? Can you remember times when you felt inferior trying to accomplish those same goals?

The Style of Life and the Life Plan

According to Adler, everyone faces difficulties in life, and they strive to overcome those difficulties. As each individual faces their unique difficulties, and strives to compensate in their own characteristic ways, given the environment (or culture) in which they live, the individual develops a sense of meaning for their life and they set a goal for their strivings. Initially Adler referred to the consistent movement toward this overriding goal as a **life plan**, but that term proved to be somewhat confusing. So, Adler chose instead to refer to the

pursuit of one's goal as the **style of life**. The style of life unifies the personality, as it is based on one's early life experiences. However, Individual Psychology looks not to the past, but rather to the future. If we know a person's style of life, we can predict their future actions (Adler, 1929a, 1931a).

It is not always easy to recognize the style of life, however, particularly in a psychologically healthy person or during times of relative calm. It is when a person faces a new situation, or a new difficulty, that the style of life becomes clear to others. For the normal person, the style of life is a framework within which the person is adapted to their society in such a way that the society benefits from the work of the person and the person themselves has the energy and courage to face any problems and difficulties that arise (Adler, 1929a). The style of life encompasses our individual creativity, the ways in which we solve problems and compensate for inferiorities, our attitudes, opinions, and goals. It unifies and expresses our personality, provides consistency for how we live our life, and helps us to find our place in the world (Adler, 1931a; Dreikurs, 1950; Lundin, 1989; Mosak & Maniacci, 1999).

The style of life is established fairly early in childhood, which can be a serious problem when it proves to be a dysfunctional style of life. The inferiority complex is, of course, one faulty style of life. When an inferiority complex arises out of an actual organ inferiority it can be particularly troublesome. Robert Lundin (1989) described the case of a senior student he knew in college who was only 5' 3" tall (very short for a man, though this would really only be a perceived inferiority). He was extremely arrogant and hostile toward younger students, claiming to be intellectually superior in every regard. He even offered to be Lundin's roommate, since Lundin obviously needed his help to improve Lundin's deficient personality! Lundin declined the offer. Adler noted that organ inferiority is not always a negative situation, and given the advances in prosthetic devices that exist today, it is even truer now that organ inferiority does not necessarily diminish one's quality of life. However, Adler emphasized that what matters most is how the individual experiences the weakness of their organ inferiority. Some try to avoid or deny the problem, others constantly "wrestle and struggle" with their difficulties. In the end, it comes down to the creative power of the individual to adapt (see below; Adler, 1932a/1964).

In addition to the style of life of that can result from organ inferiority (or perceived organ inferiority), Adler discussed two other factors that commonly lead to dysfunctional styles of life, and which can be attributed primarily to parental influence: **pampering** and **neglect**. The pampered style of life was of particular concern to Adler. He was not referring to children who are loved and cared for intimately, but to children whose parents constantly hover over them, solve every problem, and relieve the child of any duties or responsibilities. As a result, the child never learns to take care of itself or to interact with others in a cooperative manner.

The more deeply I have delved into the problem of neurosis and searched the cases presented, the more clearly have I come to see that in every individual with a neurosis some degree of pampering can be traced...Under such circumstances the child develops like a parasite… (pp. 88-89; Adler, 1932a/1964)

Extending this idea, Adler wrote that whether one is dealing with "difficult children, nervous or insane persons, suicides, delinquents, drug-addicts, or perverts, etc." there is a lack of social feeling (Adler, 1964). In other words, they simply do not function well in relationship to others because they have never had to. As for the neglected child, one who is unwanted,

they have had no opportunity for social interaction whatsoever, since their own family fails to interact with them. In cases of suicide, Adler believed that even death can be desired as a means of revenge against those who have hurt or neglected a child by showing others what they have lost in the one they failed to love (Adler, 1967). Since feelings of neglect are relative, pampered children often find themselves in situations, later in life, where they feel neglected, since they may no longer receive the pampering to which they have become accustomed (Adler, 1932a/1964).

Discussion Question: How would you describe your style of life? Adler believed that dysfunctional styles of life often result from either pampering or neglect. Do you know anyone whose style of life clearly reflects how they were raised? Are they someone you like to spend time with, or someone you would rather avoid?

Social Interest and Cooperation

Adler believed that the right way to achieve superiority was through **social interest** and the **cooperation** that naturally follows. This is not some high-minded philosophy, however, but simple reality. According to Adler, “we are in the midst of the stream of evolution.” As such, the human species as a whole has sought superiority, just as each individual seeks their own personal superiority (Adler, 1964). The individual’s weakness causes them to seek support from others, by living within a society:

All persons feel inadequate in certain situations. They feel overwhelmed by the difficulties of life and are incapable of meeting them single-handed. Hence one of the strongest tendencies in man has been to form groups in order that he may live as a member of a society and not as an isolated individual. This social life has without doubt been a great help to him in overcoming his feeling of inadequacy and inferiority. We know that this is the case with animals, where the weaker species always live in groups…On the other hand, gorillas, lions, and tigers can live isolated because nature has given them the means of self-protection. A human being has not their great strength, their claws, nor their teeth, and so cannot live apart. Thus we find that the beginning of social life lies in the weakness of the individual. (pp. 60-61; Adler, 1929a)

This evolutionary perspective provides an explanation for the paradox that Individual Psychology is focused largely on social relationships! Once again, we know (though perhaps unconsciously) that alone we are weak and inferior, but together we can accomplish great things. Adler’s hopeful vision for the future is that someday humanity’s social feeling will overcome everything that opposes it, and people will live in harmony. In the meantime, however, he acknowledges that many things still oppose it, and work to destroy the social feelings and social interest of children: sexism, racism, poverty, crime, the death penalty, suicide, greed, mistreatment of the poor, the handicapped, and the elderly, and all forms prejudice, discrimination, and intolerance (Adler, 1964). It is not an easy challenge facing humanity, but Adler suggested that the path toward harmony lies, in part, in recognizing the three main ties that every person must take into account. First, we are tied to this one world, the earth. We must learn how to survive here, given whatever advantages and disadvantages that means. Second, we are not the only member of the human race. Through cooperation and association we can find strength for all, and we can ensure the future of humanity. Finally, we must accept that there are two sexes. For Adler, this last tie is resolved primarily through love and marriage. While this may sound like a product of Adler’s cultural upbringing, it also implies caring for and respecting members of the other sex. Otherwise,

love is a word used without meaning. Adler proposed that if we give meaning to life through the recognition of these three ties to our environment, then others can share in our meaning of life, and the benefit will then return to us (Adler, 1931a).

Children often cooperate without any need for encouragement, especially when one of them has a skill that another lacks.

In more practical terms, social interest is evident in cooperation. In order for an individual to overcome their own feelings of inferiority they must know that they are valuable, which comes only from contributing to the common welfare. Adler felt that those who seek personal power are pursuing a false goal, and they will eventually disappear from life altogether. However, by contributing to family and society, either through raising children or contributing to the success of one's culture or society, one can claim a sense of immortality. Individual psychology is based on the premise that when a person realizes that the common good is essential to the development of humanity, then they will pursue personal development that is in accord with the greater good. They will recognize both the good and challenges that come their way as belonging to all, and they will cooperate in seeking to solve the challenges. They will not ask for anything in return, since they recognize that whatever they do to benefit others is ultimately to their own benefit as well (Adler, 1933/1964). This perspective is surprisingly close to Eastern philosophies and the concepts of interbeing and karma, though Adler's religious references are primarily Christian (though born Jewish, Adler later became a Christian).

In American society, work is often done by **teams**. The short definition of a team is two or more individuals, with different roles, who socially interact in order to pursue some common goal. Teams can lead to successful outcomes in a wide variety of settings, such as in software development, Olympic hockey, disease outbreak responses, or the unexpected damage to a spacecraft like Apollo 13 (for two excellent and entertaining movies on teamwork, see Miracle [O'Connor & Guggenheim, 2004] and Apollo 13 [Howard, Broyles, Jr., & Reinert, 1995]). However, teams can also lead to group failures, such as the international intelligence failures leading up to the 9/11 terrorist attack on the World Trade Center, the Space Shuttle Columbia accident, or the widely reported, storm-related deaths on Mt. Everest in 1997 (Kozlowski & Ilgen, 2006; Marks, 2006; for more on Mt. Everest see Boukreev & DeWalt, 1997; Krakauer, 1997). Given the importance of teamwork, both in personal settings and within organizations, there has been a great deal of research on teams, addressing cognitive, motivational, and behavioral factors, as well as information on effective team design, team training, and team leadership. Despite the wealth of information on both the positive and negative factors involved in teamwork, there is an interesting contradiction in the Western world:

…We school our children as individuals. We hire, train, and reward employees as individuals. And, yet, we have great faith that individuals thrown together into a team with little thought devoted to team composition, training and development, and leadership will be effective and successful. (pg. 115; Kozlowski & Ilgen, 2006)

Discussion Question: Working in teams can turn out good or bad, depending on dynamics of the team and the individuals involved. Are you actively involved in any teamwork? Does your team work well together, or do the dynamics of the team cause problems and interfere with accomplishing your goals?

The Life Tasks - Work, Communal Life, and Love

Based on the three ties described above, our ties to earth, humanity, and the opposite sex, Adler described three **life tasks**: **work**, **communal life**, and **love**. Work relates to the earth in an evolutionary sense, dating back to when our ancient ancestors were hunter/gatherers dependent on the environment for food and shelter. According to Adler, all of the questions of life can be found within these three tasks, which challenge us continuously throughout our lives. They tasks are not unrelated, since each one depends upon the successful pursuit of the other two. Given this interrelationship, Adler believed that how a person approaches each of these tasks, through their style of life, reveals a great deal about what they view as the meaning of life. It is necessary, of course, for there to be balance. For example, a person in an unhappy marriage might spend a great deal of time at work. This represents a mistaken style of life (Adler, 1931a, 1964). Worse still, is someone who fails to pursue any of the life tasks:

Suppose, for example, we consider a man whose love-life is incomplete, who makes no efforts in his profession, who has few friends and who finds contact with his fellows painful. From the limits and restrictions of his life we may conclude that he feels being alive as a difficult and dangerous thing, offering few opportunities and many defeats. (pg. 7; Adler, 1931a)

The importance of the work task is to be found in the fact that we must do something with our time. As people began to cooperate, they were able to divide their labors. Some would hunt, some would farm, some became craftsmen, some raised the young, and eventually others served in the armies that protected all the rest. In this manner, each person served a valuable role within society (even if the role was not prestigious), and everyone benefited from the ability of each person to become more of an expert in their role. Of course, this sort of social cooperation is the second task of life, the communal life or, as it is sometimes referred to, having friends. Working with others for the common good can be quite difficult if people are doing so only for their own benefit, and it they distrust or fear those they seem to be cooperating with. As societies became more advanced, and education became an important part of society, most societies encourage social interest as an aspect of education. In America, for example, we talk about children learning to be good citizens, and schools include many civics lessons. In addition, societies establish not only formal and informal guidelines and norms for acceptable behavior, but actual laws are written to punish those who act in defiance of the common good. Not that this is easy! The first amendment to the American constitution guarantees free speech, which includes the right to challenge the very existence of our form of government. However, it is generally recognized that the greater good is served by protecting the people from possible abuses of power by the government. Without getting into a discussion of politics, this balance, which seeks to serve the best interests of the community of citizens, has resulted in one of the longest lasting governments in the world today.

When Adler referred to the third task of life, love, he was primarily talking about choosing a partner to bear and raise children. When a child is first born, the love of its mother is the basis for the child’s development of social feelings. If a child is neglected, they do not learn how to relate to others, or if they are spoiled, they do not need to relate to others. An early challenge for the child is found in the nature of the father, and then any siblings who may be a part of the family. They typically do not approach the child with the same tender love as

the mother. If the mother protects the child from this, spoiling and coddling the infant, a disordered style of life develops, but if the mother leaves the child to face this new challenge on its own, they must rely on their creative powers to adapt to these different social relationships. Children readily have this capacity, if they are allowed to utilize it. Later in life, each person must choose a mate in order to have their own children, and their ability to adapt to relationships with love interests will, obviously, depend on their own development earlier in life. Active, friendly members of a community will have more opportunities to meet someone they are truly attracted to. Individuals who are successful and productive in their work will be better able to provide for a family. And of course, the ultimate existence of each member of the community depends on continued procreation of the species. Thus, work, communal life, and love come together within a healthy society for everyone's benefit (Adler, 1931a, 1964; Lundin, 1989; Mosak & Maniacci, 1999).

Connections Across Cultures: Randy Kearse and

Using Prison as an Opportunity for Change

One of the challenges to social cooperation is the ability to communicate. Communication takes place in at least two important ways: language and shared experiences/goals. If we cannot understand what a person is saying, then communication is obviously difficult. But even if we speak the same language, if our entire perspective on life is different, particularly the direction in which we are headed (our style of life), it can be even more difficult to really communicate. Randy Kearse is a man who has lived outside of what many of us consider mainstream America. He grew up in a Brooklyn, NY ghetto, where he became well versed in street talk, or what he refers to as hip-hop and urban **slanguage** (Kearse, 2006a). Despite coming from a relatively stable family (his mother was a teacher and all of his brothers and his sister graduated from high school), he descended into a life of drugs and crime, eventually spending over 13 years in a federal prison for dealing illegal drugs. This introduced Kearse to a large, and growing, subculture in America: the prison population. Kearse learned one lesson very clearly in prison: he hated it! He hated people having such power over him, he hated the disrespect his mother had to endure when she visited him, he hated the food, he hated having the guards read his mail, etc., ad infinitum.

With so many reasons to hate prison, there was no way I was gonna put myself back in the same situation again. Brothers complain about being locked up everyday all day while they're there, but when they get a chance to run the streets again their hatred for prison life fades away. That's crazy! (pg. 132; Kearse, 2006b)

But how does one stay out of prison? In Changin' Your Game Plan! (Kearse, 2006b), Kearse offers some very practical steps. But more importantly, he discusses why it matters, and how one needs to change their mindset in order to be successful. His advice fits well with Adler's emphasis on social interest and cooperation, as well as with Adler's three life tasks. As Kearse says, what good is street pride when you don't have the respect of your mother, your children, and other people who know what you're capable of accomplishing?

Kearse believes that everyone has a purpose in life. Even prisoners serving life sentences can preach to others about straightening out their lives so the ones who do get out of prison can stay out. For his own part, Kearse is trying to set a good example now that he is out. He talks about the misguided sense of pride that keeps people from working minimum wage jobs when they come home. Most people in prison do not want their children to live such a

life, but can they really set the right example when they themselves get out of prison? One of the most important things to realize is that their style of life has helped to create the problems that exist in their community:

We have a real obligation to make these changes while incarcerated because a lot of us are to blame for the condition our neighborhoods are in today. We were major contributors to the chaos, mayhem and destruction that have plagued our communities…The saddest thing you can see while incarcerated, is a youngster young enough to be your son walk through the doors. (pg. 133; Kearse, 2006b)

In order to help their communities and their families, Kearse emphasizes that individuals returning home from prison need to get a job and work for an honest living. Kearse tells New Yorkers exactly where to go and how to go about getting ID and a social security card. He recommends getting a job as a messenger, especially if you have a driver's license and can afford to buy a van (or if you save up the money to buy a van). For most of us, having these things is simply taken for granted, but not so for many poor people growing up in the city where such things may not be necessary (especially if one works illegally). One of the advantages of having a messenger/delivery job is that one gets to travel around, meet different people, and become aware of different opportunities. This proved very helpful to Kearse as he pursued his dream of publishing Street Talk: Da Official Guide to Hip-Hop & Urban Slanguage (Kearse, 2006a). Curiously, the book is not complete. In acceptance of his publisher's concerns (representing the community), the commercially available version left out the most derogatory slang pertaining to women, race, sexual preference, ethnicity, and religion. Kearse later published a supplemental version ("Da Grimy Version") through a private site. As Kearse became more experienced, he established his own publishing company for his second book, Changin' Your Game Plan!, and he is now working on an autobiography. Randy Kearse is working hard to make a better life for himself, set an example for his community, and to honor the mother who raised him to have ambition and dreams. His mother still loved Kearse when he was sent to prison, but her patience was limited:

Once I received my sentence my mother told me straight up, "I'll ride this time out with you, but if you get back out here and get caught up in them streets again don't call me," and I can't blame her for that. How much can a mother take seeing her child going back and forth to prison? (pg. 33; Kearse, 2006b)

It would be very difficult for most of us to imagine what prison life and culture is like, or even what it would be like to get caught up in the judicial system. But for people who live in difficult circumstances, it can be just as difficult to avoid getting caught up in a style of life that promises instant gratification, but which costs a lot of money. People who try to take shortcuts, such as stealing what they want, or selling drugs to make a lot of money, end up with little to show for their life except "a gang of years in prison." Escaping this style of life requires a plan, and even more so the motivation for making that plan work:

Coming out of prison you have to have a plan. If you don't, the chance of you returning to prison is great and you and I both know these people aren't playing. They'll lock your ass up for a hun'ned years and not care…Shit is real. (pg. xxiii; Kearse, 2006b)

The Creative Power of the Individual and Fictional Finalism

The science of Individual Psychology developed out of the effort to understand that mysterious creative power of life - that power which expresses itself in the desire to develop, to strive and to achieve - and even to compensate for defeats in one direction by striving for success in another. (pg. 32; Adler, 1929a)

Adler believed that we are all born with a creative force: the **creative power of the individual**. He did not reject the concepts of heredity, temperament, or disposition, but he emphasized that it not so important what we are born with, but rather what we do with it (Adler, 1932a/1964). As noted above, infants are inferior, so everyone begins life with feelings of inferiority. This leads to the striving for superiority, and the development of a style of life, which is aimed toward some goal. The nature of that style of life is unique because it is created by the child, and it is done very early in life. This is not a deterministic perspective, this creation of the style of life is just that, creative, and therefore it must be unique (hence, Individual Psychology). Since Adler believed that all thought and behavior was oriented toward some goal, there must be some goal that underlies the manner in which the style of life is created. Since a child cannot see into the future and create a specific goal in life, Adler proposed that we are guided by a fictional goal, the so-called **fictional finalism** (Adler, 1914/1963, 1928, 1929a, 1932a/1964; Lundin, 1989; Manaster & Corsini, 1982).

The fictional final goal involves the sentiment of superiority, or at least the elevation of the personality to an extent that makes life seem worth living (Adler, 1928). Thus, it does not need to be precisely defined, which is important for our consideration that it is created by a young child. And yet it exists within the child's mind, it provides the framework within which the style of life is creatively formed, and it serves as the child's goal in life (though it remains primarily unconscious). It is also important to recognize that although this goal is fictional, it is entirely positive, it is a healthy and natural motivational force (Lundin, 1989). The fictional finalism should definitely not be mistaken for **fictive superiority**. Fictive superiority is the imagination, or false belief, that one is actually superior. It is a typical neurotic symptom that stems, primarily, from having been pampered. A pampered child is superior, at least in the sense that everything is done for them. However, adult life no longer sustains that delusion, yet the child has never learned how to adapt to life's challenges, their style of life is set in the expectation of challenges being solved for them. A healthy child, on the other hand, has learned to face challenges, and to strive toward overcoming them. Thus, the healthy child develops a style of life that incorporates the process of facing and overcoming life's obstacles, and this carries over into a healthy adulthood (Adler, 1932b/1964).

Within his discussion of the creative power of the individual and the fictional finalism, Adler began to address what can be viewed as the foundation for cognitive psychology and cognitive therapy (see Chapter 12):

In a word, I am convinced that a person's behaviour springs from his idea. We should not be surprised at this, because our senses do not receive actual facts, but merely a subjective image of them - a reflection of the external world. (pg. 19; Adler, 1964)

According to Adler, the prototype of the style of life, as it points toward the fictional finalism, is set in a particular orientation. Throughout the individual's life, their perceptions of the world are then constrained to "fall into a groove established by the line of direction" (Adler, 1929a). He referred to this phenomenon as the **scheme of apperception**. As a result of this scheme, the individual interprets experiences before they are accepted, and the

interpretation always agrees with the original meaning that the individual has given to their life. When the individual has developed a mistaken meaning to life, or when experience cannot be reconciled with the meaning they hold, they may be forced to change their scheme of apperception. This is not easy, however, and only occurs when there is sufficient social pressure to do so (Adler, 1931a).

Discussion Question: Adler described fictive superiority as the mistaken belief that one is superior to others. What do you think about people who think they are great, who think they know everything about everything, and can do anything at all, but who really are no different than anyone else?

Child Development and Education

Adler agreed with Freud that personality is basically set in the early childhood years. Thus, Adler was particularly interested in child development, and also in the training of those responsible for raising children (typically, the parents, the rest of the family, and school teachers). The emphasis in much of his work was on ensuring that children are brought up in the best way possible from the very beginning of life. We have already examined the importance of the mother's role, and that she needs to be supportive but must not spoil her child. A child who is unable to resolve life's problem without the assistance of others will grow up into a neurotic person demonstrating a dysfunctional style of life (Adler, 1913a/1963). Since this is something that happens within the family, the parents obviously cannot serve as part of the solution for this problem. Thus, Adler turned his attention to school teachers.

Adler believed that a person is educated when it becomes clear that they have become more relevant to more people (Adler, 1958). In other words, they become a more active and involved member of their community, society, and perhaps even the whole world. School represents a new situation for a child. If they have been raised well, they pass this test rather easily. If not, the defects in their style of life become evident. School work requires cooperation with both the teacher and the other students, and the ability to cooperate is probably more important than the child's innate intellectual abilities. Adler did not dismiss the importance of I.Q. (though he did suggest that the child and the parents should never be told what it is), but he pointed out that a child's ability to concentrate on school subjects is primarily dependent on the child's interest in the teacher (Adler, 1930b). So once again, we see the three life tasks coming into play. School work is the child's work, and it must take place within a communal setting (the classroom). But what about love? Whether you want to call it support, encouragement, caring, motivation, or love, Adler was clear in terms of what he thought about the "peculiar position" held by teachers:

The teacher, professional or amateur, must teach the simple thing: love, and call it by its simple name: love. Almost since the beginnings of recorded history, however, the teacher has been in a peculiar position: he is facing pupils, children or adults, who do not expect that the thing they ought to learn is so simple. (pg. 115; Adler, 1958)

According to Adler, it is the role of teachers to recognize the difficulties that children cannot overcome, and to "correct the mistakes of the parents." An essential aspect of this "correction," however, is that it must never be punitive. Adler believed that if teachers scold or criticize students who cannot connect with their teacher or their classmates, then the child will realize they were right to dislike school. Rather, teachers must help children to

connect with themselves, and then reach out to connect with others (Adler, 1931a). But what about children who cannot redirect their style of life or face up to the challenges they encounter?

Discussion Question: Teachers play an important role in each person's development, according to Adler. Who were the teachers who really influenced you, and were they good teachers who helped you, or bad teachers who inspired you to be a better person than you saw in them?

Adler wrote a great deal about guiding children, and about helping them to avoid and recover from delinquency (Adler, 1918/1963, 1930a, 1931a, 1935/1964, 1963), and his daughter Alexandra joined him in this endeavor (Alexandra Adler, 1930a,b). Often the discussion addresses the primary problem in Adler's view: a lack of social interest due to having been pampered. In addition, both Adler and his daughter bring into consideration an interesting family dynamic that appears to play some role: the child's **birth order**. An **only child** is in an unfavorable situation. Although they certainly receive attention and support from their parents, since they receive all of it they tend to be pampered and they lack practice in being sociable. As they leave the pampered surroundings of the home, such as when they go to school, the child grows up fighting against their environment and trying to dominate it. Their solutions can be passive, such as being timid, anxious, or routinely getting "sick," or their solutions can be active, such as being excessively talkative, defiant, or combative (Alexandra Adler, 1930a; Adler, 1929/1964). They face the potential of becoming like parasites, people who do nothing but enjoy life while expecting others to take care of them (Adler, 1928). The situation can be much worse, however, for the **oldest child**. The oldest child was, for a time, an only child, and received all of the privilege and pampering associated with that position. With the arrival of the next child, however, they lose their privileged position, and the mother must be particularly attentive to the new infant. The oldest child is **dethroned**, and this can feel quite tragic, leading to consuming jealousy and a bitter struggle to regain the parent's attention (Adler, 1929/1964, 1931a, 1963). Dethroning is an experience that always leaves a great impression, and can lead to a critical attitude toward the mother and a turning away toward the father (Adler, 1929/1964). According to Adler, one often finds the experience of being dethroned in the past of problem children, neurotics, criminals, drunkards, and perverts (Adler, 1931a). Nonetheless, being the oldest child also has distinct advantages. Amongst the siblings, the first born is typically the biggest and more experienced child. They have a certain power over the other children, in that they are often given greater responsibility, including, perhaps, the responsibility of caring for their younger brothers and sisters. They tend to be guardians of law and order, and they have an especially high valuation of power (Adler, 1928). Adler was careful to point out, however, that too much is often made of his theories on birth order. It is not the birth order, per se, which determines the nature of development. For example, if the oldest child is not competitive, the second child may develop as if they were the first child. Similarly, if two children are born much later than their older siblings, the elder of those two may develop the characteristics of a first born.

In those cases where the challenges of adolescence become too much for a child, they begin to creatively protect themselves by doing things such as forging report cards, skipping school, etc. As they meet others doing the same things, they join with them, form gangs, and may well start out on a road that leads to a life of crime. Demonstrating his great concern for the individual, however, Adler wrote:

All this can be avoided if we accept the point of view of Individual Psychology that no child should be thought hopeless. We must feel that a method can always be found to help a child. Even in the worst of circumstances there is always a particular way of approach - but this of course needs to be found. (pg. 179; Adler, 1930b)

In a recent special issue of the American Psychologist, a series of articles were presented focusing on effective, evidence-based prevention programs designed to increase the number of children and youth who will succeed and contribute both in school and in life (Weissberg, Kumpfer, & Seligman, 2003). In accordance with Adler's theories, effective parenting seems to be the best way to reduce adolescent problem behaviors, and the family can be strengthened through approaches such as behavioral parent training, family skills training, and family therapy (Kumpfer & Alvarado, 2003). School-based prevention programs can also be beneficial, but it is important that educational approaches coordinate social and emotional learning with more traditional academic learning (Greenberg, et al., 2003). Since the problems of adolescence are so variable, including such things as alcohol, tobacco, and other drug abuse, violence, delinquency, mental illness, etc., and since they affect so many children, approaches that attempt to intervene with one child at a time may not be adequate. Accordingly, community interventions become important, and may go so far as to require coordinated national efforts, such as Head Start or the combined efforts of the Department of Health and Human Services, Department of Housing and Urban Development, and the Environmental Protection Agency to reduce lead poisoning (Ripple & Zigler, 2003; Wandersman & Florin, 2003). And last but not least, health care providers can play an important role in ensuring the psychological well-being of their patients, as well as their physical health (Johnson & Millstein, 2003). Indeed, Adler quoted the renowned Rudolf Virchow (one of the founders of cellular pathology; who coined the terms thrombosis, embolism, and leukemia, among many other accomplishments) in saying: "Physicians will eventually become the educators of humanity" (pg. 317; Adler, 1918/1963; see also Knopf & Wexberg, 1930). Perhaps the most important aspect of these studies is the concerted effort to combine scientific research with clinical practice and experience, as well as doing so in socioculturally relevant ways (Biglan, et al., 2003; Nation, et al., 2003). Echoing Adler's call from 1930, and moving toward its answer:

If researchers can foster increased use of scientific practices in these ways, it is possible to achieve a society in which the largest possible proportion of children experience healthy, happy, and successful development and arrive at adulthood with the social, emotional, and cognitive skills they need to lead healthy and successful lives. (pg. 438; Biglan et al., 2003)

The Psychology of Women

Adler's views on the psychology of women could not have been more different than those of Freud. As someone who questioned cultural discrimination long before most, Adler considered women to be equal to men, and he described the belief that they were inferior as a myth. A division of labor has always been an important part of the communal life of the human species. When neighboring groups came into conflict, it was the larger, stronger men who did most of the fighting to protect the group. According to Adler, men extended this conflict and feeling of power to the subjugation of women. Ever since, men have enjoyed privileges that were denied to women, and this has been maintained primarily through force, or the threat of force, but also through indoctrination and education (Adler, 1910/ 1978, 1927/1978, 1928). Adler himself avoided the use of the term "opposite sex," a term that

implies an adversarial relationship, preferring instead to use the term “other sex” when referring to women (Manaster & Corsini, 1982).

Adler recognized that what women really desire is the privilege that men enjoy, but this is not unique to women. There are also men, or boys, who are not dominant, and they also strive for superiority and privilege. At birth, of course, both male and female infants are helpless and inferior, and must begin to strive for superiority. The form that this striving takes is something Adler called the **masculine protest**. It was not his intention to suggest that masculine traits of dominance and aggression make men better than women, but this was the nature of the times in which he lived. It is purely cultural that the male gender role includes strength, knowledge, physical activity, etc., whereas the female gender role includes submissiveness, weakness, the desire for physical and emotional closeness, etc. All children display some degree of these traits, but society directs boys toward the male role, and girls toward the female role (Adler, 1910/1978, 1912a/1963, 1928, 1929/1964). We can now recognize what many consider Freud’s great mistake regarding the psychology of women. Women who display masculine traits were seen as neurotic by Freud, but Adler viewed them as protesting the cultural denigration of women. Still, it is not easy to challenge the nature of society, so Adler still acknowledged that women were more likely to be neurotic than men. However, Adler attributes the neurosis of most women to masculine protest, not to the inability to resolve a woman’s penis envy! In 1910, just as Adler was about to break away from Freud’s Psychoanalytic Society, Adler proposed that the great Oedipus complex is only a small part, just a stage, of the masculine protest, for both men and women (Adler, 1910/1978).

Discussion Question: Adler described masculine protest as a cultural phenomenon in which women, and even some men, strive to act masculine in order to ensure the privilege reserved only for men. Can you think of any strong women whose career or style of life fits into this theory? What about any men you know (or know of)?

Are strong women just that, or are they acting out their masculine protest?

Adlerian Psychotherapy

Adler’s approach to psychotherapy has been the topic of numerous books and chapters (e.g., Dinkmeyer, Dinkmeyer, Jr., & Sperry, 1979; Mosak, 1995; Nikelly, 1971a; Watts & Carlson, 1999), including specific books on family therapy and lifestyle counseling for people with disabilities (Rule, 1984; Sherman & Dinkmeyer, 1987). It is generally accepted that Adlerian psychotherapists have no specific technique, but rather are eclectic in their approach. There are, however, a few key elements to Individual Psychology. First is the goal of understanding the style of life. Once the therapist understands the style of life, they can understand most everything the patient does. The therapist then helps the patient to strengthen their social interest. This involves a practical application of social interest: reorienting and readjusting the patient’s style of life. Underlying the success of therapy in Individual Psychology is a supportive therapeutic relationship. Adler believed in facing his clients, on an equal basis. The therapist helps to educate the patient on the nature of therapy and the goals that might be pursued, which involves helping the client to recognize the mistaken style of life and goals they have been pursuing in the past. In this way, the therapist helps the client in their own creative process of personality change (Dinkmeyer, Dinkmeyer, Jr., & Sperry, 1979; Mosak, 1995). This approach shares many similarities with both the client-centered

approach popularized by Carl Rogers and the behavioral-cognitive approaches developed by Albert Ellis and Aaron Beck.

Understanding the Style of Life

Psychotherapy is about helping people to change their lives for the better. Thus, it involves looking forward, despite whether or not we need to know what has happened in the past (and different theories consider the importance of the past in radically different ways). According to Adler, “in order to understand a person’s future we must understand his style of life” (pg. 99; Adler, 1929a). The style of life, and the associated scheme of apperception, brings all experience into line with the person’s fictional final goal. So, if we can understand how a person is living their life, if we understand their style of life, we can help them to understand it as well, and then perhaps make changes for the better. So, how might we go about understanding the style of life?

As Freud and Breuer had before him, one area of interest for Adler was the analysis of dreams. However, Adler did not distinguish between the conscious and unconscious, or between waking and sleep. He considered dreams to be a reflection of the style of life, and the individual’s striving for superiority. Thus, dreams do not hold any special meaning (Adler, 1912b/1963, 1928, 1929a, 1932b/1964). As such, their interpretation can be relatively straight-forward:

Take, for instance, our knowledge that mankind as a whole is really cowardly. From this general fact we can presuppose that the largest number of dreams will be dreams of fear, danger, or anxiety. And so if we know a person and see that his goal is to escape the solution of life’s problems, we can guess that he often dreams that he falls down. Such a dream is like a warning to him: “Do not go on - you will be defeated.” (pg. 155; Adler, 1929a)

Of greater interest to Adler was the analysis of **early memories**. He believed that the earliest childhood recollections provided valuable information, and he did not let a single patient go without having been asked about them:

The information regarding memory is significant. Memory is an activity. It is based on the life style, which here steps in by selecting from old impressions a single one. This leads us to the question, why this single one? In it the entire life style resonates. (pg. 197; Adler, 1932b/1964)

It is not even important if the memory is, in fact, true. A “memory” may be created by a child based on their conscious experience in order to reflect an attitude, or an emotional tone, if that is necessary to pursue their unique, personal goal (Adler, 1928). Accordingly, if we can obtain a person’s earliest memories, we make reasonable predictions regarding the future course of their life. However, early memories are not reasons for behavior, but they are hints. It is also true that some forgotten memories (or unconscious memories) may have played a role in the creation of the style of life, but they are obviously more difficult to obtain, so they defy analysis. But, Adler did not consider them to be any different in content and tone from the conscious memories; the unconscious psyche enacts the same style of life toward the same final goal (as with dreams; Adler, 1929a, 1931a, 1964). There may, however, be some interesting differences in early memories based on culture. For example, Wang (2006b) examined the earliest memories of European American and Taiwanese college students in response to five cue words: self, mother, family, friend, and surroundings. In both groups, memories for ‘mother’ came from an earlier time than memories for ‘self,’ and

memories for ‘mother,’ ‘family,’ and ‘friend’ were more socially oriented. The European Americans typically had memories from earlier ages, and their memories tended to report more specific events and to focus on their personal roles and autonomy in those memories. Thus, early memories not only reflect one’s style of life, but do so within a cultural context.

In two major case studies published by Adler, The Case of Miss R. (1929b) and The Case of Mrs. A (1931b), the subtitle of each book refers to the style of life. In the latter book, he concludes by saying: “I have simply wanted to show you the COHERENCE OF A LIFE-STYLE” (pg. 46; Adler, 1931b). Moving well beyond neurosis, both Adler and his son Kurt applied the study of the style of life to patients suffering from psychotic disorders, including schizophrenia and manic phase of bipolar disorder (Adler, 1929/1964; Kurt Adler, 1959). Kurt Adler does an excellent job of summarizing Individual Psychology, and he quotes his father’s description of neurosis and psychosis as attempts by a person to avoid clashing with reality and exposing their weakness and inferiority. However, these are “failed” styles of life, because they lead to an avoidance of social interest, which is necessary for solving life’s challenges in cooperation with others. In the case of a disorder such as schizophrenia, the child begins life with extreme feelings of inferiority, which lead to an exaggerated fictional goal. Subsequently, it is more difficult to achieve any success in life, which creates a vicious cycle of failure and greater distrust of others. Finally, as the individual fails to develop social-emotional capacities, common sense, or logic, they retreat into the private world that accommodates their exaggerated fictional personal goal. The subsequent treatment of psychotic patients is difficult, particularly if the therapist expects too much. Kurt Adler extends his father’s caution that the therapist must be aware of their own style of life, set aside any personal expectations, and never expectanything of the patient (Kurt Adler, 1959). As we will see below, Harry Stack Sullivan also paid special attention to psychodynamic processes in schizophrenia.

Discussion Question: What are your earliest memories, especially of your mother, father, and sense of self? Can you see a clear relationship between the content of those memories and your style of life? How does your life plan fit with those memories?

Strengthening Social Interest and Reorienting the Style of Life

Adler believed that everyone has some degree of social interest, but in neurotic and psychotic patients there is relatively less. He described Freudian transference as an expression of the patient’s social interest, directed toward the therapist. Resistance is simply the patient lacking the courage (i.e., feeling too inferior) to return to social usefulness and cooperation with others. The therapist must, therefore, help the patient to strengthen their social interest and reorient their style of life. Since the style of life is creatively formed, it becomes important to understand how and when the failed style of life began. Just as the individual took time and was selective in the formation of their failed style of life, the therapist must take time to understand it and educate the patient about it (Adler, 1913b/1963, 1929/1964, 1930c, 1932b/1964, 1958).

Individual Psychology stresses enormously that the psychological development of a person can reach a normal condition only when he can achieve the necessary degree of ability to cooperate. (pg. 199; Adler, 1932b/1964)

It is almost impossible to exaggerate the value of an increase in social feeling…The feeling of worth and value is heightened, giving courage and an optimistic view…The individual feels

at home in life and feels his existence to be worthwhile just so far as he is useful to others… (pg. 79; Adler, 1929/1964)

There are numerous ways in which a therapist can help a client to increase their social interest. It begins with the therapist openly communicating with and accepting the patient, which is, of course, a social interaction. This is especially effective if the therapist displays social feeling themselves, while also helping the patient to recognize the patient's value as a human being (as well as the value of other people). The therapist can point out that if other people, including the therapist, had the same values, goals, and cognitive orientation as the patient then others would also have the same problems as the patient. The therapist can then nudge the patient toward their own niche, where they can feel comfortable with others without fearing being the loser in personal interactions. And finally, the therapist can encourage the patient to get involved in concerns outside themselves (Nikelly, 1971b). Each of these ways of encouraging social interest can be a part of the reorientation and readjustment process, the "working-through" phase of therapy, in which the patient gains insight into their style of life, confronts their goals, and realizes that they are capable of making choices and decisions (Nikelly & Bostrom, 1971). The patient must be encouraged to understand and believe that they have the power to implement their insights and change their life.

The client must not be allowed to think of himself as the victim of past circumstances but must come to recognize that the problem is his alone and that he alone will solve it. In contrast, emphasis on past history may lead to rationalization and intellectualization without appropriate action. The client learns more and more about his problem and does less and less about it. (pg. 103; Nikelly & Bostrom, 1971)

Group Psychotherapy and Family Therapy

Our discussion of Adlerian psychotherapy would not be complete without mentioning **group psychotherapy**, and in the same breath, **family therapy**. Group psychotherapy probably has multiple origins, having been tried by a variety of independent therapists. In 1921, Adler began interviewing and counseling parents in front of a group of professionals. This fit well within his perspective on the three life tasks and social interest, since he felt that parents, family, and other important people, such as school teachers, needed to be involved in the development of children. Adler soon discovered that these settings led to personal growth in everyone present, establishing a form of group therapy that may be the oldest still in use. Strictly speaking, Individual Psychology does not prescribe any specific techniques for group psychotherapy, but rather, there is Individual Psychology for patients in a private setting as well as for patients receiving therapy in a group setting (Corsini, 1971).

Involving the parents in a child's therapy is, quite obviously, the beginning of family therapy. However, when Adler established his child guidance centers, they were open for parents, teachers, social workers, and other interested people to be able to observe the process of psychotherapy. Adler felt it was important for everyone involved in the development of children to be educated in the important process of Individual Psychology, and the same was true of Rudolf Dreikurs, a student of Adler who opened family education centers in the United States (the first in Chicago in 1937; see Dreikurs, 1950; Sherman & Dinkmeyer, 1987). This education may be particularly important for the parents, whose attitude can have a profound influence on whether psychotherapy for a child is successful or not. As for teachers, understanding Individual Psychology can help them to put a child's academic

performance into a context that helps to explain it as a style of life aimed toward a particular goal, even if that goal is dysfunctional and interferes with schoolwork (Friedmann, 1930; Spiel & Birnbaum, 1930). Overall, the goal of family therapy is to improve communication and cooperation amongst all members of the family, and to help parents avoid the most difficult problems (Alexandra Adler, 1930a; Adler, 1930d).

In our conversations with the parents we relentlessly try to make them realize that children must not be beaten. Beating can only lead to a discouragement of the child, and we know well enough that this can make the child worse and not better…We always dissuade parents from engaging in warfare with the child, since the child inevitably remains in the end the stronger party. (pp. 110-111; Alexandra Adler, 1930a)

Discussion Question: Have you ever experienced group psychotherapy, family therapy, or anything similar (or know anyone who has)? What advantages did you experience, or what do you think you might experience if you were to work on your relationships with family members or other people?

A Brief Biography of Harry Stack Sullivan

Several aspects of Harry Stack Sullivan's life history are quite strange, and so, according to some biographers, was Harry Stack Sullivan himself. He was born on February 21st, 1892, though his medical records say he was born in 1886, in the rural community of Norwich, New York. His father's name was Timothy Sullivan and his mother's name seems to have been Ella Stack, but it remains unclear whether her name was actually Ella. That is the name that appears on Sullivan's birth certificate and on her death certificate, but in the family records her name was listed as Ellen, and on Sullivan's baptismal certificate it was listed as Ellina. Timothy Sullivan was a hired-hand on the Stack farm (or another farm nearby), whose father had died when Timothy Sullivan was young, leaving the family quite poor. There was a great deal of discrimination against Irish Catholics at the time, and Timothy and Ella struggled. They lived in a poor part of town, where there was an improperly drained canal that was believed to be a breeding ground for "black diphtheria." The Sullivans had two children before Harry, both born in February, who died in terrible convulsions before reaching the fall of their first year. When Harry was born in February, his mother was terrified that he would die as well (Alexander, 1990; Evans, 1996; Perry, 1982).

When Sullivan was 2 ½ he was sent to Smyrna, New York to live with his maternal grandmother. Where his mother was for the next few years is unknown. It was rumored that she had a nervous breakdown, and may have been kept hidden away in the attic of the barn until she recovered. Although she eventually returned to public view, she was never really Sullivan's caretaker again. Neither was his father. When Sullivan's maternal grandfather died, there was no one to run the farm, which provided the support for him and his grandmother. Being poor himself, Timothy Sullivan was granted control of the farm. However, the Stack family had always considered Timothy Sullivan to be beneath them (the Stack family had a prestigious ancestry), so the legal documents were quite demeaning to Timothy Sullivan, and the farm's name remained the Stack farm. Timothy Sullivan worked hard, but withdrew into himself and had little contact with others (Alexander, 1990; Evans, 1996; Perry, 1982).

Sullivan did very well in school, but had few friends due to the common prejudice and discrimination. At one point, the Ku Klux Klan burned a cross in front of the Stack farm. His

only friend was a boy who was 5 years older, Clarence Bellinger. The two were very close throughout school, and both went on to become psychiatrists. However, after high school, they never contacted one another again. Bellinger always spoke poorly of Sullivan, and Sullivan simply never spoke of Bellinger. The reasons why are unknown, but it may have something to do with the fact that neither one of them ever married, and Sullivan was widely regarded as a homosexual. When Sullivan was 35, he took in a foster son, who may also have been a psychotic patient. Late in life, Sullivan seems to have referred to the young man as a "lover" (Alexander, 1990). If this was their relationship when his companion was only 15, and if his companion was a former patient, it was both unethical and criminal. It is tempting to suggest that Sullivan's sexual development and alleged later actions may have been influenced by an inappropriate relationship with Bellinger while Sullivan was still quite young. However, the truth is not known, and there are other gaps in the history of Sullivan's life (Alexander, 1990; Evans, 1996; Perry, 1982).

Two family members helped Sullivan with his education during his school years. His mother's sister Margaret, a schoolteacher, brought him many books and introduced him to a wide range of intellectual ideas. His father's brother, Will, was a respected lawyer and then a judge who was influential in applying psychological issues to the law. He helped to foster Sullivan's interest in human problems. This aunt and uncle both later helped Sullivan with college finances, and they helped him to win a prestigious New York State Regents' Scholarship to Cornell University. However, Sullivan was not prepared for college. After a fair start, his grades dropped drastically. He became involved with a group of boys who were illegally obtaining and selling chemicals. Only Sullivan was caught, and he was convicted of mail fraud. His whereabouts for the next 2 years are unknown. He may have been in jail, but there is evidence to suggest that he was hospitalized at Bellevue Hospital following a psychotic break. Later in life he was friends with the renowned A. A. Brill (the psychiatrist who first translated many of Freud's books into English), and Brill was working at Bellevue at the time. Sullivan was also friends later in life with another employee who worked at Bellevue at that time, and no simple explanation can be given for his friendship with two men who worked at Bellevue Hospital during the time Sullivan's whereabouts are unknown (Alexander, 1990; Evans, 1996; Perry, 1982).

In 1911, Sullivan reappeared and entered the Chicago College of Medicine and Surgery. He finished his coursework in 1915, but did not receive a degree, possibly due to owing the school money. He worked briefly as an industrial surgeon in a steel mill, and in 1916 joined the Illinois National Guard. After only 5 months he was released on medical grounds, supposedly due to a broken jaw. He then disappeared for a while, later claiming that he received 75 hours of psychoanalysis during the disappearance. But where that occurred, or with whom, is again unknown. In 1917, he was finally awarded his medical degree, and he joined the U. S. Army. His application contained many inaccuracies and falsifications, including lying about his age. Nonetheless, he was commissioned as a lieutenant, promoted to captain at the end of World War I, and spent 2 years moving between Chicago and Washington, DC. In 1920, he left the Army, and again a year of his life seems to be missing. In November, 1921, he joined St. Elizabeth's Hospital in Washington, DC, and began his career in psychiatry (Alexander, 1990; Evans, 1996; Perry, 1982).

St. Elizabeth's Hospital was one of best psychiatric hospitals of the time (and remains so today), under the leadership of the renowned William Alanson White, a pioneer in applying dynamic psychotherapy to psychotic patients. After a year, however, there was no

permanent position for Sullivan. So, he applied for a position at the Sheppard and Enoch Pratt Hospital in Baltimore (as prestigious as St. Elizabeth's both then and now). White's letter of recommendation for Sullivan was lukewarm and vague. He described Sullivan as "better equipped than the average State Hospital assistant," but he also acknowledged that he didn't really know Sullivan because of Sullivan's personal distance from other people (Alexander, 1990). Nonetheless, Sullivan was hired for his first clinical psychiatric position at Sheppard Pratt (as it is more commonly known - Note: The author's wife had the privilege of working at Sheppard Pratt early in her career), and he began an exciting and innovative period of eight years, during which he did the most important clinical work of his career, including innovative techniques for the treatment of schizophrenia in young patients (Alexander, 1990; Evans, 1996; Perry, 1982). Today, the Sheppard Pratt Health System includes the Harry Stack Sullivan Day Hospital, a partial hospitalization program assisting adults with severe mental illness, including psychotic disorders.

During his time at Sheppard Pratt, Sullivan became close friends with a young psychiatrist named Clara Thompson. They shared an interest in questioning Freud's concepts of the feminine psyche and other challenges to orthodox psychoanalysis. As Sullivan became particularly interested in the work of Sandor Ferenczi (who accompanied Sigmund Freud and Carl Jung on Freud's only trip to America, but later split with Freud), he encourage Thompson to go to Budapest and be psychoanalyzed by Ferenczi. She did so, and upon her return she was Sullivan's training analyst, and Sullivan was later admitted to the American Psychoanalytic Society. The two remained close throughout their lives, and supposedly agreed to marry one another, but both quickly broke off the engagement the very next morning (Alexander, 1990; Evans, 1996; Perry, 1982).

In 1930, Sullivan moved to New York City to establish a private practice and conduct research. However, he failed to make much money, incurred some family debts following his father's death, and he filed for bankruptcy in 1932. The next few years were interesting, but unstable. He was the driving force behind the establishment of the William Alanson White Foundation in 1933, and he actively collaborated with two colleagues at Yale University (where they likely had an influence on Erik Erikson's work). However, one of those colleagues died in 1939, and Sullivan immersed himself in a project studying Black adolescents in Tennessee and Mississippi. He then settled in Bethesda, Maryland, where he spent the rest of his career (Alexander, 1990; Evans, 1996; Perry, 1982).

The year 1939 marked the beginning of a highly productive period for Sullivan. Dexter Bullard, the director of a psychiatric hospital named Chestnut Lodge, provided Sullivan with steady consulting opportunities. This provided Sullivan financial security, and through Chestnut Lodge he met many influential psychiatrists in the Washington, DC area. William Alanson White had died in 1937, and Sullivan delivered a series of public lectures, which became the First William Alanson White Memorial Lectures, and which were released as Sullivan's only book published while he was still alive: Conceptions of Modern Psychiatry (Sullivan, 1940). The White Foundation established the journal Psychiatry, and the Washington School of Psychiatry. Sullivan also encouraged the White Foundation to establish a second training program in New York City, and the core faculty included Clara Thompson and Erich Fromm. The second program is known today as the William Alanson White Institute. When, in 1941, Karen Horney was disqualified as a training analyst by the New York Psychoanalytic Society, Clara Thompson walked out of the meeting, and helped to establish a rival Association for the Advancement of Psychoanalysis, with Horney as its dean.

A series of political battles between opposing psychoanalytic societies followed, in many ways pitting followers of Freud against those whose interests and relationships were closer to Sullivan (Alexander, 1990; Evans, 1996; Perry, 1982).

In 1945, Sullivan became quite ill, and was urged to retire for his health. Instead, Sullivan accepted a new challenge. He was invited by Brock Chisholm, who was soon to become the first director of the World Health Organization, to serve as a consultant for the post-World War II International Congress on Mental Health. Sullivan focused on applying psychiatric principles to problems of world peace, including educating children on values for peace. He helped to establish the World Federation for Mental Health, and participated in the UNESCO Seminar on Childhood Education Toward World-Mindedness. In January 1949, following a particularly frustrating meeting of the World Federation for Mental Health, Sullivan died in a hotel room in Paris, France. He was cremated, as he had requested, and his ashes were buried at Arlington National Cemetery, honoring his service to the U. S. Army Medical Corps during World War I. A committee was established by the White Foundation to publish his papers. Between 1953 and 1972, the committee published seven books of Sullivan's work, ensuring that his theories remained available to the fields of psychiatry and psychology (Alexander, 1990; Evans, 1996; Perry, 1982).

Placing Sullivan in Context: America's Psychodynamic Theorist

Harry Stack Sullivan was an enigmatic character. His parents were poor and, apparently, both suffered from mental illness; his siblings died during their infancies. The region where he was raised was extremely prejudiced against Irish Catholics, and the KKK burned a cross in front of their home. As a student, Sullivan was very successful, but legal problems (due to his own bad choices) and presumed hospitalizations for his own mental illness delayed his success in college and medical school. And yet, he went on to become the most influential psychodynamic theorist born in America, worked at two highly prestigious psychiatric hospitals, co-founded an influential foundation, and has a hospital named after him. Like Adler, however, his contributions seldom receive proper recognition, and he remains somewhat obscure.

Sullivan's most interesting and enduring contributions relate to his relationship with William Alanson White. Although Sullivan only worked with White briefly, and White's letter of recommendation for Sullivan's next position admits that White considered Sullivan to be distant and hard to get to know, Sullivan vigorously pursued White's interest in the treatment of schizophrenia. Sullivan helped to found the William Alanson White Foundation, which established two psychiatric training institutes and the journal Psychiatry, and Sullivan delivered the First William Alanson White Memorial Lectures. Today, the Sheppard Pratt Health System includes the Harry Stack Sullivan Day Hospital (for the treatment of psychotic disorders).

Sullivan's Interpersonal Psychology

Sullivan shares two particular distinctions with Adler: his influence is extraordinarily wide ranging, and he is not very well known. It has been suggested that he was the most original and creative American-born psychiatrist (Chapman & Chapman, 1980). In Corsini & Wedding's Current Psychotherapies (1995), Sullivan is mentioned in ten of the fourteen chapters, including those written by Albert Ellis, Carl Rogers, Aaron Beck, and Rollo May, as well as the chapters written about Sigmund Freud, Alfred Adler, and Carl Jung. Due largely

to his emphasis on interpersonal relationships, he is acknowledged by Jean Baker Miller and other founding members of the Stone Center (see Chapter 9; Jordan, et al., 1991; Miller, 1976). So how can he be relatively unknown? According to Evans (1996), there are a number of reasons for Sullivan's lack of popularity, but two seem to stand out. First, Sullivan did not publish much during his lifetime, and what he did publish was somewhat poorly written and difficult to understand. Second, Sullivan was apparently a very difficult and strange man, whose writings often included derogatory statements about psychiatry and psychiatrists (even though this was his own field). Whatever the reasons are for his lack of renown, Sullivan has had a significant influence on psychiatry and psychology nonetheless.

Whereas Adler considered relationships to be an obvious consequence of social interest, the primary factor in Individual Psychology is the striving for superiority. Social interest and interpersonal relationships, of course, make healthy superiority possible. For Sullivan, however, it was the interpersonal relationships themselves that were paramount:

One achieves mental health to the extent that one becomes aware of one's interpersonal relations…It is part of the framework that supports all explanations of what is going on, what might be going on, and what will presently be going on…It is the necessary formula to which everything must be assimilable, if it is therapy. (pg. 207; Sullivan, 1940)

Euphoria, Tension, and Security

Sullivan believed that we exist somewhere between the states of absolute **euphoria** and absolute **tension**. Absolute euphoria is a state of utter well-being, which, unfortunately, is not really possible. The closest we can come to experiencing absolute euphoria is in the deep sleep of a newborn infant. Tension is the alternative state to euphoria, and tension is very much a part of our lives. It arises from two sources: **needs** and **anxiety**. There are two basic types of needs: those that arise from actual biological needs (food, water, air, etc.), and those that are cultural or learned. In real life, however, these types of needs cannot be separated. An infant cannot satisfy its biological needs, it must be cared for. Thus, Sullivan talked routinely about a mothering need, which is a need for an intimate, interpersonal relationship. When the mother does indeed care for the infant, the infant experiences this as **tenderness**, and the infant develops an ongoing need for tenderness. Thus, through the need for a mother and the need for tenderness, the infant finds itself in a world in which it needs interpersonal relationships for continued survival and psychological development (Chapman & Chapman, 1980; Lundin, 1979; Mullahy & Melinek, 1983; Sullivan, 1940, 1953).

Anxiety is the result of real or imagined threats, and can be experienced by the infant or caused by an anxious mother. In either case, it can be particularly intense in an infant because they cannot specifically do anything about it. Furthermore, unlike biological needs that can be met quite specifically (e.g., a hungry child can be fed), how can anxiety be satisfied? The answer, according to Sullivan, is through the pursuit of interpersonal **security**. In other words, a sense of security, the alternative to anxiety, can be obtained only through relationships that provide the child with tenderness and **empathy**. Sullivan used the term empathy to describe "the peculiar emotional linkage that subtends the relationship of the infant with other significant people - the mother or the nurse" (pg. 17; Sullivan, 1940). Long before infants show any sense of understanding emotional expressions, they seem to be able to share in emotional feelings, through what Sullivan considered an innate capacity for empathy. Even an infant is not, however, merely a recipient of the relationships in which

it is involved, it is an active and engaged person. Likewise, children do not simply wait and hope for security, they actively engage in thoughts and behaviors that Sullivan called **security operations**. Security operations serve to maintain our sense of self-esteem, or self-respect, and they often begin with an emphatic sense of "I." Unfortunately, this leads to an odd paradox: the concept that we can have self-esteem without being in relationship with others:

It is one's prestige, one's status, the importance which people feel one is entitled to, the respect that one can expect from people…that dominate awareness. These things are so focal in interpersonal relations of our day and age that the almost unassailable conviction develops…that each of us, as defined by the animal organism that we were at birth, are unique, isolated individuals in the human world… (pg. 219; Sullivan, 1964)

Dynamisms and the Self-System

Underlying Sullivan's emphasis on interpersonal relationships is the fundamental concept of **dynamism**. Similar to the physical universe, our psychological environment involves ongoing transfers of energy (in the psychological sense) between ourselves and the people we interact with, and this is a dynamic back-and-forth system (i.e., a relationship). A dynamism is a relatively enduring pattern of energy transformation that allows us to characterize a living organism. Each organism, or each person, has many dynamisms, but the ones of particular interest to psychiatrists, according to Sullivan, are those related to how we interact with others (Sullivan, 1953).

One of the most important dynamisms is the **self-system**. The self-system is somewhat unique, in that it integrates and provides meaning for all of the individual's interactions and experiences; Sullivan referred to it as a secondary dynamism (secondary here refers to a higher level of processing, not secondary in importance; Sullivan, 1953). The self-system also integrates the security operations, and serves to protect the individual from tension. Chapman & Chapman (1980) offer a useful description of various ways in which the self-system utilizes security operations in the actions of a 3-year-old child:

1. When faced with an emotionally threatening situation, the child may lapse into stubborn silence.

1. When there is tension between the child and the demands of older children, the child will obstinately resist the efforts of the older children to get the child to conform to their desires.

1. When stressful interpersonal situations arise, the child will withdraw into solitary play.

1. In new, anxiety-provoking environments the child will physically cling to a familiar adult.

1. When anxiety arises between the child and others, the child may engage in meticulous play with inanimate objects (toys, coloring books, etc.).

1. When the child's emotional needs are not met by others, they may regress into more immature behavior and speech.

By observing these behaviors, we can understand who this child is at this point in their life. For a 3-year-old, such behaviors are quite normal. However, such security operations would be considered less appropriate for an adult. And yet, sometimes these behaviors become part of the self-system of an adolescent, or an adult, and Sullivan referred to such instances as **dynamisms of difficulty** (Sullivan, 1956, 1972). When dynamisms of difficulty go into

action, they fail to achieve a desired goal, or at best achieve only an unsatisfactory goal. Since they are relatively enduring, as are all dynamisms, they tend to characterize states of psychological illness, and would indicate the need for psychotherapy. As a curious side-note, while discussing the dynamism of the self-system, Sullivan appears to be the first person to have used the now well-known term **significant other** (Sullivan, 1953).

Another important type of dynamism is the **personification**. A newborn infant cannot really understand who their mother is, or who they themselves are, so the infant develops an image of the mother, the father, themselves, etc. These images, which can sometimes conflict with one another, are called personifications. For instance, when the mother feeds and comforts the infant, she is the "good" mother. But when she fails to display appropriate tenderness, or her own anxiety is expressed to and experienced by the infant, she is the "bad" mother. Later, as the child develops greater intellectual capacity and experience, these personifications are fused into a single mother. Similarly, the infant begins life with personifications of itself as sometimes "bad" and sometimes "good," and later in life fuses these personifications as the adolescent begins to establish a clear identity (Chapman & Chapman, 1980; Mullahy & Melinek, 1983; Sullivan, 1953, 1964). As with all of the concepts we have discussed here, the process of personification continues into adulthood. A mother will develop her own personifications of the infant, which, as mentioned above, may be experienced by the infant. For example, if a baby is colicky (cries constantly and cannot be soothed), it can be very frustrating for a mother, especially a new mother. Her anxiety may lead to a personification of her baby as inconsolable and of herself as a "bad" mother. Her discomfort, then, in caring for her baby can be experienced by the baby, leading to the baby's personification of her as a "bad" mother. The key here, according to Sullivan's theory, is that neither the mother nor the baby is in any way bad or good, inconsolable or tender, except in relation to one another!

Sullivan described the **personified self** as those aspects of personality about which one is consciously aware. This provides important information for a psychotherapist, and pursuing information about the personified self is an important part of the **psychiatric interview** (Sullivan, 1954). During the interview, Sullivan recommended that the psychotherapist examine four aspects of personified self: what does the patient esteem and/or disparage about themselves; to what experiences is the patient's self-esteem particularly and unreasonably vulnerable; what are the characteristic security operations employed when the patient is made anxious; and how great are the patient's reserves of security? In addition to these important points, Sullivan also considered it essential for a psychotherapist to gain an impression of the whole interview situation, particularly with regard to the patient's sense of proportion in terms of where they fit into life. By understanding the personified self, the patient's knowledge of themselves, and how they view themselves within the context of their life, the psychotherapist can gain an understanding of what is realistically possible for the patient during the course of therapy (Sullivan, 1954).

Discussion Question: Sullivan described the personified self as knowing what you like or dislike about yourself, what experiences are particularly damaging to your self-esteem, your characteristic security operations, and how deep your reserves of security are. Consider each of these points, and provide yourself with a self-evaluation. Overall, do you see yourself as a relatively healthy, secure person, or someone whose life is plagues by tension and anxiety?

Developmental Epochs

Sullivan considered an understanding of the course of human development to be essential to understanding individuals. He described seven **developmental epochs**: **infancy**, **childhood**, the **juvenile era**, **preadolescence**, **early adolescence**, **late adolescence**, and **adulthood** (Sullivan, 1953, 1954, 1964). These developmental stages represent a gradual unfolding of the individual's capacity for effective interpersonal relationships or, in other words, the individual's ability to fit into the social structure of their environment (Chrzanowski, 1977; Lundin, 1979). As we all know, children maintain very different relationships than adults do, and we would not expect children to have adult-like relationships. Thus, when attempting to evaluate a person's state of mental health, which Sullivan equated with their ability to form healthy relationships, we clearly need to understand something about what constitutes the developmentally appropriate possibilities.

Infancy begins at birth, and lasts until the acquisition of speech. It is the time of the most empathic connection between the infant and its caregivers, which can be characterized by tenderness and security, or by anxiety and tension. Childhood lasts through the remainder of the preschool years, and involves extending one's interpersonal relationships outside of the immediate family to peers (friends and playmates). In addition, language is incorporated into one's experiences of approval and disapproval. The juvenile era begins as one enters grade school. Once again, interpersonal relations are expanded to include a wider variety of people and situations, including competition with one's peers and subordination to authority figures (such as teachers and the school's principal).

Adolescence involves some very dramatic changes, particularly in the direction of the intensely intimate relationships of adulthood. During preadolescence, there is a marked increase in the closeness of friendships (e.g., "best friends forever!"), which is characterized by an intimate dynamism. Early adolescence is marked by an interest in the other sex, and an erotic dynamism is formed. Late adolescence, which Sullivan placed between the middle high school years the early college years, were a time of developing full and mature interrelationships, as well as understanding of the need for responsible citizenship within one's society. When these dynamisms, including the self-system, become stabilized, and society has transformed the individual into a completely social being, the person has attained adulthood (Chrzanowski, 1977; Lundin, 1979; Sullivan, 1953, 1954, 1964).

Sullivan provided ages for the transition of these developmental epochs, but he also acknowledged that they are based on experience. Thus, it is possible for the stages to occur at different times in unique individuals. The course, however, remains the same, unless the course fails to progress. If an individual is not provided with the opportunities to advance their interpersonal development, there can be an **arrest of development** (Sullivan, 1953, 1964). An arrest of development does not mean that the personality becomes static, but rather, the freedom and velocity of constructive changes becomes markedly reduced. Later, the arrested development becomes apparent through eccentricities in one's interpersonal relations. The individual does not simply continue to act like a normal child, there is an increased likelihood that this "warp" in personality development will also lead to malevolent dynamisms and the consequent social problems (such as delinquency and crime).

Discussion Question: Do you know anyone who demonstrates arrested development (someone who seems immature, especially in relationships)? Are you able to maintain a friendship with that person, or is the situation too stressful?

Final Notes on Harry Stack Sullivan

From his early days working with William Alanson White, Sullivan was particularly interested in the treatment of schizophrenia. One of the books published by the White Foundation was entitled Schizophrenia as a Human Process (Sullivan, 1962), in which his longtime friend Clara Thompson described him as having a “genuine liking and respect” for his patients. Later the White Foundation published a series of seminars Sullivan offered to the psychiatric residents at Sheppard Pratt, as well as members of the Chestnut Lodge and the Washington School of Psychiatry, on the treatment of a young, male schizophrenic. The text includes commentary by others some 25 years after the seminar (see Kvarnes & Parloff, 1976).

Like Alfred and Kurt Adler, Sullivan recognized the difficulty of treating psychotic patients. The prognosis, in Sullivan’s opinion, depends to a large extent upon the patient’s history of successful interpersonal relationships:

…let us consider the empirical fact that the schizophrenia which appears in the form of a sudden dramatic onset is usually considered to have a more favorable prognosis than the schizophrenia with more gradual onset. But in those cases of sudden onset in which satisfactory experience with significant persons in the past is totally lacking, the patient may be practically beyond redemption; he may manifest empirically trustworthy capacity for recovery to the extent of making excellent institutional recovery; but the psychiatrist is daft who expects that he can put the patient out into the world without prompt relapse. (pg. 197; Sullivan, 1956)

Returning to Sullivan’s basic concept of personality, he offered the following definition:

Personality is the relatively enduring configuration of life-processes characterizing all of the person’s total activity pertaining to such other persons, real or fantastic, as become from time to relevant factors in his total situations. (pg. 47; Sullivan, 1972)

While this definition is rather sweeping in its coverage, it remains focused on interpersonal relationships. According to Thompson, Sullivan was very serious about his own relationships:

The quality of his friendship showed the same genuineness and tolerance so characteristic of his relation to patients. He was slow in making friends. He tested them for a long time…Once a person had passed the test he could count on Harry for absolute loyalty. No matter what your mistakes - and he might point them out to you privately - before the world he was on your side…He had a characteristic phrase when parting form a friend - “God keep you.” (pg. xxxiv; Clara Thompson in Sullivan, 1962)

Personality Theory in Real Life: Achieving Athletic Excellence

Despite Physical Challenges

Adler’s studies on inferiority began with physical problems, what he called organ inferiority (Adler, 1917). Most students of Adler look past that medical beginning, and focus instead on the psychological inferiorities that children experience during their development. However, there are many people with organ inferiority, or what we more commonly refer to as disabilities, handicaps, or “challenges.” There may be some debate as to which term is preferred, but since the phrase “politically correct” is itself a contradiction in terms, I will

use the terms **disability** and **handicap** as presented in Warren Rule's book Lifestyle Counseling for Adjustment to Disability (Rule, 1984). In his summary of previous research, Rule adopts the definition of a disability as a "relatively severe chronic impairment of function" that occurs as the result of a congenital defect, disease, or an accident. Accordingly, disability refers to actual physical, mental, or emotional impairments that become a handicap only if they cause lowered self-assessment, reduced activity, or limited opportunities. When disabilities become a handicap, they can affect the individual's entire style of life. Thus, Rule brought together a group of therapists trained in Individual Psychology, and published the aforementioned book on using lifestyle counseling for people with disabilities that have led to handicaps.

However, not everyone with a disability develops a handicap. Instead, some individuals become truly inspirational by the way in which they live their lives in spite of their disability, or rather, as if they simply were not disabled. Erik Weihenmayer (2001; see also Stoltz & Weihenmayer, 2006) was born with retinoscheses, a degenerative eye disease, which slowly destroyed his retinas, leaving him blind by the age of 13. In high school, Erik spent a month one summer at the Carroll Center for the Blind in Massachusetts. The summer camp included a weekend of rock climbing in N. Conway, New Hampshire (where the author has done a lot of rock-climbing). Weihenmayer's rock climbing experience altered his life. He continued climbing rock, and then moved on to ice-climbing and mountaineering. He didn't just followed more experienced climbers up the cliffs, he also learned to lead-climb: placing one's own protection along the climb and then clipping in the rope, what climbers call "the sharp end" of the rope. I had the pleasure of climbing with Erik in Michigan's Upper Peninsula a few years ago, while his seeing-eye dog slept in a sort of ice cave formed by the overhanging ice. It is truly extraordinary to watch him climb. He moves so smoothly, as he feels the ice above with his ice axe, and then sets the ice axe so deliberately when he finds the right spot, that you would not know he was climbing blind if you only watched for a little while. Eventually, Erik decided to pursue the Seven Summits, climbing the highest peak on each continent: Mt. McKinley (N. America), Aconcagua (S. America), Mt. Everest (Asia), Mt. Elbrus (Europe), Vinson Massif (Antarctica), Mt. Kosciusko (Australia), Kilimanjaro (Africa). He accomplished his goal in 2002.

Erik Weihenmayer is by no means the only well-known, disabled climber. In an amazing video, Beyond the Barriers (Perlman & Wellman, 1998), Erik goes climbing with Mark Wellman and Hugh Herr. Wellman was paralyzed from the waist down in a climbing accident (Wellman & Flinn, 1992), and Herr lost both of his lower legs to frostbite after being caught in a vicious winter storm on Mt. Washington, NH (Osius, 1991; Note: The author has suffered a small patch of frostbite during a winter storm on Mt. Washington). In Beyond the Barriers, Herr leads the hike toward the climb, while Erik carries Wellman. Once on the climb, Herr leads the climb, Erik follows, and they set ropes for Wellman to do pull-ups up the cliff. It simply has to be seen to be believed. One of the surprising aspects is how they joke with each other about what they are doing. As Erik is carrying Wellman, Wellman says: "I don't know man. A blind man giving a para a piggyback ride? It's a pretty scary thing!" When Herr starts climbing on a day when it was snowing, he says his hands are getting numb from the cold. So, Erik asks him how his feet feel! Humor was always an important part of Adlerian psychotherapy (Scott, 1984), so perhaps it should not be surprising that a sense of humor is an aspect of their personalities. One of the funniest stories that Erik tells is about the time he accidentally drank out of his climbing partners piss bottle (a bottle used

to urinate inside the tent during storms). Erik became quite upset that the bottle wasn't marked somehow, but his partner defended himself by saying he had clearly written on the bottle which one it was. It slowly dawned on Erik's partner that the writing was of no help to Erik. As another example of Erik's humor, consider the challenge he tried to avoid after having climbed the highest peaks in Africa and North and South America:

Emma Louise Weihenmayer was born on June 21, year 2000, at 3:57 A.M. There is so much to learn about parenthood. Sometimes being a father is about as intense as climbing Denali, Kilimanjaro, and Aconcagua, all in a day. Because I'm blind, I tried to convince Ellie that I couldn't change diapers, but for some reason, she didn't buy it. (pg. 303; Weihenmayer, 2001)

In addition to his climbing, Erik Weihenmayer is a college graduate with a teaching certificate, and he spent some time as a middle school teacher. He also tried the sport of wrestling, and was a wrestling coach. Trevon Jenifer was also a wrestler.

Trevon Jenifer was born without legs. Perhaps even more challenging, however, was the fact that he was the fourth child of a poor, single mother living in a ghetto outside of Washington, DC. Obviously, Trey (the name he goes by) began life facing difficult obstacles, but little by little, things got better. His mother, Connie, made a conscious decision to take care of him the best she could. She soon met Eric Brown, who became Trey's step-father, providing a stable home for their family. He met a wonderful special education teacher named Bob Gray, who got Trey interested in sports, and who helped to make participating in sports a realistic possibility. He eventually joined a wheelchair track and basketball team named Air Capital, and he was very successful on the track, setting national records in the 100-, 200-, and 400-meter races. It was prior to his junior year in high school, however, that his step-father, who had been a wrestler, recommended that Trey try out for the wrestling team, the regular wrestling team.

What Trey wanted more than anything was to fit in, to have a normal social life at school. Being in a wheelchair, that was not likely to happen. However, he felt that sports might help him accomplish that goal, so he did try out for the wrestling team. He worked hard, learned as much as he could, and he made the varsity team as the 103-pound competitor (actually, there was no one else that light on the team, but he didn't know that). His coach, Terry Green, did all he could to help Trey find a wrestling style that would take advantage of his relative arm strength (he made weight without legs, so his upper body was relatively large) while overcoming the disadvantage of not being able to balance or leverage his body weight by spreading out his legs. Now it was up to Trey. He was nervous in his first match, didn't assert himself, and was easily pinned. In his second match he became the aggressor and earned his first victory. The rest of his junior year continued to be a series of wins and losses, and he ended the season 17-18. Of course, it had only been his first season of wrestling.

In anticipation of his senior year in high school, Trey continued to train hard. Outside of the ring he also received recognition, and became a part of the social network of the school. He received a Medal of Courage from the National Wrestling Hall of Fame, he attended his school's prom, and he was chosen as co-captain of the wrestling team. Once again, humor played a role, as he compared his strength to a teammate from the previous year. Trey had made significant strides in how much he could bench-press, so his former teammate asked

him how much he could squat (a lift done entirely with the legs)! Both wrestlers enjoyed a good laugh at that one.

Trey was doing quite well in league wrestling, and he also began to do well in tournaments. Eventually, he won a tournament, ended his season at 26-6, and from there went on the state championship. He won his first match, but then had to face an undefeated wrestler. He lost, but in that loss there was a sense of accomplishment due to how far he had come:

I lost 5-2…I was hurt less by the fact that I lost, and more by the closeness of it. This one hurt even more because of how close I was to beating the best wrestler in the state. Sharbaugh went on to win the state championship. In fact, he won his last two matches very convincingly, 6-0, and, 12-5. He told reporters afterward that my match was his toughest of the tournament. (pp. 171-172; Trevon Jenifer in Jenifer & Goldenbach, 2006).

The next morning he had to return to the championships to wrestle for a chance at third place in the state. He began with a vengeance, scoring victories of 9-1 and 9-2. His next match, and a shot at third place, was not so easy, but he won 3-1, earning his 30th victory of the season. He then won his final match, and earned third place in the state championship. However, an even more important challenge now loomed ahead of him: college.

Coming from a poor, Black family, there was no tradition of children going to college. However, a group of concerned philanthropists became interested in supporting his dreams. His old coach at Air Capital had talked to Jim Glatch, who coached wheelchair basketball at Edinboro College in Pennsylvania, a school with a large population (10 percent) of students with disabilities. Trevon Jenifer currently attends Edinboro College and plays on the wheelchair basketball team. He does not know if he will ever wrestle again, but it is interesting to note that Edinboro College has a famous wrestler as their athletic director: two-time Olympic gold medalist Bruce Baumgartner!

Recently, Trey was kind enough to respond to an email I sent him, and he provided me with an update on how things have gone during his first year of college. He misses wrestling very much, but he has really enjoyed his return to wheelchair basketball. It probably didn’t hurt that the team is very good, and they came in second-place in the NCAA championship for wheelchair basketball. Trey maintained good grades, his family strongly supports him in pursuing his education, and he has made many new friends. But a few challenges remain. It has been a little difficult for him to get used to the weather in northwestern Pennsylvania, and he has been too busy to attend as many book signings as his publisher would like (but he says they have been very understanding). As for becoming the inspiration his mother thought he was born to be:

I think that I have inspired some people, and I think that is great, but I don’t think that it has reached all the people that I would like it too. My family says that I have done a good job, but I [think that I could do a lot better], and I will try until I feel that I have reached that. (Trevon Jenifer; personal communication, 2007)

The range of sports in which disabled individuals compete is extraordinary today. Beyond the Barriers also includes disabled individuals sailing, scuba diving, surfing, and hang gliding. A few years ago, I began practicing Tae Kwon Do, and I soon discovered that I had degenerative joint disease in both hips. I considered quitting Tae Kwon Do, but I was strongly encouraged to continue by my instructors, as well as by my orthopedic surgeon and

physical therapist. I have since learned that Dirk Robertson, a former social worker turned actor and writer, has worked hard promoting martial arts training for people with disabilities (Robertson, 1991; see also McNab, 2003). Each person simply needs to be encouraged to do their best. Adler suggested that the best way to strive for superiority was through social interest. Whether it's a climbing partner, a wrestling team, a wheelchair basketball team, a martial arts school, whatever, when people work together to help each individual achieve their potential, it can prove to be a highly rewarding experience.

Whilst it is important to be sensitive to their particular situation, their disability should not be the central focus all the time. Their ability to learn, listen and adapt should be built on and encouraged. Do not be over-protective or an instant expert on people with disabilities. The experts are the people themselves, so listen to what they have to say. (pp. 101-102; Robertson, 1991)

Review of Key Points

• Adler faced death numerous times as a child, including his brother's actual death. These events had a profound impact on the nature of his theories.

• Adler helped Freud gain recognition for Freud's personality theory. This lends credence to Adler's claim that he was never a student or follower of Freud, but rather a colleague interested in similar psychiatric/psychological questions.

• The fundamental aspect of Individual Psychology is that we are born inferior and spend our lives striving for superiority.

• Striving for superiority takes the form of compensating for our weaknesses, which, unfortunately, can sometimes lead to overcompensation.

• When individuals can not compensate, they may develop an inferiority complex. Extreme feelings of inferiority can lead to the paradoxical superiority complex.

• Adler believed that all thought and behavior was tied to some goal. Our overriding goal is the life plan, and we pursue it by living a characteristic style of life.

• Dysfunctional styles of life can result from pampering and neglect, both of which reduce our social interest.

• According to Adler, social interest is the best way to achieve superiority, and it can most easily be seen in cooperation.

• Formal examples of cooperation include teamwork, which can lead to either positive or negative outcomes, depending on the circumstances.

• Adler described three life tasks: work, communal life, and love.

• Important aspects of our ability to strive for superiority are the creative power of the individual and the goal we set as the fictional finalism.

• Each person exists within their own perception of the world, known as the scheme of apperception. This scheme guides all experience to fit into our style of life and our goals.

• Since child development is so important, Adler created child guidance centers to train both parents and schoolteachers in the principles of Individual Psychology.

• Adler was very interested in the effects of birth order and the family constellation. Being an only child is likely to result in pampering, whereas an oldest child must deal with being dethroned.

• In an attempt to understand the psychological motivation of women, and some men as well, Adler proposed the theory of masculine protest.

• The key to Adlerian psychotherapy is understanding the patient’s style of life.

• An important technique for revealing the style of life is the early memories test.

• Once the style of life is understood, Adlerian psychotherapists work to strengthen social interest and reorient the style of life.

• Adler was an early innovator in terms of both family therapy and group psychotherapy.

• Sullivan began his formal psychiatric career at two prestigious hospitals: St. Elizabeth’s and Sheppard Pratt.

• According to Sullivan, we are a constant state of tension, due to our needs and anxieties. We seek security, and we employ security operations to reduce our anxiety and tension.

• Sullivan referred to the energy transformation that underlies our personal interactions as dynamisms. Perhaps the most important dynamism is the self-system.

• When an individual develops dysfunctional security operations, Sullivan referred to them as dynamisms of difficulty.

• Before we can truly understand relationships, we develop images in our mind known as personifications. The personified self includes everything we can consciously describe about ourselves.

• Sullivan described seven developmental epochs, which provide a framework for our unfolding abilities to engage in healthy interpersonal relationships.

CHAPTER 06 / CAC.902

Horney and Fromm: Anxiety, Culture, and Character

Karen Horney and Erich Fromm extended psychodynamic thinking by insisting that culture and social conditions shape personality. They did not treat the individual as a sealed container of drives. They asked how family, gender, economic pressure, and social values create anxiety and then shape the ways people protect themselves. For counselors working with children and adolescents, this is a useful correction. A young person's distress may be a response to an environment that demands too much, offers too little safety, or confuses love with performance.

Horney: basic anxiety and protective moves

Horney described basic anxiety as the feeling of being isolated and helpless in a potentially hostile world. A child who cannot rely on warmth may develop protective strategies. Horney grouped these into moving toward people, moving against people, and moving away from people. Moving toward seeks safety through compliance and affection. Moving against seeks safety through power and control. Moving away seeks safety through distance and self-sufficiency. These are not fixed personality types. They are strategies that can become rigid when anxiety is high.

A child who moves toward may be called sweet but may be terrified of disapproval. A child who moves against may be called defiant but may expect attack. A child who moves away may be called independent but may have given up on help. The counselor asks what the strategy protects and what it costs. The goal is not to remove the strategy by force. The goal is to make the child safer so that more flexible choices become possible.

The tyranny of the should

Horney also described the tyranny of the should: internal demands that a person must be perfect, always loving, never needy, or always strong. These demands create a gap between the real self and an idealized image. The person may then hate the real self for failing to match the ideal. Adolescents are especially vulnerable because identity is forming and social comparison is intense. A teen may believe, "I should never feel jealous," and then feel shame when jealousy appears. The counselor can help the client separate a value from a tyrannical demand.

Fromm: freedom, relatedness, and character

Erich Fromm argued that human beings need relatedness, transcendence, rootedness, identity, and a frame of orientation. Freedom can be experienced as both opportunity and burden. When people cannot bear aloneness, they may escape freedom through authoritarianism, destructiveness, or automatic conformity. Fromm's social character concept describes patterns shared by members of a society because they adapt to common conditions. A marketing culture may reward having over being. A school culture may reward ranking over learning. The counselor should not reduce social analysis to individual pathology.

Fromm distinguished productive and nonproductive orientations. A productive orientation involves love, reason, and creative participation. Nonproductive orientations include receptive, exploitative, hoarding, and marketing styles. These are not diagnoses. They are lenses for asking how a person relates to the world. A teenager who treats the self as a product to be optimized may be expressing a marketing orientation. The counselor can ask, "If you were not being evaluated, what would you want?"

Clinical example

Sixteen-year-old Tolu is exhausted by extracurricular achievements. Her parents say they only want her to be happy, but every conversation returns to university admission. Tolu has headaches and says she feels empty. A Horneyan formulation might note moving toward: Tolu complies to keep love. A Frommian formulation might note a marketing self: Tolu experiences herself as a résumé. The counselor does not blame the parents. The counselor helps the family notice the pressure cycle and helps Tolu identify values that are not performance-based.

Intervention may include family sessions, values clarification, and small acts of self-definition. Tolu might drop one activity and observe the family response. The experiment tests whether love survives imperfection. The counselor also checks for depression and safety. Cultural critique does not replace clinical assessment.

Gender and culture

Horney challenged Freud's views of women, arguing that cultural restrictions, not biology alone, shape feminine psychology. She criticized penis envy as a male-centered construct and emphasized the impact of gender inequality. This critique remains relevant when counselors work with girls who are shamed for anger, boys who are shamed for vulnerability, or gender-diverse youth who are forced into narrow roles. The counselor should ask whose standards define health. A quiet girl may be praised while her needs are ignored. An aggressive boy may be punished while his fear is ignored.

Limits and integration

Horney and Fromm offer rich clinical language but less standardized treatment protocols than CBT. Their ideas can become vague if every problem is called cultural. The counselor should connect social analysis to specific mechanisms: family rules, peer norms, economic stress, discrimination, or school pressure. Integration is natural. A counselor may use Horney's protective moves in formulation, CBT for panic symptoms, and family therapy to change the pressure system.

Reflection

Write three shoulds you carry. For each, ask who taught it, what it protects, and what it costs. Then ask what value might remain if the tyranny were removed. This exercise is not about rejecting all standards. It is about distinguishing chosen values from fear-based demands.

Protective moves under anxiety



Horney's protective moves can become costly when they are the only options available.

Attributed Foundational Reading

Attribution and critical-reading note

Adapted from Personality Theory in a Cultural Context by Mark D. Kelland, OpenStax CNX / Lansing Community College (2015), CC BY 4.0. Selection, organization, and PDF layout modified for this course; new lectures and course diagrams are separate additions.

Read historical and theoretical claims critically. A theory is a lens for inquiry, not a verdict about a child or family. Where source claims are dated or culture-bound, the new lecture supplies cautions and contemporary framing.

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Karen Horney stands alone as the only women recognized as worthy of her own chapter in many personality textbooks, and the significance of her work certainly merits that honor. She did not, however, focus her entire career on the psychology of women. Horney came to believe that culture was more important than gender in determining differences between men and women. After refuting some of Freud’s theories on women, Horney shifted her focus to the development of basic anxiety in children, and the lifelong interpersonal relationship styles and intrapsychic conflicts that determine our personality and our personal adjustment.

Personally, Horney was a complex woman. Jack Rubins, who knew Horney during the last few years of her life, interviewed many people who knew her and came away with conflicting views:

She was described variously as both frail and powerful, both open and reticent, both warm and reserved, both close and detached, both a leader and needing to be led, both timid and awesome, both simple and profound. From these characterizations, the impression emerges that she was not only a complex personality but changeable and constantly changing. She was able to encompass and unify, though with struggle, many diverse attitudes and traits… (pg. 13; Rubins, 1972)

Erich Fromm, who was a lay-analyst with a Ph.D. (not an M.D. like most early psychoanalysts), focused even more than Horney on social influences, particularly one's relationship with society itself. He not only knew and worked with Horney personally, but the two were intimately involved for a number of years, and Fromm analyzed Horney's daughter Marianne. Both Horney and Fromm can be seen as extending Adler's emphasis on social interest and cooperation (or the lack thereof), and their belief that individuals pursue safety and security to overcome their anxiety is similar to Adler's concept of striving for superiority.

Brief Biography of Karen Horney

Karen Clementine Theodore Danielssen was born on September 16th, 1885, in Hamburg, Germany. Her father was Norwegian by birth, but had become a German national. A successful sailor, he had become the captain of his own ship, and his family accompanied him on a few of his voyages, including trips around Cape Horn, along the west coast of South America, and as far north as San Diego in the United States. Those trips established a life-long interest in travel, foreign customs, and diversity in the young Karen Horney. Although her father was a stern and repressive man, her mother, who was Dutch and 17 years younger than Horney's father, was a dynamic, intelligent, and beautiful woman who maintained a very happy home for the children (Kelman, 1971; Rubins, 1972, 1978).

From early childhood, Horney enjoyed reading, studying, and going to school. She was particularly interested in the novels of Karl May, who often wrote about the Native Americans, and Horney would play many games in which she pretended to be an Indian (usually, Chief Winnetou, a fictional character from May's novels). Her father believed that education was only for men, but her mother encouraged Horney's schooling, and in doing so, set an example of independence that greatly influenced Horney's life and career. Horney followed the traditional education of the day, covering science, math, French, Latin, English, and the humanities. She also took special classes in speech, and for a time was very interested in dancing, drama, and the theatre. Despite the challenging curriculum, she was an excellent student, and often placed first in her class. After being impressed by a friendly country doctor when she was 12, she decided to pursue a career in medicine. When she began college at the University of Freiburg-in-Breisgau, at the age of 20, her mother came along to get her settled in and care for her. Horney soon became good friends with Ida Grote, who moved in with Horney and her mother to help offset the costs of attending college. In 1906, Horney also met her future husband, Oskar Horney (Kelman, 1971; Rubins, 1972, 1978).

Over the next few years, she began her medical studies at the University of Gottingen, and then transferred to the University of Berlin, where she received her medical degree in 1911. In 1909 she had married Oskar Horney, who was described as a tall, slim, handsome man, a brilliant thinker, gifted organizer, and possessing great physical and emotional strength. He also attended the University of Berlin, eventually receiving doctorate degrees in Law, Economics, and Political Science! They soon had three daughters, Brigitte, Marianne, and Renate (between 1911 and 1915). Both Karen and Oskar Horney were successful in their careers during the beginning of their marriage. He worked as a lawyer for a munitions company, and did very well financially. She was actively developing her medical career, but had to work that much harder due to continued discrimination against women at the time. Still, the family spent time together on weekends, when her brother's family often visited,

and vacations. Nonetheless, the Horneys grew apart during these years. In 1923, during the turmoil following World War I, Oskar's investments collapsed, and he eventually went bankrupt. A year later, he was stricken with severe encephalomeningitis, and spent 8 months in critical condition. These events radically altered his personality, as he became a broken and depressed person. In 1926 they separated, and never got back together. It was not, however, until 1939 that Karen Horney legally divorced her husband (Kelman, 1971; Rubins, 1972, 1978).

For Karen Horney's career, the years in Berlin were important and productive. She entered into psychoanalysis with Karl Abraham, and later she was also analyzed by Hanns Sachs for a brief time. Abraham appointed her as an instructor in the Berlin Psychoanalytic Poliklinik in 1919, and brought her to the attention of Sigmund Freud (with high praise). She came to know many of the candidates for psychoanalytic training, and also became friends with many of them, including Melanie Klein, Wilhelm Reich, and Erich Fromm. She also had many friends outside psychoanalytic circles, including the existential theologian Paul Tillich and the neurologist Kurt Goldstein (who coined the term self-actualization). The psychoanalytic scene in Berlin was active and dynamic, and Horney was very much in the middle of it all, never shy about expressing her own ideas and different opinions. One such issue was that of training lay-analysts (psychologists, as opposed to psychiatrists). She favored allowing the training for the purposes of research, but clearly favored medical training for those who would actually practice therapeutic psychoanalysis. This eventually led to conflict between Horney and her close friend Erich Fromm. Despite the many favorable circumstances in Berlin at the time, in the early 1930s Hitler was elected, and the Nazi regime began. Although Horney was not Jewish, psychoanalysis was considered a "Jewish" science. So, when Franz Alexander, who had been asked to come to Chicago to establish a new psychoanalytic training institute, asked her to be the Associate Director of the newly established Chicago Institute of Psychoanalysis, she accepted (Kelman, 1971; Rubins, 1972, 1978). This dramatic turn in the events of her life did not, however, occur without a bit of chance. Alexander had first asked Helene Deutsch, one of the first women to join Freud's psychoanalytic group (see Sayers, 1991), but Deutsch was not interested at the time. Thus, Horney was the second choice for the position that brought her to America for the rest of her life (Kelman, 1971; Rubins, 1972, 1978).

Once in Chicago, however, her theoretical differences with Alexander became a clear source of disagreement. Alexander was not willing, as Horney was, to discard significant elements of Freud's original theories. So, just 2 years later, in 1934, Horney moved to New York City and joined the New York Psychoanalytic Institute. A number of her friends from Berlin had also come to New York, including Erich Fromm and Paul Tillich, and Wilhelm Reich also visited her there. She soon met Harry Stack Sullivan and Clara Thompson, as they were establishing their new training institute in New York. She also began teaching at the New School for Social Research, and the American Institute for Psychoanalysis. Her private practice grew steadily, and Alvin Johnson, the president of the New School (as it is commonly known) introduced her to W. W. Norton, who established a well-known publishing house that produced all of Horney's books. Her first book was entitled The Neurotic Personality of Our Time (1937), which was followed by perhaps her two most radical books, New Ways in Psychoanalysis (1939) and Self-Analysis (1942). Horney had pursued new techniques in psychoanalysis and self-analysis, in part, because of her dissatisfaction with her own results as both a patient and a psychoanalyst. Later, she published Our Inner

Conflicts (1945), Are You Considering Psychoanalysis (1946), and Neurosis and Human Growth: The Struggle Toward Self-Realization (1950). After her death, Harold Kelman (who was both a friend and colleague) brought together a number of her early papers in Feminine Psychology (Kelman, 1967), and, as a special tribute, Douglas Ingram published the transcripts of her final lectures, presented during a class she taught in the fall of 1952 (Ingram, 1987).

During the 1930s and 1940s, Horney's personal life was a social whirlwind. She entertained frequently, often cooking herself, and when her own home was in disarray she would arrange the party at a friend's home. She bought and sold vacation homes often, including one where Oskar Horney stayed for a time, and she traveled frequently. She enjoyed playing cards, and wanted to win so much that she would sometimes cheat! When caught, she would freely admit it, laugh, and say that her opponents should have stopped her sooner. Sometimes she would even gather her friends together and loudly sing German songs, in memory of their homeland (Kelman, 1971; Rubins, 1972, 1978).

At work, however, there was constant tension regarding theoretical and political issues in the psychoanalytic societies. In 1941, the New York Psychoanalytic Institute voted to disqualify Horney as a training analyst, due to her seemingly radical ideas on psychoanalytic techniques. Half the society did not vote, however, and they soon left to form a new institute. Immediately following the vote, Horney walked out, and a group of analysts led by Clara Thompson followed her. The very same month, twenty analysts joined Horney in forming the Association for the Advancement of Psychoanalysis, and Horney was asked to become the Dean of their soon to be established American Institute for Psychoanalysis. When Thompson suggested that Sullivan be granted honorary membership, and Horney recommended the same for Fromm, Fromm refused because he was not going to be recognized as a clinical psychoanalyst. The resulting controversy led to a committee review, which voted against Fromm's membership. Among others, Fromm, Thompson, and Sullivan left the society. There were other political battles as well, and Horney was routinely torn between her professional beliefs, her need to control the direction of the society and institute, and her personal friendships with the individuals involved. Through it all, although she held strong beliefs (such as opposing therapeutic psychoanalysis by lay-analysts like Fromm), she nonetheless encouraged challenging the original theories developed by Freud, as well as her own theories:

I recall being impressed by her response at my first meeting with her, when I indicated my own curiosity and bent for research. She had warmly hoped I would continue this way, since her views needed further work and clarification. Indeed, during an interview in 1952, she stated that she knew her ideas would be changed, if not by herself by someone else. (pg. 37; Rubins, 1972)

By 1950, Horney seemed to be feeling lonely and isolated. Perhaps the political and theoretical battles had taken their toll, perhaps it was her strained relationships with her daughters (they were never really close), or perhaps it was the beginning of the cancer that would eventually take her life. Although Horney would not consult with her physician about the abdominal pains she was experiencing (thus she did not know that she had cancer), she did begin to develop strong spiritual interests. She occasionally attended Tillich's sermons at St. John the Divine Church, though she seemed more interested in the philosophical and ethical aspects of religion than the spiritual aspects. She kept a copy of Aldous Huxley's The

Perennial Philosophy (1945/2004) by her bedside for over a year, reading daily on Huxley's interpretations of Eastern and Western mystics. A few years earlier she had met D. T. Suzuki, and she became particularly interested in Zen. She was especially impressed by a book he recommended entitled Zen in the Art of Archery (Herrigel, 1953; based on an article he wrote in 1936). In 1951, Suzuki led Horney on a trip to Japan, where she visited a number of Zen temples and had lengthy discussions with Zen monks. Although she seemed more interested in the practical aspects of being a student of Zen, she nonetheless endeavored to put Zen principles into a context she could understand (such as equating enlightenment with self-realization; Rubins, 1972, 1978). Late in 1952, her cancer became so advanced that she finally sought medical care. However, it was too late. On December 4, 1952, she died peacefully, surrounded by daughters.

Placing Horney in Context: Culture and the Female Psyche

Karen Horney's career intersected many areas of psychology, relevant both to the past and to the future. One of the first women trained in psychoanalysis, she was the first to challenge Freud's views on women. She did not, however, attempt to reject his influence, but rather, felt that she honored him by building upon his achievements. The most significant change that she felt needed to be made was a shift away from the biological/medical model of Freud to one in which cultural factors were at least as important. Indeed, she challenged Freud's fundamental belief that anxiety follows biological impulses, and instead suggested that our behaviors adapt themselves to a fundamental anxiety associated with the simple desire for survival and to cultural determinants of abnormal, anxiety-provoking situations.

Horney was also significant in the development of psychodynamic theory and psychoanalysis in America. She helped to establish psychoanalytic societies and training institutes in Chicago and New York. She was a friend and colleague to many influential psychoanalysts, including Harry Stack Sullivan and Erich Fromm. She encouraged cross-cultural research and practice through her own example, not only citing the work of anthropologists and sociologists, but also through her personal interest and support for the study of Zen Buddhism.

Although Horney herself abandoned the study of feminine psychology, suggesting instead that it represented the cultural effect of women being an oppressed minority group, her subsequent emphasis on the importance of relationships and interpersonal psychodynamic processes laid the foundation for later theories on the psychology of women (such as the relational-cultural model). Thus, her influence is still being felt quite strongly today.

Horney's Shifting Perspectives on Psychodynamic Theory

Horney did not establish a specific theory of personality. Rather, her career proceeded through a series of stages in which she addressed the issues that were of particular concern to her at the time. Accordingly, her theories can be grouped into three stages: feminine psychology, culture and disturbed human relationships, and finally, the mature theory in which she focused on the distinction between interpersonal and intrapsychic defenses (Paris, 1994).

Feminine Psychology

Horney was neither the first, nor the only, significant woman in the early days of psychodynamic theory and psychoanalysis. However, women such as Helene Deutsch, Marie

Bonaparte, Anna Freud, and Melanie Klein remained faithful to Freud's basic theories. In contrast, Horney directly challenged Freud's theories, and offered her own alternatives. In doing so, she offered a very different perspective on the psychology of women and personality development in girls and women. Her papers have been collected and published in Feminine Psychology by her friend and colleague Harold Kelman (1967), and an excellent overview of their content can be found in the biography written by Rubins (1978).

In her first two papers, On the Genesis of the Castration Complex in Women (Horney, 1923/1967) and The Flight from Womanhood (Horney, 1926/1967), Horney challenged the Freudian perspective on the psychological development of females. Although she acknowledged Freud's pioneering theories, even as they applied to women, she believed that they suffered from a male perspective, and that the men who originally offered these theories simply did not understand the feminine perspective. Horney agreed that girls develop penis envy, but not that it is the only dynamic force influencing development during the phallic stage. Girls envy the ability of boys to urinate standing up, the fact that boys can see their genitals, and the relative ease with which boys can satisfy their desire for masturbation. More important for girls than penis envy, however, was the fear and anxiety young girls experience with regard to vaginal injury were they to actually have intercourse with their fathers (which, Horney agreed, they may fantasize). Thus, they experience a unique dynamic force called **female genital anxiety**. Another element of the castration complex in women, according to Horney, was the consequence of castration fantasies that she called **wounded womanhood** (incorporating the belief that the girl had been castrated).

Far more important than these basic processes, however, was the male bias inherent in society and culture. The very name phallic stage implies that only someone with a phallus (penis) can achieve sexual satisfaction and healthy personality development. Girls are repeatedly made to feel inferior to boys, feminine values are considered inferior to masculine values, even motherhood is considered a burden for women to bear (according to the Bible, the pain of childbirth is a curse from God!). In addition, male-dominated societies do not provide women with adequate outlets for their creative drives. As a result, many women develop a **masculinity complex**, involving feelings of revenge against men and the rejection of their own feminine traits. Thus, it may be true that women are more likely to suffer from anxiety and other psychological disorders, but this is not due to an inherent inferiority as proposed by Freud. Rather, women find it difficult in a patriarchal society to fulfill their personal development in accordance with their individual personality (unless they naturally happen to fit into society's expectations).

Perhaps the most curious aspect of these early studies was the fact that Horney turned the tables on Freud and his concept of penis envy. The female's biological role in childbirth is vastly superior (if that is a proper term) to that of the male. Horney noted that many boys express an intense envy of pregnancy and motherhood. If this so-called **womb envy** is the male counterpart of penis envy, which is the greater problem? Horney suggests that the apparently greater need of men to depreciate women is a reflection of their unconscious feelings of inferiority, due to the very limited role they play in childbirth and the raising of children (particularly breast-feeding infants, which they cannot do). In addition, the powerful creative drives and excessive ambition that are characteristic of many men can be viewed, according to Horney, as overcompensation for their limited role in parenting. Thus, as wonderful and intimate as motherhood may be, it can be a burden in the sense that the men who dominate society have turned it against women. This is, of course, an illogical state

of affairs, since the children being born and raised by women are also the children of the very men who then feel inferior and psychologically threatened.

In a later paper, Horney (1932/1967) carried these ideas a step further. She suggested that, during the Oedipus stage, boys naturally judge the size of their penis as inadequate sexually with regard to their mother. They dread this inadequacy, which leads to anxiety and fear of rejection. This proves to be quite frustrating, and in accordance with the frustration-aggression hypothesis, the boy becomes angry and aggressive toward his mother. For men who are unable to overcome this issue, their adult sexual life becomes an ongoing effort to conquer and possess as many women as possible (a narcissistic overcompensation for their feelings of inadequacy). Unfortunately, according to Horney, these men become very upset with any woman who then expects a long-term or meaningful relationship, since that would require him to then prove his manhood in other, non-sexual ways.

For women, one of the most significant problems that results from these development processes is a desperate need to be in a relationship with a man, which Horney addressed in two of her last papers on feminine psychology: The Overvaluation of Love (1934/1967) and The Neurotic Need for Love (1937/1967). She recognized in many of her patients an obsession with having a relationship with a man, so much so that all other aspects of life seem unimportant. While others had considered this an inherent characteristic of women, Horney insisted that characteristics such as this **overvaluation of love** always include a significant portion of tradition and culture. Thus, it is not an inherent need in women, but one that has accompanied the patriarchal society's demeaning of women, leading to low self-esteem that can only be overcome within society by becoming a wife and mother. Indeed, Horney found that many women suffer an intense fear of not being normal. Unfortunately, as noted above, the men these women are seeking relationships with are themselves seeking to avoid long-term relationships (due to their own insecurities). This results in an intense and destructive attitude of rivalry between women (at least, those women caught up in this neurotic need for love). When a woman loses a man to another woman, which may happen again and again, the situation can lead to depression, permanent feelings of insecurity with regard to feminine self-esteem, and profound anger toward other women. If these feelings are repressed, and remain primarily unconscious, the effect is that the woman searches within her own personality for answers to her failure to maintain the coveted relationship with a man. She may feel shame, believe that she is ugly, or imagine that she has some physical defect. Horney described the potential intensity of these feelings as "self-tormenting."

In 1935, just a few years after coming to America, Horney rather abruptly stopped studying the psychology of women (though her last paper on the subject was not published until 1937). Bernard Paris found the transcript of a talk that Horney had delivered that year to the National Federation of Professional and Business Women's Clubs, which provided her reasoning for this change in her professional direction (see Paris, 1994). First, Horney suggested that women should be suspicious of any general interest in feminine psychology, since it usually represents an effort by men to keep women in their subservient position. In order to avoid competition, men praise the values of being a loving wife and mother. When women accept these same values, they themselves begin to demean any other pursuits in life. They become a teacher because they consider themselves unattractive to men, or they go into business because they aren't feminine and lack sex appeal (Horney, cited in Paris,

1994). The emphasis on attracting men and having children leads to a “cult of beauty and charm,” and the overvaluation of love. The consequence of this tragic situation is that as women become mature, they become more anxious due to their fear of displeasing men:

…The young woman feels a temporary security because of her ability to attract men, but mature women can hardly hope to escape being devalued even in their own eyes. And this feeling of inferiority robs them of the strength for action which rightly belongs to maturity.

Inferiority feelings are the most common evil of our time and our culture. To be sure we do not die of them, but I think they are nevertheless more disastrous to happiness and progress than cancer or tuberculosis. (pg. 236; Horney cited in Paris, 1994)

The key to the preceding quote is Horney’s reference to culture. Having been in America for a few years at this point, she was already questioning the difference between the greater opportunities for women in America than in Europe (though the difference was merely relative). She also emphasized that when women are demeaned by society, this had negative consequences on men and children. Thus, she wanted to break away from any perspective that led to challenges between men and women:

…First of all we need to understand that there are no unalterable qualities of inferiority of our sex due to laws of God or of nature. Our limitations are, for the greater part, culturally and socially conditioned. Men who have lived under the same conditions for a long time have developed similar attitudes and shortcomings.

Once and for all we should stop bothering about what is feminine and what is not. Such concerns only undermine our energies…In the meantime what we can do is to work together for the full development of the human personalities of all for the sake of general welfare. (pg. 238; Horney cited in Paris, 1994)

In her final paper on feminine psychology, Horney (1937/1967) concludes her discussion of the neurotic need for love with a general discussion of the relationship between anxiety and the need for love. Of course, this is true for both boys and girls. This conclusion provided a clear transition from Horney’s study of the psychology of women to her more general perspectives on human development, beginning with the child’s need for security and the anxiety that arises when that security seems threatened.

Discussion Question: After a number of years studying feminine psychology, Horney came to believe that women are no different than any other minority group, and she began to pursue different directions in her career. Are the problems faced by women different than other minority groups? If so, how are they different?

Anxiety and Culture

In the introduction to The Neurotic Personality of Our Time, Horney (1937) makes three important points. First, she acknowledged that neuroses have their roots in childhood experiences, but she also considered the experiences of adulthood to be equally important. Second, she believed that neuroses can only develop within a cultural context. They may stem from individual experience, but their form and expression are intimately tied to one’s cultural setting. And finally, she emphasized that she was not rejecting Freud’s basic theory. Though she disagreed with many of his ideas, she considered it an honor to build upon the foundation of his “gigantic achievements.” To do so, she wrote, helps to avoid the danger of stagnation. If any more evidence than her word was necessary to demonstrate her loyalty to

Freud, in this introduction we also find mention of Alfred Adler. Although Horney acknowledges some similarities with Adler's perspective, she insists that her ideas are grounded in Freudian theory, and she describes Adler's work as having become sterile and one-sided.

Horney believed that anxiety was a natural state of all living things, something the German philosophers had called **Angst der Kreatur** (anxiety of the creature), a feeling that one is helpless against such forces as illness, old age, and death. We first experience this anxiety as infants, and it remains with us throughout life. It does not, however, lead to neurotic anxiety. But if a child is not cared for, if their anxiety is not alleviated by the protection of their parents, the child may develop **basic anxiety**:

The condition that is fostered…is an insidiously increasing, all-pervading feeling of being lonely and helpless in a hostile world…This attitude as such does not constitute a neurosis but it is the nutritive soil out of which a definite neurosis may develop at any time. (pg. 89; Horney, 1937)

Thus, in contrast to Freud's belief that anxiety followed the threat of id impulses breaking free of the unconscious mind, Horney places anxiety before behavior. The child, through interactions with other people (particularly the parents), strives to alleviate its anxiety. If the child does not find support, then basic anxiety develops, and neurotic disorders become a distinct possibility. From that point forward, the child's drives and impulses are motivated by anxiety, rather than being the cause of anxiety as proposed by Freud. Basic anxiety is considered basic for two reasons, one of which is that it is the source of neuroses. The other reason is that it arises out of early, but disturbed, relationships with the parents. This leads to feelings of hostility toward the parents, and Horney considered there to be a very close connection between anxiety and hostility. And yet, the child remains dependent on the parents, so it must not exhibit that hostility. This creates a vicious circle in which more anxiety is experienced, followed by more hostility, etc. Unresolved, these psychological processes leave the child feeling not only basic anxiety, but also **basic hostility**(Horney, 1937; May, 1977). In order to deal with this basic anxiety and basic hostility, Horney proposed both interpersonal and intrapsychic strategies of defense (which we will examine in the next two sections). First, however, let's take a brief, closer look at Horney's views on culture and anxiety.

A neurotic individual, simply put, is someone whose anxiety levels and behavior are significantly different than normal. What is normal, of course, can only be defined within a cultural context. Horney cited a number of famous anthropologists and sociologists to support this claim, including Margaret Mead and Ruth Benedict. She cites H. Scudder Mekeel's somewhat famous example of Native Americans having high regard for individuals who have visions and hallucinations, since those visions are considered to be special gifts, indeed blessings, from the spirits. This is in sharp contrast to the standard Western view, which considers hallucinations to be a symptom of psychosis. And yet, Native Americans are not fundamentally different than Westerners. Only one year after Horney's book was published, Mekeel led Erik Erikson on the first of Erikson's studies of Native American development, which led Erikson to conclude that his stages of psychosocial crisis were valid, since they seemed to apply to Europeans, European-Americans, and Native Americans. After citing many such examples, from simple matters such as preferred foods to complex matters

such as attitudes toward murder, Horney concluded that every aspect of human life, including personality, was intimately tied to cultural factors:

It is no longer valid to suppose that a new psychological finding reveals a universal trend inherent in human nature…This in turn means that if we know the cultural conditions under which we live we have a good chance of gaining a much deeper understanding of the special character of normal feelings and attitudes. (pg. 19; Horney, 1937)

This emphasis on culture, however, should not be confused with the importance of individuality. Anxieties and neurotic symptoms exist within individuals, and present themselves within personal relationships. Culture, once again, merely guides the nature or form of those anxieties. In Western culture, we are driven primarily by economic and individual competition. Thus, other people are seen as competitors, or rivals. For one person to gain something, another must lose. As a result, according to Horney, there is a diffuse hostile tension pervading all of our relationships. For those who cannot resolve this tension, most likely due to having experienced the culturally determined anxieties in exaggerated form during a dysfunctional childhood, they become neurotic. Accordingly, Horney described the neurotic individual as “a stepchild of our culture” (Horney, 1937).

Interpersonal Strategies of Defense

Horney considered inner conflicts, and the personality disturbances they cause, to be the source of all psychological illness. In other words, calm, well-balanced individuals do not suffer psychological disorders (consider the stress-diathesis model of abnormal psychology). Although Freud approached this concept in his work, it was those who followed him, such as Franz Alexander, Otto Rank, Wilhelm Reich, and Harald Schultz-Nencke, who defined it more clearly. Still, Horney felt they all failed to understand the precise nature and dynamics of character structure, because they did not take into account the cultural influences. It was only during her own work on feminine psychology that Horney came to the full understanding of these psychodynamic processes (at least, in her own view; Horney, 1945).

At the core of these conflicts is a **basic conflict**, which Freud described as being between one’s desire for immediate and total satisfaction (the id) and the forbidding environment, such as the parents and society (the superego). Horney generally agreed with Freud on this concept, but she did not consider the basic conflict to be basic. Rather, she considered it an essential aspect of only the neurotic personality. Thus, it is a basic conflict in the neurotic individual, one which expresses itself in the person’s predominant style of relating to others. The three general attitudes that arise as neurotic attempts to solve conflict are known as **moving toward people**, **moving against people**, and **moving away from people** (Horney, 1945). Although they provide a way for neurotics to attempt solutions in their disturbed interpersonal relationships, they achieve only an artificial balance, which creates new conflicts. These new conflicts create greater hostility, anxiety, and alienation, thus continuing a vicious circle, which Horney believed could be broken by psychoanalysis.

Psychoanalysis is important for understanding neurotic individuals in part because they build a defensive structure around their basic conflict. Their behavior, according to Horney, reflects more of their efforts to solve conflicts, rather than the basic conflict itself. Thus, the basic conflict becomes so deeply embedded in the personality, that it can never be seen in its pure form. Nonetheless, when one of the basic character attitudes becomes predominant,

we can observe characteristic behaviors that reflect the neurotic failure to resolve one's inner conflicts.

Moving toward people, also known as the **compliant personality**, incorporates needs for affection and approval, and a special need for a partner who will fulfill all of one's expectations of life. These needs are characteristic of neurotic trends: they are compulsive, indiscriminate, and they generate anxiety when they are frustrated. In addition, they operate independently of one's feelings toward or value of the person who is the object of those needs (Horney, 1945). In order to ensure the continued support of others, the compliant individual will do almost anything to maintain relationships, but they give themselves over so completely that they may enjoy nothing for themselves. They begin to feel weak and helpless, and they subordinate themselves to others, thinking that everyone is smarter, more attractive, and more worthwhile than they are. They rate themselves by the opinions of others, so much so that any rejection can be catastrophic. Love becomes the most compulsive desire, but their lack of self-esteem makes true love difficult. Accordingly, sexual relations become a substitute for love, as well as the "evidence" that they are loved and desired.

Just as the compliant type clings to the belief that people are "nice," and is continually baffled by evidence to the contrary, so the aggressive type takes it for granted that everyone is hostile, and refuses to admit that they are not. To him life is a struggle of all against all, and the devil take the hindmost. (pg. 63; Horney, 1945)

As noted in the preceding quote, those who move against people, the **aggressive personality,** are driven by a need to control others. They view the world in a Darwinian sense, a world dominated by survival of the fittest, where the strong annihilate the weak. The aggressive person may seem polite and fair-minded, but it is mostly a front, put up in order to facilitate their own goals. They may be openly aggressive, or they may choose to manipulate others indirectly, sometimes preferring to be the power behind the throne. Love, which is such a desperate need for the compliant person, is of little consequence for the aggressive person. They may very well be "in love," and they may marry, but they are more concerned in what they can get out of the relationship. They tend to choose mates for their attractiveness, prestige, or wealth. What is most important is how their mate can enhance their own social position. They are keen competitors, looking for any evidence of weakness or ambition in others. Unfortunately, they also tend to suppress emotion in their lives, making it difficult, if not impossible, to enjoy life.

Those who move away from people, the **detached personality**, are not merely seeking meaningful solitude. Instead, they are driven to avoid other people because of the unbearable strain of associating with others. In addition, they are estranged from themselves, they do not know who they are, or what they love, desire, value, or believe. Horney described them as zombies, able to work and function like living people, but there is no life in them. A crucial element appears to be their desire to put emotional distance between themselves and others. They become very self-sufficient and private. Since these individuals seek negative goals, not to be involved, not to need help, not to be bothered, as opposed to having clear goals (needing a loving partner or needing to control others) their behavior is more subject to variability, but the focus remains on being detached from others in order to avoid facing the conflicts within their psyche (Horney, 1945).

Each of these three character attitudes has within it some value. It is important and healthy to maintain relationships with others (moving toward), ambition and a drive to excel have definite benefits in many cultures (moving against), and peaceful solitude, a chance to get away from it all, can be very refreshing (moving away). The healthy individual is likely able to make use of each of these solutions in the appropriate situations. When someone needs our help, we reach out to them. If someone tries to take advantage of us, we stand up for ourselves. When the daily hassles of life wear us down, we retreat into solitude for a short time, maybe exercising, going to a movie, or listening to our favorite music. As Horney attempted to make very clear, the neurotic individual is marked by a compulsion to use one style of relating to others, and they do so to their own detriment.

Connections Across Cultures: Cultural Differences in

Interpersonal Relationship Styles

As Horney repeatedly pointed out, neurotic behavior can only be viewed as such within a cultural context. Thus, in the competitive and individualistic Western world, our cultural tendencies are likely to favor moving against and moving away from others. The same is not true in many other cultures.

Relationships can exist in two basic styles: exchange or communal relationships. **Exchange relationships** are based on the expectation of some return on one's investment in the relationship. **Communal relationships**, in contrast, occur when one person feels responsible for the well-being of the other person(s). In African and African-American cultures we are much more likely to find communal relationships, and interpersonal relationships are considered to be a core value amongst people of African descent (Belgrave & Allison, 2006). While there may be a tendency in Western culture to consider this dependence on others as somehow "weak," it provides a source of emotional attachment, need fulfillment, and the influence and involvement of people in each other's activities and lives.

Cultural differences also come into play in love and marriage. In America, passionate love tends to be favored, whereas in China companionate love is favored. African cultures seem to fall somewhere in between (Belgrave & Allison, 2006). When considering the divorce rate in America, as compared to many other countries, it has been suggested that Americans marry the person they love, whereas people in many other cultures love the person they marry. In a study involving people from India, Pakistan, Thailand, Mexico, Brazil, Japan, Hong Kong, the Philippines, Australia, England, and the United States, it was found that individualistic cultures placed greater importance on the role of love in choosing to get married, and also on the loss of love as sufficient justification for divorce. For intercultural marriages, these differences are a significant, though not insurmountable, source of conflict (Matsumoto & Juang, 2004). Attempting to maintain awareness of cultural differences when relationship conflicts occur, rather than attributing the conflict to the personality of the other person, can be an important first step in resolving intercultural conflict. However, it must also be remembered that different cultures acknowledge and tolerate conflict to different extents (Brislin, 2000; Matsumoto, 1997; Okun, Fried, & Okun, 1999; for a brief discussion of intergroup dialogue and conflict resolution options, see Miller & Garran, 2008).

These cultural differences are so fundamental, that even at the level of considering basic intelligence we see the effects of these contrasting perspectives. In a study on the Kiganda culture (within the country of Uganda, in Africa), Wober (1974) found that they consider intelligence to be more externally directed than we do, and they view successful social climbing and social interaction as evidence of intelligent behavior. This matches the attitude amongst Mediterranean cultures that notable people will be devoted to a life of public service (in contrast, the word “idiot” is derived from a Greek word meaning a private man).

Thus, moving toward others would be favored much more in other cultures than it might be in the Western world. Consequently, a significant attitude and the behavior of moving toward others would be less likely to be viewed as neurotic. Such issues are, of course, very important as we interact with people of other cultures, as we may consider their behavior to be odd according to our standards. Naturally, they may be thinking the same thing about us. What is probably most important is that we learn about and experience other cultures, so that differences in customs and behavior are not surprising when they occur.

There are two other mechanisms that Horney suggested are used by people in their attempts to resolve inner conflict: the **idealized image**, and **externalization** (Horney, 1945). The idealized image is a creation of what the person believes themselves to be, or what they feel they can or ought to be. It is always flattering, and quite removed from reality. The individual may see themselves as beautiful, powerful, saintly, or a genius. Consequently, they become quite arrogant. The more unrealistic their view is, the more compulsive their need for affirmation and recognition. Since they need no confirmation of what they know to be true, they are particularly sensitive when questioned about their false claims! The idealized image is not to be confused with authentic ideals. Ideals are goals, they have a dynamic quality, they arouse incentive to achieve those goals, and they are important for personal growth and development. Having genuine ideals tends to result in humility. The idealized image, in contrast, is static, and it hinders growth by denying or condemning one’s shortcomings.

The idealized image can provide a temporary refuge from the basic conflict, but when the tension between the actual self and the idealized image becomes unbearable, there is nothing within the self to fall back on. Consequently, an extreme attempt at a solution is to run away from the self entirely. Externalization is the tendency to experience one’s own psychodynamic processes as having occurred outside oneself, and then blaming others for one’s own problems. Such individuals become dependent on others, because they become preoccupied with changing, reforming, punishing, or impressing those individuals who are responsible for their own well-being. A particularly unfortunate consequence of externalization is a feeling Horney described as a “gnawing sense of emptiness and shallowness” (pg. 117; Horney, 1945). However, rather than allowing themselves to feel the emotion, they might experience it as an empty feeling in the stomach, and attempt to satisfy themselves by, for example, overeating. Overall, the self-contempt they feel is externalized in two basic ways: either despising others, or feeling that others despise them. Either way, it is easy to see how damaged the individual’s personal relationships would become. Horney described externalization as a process of self-elimination, which aggravates the very process with set it in motion: the conflict between the person and their environment.

Discussion Question: Horney described three basic attitudes regarding other people: moving toward, moving against, or moving away from them. Do you easily use all three

styles of relating to others, or do you tend to rely on one more than the others? Does this create problems in your relationships?

Intrapsychic Strategies of Defense

In Neurosis and Human Growth, Horney (1950) addressed the psychodynamic struggle toward self-realization. She described a series of psychological events that occur in the development of a neurotic personality, and how they interfere with the healthy psychological growth of the **real self**. Indeed, neurotic symptoms arise out of the conflict between the real self, our deep source of growth, and the idealized image. She began this book with a simple statement as to why she focused so much of her work on neurotic personalities:

The neurotic process is a special form of human development, and - because of the waste of constructive energies which it involves - is a particularly unfortunate one. (pg. 13; Horney, 1950)

Horney believed in an innate potentiality within all people, which she referred to as **growth toward self-realization**. The real self underlies this tendency toward self-realization, but it can be diverted by the development of basic anxiety. In order to overcome basic anxiety, the child adopts one of the strategies described above, attempting to solve its conflicts by moving toward, against, or away from others. Under adverse conditions, the child adopts one of these strategies in a rigid and extreme fashion, and begins the neurotic development. And yet, the tendency toward self-realization remains deep within the psyche, demanding that the neurotic development seek some higher level. Thus, the idealized image is formed, and a variety of intrapsychic processes begin an attempt to justify oneself based upon that idealized image.

The establishment of the idealized image involves self-glorification, and it reflects a need to lift oneself above others. The psychic energy associated with self-realization is shifted toward realization of the idealized image, establishing a general drive that Horney called the **search for glory** (Horney, 1950). The search for glory includes several elements, which are manifested as drives or needs. There is a **need for perfection**, which aims at the complete molding of the personality into the idealized self, and a drive for **neurotic ambition**, or striving for external success. The most damaging element of the search for glory, however, is the drive toward **vindictive triumph**. The aim of vindictive triumph is to put others to shame, or to defeat them, through one’s own success. Horney considered this drive to be vindictive because its motivating source is the desire to take revenge for humiliations suffered in childhood (i.e., to pay others back for the circumstances that created basic anxiety).

The elements of the search for glory are not necessarily bad. Who wouldn’t want to be perfect, ambitious, and triumphant? However, in their compulsive and neurotic form, Horney believed that people came to expect these elements, creating what she called the **neurotic claims**. When simple desires or needs become claims, individual feel they have a right to those things, they feel they are entitled. They fully expect to be satisfied in every way, and they also expect, indeed feel they are entitled, to never be criticized, doubted, or questioned (Horney, 1950). These claims are not only made on other people, but also on institutions, such as the workplace or society as a whole. The individual becomes highly egocentric, reminding others of a spoiled child, and they expect their needs to be satisfied without putting forth any effort of their own. Obviously, it is highly unlikely that such a

person’s needs are going to be fulfilled, creating a diffuse state of frustration and discontent, so all-encompassing that Horney suggested it can actually be viewed as a character trait in the neurotic individual. From the therapist’s point of view, neurotic claims are particularly serious because they take the place of the patient’s actual personality growth. In other words, the patient believes that merely wanting or intending to change is enough, and no effort is necessary. Indeed, the claims themselves are the neurotic’s guarantee of future glory (Horney, 1950).

While these neurotic claims and the feelings of entitlement that accompany them may seem to be just a personal problem, the fact is that many people make seriously flawed self-assessments of their abilities, attributes, and future behavior. Indeed, the “average” person typically rates themselves as “above average” in many areas of their lives. These flawed self-assessments come into play in many aspects of our lives, and can easily affect others (Dunning, Heath, & Suls, 2004; Williams, 2004). For example, the United States spends more of its gross domestic product on health care than any other major industrialized country, and yet many people seriously underestimate the consequences of a wide range of unhealthy behaviors, such as smoking, drinking alcohol, overeating to the point of obesity, and avoiding exercise. The poor physical health of many Americans has become a regular topic in the mainstream media, as it threatens both individual lives (and, consequently, the family and friends of those who die) and our ability to fund healthcare for those who are poor or aged. In education, students dramatically overestimate the extent to which they have learned, limiting the likelihood that they will take fuller advantage of their education. And in business, the consequences can be severe for many employees, and therefore their families, when a President or CEO is so over-confident that they make poor decisions that bankrupt the company. As suggested above, these problems are common, not just confined to those who are neurotic. Thus, the problem of overconfidence, whether the result of an unreasonable trend in society to ensure everyone’s self-esteem or the result of neurotic claims, as well as the extent to which individuals are able to know themselves and, therefore, function in the real world, is critical to everyone (Dunning, Heath, & Suls, 2004; Williams, 2004).

Whereas the neurotic claims are directed outward, the individual then turns their attention back into the self. They begin to tell themselves (though this may unconscious) to forget about the worthless creature they believe they are, and start behaving as they should. In order to match up with the idealized image, they should be honest, generous, and just, they should be able to endure any misfortune, they should be the perfect friend and lover, they should like everyone, they should never feel hurt, they should never be attached to anyone or anything, they should know, understand, and foresee everything, they should be able to overcome any difficulty, etc. Obviously, no one can be everything at all times. Horney described this tragic state as the **tyranny of the should**. Since it is virtually impossible for anyone to maintain such discipline in their life, rather than developing real self-confidence, the neurotic individual develops a questionable alternative: **neurotic pride**. However, the pride is not in who the individual is, but rather in who the individual believes they should be (Horney, 1950).

Sooner or later, it is inevitable that the neurotic individual will have their pride hurt in real life. When this happens, the other side of neurotic pride comes out: **self-hate**. Indeed, Horney believed that pride and self-hate are a single entity, which she called the **pride system**. As the neurotic individual becomes more aware of their failure to live up to the

idealized self, they develop self-hate and self-contempt. According to Horney, the battle lines are now drawn between the pride system and the real self. It is not the real self that is hated, however, but the emerging constructive forces of the real self (the actual aim of psychotherapy!). This conflict, between the pride system and the constructive forces for change inherent in the real self, are so profound, that Horney named it the **central inner conflict**! In her earlier writings, Horney used the term neurotic conflict to refer to conflicts between incompatible compulsive drives. The central inner conflict is unique, in that it sets up a conflict between a neurotic drive (the pride system) and a healthy drive (the trend toward self-realization). Horney believed that individuals who have arrived at this psychological state of affairs were indeed in a difficult situation:

Surveying self-hate and its ravaging force, we cannot help but see in it a great tragedy, perhaps the greatest tragedy of the human mind. Man in reaching out for the Infinite and Absolute also starts destroying himself. When he makes a pact with the devil, who promises him glory, he has to go to hell - to the hell within himself. (pg. 154; Horney, 1950).

Discussion Question: Horney defined the central inner conflict as the battle between the constructive forces for change inherent in the real self and the self-hate that arises out of the pride system. Have you ever found yourself giving up on something important because you feel incapable, unworthy, or overly self-critical? If you have ever been aware of these feelings at the time they occurred, what, if anything, did you do about them?

Horney’s Challenge for Psychoanalysis

One of the actions that made Horney most controversial was her willingness to challenge how psychoanalysis should be conducted with patients. In New Ways in Psychoanalysis (Horney, 1939), Horney made it very clear why she thought that psychoanalysis needed to be questioned:

My desire to make a critical re-evaluation of psychoanalytical theories had its origin in a dissatisfaction with therapeutic results. (pg. 7; Horney, 1939)

Simply put, she had asked many leading psychoanalysts questions about problems in treating her patients, and none of them could offer meaningful answers (at least, they had no meaning for Horney). In addition, a few of them, such as Wilhelm Reich, encouraged her to question orthodox psychoanalytic theory. As always, Horney did not see this as a rejection of Freud. Indeed, she felt that as she pursued new ideas, she found stronger reasons to admire the foundation that Freud had established. More importantly, she was upset that those who criticized psychoanalysis often simply ignored it, rather than looking more deeply into the valuable insights she believed it still had to offer for any therapist. As before, she saved her most serious critiques for the study of feminine psychology, though she still considered psychoanalysis with an emphasis on culture to be a valid therapeutic approach:

The American woman is different from the German woman; both are different from certain Pueblo Indian women. The New York society woman is different from the farmer’s wife in Idaho. The way specific cultural conditions engender specific qualities and faculties, in women as in men - this is what we may hope to understand. (pg. 119; Horney, 1939)

In her second book on therapy, Horney proposed something quite radical: the possibility of Self-Analysis (Horney, 1942). She considered **self-analysis** important for two main reasons. First, psychoanalysis was an important means of personal development, though not the only

means. In this assertion, she was both emphasizing the value of psychoanalysis for many people, while at the same time saying that it wasn't so important that it had to conducted in the orthodox manner by an extensively trained psychoanalyst, since there are many paths to self-development (e.g., good friends and a meaningful career). Second, even if many people sought traditional psychoanalysis, there simply aren't enough psychoanalysts to go around. So, Horney provided a book to help those willing to pursue their own self-analysis, even if they do so only occasionally (which she believed could be quite effective for specific issues). She did not suggest that self-analysis was by any means easy, but more important was the realization that it was possible. With regard to the possible criticism that self-analysts might not finish the job, that they might not delve into the darkest and most repressed areas of their psyche, she simply suggested that no analysis is ever complete. What matters more than being successful is the desire to continue (Horney, 1942).

When the Association for the Advancement of Psychoanalysis was established, an important part of their mission was community education. One of the courses was entitled Are You Considering Psychoanalysis? This course was so popular, that the instructors decided to publish a book by the same name, and Horney was chosen the editor-in-chief (Horney, 1946). The chapters present very practical topics, such as: What Are Your Doubts About Psychoanalysis? (Kelman, 1946); What Do You Do in Analysis? (Kilpatrick, 1946); and How Does Analysis Help? (Ivimey, 1946). Perhaps reflecting her own concerns about the ability of psychoanalysis to "cure" a person's problems, Horney entitled the final chapter, which she wrote herself: How Do You Progress After Analysis? She begins the chapter by addressing the concern that many of her patients had: why would a person need more progress after psychoanalysis? Isn't psychoanalysis supposed to resolve all of a person's psychological problems? As noted above, however, Horney felt that no analysis is ever complete. But this time the reasoning is not based on questioning the effectiveness of psychoanalysis itself. Rather, it is based on the potential for human growth, a potential that is boundless:

Your growth as a human being, however, is a process that can and should go on as long as you live…analytical therapy merely sets this process in motion… (pg. 236; Horney, 1946)

Discussion Question: Have you ever tried self-analysis, in either a formal or an informal way? If yes, were your efforts based on any personal experience or knowledge, and did it prove to be helpful?

Brief Biography of Erich Fromm

Erich Fromm was a colleague and long-time friend of Horney. He became interested in psychoanalysis at the beginning of World War I, when he was amazed at how readily so many people seemed eager for war. Unlike most other psychoanalysts, however, he earned a Ph.D., not an M.D. This eventually proved to be a source of conflict between Fromm and Horney, as she believed that lay-analysts should not be allowed to conduct therapy. Still, Fromm acknowledged Horney as influencing his career and sharing his own interests in culture and particularly in society itself (Evans, 1981a). Fromm also considered himself as remaining especially true to the theories of Sigmund Freud, though some authors consider him to be more of a philosopher than a psychologist (Evans, 1981a; Lundin, 1979; see also Funk, 1982, 2000).

Fromm was born on March 23, 1900, the only son of Orthodox Jewish parents, in Frankfurt, Germany. He studied the Talmud and law, but eventually switched from the University of

Frankfurt to the University of Heidelberg and changed his major to sociology and economics. In 1922 he received his doctorate, and in 1924 he was psychoanalyzed by Frieda Reichmann. He turned away from Orthodox Judaism, married Frieda Reichmann (whom he later divorced), and became active in the Berlin psychoanalytic community (where he completed his psychoanalytic training). In 1933, Horney invited Fromm to guest lecture in Chicago. A year later, he moved to New York. There he collaborated with Horney, Harry Stack Sullivan (whom he also acknowledged as a significance influence on his thinking; see, e.g., Evans, 1981a and Fromm, 1941, 1955a), and Clara Thompson. In 1940 he became a United States citizen, then in 1941 he published Escape from Freedom (Fromm, 1941) and began teaching at the New School (Funk, 1982, 2000).

After his break with Horney (both personally and professionally), Fromm married his second wife and spent some time teaching at Yale University. A few years later his wife died, Fromm soon married for the third time, and that marriage lasted until his death. Shortly after his third marriage, Fromm moved to Mexico City, Mexico, where he lived for the next 24 years. He joined the medical faculty at the National Autonomous University of Mexico, and co-founded a Mexican psychoanalytic society. In 1956, he published his acclaimed book The Art of Loving (Fromm, 1956). He taught a seminar with D. T. Suzuki, and their friendship led to the publication of Zen Buddhism & Psychoanalysis (Suzuki, Fromm, and De Martino, 1960). He also conducted important cross-cultural studies in a Mexican peasant village, resulting in the publication of Social Character in a Mexican Village (Fromm & Maccoby, 1970),

In 1966, Fromm suffered a heart attack and began spending more time back in Europe. In 1974, he sold his home in Mexico and settled permanently in Switzerland (where he had been spending his summers). After a series of three more heart attacks, Fromm died in 1980 (Funk, 1982, 2000).

Placing Fromm in Context: Individuality in Relation to Society

Erich Fromm was a colleague and close personal friend of Karen Horney for many years. He shared her interest in the role of culture in personality, and was even more interested in the interactions between the individual and society as a whole. Fromm viewed societies as forces that lead to alienation from a more natural, primitive way of life. As a result, freedom and individuality actually create psychological problems, as we become disconnected from our immediate social groups (such as the family or local community). This often leads to unfortunate consequences, such as seeking fellowship within a society at the expense of one’s regard for self and others, providing a framework within which dictatorships can develop (as individuals completely surrender their freedom).

Fromm examined and combined many different interests in his career, including philosophy, economics, and psychology, and he felt that such a combination of interests was essential for the study of psychology to have real meaning. In one of the longest projects of his life, he and a number of colleagues applied a unique form of “psychoanalysis” to an entire village in rural Mexico. He then described how an understanding of social character can lead to an understanding of individual character, providing guidance for future considerations on planning social development during times of dramatic socioeconomic change.

Our Relationship to Society

Fromm was a prolific writer, whose interests included psychoanalysis, economics, religion, ethics, culture, and societal systems. He evaluated both Freud the man and Freud's theories in Sigmund Freud's Mission (Fromm, 1978) and Greatness and Limitations of Freud's Thought(Fromm, 1980). His religious works include such provocative titles as The Dogma of Christ (Fromm, 1955b) and You Shall Be as Gods (Fromm, 1966). He addressed the person's place within society in books such as The Sane Society (1955a) and The Revolution of Hope (1968). And a collection of his works on gender psychology, Love, Sexuality, and Matriarchy, was edited by Rainer Funk (1977). The unifying theme throughout Fromm's writings is each person's relationship to society, which he addressed most directly in Escape from Freedom (Fromm, 1941).

Fromm interpreted Freud's theories on the satisfaction of drives as necessarily involving other people, but for Freud those relationships are only a means to an end. Although hunger, thirst, and sex may be common needs, Fromm suggested that the needs that lead to differences in people's character, such as love and hatred, lusting for power or yearning to submit, or the enjoyment of sensuous pleasure as well as the fear of it, are all the result of social processes. One's very nature is a product of the interaction between the individual and their cultural setting. We are the creation and achievement of human history, and at the same time we influence the course of that history and culture. In modern times, particularly in the Western world, our pursuit of individuality has alienated us from others, from the very social structure that is inherent to our nature. Consequently, our **freedom** has become a psychological problem, it has isolated us from the connections necessary for our survival and development (Fromm, 1941). The danger with this situation, according to Fromm, is that when an entire society is suffering from feelings of isolation and disconnection with the natural order (from nature itself, in Fromm's view), the members of that society may seek connection with a societal structure that destroys their freedom and, thus, integrates their self into the whole (albeit in a dysfunctional way). The three ways in which individuals escape from freedom are **authoritarianism**, or giving oneself up to some authority in order to gain the strength that the individual lacks, **destructiveness**, in which the individual tries to destroy the object causing anxiety (e.g., society), and **automaton conformity**, in which the person renounces their individual integrity. Fromm believed that these phenomena provided an explanation for the development of dictatorships, such as the rise of Fascism in Europe during the 1920s and 1930s. For the leaders of these societies, these processes are such a deeply ingrained aspect of their character that Fromm actually described Adolf Hitler's destructiveness as evidence of a necrophilous character (a necrophiliac is someone sexually attracted to the dead; Fromm, 1973).

In order to approach a solution for this problem, Fromm pursued an overall integration of the person and society. He believed that psychology cannot be divorced from philosophy, sociology, economics, or ethics. The moral problem facing people in the modern world is their indifference to themselves. Although democracy and individuality seem to offer freedom, it is only a promise of freedom. When our insecurities and anxieties lead us to submit to some source of power, be it a political party, church, club, whatever, we surrender our personal power (Fromm, 1947). Consequently, we become subject to the undue influence of others (and in extreme situations, to a Hitler or a Stalin). The solution may be as simple as love, but Fromm suggests that love is by no means an easy task, and it is not simply a relationship between two people:

…love is not a sentiment which can be easily indulged in by anyone, regardless of the level of maturity reached by him. It [Fromm's book] wants to convince the reader that all his attempts for love are bound to fail, unless he tries most actively to develop his total personality, so as to achieve a productive orientation; that satisfaction in individual love cannot be attained without the capacity to love one's neighbor, without true humility, courage, faith and discipline. (pg. xxi; Fromm, 1956)

An individual's capacity for love is a reflection of the extent to which their culture encourages the development of the capacity for love as part of the character of each person. Capitalist societies, according to Fromm, emphasize individual freedom and economic relations. Thus, a capitalist society values economic gain (amassed wealth) over labor (the power of people). And yet, such an economy needs large groups of people working together (the labor force). As individuals become anxious in their pursuit of life, they become psychologically invested in the capitalist system, they surrender themselves to capitalism, and become the labor force that leads to the wealth of those who own the company. Fromm believed this alienated us from ourselves, from others, and from nature (or, the natural order). In order to regain our connection to others in a healthy way, we need to practice the art of love, love both for ourselves and for others. Doing so requires discipline, concentration, and patience, personal strengths that are all taught in the practice of Zen. Indeed, Fromm recommends one of Horney's favorite books: Zen in the Art of Archery (Herrigel, 1953). We will examine the relationship between Zen and the approaches of Horney and Fromm to solving society's problems in more detail in "Personality Theory in Real Life." But first, Fromm chose to examine whether the principles of psychoanalysis could be used to examine the relationship between individuals and society. He and his colleagues addressed this question in a Mexican village, a study we will examine in the next section.

Discussion Question: Fromm believed that the freedom we have in modern, Western societies actually separates and alienates us from others, becoming a source of great anxiety. Can you agree that freedom can become a problem? Can you agree that people within an entire society could become so anxious that they support the rise of a dictator?

Fromm's Cross-Cultural Studies in Mexico

Fromm believed that in addition to individual's having a certain character structure, there is also **social character**. Social character is common to groups or classes within a society, and provides a framework within which psychic energy in general is transformed into the specific psychic energy of each person within the group. From 1957 to 1963, Fromm, Michael Maccoby, and numerous colleagues interviewed every adult member of a Mexican village, and about half the children, with a focus on applying psychodynamic theory in order to understand the social character of the village and its role in determining the personality of each person. The village was chosen as representative of many small villages (this village had approximately 800 residents) in Mexico that underwent substantial changes in socioeconomic structure following the Mexican revolution. The primary, and most controversial, purpose of this study was to determine whether a society could be "psychoanalyzed" in order to understand the character of individual's within that society. Fromm & Maccoby also hoped that their study would provide information to help predict and plan social change during times of dramatic socioeconomic change, such as the transition from a non-democratic to a democratic society (Fromm & Maccoby, 1970).

While it took an entire book for Fromm and Maccoby to report their results, a few key findings can be summarized. First, although they began their study with a questionnaire that had been developed for a previous study, the level of interpretation needed for psychoanalytic theorizing required additional information. This was obtained by also having the participants take the Rorschach inkblot test. Second, the theory of social character, as an adaptation to the socioeconomic conditions of a society that serve to stabilize and maintain that society, was confirmed. Of particular interest were those individuals whose character was typically viewed as deviant, because they seek change and opportunity. When external socioeconomic conditions force changes upon a society, the previously "deviant" individuals are among those who flourish under this new opportunity for change. In other words, their so-called deviance now becomes advantageous, and they lead others toward new adaptive changes in social character (though this may occur slowly for most members of the society). In a manner similar to natural selection in evolution, Fromm and Maccoby referred to this type of change in a society as **social selection**. Unfortunately, if the individuals leading these changes are dysfunctional or cruel individuals, such as the leaders of the fascist groups in Europe during the 1920s and 1930s, the consequences can be tragic. It was for this very reason that Fromm sought to understand how people are drawn into groups following their alienation and anxiety due to changes in the course of society.

Personality Theory in Real Life: Feminine Psychology, Zen Mindfulness,

Psychoanalysis, and Everyday Relationships

The ancient practice of mindfulness, which is associated with Buddhism but also has roots in other spiritual practices and religions, has become an important and fairly common psychotherapeutic technique (see, e.g., Germer, Siegel, & Fulton, 2005; Richards & Bergin, 2000; Sperry & Shafranske, 2005). There are also some interesting connections between the practice of Buddhist mindfulness and those who established feminine psychology. At the end of her life, Karen Horney went to Japan to study Zen Buddhism with the renowned Buddhist scholar D. T. Suzuki, and Janet Surrey, one of the founding members of the Stone Center (which will be introduced in the next chapter), has been practicing mindfulness and working to synthesize Buddhist practices with relational-cultural approaches to psychology for over 20 years (Surrey, 2005). Surrey is also on the faculty of the Institute for Meditation and Psychotherapy, and teaches seminars on the use of mindfulness in conjunction with relational-cultural therapy (and I can personally attest to the wonderful job she does).

Since Horney became interested in Zen near the end of her life, she wrote very little about it. Indeed, most of what is recorded is in the book Final Lectures (Ingram, 1987), which was published by Douglas Ingram many years after Horney died. However, her close friend and colleague Erich Fromm also worked with Suzuki. Fromm mentions Yoga and Buddhism often in his books, and Suzuki and Fromm (along with another colleague) co-authored Zen Buddhism & Psychoanalysis in 1960.

Horney equated Zen mindfulness with living fully in each moment, with wholehearted concentration (Horney, 1945, 1950; Ingram, 1987). This general theme is expressed quite eloquently in one of Horney's favorite books, Zen in the Art of Archery (Herrigel, 1953), as well as in Herrigel's other book, The Method of Zen (Herrigel, 1960). In the latter book, Herrigel expresses the essence of Zen from his perspective, presenting a psychological view that fits well with the relational-cultural perspective we will examine in the next chapter:

…the Zen Buddhist is far from limiting his feelings of joy and compassion to human beings and to every aspect of human existence. He embraces in these feelings everything that lives and breathes…The Zen Buddhist is constantly confirmed in his experience that there is a fundamental communication which embraces all forms of existence…He does not pass by the joys and sufferings of others without taking them to himself and reinforcing them with his own feelings… (pp. 119-120; Herrigel, 1960)

Fromm knew Suzuki at the same time as Horney, but the two men really got to know each other when Suzuki spent a week in Mexico in 1956, and Fromm then visited Suzuki in New York. In 1964, Fromm wrote to Suzuki that every morning he read a passage on Zen or something by Meister Eckhart (a well-known Christian mystic). In addition, Fromm was interested in Kabbalah and Sufism, as well as other spiritual approaches to understanding people (Funk, 2000). Fromm examined many of these diverse perspective in books such as The Nature of Man (Fromm & Xirau, 1968) and Psychoanalysis and Religion (Fromm, 1950), and he drew interesting connections between the physical activities of Yoga and Wilhelm Reich's somatic psychology (Fromm, 1992). He was by no means an unqualified supporter, however, suggesting that some self-proclaimed gurus can do more harm than good when seeking to serve their own selfish interests (usually in order to make money; Fromm, 1994).

In their work together, Suzuki provided a brief overview of the essentials of Zen practice, which focuses on living life:

Zen may occasionally appear too enigmatic, cryptic, and full of contradictions, but it is after all a simple discipline and teaching:

To do goods,

To avoid evils,

To purify one's own heart:

This is the Buddha-Way.

Is this not applicable to all human situations, modern as well as ancient, Western as well as Eastern? (pg. 76; Suzuki, Fromm, & DeMartino, 1960)

Fromm, for his part, identified ways in which Zen principles appeared to be compatible with psychoanalysis. He considered psychoanalysis to be the Western parallel to Zen, since Zen arose from Indian rationality and abstraction mixed with Chinese concreteness and realism, whereas psychoanalysis arose from Western humanism and rationalism. Fromm described the Western world as suffering from a spiritual crisis, resulting from a change in the pursuit of the perfection of humanity to the pursuit of the perfection of things (e.g., technology). Since we have lost our connection to nature, and to ourselves and our communities, we have become anxious and depressed. Psychoanalysis was developed to help us deal with these anxieties, as an alternative to the flawed ways in which we had been dealing with them in the past: religion (according to Freud). As described very simply in the quote above, Zen Buddhism also seeks to resolve human anxiety, simply by doing good and avoiding evil. In Freudian terms, doing good results from knowing oneself, and one can only know oneself through the process of psychoanalysis. Then, a person can act in accordance with reality, rather than being influenced by unconscious, repressed, and dysfunctional psychological processes. Therefore, Fromm considered the essential nature of psychoanalysis to be compatible with Zen (Suzuki, Fromm, & DeMartino, 1960), a perspective supported more

recently by Mark Epstein in his comparison of Buddhist meditation and psychoanalysis, Thoughts Without a Thinker (Epstein, 1995).

Fitting even more closely with Fromm's perspective on human development and psychoanalysis, Zen art is intimately involved with nature, and with humanity's relationship with nature (Herrigel, 1953, 1960; Suzuki, Fromm, & DeMartino, 1960). Fromm used Zen perspectives to reform his views on psychoanalysis and development. He considered the development of the individual to be a re-enactment of the development of the species (i.e., ontogeny recapitulates phylogeny). Prior to birth there is no anxiety, following birth we must deal with anxiety. We can try to deal with our anxieties by regressing to our earliest state, or we can attempt to complete the process of birth, which Fromm described as a lifelong process:

Birth is not one act; it is a process. The aim of life is to be fully born, though its tragedy is that most of us die before we are thus born. To live is to be born every minute. (pg. 88; Suzuki, Fromm, & DeMartino, 1960)

Fromm does not suggest that this is easy, but it is possible. However, which method is to be preferred: psychoanalysis or the practice of Zen Buddhism? That would appear to be a personal matter, since both psychoanalysis and Zen Buddhism aim toward the same goal:

This description of Zen's aim could be applied without change as a description of what psychoanalysis aspires to achieve; insight into one's own nature, the achievement of freedom, happiness and love, liberation of energy, salvation from being insane or crippled…The aim of Zen transcends the goal of ethical behavior, and so does psychoanalysis. It might be said that both systems assume that the achievement of their aim brings with it an ethical transformation, the overcoming of greed and the capacity for love and compassion. (pp. 122-123; Suzuki, Fromm, & DeMartino, 1960)

Just as love is considered an essential element of being Christian, compassion is essential to Buddhism. In The Art of Loving, Fromm (1956) noted that a person cannot love themselves if they do not love others. Thus, love and compassion are intertwined, one must love and care for all people, indeed for all things, to be fulfilled. Zen teaches this peace in many ways, even sword fighting and archery become art when performed by a Zen master. Fromm acknowledged that a Zen master of sword fighting has no wish to kill and experiences no hate for his opponent. Although a classic psychoanalyst might insist that the sword master is motivated by some unconscious hatred or anger, Fromm says that such a psychoanalyst simply does not grasp the spirit of Zen. Likewise, citing Herrigel's Zen in the Art of Archery again, Fromm notes how archery has been transformed from a military skill into an exercise of spirituality, or in non-spiritual terms, a form of playful violence (as opposed to aggressive violence; Fromm, 1964, 1973).

Thus, the practice of mindfulness, the art of love, compassion, all play similar roles in helping people to be aware of who they are and of their relationships with others. In addition, they encourage and support a genuine desire to be connected to others, and to maintain healthy interpersonal connections, even in such diverse activities as eating breakfast, going to work, or practicing archery.

Review of Key Points

• Horney was the first psychodynamic theorist to challenge Freud’s perspective on the psychology of women.

• Horney did not deny that girls envy boys for certain anatomical advantages that boys have, but she suggested that another dynamic force comes into play: female genital anxiety.

• In addition to female genital anxiety, an important consequence of the castration complex in women is the experience of wounded womanhood.

• Because girls are repeatedly made to feel inferior to boys, and few outlets are available for their creative drives, many women develop a masculinity complex. This complex is manifested by a desire for revenge against men and the rejection of feminine traits.

• Horney suggested that boys are actually more envious of girls, since boys place such a minimal role in pregnancy, childbirth, and nurturing an infant. Indeed, they may feel so inadequate that they seek to conquer many women in relationships, but avoid any long-term or meaningful relationships.

• As the result of cultural pressure, Horney felt that many women overvalue love, and experience a desperate need to be with a man.

• Horney eventually moved away from studying feminine psychology, considering it more of a cultural issue than a gender issue. She believed that demeaning women also hurt children and men, and she did not want to further the conflict between men and women.

• Horney believed that all children are born with an anxiety related to survival. When they are not nurtured and cared for fully, they then develop a basic anxiety toward the seemingly unsafe world.

• Basic anxiety leads to basic hostility, and the child must then choose some strategy for dealing with other people. The three basic strategies are moving toward people, moving against people, and moving away from people.

• As the neurotic personality progresses, due to the continued failure to resolve one’s conflicts, an idealized image is formed. In addition, the neurotic individual externalizes their anxiety.

• Horney suggested that the real self contains an innate trend toward self-realization. However, in the neurotic individual, this energy is turned toward realizing the idealized image.

• The neurotic individual then embarks on a search for glory, which includes neurotic claims. The neurotic claims then lead to a powerful compulsion known as the tyranny of the should.

• As this dysfunctional state continues, neurotic pride leads to self-hate, the two phenomena being opposite sides of the pride system. This conflict between the real self and the pride system is the central inner conflict.

• Horney felt that psychoanalysis needed to be re-evaluated because in many cases it was ineffective. Her most radical suggestion involved the possibility of teaching people about self-analysis.

• Fromm believed that the freedom associated with modern societies had alienated us from the natural order. Consequently, freedom had become a psychological problem, resulting in alienation and anxiety.

• Individuals who feel anxious and alienated seek connection by any means, according Fromm, including submitting themselves to authoritarian regimes.

• Fromm encouraged practicing the art of love, or a deep concern for others, as a way to feel connected without surrendering one’s personal freedom.

• In his extensive study of a Mexican village, Fromm believed that psychodynamic principles could be used to study the social character of groups. He also felt that the evolution of groups, or societies, over time reflects a type of social selection.

• Both Horney and Fromm believed that Zen Buddhism and psychoanalysis shared common elements, and that each discipline could benefit from studying the other. They attempted to incorporate these ideas in ways that might enhance interpersonal relationships in our everyday lives.

CHAPTER 07 / CAC.902

Jung and Analytical Psychology

Carl Jung's analytical psychology introduces a different emphasis from Freud's. Jung focused on the collective unconscious, archetypes, individuation, and the symbolic life. His ideas have influenced art, religion, literature, and psychotherapy. They are also controversial. A doctoral counselor should approach Jung with both openness and critical discipline. Symbolic work can be meaningful for some clients, but archetypal claims should not be presented as settled empirical science.

Persona, shadow, and self

Jung described the persona as the social mask a person presents to the world. The persona is necessary; every society requires roles. Problems arise when a person identifies completely with the mask and loses contact with other parts of the self. The shadow contains qualities the ego rejects or does not recognize. The shadow is not only evil. It may contain creativity, anger, tenderness, or ambition that the person was taught to hide. The Self, in Jung's theory, is an organizing center of the whole psyche, often represented in symbols of wholeness.

A teenager who is always the helper may have a shadow full of unmet need. A teenager who is always tough may have a shadow full of fear. The counselor does not announce, "This is your shadow." The counselor invites the client to notice what is excluded. "What part of you is not allowed in this story?" The question can open space without imposing a symbol.

Archetypes and the collective unconscious

Archetypes are proposed universal patterns of image and behavior, such as the hero, the mother, the trickster, or the wise old person. Jung argued that they arise from the collective unconscious, a layer of psyche shared by humanity. This claim is difficult to test scientifically. A counselor can use archetypal images as metaphors without asserting that they are biologically inherited structures. A client may find the hero story useful for thinking about courage. Another client may find it oppressive because it demands constant bravery. The meaning belongs to the client.

Individuation

Individuation is the process of becoming a whole person, integrating conscious and unconscious aspects rather than remaining split. It is not the same as individualism. Jung did not mean selfish isolation. He meant a more complete relationship with oneself and the world. In adolescence, identity formation can be seen as an individuation process, but counselors should not force Jung's stages onto every teen. Cultural traditions differ in how they understand selfhood. Some emphasize relational identity more than separate identity.

Dreams and active imagination

Jung placed great weight on dreams. He saw them as compensatory, offering what consciousness lacks. A client who feels powerful may dream of being small. A client who feels hopeless may dream of a hidden door. The counselor explores the dream's images, emotions, and associations rather than applying a fixed dictionary. Active imagination is a waking practice of engaging inner images through writing, drawing, or dialogue. It can be useful for creative clients but may be destabilizing for clients with psychosis or severe dissociation unless carefully adapted and supervised.

Clinical example

Seventeen-year-old Priya draws a recurring image of a walled garden. In therapy, she associates the wall with family protection and the garden with a part of herself no one sees. A Jungian lens might explore the symbol of enclosed potential. A person-centered counselor would follow Priya's meaning without naming archetypes. A CBT counselor might explore beliefs about exposure and judgment. The counselor can integrate: use the image as a shared metaphor, ask what the gate would need to open safely, and connect the metaphor to real-life steps.

The risk is overinterpretation. If the counselor says, "The garden is the Great Mother archetype," Priya may feel invaded. The counselor should ask, "What does the garden mean to you?" The client's association is primary.

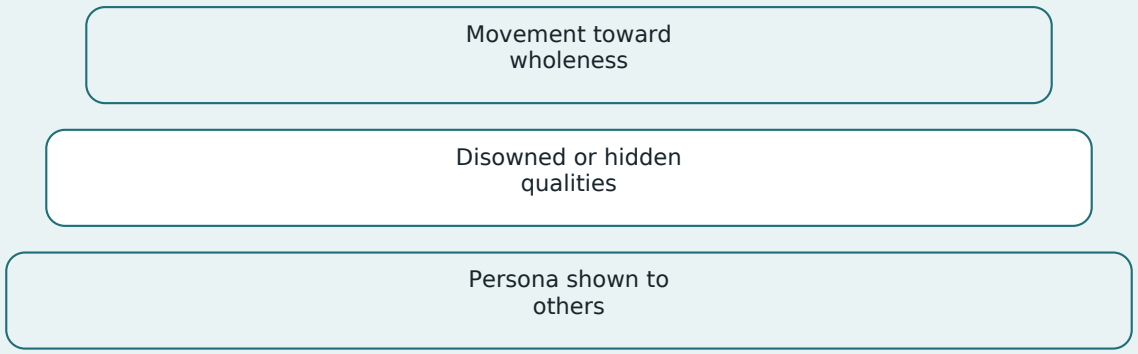
Evidence and caution

Jungian therapy has a smaller empirical base than CBT for specific disorders. Some clients report meaning and growth, but meaning is not the same as symptom reduction. Counselors should not use Jungian ideas to bypass diagnosis, safety planning, or evidence-based treatment when those are needed. Spiritual emergency language should not be used to dismiss psychosis. Cultural humility is essential because symbols do not mean the same thing across traditions.

Reflection

Choose a recurring image from a dream, story, or drawing. Do not interpret it immediately. Write what it feels like, what it protects, and what it might invite. Then ask what evidence would support or challenge your interpretation. Symbolic work is strongest when it remains curious.

Persona and shadow



Jungian work explores what is excluded from the social mask without forcing symbols.

Attributed Foundational Reading

Attribution and critical-reading note

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Read historical and theoretical claims critically. A theory is a lens for inquiry, not a verdict about a child or family. Where source claims are dated or culture-bound, the new lecture supplies cautions and contemporary framing.

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Carl Jung brought an almost mystical approach to psychodynamic theory. An early associate and follower of Freud, Jung eventually disagreed with Freud on too many aspects of personality theory to remain within a strictly Freudian perspective. Subsequently, Jung developed his own theory, which applied concepts from natural laws (primarily in physics) to psychological functioning. Jung also introduced the concept of personality types, and began to address personality development throughout the lifespan. In his most unique contribution, at least from a Western perspective, Jung proposed that the human psyche contains within itself psychological constructs developed throughout the evolution of the human species.

Jung has always been controversial and confusing. His blending of psychology and religion, as well as his openness to different religious and spiritual philosophies, was not easy to accept for many psychiatrists and psychologists trying to pursue a purely scientific explanation of personality and mental illness. Perhaps no one was more upset than Freud, whose attitude toward Jung changed dramatically over just a few years. In 1907, Freud wrote a letter to Jung in which Freud offered high praise:

…I have already acknowledged…above all that your person has filled me with trust in the future, that I now know that I am dispensable like everyone else, and that I wish for no one other or better than you to continue and complete my work. (pg. 136; cited in Wehr, 1989)

In 1910, hoping that others would also support Jung, Freud wrote to Oskar Pfister:

I hope you will loyally support Jung, I want him to acquire the authority that will entitle him to leadership of the whole movement. (pg. 136; cited in Wehr, 1989)

However, in a dramatic shift just three years later, Freud wrote to Jung:

I suggest to you that we completely give up our private relationship. By this I lose nothing, since for a long time I have been bound to you emotionally only by the thin thread of previously experienced disappointments. … Spare me the supposed “duties of friendship.” (pg. 136; cited in Wehr, 1989)

Later, in 1922, Freud thanked Oskar Pfister for his help in trying to eliminate Jung’s influence on the psychoanalytic community:

With your ever more thorough and ever more clearly demonstrated dismissal of Jung and Adler, you have for a long time given me great satisfaction. (pg. 136; cited in Wehr, 1989)

Who was this man who inspired such profound confidence from Sigmund Freud, only to later inspire such contempt? And were his theories that difficult for the psychodynamic community, or psychology in general, to accept? Hopefully, this chapter will begin to answer those questions. As evidence of his character, and in contrast to Freud, Jung did not turn his back on his former mentor. Following Freud’s death in 1939, and later in 1957, Jung wrote the following:

[Freud’s work was]…surely the boldest attempt ever made on the apparently solid ground of empiricism to master the riddle of the unconscious psyche. For us young psychiatrists, it was a source of enlightenment… (pg. 29; cited in Wehr, 1989)

…Despite the resounding censure I suffered at the hands of Freud, I cannot, even despite my resentment toward him, fail to recognize his importance as a critical analyst of culture and as a pioneer in the field of psychology. (pg. 39; cited in Wehr, 1989)

A Brief Biography of Carl Jung

At the beginning of his autobiography, entitled Memories, Dreams, Reflections, Jung (1961) described his life as “a story of the self-realization of the unconscious.” Jung believed that our personality begins with a collective unconscious, developed within our species throughout time, and that we have only limited ability to control the psychic process that is our own personality. Thus, our true personality arises from within as our collective unconscious comes forth into our personal unconscious and then our consciousness. It can be helpful to view these concepts from an Eastern perspective, and it is interesting to note that “self-realization” was used in the name of the first Yoga society established in America (in 1920 by Paramahansa Yogananda).

Carl Gustav Jung was born on July 26th, 1875, in the small town of Kesswil, Switzerland, into an interesting and notable family. His grandfather of the same name had been a physician, and had established the psychiatric clinic at the University of Basel and the “Home of Good Hope” for mentally retarded children. At an early age he had been imprisoned for over a year, for the crime of having participated in a demonstration supporting democracy in Germany. Rumored to be an illegitimate son of the great Johann Wolfgang von Goethe, though there is no convincing evidence, the elder Carl Jung died before his namesake grandson ever knew him. Nonetheless, Jung was greatly influenced by

stories he heard about his grandfather. His maternal grandfather, Samuel Preiswerk, was the dean of the Basel (Switzerland) clergy and pastor of a major church. He was one the first people in Europe to suggest a restoration of Palestine to the Jews, thus establishing himself as a forerunner to the Zionists. Samuel Preiswerk also believed that he was regularly surrounded by spirits (or ghosts), something that likely had quite an influence on Jung's theories (Jaffe, 1979; Wehr, 1989).

Jung's father, Johann Paul Achilles Jung, married Emilie Preiswerk in 1874. Johann Jung was a scholar of Oriental languages, studied Arabic, and was ordained a minister. In addition to being a pastor at two churches during Jung's childhood, Johann Jung was the pastor at Friedmatt, the insane asylum in Basel. During Jung's early childhood he did not always have the best of relationships with his parents. He considered his mother to be a good mother, but he felt that her true personality was always hidden. She spent some time in the hospital when he was three years old, in part due to problems in her marriage. Jung found this separation from his mother deeply troubling, and he became mistrustful of the spoken word "love." Since his father was a pastor, there were often funerals and burials, all of which was very mysterious to the young Jung. In addition, his mother was considered a spiritual medium, and often helped Jung with his later studies on the occult. Perhaps most troubling of all, was Jung's belief that his father did not really know God, but rather, had become a minister trapped in the performance of meaningless ritual (Jaffe, 1979; Jung, 1961; Wehr, 1989).

An only child until he was 9, Jung preferred to be left alone, or at least he came to accept his loneliness. Even when his parent's guests brought their children over for visits, Jung would simply play his games alone:

…I recall only that I did not want to be disturbed. I was deeply absorbed in my games and could not endure being watched or judged while I played them. (pg. 18; Jung, 1961)

He also had extraordinarily rich and meaningful dreams, many of which were quite frightening, and they often involved deeply religious themes. This is hardly surprising, since two uncles on his father's side of the family were ministers, and there were six more ministers on his mother's side. Thus, he was often engaged in religious discussions at home. He was particularly impressed with a richly illustrated book on Hinduism, with pictures of Brahma, Vishnu, and Shiva (the Hindu trinity of gods). Even at 6 years old, he felt a vague connection with the Hindu gods, something that once again would have an interesting influence on his later theories. These dreams led Jung into deep religious speculations, something he considered to be a secret that he could not share with anyone else (Jaffe, 1979; Jung, 1961; Wehr, 1989).

Jung's school-age years were a mixture of experiences. He enjoyed school, in the sense that it was easy for him and he found other children to play with. However, he also began studying Latin with his father and taking divinity classes. He found the classes on religion terribly boring, and the more he got to know his father, the less he believed that his father understood either God, religion, or spirituality. It didn't help that he was well-aware of the continued turmoil in his parent's marriage. In a cave in the garden he tended a fire that he meant to keep burning forever, and although he allowed other children to help gather the wood, only Jung himself could tend the fire. At the age of 11 he began attending the Gymnasium in Basel (something like an advanced high school). The other children were quite wealthy, and Jung became aware of how poor they were. Although this led him to feel

some compassion for his father, the Gymnasium created a number of problems. Jung simply did not understand mathematics, his divinity classes became unbearably boring, and so, school itself became boring. This led to a severe neurosis at the age of 12 (Jaffe, 1979; Jung, 1961; Wehr, 1989).

Jung had been knocked down by another boy on the way home from school. He hit his head on a rock, and was nearly knocked out. He was so dizzy that others had to help him, and he suddenly realized that he did not have to go to school if he was ill. Consequently, he began having fainting spells any time he was sent to school or to do his homework. He missed 6 months of school due his psychological problems, and Jung loved the opportunity to spend his days exploring the world in any way he wished. He was eventually diagnosed with epilepsy, though Jung himself knew the diagnosis was ridiculous. One day he heard his father expressing great fear to a friend about what would become of Jung if he were unable to earn his own living. The reality of this statement was shocking to Jung, and “From that moment on I became a serious child.” He immediately went to study Latin, and began to feel faint. However, he consciously made himself aware of his neurosis, and cognitively fought it off. He soon returned to school, recognizing “That was when I learned what a neurosis is” (Jaffe, 1979; Jung, 1961; Wehr, 1989).

As he continued through school, his personal life continued to be quite strange. He began to believe that he was two people, one having lived 100 years earlier. He also had heated religious debates with his father. Fueling his courage during these debates was his belief that a vision had led to his understanding of true spirituality:

One fine summer day that same year I came out of school at noon and went to the cathedral square. The sky was gloriously blue, the day one of radiant sunshine. The roof of the cathedral glittered, the sun sparkling from the new, brightly glazed tiles. I was overwhelmed by the beauty of the sight, and thought: “The world is beautiful and the church is beautiful, and God made all this and sits above it far away in the blue sky on a golden throne and …” Here came a great hole in my thoughts, and a choking sensation. I felt numbed, and knew only: “Don’t go on thinking now! Something terrible is coming, something I do not want to think, something I dare not even approach. Why not? Because I would be committing the most frightful of sins. What is the most terrible sin? Murder? No, it can’t be that. The most terrible sin is the sin against the Holy Ghost, which cannot be forgiven. Anyone who commits that sin is damned to hell for all eternity. That would be very sad for my parents, if their only son, to whom they are so attached, should be doomed to eternal damnation. I cannot do that to my parents. All I need do is not go on thinking.” (pg. 36; Jung, 1961)

However, Jung was not able to ignore his vision. He was tormented for days, and spent sleepless nights wondering why he would have to think something unforgivable as a result of praising God for the beauty of all creation. His mother saw how troubled he was, but Jung felt that he could not dare confide in her. Finally, he decided that it was God’s will that he should face the meaning of this vision:

I thought it over again and arrived at the same conclusion. “Obviously God also desires me to show courage,” I thought. “If that is so and I go through with it, then He will give me His grace and illumination.”

I gathered all my courage, as though I were about to leap forthwith into hell-fire, and let the thought come. I saw before me the cathedral, the blue sky. God sits on His golden throne,

high above the world - and from under the throne an enormous turd falls upon the sparkling new roof, shatters it, and breaks the walls of the cathedral asunder. (pg. 39; Jung 1961)

Jung was overjoyed by his understanding of this vision. He believed that God had shown him that what mattered in life was doing God's will, not following the rules of any man, religion, or church. This was what Jung felt his own father had never come to realize, and therefore, his father did not know the "immediate living God." This conviction that one should pursue truth, rather than dogma, was an essential lesson that returned when Jung faced his dramatic split with Sigmund Freud.

When Jung decided to enter medical school, he did not leave his interest in strange spiritual matters behind. His cousin Helene Preiswerk led séances in which she would fall into a trance and channel strange spirits. The climax of these trances was often a mandala (a mandala is a geometric figure that represents wholeness, completeness, and perfection), which she would dictate to Jung, and then attempt to translate what was told to her by the spirits. Eugen Bleuler urged Jung to publish his studies on occult phenomena (remember that Bleuler defined schizophrenia), which Jung did, under the title On the Psychology and Pathology of So-called Occult Phenomena. Another important event that occurred early during Jung's medical training was the death of his father. The church had no provisions for the family of a deceased minister, but one of his uncles loaned Jung the money he needed to continue his studies. Upon completing medical school, he joined Dr. Bleuler in Zurich at the Burgholzli Mental Hospital, and soon became the first assistant physician. The Burgholzli clinic was a renowned institution. Bleuler was considered one of the two most influential psychiatrists of the day, and the clinic had come to prominence under his predecessor Auguste Forel, who was the first person to formally publish the theory that neurons communicate through synaptic junctions (though just how was not well understood at the time; Finger, 1994). Jung worked hard at Burgholzli, as Bleuler expected nothing less. He also spent some time in France, at the internationally recognized Salpetriere hospital, where he met Pierre Janet. Janet is a curious figure in the history of psychoanalysis. He claimed that he developed everything good in psychoanalysis, and that everything Freud developed was bad. Janet also apparently suggested that only the corrupt city of Vienna could be the source of a theory that traces the development of personality to sexual urges (Freud, 1914/1995). Jung spoke favorably of what he learned from Janet; Freud soundly rejected Janet's claims, but did grudgingly acknowledge that Janet did some important work on understanding neuroses (Freud, 1914/1995; Jung, 1961).

In 1906, Jung sent Freud a copy of his book The Psychology of Dementia Praecox (an earlier term for schizophrenia), which Freud found quite impressive. The two met in February, 1907, and talked for nearly 13 straight hours. According to Jung, "Freud was the first man of real importance I had encountered…no one else could compare with him." Very quickly, as evidenced in the letters quoted at the beginning of this chapter, Freud felt that Jung would become the leader of the psychoanalytic movement. In 1909, Jung's psychoanalytic practice was so busy that he resigned from the Burgholzli clinic, and he traveled to America with Freud. During this trip the two men spent a great deal of time together. It quickly became evident to Jung that he could not be the successor that Freud was seeking; Jung had too many differences of opinion with Freud. More importantly, however, Jung described Freud as neurotic, and wrote that the symptoms were sometimes highly troublesome (though Jung failed to identify those symptoms). Freud taught that everyone was a little neurotic, but Jung wanted to know how to cure neuroses:

Apparently neither Freud nor his disciples could understand what it meant for the theory and practice of psychoanalysis if not even the master could deal with his own neurosis. When, then, Freud announced his intention of identifying theory and method and making them into some kind of dogma, I could no longer collaborate with him; there remained no choice for me but to withdraw. (pg. 167; Jung, 1961)

Clearly Jung could not accept a dogmatic approach to psychoanalysis, since he believed that God Himself had told Jung not to follow any rigid system of rules. Even worse, this was when Jung first published his “discovery” of the collective unconscious. Freud wholly rejected this concept, and Jung felt that his creativity was being rejected. He offered to support Freud in public, while extending honest opinions in so-called “secret letters.” Freud wanted none of it. Almost as quickly as their relationship had grown, it fell apart (Jaffe, 1979; Jung, 1961; Wehr, 1989).

The loss of his relationship with Freud, following the loss of his father, led Jung in a period of personal crisis. He resigned his position at the University of Zurich, and began a lengthy series of experiments in order to understand the fantasies and dreams that arose from his unconscious. The more he studied these phenomena, the more he realized they were not from his own memories, but from the collective unconscious. He was particularly curious about mandala drawings, which date back thousands of years in all cultures. He studied Christian Gnosticism, alchemy, and the I Ching (or: Book of Changes). After meeting Richard Wilhelm, an expert on Chinese culture, Jung studied more Taoist philosophy, and he wrote a glowing foreword for Wilhelm’s translation of the I Ching (Wilhelm, 1950). These extraordinarily diverse interests led Jung to seek more in-depth knowledge from around the world. He traveled first to North Africa, then to America (to visit Pueblo Indians in New Mexico), next came East Africa (Uganda and Kenya), and finally India. Jung made every effort to get away from civilized areas, which might have been influenced by other cultures, in order to get a more realistic impression of the local culture, and he was particularly successful in this regard in meeting gurus in India (Jaffe, 1979; Jung, 1961; Wehr, 1989).

Through it all, he continued his work in psychology. He had developed his concept of psychological types, one of his most significant contributions, and published his work shortly after the break with Freud. He continued to develop his own form of psychoanalysis. Jung’s family was also an important part of his life. He had married Emma Rauschenbach in 1903. They had four daughters and one son, followed by nineteen grandchildren and many great-grandchildren. Emma Jung was very supportive of her husband, especially during the more turbulent periods of his career (including the break with Freud), and she was no stranger to his work. She had done some analytical work with Freud herself, she wrote essays on Jung’s concept of anima and animus, and she was the first president of the Psychological Club of Zurich. When his wife Emma died in 1955, Jung wrote in a letter that the loss had taken a lot out of him, and that at his age (80 years old) it wasn’t easy to recover. Yet two years later, he began dictating his autobiography to Aniela Jaffe. Looking ahead to the end of his life, Jung said:

The world into which we are born is brutal and cruel, and at the same time of divine beauty. Which element we think outweighs the other, whether meaninglessness or meaning is a matter of temperament. If meaninglessness were absolutely preponderant, the meaningfulness of life would vanish to an increasing degree with each step in our development. But that is - or seems to me - not the case. Probably, as in all metaphysical

questions, both are true: Life is - or has - meaning and meaninglessness. I cherish the anxious hope that meaning will preponderate and win the battle.

When Lao-tzu says: “All are clear, I alone am clouded,” he is expressing what I now feel in advanced old age. Lao-tzu is the example of a man with superior insight who has seen and experienced worth and worthlessness, and who at the end of his life desires to return into his own being, into the eternal unknowable meaning. The archetype of the old man who has seen enough is eternally true. (pp. 358-359; Jung, 1961)

Carl Jung died at home in 1961, in Kusnacht, Switzerland, at the age of 85. As psychologists today examine more deeply the relationship between Eastern and Western perspectives, it may be that Jung’s legacy has yet to be fulfilled.

Discussion Question: Even as a child, Jung had vivid dreams that he believed were giving him insight and guidance for the future. Have you ever had dreams so vivid, dreams that left such a powerful impression on you, that you felt they must have some special meaning? How did you respond, and what consequences, if any, followed your responses?

Placing Jung in Context: A Psychodynamic Enigma

Carl Jung holds an extraordinary place in the histories of psychiatry and psychology. Having already been an assistant to the renowned psychiatrist Eugen Bleuler, he went to Vienna to learn more about the fledgling science of psychoanalysis. He became Freud’s hand-picked heir to the psychoanalytic throne, and was one of the psychiatrists who accompanied Freud to America. Later, however, as he developed his own theories, he parted ways with Freud. Freud eventually came to describe Jung’s theories as incomprehensible, and Freud praised other psychiatrists who also opposed Jung’s ideas.

The most dramatic contribution that Jung made to psychodynamic thought was his concept of the collective unconscious, a mysterious reservoir of psychological constructs common to all people. Jung traveled extensively, including trips to Africa, India, and the United States (particularly to visit the Pueblo Indians in New Mexico), and he studied the cultures in those places. He also observed many basic similarities between different cultures. Those similarities led Jung to propose the collective unconscious. How else could so many significant cultural similarities have arisen within separate and distant lands? Jung did not reject the concepts already developed by Freud and Adler, including the dynamic interaction between the conscious mind and the personal unconscious, but he extended them in order to connect them with his own theory of our underlying collective unconscious. As strange as this theory seemed to Freud, and Freud wondered whether it even made sense to Jung, such a concept is not difficult to understand from an Eastern perspective.

Initially Jung’s theories had more influence on art, literature, and anthropology than they did on psychiatry and psychology. More recently, however, cognitive-behavioral theorists have begun to explore mindfulness as an addition to more traditional aspects of cognitive-behavioral therapies. As psychologists today study concepts from Yoga and Buddhism that are thousands of years old, Jung deserves the credit for bringing such an open-minded approach to the modern world of psychotherapy. Many famous and influential people admired Jung’s work, including psychiatrist Viktor Frankl, psychologist Erich Fromm, the authors Hermann Hesse and H. G. Wells, and Nobel Laureate (Physics) Wolfgang Pauli (for a number of interesting testimonials see Wehr, 1989). In addition, Jung’s discussion of how the libido has transformed throughout the evolution of the human species sounds very

much like sociobiology, which was not an established field until the 1970s. Clearly Jung did not simply dabble in a wide range of ideas, but rather, he had an extraordinary vision of the complexity of the human psyche.

Basic Concepts

In order to distinguish his own approach to psychology from others that had come before, Jung felt that he needed a unique name. Freud, of course, had chosen the term "psychoanalysis," whereas Alfred Adler had chosen "individual psychology." Since Jung admired both men and their theories, he chose a name intended to encompass not only their approaches, but others as well. Thus, he chose to call his approach **analytical psychology** (Jung, 1933).

Analytical psychology, as presented by Jung, addresses the question of the psyche in an open-minded way. He laments the overly scientific approach of the late 1800s and efforts to explain away the psyche as a mere epiphenomenon of brain function. Curiously, that debate remains with us today, and is still unanswered in any definitive way. Jung did not accept the suggestion that the psyche must come from the activity of the brain. This allowed him to consider the possibility of a collective unconscious, and fit well with his acceptance of the wisdom of Eastern philosophers. Indeed, Jung suggests that psychology will find truth only when it accepts both Eastern and Western, as well as both scientific and spiritual, perspectives on the psyche (Jung, 1933).

Dynamic Psychic Energy

Jung believed in a dynamic interaction between the conscious and unconscious minds, in a manner quite similar to that proposed by Freud. However, as we will examine below, his concept of the psyche included elements of an unconscious mind that transcends the individual, and may be considered a combination of the spirit, or soul, and one's thoughts and sensations. This inner psychic realm is capable of affecting the brain and its functions and, therefore, can influence one's perception of external reality. In addition, Jung thought of the libido somewhat differently than Freud. Although Jung considered sexuality to be an important aspect of the libido, primarily he thought of libido as a more generalized life energy (Douglas, 1995; Jarvis, 2004). Jung believed that as the human species evolved, the nature of sexual (or survival) impulses transformed. For example, early in human evolution we needed, as do other species, to be able to attract mates for procreation. Over time, these attraction behaviors generalized to behaviors such as art or music. Thus, a Freudian might say that creating music is a sexual act, but according to Jung "it would be a poor, unaesthetic generalization if one were to include music in the category of sexuality" (Jung, 1916/1963).

An important element of Jung's conception of the psyche and libido is found in the nature of opposites. Indeed, all of nature is composed of opposites:

…The concept of energy implies that of polarity, since a current of energy necessarily presupposes two different states, or poles, without which there can be no current. Every energic phenomenon…consists of pairs of opposites: beginning and end, above and below, hot and cold, earlier and later, cause and effect, etc. The inseparability of the energy concept from that of polarity also applies to the concept of libido. (pg. 202; Jung, 1971)

…opposites are the ineradicable and indispensable preconditions of all psychic life… (pg. 170; Jung, 1970)

In accordance with this view, Jung felt that the psyche sought balance, much like the concept of **entropy** from the field of physics. Entropy, in simple terms, is a thermodynamic principle that all energy within a system (including the universe) will eventually even out. Jung applied this principle to motivation, believing that we are driven forward through our lives in such a way that we might reduce the imbalance of psychic energy between opposing pairs of emotions (such as love and hate; Jarvis, 2004; Jung, 1971). Borrowing concepts from physics was certainly not a strange thing for Jung to do. As mentioned in Chapter 3, Freud's theories were motivated in part by advancements in science and Darwin's theory of evolution. Jung was personally acquainted with the Nobel Prize winning physicist Wolfgang Pauli, and the two published essays blending psychology and physics in a book entitled The Interpretation of Nature and the Psyche (Jung & Pauli, 1955). However, entropy and motivation are focused forward in time. Such an orientation toward the future marks another distinction between the theories of Freud and Jung.

Jung did not simply study symbolism, such as that in dreams, to uncover evidence of past repression. Jung believed that dreams could guide our future behavior, because of their profound relationship to the past, and their profound influence on our conscious mental life. Jung proposed that dreams can tell us something about the development and structure of the human psyche, and that dreams have evolved with our species throughout time. Since consciousness is limited by our present experience, dreams help to reveal much deeper and broader elements of our psyche than we can be aware of consciously. As such, dreams cannot easily be interpreted. Jung rejected the analysis of any single dream, believing that they belong within a series. He also rejected trying to learn dream analysis from a book. When done properly, however, dream analysis can provide unparalleled realism (see Jacobi & Hull, 1970; Jung, 1933):

…I cannot prove in every case that dreams are meaningful, for there are dreams that neither doctor nor patient understands. But I must regard them as hypothetically meaningful in order to find courage to deal with them at all…We must never forget in dream-analysis, even for a moment, that we move on treacherous ground where nothing is certain but uncertainty…When we take up an obscure dream, our first task is not to understand and interpret it, but to establish the context with minute care. What I have in mind is not a boundless sweep of "free associations" starting from any and every image in the dream, but a careful and conscious illumination of those chains of associations that are directly connected with particular images. (pp. 11-12; Jung, 1933)

In the final analysis, there is a particular challenge to understanding what dreams point to, and that is the situation under which a therapist typically learns of someone's dreams: in therapy. Jung suggested that if therapists could continue to observe the journey of one's dreams after therapy was successful, then the therapist, and possibly the client as well, might begin to more clearly understand the meaning and direction of the dreams. Still, dreams themselves are about both health and sickness, in keeping with Jung's principle of opposites. As such, Jung wrote that "dreams are the natural reaction of the self-regulating psychic system." The theory of entropy allows for an imbalance of energy in a closed-system. We may think of our conscious mind as just such a closed system. When we dream, however, the ongoing effort of our psyche to balance itself takes over, and the dreams counteract

what we have done to imbalance our psychological selves. Thus, it is within the context of dreams, not the details, that meaning is to be found (Jung, 1959a, 1968).

Discussion Question: Jung believed that the source of our motivation was a psychological drive to achieve balance (the effect of entropy on the psyche). Have you ever felt that you were being pushed or pulled in the wrong direction, or in too many directions at once, and simply wanted to achieve some balance in your life? In contrast, have there been times that your life was unfulfilling, and you needed something more in order to feel whole?

The Unconscious Mind

Perhaps Jung's most unique contribution to psychology is the distinction between a **personal unconscious** and a **collective unconscious**. The personal unconscious is not entirely different than that proposed by Freud, but is more extensive. In addition to repressed memories and impulses, the personal unconscious contains undeveloped aspects of the personality and material arising from the collective unconscious that is not yet ready for admission into conscious awareness. The personal unconscious is revealed through clusters of emotions, such as those resulting in a particular attitude toward one's father or other father figures, which Jung referred to as a **complex** (Douglas, 1995; Jarvis, 2004). In this sense, a complex is not synonymous with a psychological problem, as the term is often used today, but rather any general state of mind common to certain situations. In this context, it is quite similar to the **schemas** discussed by cognitive theorists.

Jung arrived at his theory of complexes as a result of his research into schizophrenia, under the direction of Dr. Bleuler. Bleuler had assigned Jung the task of studying the **Word Association Test**, a test in which a list of 100 words is read to the patient, and the therapist watches for evidence of emotional arousal, such as pauses, failures to respond, or physical acts. In addition, Jung noticed that even with schizophrenic patients, the patterns of word association were often centered on a particular theme. That theme could then be regarded as a complex (which, again, could be either positive or negative). Sometimes, complexes remain unresolved, such as one's feelings about parents, if the parents have died. In relatively healthy individuals, these unresolved complexes could result in dreams, visions, or similar phenomena pertaining to the object(s) of the unresolved complex. These complexes might even become personified. In the extreme situation of personified complexes, such as in a person suffering from schizophrenia, the patient cannot distinguish the personification of the unresolved complex from the seeming reality of being another person. Hence, the schizophrenic "hears" voices in their head, "spoken" by someone else. As Jung further investigated the nature and themes of complexes in psychiatric patients, he found common themes that could not always be attributed to the patient's personal history. And so, he began to form his concept of the collective unconscious (Douglas, 1995; Jarvis, 2004; Jung, 1959b, 1961; Storr, 1983).

…from the standpoint of the psychology of the personality a twofold division ensures: an "extra-conscious" psyche whose contents are personal, and an "extra-conscious" psyche whose contents are impersonal and collective. The first group comprises contents which are integral components of the individual personality and could therefore just as well be conscious; the second group forms, as it were, an omnipresent, unchanging, and everywhere identical quality or substrate of the psyche per se. This is, of course, no more than a hypothesis. But we are driven to it by the peculiar nature of the empirical material…

Whereas the contents of the personal unconscious are acquired during the individual's lifetime, the contents of the collective unconscious are invariably archetypes that were present from the beginning. (pp. 7-8; Jung, 1959c)

Thus, according to Jung, the collective unconscious is a reservoir of psychic resources common to all humans (something along the lines of psychological instinct). These psychic resources, known as **archetypes**, are passed down through the generations of a culture, but Jung considered them to be inherited, not learned. As generation after generation experienced similar phenomena, the archetypal images were formed. Despite cultural differences, the human experience has been similar in many ways throughout history. As such, there are certain archetypes common to all people. According to Jung, the most empirically valid archetypes, and therefore the most powerful, are the **shadow**, the **anima**, and the **animus** (Jung, 1959c).

Jung described the shadow as "the inferior and less commendable part of a person," and "a moral problem that challenges the whole ego-personality" (Jung, 1940, 1959c). It encompasses desires and feelings that are not acceptable to society or the conscious psyche. With effort the shadow can be somewhat assimilated into the conscious personality, but portions of it are highly resistant to moral control. As a result, we tend to project those thoughts, feelings, or emotions onto other people. When they have moved beyond one's control, such as when we lose our temper, these projections isolate the individual from their environment, since they are no longer approaching situations realistically. Jung described the circumstance as tragic when people continue to ruin their lives, and the lives of others, because they cannot see through the illusion of how their shadow has been projected, and consequently interfered with their ability to live a healthy life (Jung, 1959c).

The shadow is not, however, entirely evil. Rather, Jung described it as un-adapted and awkward, much like a child trying to function in the company of adults. Trying to entirely suppress the shadow is not the appropriate solution, since the shadow is driving us forward in our efforts to achieve balance between the unconscious and conscious realities. In other words, just as a child may act inappropriately while trying to grow up, the shadow may cause inappropriate behavior in opposition to the accepted rules of society. Nonetheless, it is important for us to have that driving force pushing us toward self-development (and the development of the human species), so that we don't simply live a life of passivity and/or reaction to outside events. It is the shadow that pushes us forward (Jacobi & Hull, 1970; Jung, 1961).

Although many people emphasize the differences between men and women, psychologically their common traits can readily be observed. Jung described the anima as the female aspect of the male psyche, and the animus as the male aspect of the female psyche. Jung intentionally addresses this difficult concept in mythological terms, but he also makes it clear that this is a natural phenomenon for each person, and not a substitute for one's mother (in the case of the anima) or father (in the case of the animus). While the presence of a feminine aspect within the male psyche and the presence of a masculine aspect within the female psyche have some positive benefits, such as making it possible for men and women to relate to one another, the unfortunate reality is often the opposite. In 1959(c), Jung described the difficulties that men and women have relating to family and friends of the opposite sex, due to fundamental differences in style. Although men may contain the anima, they are still primarily masculine, whereas women, despite the animus, are still primarily

feminine. As with the shadow, relationship problems can arise from the anima or animus when we allow our archetypal image to be projected onto others. As Jung himself noted, many men project a desired image onto a woman that would require her to be a sexually vivacious virgin, something of a contradiction in terms. Thus, over time, such a man's relationships may suffer as a result of his learning more about the real life of his companion, even though she has done nothing but be herself (Jung, 1940, 1959c).

Jung did not place a limit on the number of possible archetypes, and he described quite a few in his writings. It did not matter to Jung whether archetypes were, in fact, real. In a perspective quite similar to cognitive theorists, he wrote that "insofar as the archetypes act upon me, they are real and actual to me, even though I do not know what their real nature is" (Jung, 1961). One of the more important archetypes is the self, which represents the integration of the whole personality. Indeed, Jung described the self as the goal of all psychic development. A special type of image often associated with the self, and with Jung himself, is the **mandala**. A mandala is a geometric figure that represents wholeness, completeness, perfection (Jung, 1958). They also tend to be symmetrical, representing the natural balance of opposites. Although they typically have religious or spiritual significance, it is not required. Jung was very interested in mandalas, and from 1916 to 1918 he draw a new one every morning (Wehr, 1989). Mandalas can appear in dreams as an image of wholeness, or in times of stress they may appear as compensatory images (Douglas, 1995). Their potential healing ability stems from their connection between the uniqueness of our present consciousness and the depths of our primordial past:

…The psyche is not of today; its ancestry goes back many millions of years. Individual consciousness is only the flower and the fruit of a season, sprung from the perennial rhizome beneath the earth; and it would find itself in better accord with the truth if it took the existence of the rhizome into its calculations. For the root matter is the mother of all things. (pg. xxiv; Jung, 1956)

It is important to note that archetypal images are considered to be ancient. Although we talk about them as if they are still forming, and that may well be possible, the fact is that there were countless human generations long before recorded history. Jung has referred to archetypes as primordial images, "impressed upon the mind since of old" (Jung, 1940). Archetypes have been expressed as myths and fables, some of which are thousands of years old even within recorded history. As the eternal, symbolic images representing archetypes were developed, they naturally attracted and fascinated people. That, according to Jung, is why they have such profound impact, even today, in our seemingly advanced, knowledgeable, and scientific societies.

Table 4.1: Common Archetypes in Jung's Theory of the Collective Unconscious*	
Self	Integration and wholeness of the personality, the center of the totality of the psyche; symbolically represented by, e.g., the mandala, Christ, or by helpful animals (such as Rin Tin Tin and Lassie or the Hindu monkey god Hanuman)
Shadow	The dark, inferior, emotional, and immoral aspects of the psyche; symbolically represented by, e.g., the Devil (or an evil character such as Dracula), dragons, monsters (such as Godzilla)

Anima	Strange, wraithlike image of an idealized women, yet contrary to the masculinity of the man, draws the man into feminine (as defined by gender roles) behavior, always a supernatural element; symbolically represented by, e.g., personifications of witches, the Greek Sirens, a femme fatale, or in more positive ways as the Virgin Mary, a romanticized beauty (such as Helen of Troy) or a cherished car
Animus	A source of meaning and power for women, it can be opinionated, divisive, and create animosity toward men, but also creates a capacity for reflection, deliberation, and self-knowledge; symbolically represented by, e.g., death, murderers (such as the pirate Bluebeard, who killed all his wives), a band of outlaws, a bewitched prince (such as the beast in “Beauty and the Beast”) or a romantic actor (such as Rudolph Valentino)
Persona	A protective cover, or mask, that we present to the world to make a specific impression and to conceal our inner self; symbolically represented by, e.g., a coat or mantle
Hero	One who overcomes evil, destruction, and death, often has a miraculous but humble birth; symbolically represented by, e.g., angels, Christ the Redeemer, or a god-man (such as Hercules)
Wise Old Man	Typically a personification of the self, associated with saints, sages, and prophets; symbolically represented as, e.g., the magician Merlin or an Indian guru
Trickster	A childish character with pronounced physical appetites, seeks only gratification and can be cruel and unfeeling; symbolically represented by, e.g., animals (such as Brer Rabbit, Wile E. Coyote or, often, monkeys) or a mischievous god (such as the Norse god Loki)
For more information read The Integration of the Personality (Jung, 1940), Aion: Researches Into the Phenomenology of the Self (Jung, 1959c), and Man and His Symbols* (Jung, et al., 1964).	

Discussion Question: What is your impression of the concept of archetypes? Think about mythic heroes and gods, or concepts of motherhood or being a father. Can you identify commonalities between different cultures, now or throughout time, which seem to suggest themes that are common to all people?

Connections Across Cultures: Symbolism

Throughout Time and Around the world

Near the end of Jung’s life, he was asked to write a book that might make his theories more accessible to common readers. Jung initially refused, but then he had an interesting dream, receiving advice from his unconscious psyche that he should reconsider his refusal:

…He dreamed that, instead of sitting in his study and talking to the great doctors and psychiatrists who used to call on him from all over the world, he was standing in a public place and addressing a multitude of people who were listening to him with rapt attention and understanding what he said… (pg. 10; John Freeman, in his introduction to Man and His Symbols, Jung et al., 1964)

Jung then agreed to write the book that became known as Man and His Symbols, but only if he could hand-pick the co-authors who would help him. Jung supervised every aspect of the book, which was nearly finished when he died. Written purposefully to be easily understood by a wide audience, the book presents an astonishingly wide variety of symbolism from art, archaeology, myth, and analysis within the context of Jung's theories. Many of the symbols were represented in dreams, and symbolic dreams are the primary means by which our unconscious psyche communicates with our conscious psyche, or ego. It is extraordinary to see how similar such symbolism has been throughout time and across cultures, even though each individual example is unique to the person having the dream or expressing themselves openly.

Symbols, according to Jung, are terms, names, images, etc. that may be familiar in everyday life, but as symbols they come to represent something vague and unknown, they take on meaning that is hidden from us. More specifically, they represent something within our unconscious psyche that cannot ever be fully explained. Exploring the meaning will not unlock the secrets of the symbol, because its meaning is beyond reason. Jung suggests that this should not seem strange, since there is nothing that we perceive fully. Our eyesight is limited, as is our hearing. Even when we use tools to enhance our senses, we still only see better, or hear better. We don't comprehend the true nature of visual objects or sounds, we only experience them differently, within our psychic realm as opposed to their physical reality. And yet, the symbols created by our unconscious psyche are very important, since the unconscious is at least half of our being, and it is infinitely broader than our conscious psyche (Jung et al., 1964).

Jung believed that the symbols created in dreams have a deeper meaning than Freud recognized. Freud believed that dreams simply represent the unconscious aspects of one's psyche. Jung believed, however, that dreams represent a psyche all their own, a vast and ancient psyche connected to the entire history of humanity (the collective unconscious). Therefore, dreams can tell a story of their own, such as Jung's dream encouraging him to write a book for a common audience. Thus, his dream did not reflect some underlying neurosis connected to childhood trauma, but rather, his unconscious psyche was pushing him forward, toward a sort of wholeness of self by making his theories more readily accessible to those who are not sufficiently educated in the wide variety of complex topics that are typically found in Jung's writings. By virtue of the same reasoning, Jung considered dreams to be quite personal. They could not be interpreted with dream manuals, since no object has any fixed symbolic meaning.

What makes the symbolism within dreams, as well as in everyday life, most fascinating, however, is how common it is throughout the world, both in ancient times and today. In their examination of symbols and archetypes, Jung and his colleagues offer visual examples from: Egypt, England, Japan, the Congo, Tibet, Germany, Belgium, the United States, Bali, Haiti, Greece, Switzerland, Spain, Italy, Cameroon, Java, France, Kenya, India, Sweden, Russia, Poland, Australia, China, Hungary, Malaysia, Borneo, Finland, the Netherlands, Rhodesia,

Israel, Saudi Arabia, Scotland, Ireland, Brazil, Monaco, Burma, Bolivia, Cambodia, Denmark, Macedonia, and Peru, as well as from Mayan, Celtic, Babylonian, Persian, Navaho, and Haidu cultures. There are also many Biblical references. It would be safe to say that no one else in the history of psychology has so clearly demonstrated the cross-cultural reality of their theory as is the case with Carl Jung.

Of course, as with dreams, many of these symbols are unique to the culture in which they have arisen. Therefore, it takes a great deal of training and experience for a psychotherapist to work with patients from different cultures. Nonetheless, the patterns represent the same basic concepts, such as self, shadow, anima, animus, hero, etc. Once recognized in their cultural context, the analyst would have a starting point from which to begin working with their patient, or the artist would understand how to influence their audience. One important type of art that relies heavily on cultural images and cues is advertising. Cultural differences can create problems for companies pursuing global marketing campaigns. Jung's theory suggests that similarities in how we react to certain archetypal themes should be similar in different countries, but of course the images themselves must be recognizable, and we may still be a long way from understanding those fundamental images:

…Our actual knowledge of the unconscious shows that it is a natural phenomenon and that, like Nature herself, it is at least neutral. It contains all aspects of human nature – light and dark, beautiful and ugly, good and evil, profound and silly. The study of individual, as well as of collective, symbolism is an enormous task, and one that has not yet been mastered. (pg. 103; Jung et al., 1964)

Personality Types

One of Jung's most practical theories, and one that has been quite influential, is his work on **personality types**. Jung had conducted an extensive review of the available literature on personality types, including perspectives from ancient Brahmanic conceptions taken from the Indian Vedas (see below) and types described by the American psychologist William James. In keeping with one of Jung's favorite themes, James had emphasized opposing pairs as the characteristics of his personality types, such as rationalism vs. empiricism, idealism vs. materialism, or optimism vs. pessimism (see Jung, 1971). Based on his research and clinical experience, Jung proposed a system of personality types based on **attitude-types** and **function-types** (more commonly referred to simply as attitudes and functions). Once again, the attitudes and functions are based on opposing ways of interacting with one's environment.

The two attitude-types are based on one's orientation to external objects (which includes other people). The **introvert** is intent on withdrawing libido from objects, as if to ensure that the object can have no power over the person. In contrast, the **extravert** extends libido toward an object, establishing an active relationship. Jung considered introverts and extraverts to be common amongst all groups of people, from all walks of life. Today, most psychologists acknowledge that there is a clear genetic component to these temperaments (Kagan, 1984, 1994; Kagan, Kearsley, & Zelazo 1978), a suggestion proposed by Jung as well (Jung, 1971). Of course, one cannot have an orientation to objects without consciousness, and consciousness cannot exist without an **ego**. For Jung, the ego is a complex, so it is associated with both the conscious psyche and the personal unconscious. According to Jung, "it is always in the center of our attention and of our desires, and it is the absolutely indispensable centre of consciousness" (Jung, 1968).

Jung's four functions describe ways in which we orient ourselves to the external environment, given our basic tendency toward introversion or extraversion. The first opposing pair of functions is **thinking** vs. **feeling**. Thinking involves intellect, it tells you whata thing is, whereas feeling is values-based, it tells what a thing is worth to you. For example, if you are trying to choose classes for your next semester of college, perhaps you need to choose between a required general education course as opposed to a personally interesting course like Medical First Responder or Interior Design. If you are guided first by thinking, you will probably choose the course that fulfills a requirement, but if you are guided by feeling, you may choose the course that satisfies your more immediate interests. The second opposing pair of functions is **sensing** vs. **intuition**. Sensing describes paying attention to the reality of your external environment, it tells you that something is. In contrast, intuition incorporates a sense of time, and allows for hunches. Intuition may seem mysterious, and Jung freely acknowledges that he is particularly mystical, yet he offers an interesting perspective on this issue:

…Intuition is a function by which you see round corners, which you really cannot do; yet the fellow will do it for you and you trust him. It is a function which normally you do not use if you live a regular life within four walls and do regular routine work. But if you are on the Stock Exchange or in Central Africa, you will use your hunches like anything. You cannot, for instance, calculate whether when you turn round a corner in the bush you will meet a rhinoceros or a tiger - but you get a hunch, and it will perhaps save your life… (pg. 14; Jung, 1968)

The two attitudes and the four functions combine to form eight personality types. Jung described a so-called cross of the functions, with the ego in the center being influenced by the pairs of functions (Jung, 1968). Considering whether the ego's attitude is primarily introverted or extraverted, one could also propose a parallel pair of crosses. Jung's theory on personality types has proven quite influential, and led to the development of two well-known and very popular instruments used to measure one's personality type, so that one might then make reasoned decisions about real-life choices.

Jung proposed a "cross of the functions," in which the ego sits at the center of the opposing pairs of functions (Jung, 1968). When the attitudes of introversion and extraversion are included, one can represent Jung's view as parallel crosses of the functions.

Discussion Question: Jung described two attitudes (introversion-extraversion) and four functions (thinking-feeling, sensing-intuition) as the primary basis for psychological types. Think about yourself and/or some of your close friends and family members. Can you use the eight types described by Jung to get a reasonable impression of the people you know, or does Jung's theory seem to fall short?

In 1923, Katharine Briggs and her daughter Isabel Briggs Myers learned of Jung's personality types and became quite interested in his theory. After spending 20 years observing individuals of different types, they added one more pair of factors based on a person's preference for either a more structured lifestyle, called **judging**, or a more flexible or adaptable lifestyle, called **perceiving**. There were now, according to Briggs and Myers, sixteen possible personality types. In the 1940s, Isabel Myers began developing the **Myers-Briggs Type Indicator** (MBTI) in order to help people learn about their personality type. To provide just one example of an MBTI profile, an individual who is extraverted and prefers sensing, thinking, and judging (identified by the initials ESTJ) would be described as:

“Practical, realistic, matter-of-fact. Decisive, quickly move to implement decisions. Organize projects and people to get things done…Forceful in implementing their plans” (Myers, 1993; Myers & McCaulley, 1985; see also the website for the Myers & Briggs Foundation, at www.myersbriggs.org). While it is relatively easy to find shortcut tests or variations of the MBTI online, if one plans to make any meaningful decisions based on their personality type they should consult a trained MBTI administrator. What sort of decision might one make? The MBTI has become a popular tool for looking at career choices and workplace relationships. A number of popular books, such as Do What You Are (Tieger & Barron-Tieger, 2001) and Type Talk at Work (Kroeger, Thuesen, & Rutledge, 2002), are available that provide information intended to help people choose satisfying careers and be successful in complex work environments. In addition to its use in career counseling, the MBTI has been used in individual counseling, marriage counseling, and in educational settings (Myers, 1993; Myers & McCaulley, 1985; Myers & Myers, 1980). Another popular instrument, based once again on Jung’s theory and compared directly to the MBTI, is the **Keirsey Temperament Sorter**. David Keirsey uses plain language in an effort to make personality types easy to comprehend. Setting aside introversion vs. extraversion, he has identified eight character portraits: mentors, organizers, monitors, operators, advocates, engineers, conservators, and players. When Keirsey adds extraversion and introversion back into the mixture, he can identify one’s personality type even more clearly: monitors become either supervisors (E) or inspectors (I), players become performers (E) or composers (I), engineers become inventors (E) or designers (I), etc. As with the MBTI, Keirsey provides concrete recommendations regarding how one might use the results of his temperament sorter to make decisions about one’s choices in life (Keirsey, 1987).

Table 4.2: Jung’s Eight Personality Types*	
Introverted Thinking	Focused on own internal thoughts and ideas, do not communicate well, can be highly conflicted and will lash out at critics, generally stubborn and do not get along well with others
Introverted Feeling	Tend to be silent, inaccessible, and melancholy, have deep emotions but hide them and appear cold and reserved on the surface, tend to be suspicious of others, most are women
Introverted Sensing	Guided by subjective impression of real-life objects, often express their sensations through artistic endeavors, the objective world may seem make-believe and comical
Introverted Intuitive	Tend to be peculiar and lack contact with reality, may be completely misunderstood even by those who are close to them, may seem like a mystical dreamer and seer on one hand but just a cranky person on the other, may have vision but lack convincing power of reason
Extraverted Thinking	Seek intellectual conclusions based on objective reality, seek to influence others, suppress emotion, can be rigid and dogmatic (tyrannical when others penetrate their power province)

Extraverted Feeling	Feelings harmonize with objective situations, can be highly emotional, will avoid thinking when it proves upsetting, most are women
Extraverted Sensing	Immersed in realism and seek new experiences, whole aim is concrete enjoyment, most are men
Extraverted Intuitive	Always seek new opportunities, may seize new opportunity with enthusiasm and just as quickly abandon it if not promising, has vision, often found among business tycoons and politicians, but have little regard for welfare of others
For more information read Psychological Types* (Jung, 1971).	

Personality Development

Jung believed that “everyone’s ultimate aim and strongest desire lie in developing the fullness of human existence that is called personality” (Jung, 1940). However, he lamented the misguided attempts of society to educate children into their personalities. Not only did he doubt the abilities the average parent or average teacher to lead children through the child’s personality development, given their own personal limitations, he considered it a mistake to expect children to act like young adults:

It is best not to apply to children the high ideal of education to personality. For what is generally understood by personality - namely, a definitely shaped, psychic abundance, capable of resistance and endowed with energy - is an adult ideal…No personality is manifested without definiteness, fullness, and maturity. These three characteristics do not, and should not, fit the child, for they would rob it of its childhood. (pp. 284-285; Jung, 1940)

This is not to suggest that childhood is simply a carefree time for children:

No one will deny or even underestimate the importance of childhood years; the severe injuries, often lasting through life, caused by a nonsensical upbringing at home and in school are too obvious, and the need for reasonable pedagogic methods is too urgent…But who rears children to personality? In the first and most important place we have the ordinary, incompetent parents who are often themselves, all their lives, partly or wholly children. (pg. 282; Jung, 1940)

So, if childhood is a critical time, but most adults never grow up themselves, what hope does Jung see for the future? The answer is to be found in midlife. According to Jung, the middle years of life are “a time of supreme psychological importance” and “the moment of greatest unfolding” in one’s life (cited in Jacobi & Hull, 1970). In keeping with the ancient tradition of the Vedic stages of life, from Hindu and Indian culture, the earlier stages of life are about education, developing a career, having a family, and serving one’s proper role within society:

Man has two aims: The first is the aim of nature, the begetting of children and all the business of protecting the brood; to this period belongs the gaining of money and social position. When this aim is satisfied, there begins another phase, namely, that of culture. For the attainment of the former goal we have the help of nature, and moreover of education;

but little or nothing helps us toward the latter goal... (pg. 125; Jung, cited in Jacobi & Hull, 1970)

So where does one look for the answers to life? Obviously, there is no simple answer to that question, or rather, than are many answers to that question. Some pursue spiritual answers, such as meditating or devoting themselves to charitable causes. Some devote themselves to their children and grandchildren, others to gardening, painting, or woodworking. The answer for any particular individual is based on that person's **individuation**.

Individuation is the process by which a person actually becomes an "individual," differentiated from all other people. It is not to be confused with the ego, or with the conscious psyche, since it includes aspects of the personal unconscious, as influenced by the collective unconscious. Jung also described individuation as the process by which one becomes a "whole" person. To some extent, this process draws the individual away from society, toward being just that, an individual. However, keeping in mind the collective unconscious, Jung believed that individuation leads to more intense and broader collective relationships, rather than leading to isolation. This is what is meant by a whole person, one who successfully integrates the conscious psyche, or ego, with the unconscious psyche. Jung also addresses the Eastern approaches, such as meditation, as being misguided in their attempts to master the unconscious mind. The goal of individuation is wholeness, wholeness of ego, unconscious psyche, and community (Jung, 1940, 1971):

Consciousness and the unconscious do not make a whole when either is suppressed or damaged by the other. If they must contend, let it be a fair fight with equal right on both sides. Both are aspects of life…It is the old play of hammer and anvil: the suffering iron between them will in the end be shaped into an unbreakable whole, the individual. (pg. 27; Jung, 1940)

Jimmy Carter was a successful farmer, Governor of Georgia, and President of the United States. As president he brokered a peace treaty between Egypt and Israel that earned him a Nobel Peace Prize nomination. Despite such an illustrious life to that point, he has, perhaps, done even more good since then. After devoting himself to a wide variety of social and political activities focusing on peace and justice, including personally building houses with Habitat for Humanity, the Honorable Mr. Carter received a well-earned Nobel Peace Prize in 2002.

Discussion Question: Jung identified the first half of life as a time to take care of the biological needs of the species (e.g., raising children). It is in the second half of life, according to Jung, that one looks within oneself for the guidance and motivation to fulfill the needs of society and one's culture.

Jungian Analysis

Jung was deeply moved by his ability to help patients, and he took the process of analysis very seriously. This is not to say, however, that he considered a specific process to be necessary. He was suspicious of theoretical assumptions, he focused on each individual in therapy, and was just as likely to adapt Adlerian techniques as he was to adapt Freudian techniques. In fact, he considered general professional experience to be an important aspect of one's ability to be a good analyst. He did consider training to be very important, and that it should include medical training. He was not opposed to lay analysts (those with a Ph.D., rather than an M.D.), but felt that they should be supervised by a psychiatrist (Jung, 1961).

He built his general system of psychotherapy on four tenets: the psyche is a self-regulating system, the unconscious has creative and compensatory components, the doctor-patient relationship is crucial, and personality growth takes place throughout the lifespan. In addition, the process of psychotherapy involves four stages: **confession**, **elucidation**, **education**, and **transformation** (see Douglas, 1995). The confession and elucidation stages involve the patient recounting elements of their personal history, dreams, and fantasies, followed by the analyst bringing attention to symptoms, transferences, and attempting to help the patient gain insight on both intellectual and emotional levels. The education stage then involves moving the patient into the realm of an individual, hopefully as an adapted social being. Education focuses mostly on the persona and the ego, whereas confession and elucidation serve the role of exploring the personal unconscious. The education stage also involves trying to provide the patient with realistic options for changing their behavior. The final stage, transformation, was described by Jung as being similar to self-actualization. Also like self-actualization, not every patient (or person) makes it to this stage. Addressing the archetypal image of the self, the image of wholeness, requires working with the whole range of the conscious, personal unconscious, and collective unconscious psyches. The final goal is to inspire the patient to become a uniquely individual self without losing a sense of responsible integrity (the collectiveness inherent in the collective unconscious; see Douglas, 1995).

Since Jung did not want to be tied to any specific technique(s), he incorporated a variety of techniques as appropriate for each patient. Like Freud, Jung experimented with hypnosis early in his career, but discarded the technique as ineffective. He used dream analysis regularly, but did not consider each dream to be necessarily important. Instead, he looked for patterns in dreams over time, particularly recurring dreams. Jung taught his patients to get in touch with their own unconscious psyche through an active imagination technique. This meditative imagery procedure is somewhat similar to that of the Buddhist mindfulness techniques taught by Gotama Buddha some 2,600 years ago, but involves much more active cognition. Jung’s active imagery technique was also extended into actual physical activity. Jung found it helpful, especially with particularly withdrawn patients, to have them act out their thoughts and feeling. Jung would even mimic their movements to help himself better understand what his patients were trying to communicate (Douglas, 1995). The importance of physical states, as reflective of psychological states, was developed in more detail in the somatic psychology theories of Wilhelm Reich. In addition, Jung extended his therapeutic approach to group therapy, family and marital therapy, art therapy, child therapy, and the recurrent nightmares of patients suffering from post-traumatic stress. Through it all, Jung paid special attention to complexes, as representative of the psychic processes of the patient. Once an analyst understands both the symptoms and the complexes of the patient, Jung believed, the analyst has found the key to treatment (Douglas, 1995).

Like Freud, Jung felt that analytical psychology could serve a greater purpose beyond just helping individuals, so he turned his attention to society in several of his books. In Modern Man in Search of a Soul (Jung, 1933), Jung addressed the spiritual problems of our times. A modern person, according to Jung, is one who is whole, aware of their conscious psyche and their unconscious psyche (both personal and collective). Though such people are few and far between, Jung believed he saw evidence of a need for understanding the unconscious psyche. One obvious piece of evidence was the rise of psychology as a discipline at the beginning of the twentieth century. However, in contrast to Jung’s personal preference,

psychology has largely moved away spiritual pursuits, with its preference for a scientific evaluation of the human mind. Religion, in Jung's opinion, has also moved away from spirituality, in favor of the dogma of human rules and regulations. Jung believed that we can learn a great deal from Yoga and Buddhism in terms of blending psychology and spirituality in order to understand the whole nature of human beings. In The Undiscovered Self (Jung, 1957), Jung continues this argument in a more personal way. He suggests that modern psychological approaches to "self-knowledge" are at best superficial, and don't address the real psychic processes that lead to individuality. As a general condemnation of our obsession with science, he questions any theory based on statistical averages, since those averages say very little at all about the unique units being studied, whether they are people or some other object. In Civilization in Transition (Jung, 1964), the tenth volume of Jung's collected works, he addresses the problems of American psychology, and what all Westerners can learn from the ancient wisdom of India about studying our own unconscious psyche. The importance of understanding the true and complete nature of our psyche is that until we do we cannot live our lives to the fullest:

Our souls as well as our bodies are composed of individual elements which were all already present in the ranks of our ancestors. The "newness" in the individual psyche is an endlessly varied recombination of age-old components…Once the past has been breached, it is usually annihilated, and there is no stopping the forward motion. But it is precisely the loss of connection with the past, our uprootedness, which has given rise to the "discontents" of civilization and to such a flurry and haste that we live more in the future and its chimerical promises of a golden age than in the present, with which our whole evolutionary background has not yet caught up. (pp. 235-236; Jung, 1961)

A Final Note on Carl Jung

It can be something of a challenge to view Jung's work as psychological. It lends itself more readily to, perhaps, the study of the humanities, with elements of medieval pseudo-science, Asian culture, and native religions (an odd combination, to be sure). With titles such as Aion: Researches into the Phenomenology of the Self (Jung, 1959c) and Mysterium Conjunctionis: An Inquiry into the Separation and Synthesis of Psychic Opposites in Alchemy (Jung, 1970), Jung is not exactly accessible without a wide range of knowledge in areas other than psychology. **Alchemy**was of particular interest to Jung, but not in terms of turning base metals into gold (alchemy is a strange mixture of spirituality and chemistry). Rather, Jung believed that psychology could find its base in alchemy, and that it was the collective unconscious that came forth in the ongoing human effort to understand the nature of matter (Jaffe, 1979; Jung, 1961; Wehr, 1989). He even went so far as to write about flying saucers, the astrological seasons of time, and the prophesies of Nostradamus (Jung, 1959c; Storr, 1983).

And yet, Jung addressed some very important and interesting topics in psychology. His theory of psychological types is reflected in trait descriptions of personality and corresponding trait tests, such as Cattell's 16-PF and the MMPI. The value Jung placed on mid-life and beyond, based largely on the ancient Vedic stages of life, suggests that one is not doomed to the negative alternative in Erikson's final psychosocial crises. So Jung's personal interests, and his career as a whole, straddled the fence between surreal and practical. He may always be best-known for his personal relationship with Freud, brief as it was, but the blending of Eastern and Western thought is becoming more common in

psychology. So perhaps Jung himself will become more accessible to the field of psychology, and we may find a great deal to be excited about in his curious approach to psychodynamic theory.

Personality Theory in Real Life: Synchronicity - Coincidence or an Experience with Mystical Spirituality?

Many people are deeply religious and many others consider themselves to be just as deeply spiritual, though not connected to any specific religion. As important as religion and spirituality are in the lives of many people, psychology has tended to avoid these topics, primarily because they do not lend themselves readily to scientific investigation. Jung certainly did not avoid these topics, and he studied a wide range of spiritual topics. For example, he wrote the foreword for Richard Wilhelm's translation of the I Ching (Wilhelm, 1950), he wrote psychological commentary for a translation of The Tibetan Book of the Great Liberation (Evans-Wentz & Jung, 1954), he discussed the psychology of evil in Answer to Job(Jung, 1954), he wrote about Gnostic traditions at length (see Segal, 1992), and one of the volumes of his collected works is entitled Psychology and Religion: West and East (Jung, 1958). In addition to his varied spiritual interests, Jung became interested in psychological phenomena that could not be explained in scientific terms. Such phenomena do not necessarily require a spiritual explanation, but in the absence of any other way to explain them, they are often thought of in spiritual terms. One such topic is **synchronicity**.

Jung uses the term synchronicity to describe the "coincidence in time of two or more causally unrelated events which have the same or a similar meaning" (Jung & Pauli, 1955). In particular, it refers to the simultaneous occurrence of a particular psychic state with one or more external events that have a meaningful parallel to one's current experience or state of mind. I would like to share with you two experiences of synchronicity from my own life. Having a Ph.D. in physiological psychology, and having spent a number of years conducting biomedical research, it seems rather strange to be sharing experiences that can be classified as extrasensory perception, or ESP. Jung also found it difficult to address this topic:

…If I have now conquered my hesitation and at last come to grips with my theme, it is chiefly because my experiences of the phenomenon of synchronicity have multiplied themselves over the decades…As a psychiatrist and psychotherapist I have often come up against the phenomena in question and could convince myself how much these inner experiences meant to my patients. In most cases they were things which people do not talk about for fear of exposing themselves to thoughtless ridicule. I was amazed to see how many people have had experiences of this kind and how carefully the secret was guarded. So my interest in this problem has a human as well as a scientific foundation. (pp. 5-6; Jung, in Jung & Pauli, 1955)

Synchronicity Experience #1: In February, 1992, our first son, Mark David Kelland, Jr., died at birth. Later that year, I was traveling across the country, taking a whole week to get to a neuroscience convention in California, and I decided to hike up Wheeler Peak, the highest point in New Mexico (13,161 feet). I arrived a little late in the day, but decided I had enough time to make the climb and get back down off the ridge before dark. I just made it, but still had a few miles to go on the jeep trail down to the parking lot. Along the way, I stopped and looked back up at Wheeler Peak. I turned off my flashlight, and in the dark I could just make out the outline of the mountain against the night sky. I prayed to God for a sign that our son

was in Heaven. The instant I said “Amen,” a brilliant shooting star streaked across the sky and descended behind Wheeler Peak! I took it to be the sign I had asked for.

Synchronicity Experience #2: While growing up, we lived next door to Bill and Jackie O’Reilly, who co-owned the corner drugstore in the center of Foxborough, MA. When I was young I mowed their lawn in the summer and shoveled their driveway in the winter. When I was old enough to get a regular job, I asked Mr. O’Reilly for a letter of recommendation for a job at the local newspaper. He declined, saying he wanted me to work at their drugstore. But he didn’t say anything else, until Mrs. O’Reilly told him to give me a job. So, I worked at the drugstore from the age of 15 to 20 years old. I even began college at the same school they had attended: the Massachusetts College of Pharmacy. Although I changed schools and switched my major to psychology, I was always hopeful that the O’Reilly’s would be proud of me. Unfortunately, Bill O’Reilly died a week after I graduated from college. Jackie O’Reilly retired and sold their building, and the old corner drugstore in the center of town ceased to exist.

Eventually I moved away, pursued my career in psychology, got married and had children, and visits to Massachusetts became few and far between. One night, about 25 years after I had worked at the drugstore, I had a very vivid and moving dream. I was standing in the center of Foxborough, looking at the building where the drugstore had been. I was overwhelmed by a profound sense of sadness, sad that things must change with time and cannot remain the same, no matter how much we may long for the past. I awoke from that dream astonished by its sense of reality and its emotional impact. The next morning my mother called me, and told me that Jackie O’Reilly had died during the night! Was it merely a coincidence that I dreamt about O’Reilly’s Pharmacy and felt the sadness of watching time pass away, while Jackie O’Reilly was in reality passing away, or was it something more? There can be no scientific explanation. I searched my mind for anything that might have coincidentally caused me to think about the drugstore the day before, but nothing came to mind except for an alternative explanation, which was not at all scientific. Had Jackie O’Reilly’s spirit passed by and said goodbye, on her way to the great beyond?

In the quote cited above, Jung wrote that he was amazed by how many people have had experiences of synchronicity. The questions I would pose to you are quite simple. Have you ever experienced synchronicity? If you have not, do you consider it possible that such events occur as something more than simple (if improbable) coincidence?

Before dismissing synchronicity as non-scientific, keep in mind the circumstances that led Jung to this theory. In addition to personally knowing Wolfgang Pauli, Jung also knew Nils Bohr and Albert Einstein (both of whom, like Pauli, had won a Nobel Prize in physics). Although these men are considered among the greatest scientists of modern times, Einstein perhaps the greatest, consider some of their theories. For instance, Einstein proposed that time isn’t time, it’s relative, except for the speed of light, which alone is always constant. In recent years, experimental physicists have exceeded the speed of light, broken Heisenberg’s Uncertainty Principle (which, by definition, couldn’t be broken), and proposed that it might be possible to get something colder than absolute zero. How can we accept things that cannot be observed or proved as scientific, while rejecting something that Jung and many others have observed time and time again? Jung was impressed by the possibility of splitting atoms, and wondered if such a thing might be possible with the psyche. As physics

suggested strange new possibilities, Jung held out the same hope for humanity (Progoff, 1973).

Regardless of whether the strangest of Jung's theories are ever proven right or wrong, at the very least they provide an opportunity for interesting discussions! There also happens to be another well-known person in the history of psychology who has experienced synchronicity and who talked about many of her patients having had out-of-body and near-death experiences: Elisabeth Kubler-Ross. In her book On Children and Death (Kubler-Ross, 1983), Kubler-Ross describes even more serious concerns than Jung about discussing this topic, but as with Jung, she has also met many, many patients who have had these experiences:

…I have been called every possible name, from Antichrist to Satan himself; I have been labeled, reviled, and otherwise denounced…But it is impossible to ignore the thousands of stories that patients - children and adults alike - have shared with me. These illuminations cannot be explained in scientific language. Listening to these experiences and sharing many of them myself, it would seem hypocritical and dishonest to me not to mention them in my lectures and workshops. So I have shared all of what I have learned from my patients for the last two decades, and I intend to continue to do so. (pg. 106; Kubler-Ross, 1983)

Discussion Question: Jung studied and wrote about topics as diverse as alchemy, astrology, flying saucers, ESP, the prophecies of Nostradamus, and synchronicity. Does this make it difficult for you to believe any of his theories? If you don't believe anything about any of these topics, are you still able to find value in other theories proposed by Jung?

Review of Key Points

• As a child, Jung was introduced to a wide variety of cultural and religious perspectives from around the world. As a result of these experiences, he was open to many different perspectives throughout his career.

• Jung had extremely vivid dreams, many of which he interpreted as visions (or unconscious communications) intended to guide his actions.

• Jung called his approach "analytical psychology" in order to distinguish it from Freud's "psychoanalysis" and Adler's "individual psychology."

• An important starting point for Jung's theories was the concept of entropy, which proposes an eventual balance of all energy. Jung applied this concept to the psychic energy present in the conscious and unconscious psyches.

• Jung proposed two distinct realms within the unconscious psyche, the personal and the collective.

• According to Jung, the personal unconscious is revealed through its complexes.

• Jung advanced the Word Association Test as a means of examining the complexes contained within the personal unconscious.

• The collective unconscious communicates through archetypal images. Jung believed the most readily observed archetypes are the shadow, the anima, and the animus.

• Another important archetype is the self, the representation of wholeness and the completed development of the personality. The self is often symbolically represented by mandalas.

• Jung developed a framework for recognizing particular personality types. He proposed two attitudes, introversion and extraversion, and four functions, thinking, feeling, sensing, and intuition.

• Jung’s type theory provided the basis for some practical personality tests. The Myers-Briggs Type Indicator and the Keirsey Temperament Sorter are both well-known in the field of psychology.

• Jung believed that everyone’s ultimate goal is to fully develop the potential of their personality. Jung called this process individuation.

• Development during the first half of life involves the natural aims of survival and procreation. The second half of life offers the opportunity to seek cultural development and the fulfillment of one’s self.

• Jungian analysis follows a basic series of stages, involving confession, elucidation, education, and transformation. However, Jung suggested it was better to avoid being locked into a rigid procedure. As a result, he utilized many different techniques, based on each individual patient.

• As Freud had before him, Jung developed a grand vision of how analytical psychology might help society as a whole. One unique proposition was that the Western world had much to learn from Eastern cultures.

• Jung’s interest in topics such as alchemy and extrasensory perception did not sit well with colleagues seeking to establish psychology as a scientific discipline. This opposition to Jung remains quite strong today, though Western psychology is broadening its perspective.

CHAPTER 08 / CAC.902

Person-Centered Therapy: Rogers and Maslow

Carl Rogers developed person-centered therapy around a radical proposition: the client has a formative tendency toward growth, and the therapist's task is to provide conditions in which that tendency can operate. The core conditions are congruence, unconditional positive regard, and empathic understanding. Rogers did not see these as polite extras. He argued that they are central to therapeutic change. Abraham Maslow, working in a related humanistic tradition, studied self-actualization, peak experiences, and human motivation.

The core conditions in practice

Congruence means the therapist is real and transparent within professional limits. Unconditional positive regard means the therapist values the client without making worth conditional on behavior. Empathy means the therapist accurately senses the client's inner frame of reference and communicates that understanding. In practice, these conditions are not passive. The therapist must listen carefully, check understanding, and remain present when the client's experience is confusing or painful.

A child-centered application might look like play therapy in which the counselor follows the child's lead, reflects feeling, and sets limits only when needed for safety. The counselor does not direct the play toward a predetermined lesson. The child uses play to express what cannot yet be said. A counselor might say, "That tower keeps falling, and you look frustrated." The reflection is not a technique to produce insight. It is a way of being with the child's experience.

Conditions of worth and the self-concept

Rogers argued that children need positive regard. When regard is conditional, children may develop conditions of worth: "I am lovable only if I am quiet," or "I am acceptable only if I achieve." The self-concept then becomes organized around approval rather than organismic experience. Incongruence arises between the self-concept and lived experience. A teen who believes "I never get angry" may experience rage as threatening and distort it. Therapy offers a relationship in which the experience can be admitted without losing love.

Maslow and motivation

Maslow's hierarchy of needs is widely known but often oversimplified. The idea that lower needs must be fully satisfied before higher needs appear is not a rigid law. People seek meaning, love, and creativity even in hardship. Still, the hierarchy reminds counselors that safety, food, sleep, and belonging matter. A child cannot use a cognitive worksheet effectively when hungry or terrified. Doctoral assessment includes basic needs and social context.

Self-actualization refers to the realization of potential. Maslow studied people he considered psychologically healthy and described qualities such as acceptance, spontaneity, problem-centering, and peak experiences. Critics note that his sample was selective and culturally shaped. The counselor should use the idea as an invitation to growth, not as a standard to judge clients.

Nondirectivity and its limits

Person-centered therapy is often called nondirective. This does not mean the therapist has no structure. The frame, confidentiality, and limits are structured. Nondirectivity means the therapist does not impose an agenda on the client's experience. With young children, some direction may be needed for safety. With adolescents, a purely nondirective stance may be experienced as indifference if the teen wants help with a concrete problem. The counselor can remain person-centered while offering a rationale and collaborating on tasks.

Evidence and application

The therapeutic relationship is one of the most consistently supported factors in psychotherapy outcome. Person-centered therapy has evidence for various concerns, and its core conditions influence many other approaches. However, for some specific disorders, structured treatments such as exposure-based CBT have stronger symptom-specific evidence. A counselor can honor Rogers while using structured methods. The question is whether the method is offered within a relationship of respect and collaboration.

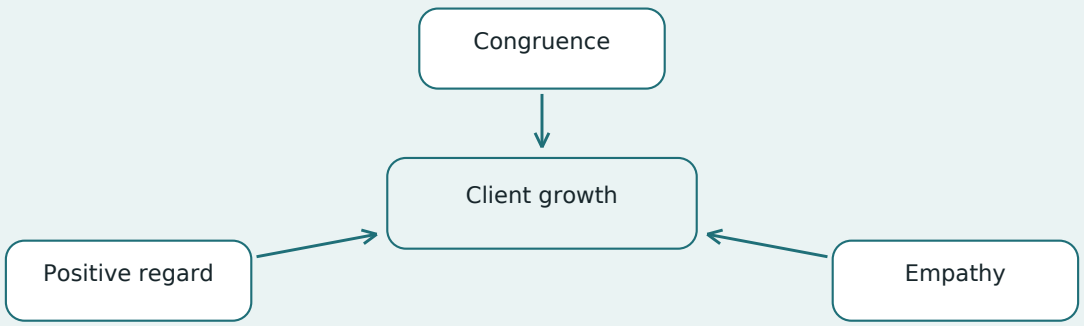
Clinical example

Eleven-year-old Sam is referred for selective mutism. A directive plan that pressures speech may increase anxiety. A person-centered counselor builds safety through play, accepts nonverbal communication, and does not make speech a condition of regard. At the same time, the counselor may collaborate with a behavioral plan for gradual communication. The person-centered condition protects the child's dignity; the behavioral plan provides steps. The child is not a project to be fixed. The child is a person whose voice will emerge when safety and skill meet.

Reflection

Think of a relationship in which you felt accepted even when you failed. What did that acceptance make possible? Then think of a relationship in which you felt valued only when you performed. How did your behavior change? Clients bring these histories into the room. The counselor's task is to offer a different condition.

Core conditions for growth



Person-centered therapy treats the relationship conditions as central, not decorative.

Attributed Foundational Reading

Attribution and critical-reading note

Adapted from Personality Theory in a Cultural Context by Mark D. Kelland, OpenStax CNX / Lansing Community College (2015), CC BY 4.0. Selection, organization, and PDF layout modified for this course; new lectures and course diagrams are separate additions.

Read historical and theoretical claims critically. A theory is a lens for inquiry, not a verdict about a child or family. Where source claims are dated or culture-bound, the new lecture supplies cautions and contemporary framing.

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In contrast to both the often dark, subconscious emphasis of the psychodynamic theorists and the somewhat cold, calculated perspectives of behavioral/cognitive theorists, the humanistic psychologists focus on each individual's potential for personal growth and self-actualization. Carl Rogers was influenced by strong religious experiences (both in America and in China) and his early clinical career in a children's hospital. Consequently, he developed his therapeutic techniques and the accompanying theory in accordance with a positive and hopeful perspective. Rogers also focused on the unique characteristics and viewpoint of individuals.

Abraham Maslow is best known for his extensive studies on the most salient feature of the humanistic perspective: self-actualization. He is also the one who referred to humanistic psychology as the third force, after the psychodynamic and behavioral/cognitive perspectives, and he specifically addressed the need for psychology to move beyond its study of unhealthy individuals. He was also interested in the psychology of the work place, and his recognition in the business field has perhaps made him the most famous psychologist.

Henry Murray was an enigmatic figure, who seemingly failed to properly acknowledge the woman who inspired much of his work, and who believed his life had been something of a failure. Perhaps he felt remorse as a result of maintaining an extramarital affair with the aforementioned woman, thanks in large part to the advice and help of Carl Jung! Murray extended a primarily psychodynamic perspective to the study of human needs in normal individuals. His Thematic Apperception Test was one of the first psychological tests applied outside of a therapeutic setting, and it provided the basis for studying the need for achievement (something akin to a learned form of self-actualization).

Carl Rogers and Humanistic Psychology

Carl Rogers is the psychologist many people associate first with humanistic psychology, but he did not establish the field in the way that Freud established psychoanalysis. A few years older than Abraham Maslow, and having moved into clinical practice more directly, Rogers felt a need to develop a new theoretical perspective that fit with his clinical observations and personal beliefs. Thus, he was proposing a humanistic approach to psychology and, more specifically, psychotherapy before Maslow. It was Maslow, however, who used the term humanistic psychology as a direct contrast to behaviorism and psychoanalysis. And it was Maslow who contacted some friends, in 1954, in order to begin meetings that led to the

creation of the American Association for Humanistic Psychology. Rogers was included in that group, but so were Erich Fromm and Karen Horney, both of whom had distinctly humanistic elements in their own theories, elements that shared a common connection to Alfred Adler's Individual Psychology (Stagner, 1988). In addition, the spiritual aspects of humanistic psychology, such as peak experiences and transcendence, have roots in the work of Carl Jung and William James, and go even further back in time to ancient philosophies of Yoga and Buddhism.

In at least one important way, Rogers' career was similar to that of Sigmund Freud. As he began his clinical career, he found that the techniques he had been taught were not very effective. So, he began experimenting with his own ideas, and developing his own therapeutic approach. As that approach developed, so did a unique theory of personality that aimed at explaining the effectiveness of the therapy. Rogers found it difficult to explain what he had learned, but he felt quite passionately about it:

…the real meaning of a word can never be expressed in words, because the real meaning would be the thing itself. If one wishes to give such a real meaning he should put his hand over his mouth and point. This is what I should most like to do. I would willingly throw away all the words of this manuscript if I could, somehow, effectively point to the experience which is therapy. It is a process, a thing-in-itself, an experience, a relationship, a dynamic… (pp. ix; Rogers, 1951)

Brief Biography of Carl Rogers

Carl Ransom Rogers was born on January 8, 1902, in Chicago, Illinois. His parents were well-educated, and his father was a successful civil engineer. His parents loved their six children, of whom Rogers was the fourth, but they exerted a distinct control over them. They were fundamentalist Christians, who emphasized a close-knit family and constant, productive work, but approved of little else. The Rogers household expected standards of behavior appropriate for the 'elect' of God: there was no drinking of alcohol, no dancing, no visits to the theater, no card games, and little social life at all (DeCarvalho, 1991; Thorne, 2003).

Rogers was not the healthiest of children, and his family considered him to be overly sensitive. The more his family teased him, the more he retreated into a lonely world of fantasy. He sought consolation by reading books, and he was well above his grade level for reading when he began school. In 1914 the family moved to a large farm west of Chicago, a move motivated primarily by a desire to keep the children away from the temptations of suburban city life. The result was even more isolation for Rogers, who lamented that he'd only had two dates by the end of high school. He continued to learn, however, becoming something of an expert on the large moths that lived in the area. In addition, his father encouraged the children to develop their own ventures, and Rogers and his brothers raised a variety of livestock. Given these interests, and in keeping with family tradition, Rogers enrolled in the University of Wisconsin-Madison to study scientific agriculture (DeCarvalho, 1991; Thorne, 2003).

During his first year of college, Rogers attended a Sunday morning group of students led by Professor George Humphrey. Professor Humphrey was a facilitative leader, who refused to be conventional and who encouraged the students to make their own decisions. Rogers found the intellectual freedom very stimulating, and he also began to make close friends.

This increased intellectual and emotional energy led Rogers to re-examine his commitment to Christianity. Given his strong religious faith, he decided to change his major to history, in anticipation of a career as a Christian minister. He was fortunate to be chosen as one of only twelve students from America to attend a World Student Christian Federation conference in Peking, China. He traveled throughout China (also visiting Korea, Hong Kong, Japan, the Philippines, and Hawaii) for 6 months, surrounded by other intelligent and creative young people. He kept a detailed journal, and wrote lengthy letters to his family and Helen Elliott, a childhood friend whom he considered to be his “sweetheart.” His mind was stretched in all directions by this profound cross-cultural experience, and the intellectual and spiritual freedom he was embracing blinded him to the fact that his fundamentalist family was deeply disturbed by what he had to say. However, by the time Rogers was aware of his family’s disapproval, he had been changed, and he believed that people of very different cultures and faiths can all be sincere and honest (Kirschenbaum, 1995; Thorne, 2003). As a curious side note, Rogers’ roommate on the trip was a Black seminary professor. Rogers was vaguely aware that it was strange at that time for a Black man and a White man to room together, but he was particularly surprised at the stares they received from the Chinese people they met, who had never seen a Black person before (Rogers & Russell, 2002). After his return from China, Rogers graduated from college, and 2 months later he married Helen. Again his family disapproved, believing that the young couple should be more established first. But Rogers had been accepted to the Union Theological Seminary in New York City, and both he and Helen wanted to be together. His family may have wanted them to wait because Union Theological Seminary was, perhaps, the most liberal seminary in America at the time (DeCarvalho, 1991; Rogers & Russell, 2002; Thorne, 2003).

Rogers spent 2 years at the seminary, including a summer assignment as the pastor of a small church in Vermont. However, his desire not to impose his own beliefs on others, made it difficult for him to preach. He began taking courses at nearby Teachers’ College of Columbia University, where he learned about clinical and educational psychology, as well as working with disturbed children. He then transferred to Teachers’ College, and after writing a dissertation in which he developed a test for measuring personality adjustment in children, he earned his Ph.D. in Clinical Psychology. Then, in 1928, he began working at the Rochester Society for the Prevention of Cruelty to Children (DeCarvalho, 1991; Thorne, 2003).

Rogers was immersed in his work in Rochester for 12 years. He found that even the most elaborate theories made little sense when dealing with children who had suffered severe psychological damage after traveling through the courts and the social work systems. So Rogers developed his own approach, and did his best to help them. Many of his colleagues, including the director, had no particular therapeutic orientation:

When I would try to see what I could do to alter their behavior, sometimes they would refuse to see me the next time. I’d have a hard time getting them to come from the detention home to my office, and that would cause me to think, “What is it that I did that offended the child?” Well, usually it was overinterpretation, or getting too smart in analyzing the causes of behavior…So we approached every situation with much more of a question of “What can we do to help?” rather than “What is the mysterious cause of this behavior?” or “What theory does the child fit into?” It was a very good place for learning in that it was easy to be open to experience, and there was certainly no pressure to fit into any particular pattern of thought. (pg. 108; Rogers & Russell, 2002)

Eventually Rogers wrote a book outlining his work with children, The Clinical Treatment of the Problem Child (Rogers, 1939), which received excellent reviews. He was offered a professorship at Ohio State University. Beginning as a full professor gave Rogers a great deal of freedom, and he was frequently invited to give talks. It has been suggested that one such talk, in December 1940, at the University of Minnesota, entitled “Newer Concepts in Psychotherapy,” was the official birthday of client-centered therapy. Very popular with his students, Rogers was not so welcome amongst his colleagues. Rogers believed that his work was particularly threatening to those colleagues who believed that only their own expertise could make psychotherapy effective. After only 4 years, during which he published Counseling and Psychotherapy (Rogers, 1942), Rogers moved on to the University of Chicago, where he established the counseling center, wrote Client-Centered Therapy (Rogers, 1951) and contributed several chapters to Psychotherapy and Personality Change (Rogers & Dymond, 1954), and in 1956 received a Distinguished Scientific Contribution Award from the American Psychological Association. Then, in 1957, he accepted a joint appointment in psychiatry and psychology at the University of Wisconsin to study psychotic individuals. Rogers had serious doubts about leaving Chicago, but felt that the joint appointment would allow him to make a dramatic contribution to psychotherapy. It was a serious mistake. He did not get along with his colleagues in the psychology department, whom he considered to be antagonistic, outdated, “rat-oriented,” and distrustful of clinical psychology, and so he resigned. He kept his appointment in the psychiatry department, however, and in 1961 published perhaps his most influential book, On Becoming a Person (Rogers, 1961).

In 1963, Rogers moved to California to join the Western Behavioral Sciences Institute, at the invitation of one of his former students, Richard Farson. This was a non-profit institute dedicated to the study of humanistically-oriented interpersonal relations. Rogers was leery of making another major move, but eventually agreed. He became very active in research on encounter groups and educational theory. Five years later, when Farson left the institute, there was a change in its direction. Rogers was unhappy with the changes, so he joined some colleagues in leaving and establishing the Center for Studies of the Person, where he remained until his death. In his later years, Rogers wrote books on topics such as personal power and marriage (Rogers, 1972, 1977). In 1980, he published A Way of Being (Rogers, 1980), in which he changed the terminology of his perspective from “client-centered” to “person-centered.” With the assistance of his daughter Natalie, who had studied with Abraham Maslow, he held many group workshops on life, family, business, education, and world peace. He traveled to regions where tension and danger were high, including Poland, Russia, South Africa, and Northern Ireland. In 1985 he brought together influential leaders of seventeen Central American countries for a peace conference in Austria. The day he died, February 4, 1987, without knowing it, he had just been nominated for the Nobel Peace Prize (DeCarvalho, 1991; Kirschenbaum, 1995; Thorne, 2003).

Placing Rogers in Context: A Psychology 2,600 Years in the Making

Carl Rogers was an extraordinary individual whose approach to psychology emphasized individuality. Raised with a strong Christian faith, exposed to Eastern culture and spirituality in college, and then employed as a therapist for children, he came to value and respect each person he met. Because of that respect for the ability of each person to grow, and the belief that we are innately driven toward actualization, Rogers began the distinctly humanistic approach to psychotherapy that became known as client-centered therapy.

Taken together, client-centered therapy and self-actualization offer a far more positive approach to fostering the growth of each person than most other disciplines in psychology. Unlike the existing approaches of psychoanalysis, which aimed to uncover problems from the past, or behavior therapies, which aimed to identify problem behaviors and control or "fix" them, client-centered therapy grew out of Rogers' simple desire to help his clients move forward in their lives. Indeed, he had been trained as a psychoanalyst, but Rogers found the techniques unsatisfying, both in their goals and their ability to help the children he was working with at the time. The seemingly hands-off approach of client-centered therapy fit well with a Taoist perspective, something Rogers had studied, discussed, and debated during his trip to China. In A Way of Being, Rogers (1980) quotes what he says is perhaps his favorite saying, one which sums up many of his deeper beliefs:

If I keep from meddling with people, they take care of themselves,

If I keep from commanding people, they behave themselves,

If I keep from preaching at people, they improve themselves,

If I keep from imposing on people, they become themselves.

Lao Tsu, c600 B.C.; Note: This translation differs somewhat from the one

cited in the References. I have included the translation Rogers quoted,

since the difference likely influenced his impression of this saying.

Rogers, like Maslow, wanted to see psychology contribute far more to society than merely helping individuals with psychological distress. He extended his sincere desire to help people learn to really communicate, with empathic understanding, to efforts aimed at bringing peace to the world. On the day he died, he had just been nominated for the Nobel Peace Prize. Since a Nobel Prize cannot be awarded to someone who has died, he was not eligible to be nominated again. If he had lived a few more years, he may well have received that award. His later years were certainly committed to peace in a way that deserved such recognition.

Basic Concepts

Rogers believed that each of us lives in a constantly changing private world, which he called the **experiential field**. Everyone exists at the center of their own experiential field, and that field can only be fully understood from the perspective of the individual. This concept has a number of important implications. The individual's behavior must be understood as a reaction to their experience and perception of the field. They react to it as an organized whole, and it is their reality. The problem this presents for the therapist is that only the individual can really understand their experiential field. This is quite different than the Freudian perspective, in which only the trained and objective psychoanalyst can break through the defense mechanisms and understand the basis of the patient's unconscious impulses. One's perception of the experiential field is limited, however. Rogers believed that certain impulses, or sensations, can only enter into the conscious field of experience under certain circumstances. Thus, the experiential field is not a true reality, but rather an individual's potential reality (Rogers, 1951).

The one basic tendency and striving of the individual is to actualize, maintain, and enhance the experiencing of the individual or, in other words, an **actualizing tendency**. Rogers

borrowed the term **self-actualization**, a term first used by Kurt Goldstein, to describe this basic striving.

The tendency of normal life is toward activity and progress. For the sick, the only form of self-actualization that remains is the maintenance of the existent state. That, however, is not the tendency of the normal…Under adequate conditions the normal organism seeks further activity. (pp. 162-163; Goldstein, 1934/1995).

For Rogers, self-actualization was a tendency to move forward, toward greater maturity and independence, or self-responsibility. This development occurs throughout life, both biologically (the differentiation of a fertilized egg into the many organ systems of the body) and psychologically (self-government, self-regulation, socialization, even to the point of choosing life goals). A key factor in understanding self-actualization is the experiential field. A person's needs are defined, as well as limited, by their own potential for experience. Part of this experiential field is an individual's emotions, feelings, and attitudes. Therefore, who the individual is, their actual **self**, is critical in determining the nature and course of their self-actualization (Rogers, 1951). We will examine Maslow's work on self-actualization in more detail below.

What then, is the self? In Rogers' (1951) initial description of his theory of personality, the experiential field is described in four points, the self-actualizing tendency in three points, and the remaining eleven points attempt to define the self. First and foremost, the self is a differentiated portion of the experiential field. In other words, the self is that part of our private world that we identify as "me," "myself," or "I." Beyond that, the self remains somewhat puzzling. Can the self exist in isolation, outside of relationships that provide some context for the self? Must the self be synonymous with the physical body? As Rogers' pointed out, when our foot "goes to sleep" from a lack of circulation, we view it as an object, not as a part of our self! Despite these challenging questions, Rogers tried to define and describe the self.

Rogers believed the self is formed in relation to others; it is an organized, fluid, yet consistent conceptual pattern of our experiential interactions with the environment and the values attached to those experiences. These experiences are symbolized and incorporated into the structure of the self, and our behavior is guided largely by how well new experiences fit within that structure. We may behave in ways inconsistent with the structure of our self, but when we do we will not "own" that behavior. When experiences are so inconsistent that we cannot symbolize them, or fit them into the structure of our self, the potential for psychological distress arises. On the other hand, when our concept of self is mature enough to incorporate all of our perceptions and experiences, and we can assimilate those experiences symbolically into our self, our psychological adjustment will be quite healthy. Individuals who find it difficult to assimilate new and different experiences, those experiences that threaten the structure of the self, will develop an increasingly rigid **self-structure**. Healthy individuals, in contrast, will assimilate new experiences, their self-structure will change and continue to grow, and they will become more capable of understanding and accepting others as individuals (Rogers, 1951).

The ability of individuals to make the choices necessary for actualizing their self-structure and to then fulfill those choices is what Rogers called **personal power** (Rogers, 1977). He believed there are many self-actualized individuals revolutionizing the world by trusting their own power, without feeling a need to have "power over" others. They are also willing to

foster the latent actualizing tendency in others. We can easily see the influence of Alfred Adler here, both in terms of the creative power of the individual and seeking superiority within a healthy context of social interest. **Client-centered therapy** was based on making the context of personal power a clear strategy in the therapeutic relationship:

…the client-centered approach is a conscious renunciation and avoidance by the therapist of all control over, or decision-making for, the client. It is the facilitation of self-ownership by the client and the strategies by which this can be achieved…based on the premise that the human being is basically a trustworthy organism, capable of…making constructive choices as to the next steps in life, and acting on those choices. (pp. 14-15; Rogers, 1977)

Discussion Question: Rogers claimed that no one can really understand your experiential field. Would you agree, or do you sometimes find that close friends or family members seem to understand you better than you understand yourself? Are these relationships congruent?

Personality Development

Although Rogers described personality within the therapist-client relationship, the focus of his therapeutic approach was based on how he believed the person had arrived at a point in their life where they were suffering from psychological distress. Therefore, the same issues apply to personality development as in therapy. A very important aspect of personality development, according to Rogers, is the parent-child relationship. The nature of that relationship, and whether it fosters self-actualization or impedes personal growth, determines the nature of the individual's personality and, consequently, their self-structure and psychological adjustment.

A child begins life with an actualizing tendency. As they experience life, and perceive the world around them, they may be supported in all things by those who care for them, or they may only be supported under certain conditions (e.g., if their behavior complies with strict rules). As the child becomes self-aware, it develops a need for **positive regard**. When the parents offer the child **unconditional positive regard**, the child continues moving forward in concert with its actualizing tendency. So, when there is no discrepancy between the child's **self-regard** and its positive regard (from the parents), the child will grow up psychologically healthy and well-adjusted. However, if the parents offer only **conditional positive regard**, if they only support the child according the desires and rules of the parents, the child will develop **conditions of worth**. As a result of these conditions of worth, the child will begin to perceive their world selectively; they will avoid those experiences that do not fit with its goal of obtaining positive regard. The child will begin to live the life of those who set the conditions of worth, rather than living its own life.

As the child grows older, and more aware of its own condition in the world, their behavior will either fit within their own self-structure or not. If they have received unconditional positive regard, such that their self-regard and positive regard are closely matched, they will experience **congruence**. In other words, their sense of self and their experiences in life will fit together, and the child will be relatively happy and well-adjusted. But, if their sense of self and their ability to obtain positive regard do not match, the child will develop **incongruence**. Consider, for example, children playing sports. That alone tells us that parents have established guidelines within which the children are expected to "play." Then we have some children who are naturally athletic, and other children who are more awkward and/or clumsy. They may become quite athletic later in life, or not, but during childhood

there are many different levels of ability as they grow. If a parent expects their child to be the best player on the team, but the child simply isn't athletic, how does the parent react? Do they support the child and encourage them to have fun, or do they pressure the child to perform better and belittle them when they can't? Children are very good at recognizing who the better athletes are, and they know their place in the hierarchy of athletics, i.e., their athletic self-structure. So if a parent demands dominance from a child who knows they just aren't that good, the child will develop incongruence. Rogers believed, quite understandably, that such conditions are threatening to a child, and will activate defense mechanisms. Over time, however, excessive or sudden and dramatic incongruence can lead to the breakdown and disorganization of the self-structure. As a result, the individual is likely to experience psychological distress that will continue throughout life (Rogers, 1959/1989).

Discussion Question: Conditions of worth are typically first established in childhood, based on the relationship between a child and his or her parents. Think about your relationship with your own parents and, if you have children, think about how you treat them. Are most of the examples that come to mind unconditional positive regard, or conditional positive regard? How has that affected your relationship with your parents and/or your own children?

Sometimes parents create conditions of worth by overemphasizing the importance of competition. Even a trip to the pumpkin patch can be a challenge to get the biggest, best pumpkin.

Another way in which Rogers approached the idea of congruence and incongruence was based on an individual's dual concept of self. There is, of course, the actual self-structure, or **real self**. In addition, there is also an **ideal self**, much like the fictional finalism described by Adler or the idealized self-image described by Horney. Incongruence develops when the real self falls far short of the accomplishment expected of the ideal self, when experience does not match the expectations of the self-structure (Rogers, 1951, 1959/1989). Once again, the relationship between parents and their children plays an important role in this development. If parents expect too much, such as all A's every marking period in school, but the child just isn't academically talented, or if the parents expect their child to be the football team's quarterback, but the child isn't a good athlete, then the ideal self will remain out of reach. Perhaps even worse, is when a child is physically or emotionally abused. Such a child's ideal self may remain at a relatively low standard, but the real self may be so utterly depressed that incongruence is still the result. An important aspect of therapy will be to provide a relationship in which a person in this unfortunate condition can experience the unconditional positive regard necessary to begin reintegrating the self-structure, such that the gap between the real self and the ideal self can begin to close, allowing the person to experience congruence in their life.

What about individuals who have developed congruence, having received unconditional positive regard throughout development or having experienced successful client-centered therapy? They become, according to Rogers (1961), a **fully functioning person**. He also said they lead a good life. The good life is a process, not a state of being, and a direction, not a destination. It requires psychological freedom, and is the natural consequence of being psychologically free to begin with. Whether or not it develops naturally, thanks to a healthy and supportive environment in the home, or comes about as a result of successful therapy,

there are certain characteristics of this process. The fully functioning person is increasingly open to new experiences, they live fully in each moment, and they trust themselves more and more. They become more able and more willing to experience all of their feelings, they are creative, they trust human nature, and they experience the richness of life. The fully functioning person is not simply content, or happy, they are alive:

I believe it will become evident why, for me, adjectives such as happy, contented, blissful, enjoyable, do not seem quite appropriate to any general description of this process I have called the good life, even though the person in this process would experience each one of these feelings at appropriate times. But the adjectives which seem more generally fitting are adjectives such as enriching, exciting, rewarding, challenging, meaningful. This process…involves the courage to be. …the deeply exciting thing about human beings is that when the individual is inwardly free, he chooses as the good life this process of becoming. (pp. 195-196; Rogers, 1961)

Discussion Question: Rogers described self-actualized people as fully functioning persons who are living a good life. Do you know anyone who seems to be a fully functioning person? Are there aspects of their personality that you aspire to for yourself? Does it seem difficult to be fully functioning, or does it seem to make life both easier and more enjoyable?

Connections Across Cultures: Self-Realization as the

Path to Being a Fully Functioning Person

Rogers described an innate drive toward self-actualization, he talked about an ideal self, and he said that a fully functioning person lived a good life. But what does this actually mean? In the Western world we look for specific, tangible answers to such questions. We want to know what the self-actualization drive is, we want to know which ideals, or virtues, are best or right, and we want to define a “good life.” All too often, we define a good life in terms of money, power, and possessions. The Eastern world has, for thousands of years, emphasized a very different perspective. They believe there is a natural order to life, and it is important that we let go of our need to explain the universe, and it is especially important that we let go of our need to own pieces of the universe. In the Tao Te Ching, Lao Tsu (c. 600 B.C./1989) writes:

Something mysteriously formed,

Born before heaven and earth.

In the silence and the void,

Standing alone and unchanging,

Ever present and in motion.

Perhaps it is the mother of ten thousand things.

I do not know its name,

Call it Tao.

For lack of a better word, I call it great…

The greatest Virtue is to follow Tao and Tao alone…

Tao follows what is natural.

At about the same time, some 2,600 years ago, the Bhagavad Gita was also written down (Mitchell, 2000). In the second chapter one finds:

When a man gives up all desires

That emerge from the mind, and rests

Contented in the Self by the Self,

He is called a man of firm wisdom…

In the night of all beings, the wise man

Sees only the radiance of the Self;

But the sense-world where all beings wake,

For him is as dark as night.

In each of these sacred books, we are taught that there is something deeper than ourselves that permeates the universe, but it is beyond our comprehension. It is only when we stop attempting to explain it, our way of trying to control it, and be content to just be ourselves, that we can actually attain that goal. To achieve this goal seems to require the absence of conditions of worth. If someone has been given unconditional positive regard throughout their life, they will be content to live that life as it is. Rogers was well aware of this challenge, and he described the good life as a process, not something that you could actually get, but something that you had to "Be." Still, is it possible that a fully functioning person might have the insight necessary to understand the essence of the universe? Not according to Swami Sri Yukteswar:

Man possesses eternal faith and believes intuitively in the existence of a Substance, of which the objects of sense - sound, touch, sight, taste, and smell, the component parts of this visible world - are but properties. As man identifies himself with his material body, composed of the aforesaid properties, he is able to comprehend by these imperfect organs these properties only, and not the Substance to which these properties belong. The eternal Father, God, the only Substance in the universe, is therefore not comprehensible by man of this material world, unless he becomes divine by lifting his self above this creation of Darkness or Maya. See Hebrews 11:1 and John 8:28.

"Now faith is the substance of things hoped for, the evidence of things not seen."

"Then said Jesus unto them, When ye have lifted up the son of man, then shall ye know that I am he."

Jnanavatar Swami Sri Yukteswar Giri, 1894/1990

So whether we believe in God, Tao, an eternal Self, a mortal Self, or merely an actualizing tendency, for thousands of years there has been the belief, amongst many people, that our lives are about more than just being alive for a limited period of time. And it is in the recognition and acceptance, indeed the embracing, of that something more, even if we can't conceive it in our conscious mind, that we find and live a good life. When Paramahansa Yogananda, a direct disciple of Swami Yukteswar, came to the United States in 1920 to establish a permanent Yoga society, it was suggested that he name his society God-Realization. However, since he believed life is about realizing (or actualizing, in

psychological terms) our selves, he established his organization as the Self-Realization Fellowship (Yogananda, 1946).

Self-realization, in the context of Yoga, refers to becoming aware of one's connection to the spark of divinity that exists within us, which may well be the source of our actualizing tendency. It is not the same as the sense of "I" or "me" that we normally think of. After all, are we our body or our mind? Consider the body. Is it the body we were born with, or the body we have now? Is our mind what we are thinking now, or what we were thinking 2 years ago? Both the body and the mind are transient, but the Self continues. It is that Self that Yogis, Buddhists, and Taoists seek to realize, and it may well be that Self which seeks its own actualization (separate from the consciousness created by the brain underlying our mind; see Feuerstein, 2003; Kabat-Zinn, 1994). This is also the Self of Being and transcendence, as described by Maslow.

Social Relationships and Marriage

Social and personal relationships were very important to Rogers, both in therapy and in everyday life. During each moment, we have our awareness (or consciousness), our experience (our perception of what is happening), and our communication (our relational behavior). For the fully functioning person, there is congruence between each of these phenomena. Unfortunately, we tend to be a poor judge of our own congruence. For example, if someone becomes angry with another person at a meeting or in a therapy group, they may remain unaware of their anger, even though it may be quite obvious to everyone else in the room. Thus, our relationship with others can reflect the true nature of our own personality, and the degree to which we are congruent. If others are congruent, and therefore are willing to talk to us openly and honestly, it will encourage us to become more congruent and, consequently, more psychologically healthy (Rogers, 1961, 1980). Curiously, the reason this became so important to Rogers was the lack of such meaningful relationships in his own life. Because his family followed strict, fundamentalist rules, they discouraged relationships with people outside their family. The consequences were rather disturbing for Rogers:

…the attitudes toward persons outside our large family can be summed up schematically in this way: "Other persons behave in dubious ways which we do not approve in our family. Many of them play cards, go to movies, smoke, dance, drink, and engage in other activities, some unmentionable. So the best thing to do is to be tolerant of them, since they may not know better, but to keep away from any close communication with them and to live your life within the family…"

I could sum up these boyhood years by saying that anything I would today regard as a close and communicative interpersonal relationship with another was completely lacking during that period…I was peculiar, a loner, with very little place or opportunity for a place in the world of persons. I was socially incompetent in any but superficial contacts. My fantasies during this period were definitely bizarre, and probably would be classed as schizoid by a diagnostician, but fortunately I never came in contact with a psychologist. (pp. 28-30; Rogers, 1980)

As noted above, the development of healthy relationships takes place whenever one person in the relationship is congruent. Their congruence encourages the other person to be more congruent, which supports the continued open communication on behalf of the first person. This interplay goes back and forth, encouraging continued and growing congruence in the

relationship. As we will see below, this is basically the therapeutic situation, in which the therapist is expected to be congruent. However, it certainly does not require a trained therapist, since it occurs naturally in any situation in which one person is congruent from the beginning of the relationship.

One of the most important, and hopefully meaningful, relationships in anyone's life is marriage. Rogers was married for 55 years, and as the end of his wife's life approached he poured out his love to her with a depth that astonished him (Rogers, 1980). As relationships became more and more meaningful to him, he wanted to study the extraordinary relationships that become more than temporary. Although this is not necessarily synonymous with marriage, it most typically is. So he conducted a series of informal interviews with people who were, or had been, in lengthy relationships (at least 3 years). In comparing the relationships that seemed successful, as compared to those that were unhappy or had already come to an end, Rogers identified four factors that he believed were most important for long-term, healthy relationships: dedication or commitment, communication, the dissolution of roles, and becoming a separate self (Rogers, 1972).

Dedication, Commitment: Marriage is challenging: love seems to fade, vows are forgotten or set aside, religious rules are ignored (e.g., "What therefore God has joined together, let no man put asunder."; Matthew 19:6; Holy Bible, 1962). Rogers believed that in order for a relationship to last, each person must be dedicated to their partnership. They must commit themselves to working together throughout the changing process of their relationship, which is enriching their love and their life.

Communication: Communication encompasses much of human behavior, and it can be both subtle and complex. Communication itself is not a good thing, since many negative and hurtful things can be communicated. However, Rogers believed that we need to communicate persistent feeling, whether positive or negative, so that they don't overwhelm us and come out in inappropriate ways. It is always important to express such communication in terms of your own thoughts and feelings, rather than projecting those feelings onto others (especially in angry and/or accusatory ways). This process involves risk, but one must be willing to risk the end of a relationship in order to allow it to grow.

Dissolution of Roles: Culture provides many expectations for the nature of relationships, whether it be dating or something more permanent like marriage. According to Rogers, obeying the cultural rules seems to contradict the idea of a growing and maturing relationship, a relationship that is moving forward (toward actualization). However, when individuals make an intentional choice to fulfill cultural expectations, because they want to, then the relationship can certainly be actualizing for them.

Becoming a Separate Self: Rogers believed that "a living partnership is composed of two people, each of whom owns, respect, and develops his or her own selfhood" (pg. 206; Rogers, 1972). While it may seem contradictory that becoming an individual should enhance a relationship, as each person becomes more real and more open they can bring these qualities into the relationship. As a result, the relationship can contribute to the continued growth of each person.

Discussion Question: Consider Rogers' criteria for a successful marriage, which begins with commitment to the marriage. Given the divorce rate (which studies now place at over

60%), and ongoing political debates about what marriage is or is not, what is your opinion of the status of marriage in society today?

Client-Centered and Person-Centered Therapy

Central to Rogers' view of psychotherapy is the relationship between the therapist and the client, and we must again emphasize the distinction between a client and a patient. This involves shifting the emphasis in therapy from a psychologist/psychiatrist who can "fix" the patient to the client themselves, since only the client can truly understand their own experiential field. The therapist must provide a warm, safe environment in which the client feels free to express whatever attitude they experience in the same way that they perceive it. At the same time, the client experiences the therapist as someone temporarily divested of their own self, in their complete desire to understand the client. The therapist can then accurately and objectively reflect the thoughts, feelings, perceptions, confusions, ambivalences, etc., of the client back to the client. In this open, congruent, and supportive environment, the client is able to begin the process of reorganizing and reintegrating their self-structure, and living congruently within that self-structure (Rogers, 1951).

In 1957, Rogers published an article entitled The Necessary and Sufficient Conditions of Therapeutic Personality Change (Rogers, 1957/1989). The list is fairly short and straightforward:

1. The client and the therapist must be in psychological contact.

1. The client must be in a state of incongruence, being vulnerable or anxious.

1. The therapist must be congruent in the relationship.

1. The therapist must experience unconditional positive regard for the client.

1. The therapist must experience empathic understanding of the client's frame of reference and endeavor to communicate this experience to the client.

1. The client must perceive, at least to a minimal degree, the therapist's empathic understanding and unconditional positive regard.

According to Rogers, there is nothing else that is required; if these conditions are met over a period of time, there will be constructive personality change. What Rogers considered more remarkable are those factors that do not seem necessary for positive therapeutic change. For example, these conditions do not apply to one type of client, but to all clients, and they are not unique to client-centered therapy, but apply in all types of therapy. The relationship between the therapist and client is also not unique, these factors hold true in any interpersonal relationship. And most surprisingly, these conditions do not require any special training on the part of therapist, or even an accurate diagnosis of the client's psychological problems! Any program designed for the purpose of encouraging constructive change in the personality structure and behavior of individuals, whether educational, military, correctional, or industrial, can benefit from these conditions and use them as a measure of the effectiveness of the program (Rogers, 1957).

Can any one of these conditions be considered more important than the others? Although they are all necessary, Rogers came to believe that the critical factor may be the therapist's **empathic understanding** of the client (Rogers, 1980). The Dalai Lama (2001) has said that empathy is an essential first step toward a compassionate heart. It brings us closer to

others, and allows us to recognize the depth of their pain. According to Rogers, empathy refers to entering the private world of the client, and moving about within it without making any judgments. It is essential to set aside one's own views and values, so that the other person's world may be entered without prejudice. Not just anyone can accomplish this successfully:

In some sense it means that you lay aside your self; this can only be done by persons who are secure enough in themselves that they know they will not get lost in what may turn out to be the strange or bizarre world of the other, and that they can comfortably return to their own world when they wish. (pg. 143; Rogers, 1980)

Finally, let us consider group therapy situations. Within a group, all of the factors described above hold true. Rogers, who late in his career was becoming more and more interested in the growth of all people, including those reasonably well-adjusted and mature to begin with, became particularly interested in **T-groups** and **encounter groups**. These groups were developed following the proposition by Kurt Lewin that modern society was overlooking the importance of training in human relations skills (the "T" in T-group stands for "training"). Encounter groups were quite similar to T-groups, except that there was a greater emphasis on personal growth and improved interpersonal communication through an experiential process. Each group has a leader, or facilitator, who fosters and encourages open communication. The group serves as a reflection of the congruence, or lack thereof, in the communication of whoever is currently expressing themselves. As a result, the group hopefully moves toward congruence, and the subsequent personal growth and actualization of the each individual (Rogers, 1970).

Given the usefulness of T-groups and encounter in a variety of settings, as well as the importance of continued personal growth and actualization for the well-adjusted as well as those suffering psychological distress, Rogers shifted his focus from simply client-centered therapy to a more universal **person-centered approach**, which encompasses client-centered therapy, student-centered teaching, and group-centered leadership (Rogers, 1980; see also Rogers & Roethlisberger, 1952/1993). Rogers believed that all people have within them vast resources for self-understanding and for changing their self-concepts, attitudes, and behaviors. In all relationships, whether therapist-client, parent-child, teacher-student, leader-group, employer-employee, etc., there are three elements that can foster personal growth: genuineness or congruence, acceptance or caring, and empathic understanding. When these elements are fostered in any setting, "there is greater freedom to be the true, whole person." The implications go far beyond individual relationships. We live in what seems to be an increasingly dangerous world. Globalism has brought with it global tension and conflict. However, Rogers argued that a person-centered approach would help to ease intercultural tension, by helping each of us to learn to appreciate and understand others. Whether the cultural differences are political, racial, ethnic, economic, whatever, as more leaders become person-centered there is the possibility for future growth of intercultural understanding and cooperation (Rogers, 1977).

Abraham Maslow and Holistic-Dynamic Psychology

Maslow stands alongside Rogers as one of the founders of humanistic psychology. Although he began his career working with two of the most famous experimental psychologists in America, he was profoundly influenced by the events that led into World War II. He became devoted to studying the more virtuous aspects of personality, and he may be viewed as one

of the founders of positive psychology. Well-known primarily for his work on self-actualization, Maslow also had a significant impact on the field of management. His fame in both psychology and business makes him a candidate for being, perhaps, the best-known psychologist of all time (Freud is certainly more famous, but remember that he was a psychiatrist). According to Maslow, his **holistic-dynamic theory** of personality was a blend of theories that had come before his:

This theory is, I think, in the functionalist tradition of James and Dewey, and is fused with the holism of Wertheimer, Goldstein, and Gestalt psychology, and with the dynamicism of Freud, Fromm, Horney, Reich, Jung, and Adler. This integration or synthesis may be called a holistic-dynamic theory. (pg. 35; Maslow, 1970)

Brief Biography of Abraham Maslow

Abraham H. Maslow was born on April 1, 1908 in Brooklyn, New York, the first of seven children. His father, Samuel, had left Kiev, Russia at just 14 years old. When Samuel Maslow arrived in America he had no money and did not speak English. Samuel Maslow spent a few years in Philadelphia, doing odd jobs and learning the language, before moving to New York City, where he married his first cousin Rose and began a cooperage business (a cooper builds and repairs barrels). Samuel and Rose Maslow did not have a happy marriage, and Abraham Maslow was particularly sensitive to this fact. Maslow resented his father's frequent absences, and apparently hated his mother. His mother was a superstitious woman, who severely punished Maslow for even minor misbehavior by threatening him with God's wrath. Maslow developed an intense distrust of religion, and was proud to consider himself an atheist (Gabor, 2000; Hoffman, 1988; Maddi & Costa, 1972).

Maslow's childhood was no better outside the home. Anti-Semitism was rampant in New York. Many teachers were cruel, and he overheard them say nasty things about him. He had no friends, and there were anti-Semitic gangs that would find and beat up Jewish children. At one point he decided to join a Jewish gang for protection, but he didn't have the "right" attitude:

I wanted to be a member of the gang, but I couldn't: they rejected me because I couldn't kill cats…We'd stake out a cat on a [clothesline] and stand back so many paces and throw rocks at it and kill it.

And the other thing was to throw rocks at the girls on the corner. Now I knew that the girls liked it, and yet I couldn't throw rocks at girls and I couldn't kill cats, so I was ruled out of the gang, and I could never be the gangster that I wanted to become. (pg. 4; Maslow, cited in Hoffman, 1988)

With six more children joining the family, one every couple of years, the family was constantly moving and, following the troubling death of one of his little sisters (Maslow blamed her illness, in part, on their mother's neglect), Maslow became a very unhappy and shy child. He also thought he was terribly ugly, something his father said openly at a large family gathering! Perhaps worst of all, he felt profoundly strange and different than other children, largely because he was so intellectual. Maslow reconciled with his father later in life. During the depression, Samuel Maslow lost his business. By that time he had divorced Maslow's mother, Rose, and he moved in with his son. The two became close, and after Samuel Maslow died, his son remembered him fondly. Maslow never forgave his mother, however. Some of the childhood stories he related were shockingly cruel. Once, he had

searched through second-hand record shops for some special 78-RPM records. When he failed to put them away soon after returning home, his mother stomped them into pieces on the living room floor. Another time, Maslow brought home two abandoned kittens he had found. When his mother caught him feeding them a saucer of milk, she grabbed the kittens and smashed their heads against a wall until they were dead! Later in life, he refused to even attend her funeral.

What I had reacted to and totally hated and rejected was not only her physical appearance, but also her values and world view…I've always wondered where my utopianism, ethical stress, humanism, stress on kindness, love, friendship, and all the rest came from. I knew certainly of the direct consequences of having no mother-love. But the whole thrust of my life-philosophy and all my research and theorizing also has its roots in a hatred for and revulsion against everything she stood for. (pg. 9; Maslow cited in Hoffman, 1988)

Maslow spent much of his childhood reading, and despite the treatment he received from many of his prejudiced teachers, he loved to learn. After high school Maslow won a scholarship to Cornell University, but encountered pervasive anti-Semitism throughout his first year. So he transferred to City College, where he first studied the work of behavioral scientists like John B. Watson. He was impressed by Watson's desire to use the newly created science of behaviorism to fight social problems, such as racial and ethnic discrimination. At the same time, however, Maslow had fallen in love with his first cousin Bertha Goodman, a relationship his parents strongly opposed. So Maslow left for the University of Wisconsin (Gabor, 2000; Hoffman, 1988; Maddi & Costa, 1972). Bertha Goodman followed, and they were soon married. Marriage boosted Maslow's self-esteem, and provided him with a sense of purpose in life. He later said that "life didn't really start for me until I got married and went to Wisconsin" (pg. 128; cited in Maddi & Costa, 1972).

In Wisconsin, Maslow studied the behavior of primates under the supervision of the renowned Harry Harlow (most famous for his studies on **contact comfort**). One day, while watching some monkeys seemingly enjoy munching on peanuts and other treats, Maslow recognized that appetite and hunger are two different things. Thus, motivation must be comprised of separate elements as well. In another study, Maslow tried to address the different aspects of Freud and Adler's psychodynamic perspectives by observing dominance behavior amongst the monkeys. His colleagues and professors, however, had little interest in the psychoanalytic science that they considered to be a European endeavor. Maslow completed his Ph.D. at Wisconsin in 1934, and then returned to New York. He earned a position at Columbia University with the renowned Edward Thorndike, and began studying the relative contributions of heredity and environment on social behavior, as part of a project to study factors involved in poverty, illiteracy, and crime. As a curious side note, Thorndike had also developed an IQ test; Maslow scored 195 on this test, one of the highest scores ever recorded. During this time at Columbia University, Maslow also began relationships with many of the psychologists, sociologists, and anthropologists who had fled Nazi Germany. He was very impressed with Max Wertheimer, one of the founders of Gestalt psychology, and who helped to lay the foundation for positive psychology:

"Are there not tendencies in men and in children to be kind, to deal sincerely [and] justly with the other fellow? Are these nothing but internalized rules on the basis of compulsion and fear?" he asked rhetorically. (pg. 159; Wertheimer, cited in Gabor, 2000)

Maslow was one of the first students to study with Alfred Adler in America, being particularly impressed with Adler's work helping academically-challenged children to succeed despite their low IQ scores. Maslow also studied with Erich Fromm, Karen Horney, and Ruth Benedict. Benedict was an anthropologist who encouraged Maslow to gain some field experience. She sponsored a grant application that Maslow received to study the Blackfoot Indians. During the summer of 1938, Maslow examined the dominance and emotional security of the Blackfoot Indians. He was impressed by their culture, and recognized what he believed was an innate need to experience a sense of purpose in life, a sense of meaning. A few years later, shortly after the beginning of World War II, Maslow had an epiphany regarding psychology's failure to understand the true nature of people. He devoted the rest of his life to the study of a hopeful psychology (Gabor, 2000; Hoffman, 1988; Maddi & Costa, 1972).

Maslow taught for a few years at Brooklyn College, and also served as the plant manager for the Maslow Cooperage Corporation (from 1947-1949). In 1951 he was appointed Professor and Chair of the Department of Psychology at Brandeis University, where he conducted the research and wrote the books for which he is most famous. By the late 1960s, Maslow had become disillusioned with academic life. He had suffered a heart attack in 1966, and seemed somewhat disconnected from the very department he had helped to form. In 1969, however, he accepted a four year grant from the Laughlin Foundation, primarily to study the philosophy of democracy, economics, and ethics as influenced by humanistic psychology. He had been troubled by what he viewed as a loss of faith in American values, and he was greatly enjoying his time working in California. He also attended management seminars at the Saga Corporation, urging the participants to commit themselves to humanistic management. One day in June, 1970, he was jogging slowly when he suffered a massive heart attack. He was already dead by the time his wife rushed over to him (Gabor, 2000; Hoffman, 1988; Maddi & Costa, 1972). He was only 62 years old. Shortly after his death, the International Study Project of Menlo Park, CA published a memorial volume in tribute to Abraham Maslow (International Study Project, 1972).

Placing Maslow in Context: Beyond Humanistic Psychology

Whereas Carl Rogers is often thought of as the founder of humanistic psychology, in large part because of his emphasis on psychotherapy, it was Maslow who studied in great detail the most significant theoretical aspect of it: self-actualization. In addition to studying self-actualization, he applied it both in psychology and beyond. His application of self-actualization to management continued the classic relationship between psychology and business (which began with John B. Watson and his application of psychological principles to advertising). Unfortunately, Maslow died just as he was beginning to study his proposed fourth force: transpersonal psychology. Transpersonal psychology offered a connection between psychology and many of the Eastern philosophies associated with Yoga and Buddhism, and also provided a foundation for the study of positive psychology.

Maslow's interest in business and management has quite possibly led to his being the most famous psychologist of all time, since he is well-known in both psychology and business. If he had continued being a vocal advocate for transpersonal psychology (if not for his untimely death at an early age), given today's growing interest in Eastern philosophy and psychology and the establishment of positive psychology as a goal for the field of psychology by former APA President Martin Seligman, Maslow may well have become even more famous. It is

interesting to note that someone so truly visionary seems to have become that way as a result of studying people whom he felt were themselves self-actualized. If positive psychology, the psychology of virtue and values, becomes the heir of Maslow's goal, it should become a significant force in the field of psychology. That will be Maslow's true legacy.

The Importance of Values in the Science of Psychology

A common criticism leveled against many personality theorists is that they have not confirmed their theories in a strict, scientific manner. When one goes so far as to consider **values**, which are typically associated with religious morality, there is even greater resistance on the part of those who would have psychology become "truly" scientific to consider such matters worthy of examination. However, Maslow felt that:

Both orthodox science and orthodox religion have been institutionalized and frozen into a mutually excluding dichotomy…One consequence is that they are both pathologized, split into sickness, ripped apart into a crippled half-science and a crippled half-religion…As a result…the student who becomes a scientist automatically gives up a great deal of life, especially its richest portions. (pg. 119; Maslow, 1966)

Consequently, Maslow urged that we need to be fully aware of our values at all times, and aware of how our values influence us in our study of psychology. Although people approach the world in common ways, they also pay selective attention to what is happening, and they reshuffle the events occurring around them according to their own interests, needs, desires, fears, etc. Consequently, Maslow believed that paying attention to human values, particularly to an individual's values, actually helps the psychological scientist achieve the goal of clearly understanding human behavior (Maslow, 1970). In a similar vein, when Maslow co-authored an abnormal psychology text early in his career, he included a chapter on normal psychology. His description of the characteristics of a healthy, normal personality provides an interesting foreshadowing of his research on self-actualization (Maslow & Mittelmann, 1941).

Maslow felt so strongly about the loss of values in our society that he helped to organize a conference and then served as editor for a book entitled New Knowledge in Human Values (Maslow, 1959). In the preface, Maslow laments that "…the ultimate disease of our time is valuelessness…this state is more crucially dangerous than ever before in history…" (pg. vii; Maslow, 1959). Maslow does suggest, however, that something can be done about this loss of values, if only people will try. In the book, he brought together an interesting variety of individuals, including: Kurt Goldstein, a well-known neurophysiologist who studied the holistic function of healthy vs. brain-damaged patients and who coined the term self-actualization; D. T. Suzuki, a renowned Zen Buddhist scholar; and Paul Tillich, a highly respected existential theologian (who had a direct and significant influence on the career of Rollo May). There are also chapters by Gordon Allport and Erich Fromm. In his own chapter, Maslow concludes:

If we wish to help humans to become more fully human, we must realize not only that they try to realize themselves but that they are also reluctant or afraid or unable to do so. Only by fully appreciating this dialectic between sickness and health can we help to tip the balance in favor of health. (pg. 135; Maslow, 1959)

Discussion Question: Maslow believed that values are very important, not only in the study of psychology, but in society as well. Do you agree? When politicians or religious leaders talk about values, do you think they represent meaningful, true values, or do they just support the values that are an advantage to their own goal or the goals of their political party or church?

The Hierarchy of Needs

Maslow's is undoubtedly best known for his **hierarchy of needs**. Developed within the context of a theory of human motivation, Maslow believed that human behavior is driven and guided by a set of **basic needs**: **physiological needs, safety needs, belongingness and love needs, esteem needs,** and the **need for self-actualization**. It is generally accepted that individuals must move through the hierarchy in order, satisfying the needs at each level before one can move on to a higher level. The reason for this is that lower needs tend to occupy the mind if they remain unsatisfied. How easy is it to work or study when you are really hungry or thirsty? But Maslow did not consider the hierarchy to be rigid. For example, he encountered some people for whom self-esteem was more important than love, individuals suffering from antisocial personality disorder seem to have a permanent loss of the need for love, or if a need has been satisfied for a long time it may become less important. As lower needs are becoming satisfied, though not yet fully satisfied, higher needs may begin to present themselves. And of course there are sometimes multiple determinants of behavior, making the relationship between a given behavior and a basic need difficult to identify (Maslow, 1943/1973; Maslow, 1970).

The physiological needs are based, in part, on the concept of **homeostasis**, the natural tendency of the body to maintain critical biological levels of essential elements or conditions, such as water, salt, energy, and body temperature. Sexual activity, though not essential for the individual, is biologically necessary for the human species to survive. Maslow described the physiological needs as the most **prepotent**. In other words, if a person is lacking everything in life, having failed to satisfy physiological, safety, belongingness and love, and esteem needs, their consciousness will most like be consumed with their desire for food and water. As the lowest and most clearly biological of the needs, these are also the most animal-like of our behavior. In Western culture, however, it is rare to find someone who is actually starving. So when we talk about being hungry, we are really talking about an appetite, rather than real hunger (Maslow, 1943/1973; Maslow, 1970). Many Americans are fascinated by stories such as those of the ill-fated Donner party, trapped in the Sierra Nevada mountains during the winter of 1846-1847, and the Uruguayan soccer team whose plane crashed in the Andes mountains in 1972. In each case, either some or all of the survivors were forced to cannibalize those who had died. As shocking as such stories are, they demonstrate just how powerful our physiological needs can be.

The safety needs can easily be seen in young children. They are easily startled or frightened by loud noises, flashing lights, and rough handling. They can become quite upset when other family members are fighting, since it disrupts the feeling of safety usually associated with the home. According to Maslow, many adult neurotics are like children who do not feel safe. From another perspective, that of Erik Erikson, children and adults raised in such an environment do not trust the environment to provide for their needs. Although it can be argued that few people in America seriously suffer from a lack of satisfying physiological needs, there are many people who live unsafe lives. For example, inner city crime, abusive

spouses and parents, incurable diseases like HIV/AIDS, all present life threatening dangers to many people on a daily basis.

One place where we expect our children to be safe is in school. However, as we saw in the last chapter (in the section on the martial arts), 160,000 children each day are too frightened to attend school (Nathan, 2005). Juvonen et al. (2006) looked at the effects of ethnic diversity on children's perception of safety in urban middle schools (Grade 6). They surveyed approximately 2,000 students in 99 classrooms in the greater Los Angeles area. The ethnicity of the students in this study was 46 percent Latino (primarily of Mexican origin), 29 percent African American, 9 percent Asian (primarily East Asian), 9 percent Caucasian, and 7 percent multiracial. When a given classroom, or a given school, is more ethnically diverse, both African American and Latino students felt safer, were harassed less by peers, felt less lonely, and they had higher levels of self-worth (even when the authors controlled for differences in academic engagement). Thus, it appears that ethnic diversity in schools leads toward satisfaction of the need for safety, at least in one important area of a child's life. Unfortunately, most minority students continue to be educated in schools that are largely ethnically segregated (Juvonen, et al., 2006).

Throughout the evolution of the human species we found safety primarily within our family, tribal group, or our community. It was within those groups that we shared the hunting and gathering that provided food. Once the physiological and safety needs have been fairly well satisfied, according to Maslow, "the person will feel keenly, as never before, the absence of friends, or a sweetheart, or a wife, or children" (Maslow, 1970). Although there is little scientific confirmation of the belongingness and love needs, many therapists attribute much of human suffering to society's thwarting of the need for love and affection. Most notable among personality theorists who addressed this issue was Wilhelm Reich. An important aspect of love and affection is sex. Although sex is often considered a physiological need, given its role in procreation, sex is what Maslow referred to as a multidetermined behavior. In other words, it serves both a physiological role (procreation) and a belongingness/love role (the tenderness and/or passion of the physical side of love). Maslow was also careful to point out that love needs involve both giving and receiving love in order for them to be fully satisfied (Maslow, 1943/1973; Maslow, 1970).

Maslow believed that all people desire a stable and firmly based high evaluation of themselves and others (at least the others who comprise their close relationships). This need for self-esteem, or self-respect, involves two components. First is the desire to feel competent, strong, and successful (similar to Bandura's self-efficacy). Second is the need for prestige or status, which can range from simple recognition to fame and glory. Maslow credited Adler for addressing this human need, but felt that Freud had neglected it. Maslow also believed that the need for self-esteem was becoming a central issue in therapy for many psychotherapists. However, as we saw in Chapter 12, Albert Ellis considers self-esteem to be a sickness. Ellis' concern is that self-esteem, including efforts to boost self-esteem in therapy, requires that people rate themselves, something that Ellis felt will eventually lead to a negative evaluation (no one is perfect!). Maslow did acknowledge that the healthiest self-esteem is based on well-earned and deserved respect from others, rather than fleeting fame or celebrity status (Maslow, 1943/1973; Maslow, 1970).

When all of these lower needs (physiological, safety, belongingness and love, and esteem) have been largely satisfied, we may still feel restless and discontented unless we are doing

what is right for ourselves. “What a man can be, he must be” (pg. 46; Maslow, 1970). Thus, the need for self-actualization, which Maslow described as the highest of the basic needs, can also be referred to as a **Being-need**, as opposed to the lower **deficiency-needs** (Maslow, 1968). We will examine self-actualization in more detail in the following section.

Maslow’s hierarchy of needs is based on a theory of motivation. Individuals must essentially satisfy the lower deficiency needs before they become focused on satisfying the higher Being needs. Beyond even the Being needs there is something more, a state of transcendence that ties all people and the whole of creation together.

Although Maslow recognized that humans no longer have instincts in the technical sense, we nonetheless share basic drives with other animals. We get hungry, even though how and what we eat is determined culturally. We need to be safe, like any other animal, but again we seek and maintain our safety in different ways (such as having a police force to provide safety for us). Given our fundamental similarity to other animals, therefore, Maslow referred to the basic needs as **instinctoid**. The lower the need the more animal-like it is, the higher the need, the more human it is, and self-actualization was, in Maslow’s opinion, uniquely human (Maslow, 1970).

In addition to the basic needs, Maslow referred to **cognitive needs** and **aesthetic needs**. Little is known about cognitive needs, since they are seldom an important focus in clinic settings. However, he felt there were ample grounds for proposing that there are positive impulses to know, to satisfy curiosity, to understand, and to explain. The eight-fold path described by the Buddha, some 2,600 years ago, begins with right knowledge. The importance of mental stimulation for some people is described quite vividly by Maslow:

I have seen a few cases in which it seemed clear to me that the pathology (boredom, loss of zest in life, self-dislike, general depression of the bodily functions, steady deterioration of the intellectual life, of tastes, etc.) were produced in intelligent people leading stupid lives in stupid jobs. I have at least one case in which the appropriate cognitive therapy (resuming part-time studies, getting a position that was more intellectually demanding, insight) removed the symptoms.

I have seen many women, intelligent, prosperous, and unoccupied, slowly develop these same symptoms of intellectual inanition. Those who followed my recommendation to immerse themselves in something worthy of them showed improvement or cure often enough to impress me with the reality of the cognitive needs. (pg. 49; Maslow, 1970)

There are also classic studies on the importance of environmental enrichment on the structural development of the brain itself (Diamond et al., 1975; Globus, et al., 1973; Greenough & Volkmar, 1973; Rosenzweig, 1984; Spinelli & Jensen, 1979; Spinelli, Jensen, & DiPrisco, 1980). Even less is known about the aesthetic needs, but Maslow was convinced that some people need to experience, indeed they crave, beauty in their world. Ancient cave drawings have been found that seem to serve no other purpose than being art. The cognitive and aesthetic needs may very well have been fundamental to our evolution as modern humans.

Self-Actualization

Maslow began his studies on self-actualization in order to satisfy his own curiosity about people who seemed to be fulfilling their unique potential as individuals. He did not intend to

undertake a formal research project, but he was so impressed by his results that he felt compelled to report his findings. Amongst people he knew personally and public and historical figures, he looked for individuals who appeared to have made full use of their talents, capacities, and potentialities. In other words, “people who have developed or are developing to the full stature of which they are capable” (Maslow, 1970). His list of those who clearly seemed self-actualized included Abraham Lincoln, Thomas Jefferson, Albert Einstein, Eleanor Roosevelt, Jane Addams, William James, Albert Schweitzer, Aldous Huxley, and Baruch Spinoza. His list of individuals who were most-likely self-actualized included Goethe (possibly the great-grandfather of Carl Jung), George Washington, Benjamin Franklin, Harriet Tubman (born into slavery, she became a conductor on the Underground Railroad prior to the Civil War), and George Washington Carver (born into slavery at the end of the Civil War, he became an agricultural chemist and prolific inventor). In addition to the positive attributes listed above, Maslow also considered it very important that there be no evidence of psychopathology in those he chose to study. After comparing the seemingly self-actualized individuals to people who did not seem to have fulfilled their lives, Maslow identified fourteen characteristics of self-actualizing people (Maslow, 1950/1973, 1970), as follows:

More Efficient Perception of Reality and More Comfortable Relations with It: Self-actualizing people have an ability to recognize fakers, those who present a false persona. More than that, however, Maslow believed they could recognize hidden or confused realities in all aspects of life: science, politics, values and ethics, etc. They are not afraid of the unknown or people who are different, they find such differences to be a pleasant challenge. Although a high IQ may be associated with this characteristic, it is not uncommon to find those who are seemingly intelligent yet unable to be creative in their efforts to discover new phenomena. Thus, the perception of reality is not simply the same as being smart.

Acceptance (Self, Others, Nature): Similar to the approach Albert Ellis took with REBT (and his hypothesized dangers inherent in self-esteem), Maslow believed that self-actualizing people accept themselves as they are, including their faults and the differences between their personal reality and their ideal image of themselves. This is not to say that they are without guilt. They are concerned about personal faults that can be improved, any remaining habits or psychological issues that are unhealthy (e.g., prejudice, jealousy, etc.), and the shortcomings of their community and/or culture.

Spontaneity: The lives of self-actualizing people are marked by simplicity and a natural ease as they pursue their goals. Their outward behavior is relatively spontaneous, and their inner life (thoughts, drives, etc.) is particularly so. In spite of this spontaneity, they are not always unconventional, because they can easily accept the constraints of society and find their own way to fit in without being untrue to their own sense of self.

Problem-Centering: Self-actualizing individuals are highly problem-centered, not ego-centered. The problems they focus on are typically not their own, however. They focus on problems outside themselves, on important causes they would describe as necessary. Solving such problems is taken as their duty or responsibility, rather than as something they want to do for themselves.

The Quality of Detachment; the Need for Privacy: Whereas social withdrawal is often seen as psychologically unhealthy, self-actualizing people enjoy their privacy. They can

remain calm as they separate themselves from problematic situations, remaining above the fray. In accordance with this healthy form of detachment, they are active, responsible, self-disciplined individuals in charge of their own lives. Maslow believed that they have more free will than the average person.

Self-actualized individuals need their privacy. This may help them put life in perspective and prepare for each day.

Autonomy, Independence of Culture and Environment: As an extension of the preceding characteristics, self-actualizing individuals are growth-motivated as opposed to being deficiency-motivated. They do not need the presence, companionship, or approval of others. Indeed, they may be hampered by others. The love, honor, esteem, etc., that can be bestowed by others has become less important to someone who is self-actualizing than self-development and inner growth.

Continued Freshness of Appreciation: Self-actualizing people are able to appreciate the wonders, as well as the common aspects, of life again and again. Such feelings may not occur all the time, but they can occur in the most unexpected ways and at unexpected times. Maslow offered a surprising evaluation of the importance of this characteristic of self-actualization:

I have also become convinced that getting used to our blessings is one of the most important nonevil generators of human evil, tragedy, and suffering. What we take for granted we undervalue, and we are therefore too apt to sell a valuable birthright for a mess of pottage, leaving behind regret, remorse, and a lowering of self-esteem. Wives, husbands, children, friends are unfortunately more apt to be loved and appreciated after they have died than while they are still available. Something similar is true for physical health, for political freedoms, for economic well-being; we learn their true value after we have lost them. (pp. 163-164; Maslow, 1970)

The “Mystic Experience” or “Oceanic Feeling;” Peak Experiences: The difference between a **mystic experience** (also known as an **oceanic feeling**) and a **peak experience** is a matter of definition. Mystic experiences are viewed as gifts from God, something reserved for special or deserving (i.e., faithful) servants. Maslow, however, believed that this was a natural occurrence that could happen for anyone, and to some extent probably did. He assigned the psychological term of peak experiences. Such experiences tend to be sudden feelings of limitless horizons opening up to one’s vision, simultaneous feelings of great power and great vulnerability, feelings of ecstasy, wonder and awe, a loss of the sense of time and place, and the feeling that something extraordinary and transformative has happened. Self-actualizers who do not typically experience these peaks, the so-called “**non-peakers**,” are more likely to become direct agents of social change, the reformers, politicians, crusaders, and so on. The more transcendent “**peakers**,” in contrast, become the poets, musicians, philosophers, and theologians.

Maslow devoted a great deal of attention to peak experiences, including their relationship to religion. At the core of religion, according to Maslow, is the private illumination or revelation of spiritual leaders. Such experiences seem to be very similar to peak experiences, and Maslow suggests that throughout history these peak experiences may have been mistaken for revelations from God. In his own studies, Maslow found that people who were spiritual, but not religious (i.e., not hindered by the doctrine of a specific faith or

church), actually had more peak experiences than other people. Part of the explanation for this, according to Maslow, is that such people need to be more serious about their ethics, values, and philosophy of life, since their guidance and motivation must come from within. Individuals who seek such an appreciation of life may help themselves to experience an extended form of peak experience that Maslow called the **plateau experience**. Plateau experiences always have both noetic and cognitive elements, whereas peak experiences can be entirely emotional (Maslow, 1964). Put another way, plateau experiences involve serene and contemplative **Being-cognition**, as opposed to the more climactic peak experiences (Maslow, 1971).

Gemeinschaftsgefuhl: A word invented by Alfred Adler, **gemeinschatfsgefuhl** refers to the profound feelings of identification, sympathy, and affection for other people that are common in self-actualization individuals. Although self-actualizers may often feel apart from others, like a stranger in a strange land, becoming upset by the shortcomings of the average person, they nonetheless feel a sense of kinship with others. These feelings lead to a sincere desire to help the human race.

Interpersonal Relations: Maslow believed that self-actualizers have deeper and more profound personal relationships than other people. They tend to be kind to everyone, and are especially fond of children. Maslow described this characteristic as “compassion for all mankind,” a perspective that would fit well with Buddhist and Christian philosophies.

The Democratic Character Structure: Self-actualizing people are typically friendly with anyone, regardless of class, race, political beliefs, or education. They can learn from anyone who has something to teach them. They respect all people, simply because they are people. They are not, however, undiscriminating:

The careful distinction must be made between this democratic feeling and a lack of discrimination in taste, of an undiscriminating equalizing of any one human being with any other. These individuals, themselves elite, select for their friends elite, but this is an elite of character, capacity, and talent, rather than of birth, race, blood, name, family, age, youth, fame, or power. (pg. 168; Maslow, 1970)

Discrimination Between Means and Ends, Between Good and Evil: Self-actualizers know the difference between right and wrong. They are ethical, have high moral standards, and they do good things while avoiding doing bad things. They do not experience the average person’s confusion or inconsistency in making ethical choices. They tend to focus on ends, rather than means, although they sometimes become absorbed in the means themselves, viewing the process itself as a series of ends.

Philosophical, Unhostile Sense of Humor: The sense of humor shared by self-actualizers is not typical. They do not laugh at hostile, superior, or rebellious humor. They do not tell jokes that make fun of other people. Instead, they poke fun at people in general for being foolish, or trying to claim a place in the universe that is beyond us. Such humor often takes the form of poking fun at oneself, but not in a clown-like way. Although such humor can be found in nearly every aspect of life, to non-self-actualizing people the self-actualizers seem to be somewhat sober and serious.

Creativeness: According to Maslow, self-actualizing people are universally creative. This is not the creativity associated with genius, such as that of Mozart or Thomas Edison, but rather the fresh and naive creativity of an unspoiled child. Maslow believed that this

creativity was a natural potential given to all humans at their birth, but that the constraints on behavior inherent in most cultures lead to its suppression.

As desirable as self-actualization may seem, self-actualizing individuals still face problems in their lives. According to Maslow, they are typically not well adjusted. This is because they resist being enculturated. They do not stand out in grossly abnormal ways, but there is a certain inner detachment from the culture in which they live. They are not viewed as rebels in the adolescent sense, though they may be rebels while growing up, but rather they work steadily toward social change and/or the accomplishment of their goals. As a result of their immersion in some personal goal, they may lose interest in or patience with common people and common social practices. Thus, they may seem detached, insulting, absent-minded, or humorless. They can seem boring, stubborn, or irritating, particularly because they are often superficially vain and proud only of their own accomplishments and their own family, friends, and work. According to Maslow, outbursts of temper are not rare. Maslow argued that there are, in fact, people who become saints, movers and shakers, creators, and sages. However, these same people can be irritating, selfish, angry, or depressed. No one is perfect, not even those who are self-actualizing (Maslow, 1950/1973, 1970).

Discussion Question: Consider Maslow's characteristics of self-actualizing people. Which of those characteristics do you think are part of your personality? Are there any characteristics that you think may be particularly difficult for you to achieve?

Obstacles to Self-Actualization

In The Farther Reaches of Human Nature (Maslow, 1971), which was completed by Maslow's wife and one of his colleagues shortly after Maslow's death, Maslow described self-actualization as something that one does not obtain or fulfill at a specific point in time. Rather, it is an ongoing process of self-actualizing, characterized for some by brief periods of self-actualization (the peak experiences, for example). Maslow also described two major obstacles to achieving self-actualization: **desacralizing** and the **Jonah complex**. The Jonah complex, a name suggested by Maslow's friend Professor Frank Manuel, refers to being afraid of one's own greatness, or evading one's destiny or calling in life. Maslow specifically described this as a non-Freudian defense mechanism in which a person is as afraid of the best aspects of their psyche as they are afraid of the worst aspects of their psyche (i.e., the socially unacceptable id impulses). He described the process of this fear as a recognition, despite how much we enjoy the godlike possibilities revealed by our finest accomplishments, of the weakness, awe, and fear we experience when we achieve those accomplishments. According to Maslow, "great emotions after all can in fact overwhelm us" (Maslow, 1971). Nonetheless, he encouraged people to strive for greatness, within a reasonable sense of their own limitations.

A very important defense mechanism, which affects young people in particular, is what Maslow called desacralizing. The source of this problem is usually found within the family:

These youngsters mistrust the possibility of values and virtues. They feel themselves swindled or thwarted in their lives. Most of them have, in fact, dopey parents whom they don't respect very much, parents who are quite confused themselves about values and who, frequently, are simply terrified of their children and never punish them or stop them from doing things that are wrong. So you have a situation where the youngsters simply despise their elders - often for good and sufficient reason. (pg. 49; Maslow, 1971)

As a result, children grow up without respect for their elders, or for anything their elders consider important. The values of the culture itself can be called into question. While such a situation may sometimes be important for changing social conventions that unfairly discriminate against some people, can we really afford to live in a society in which nothing is sacred? Indeed, can such a society or culture continue to exist? Thus, Maslow emphasized a need for **resacralizing**. Maslow noted that he had to make up the words desacralizing and resacralizing “because the English language is rotten for good people. It has no decent vocabulary for the virtues” (Maslow, 1971). Resacralizing means being willing to see the sacred, the eternal, the symbolic. As an example, Maslow suggested considering a medical student dissecting a human brain. Would such a student see the brain simply as a biological organ, or would they be awed by it, also seeing the brain as a sacred object, including even its poetic aspects? This concept is particularly important for counselors working with the aged, people approaching the end of their lives, and may be critical for helping them move toward self-actualization. According to Maslow, when someone asks a counselor for help with the self-actualizing process, the counselor had better have an answer for them, “or we’re not doing what it is our job to do” (Maslow, 1971).

Discussion Question: Maslow believed that desacralizing was particularly challenging for young people. Do you think our society has lost its way, have we lost sight of meaningful values? Is nothing sacred anymore? Is there anything that you do in your life to recognize something as sacred in a way that has real meaning for your community?

Maslow had something else interesting to say about self-actualization in The Farther Reaches of Human Nature: "What does self-actualization mean in moment-to-moment terms? What does it mean on Tuesday at four o'clock?" (pg. 41). Consequently, he offered a preliminary suggestion for an operational definition of the process by which self-actualization occurs. In other words, what are the behaviors exhibited by people on the path toward fulfilling or achieving the fourteen characteristics of self-actualized people described above? Sadly, this could only remain a preliminary description, i.e., they are "ideas that are in midstream rather than ready for formulation into a final version," because this book was published after Maslow's death (having been put together before his sudden and unexpected heart attack).

What does one do when he self-actualizes? Does he grit his teeth and squeeze? What does self-actualization mean in terms of actual behavior, actual procedure? I shall describe eight ways in which one self-actualizes. (pg. 45; Maslow, 1971)

• They experience full, vivid, and selfless concentration and total absorption.

• Within the ongoing process of self-actualization, they make growth choices (rather than fear choices; progressive choices rather than regressive choices).

• They are aware that there is a selfto be actualized.

• When in doubt, they choose to be honestrather than dishonest.

• They trust their own judgment, even if it means being different or unpopular (being courageous is another version of this behavior).

• They put in the effort necessary to improve themselves, working regularly toward self-developmentno matter how arduous or demanding .

• They embrace the occurrence of peak experiences, doing what they can to facilitate and enjoy more of them (as opposed to denying these experiences as many people do).

• They identify and set aside their ego defenses (they have "the courage to give them up"). Although this requires that they face up to painful experiences, it is more beneficial than the consequences of defenses such as repression.

Being and Transcendence

Maslow had great hope and optimism for the human race. Although self-actualization might seem to be the pinnacle of personal human achievement, he viewed Humanistic Psychology, or **Third Force Psychology**, as just another step in our progression:

I should say also that I consider Humanistic, Third Force Psychology to be transitional, a preparation for a still “higher” Fourth Psychology, transpersonal, transhuman, centered in the cosmos rather than in human needs and interest, going beyond humanness, identity, self-actualization, and the like…These new developments may very well offer a tangible, usable, effective satisfaction of the “frustrated idealism” of many quietly desperate people, especially young people. These psychologies give promise of developing into the life-philosophy, the religion-surrogate, the value-system, the life-program that these people have been missing. Without the transcendent and the transpersonal, we get sick, violent, and nihilistic, or else hopeless and apathetic. We need something “bigger than we are” to be awed by and to commit ourselves to in a new, naturalistic, empirical, non-churchly sense, perhaps as Thoreau and Whitman, William James and John Dewey did. (pp. iii-iv; Maslow, 1968)

Although Maslow wrote about this need for a **Fourth Force Psychology** in 1968, it was not until the year 1998 that APA President Martin Seligman issued his call for the pursuit of positive psychology as an active force in the field of psychology. Maslow believed that all self-actualizing people were involved in some calling or vocation, a cause outside of themselves, something that fate has called them to and that they love doing. In so doing, they devote themselves to the search for **Being-values** (or B-values; Maslow, 1964, 1967/2008, 1968). The desire to attain self-actualization results in the B-values acting like needs. Since they are higher than the basic needs, Maslow called them **metaneeds**. When individuals are unable to attain these goals, the result can be **metapathology**, a sickness of the soul. Whereas counselors may be able to help the average person with their average problems, metapathologies may require the help of a **metacounselor**, a counselor trained in philosophical and spiritual matters that go far beyond the more instinctoid training of the traditional psychoanalyst (Maslow, 1967/2008). The B-values identified by Maslow (1964) are an interesting blend of the characteristics of self-actualizing individuals and the human needs described by Henry Murray: truth, goodness, beauty, wholeness, dichotomy-transcendence, aliveness, uniqueness, perfection, necessity, completion, justice, order, simplicity, richness, effortlessness, playfulness, self-sufficiency.

Transcendence is typically associated with people who are religious, spiritual, or artistic, but Maslow said that he found transcendent individuals amongst creative people in a wide variety of vocations (including business, managers, educators, and politicians), though there are not many of them in any field. Transcendence, according to Maslow, is the very highest and most holistic level of human consciousness, which involves relating to oneself, to all others, to all species, to nature, and to the cosmos as an end rather than as a means

(Maslow, 1971). It is essential that individuals not be reduced to the role they play in relation to others, transcendence can only be found within oneself (Maslow, 1964, 1968). Maslow's idea is certainly not new. Ancient teachings in Yoga tell us that there is a single universal spirit that connects us all, and Buddhists describe this connection as **interbeing**. The Abrahamic religions teach us that the entire universe was created by, and therefore is connected through, one god. It was Maslow's hope that a transcendent Fourth Force in psychology would help all people to become self-actualizing. In Buddhist terms, Maslow was advocating the intentional creation of psychological Bodhisattvas. Perhaps this is what Maslow meant by the term metacounselor.

Connections Across Cultures: Is Nothing Sacred?

Maslow described some lofty ambitions for humanity in Toward a Psychology of Being(1968) and The Farther Reaches of Human Nature (1971), as well as some challenges we face along the way. Transcendence, according to Maslow, is a loss of our sense of Self, as we begin to feel an intimate connection with the world around us and all other people. But transcendence is exceedingly difficult when we are hindered by the defense mechanism of desacralization. What exactly does the word "sacred" mean? It is not easily found in psychological works. William James often wrote about spiritual matters, but not about what is or is not sacred. Sigmund Freud mentioned sacred prohibitions in his final book, Moses and Monotheism (Freud, 1939/1967), but he felt that anything sacred was simply a cultural adaptation of all children's fear of challenging their father's will (and God was created as a symbol of the mythological father). A dictionary definition of sacred says that it is "connected with God (or the gods) or dedicated to a religious purpose and so deserving veneration." However, there is another definition that does not require a religious context: "regarded with great respect and reverence by a particular religion, group, or individual" (The Oxford American College Dictionary, 2002). Maslow described desacralization as a rejection of the values and virtues of one's parents. As a result, people grow up without the ability to see anything as sacred, eternal, or symbolic. In other words, they grow up without meaning in their lives.

The process of resacralization, which Maslow considered an essential task of therapists working with clients who seek help in this critical area of their life, requires that we have some concept of what is sacred. So, what is sacred? Many answers can be found, but there does seem to be at least one common thread.

Christians have long believed that forgiveness lies at the heart of faith. Psychologists have recently found that forgiveness may also lie at the heart of emotional and physical well-being.

David Myers & Malcolm Jeeves (2003)

…Compassion is the wish that others be free of suffering. It is by means of compassion that we aspire to attain enlightenment. It is compassion that inspires us to engage in the virtuous practices that lead to Buddhahood. We must therefore devote ourselves to developing compassion.

The Dalai Lama (2001)

I have been engaged in peace work for more than thirty years: combating poverty, ignorance, and disease; going to sea to help rescue boat people; evacuating the wounded

from combat zones; resettling refugees; helping hungry children and orphans; opposing wars; producing and disseminating peace literature; training peace and social workers; and rebuilding villages destroyed by bombs. It is because of the practice of meditation - stopping, calming, and looking deeply - that I have been able to nourish and protect the sources of my spiritual energy and continue this work.

Thich Nhat Hanh (1995)

…Our progress is the penetrating of the present moment, living life with our feet on the ground, living in compassionate, active relationship with others, and yet living in the awareness that life has been penetrated by the eternal moment of God and unfolds in the power of that moment.

Fr. Laurence Freeman (1986)

Keep your hands busy with your duties in this world, and your heart busy with God.

Sheikh Muzaffer (cited in Essential Sufism by Fadiman & Frager, 1997)

Forgiveness is a letting go of past suffering and betrayal, a release of the burden of pain and hate that we carry.

Forgiveness honors the heart’s greatest dignity. Whenever we are lost, it brings us back to the ground of love.

Jack Kornfield (2002)

And he said to him, “You shall love the Lord your God with all your heart, and with all your soul, and with all your mind. This is the great and first commandment. And a second is like it, You shall love your neighbor as yourself…”

Jesus Christ (The Holy Bible, 1962)

In examining self-actualizing people directly, I find that in all cases, at least in our culture, they are dedicated people, devoted to some task “outside themselves,” some vocation, or duty, or beloved job. Generally the devotion and dedication is so marked that one can fairly use the old words vocation, calling, or mission to describe their passionate, selfless, and profound feeling for their “work.”

The spiritual life is then part of the human essence. It is a defining-characteristic of human nature, without which human nature is not full human nature. It is part of the Real Self, of one’s identity, of one’s inner core, of one’s specieshood, of full humanness.

Abraham Maslow (1971)

Christians, Buddhists, Muslims, as well as members of other religions and humanists, all have some variation of what has been called The Golden Rule: treating others as you would like to be treated. If that is sacred, then even amongst atheists, young people can evaluate the values and virtues of their parents, community, and culture, and then decide whether those values are right or wrong, whether they want to perpetuate an aspect of that society based on their own thoughts and feelings about how they, themselves, may be treated someday by others. This resacralization need not be religious or spiritual, but it commonly is, and some psychologists are comfortable embracing spirituality as such.

Kenneth Pargament and Annette Mahoney (2005) wrote a chapter entitled Spirituality: Discovering and Conserving the Sacred, which was included in the Handbook of Positive Psychology (Snyder & Lopez, 2005). First, they point out that religion is an undeniable fact in American society. Some 95 percent of Americans believe in God, and 86 percent believe that He can be reached through prayer and that He is important or very important to them. Spirituality, according to Pargament and Mahoney, is the process in which individuals seek both to discover and to conserve that which is sacred. It is interesting to note that Maslow and Rogers consider self-actualization and transcendence to be a process as well, not something that one can get and keep permanently. An important aspect of defining what is sacred is that it is imbued with divinity. God may be seen as manifest in marriage, work can be seen as a vocation to which the person is called, the environment can been seen as God's creation. In each of these situations, and in others, what is viewed as sacred has been sanctified by those who consider it sacred. Unfortunately, this can have negative results as well, such as when the Heaven's Gate cult followed their sanctified leader to their deaths. Thus, spirituality is not necessarily synonymous with a good and healthy lifestyle.

Still, there is research that has shown that couples who sanctify their marriage experience greater marital satisfaction, less marital conflict, and more effective marital problem-solving strategies. Likewise, mothers and fathers who sanctify the role of parenting report less aggression and more consistent discipline in raising their children. For college students, spiritual striving was more highly correlated with well-being than any other form of goal-setting (see Pargament & Mahoney, 2005). So there appear to be real psychological advantages to spiritual pursuits. This may be particularly true during challenging times in our lives:

…there are aspects of our lives that are beyond our control. Birth, developmental transitions, accidents, illnesses, and death are immutable elements of existence. Try as we might to affect these elements, a significant portion of our lives remains beyond our immediate control. In spirituality, however, we can find ways to understand and deal with our fundamental human insufficiency, the fact that there are limits to our control… (pg. 655; Pargament & Mahoney, 2005)

Eupsychian Management and Theory Z

It is not merely a coincidence that Maslow is well-known in the field of business. He spent 3 years as the plant manager for the Maslow Cooperage Corporation, and later he spent a summer studying at an electronics firm in California (Non-Linear Systems, Inc.) at the invitation of the company's president. He became very interested in industrial and managerial psychology, and the journal he kept in California was published as Eupsychian Management (Maslow, 1965). **Eupsychia** refers to real possibility and improvability, and a movement toward psychological health, as opposed to the vague fantasies of proposed Utopian societies. More precisely, though this is something of a fantasy itself, Maslow described Eupsychia as the culture that would arise if 1,000 self-actualizing people were allowed to live their own lives on a sheltered island somewhere. Maslow applied his psychological theories, including both the hierarchy of needs and self-actualization, to a management style that takes advantage of this knowledge to maximize the potential of the employees in a company (also see the collection of Maslow's unpublished papers by Hoffman, 1996).

Maslow introduced a variety of terms related to his theories on management, one of the most interesting being **synergy**. Having borrowed the term from Ruth Benedict, synergy refers to a situation in which a person pursuing their own, selfish goals is automatically helping others, and a person unselfishly helping others is, at the same time, helping themselves. According to Maslow, when selfishness and unselfishness are mutually exclusive, it is a sign of mild psychopathology. Self-actualizing individuals are above the distinction between selfishness and unselfishness; they enjoy seeing others experience pleasure. Maslow offered the personal example of feeding strawberries to his little daughter. As the child smacked her lips and thoroughly loved the strawberries, an experience that thrilled Maslow, what was he actually giving up by letting her eat the strawberries instead of eating them himself? In his experience with the Blackfoot tribe, a member named Teddy was able to buy a car. He was the only one who had one, but tradition allowed anyone in the tribe to borrow it. Teddy used his car no more often than anyone else, but he had to pay the bills, including the gas bill. And yet, everyone in the tribe was so proud of him that he was greatly admired and they elected him chief. So, he benefited in other ways by following tradition and letting everyone use his car (Maslow, 1965). In the business field, when managers encourage cooperation and communication, everyone benefits from the healthy growth and continuous improvement of the company. And this leads us to **Theory Z** (which is Eupsychian management).

Douglas McGregor, a professor of industrial relations at the Massachusetts Institute of Technology, was greatly impressed with Maslow's work, and McGregor had used Motivation and Personality as a textbook in his business classes. Based on Maslow's theories, McGregor published a book in 1960 in which he outlined two managerial models, **Theory X** and **Theory Y** (Gabor, 2000; Hoffman, 1996). Maslow described the two theories as follows:

…To put it succinctly, Theory Y assumes that if you give people responsibilities and freedom, then they will like to work and will do a better job. Theory Y also assumes that workers basically like excellence, efficiency, perfection, and the like.

Theory X, which still dominates most of the world's workplace, has a contrasting view. It assumes that people are basically stupid, lazy, hurtful, and untrustworthy and, therefore, that you have got to check everything constantly because workers will steal you blind if you don't. (pg. 187; Maslow, 1996a)

The Theory X/Theory Y strategy was intentionally put into practice at Non-Linear Systems, hence Maslow's invitation to study there. Maslow concluded, however, that even Theory Y did not go far enough in maximizing people's potential. People have metaneeds*(**the need for B-values), needs that go beyond simply offering higher salaries. When employees have their basic needs met, but recognize inefficiency and mismanagement in the company, they will still complain, but these higher level complaints can now be described as** metagrumbles*(as opposed to the lower level grumbles about lower level needs). Theory Z attempts to transcend Theory Y and actively facilitate the growth of a company's employees toward self-actualization (Hoffman, 1996; Maslow, 1971; Maslow 1996b).

Discussion Question: How's your job (or any job you have had)? Would you describe your supervisor or boss as someone who uses Eupsychian or Theory Z management? Does the workplace foster synergy amongst the employees? If not, can you imagine how the job would be different if they did?

Henry Murray and Personology

Henry Murray was primarily psychodynamic in his orientation. However, the fundamental aspect of his theory is the presence of needs in our lives, and there was a distinctly humanistic aspect to his theories as well (Maddi & Costa, 1972). Thus, it seems appropriate to include Murray alongside Maslow's discussion of human needs. In addition, Murray developed a practical application of his famous test, the **Thematic Apperception Test** (or **TAT**), for screening candidates for special work assignments. Once again, this is similar to Maslow's forays into the field of industrial/organizational psychology. Although it is common to present different fields as fundamentally opposed, such as humanistic psychology **vs.** psychodynamic psychology, Murray and Maslow provide an ideal opportunity to see the commonalities that often exist between different areas in psychology. It must also be remembered that Murray was no strict adherent to the dogmatic view of psychoanalysis presented by Freud:

…psychoanalysis stands for a conceptual system which explains, it seems to me, as much as any other. But this is no reason for going in blind and swallowing the whole indigestible bolus, cannibalistically devouring the totem father in the hope of acquiring his genius, his authoritative dominance, and thus rising to power in the psychoanalytic society, that battle-ground of Little Corporals. No; I, for one, prefer to take what I please, suspend judgment, reject what I please, speak freely. (pg. 31; Murray, 1940/2008).

Brief Biography of Henry Murray

Henry Alexander Murray, Jr. was born in 1893 in New York City. He had many nicknames, and typically asked his friends to call him Harry. His family was quite wealthy, and had a noble history. He was a descendant of John Murray, the fourth Earl of Dunmore, the last Royal Governor of Virginia, and his mother's great-grandfather, Colonel Harry Babcock, had served on General George Washington's staff during the Revolutionary War. Murray lived a life of luxury, spending the summers on Long Island and often traveling throughout Europe. He was educated at exclusive private schools. However, his childhood was not without challenges. He felt abandoned by his mother, who suffered from depression much of her life, when Murray was quite young. He stuttered, and was cross-eyed. The operation to help cure his internal strabismus accidentally left him with an external strabismus. This created problems for Murray when it came to competing in athletics, but Murray worked hard to overcome his difficulties and he excelled at sports. He became the quarterback of his football team and won a featherweight boxing championship at school. In college, he made the rowing team at Harvard University (Maddi & Costa, 1972; Robinson, 1992).

In spite of his athletic success at Harvard, or perhaps because of it, he did not do well academically, receiving below average grades. Nonetheless, he earned a degree in history in 1915. While at Harvard he also married Josephine Rantoul, after a lengthy courtship. Despite his mediocre grades at Harvard, Murray was accepted into the Columbia College of Physicians and Surgeons, and graduated first in his class in 1919. He then completed a surgical internship at Presbyterian Hospital in New York, where he once treated the future president Franklin D. Roosevelt, followed by a period of research at the Rockefeller Institute for Medical Research and Cambridge University, which culminated in a Ph.D. in biochemistry in 1927. He then accepted a position as assistant to Morton Prince, and became the director of Harvard University's psychology clinic. Murray had never taken a

psychology course, but he had some interesting experience (Maddi & Costa, 1972; Robinson, 1992).

Murray had a psychiatry course in medical school, and had read Freud's Interpretation of Dreams. He also had a research assistant from Vienna, Alma Rosenthal, who had been a long-time friend of Anna Freud. While both working together and having an intimate love affair, Rosenthal introduced Murray to the deeper dimensions of the unconscious mind. However, it was Murray's lifelong mistress, Christiana Morgan, who introduced him to Jung's book Psychology Types. Murray was deeply impressed by Jung's book, but even more by Jung himself. Murray was troubled by the intense love affair he had developed with Morgan, so he went to Zurich in order to be psychoanalyzed by Jung. Jung managed to help Murray understand his stuttering and accept having his affair with Morgan. After all, Jung had maintained a mistress of his own for many years. Jung also managed to convince Murray's wife and Morgan's husband to accept the affair as well, and Christiana Morgan remained a very important colleague throughout Murray's life. It has been suggested that she played a far more important role in his theories, and in the development of the TAT, than she has been given credit for (Maddi & Costa, 1972; Robinson, 1992). Partly because Jung had directly helped him with a psychological problem, and partly because of the extraordinary range of ideas that Jung was open to, Murray always spoke highly of Jung (though he believed that Jung tended toward being psychotic, just as Freud tended toward being neurotic; see Brian, 1995).

Initially, Murray's reappointment as clinic director was challenged by the experimental psychologists Edwin Boring and Karl Lashley, but he was supported by the clinical psychologists, who were led by Gordon Allport (Stagner, 1988). As his work continued he was quite productive (it was during this time that he developed the TAT), and many important clinicians passed through the clinic. Included among them was Erik Erikson, who came to the clinic after having been psychoanalyzed by Anna Freud in Vienna. Murray also spent a great deal of time traveling and studying in Europe, and enjoyed a memorable evening with Sigmund and Anna Freud. As he was preparing to return to the clinic, World War II began. Murray joined the Army Medical Corps, and eventually worked for the Office of Strategic Services (OSS). Of particular interest was his use of the TAT to screen OSS agents for sensitive missions (the OSS was the precursor to the CIA, so in peacetime these agents would be called spies). He was in China studying errors they had made in their assessments when the atomic bomb was dropped on Hiroshima. Murray was shocked, and devoted the rest of his life to seeking alternatives to war (Maddi & Costa, 1972; Robinson, 1992).

As his career and life approached their ends, Murray received the Distinguished Scientific Contribution Award from the American Psychological Association, and the Gold Medal Award from the American Psychological Foundation. He received numerous honorary degrees, and collections of papers have been published in his honor (e.g., White, 1963; Zucker, Rabin, Aronoff, & Frank, 1992). In June, 1988, Murray told his nurse that he was dead. She disagreed with him, and pinched him gently on the cheek to prove her point. He curtly disagreed with her, declaring that he was the doctor, she was the nurse, and he was dead. A few days later he was right (Robinson, 1992).

Placing Murray in Context: A Challenging Task

There does not seem to be a consensus on where Murray fits within the field of personality theory. Trained as a Freudian psychoanalyst, he is often grouped with the neo-Freudians. However, he has also been placed with the trait theorists, and he was a colleague of Gordon Allport. However, many personality theory textbooks don't consider Murray worthy of significant attention. He is included alongside Maslow in this textbook because his work focused primarily on needs. In addition, the practical application of his Thematic Apperception Test in screening candidates for OSS assignments was similar to Maslow's application of psychological principles in the business field.

The Thematic Apperception Test is certainly Murray's claim to fame. It remains one of the best-known tests in psychology, having been applied in research, business, and therapeutic settings. Since Murray used the TAT in combination with the Rorschach Inkblot Test, he maintained his ties to traditional psychoanalysis and helped to advance the fame of the other renowned projective test. As such, his practical contributions to psychology seem to outweigh his theoretical contributions.

It has been said that the value of a theory can be measured by the research that follows. David McClelland's use of the TAT to study the need for achievement is a common topic in introductory psychology textbooks. Thus, Murray's contributions have inspired classic research in psychology. That alone should ensure a place of significance for Murray in the history of personality theory.

Human Needs

In Explorations in Personality (Murray, 1938), Murray describes people as "today's great problem". What can we know about someone, and how can we describe it in a way that has clear meaning? Nothing is more important in the field of psychology:

The point of view adopted in this book is that personalities constitute the subject matter of psychology, the life history of a single man being a unit with which this discipline has to deal… Our guiding thought was that personality is a temporal whole and to understand a part of it one must have sense, though vague, of the totality. (pgs. 3-4; Murray, 1938)

Thus, Murray and his colleagues sought to understand the nature of personality, in order to help them understand individuals. He referred to this direct study of personality as **personology**, simply because he considered it clumsy to refer to "the psychology of personality" instead.

Murray described the very elegant process by which the Harvard Clinic group systematically approached their studies, and then presented a lengthy series of propositions regarding a theory of personality. The primary focus of these propositions came down to what Murray called a press-need combination. A **need**, according to Murray, is a hypothetical process that is imagined to occur in order to account for certain objective and subjective facts. In other words, when an organism reliably acts in a certain way to obtain some goal, we can determine that the organism had a need to achieve that goal. Needs are often recognized only after the fact, the behavior that satisfies the need may be a blind impulse, but it still leads toward satisfying the needed goal. **Press** is the term Murray applied to environmental objects or situations that designate directional tendencies, or that guide our needs. Anything in the environment, either harmful or beneficial to the organism, exerts press. Thus, our current needs, in the context of current environmental press, determine our ongoing behavior (Murray, 1938).

Like Maslow, Murray separated needs into biological and psychology factors based on how essential they were to one's survival. The primary, or **viscerogenic needs**, include air, water, food, sex, harm-avoidance, etc. The secondary or **psychogenic needs**, which are presumed to derive from the primary needs, are common reaction systems and wishes. Although Murray organizes the psychogenic needs into groups, they are not rank-ordered as was Maslow's hierarchy, so we will not consider the groups any further. Individually, there are a total of twenty-eight human needs (Murray, 1938). A partial list, with definitions, includes the following:

• Acquisition: the need to gain possessions and property

• Retention: the need to retain possession of things, to refuse to give or lend

• Order: the need to arrange, organize, put away objects, to be tidy and clean

• Construction: the need to build things

• Achievement: the need to overcome obstacles, to exercise power, to strive to do something difficult as well and as quickly as possible

• Recognition: the need to excite praise and commendation, to demand respect

• Exhibition: the need to attract attention to oneself

• Defendance: the need to defend oneself against blame or belittlement

• Counteraction: the need to proudly overcome defeat by restriving and retaliating, to defend one's honor

• Dominance: the need to influence or control others

• Deference: the need to admire and willingly follow a superior

• Aggression: the need to assault or injure another, to harm, blame, accuse, or ridicule a person

• Abasement: the need to surrender, to comply and accept punishment

• Affiliation: the need to form friendships and associations, to greet, join, and live with others, to love

• Rejection: the need to snub, ignore, or exclude others

• Play: the need to relax, amuse oneself, seek diversion and entertainment

• Cognizance: the need to explore, to ask questions, to satisfy curiosity

According the Murray, in the course of daily life these needs are often interrelated. When a single action can satisfy more than one need, we can say that the needs are fused. However, needs can also come into conflict. For example, an individual's need for dominance may make it difficult to satisfy their need for affiliation, unless they can find someone with a powerful need for abasement. Such a situation is one of the ways in which psychologists have tried to understand abusive relationships. In other words, when someone with a strong need for affiliation and debasement becomes involved with someone with a strong need for affiliation and dominance (particularly in a pathological sense), the results can be very unfortunate.

Anyone who has children is often reminded of their need for playing, and most any setting can provide an opportunity for play. Here, the author's children are playing cards.

Any object, or person, that evokes a need is said to "be cathected" by the person being studied. In other words, they have invested some of their limited psychic energy (libido) into that object. Murray believed that an individual's personality is revealed by the objects to which that person is attached by the cathexis of libido, especially if you can recognize the intensity, endurance, and rigidity of the cathexis. This process not only applies to individuals, but institutions and cultures also have predictable patterns in terms of their cathected objects. Put more simply, we can strive to understand individuals, including doing so from a cross-cultural perspective, by examining the nature and pattern of needs they seek to satisfy in their daily lives (Murray, 1938).

Morris Stein, who worked with Murray in the OSS and then earned a Ph.D. at the Harvard Clinic, combined Murray's work on identifying human needs and Jung's concept of psychological types. By looking at patterns in the rank-order of needs among industrial chemists and Peace Corps volunteers, Stein was able to divide each group into separate psychological types (Stein, 1963). For example, there were five basic types of industrial chemists: Type A was achievement oriented but still worked well with others; Type B focused on pleasing others, often at the expense of their own ideas; Type C was achievement oriented, but more driven and hostile than Type A; Type D was motivated by achievement and affiliation, but with an emphasis on order that protected them from criticism or blame; and Type E was particularly focused on relationships marked by cooperation and trust. As interesting as these types may be, they are quite different than the personality types identified amongst the Peace Corps volunteers (Stein, 1963). Thus, although Stein's investigation suggests that personality types can be identified based on patterns of need, this approach probably would not provide a general theory of personology that could be applied to anyone.

Discussion Question: Consider Murray's list of psychogenic needs. Which needs are the ones that affect you the most? Are you able to fulfill those needs?

The TAT and the OSS

Murray is typically credited with the development the TAT. However, the original article has Christiana Morgan as the first author (Morgan & Murray, 1935), and in Explorations in Personality most of the TAT work is described by Morgan (Murray, 1938). Apparently, when the test was revised and republished in 1943, Murray did most of the revision, partly because Morgan was quite ill at the time. The TAT consists of a series of pictures depicting potentially dramatic events (although the pictures are actually rather vague). The person taking the test is asked to provide a story that relates events preceding the picture to some final outcome of the situation. It is expected that the subject will project their own thoughts and feelings into the picture as they create their story. In order for this to be possible, Morgan and Murray made sure that in most pictures there was at least one person with whom the subject could easily empathize and identify themselves. The TAT became one of the most popular projective tests ever developed, and continues to be widely used today.

The TAT has been used in two particularly interesting settings outside of clinical psychology: to study the need for achievement (see the next section), and to screen agents for the Office of Strategic Services during World War II. Murray used the TAT as part of a program to help

select members of the OSS for critical, dangerous missions. Even before joining the OSS, Murray worked for the government in support of the war effort. In conjunction with Gordon Allport, he provided an analysis of the personality of Adolf Hitler, along with predictions as to how Hitler might react after Germany was defeated. He also helped to develop a series of questions for the crew of a captured German U-boat. The OSS program involved assessing candidate's responses to highly stressful situations. In addition to psychological testing, using instruments such as the TAT, the candidates were put into highly stressful situations. For example, they were told to pick two men to help them put together a five-foot cube with wooden poles, blocks, and pegs. However, the available men were all secretly on Murray's staff. One of them would act helpless and passive, whereas the other made stupid suggestions and constantly criticized the recruit. The task was, of course, never completed, but it provided Murray with the information he needed on how the candidate performed under stress (Brian, 1995; Robinson, 1992).

In the next chapter we will see that the existential psychologist Rollo May talked about our need for myths, in order to make sense out of our often senseless world. Although this was not a need included by Murray, he did have an interest in mythology. The imagination that is necessary to create a story around a picture in the TAT often involves symbolism that arises from the depths of the whole self (Murray, 1960). In this regard, Murray sounds quite similar to Jung and his theory of archetypes, and Murray discussed some classic images from our historical mythology. Of particular interest to Murray, however, is whether or not we will establish new myths in the future. There are older myths that remain oriented to our future, such as the apocalyptic myths or the myth of the Promised Land (Murray, 1960). The existential philosopher Jean-Paul Sartre lamented the demythologizing of the universe by science, and he advocated a remythologizing of the self (see McAdams, 1992). Given that Murray did include a need for cognizance, the need to explore, to ask questions, and to satisfy curiosity, perhaps there will be new myths created in our future. If so, psychologists will need to keep current with the cultural phenomena that influence people's unconscious projections onto the TAT and other projective tests.

David McClelland and the Need for Achievement

David McClelland, who joined the faculty of Harvard University a few years before Murray retired, conducted some well-known research utilizing the TAT to examine the **need for achievement**. The research began shortly after World War II, and was supported by the Office of Naval Research. McClelland and his colleagues made an interesting point, in the preface to their book The Achievement Motive (McClelland, Atkinson, Clark, & Lowell, 1953), about studying just one of Murray's needs: "concentration on a limited research problem is not necessarily narrowing; it may lead ultimately into the whole of psychology." Indeed, they felt that they learned a great deal about personality by studying one of the most important of human needs.

McClelland and his colleagues used the TAT and borrowed heavily from Murray's procedures and scoring system. However, they made a number of modifications. They used additional pictures of their own, they often presented the pictures on a screen to a group of subjects, those subjects were all male college students, and some of their experimental conditions were designed to evoke achievement-oriented responses, or responses based on success or failure. An important aspect of this study was that the TAT (and similar pictures developed by McClelland) requires writing imaginative stories of what the subject projects

onto the picture. Therefore, situations that stimulate achievement-oriented imagination can result in higher scores on the need for achievement, something that McClelland and his colleagues confirmed in Navaho children during the course of their research (suggesting it is a universal phenomenon). Overall, they found that individuals who are high in their need for achievement perform more tasks during timed tests, improve more quickly in their ability to perform those tasks, set higher levels of aspirations, remember more of the tasks they failed to perform, and they are more future-oriented and recognize achievement-oriented situations (McClelland et al., 1953). In addition, they found a positive correlation between the need for achievement and cultures and families in which there is an emphasis on the individual development of children, with early childhood being of particular importance. After examining eight Native American cultures (Navaho, Ciricahua-Apache, Western Apache, Hopi, Comanche, Sanpoil, Paiute, and Flatheads), McClelland and his colleagues determined that the need for achievement in each culture (measured from classic legends involving the archetypal trickster “coyote”) correlates highly with both an early age onset and the severity of independence training (McClelland et al., 1953). In summary, the need for achievement is a motivational force that develops in early childhood, and which pushes individuals toward accomplishing life’s tasks.

An excellent essay on the need for achievement, which addresses some of the criticism this concept has endured, was written by McClelland in a new introduction for the second printing of his book The Achieving Society (McClelland, 1976). This book also adds to the cross-cultural reach of McClelland’s work, since as he extends his theory on the need for achievement to the societies in which individuals live he also extends his theory to other societies around the world. First, the concept itself has typically been misunderstood:

…the word “achievement” cues all sorts of surplus meanings that the technically defined n Achievement variable does not have. It refers specifically to the desire to do something better, faster, more efficiently, with less effort. It is not a generalized desire to succeed… (pg. A; McClelland, 1976)

In studying the role of need for achievement within societies, McClelland focused on business and economic development as one of the most easily compared aspects of different cultures. He believed that nations possess something like a “group mind,” which can lead the nation in certain directions. Again using literary sources as examples of cultural perspectives on the need for achievement, McClelland found support for his theory that high need for achievement preceded dramatic societal development in ancient Greece, pre-Incan Peru, Spain in the late middle ages, England leading up to the industrial revolution, and during the development of the United States (particularly in the 1800s). Once again, McClelland cautions against over-generalizing the meaning of need for achievement:

It is a very specific, rather rare, drive which focuses on the goal of efficiency and which expresses itself in activities available in the culture which permit or encourage one to be more efficient; and across cultures the most common form such activity takes is business. (pg. B; McClelland, 1976)

The question of where the need for achievement comes from continued to perplex McClelland. Although early childhood appears to be when a lasting need for achievement develops, the need for achievement can be enhanced in adults through training seminars. More importantly, however, is the question of where need for achievement comes from in the first place, how does it develop within a society? When McClelland was working in

Ethiopia with the Peace Corps, he studied the Gurage. This small tribal group was treated with disdain by both the dominant Christian Amhara and the Muslim Galla tribes. And yet the Gurage were recognized for their clever business strategies, and their children wrote stories filled with imagery indicative of a high need for achievement. Since the Gurage had developed without contact with Western Christian, Muslim, or Greco-Roman cultures, they seemed to have developed their own need for achievement. Unfortunately, so little is known about their history, that McClelland was unable to identify the source of their motivation (McClelland, 1976).

In support of the contention that studying the need for achievement could provide insights into many aspects of personality, McClelland pursued a number of interesting topics throughout his career, including how societies can motivate economic growth and identify talent (McClelland, Baldwin, Bronfenbrenner, & Strodtbeck, 1958; McClelland & Winter, 1969), the power motive (McClelland, 1975), the development of social maturity and values (McClelland, 1982a; McClelland, 1982b), and a cross-cultural study on the role of alcohol in society (McClelland, Davis, Kalin, & Wanner, 1972). Moving in a quite different direction, McClelland also wrote a book entitled The Roots of Consciousness (McClelland, 1964), in which he argues that Sigmund Freud's psychoanalysis is really an expression of Jewish spiritual mysticism known as Kabbalah. We will examine Kabbalah, as well as Christian and Islamic mysticism, as a positive approach to one's lifestyle in Chapter 18.

Discussion Question: McClelland found support for his ideas on the development of the need for achievement amongst Native Americans, but he did not find that same support among the Gurage tribe in Ethiopia (they had a strong need for achievement, but the source was unclear). How important do you think it is for us to re-examine psychological theories in multiple cultures, and what would it mean for psychology if we often find contradictions?

A Final Note: Humanistic or Existential?

In this chapter we have examined the humanistic theories of Carl Rogers and Abraham Maslow. In the next chapter we will examine the existential theories of Viktor Frankl and Rollo May. What really is the difference? The distinction is subtle, based on definition, and may seem nonexistent at first glance. Indeed, both the humanistic and existential theorists have been influenced by the likes of Adler, Horney, Fromm, and Otto Rank, and Rogers in particular often writes about existential choices in his books. Even the cognitive therapist Albert Ellis, himself profoundly influenced by Adler, considered Rational Emotive Behavior Therapy to be distinctly humanistic (see Humanistic Psychotherapy; Ellis, 1973). In 1986, the Saybrook Institute republished a series of essays, which had appeared in the Journal of Humanistic Psychology, under the title Politics and Innocence: A Humanistic Debate (May, Rogers, Maslow, et al., 1986). In this volume, Rogers refers to May as "the leading scholar of humanistic psychology." May, for his part, concluded an open letter to Rogers in which he expressed "profound respect for you and your contribution in the past to all of us." May also maintained a friendship and correspondence with Maslow (May, 1991). Clearly, the humanistic and existential psychologists have much in common, and the important figures here in America communicated actively and with respect for the contributions of each other.

Personality Theory in Real Life: Seeking Self-Actualization

Carl Rogers described the actualizing tendency as something that exists within every living organism. It is a tendency to grow, develop, and realize one's full potential. It can be

thwarted, but it cannot be destroyed without destroying the organism itself. His person-centered approach was based on this belief, and the resulting trust that one can place in each person. In other words, we can trust that each person is driven forward by this actualizing tendency, and that under the right conditions it will flourish (Rogers, 1977, 1986/1989).

According to Abraham Maslow, life is a process of choices. At each point, we must choose between a progression choice and a regression choice. Although many people make safe, defensive choices, self-actualizing people regularly make **growth choices** (Maslow, 1971). Each growth choice moves the person closer to self-actualization, and the process continues throughout life.

So, consider your own life. Do you feel the actualizing tendency within you? Do you aspire to accomplish something great, or simply to be a good person in whatever path you choose? Think about your educational and/or career plans. Think about your life plans, and whether they include a family or special friends. Do you feel a calling that is pulling in one direction or another? The drive to accomplish, to make a contribution to your community or society, the belief that you are meant for great things, or simply that you are meant to be a source of support for others, all of these might be aspects of your actualizing tendency. Or are you moving through life without a plan, without goals? Do you skate along from day to day, with no destination in mind?

If you do feel your actualizing tendency, consider how you are living your life. Are you pursuing the steps necessary to accomplish your goals? Have you made choices, perhaps difficult choices, which have moved you forward toward those goals?

Basically, do you feel that you are on a path toward self-actualization, and do you think you should be? Is it reasonable to expect, or hope, that everyone might become self-actualized?

What might it be like to live a fully transcendent, self-actualized life? Although there are many different, and individual, answers to that question, we can find one example in the remarkable life of Peace Pilgrim (Friends of Peace Pilgrim, 1982). No one knows her original name, or exactly where or when she was born (other than it was on a small farm in the Eastern United States in the early 1900s). Her family was poor, but happy, and she enjoyed her childhood. Her life was fruitful, but eventually she found the world’s focus on self-centeredness and material goods to be unfulfilling. In 1953, she chose to leave her life behind. She adopted the name Peace Pilgrim, and began walking across America as a prayer for peace.

A pilgrim is a wanderer with a purpose…Mine is for peace, and that is why I am a Peace Pilgrim…My pilgrimage covers the entire peace picture: peace among nations, peace among groups, peace within our environment, peace among individuals, and the very, very important inner peace - which I talk about most often because that is where peace begins…I have no money. I do not accept any money on my pilgrimage. I belong to no organization…I own only what I wear and carry. There is nothing to tie me down. I am as free as a bird soaring in the sky.

I walk until given shelter, fast until given food. I don’t ask - it’s given without asking. Aren’t people good! There is a spark of good in everybody, no matter how deeply it may be buried, it is there. It’s waiting to govern your life gloriously. (pg. 25; Peace Pilgrim cited in Friends of Peace Pilgrim, 1982)

Between 1953 and her death in 1981, she walked, and walked, and walked. By 1964, she had walked 25,000 miles, including walking across the United States twice and through every Canadian province. After that, she no longer kept track of her mileage, but she completed at least four more pilgrimages, including Alaska, Hawaii, and a pilgrimage in Mexico. Among the many friends and admirers she met along the way, there are two notable people (whom psychology students should be familiar with) who provided comments for the cover of her book: Elisabeth Kubler-Ross called her “a wonderful lady,” and the popular author/counselor Wayne Dyer said “she is my hero.” As for your own life, Peace Pilgrim has some simple advice:

There is no glimpse of the light without walking the path. You can’t get it from anyone else, nor can you give it to anyone. Just take whatever steps seem easiest for you, and as you take a few steps it will be easier for you to take a few more. (pg. 91; Peace Pilgrim cited in Friends of Peace Pilgrim, 1982).

Review of Key Points

• Rogers began his clinical career searching for effective ways of conducting psychotherapy, since the techniques he had been taught were not providing adequate results.

• Rogers believed that each person exists in their own, unique experiential field. Only they can see that field clearly, although even they may not perceive it accurately (incongruence).

• Everyone has an actualizing tendency, according to Rogers. The term commonly applied to this tendency is self-actualization.

• The self is that portion of the experiential field that is recognized as “I” or “me.” It is organized into a self-structure.

• Rogers used the term personal power to describe each person’s ability to make choices necessary for the actualization of their self-structure and to then fulfill those choices or goals.

• In order for a person to grow, they must fulfill a need for positive regard. This can only come from receiving unconditional positive regard from important family members and friends (typically beginning with the parents).

• When people receive only conditional positive regard, they develop conditions of worth. Their self-regard then becomes tied to those conditions of worth.

• When an individual’s self-regard and positive regard are closely related, the person is said to be congruent. If not, they are said to be incongruent.

• Congruence and incongruence can be measured by understanding the gap between a person’s real self and their ideal self.

• Rogers described individuals who are congruent and continuing to grow as fully functioning persons.

• Relationships can serve to mirror our true personality, and to reveal incongruence we are unaware of ourselves.

• Successful marriages, according to Rogers, seem to be based on dedication/commitment, communication, dissolution of roles, and maintaining each person’s separate self.

• Rogers identified six necessary and sufficient conditions for positive therapeutic change, conditions that can exist in any interpersonal relationships (not just in therapy). The key factor in these relationships may be empathic understanding.

• Rogers extended his study of clinical psychology into other groups designed to help all people grow and self-actualize, such as T-groups and encounter groups. He described his shift from purely clinical work to fostering growth in all people as a person-centered approach.

• Maslow worked with an amazing range of people, from the renowned experimental psychologists Harry Harlow and Edward Thorndike, to the Gestalt psychologist Max Wertheimer and the personality theorists/clinicians Alfred Adler, Karen Horney, and Erich Fromm.

• Values were very important to Maslow in his approach to psychology. He did not, however, advocate his own values. He reached beyond humanistic psychology to include areas of study such as existential psychology, existential theology, and Zen Buddhism.

• Maslow described a hierarchy of needs, as follows: physiological needs, safety needs, belongingness and love needs, esteem needs, and the need for self-actualization. Lower needs must be largely satisfied before the individual begins to focus on higher needs.

• The lower needs can be described as deficiency-needs, whereas self-actualization is a Being-need.

• In addition to the basic needs, there are also cognitive needs and aesthetic needs.

• Maslow described fourteen characteristics of self-actualizing people. He developed his list by studying both contemporary and historical people who seemed to him to be self-actualizing.

• Perhaps the best know characteristic of self-actualizing is the peak experience. This experience is often described in mystical terms, and Maslow believed it may have provided a basis for the creation of religion in the early history of the human species.

• Maslow described two defense mechanisms that interfere with the process of self-actualizing: desacralizing and the Jonah complex.

• Maslow proposed a Fourth Force Psychology based on Being-values and metaneeds. He felt that some people could suffer from a sickness of the soul, a so-called metapathology, and Maslow suggested a need for metacounselors.

• Some individuals experience profound peak experiences, which Maslow described as transcendent. His concept of transcendence seems very close to the Buddhist perspective of interbeing.

• Maslow proposed that organizations should seek Eupsychia, a realistically attainable environment in which the actualizing tendency of all the organization’s members are supported.

• When Eupsychian management does support self-actualization, the actualization of each person benefits the others around them. The process is known as synergy.

• Based on a management model that described Theory X and theory Y management styles, Maslow proposed Theory Z. Theory Z management seeks a transcendent management style that encourages and maximizes self-actualization and synergy in the work place.

• Murray based “personology” on the study of needs. He distinguished between viscerogenic needs and psychogenic needs.

• Christiana Morgan and Murray developed the Thematic Apperception Test, a famous projective psychological test. Murray used the test during World War II to select special agents for highly sensitive, dangerous missions.

• Murray believed that a person’s ability to create a story around a picture in the TAT was based in large part on their personal mythology. He shared this interest in myth, and its role in psychology, with Carl Jung and Rollo May.

• McClelland used the TAT to study the need for achievement. Initially, McClelland considered parental influence very important for the development of the achievement need, a finding he confirmed in Native Americans. However, he found contradictory evidence when he studied the Gurage tribe in Ethiopia. Thus, he considered the true source of the achievement need as something needing further research.

• The distinction between humanistic psychology and existential psychology is not clear, and there is significant overlap in the thinking of representatives from both fields. In addition, there is a distinct humanistic element in the psychodynamic theories of Adler, Horney, Fromm, Murray, and others.

CHAPTER 09 / CAC.902

Existential Therapy: Frankl, May, and Meaning

Existential therapy focuses on the conditions of human existence: freedom, responsibility, isolation, meaning, and death. It is less a set of techniques than a stance toward the client's life. Viktor Frankl developed logotherapy after surviving Nazi concentration camps. Rollo May brought existential ideas into American psychology. Irvin Yalom later organized existential concerns into four ultimate concerns: death, freedom, existential isolation, and meaninglessness. For adolescents, these concerns are not abstract. They appear as questions about identity, fairness, mortality, and whether effort matters.

Frankl and meaning

Frankl argued that the primary motivational force is the will to meaning. Meaning can be found through creative work, experience, and attitude toward unavoidable suffering. This does not mean suffering is good or that clients should accept abuse. Frankl's point is that even when circumstances cannot be changed, a person may retain some freedom in how they respond. The counselor must be careful not to use meaning language to silence protest. A child in an unsafe home needs protection, not a lecture about attitude.

Logotherapy uses techniques such as dereflection and paradoxical intention in selected cases. Dereflection shifts attention from hyperreflection to meaningful action. Paradoxical intention invites the client to wish for the feared symptom in a way that disrupts anticipatory anxiety. These techniques require skill and are not appropriate for every client. With children, meaning work is usually concrete: caring for a pet, helping a sibling, creating art, or choosing a small act of courage.

May and anxiety

Rollo May distinguished normal anxiety from neurotic anxiety. Normal anxiety is part of growth and freedom. Neurotic anxiety is disproportionate, repressed, and managed through rigid defenses. May argued that avoiding all anxiety can shrink the self. The counselor helps the client face the anxiety that belongs to living. For an adolescent leaving home, anxiety may be a sign of meaningful transition. For a child with panic attacks, anxiety may need symptom-focused treatment first. The existential lens does not replace assessment.

Freedom and responsibility

Existential therapy emphasizes that people are always choosing, even when choices are constrained. Responsibility does not mean blaming the client for oppression. It means helping the client notice the responses that remain possible. A teenager in poverty did not choose poverty. The teenager can still choose how to seek support, what values to protect, and what future to work toward. The counselor must hold both realities: structural injustice and personal agency.

Existential isolation

Existential isolation refers to the unbridgeable gap between persons. No one can fully live another's life. This can be frightening, but it also makes relationship meaningful. Adolescents often feel that no one understands them. The counselor does not claim total understanding. The counselor says, "I cannot know exactly what it is like, and I want to understand as much as you can show me." That honesty can be more connecting than false certainty.

Death and finitude

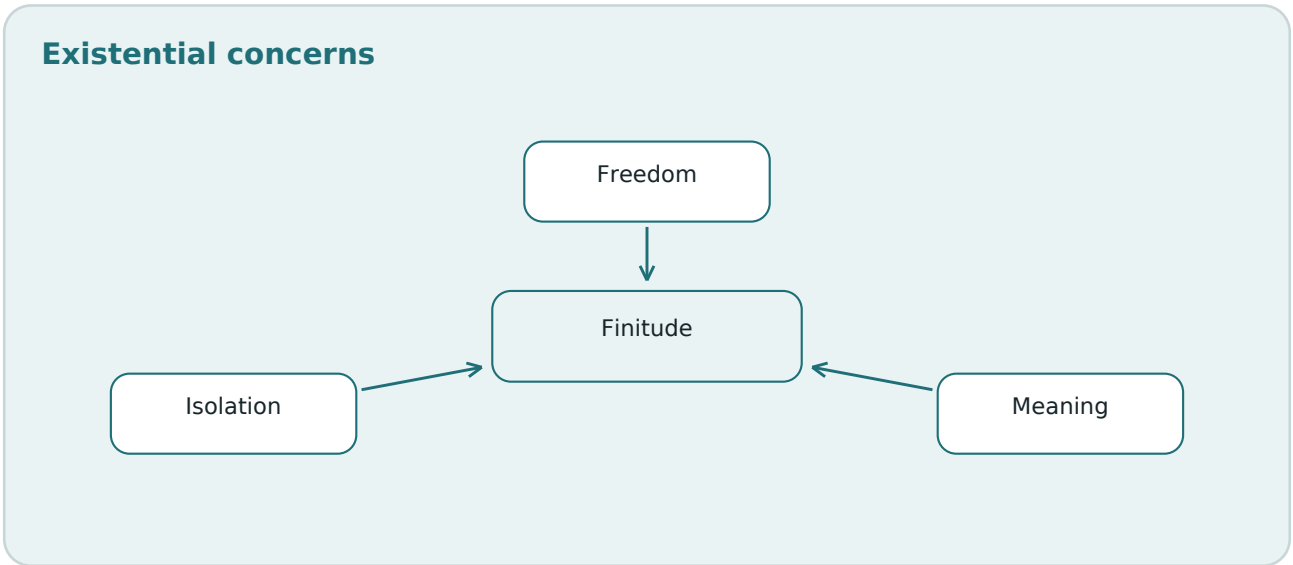
Children encounter death through pets, relatives, news, and illness. Avoiding the topic can increase fear. Existential work helps the client face finitude in developmentally appropriate ways. A child may ask, "Will you die?" The counselor answers honestly and gently, then explores the fear. With adolescents, death concerns may appear in risk-taking, nihilism, or intense activism. The counselor listens for the meaning question beneath the behavior.

Clinical example

Eighteen-year-old Aisha says, "Nothing matters. We're all going to die anyway." A purely cognitive response might challenge the thought as distorted. An existential response first validates the question: "You are looking at the big picture and wondering why any of it counts." The counselor then explores what Aisha values, what relationships matter, and what actions feel consistent with those values. If depression is present, the counselor also assesses safety and may use CBT or other evidence-based treatment. Existential dialogue and symptom treatment can coexist.

Limits and reflection

Existential therapy can be vague if it stays at the level of philosophy. It can also be culturally narrow if it assumes that individual choice is the highest value. Some clients find meaning in community, tradition, or religious faith. The counselor should follow the client's frame. Reflection: Write about a moment when you faced a limit you could not change. What response remained possible? What did that moment teach you about freedom?



Existential therapy helps clients face the conditions of living rather than only remove symptoms.

Attributed Foundational Reading

Attribution and critical-reading note

Adapted from Personality Theory in a Cultural Context by Mark D. Kelland, OpenStax CNX / Lansing Community College (2015), CC BY 4.0. Selection, organization, and PDF layout modified for this course; new lectures and course diagrams are separate additions.

Read historical and theoretical claims critically. A theory is a lens for inquiry, not a verdict about a child or family. Where source claims are dated or culture-bound, the new lecture supplies cautions and contemporary framing.

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Man is nothing else but what he makes of himself. Such is the first principle of Existentialism…For we mean that man first exists, that is, that man first of all is the being who hurls himself toward a future and who is conscious of imagining himself as being in the future…Thus, Existentialism's first move is to make every man aware of what he is and to make the full responsibility of his existence rest on him. (pg. 456; Jean-Paul Sartre, 1947/1996)

Existential psychology is the area within psychology most closely linked to the field of philosophy. Curiously, this provides one of the most common complaints against existential psychology. Many historians identify the establishment of Wilhelm Wundt's experimental laboratory in Germany in 1879 as the official date of the founding of psychology. Sigmund Freud, with his strong background in biomedical research, also sought to bring scientific methodology to the study of the mind and mental processes, including psychological disorders and psychotherapy. Shortly thereafter, Americans such as Edward Thorndike and John Watson were establishing behaviorism, and its rigorous methodology, as the most influential field in American psychology. So, as existential psychology arose in the 1940s and 1950s it was viewed as something of a throwback to an earlier time when psychology was not distinguished from philosophy (Lundin, 1979).

However, as with those who identify themselves as humanistic psychologists, existential psychologists are deeply concerned with individuals and the conditions of each unique human life. The detachment that seems so essential to experimental psychologists is unacceptable to existential psychologists. The difference can easily be seen in the titles of two influential books written by the leading existential psychologists: Man's Search for Meaning by Viktor Frankl (1946/1992) and Man's Search for Himself by Rollo May (1953). Existential psychology differs significantly from humanistic psychology, however, in focusing on present existence and the fear, anguish, and sorrow that are so often associated with the circumstances of our lives (Lundin, 1979).

Understanding the Philosophy of Existentialism

The roots of **existentialism** as a philosophy began with the Danish philosopher Søren Kierkegaard (1813-1855). Kierkegaard was intensely interested in man's relationship with God, and its ultimate impossibility. Man is finite and individual, whereas God is infinite and absolute, so the two can never truly meet. In pursuing the relationship, however, man goes through three stages or modes of existence: the **aesthetic mode**, the **ethical mode**, and the **religious mode**. The aesthetic mode is concerned with the here and now, and focuses primarily on pleasure and pain. Young children live primarily in this mode. The ethical mode involves making choices and wrestling with the concept of responsibility. An individual in the ethical mode must choose whether or not to live by a code or according to the rules of society. This submission to rules and codes may prove useful in terms of making life simple, but it is a dead end. In order to break out of this dead end, one must live in the religious mode by making a firm commitment to do so. While this may lead to the recognition that each of us is a unique individual, it also brings with it the realization of our total inadequacy relative to God. As a result, we experience loneliness, anxiety, fear, and dread. All of this anguish, however, allows us to know what is really true, and for Kierkegaard truth was synonymous with faith (in God). However, as important as man's relationship to God was for Kierkegaard, he was adamantly opposed to organized religion. Kierkegaard rejected objective, so-called **"truth"** in the form of religious dogma in favor of the subjective "truth" that each person "knows" within themselves. While this subjective, personal truth brings with it the responsibility that leads to anxiety, it can also elevate a person to an authentic existence (Breisach, 1962; Frost, 1942).

Another philosopher considered essential to the foundation of existentialism was the enigmatic German philosopher Friedrich Nietzsche (1844-1900). A key element of Nietzsche's philosophy is the **will-to-power**. He believed this will-to-power is the fundamental force in the universe (Alfred Adler considered it fundamental to personality development). Ironically, according to Nietzsche, the universe has no regard for humanity. Natural forces (such as disaster and disease) destroy people, life is extremely difficult, and even those who struggle on attempting to realize their will eventually succumb to death. There is no hope to be found in an afterlife, since Nietzsche is famous for declaring that God is dead! Neither is there much hope within society for many people. Nietzsche believed that inequality was the natural state of humanity, so he considered slavery to be perfectly understandable and he felt that women (who are physically weaker than men) should never expect the same rights as men. Nonetheless, Nietzsche saw a great future for humanity, in the belief, indeed the faith, that we would create a **superman**(or superwoman, as the case may be). It is the creation of the superman that gives purpose to existence. Although the concept of the superman helped to fuel Nazi views on creating a German master race, it also made its way into American comic books as the great hero Superman (Fritzsche, 2007; Frost, 1942; Jaspers, 1965). In perhaps Nietzsche's most famous work, Thus Spoke Zarathustra, the would-be prophet Zarathustra encouraged people to seek a better future for humanity:

I will teach you about the superman. Man is something that should be overcome. What have you done to overcome him? (pg. 81, Nietzsche, quoted in Fritzsche, 2007)

The German existentialists Martin Heidegger (1889-1976) and Karl Jaspers (1883-1969) focused on human existence itself and our role in the world. In a sense, Heidegger trivialized the nature of God, equating God with little more than the greatest being in the world, but a

being nonetheless (just as humans are). Jaspers was not an atheist, but still his existential theory focused on the human journey toward a **freedom** that has meaning only when it reveals itself in union with God (Breisach, 1962; Lescoe, 1974). Heidegger considered individuals as beings who are all connected in **Being**, thus distinguishing between mere beings (including other animals) and the nature of truth or Being. Only humans are capable of understanding this connection between all beings, and Heidegger referred to this discovery as **Dasein** (“being here,” or existence). On one level, Dasein is common to all creatures, but the possibility of being aware of one’s connection to Being is uniquely human. For those who ask the big questions, Dasein can become authentic existence. This experience comes in the fullness of life, but only if one adopts the mode of existence known as **being-in-the-world**. Heidegger insisted that Dasein and being-in-the-world are equal. Being-in-the-world is an odd concept, however, since Heidegger believed that Being can only arise from nothingness, and so we ourselves arise as **being-thrown-into-this-world**. Having been thrown into this mysterious world we wish to make it our own, but our desire for connection with Being leads to anxiety. This anxiety cannot be overcome, because we are aware that we will die! Surprisingly, however, Heidegger considers death to be something positive. It is only because we are going to die that some of us strive to experience life fully. If we can accept that death will come, and nothing will follow, we can be true to ourselves and live an authentic life (Breisach, 1962; Lundin, 1979).

Finally we come to the French existentialist Jean-Paul Sartre (1905-1980). Sartre was an extraordinary author and one of the most important philosophers of the twentieth century. He was awarded the Nobel Prize in Literature in 1964, but chose to reject it. More importantly for us, however, is the fact that he carried existential philosophy directly into psychology, with books such as The Transcendence of the Ego (Sartre, 1937/1957) and a section entitled “Existential Psychoanalysis” in his extraordinary work Being and Nothingness(Sartre, 1943). Whereas Kierkegaard believed that man could never truly be one with God, and Heidegger trivialized God, Sartre simply stated that God does not exist. But this is not inconsequential:

The Existentialist, on the contrary, thinks it very distressing that God does not exist, because all possibility of finding values in a heaven of ideas disappears along with Him; there can no longer be an a priori Good, since there is no infinite and perfect consciousness to think it. (pg. 459; Sartre, 1947/1996).

If one looks at the title of Sartre’s most famous philosophical work, Being and Nothingness(Sartre, 1943), you might get the impression that Sartre followed in the footsteps of Heidegger. However, Sartre did not agree with Heidegger (see Sartre, 1943). Sartre divided the world into **en-soi** (the in-itself) and **pour-soi** (the for-itself). Pour-soi can be defined as conscious beings, of which there is only one kind: human beings. Everything else is en-soi, things (including non-human animals) that are silent and dead, and from which come no meaning, they only are (Breisach, 1962). For Sartre, there is no mystery, no Being, tying all of creation together. Man’s consciousness is not a connection to God that can be realized, it is simply a unique characteristic of the human species. The nothingness to which Sartre refers is a shell around the pour-soi, the individual, which separates it from the en-soi. People who try to deny living authentically, those who try to deny the responsibility that comes with being conscious and settle into being nothing more than en-soi will have a shattering experience and be totally destroyed (since there is no Being, as described by Heidegger, beyond the shell surrounding the pour-soi; Breisach, 1962). This establishes

critical ethical implications for the individual, since their life will be what they make of it, and nothing more.

Unfortunately, many people do reject their unique consciousness and desire to be en-soi, just letting life happen around them. As the en-soi closes in around them, they begin to experience nausea, forlornness, anxiety, and despair. Herein lays the need for **existential psychoanalysis**:

Existential psychoanalysis is going to reveal to man the real goal of his pursuit, which is being as a synthetic fusion of the in-itself with the for-itself; existential psychoanalysis is going to acquaint man with his passion…Many men, in fact, know that the goal of their pursuit is being; and … they refrain from appropriating things for their own sake and try to realize the symbolic appropriations of their being-in-itself…existential psychoanalysis…must reveal to the moral agent that he is the being by whom values exist. It is then that his freedom will become conscious of itself… (pg. 797; Sartre, 1943)

Sartre proposed that individuals become conscious, and through that consciousness create the world itself, but also that we are “condemned to despair” and “doomed to failure” when we realize that all human activities are merely equivalent. This philosophical approach leads into Sartre’s criticism of the psychology of his time. Sartre believed that psychologists, and even most philosophers, stopped short of really understanding people:

For most philosophers the ego is an “inhabitant” of consciousness...Others - psychologists for the most part - claim to discover its material presence, as the center of desires and acts, in each moment of our psychic life. We should like to show here that the ego is neither formally nor materially inconsciousness: it is outside, in the world. It is a being of the world, like the ego of another. (pg. 31; Sartre, 1937/1957)

So, Sartre believed that an existential psychoanalysis was needed to go beyond the limits of Freudian psychoanalysis. It is not enough, according to Sartre, to stop at describing mere patterns of desires and tendencies (Sartre, 1943). In critiquing the psychoanalytic biography of a famous author named Flaubert, Sartre asked very meaningful questions about this individual's life: why did Flaubert become a writer instead of a painter, why did he come to feel exalted and self-important instead of gloomy, why did his writing emphasize violence, or amorous adventures, etc.? Sartre’s point is a common criticism of Freudian psychoanalytic theory. If most any result can come from an individual’s experiences, then what does psychoanalysis really tell us about anyone? Sartre proposed a deeper form of psychoanalysis:

This comparison allows us to understand better what an existential psychoanalysis must be if it is entitled to exist. It is a method destined to bring to light, in a strictly objective form, the subjective choice by which each living person makes himself a person; that is, makes known to himself what he is. Since what the method seeks is a choice of being at the same time as a being, it must reduce particular behavior patterns to fundamental relations - not of sexuality or of the will-to-power, but of being - which are expressed in this behavior. It is then guided from the start toward a comprehension of being and must not assign itself any other goal than to discover being and the mode of being of the being confronting this being. It is forbidden to stop before attaining this goal…This psychoanalysis has not yet found its Freud. (pp. 733-734; Sartre; 1943)

Placing Existential Psychology in Context: Height Psychology

Goes Deeper Than Depth Psychology

The two theorists highlighted in this chapter were truly extraordinary individuals. Both Viktor Frankl (who coined the term “height psychology”) and Rollo May were well immersed in existential thought and its application to psychology when they faced seemingly certain death. For Frankl, who was imprisoned in the Nazi concentration camps, death was expected. For May, who was confined to a sanitarium with tuberculosis, death was a very real possibility (and indeed many died there). But Frankl and May were intelligent, observant, and thoughtful men. They watched as many died, while some lived, and they sought answers that might explain who was destined for each group. Both men observed that for those who resigned themselves to death, death came soon. But for those who chose to live, they had a real chance to survive despite the terrible conditions in which they existed.

Frankl and May also shared their training in traditional psychoanalysis, and both had studied with Alfred Adler, at least somewhat. However, they found the so-called depth psychology as lacking, since it did not address the true potential for humans to rise above their conditions. In this regard, existential psychologists have typically been viewed as belonging within the humanistic psychology camp. However, both Frankl and May considered humanistic psychology to also be lacking, in that it neglected the true potential for humans to make bad choices, and to harm both themselves and others. So for existential psychologists, the center of their focus is on the immediate existence of the individual, in the context of their relationship to others. It is this seeming paradox, and the drive to resolve it, that provides the motivation and energy for life.

There is also a natural connection between existentialism and Eastern schools of thought, including yoga, Buddhism, and Taoism. Some of the comparisons are so striking that, shortly after discussing Taoism, May wrote “one gets the same shock of similarity in Zen Buddhism” (May, 1983). And so, this chapter should provide an interesting transition to the final section of this text, in which we will examine both Eastern and Western spiritual approaches to making positive choices in one’s life.

Viktor Frankl and Logotherapy

Viktor Frankl (1905-1997) was truly an extraordinary man. His first paper was submitted for publication by Sigmund Freud; his second paper was published at the urging of Alfred Adler. Gordon Allport was instrumental in getting Frankl’s book Man’s Search for Meaning(Frankl, 1946/1992) published in English, a book that went on to be recognized by the Library of Congress as one of the ten most influential books in America. He lectured around the world, and received some thirty honorary doctoral degrees in addition to the medical degree and the Ph.D. he had earned as a student. He was invited to a private audience with Pope Paul VI, even though Frankl was Jewish. All of this was accomplished in spite of, and partly because of, the fact that he spent several years in Nazi concentration camps during World War II, camps where his parents, brother, wife, and millions of other Jews died.

A Brief Biography of Viktor Frankl

Viktor Frankl was born in Vienna, Austria on March 26, 1905. Although his father had been forced to drop out of medical school for financial reasons, Gabriel Frankl held a series of positions with the Austrian government, working primarily with the department of child protection and youth welfare. He instilled in his son the importance of being intensely

rational and having a firm sense of social justice, and Frankl became something of a perfectionist. Frankl described his mother Elsa as a kindhearted and deeply pious woman, but during his childhood she often described him as a pest, and she even changed the words of Frankl's favorite childhood lullaby to include calling him a pest. This may have been due to the fact that Frankl was often asking questions, so much so that a family friend nicknamed him "The Thinker" (Frankl, 1995/2000). From his mother, Frankl inherited a deep emotionality. One aspect of this emotionality involved a deep attachment to his childhood home, and he often felt homesick as his responsibilities kept him away. And those responsibilities began at an early age (Frankl, 1995/2000; Pattakos, 2004).

Even in high school Frankl was developing a keen interest in existential philosophy and psychology. At the age of 16 he delivered a public lecture "On the Meaning of Life" and at 18 he wrote his graduation essay "On the Psychology of Philosophical Thought." Throughout his high school years he maintained a correspondence with Sigmund Freud (letters that were later destroyed by the Gestapo when Frankl was deported to his first concentration camp). When Frankl was just 19, Freud submitted one of Frankl's papers for publication in the International Journal of Psychoanalysis, afterward hoping that Frankl would agree and give his belated consent. Despite having impressed Freud, Frankl himself was already impressed by Alfred Adler. Frankl became active in Adler's individual psychology group, and as he began medical school he was urged by Adler to publish a paper in the International Journal of Individual Psychology. It is hard to imagine that many people could have come into the favor of both Freud and Adler by such a young age, even before having begun medical school or a career in psychiatry. Despite Frankl's young age and somewhat limited experience, the paper published by Adler was dealing with difficult material, specifically the "border area that lies between psychotherapy and philosophy, with special attention to the problems of meanings and values in psychology" (Frankl, 1995/2000). Eventually, however, Frankl fell out of favor with Adler. Frankl had been impressed with two men, Allers and Schwarz, whose views were at odds with Adler. On the evening when Allers and Schwarz announced to the society that they could not agree with Adler, Adler challenged Frankl and a friend to speak up. Frankl chose to do so, and he defended Allers and Schwarz, believing that a middle ground could be found. Adler never spoke to Frankl again, even when Frankl said hello in the local coffee shop. For a few months Adler had other people suggest to Frankl that he should quit the society. When Frankl did not, he was expelled by Adler (Frankl, 1995/2000; Pattakos, 2004).

Frankl proceeded to develop his own practice and his own school of psychotherapy, known as **logotherapy** (the therapy of meaning, as in finding meaning in one's life). As early as 1929, Frankl had begun to recognize three possible ways to find meaning in life: a deed we do or a work we create; a meaningful human encounter, particularly one involving **love**; and choosing one's attitude in the face of unavoidable suffering. Logotherapy eventually became known as the third school of Viennese psychotherapy, after Freud's psychoanalysis and Adler's individual psychology. During the 1930s Frankl did much of his work with suicidal patients and teenagers. He had extensive talks with Wilhelm Reich in Berlin, who was also involved in youth counseling by that time. As the 1930s came to an end, and Austria had been taken over by the Nazis, Frankl sought a visa to emigrate to the United States, which was eventually granted. However, Frankl's parents could not get a visa, so he chose to remain in Austria with them. He also began work on his first book, eventually published in English under the title The Doctor and the Soul (Frankl, 1946/1986), which provided the

foundation for logotherapy. He fell in love with Tilly Grosser, and they were married in 1941, the last legal Jewish marriage in Vienna under the Nazis.

Shortly thereafter, the realities of Nazi Germany overcame what little privilege Frankl had enjoyed as a doctor at a major hospital. Since it was illegal for Jews to have children, Tilly Frankl was forced to abort their first child. Frankl later dedicated The Unheard Cry for Meaning“To Harry or Marion an unborn child” (Frankl, 1978). Then the entire Frankl family, except for his sister who had gone to Australia, was deported to the Theresienstadt concentration camp (the same camp from which Anna Freud cared for orphans after the war). As they marched into the camp with hundreds, perhaps thousands, of other prisoners, his father tried to calm those who panicked by saying again and again: “Be of good cheer, for God is near.” Frankl’s parents, his only brother, and his wife Tilly died in the concentration camps. Most tragically, Frankl believed that his wife died after the war, but before the liberating Allied forces could care for all of the many, many suffering people (Frankl, 1995/2000).

When Frankl was deported, he tried to hide and save his only copy of The Doctor and the Soul by sewing it into the lining of his coat. However, he was forced to trade his good coat for an old one, and the manuscript was lost. While imprisoned, he managed to obtain a few scraps of paper on which to make notes. Those notes later helped him to recreate his book, and that goal gave such meaning to his life that he considered it an important factor in his will to survive the horrors of the concentration camps. It would be difficult to adequately describe the conditions of the concentration camps, or how they affected the minds of those imprisoned, especially since the effects were quite varied. Frankl describes those conditions in Man’s Search for Meaning. The book is rather short, but its contents are deep beyond comprehension. Frankl himself, however, might take exception to referring to his book as “deep.” Depth psychology was a term used for psychodynamically-oriented psychology. In 1938 Frankl coined the term **“height psychology”** in order to supplement, but not replace, depth psychology (Frankl, 1978).

After the war, Frankl’s life was nothing less than amazing. He returned to his home city of Vienna, married Eleonore Katharina, née Schwindt, and raised a daughter named Gabriele, whose husband and the Frankl’s grandchildren all lived in Vienna. He lectured around the world, received many honors, wrote numerous books, all while continuing to practice psychiatry and teach at the University of Vienna, Harvard, and elsewhere. He had a great interest in humor and in cartooning. Throughout his life, Frankl steadfastly refused to acknowledge the validity of collective guilt toward the German people. When asked repeatedly how he could return to Vienna, after all that happened to him and his family, Frankl replied:

…I answered with a counter-question: “Who did what to me?” There had been a Catholic baroness who risked her life by hiding my cousin for years in her apartment. There had been a Socialist attorney (Bruno Pitterman, later vice chancellor of Austria), who knew me only casually and for whom I had never done anything; and it was he who smuggled some food to me whenever he could. For what reason, then, should I turn my back on Vienna? (pp. 101-102; Frankl, 1995/2000)

Viktor Frankl died peacefully on September 2, 1997. He was 92 years old. During his life, his work influenced many people, from the ordinary to the famous and influential. “Viktor Frankl, to be sure, leaves a profound legacy” (pg. 24; Pattakos, 2004).

The Theoretical Basis for Logotherapy

While Frankl was in medical school, he considered specializing in dermatology or obstetrics. A fellow student who was aware of Frankl's wide-ranging interests, however, introduced Frankl to the works of Kierkegaard. This friend had been reminded of Kierkegaard's emphasis on living an authentic life, and he urged Frankl to pursue his interest in psychiatry. While still in medical school Frankl delivered a lecture to the Academic Society for Medical Psychology, of which Frankl was the founding vice-president, and used the term logotherapy for the first time (a few years later he first used the alternative term existential analysis; Frankl, 1995/2000). The word logos is Greek for "meaning," and this third Viennese school of psychotherapy focuses on the meaning of human existence and man's search for such a meaning. Logotherapy, therefore, focuses on man's **will-to-meaning**, in contrast to Freud's **will-to-pleasure** (the drive to satisfy the desires of the id, the pleasure principle) or Adler's **will-to-power** (the drive to overcome inferiority and attain superiority; adopted from Nietzsche) (Frankl, 1946/1986, 1946/1992).

The will-to-meaning is, according to Frankl, the primary source of one's motivation in life. It is not a secondary rationalization of the instinctual drives, and meaning and values are not simply defense mechanisms. As Frankl eloquently points out:

…as for myself, I would not be willing to live merely for the sake of my "defense mechanisms," nor would I be ready to die merely for the sake of my "reaction formations." Man, however, is able to live and even to die for the sake of his ideals and values! (pg. 105; Frankl, 1946/1992)

Unfortunately, one's search for meaning can be frustrated. This **existential frustration** can lead to what Frankl identified as a **noogenic neurosis**(a neurosis of the mind or, in other words, the specifically human dimension). Frankl suggested that when neuroses arise from an individual's inability to find meaning in their life, what they need is logotherapy, not psychotherapy. More specifically, they need help to find some meaning in their life, some reason to be. When reading Frankl's examples of how he helps such people, and Frankl offers many of these examples in his writings, it seems so simple. But it must be remembered that it takes a great deal of experience, knowledge, and maturity, as well as an ability to put oneself in another's shoes, in order to creatively think of how another person can find meaning in their life. It would be safe to say that many of us find it difficult to find meaning in our own lives, and research has indeed shown that the will-to-meaning is a significant concern throughout the world (Frankl, 1946/1992). In order to make sense of this problem, Frankl has suggested that we should not ask what we expect from life, but rather, we should understand that life expects something from us:

A colleague, an aged general practitioner, turned to me because he could not come to terms with the loss of his wife, who had died two years before. His marriage had been very happy, and he was now extremely depressed. I asked him quite simply: "Tell me what would have happened if you had died first and your wife had survived you?" "That would have been terrible," he said. "How my wife would have suffered?" "Well, you see," I answered, "your wife has been spared that, and it was you who spared her, though of course you must now pay by surviving and mourning her." In that very moment his mourning had been given a meaning - the meaning of a sacrifice. (pg. xx; Frankl, 1946/1986)

The latter point brings us back to Frankl's discussion of how one can find meaning in life: through creating a work or doing a deed; by experiencing something or encountering someone, particularly when love is involved; or by choosing one's attitude toward unavoidable suffering. Those of us who have lost someone dear know how easily it leads to deep suffering. Frankl had already written the first version of The Doctor and the Soulwhen he entered the Theresienstadt concentration camp, so his views on how one should choose their attitude toward unavoidable suffering were put to a test that no research protocol could ever hope to achieve! His observations form the basis for much of Man's Search for Meaning. Both his observations of others and his own reactions in this unimaginably horrible and tragic situation are quite fascinating:

…as we stumbled on for miles, slipping on icy spots, supporting each other time and again, dragging one another up and onward, nothing was said, but we both knew: each of was thinking his wife…my mind clung to my wife's image…Real or not, her look was then more luminous than the sun…A thought transfixed me: for the first time in my life I saw the truth as it is set into song by so many poets, proclaimed as the final wisdom by so many thinkers. The truth - that love is the ultimate and the highest goal to which man can aspire. Then I grasped the meaning of the greatest secret that human poetry and human thought and belief have to impart: The salvation of man is through love and in love. (pp. 48-49; Frankl, 1946/1992)

…One evening, when we were already resting on the floor of our hut, dead tired, soup bowls in hand, a fellow prisoner rushed in and asked us to run out to the assembly grounds and see the wonderful sunset. Standing outside we saw sinister clouds glowing in the west and the whole sky alive with clouds of ever-changing shapes and colors, from steel blue to blood red…Then, after minutes of moving silence, one prisoner said to another, "How beautiful the world could be!" (pg. 51; Frankl, 1946/1992)

…The experiences of camp life show that man does have a choice of action. There were enough examples, often of a heroic nature, which proved that apathy could be overcome, irritability suppressed. Man can preserve a vestige of spiritual freedom, of independence of mind, even in such terrible conditions of psychic and physical stress.

We who lived in concentration camps can remember the men who walked through the huts comforting others, giving away their last piece of bread. They may have been few in number, but they offer sufficient proof that everything can be taken from a man but one thing: the last of the human freedoms - to choose one's attitude in any given set of circumstances, to choose one's own way. (pp. 74-75; Frankl, 1946/1992)

Discussion Question: Frankl considered the most important aspect of survival to be the ability to find meaning in one's life. Have you found meaning in your life? Are there goals you have that you believe might add meaning to your life? Do you know anyone personally whose life seems to be filled with meaning, and if so, how does it appear to affect them?

Logotherapy as a Technique

Unfortunately, as noted by Frankl, not everyone can successfully accomplish the will-to-meaning. Those who rapidly declined toward death itself had lost the ability to have faith in the future; they could not identify any goal that provided meaning for their future. Such individual's exist in what Frankl called an existential vacuum. We have no instincts that tell us what we have to do, fewer and fewer traditions that tell us what we should do, and we

often don’t even know what we want to do. Therein lays the need for logotherapy. As a technique, logotherapy relies primarily on **paradoxical intention**and **dereflection** (Frankl, 1946/1986, 1946/1992). Paradoxical intention is based on a simple trap in which neurotic individuals often find themselves. When a person thinks about or approaches a situation that provokes a neurotic symptom, such as fear, the person experiences **anticipatory anxiety**. This anticipatory anxiety takes the form of the symptom, which reinforces their anxiety. And so on… In order to help people break out of this negative cycle, Frankl recommends having them focus intently on the very thing that evokes their symptoms, even trying to exhibit their symptoms more severely than ever before! As a result, the patient is able to separate themselves from their own neurosis, and eventually the neurosis loses its potency.

Similar to anticipatory anxiety, people often experience a compulsive inclination to observe themselves, resulting in hyper-reflection. For example, people who suffer from insomnia focus on their efforts to sleep, or people who cannot enjoy a sexual relationship often focus on their physical, sexual responses. Because of this intense focus on sleep, or having an orgasm, these very things are unattainable. In dereflection, patients are taught not to pay attention to what they desire. A person who cannot sleep might read in bed, they will eventually fall asleep. A person who cannot enjoy intimate sexuality could focus on their partner, and as a result they should experience satisfaction that they did not expect. In essence, whereas paradoxical intention teaches the patient to ridicule their symptoms, dereflection teaches the patient to ignore his or her symptoms (Frankl, 1946/1986).

Discussion Question: Logotherapy relies on paradoxical intention and dereflection to break the anticipatory anxiety that often leads to failure (and then, more anxiety). Are there situations where you find yourself getting anxious or nervous even before the situation begins? What steps, if any, have you taken to break out of that pattern?

The Search for Ultimate Meaning

Kierkegaard believed that man could never truly be in contact with the infinite and absolute God. Similarly, Frankl talked about a **super-meaning** to life, something that goes far deeper than logic. When Frankl told his daughter that the goodLord had cured her measles, his daughter reminded him that the goodLord had given her the measles in the first place. Since children may not benefit from the challenges of suffering as adults might, and even adults find it difficult to find meaning in truly horrible situations like the concentration camps of Nazi Germany or the gulags of the former Soviet Union, the meaning of life in the greater context of human societies is often not readily apparent. But as Frankl says:

This ultimate meaning necessarily exceeds and surpasses the finite intellectual capacities of man; in logotherapy, we speak in this context of a super-meaning. What is demanded of man is not, as some existential philosophers teach, to endure the meaninglessness of life, but rather to bear his incapacity to grasp its unconditional meaningfulness in rational terms. Logos is deeper than logic. (pg. 122; Frankl, 1946/1992)

In discussing the value of logotherapy, Frankl offered critiques of other popular fields in psychology and psychiatry. His most serious critique was of the deterministic nature of psychoanalysis. Frankl fervently believed in an individual’s freedom to transcend their self and choose to make the best of any situation. And he had plenty of experience to back up his opinion: “…I am a survivor of four camps - concentration camps, that is - and as such I also bear witness to the unexpected extent to which man is capable of defying and braving even

the worst conditions conceivable" (pg. 47; Frankl, 1978). He did not, however, reject determinism entirely. Instead, he attributed determinism to the psychological dimension, whereas freedom exists within the noölogical dimension. He acknowledged Freud and Adler for teaching us to "unmask the neurotic." As for behaviorism, Frankl acknowledged that it helped to "demythologize" neurosis, by pointing out that not every psychological problem is due to unconscious forces from early childhood, and he included Pavlov, Watson, and Skinner as great pioneers. Still, both psychoanalysis and behaviorism ignore the essential humanness of the individual.

Despite his emphasis on the individual human, Frankl did not consider logotherapy as belonging within humanistic psychology (or at least not within what he called pseudo-humanism; Frankl, 1978). He believed that humanistic psychology focused so much on the humanity of individuals, that they did not quite appreciate the uniqueness of each person. It is not enough to merely encounter another person. In order to be moved on the personal level there must be an element of love (love for another person, if not a more intimate and personal love as for a spouse or a child). As we will see below, Rollo May also considered love to be of great importance to our lives. The emphasis that Frankl placed on love may have something to do with his deep spirituality. Frankl believed in a spiritual unconscious, separate from the instinctual unconscious described by Freud (Frankl, 1948/2000). In order for an individual to experience an authentic existence, they must determine whether a given phenomenon (thoughts, feelings, impulses, etc.) is instinctual or spiritual, and then freely choose how to behave or respond. Frankl returned to Heidegger's concept of Dasein, living according to the understanding that one is connected to Being. Although this concept may seem reminiscent of Jung's collective unconscious, nothing could be further from the truth:

…It cannot be emphasized strongly enough that not only is the unconscious neither divine nor omniscient, but above all man's unconscious relation to God is profoundly personal. The "unconscious God" must not be mistaken as an impersonal force operant in man. This understanding was the great mistake to which C. G. Jung fell prey. Jung must be credited with having discovered distinctly religious elements within the unconscious. Yet he misplaced this unconscious religiousness of man, failing to locate the unconscious God in the personal and existential region. Instead, he allotted it to the region of drives and instincts, where unconscious religiousness no longer remained a matter of choice and decision. According to Jung, something within me is religious, but it is not I who then is religious; something within me drives me to God, but it is not I who makes the choice and takes the responsibility. (pg. 70; Frankl, 1948/2000)

According to Frankl, the most human of all human phenomena is the will-to-meaning. Religion, or spirituality, seeks a **will-to-ultimate-meaning**. Once again, Frankl believed that Freudian psychoanalysis and Adlerian individual psychology had failed to sufficiently credit the self-transcendent quality of individuals who live authentic lives. It is in the study of authentic lives that "height psychology," as Frankl called it, can address the higher aspirations of the human psyche. In other words, beyond seeking pleasure and/or power, there is man's search for meaning (Frankl, 1948/2000).

Discussion Question: Frankl was a very spiritual man. He talked about super-meaning and a will-to-ultimate-meaning. Are you a spiritual and/or religious person? If yes, does your faith help to give meaning to your life?

Rollo May and Existential Psychology

Rollo May (1909-1994) introduced existentialism to American psychologists, and has remained the best known proponent of this approach in America. Trained in a fairly traditional format as a psychoanalyst, May considered the detachment with which psychoanalysts approached their patients as a violation of social ethics. For example, if a psychoanalyst helps a patient to be the best they can be, and the person happens to earn their living in an unseemly or criminal way, it hardly seems proper (Stagner, 1988). On the other hand, who is to decide which values should be preferred in a particular society? In the pursuit of freedom, May suggested that sometimes individuals might reasonably oppose the standards or morality of their society. Politics, a wonderful topic for lively debates, is dependent on opposing viewpoints. Only when an individual lives an authentic life, however, should their opinion be considered valid, and existential psychology seeks to help individuals live authentic lives.

A Brief Biography of Rollo May

Rollo Reese May was born on April 21, 1909, in Ohio, and grew up in Marine City, Michigan. He attended Oberlin College in Ohio, graduating in 1930. Having always been interested in art and artistic creativity, he joined with a small group of artists and traveled to Europe, where they studied the local art of Poland. In order to remain in Europe, May took a teaching position with the American College at Salonika in Greece. When not teaching, he traveled widely throughout Greece, Poland, Romania, and Turkey. He attended the summer school taught by Alfred Adler. Deeply impressed by Adler (as Frankl had been), he nonetheless considered Adler’s theories overly simplistic and too general. This may well have been due to his awakening awareness of the tragic side of human life, keeping in mind that much of Europe suffered greatly during the depression between World War I and World War II (Reeves, 1977).

Upon returning to the United States, May worked as a student advisor and the editor of a student magazine at Michigan State University. In 1936, he enrolled at Union Theological Seminary in New York, with the intention of asking, and most likely hoping to find answers to, the ultimate questions about human life. Despite having no particular desire to become a minister, he did serve in a parish in Montclair, New Jersey for a while. While at the seminary, he became a lifelong friend of Paul Tillich, a well-known existential theologian. Tillich, whose classes May regularly attended, introduced May to the works of Kierkegaard and Heidegger. May also met Kurt Goldstein during this time, and became acquainted with Goldstein’s theories of self-actualization and anxiety as a reaction by organisms to catastrophic events. Regarding his time as a minister, May reflected that the only events which seemed to include an element of reality were the funerals (Reeves, 1977).

Shortly after graduating from the seminary, May began writing books on counseling and creative living. He worked as a counselor at the College of the City of New York, and trained as a psychoanalyst at the William Alanson White Institute of Psychiatry, Psychoanalysis, and Psychology in New York. His time at the training institute overlapped with Harry Stack Sullivan being the president of the William Alanson White Foundation, and Erich Fromm as a fellow associate. In 1946, May began a private practice in psychoanalysis, in 1948 he became a faculty member at the institute, and in 1949 he received the first Ph.D. in clinical psychology at Columbia University. His doctoral dissertation was published as The Meaning

of Anxiety(May, 1950), a book that heavily cites the work of Freud and Kierkegaard on anxiety, as well as Fromm, Horney, and Tillich (May, 1950; Reeves, 1977).

Similar to Viktor Frankl, May's life had taken a dramatic turn during this time, an uncontrollable event that threatened his life: May contracted tuberculosis. At the time, there were no effective treatments for this contagious disease, many people died from it, and like many others May had to spend several years at a sanitarium (Saranac Sanitarium in upstate New York). It was during his time in the sanitarium that May theorized about anxiety and came to one of the most important conclusions in his career. He determined that although Freud had done a masterful job of characterizing the effects of anxiety on the individual, it was Kierkegaard who had truly identified what anxiety is: the threat of becoming nothing. From this point on May could clearly be identified as an existential psychologist. He collaborated with Abraham Maslow, Carl Rogers, and Gordon Allport to present a symposium on existential psychology, in conjunction with the 1959 annual convention of American Psychological Association, which led to the publication of a book on the subject (Reeves, 1977).

As May's career continued, he became a supervisory and training analyst at the William Alanson White Institute, and an adjunct professor of psychology in the graduate school at New York University. He gave a series of radio talks on existential psychology on a Canadian Broadcasting Corporation show, he served as a visiting professor at Harvard and Princeton, and he continued writing. His later books include works on dreams, symbolism, religion, and love. He eventually settled in California, where he died in 1994.

Anxiety

May considered **anxiety** to be the underlying cause of nearly every crisis, whether domestic, professional, economic, or political. He described the world we live in as an age of anxiety. Even though May published The Meaning of Anxiety in 1950, it is safe to say that his concerns are even more relevant today, particularly with the advent of the depersonalization of our world due to the computer age (Reeves, 1977). May considered a wide range of theories on anxiety, including philosophers, neurologists (Kurt Goldstein), and the major psychodynamic theorists (including Freud, Adler, Jung, Horney, Sullivan, and Fromm). He came to the conclusion that Freud had done the best job of explaining anxiety, but it was Kierkegaard who best understood anxiety. May was particularly impressed by Kierkegaard's idea that anxiety must be understood in the context of an orientation toward freedom. Freedom is the goal of personality development, and although this freedom brings with it anxiety, it is through facing this anxiety that the possibility of freedom arises (May, 1950). In praise of Kierkegaard, May wrote:

…Kierkegaard is proclaiming that "self-strength" develops out of the individual's successful confronting of anxiety-creating experiences; this is the way one becomes educated to maturity as a self. What is amazing in Kierkegaard is that despite his lack of the tools for interpreting unconscious material - which tools have been available in their most complete form only since Freud - he so keenly and profoundly anticipated modern psychoanalytic insight into anxiety; and that at the same time he placed these insights in the broad context of a poetic and philosophical understanding of human experience. (pg. 45; May, 1950)

In defining anxiety, May distinguished between anxiety and fear, and between **normal anxiety** and **neurotic anxiety**. According to May, "anxiety is the apprehension cued off by a

threat to some value which the individual holds essential to his existence as a personality" (pg. 191; May, 1950). The threat may be either physical or psychological, such as facing death from tuberculosis or being imprisoned in a concentration camp (which, of course, brought the threat of death in addition to the loss of freedom), or the threat may challenge some other value that the individual identifies with their existence or personal identity (such as the loss of a career, a divorce, a challenge to patriotism in time of war, etc.). What differentiates anxiety from fear, is that fear is a reaction to a specific event, whereas anxiety is vague and diffuse. For example, during a robbery you may fear a man with a gun, but in America today many people are anxious about terrorism. No one can tell when or where terrorists may strike, or even whether they will be foreign terrorists (such as in the World Trade Center attacks) or American terrorists (such as the bombing of the federal building in Oklahoma City or the D.C. sniper killings). May carefully pointed out that using the terms "vague" and "diffuse" to describe anxiety should in no way diminish our understanding of the intensity and painfulness that anxiety can bring. Therein lies the difference between normal vs. neurotic anxiety (May, 1950).

Everyone faces challenges in life, but not everyone sees the same challenges as actual threats. Losing one's job can be an opportunity to begin a new career, perhaps to go back to school to pursue that new career. However, the transition is often difficult, especially when one is used to being the primary wage earner in the family, and also if the family has to cut back on items they can no longer afford. So anxiety would be a reasonable reaction. That anxiety is considered normal if it is 1) not disproportionate to the objective threat, 2) does not involve mechanisms of intrapsychic conflict, and 3) does not require defense mechanisms for its management (May, 1950). Normal anxiety is often overlooked in adults since it is not particularly intense, especially compared to neurotic anxiety, and it can be managed constructively. It does not show itself in panic or other dramatic symptoms. Neurotic anxiety is, simply, the opposite of normal anxiety. It is disproportionate to the objective threat, it does require intrapsychic defense mechanisms, and it results in neurotic symptoms in spite of those defense mechanisms. It is important to keep in mind that we should not consider individuals who suffer from neurotic anxiety as suffering from objective weaknesses, but rather they suffer from inner psychological patterns and conflicts that prevent them from using their powers to cope.

True to his training in psychodynamic theory, May believed that the psychological patterns resulting in the inability to cope have their origin in childhood, particularly due to poor early relations between the infant and its parents, since an infant's essential values arise from the security patterns established between the infant and its caregivers (as in Erikson's first psychosocial crisis: trust vs. mistrust, see Chapter 7). One of the most important factors seems to be the infant's subjective interpretation of rejection by its primary caregiver, and that subjectivity is influenced by expectations that form later in life (e.g., middle- and upper-class children, who expect more support from their parents, are especially prone to react to rejection with neurotic anxiety; May, 1950).

Discussion Question: May felt that we must understand anxiety in relation to freedom, or rather, as the fear that we will lose our freedom. He said that some of this anxiety is normal, and only in extreme cases does it become neurotic anxiety. What are some of the situations in your life that make you anxious, and how might they be a threat to your personal freedom? Do you think the level of these anxieties is normal, or is it severe enough to perhaps be considered neurotic?

Culture, Anxiety, and Hostility

May also addressed the effects of culture on anxiety, and the close interrelationship between anxiety and **hostility**. Culture affects both the kinds and the quantities of anxiety experienced by individuals. Beyond the essential relationship between infant and caregiver, the determinants of personality that each of us consider essential to our existence as a personality are largely cultural. Indeed, even the nature of the infant/caregiver relationship is subject to cultural influence. The amount of anxiety most people are likely to experience is determined, in part, by the stability of the culture. For example, if a culture is relatively stable and unified, there will be less anxiety throughout that culture (May, 1950). Today, however, many societies are in dramatic flux, due in large part to the powerful trend toward globalization.

As psychologists have begun to examine anxiety in different groups around the world, a variety of interesting, and sometimes disturbing, results have been found. Keep in mind, however, that these are generalities, and do not necessarily apply to each individual within any group. Generally, Asians are more anxious than Europeans and White Americans, who are more anxious than Black Americans and Africans, and there may be a neurological basis for these relative anxiety levels (Rushton, 1999). However, when looking at the specific form of anxiety related to taking academic tests, Black Americans and Chilean students demonstrate higher levels of test anxiety than White Americans (Clawson, Firment, & Trower, 1981; Guida & Ludlow, 1989). One suggestion for the higher levels of anxiety among Blacks in America is that our society is much less sociocentric than most African cultures. Thus, Blacks in America, even if they have lived here for generations, still experience the effects of their displacement from Africa when the culture they carried with them is at odds with Western cultural expectations (Okeke at al., 1999), and even more so when an individual seems to be at odds with most members of their own cultural group (Copeland, 2006). Indeed, the greater the discrepancy between one’s individual cultural expectations and the cultural expectations of the majority of society, the greater the anxiety an individual experiences. This is particularly true during attempts at intercultural communication (Matsumoto & Juang, 2004). Any subsequent breakdown of intercultural communication, which is more likely during periods of high anxiety, can either lead to or enhance pre-existing hostility, prejudice, discrimination, and scapegoating (Whitley & Kite, 2006). One important challenge to intercultural communication in psychology is the need for clinical psychologists to recognize the growing number of anxiety disorders unique to non-Western cultures, such as: **hwa-bung** (Korea), **koro** (Malaysia and Southern China), **nervios** (Latin America), **dhat syndrome** (India), **susto** (Latin America), and **taijin kyofusho**(Japan) (Castillo, 1997).

Culture can influence individuals in a wide variety of ways. May (1950) used the example of competitive individual success in the Western world as his main example, which he considered to be the dominant goal in America. There are many negative effects of this competition, including the high incidences of gastric ulcers and heart disease in our society. Less than a decade later, Freidman and Rosenman (1959) published their classic study on the relationship between Type A behavior (studied in highly competitive businessmen) and cardiovascular disease. Subsequent studies have shown that the key component of Type A behavior predictive of heart disease is hostility, which we will discuss in more detail below (Dembrowski et al., 1985; Lachar, 1993; MacDougal et al., 1985). There has also been a great deal of discussion in our society about media influences on body image, the

relationship between unreasonable expectations for women to be thin and the incidence of eating disorders in girls and women, and the repression of female sexuality in many cultures. Goldenberg (2005) recently presented an existential perspective on the body itself as a threat. Cultural beliefs often help to overcome fears of mortality by convincing individuals that they are of greater value than other, lower animals. However, despite the beliefs of many that only humans have a soul, our body is still a mortal animal. As a reaction to the anxiety presented by the reality of our mortal body, many people act in a hostile fashion toward their own bodies, ranging from denying themselves healthy physical relationships with others (e.g., sexual repression) to outright self-destructive behavior (e.g., anorexia nervosa). The problem reaches its extreme, however, when one powerful group directs its hostility in an organized fashion toward another group.

The relationship between anxiety and hostility, according to May, involves a vicious circle. Anxiety gives rise to hostility, and hostility gives rise to increased anxiety. But which comes first? May believed that it was anxiety that underlies hostility, and the evidence can be found in clinical cases involving repressed hostility:

Granted the interrelation between hostility and anxiety, which affect is generally basic? There is ground for believing that, even though hostility may be the specific affect present in many situations, anxiety is often present below the hostility…For one example, in some of the psychosomatic studies of patients with hypertension…it has been found that the reason the patients repressed their hostility was that they were anxious and dependent...The hostility would not have to be repressed in the first place except that the individual is anxious and fears counter-hostility or alienation… (pg. 223; May, 1950)

In Reeves' analysis of May's theory (1977), Reeves discusses one of the most important social issues to have faced the United States: the civil rights movement of the 1960s. When an individual's sense of selfhood is challenged by dramatic changes in society, it can be a very painful experience. And one is likely to resent those responsible for those changes. While it is true that many White people in America supported the civil rights movement, White people in the Deep South (and elsewhere, of course) turned their anxiety, and its associated hostility, toward Blacks. It should not be necessary here to describe the many terrible acts of violence that followed. Suffice it to say that the federal government had to use military troops to intervene in some of the worst cases. Today, we face a similar problem in the war on terrorism. Given the often unequal and unfair manner in which globalization brings vastly different cultures into conflict, and the ease with which so many people can travel the globe, perhaps we should not be surprised at the dramatic level of terrorism in the world today.

Connections Across Cultures: Terrorists and Terrorism

Since September 11, 2001, when agents of the terrorist organization Al Qaeda destroyed the World Trade Center in New York City and killed some 3,000 people, the United States has been involved in what has been called an international war on terrorism. As the war on terrorism developed, it had two main goals: to capture Osama bin Laden, leader of Al Qaeda and mastermind of the World Trade Center bombings, and to overthrow Saddam Hussein, the dictator of Iraq (for his alleged role in supporting international terrorism). To date, this war has lasted much longer than World War II, we have spent hundreds of billions of dollars, and thousands more young American men and women have died fighting in Afghanistan and Iraq. Many Iraqi and Afghan civilians, as well as additional coalition military personnel, have

also died. Saddam Hussein was removed from power in Iraq; he was also tried, convicted, and executed. It took nearly 10 years, but Osama bin Laden was finally tracked down and killed in a raid in Pakistan by U.S. Navy Seals. However, Al Qaeda is still committing acts of terrorism, Iraq is descending once again into bitter sectarian violence (rising to the level of civil war), and Americans continue to die fighting in Afghanistan as our intended date for withdrawal slowly draws near (after 13 years!). One thing that will not be addressed in this section, because it does not exist, is an easy answer to these problems.

Please allow me to share a little personal history here. When the Iranian revolution under Ayatollah Khomeini overthrew the Shah of Iran, and the revolutionaries captured the American embassy in Tehran and took sixty-six people hostage (fifty-two of those hostages were held for well over a year before being released), I was in the United States Marine Corps Reserve. I received a phone call at 2:00 a.m. on a Friday morning at my apartment in Cambridge, MA. By midnight, that same day, my reserve unit was in Camp Lejeune, NC, with full combat gear, ready to go to war in Iran. We spent the weekend preparing, though President Carter ultimately chose not to send us overseas. Approximately 10 years later, when the first Gulf War erupted after Iraq invaded Kuwait, my sister took part in Operation Desert Storm. As an Air Force nurse, she was sent to England to help prepare a hospital for wounded military personnel being evacuated from the Middle East (fortunately casualties were minimal). I considered re-enlisting in the Marine Corps at that time, since I certainly wasn't going to sit at home while my own sister "fought" for our country and our allies. Thankfully, that first Gulf War was brief and, seemingly, simple. So I have followed events in the Middle East carefully ever since, and when Al Qaeda attacked us in New York, I saw it as the latest in a continuation of events in my own life since 1979. For people in the Middle East, however, it was a continuation of events that have lasted for thousands of years.

What I believe matters most for Americans today is to begin to make an honest effort to understand terrorism, its causes, its goals, and how best to deal with it around the world. First, we must dispense with misconceptions. Terrorism and Islam are **not** one and the same. In an insightful and easily readable book entitled Islam versus Terrorism, Firooz Zadeh (2002) discusses how Islam opposes violence and murder, especially of innocent women and children. He also attempts to identify what is and is not terrorism, and in that effort he identifies eight types of terrorism: state terrorism, religious terrorism, criminal terrorism, terrorism by those who are mentally sick, political terrorism, oppositional terrorism, copy cat terrorism, and victim terrorism. According to Zadeh, the highest cost to society results from state terrorism. When the United States supports corrupt, terrorist governments in other parts of the world, our credibility as a nation fighting terrorism is suspect at best. Has this been the case? Yes, and in the worst possible way: we switch sides as it serves our political and economic interests. The United States helped to train Osama bin Laden and the Taliban fighters in Afghanistan when we wanted them to fight the Russians. Now we call them enemies. We provided weapons and training to Saddam Hussein's army when they were fighting the Iranians, because of the hostages taken in Tehran. Now we have deposed Hussein. We also sold weapons to Iran, and used the money to help support the Contras (freedom fighters or terrorists, depending on your point of view) trying to overthrow the leftist Sandinista government of Nicaragua. Zadeh proposes that people in the Middle East cannot trust the United States, except in one area: our support of Israel. And since other Middle Eastern countries see Israel as the one obstacle to a Palestinian homeland, they disapprove of that support. It does not matter whether the

actions of the United States were right or wrong, whether they really were in our best interests or not. What matters is how the rest of the world sees us now, and whether our top government officials are willing to consider how we are viewed globally and to act responsibly in terms of foreign policy in order to ensure what is best for all people around the world. In addressing the Middle East in particular, Fathali Moghaddam wrote:

Islamic communities in many parts of the world are experiencing a profound and historic identity crisis, one tragic manifestation of which is terrorism. In order to understand and avert this destructive trend, we must come to grips with the monumental crisis of identity that is paralyzing moderate movements but energizing fanatic forces in Islamic communities.

…Why do we need to understand how the terrorists see the world? Because this is the best way for us to find an effective means to end terrorism…Seeing the world from the terrorists' point of view does not mean condoning terrorism; rather, it means better understanding terrorism so as to end it. (pg. ix; Moghaddam, 2006)

As mentioned above, there are many different forms of terrorism, so it is difficult to define exactly what it is. Nevertheless, in an effort to do so, Moghaddam (2005) defines terrorism as "politically motivated violence, perpetrated by individuals, groups, or state-sponsored agents, intended to instill feelings of terror and helplessness in a population in order to influence decision making and to change behavior." Moghaddam suggests that psychologists need to play an important role in understanding terrorism for two main reasons: the basis for terrorist actions is typically subjectively interpreted values and beliefs, and the actions of terrorists are designed to cause specific psychological experiences, i.e., terror and helplessness. Moghaddam (2005, 2006) proposes a metaphor for how one becomes a terrorist, based on climbing a staircase, in which options are perceived to become more and more limited as one climbs the stairs. The most significant factor is the condition in which many people live on the ground floor, before they even consider climbing that staircase. Many people in this world live in abject poverty, under repressive governments that are unjust. When individuals see no hope within the system, and they lack any political means to effect change, then a path toward terrorism becomes perhaps the only reasonable possibility. Still, very few people are likely to become suicide bombers.

Individuals living in desperate conditions may move to the first floor on the staircase toward terrorism, where they evaluate their perceived options to fight unfair treatment. If there appear to be no options for justice within one's society, no opportunity to be heard, and no opportunity for personal mobility, the individual may then move to the second floor. Here the individual begins to displace their aggression. This often involves education/propaganda that identifies a clear target, for example the United States, also known as the "Great Satan." This is the important beginning of an us-versus-them mentality. On the third floor, individuals become morally engaged with the terrorist organization. While we may see terrorists as immoral, they are beginning to believe that they are fighting for a just cause, against the immoral repression of their chosen target. As they move to the fourth floor, they solidify their categorical thinking (the us-versus-them mentality) and begin to see the terrorist organization, and terrorist acts, as legitimate. At this point there is little chance that they can leave the terrorist organization alive. For specific individuals, the training necessary to carry out a terrorist act takes place, often very quickly. Not only does a terrorist need to learn about weapons and tactics, they must also be trained to sidestep the

natural, biological inhibition against killing other human beings. Two factors in helping to prepare people to kill are the intense indoctrination in the belief that their actions are for a greater good and secrecy. If an attack is done suddenly and without warning, victims have no opportunity to submit or to beg for mercy. The act occurs before the terrorist might become compassionate as he or she faces their intended victims (Moghaddam, 2005, 2006). Based on this model, Moghaddam proposes four steps that are necessary to stop terrorism by interrupting the formation of new terrorists. First, there must be prevention. Unfortunately, our government has a long history of choosing short-term fixes, rather than long-term preventative measures. Case in point: America's failure in the war on drugs. Aggressive responses aimed at individuals only provide an opening for someone new to step in and continuing using and/or selling drugs, and the same is true of terrorists. We need to work toward eliminating the pathway to terrorism, so we will not need to use the military and/or FBI to track down individuals (except, of course, in extreme cases such as terrorism that results from psychological disorder - e.g., consider the case of Theodore Kaczynski, the Unabomber). In addition, Moghaddam suggests supporting contextualized democracy, educating against categorical thinking, and promoting interobjectivity and justice. In order for there to be a long-term solution, there must be international dialogue and improved intercultural understanding (Moghaddam, 2005).

Returning to the misconception in the minds of many Americans that terrorism is synonymous with Islam, let's examine where known terrorist organizations are located around the world. Fairly notorious organizations have come from Northern Ireland (e.g., the Irish Republican Army and the Ulster Defense Association), throughout mainland Europe (e.g., the Red Army Faction in Germany, the Red Brigades in Italy, and Action Directe in France), throughout the Middle East (e.g., Hezbollah in Lebanon, the Palestinian group Hamas, the Stern Gang that fought for the establishment of Israel, and Al Qaeda), Africa, Asia, Latin America, Canada, and the United States (e.g., the Animal Liberation Front, Aryan Nations, the Black Panthers, and the Ku Klux Klan). As of 1999, at least twenty-eight well-organized terrorist groups existed, and when one takes into account factions within those groups and smaller, yet still identifiable, groups, as many as eighty-three terrorists groups have been identified around the world (Henderson, 2001). Some are primarily political, and some are primarily religious. Some are global, and some are more local. They include people and cultures of great diversity: Black, White, Asian, Latin, Catholic, Protestant, Muslim, Hindu, etc. Although terrorism appears to arise out of poverty and desperation, terrorists themselves, or at least the leaders, tend to be better educated than most and they are well versed in propaganda and well trained in weapons and tactics (Moghaddam, 2005; Zadeh, 2002). The only characteristic that all terrorists seem to share is an extreme commitment to violence, which arises out of desperation and perceived injustice, and is viewed as the only means to be heard and to effect change.

So can terrorism effect change, is terrorism effective? One can easily find authors who argue that it does indeed work (Dershowitz, 2002) or that it always fails (Carr, 2002). Alan Dershowitz (2002) argues that the very reason terrorism works is everything we have looked at so far: an effort to understand the root causes of terrorism and the terrorists themselves. Accordingly, he says:

We must take precisely the opposite approach to terrorism. We must commit ourselves never to try to understand or eliminate its alleged root causes, but rather to place it beyond the pale of dialogue and negotiation. Our message must be this: even if you have legitimate

grievances, if you resort to terrorism as a means toward eliminating them we will simply not listen to you, we will not try to understand you, and we will certainly never change any of our policies toward you. Instead, we will hunt you down and destroy your capacity to engage in terror. (pp. 24-25; Dershowitz, 2002)

As a case in point, Dershowitz cites the awarding of observer status at the United Nations to the Palestinian Liberation Organization only after Palestinian terrorists began hijacking commercial airliners. Prior to the hijackings, 20 years of pleading their case to the United Nations had little effect. Dershowitz then offers a timeline that appears to clearly establish an effective relationship in which terrorism became more and more effective over time (from 1968-1999) in eliciting international recognition and support for the Palestinian cause. In contrast, Caleb Carr (2002) views terrorism entirely within the discipline of military history. He considers today's terrorism to be nothing more than a modern permutation of warfare against civilians in order to break their support for either leaders or policies that the terrorists oppose, the origins of which are as old as human conflict itself. Viewing terrorism as warfare has certain interesting implications. Throughout history, those who wage war against civilians ultimately defeated themselves by turning sentiment against them. On 9/11, Al Qaeda attacked civilians to a degree that has not been seen in ages:

…In so doing, the organizers, sponsors, and foot soldiers of every terrorist group involved in the September 11 attacks have unwittingly ensured that their extremist cause will be discredited among many of their sympathizers, disowned by most of their former sponsors, and finally defeated by their enemies: two thousand years of the lessons of terror dictate that this is the ultimate fate that awaits the attackers, no matter how many noncombatants they manage to kill along the way. (pp. 223-224; Carr, 2002).

Carr also addresses the other most important implication of treating terrorism as warfare: it must be met with warfare, but that warfare must not be excessive, such that it might also be viewed as terrorism. If our response to terrorism is excessive military might, then the tide of public opinion can swing back in favor of Al Qaeda, especially in Muslim countries where the United States is not trusted.

Echoing Carr's concerns about the extent and nature of our military actions in the war on terror, one way in which terrorism might work against us, without seeming to have gained what was intended (if we can even know what was intended), is if our fundamental democratic principles change. In The Lesser Evil: Political Ethics in an Age of Terror, Michael Ignatieff (2004) argues that terrorism must be met with force, and that such force is a lesser evil than the terrorism that necessitated the response. The danger lies in succumbing to the greater evil of seeking revenge. Dershowitz (2002) provides a compelling case for how an amoral society could control and possibly eliminate all terrorism, but America is not an amoral society. Our responses are constrained by the constitution and by the political debate that forms the very basis of our democracy. When we respond to terrorist acts, we must consider what we want that response to accomplish:

Terrorism requires us to think carefully about who we are as free peoples and what we need to do in order to remain so. When we are confronted with terrorist violence, we cannot allow the claims of national security to trump the claims of liberty, since what we are trying to defend is our continued existence as a free people. Freedom must set a limit to the measures we employ to maintain it. (pg. 145; Ignatieff, 2004)

Finally, can the ultimate answer to terrorism be found in promoting democratic governments in every nation? The war on terror has led us to depose both Saddam Hussein in Iraq and the Taliban in Afghanistan, and to replace them with democratically elected governments. Only time will tell whether those governments will survive, but there is reason for caution. Religious turmoil continues in the Middle East. In America, our constitution provides for separation of church and state, and that separation has become an important tradition. But for Muslims, the idea of a secular democracy, one that is not guided by Allah, is simply inconceivable. They are not opposed to democracy per se, indeed it has been argued that Islam is likely to eventually lead to pluralist democracies (Aslan, 2005). But to pressure Islamic countries into accepting the secular democracy that we hold so dear is, according to Robert Shedinger (2004), equivalent to declaring war on Islam. So what appears to be essential to promoting stability in the Middle East, and elsewhere, is an effort to support contextual democracy, that is, forms of democracy that fit with the culture of the people who will create and participate in that democracy (Aslan, 2005; Moghaddam, 2005, 2006; Shedinger, 2004; Zadeh, 2002).

Integration and the Human Dilemma

In the preface to Man's Search for Himself (May, 1953), May presents the existential philosophy that there is meaning to be found in challenges and suffering, and that psychologists in particular may find a special opportunity in such circumstances:

When our society, in its time of upheaval in standards and values, can give us no clear picture of "what we are and what we ought to be,"…we are thrown back on the search for ourselves. The painful insecurity on all sides gives us new incentive to ask, Is there perhaps some important source of guidance and strength we have overlooked?…How can anyone undertake the long development toward self-realization in a time when practically nothing is certain, either in the present or the future?…The psychotherapist has no magic answers…But there is something in addition to his technical training and his own self-understanding…This something is the wisdom the psychotherapist gains in working with people who are striving to overcome their problems. He has the extraordinary, if often taxing, privilege of accompanying persons through their intimate and profound struggles to gain new integration. (pg. 7; May, 1953)

Integration, according to May, is similar to Heidegger's concept of Dasein (being-in-the-world). As conscious, free, and responsible beings our goal should be to separate ourselves from the conformist, automaton masses (the en-soi, according to Sartre) and progressively integrate with others in freely chosen love and creative work (May, 1953), or as Clement Reeves puts it: "To understand and elucidate the specific, distinguishing characteristics of the human being, and to grasp what it is to achieve courageous, decisive, integrated response to the challenge inherent in existence…" (Reeves, 1977). The process of integration is lifelong, and should be appropriate for whatever age each one of us happens to be right now. May suggests that a healthy child of eight, who is fulfilling his capacity of self-conscious choice for a child of eight years old, is more of a person than a neurotic adult who is 30 years old. Likewise, a person who can face death courageously at the age of thirty is more mature than someone 80 years old who "cringes and begs still to be shielded from reality" (May, 1953). Thus, it is important to live each moment with freedom, honesty, and responsibility. If each of us lives within the present moment, working to fulfill our potential,

being true to whom we are and the situations within which we live, May proposes that we will experience joy and gratification:

…Does not the uncertainty of our time teach us the most important lesson of all - that the ultimate criteria are the honesty, integrity, courage and love of a given moment of relatedness? If we do not have that, we are not building for the future anyway; if we do have it, we can trust the future to itself. (pg. 276; May, 1953)

One of the challenges to living an integrated life is seen in what May described as the **human dilemma** (May, 1967). Are we the subject of our lives, or are we an object in our world? When we become absorbed in the details of our responsibilities and actions, when we allow ourselves to be controlled and directed in order to accomplish our assigned tasks, when we become slaves to the clock, doing this and that, going here and there, as others expect us to, we are viewing ourselves as objects. This is reminiscent of what Karen Horney called the tyranny of the should. On the other hand, when we consider our feelings, wishes, and desires, when we are true to ourselves, or living authentically, then we are viewing ourselves as subjects, as active participants in our own lives. According to May (1967), the human dilemma arises out of our capacity to experience ourselves as both subject and object at the same time. But how can opposite poles of the human experience both be true? It is in the process between the two poles that development of human consciousness develops, both deepening and widening that consciousness. This is essentially the same idea, though in different form, used by Heidegger and Sartre in describing the unique nature of human beings. For Heidegger this nothingness was the undefined distinction between Being and beings, for Sartre it was the shell that surrounded the pour-soi.

May believed that existential psychology occupied a space somewhere between the two extremes that existed, and continue to exist, in psychology: behaviorism vs. humanism. May rejected Skinner’s arguments that all human behavior can be understood in terms of stimuli and responses, declaring that there is ample evidence in both clinical practice and everyday life of people being active participants in their view of, actions in, and reactions to their world. He was equally critical of Carl Rogers, believing that humanistic psychologists no longer recognized very real irrational behavior, as well as aggression and hostility (May, 1967). He believed that psychology had become trapped in a misguided desire to define everything scientifically, and according to rules that then determined each psychologist’s view of the world and their patients. As a caution to those psychologists who cannot see beyond their theories, May wrote:

Now I am certainly aware, if I may say so without sounding patronizing, that the compelling need for honesty is one of the motives which leads psychologists to seek quantitative measures…I am also aware that research in our day has to be carefully set up so that the results are teachable and can be built upon by others. The compelling drive to get at the truth is what improves us all as psychologists, and is part and parcel of intellectual integrity. But I do urge that we not let the drive for honesty put blinders on us and cut off our range of vision so that we miss the very thing we set out to understand - namely, the living human being. (pg. 14; May, 1967)

Discussion Question: May suggested that we need to separate ourselves from the conformist masses, and then integrate ourselves with others in free and responsible ways. Are you a follower, or a leader? Either way, do you consciously choose the role you play, thereby living an authentic life?

Love and Intentionality

Lovewas a very important topic for May. Simply put, “To be capable of giving and receiving mature love is as sound a criterion as we have for the fulfilled personality” (May, 1953). He was certainly not alone. Harry Harlow, best known for his studies on **contact comfort**, described love as “a wondrous state, deep, tender, and rewarding,” and Abraham Maslow said “We must understand love; we must be able to teach it, to create it, to predict it, or else the world is lost to hostility and to suspicion” (Harlow, 1975; Maslow, 1975). However, there are “a million and one” types of relationships that people call love, so it remains a perplexing issue (May, 1953).

May talked about four types of love in Western tradition: **sex, eros, philia**, and **agape** (May, 1969). Sex and eros are closely related, but they are different. Sex is what we also call lust or libido, whereas eros is the drive of love to procreate or create. As changes in society allowed the more open study of sex, prompted by the work of people like Sigmund Freud and Wilhelm Reich, May noted three particular paradoxes. First, our so-called enlightenment has not removed the sexual problems in our culture. In the past, an individual could refrain from sexual activity using the moral guidelines of society as an explanation. As casual sex became common, even expected, individuals had to face expressing their own morality as just that: their own! This also created a new source of anxiety for some, namely the possibility that their personal relationships might carry an expectation of sexual activity, and that if they did not comply they might not be able to continue dating someone they liked. The second paradox is that “the new emphasis on technique in sex and love-making backfires” (May, 1969). Emphasizing technique (or prowess) can result in a mechanistic attitude toward making love, possibly leading to alienation, feelings of loneliness, and depersonalization. This can lead to the anticipatory anxiety described by Frankl. Finally, May believed that our sexual freedom was actually a new form of Puritanism. There is a state of alienation from the body, a separation of emotion from reason, and the use of the body as a machine. Whereas in the Victorian era people tried to be in love without falling into sex, today many people try to have sex without falling in love.

Philia and agape are also related to one another, as with sex and love. Philia refers to feelings of friendship or brotherly love, whereas agape is the love devoted to caring for others. Friendship during childhood is very important, and May believed it was essential for meaningful and loving relationships as adults, including those involving eros. Indeed, the tension created by eros in terms of continuous attraction and continuous passion would be unbearable if philia did not enter into the equation and allow one to relax in the pleasant and friendly company of the object of one’s desires. Harry Harlow, once again, showed that the opportunity to make friends was as essential in the development of young monkeys as it appears to be in humans (cited in May, 1969). In the West, however, given our highly individualistic and competitive society, deep, meaningful friendships seem to be something of the past, especially among men. May cautions, however, that since the evidence shows the importance of friendship during development perhaps we should remember the value of having good friends.

Finally we have agape, a selfless love beyond any hope of gain for oneself. May compared this love to the biological aspect of nature in which a parent will fight to the death in defense of their offspring. With agape, we run the risk of being like God, in the sense that we know others never act without some degree of their own interests in mind. Similarly, we don’t

want to be loved in an ethereal sense, or on the other hand only for our body. We want to be loved completely. So, all true love involves some element of the other types of love, no matter how little or how obscured it may be (May, 1969).

Agape is exemplified in the bond between a parent and their child.

In the foreword to Love and Will (May, 1969) May acknowledged that some of his readers might find it odd that he combined the two topics in one book, but he felt strongly that the topics belong together. He considered both love and will to be interdependent, they are processes in which people reach out to influence others, to help to mold and create the consciousness of others. Love without will is sentimental and experimental, whereas will without love is manipulative. Only by remaining open to the influence of others can we likewise influence them, so love must have an honest purpose, and purpose must be taken with care.

Will, or **will power** as it is more commonly known, was one of the earliest subjects in American psychology, having been examined in detail by William James as early as 1890 (see James, 1892/1992) and again in 1897 in The Will to Believe (James, 1897/1992). May considered Sigmund Freud's greatest discovery to be the uncovering of unconscious desires and motives. Although many people may believe themselves to be acting out of higher ideals, most of us are, in reality, acting according to psychologically determined factors of which we are unaware. Nonetheless, May considered this to be one of the most unfortunate results of Freud's work. By accepting determinism, we undermine the influence of will and making decisions. As May put it, Freud's theory suggests that we are "not driving any more, but driven" (May, 1969).

The suggestion that we are no longer in charge of our own lives, that we are driven by psychological determinism, seems strange to those who believe that never before have people had such power, both in terms of individual freedom and in the collective conquest of nature. But May referred to a **contradiction in will**, the contrast between our feelings of powerlessness and self-doubt and the societal assurances that we can do anything we set our minds to. May believed that we exist in a "curious predicament," in that the technical wonders that make us feel so powerful are the very same processes that overwhelm us (May, 1969):

Thus, the crisis in will does not arise from either the presence or absence of power in the individual's world. It comes from the contradiction between the two - the result of which is a paralysis of will. (pg. 189; May, 1969)

Will alone is not the driving force that leads us to responsible and authentic lives. Underlying will is something May called **intentionality**. Intentionality is the structure that gives meaning to experience, it is both how we perceive the world and how the world can be perceived by us. In other words, through our perceptual processes we influence the world around us; we affect the very things that we perceive. Intentionality is a bridge between subject and object (May, 1969). Compare this once again to the nothingness between beings and Being (à la Heidegger), or between the en-soi and the pour-soi (à la Sartre). Still, our ability to reach and form the very objects that we perceive, in other words, to participate actively in our lives, can be dramatically curtailed by the problem addressed by May early in his career, anxiety:

Overwhelming anxiety destroys the capacity to perceive and conceive one's world, to reach out toward it to form and re-form it. In this sense, it destroys intentionality. We cannot hope, plan, promise, or create in severe anxiety; we shrink back into a stockade of limited consciousness hoping only to preserve ourselves until the danger is past. (pp. 244; May, 1969)

Discussion Question: Consider the different loves in your life. How do they differ? How have they brought meaning to your life? Has your view of what love is changed during your life, in either good or bad ways?

The Daimonic: Source of Violence and Creativity

The **daimonic**, according to May, is "any natural function which has the power to take over the whole person" (May, 1969). It can be either destructive or creative, and is often both. In this way it is similar to Jung's concept of the shadow, and May himself made that comparison (May, 1991; see also Diamond, 1996, Reeves, 1977). In fact, it is the mixture of good and evil in the daimonic that protects us from the dangers of excess, whether excess good or the passivity of feeling powerless. When May did not know whether he would live or die from tuberculosis, he realized that his feelings of helplessness were turning into passivity, and that this was sure to lead to his death (as he had seen with others). He described this experience as the product of his innocence, and that because he was innocent he allowed the bacteria infecting his body to do violence to him. However, when he chose to fight the disease, when he asserted his will to live, he began to make steady progress and, indeed, he recovered. In this sense, May had chosen to allow the daimonic to take over his self in the interest of self preservation. In each instance, how one allows the daimonic to take over is influenced by personal responsibility (Reeves, 1977).

When the daimonic takes over without one having made a responsible choice, however, it can lead to violence toward others. Our lives often involve conflict between those who have power and those who do not. When a person feels powerless, helpless, insignificant, they can lash out under the control of the daimonic. According to May, violence is bred in impotence and apathy (May, 1972). This can be particularly important for those who have little or no advantage in our society. In Power and Innocence (May, 1972), May described a patient who was a young, Black woman. Being both Black and female, born before the civil rights movement, she was about as powerless as one could be in America. Her stepfather had forced her to serve as a prostitute for years. Although quite intelligent, and successful in school and college, she felt so helpless that May described her as having "no active belief that she deserved to be helped." An important aspect of therapy for this patient was to get in touch with her anger, to get in touch with the violence that had been done to her and that she wished to do to others.

In considering the case of this young woman, May concluded that we must not simply condemn all violence and try to eliminate even the possibility of it. To do so would be to take away a part of full humanity. In this context, May criticizes humanistic psychology and its emphasis on fulfilling self-actualization, an emphasis that May felt moved toward greater moral perfection. However, the recognition that we are not perfect, that each of us has good and evil within, prohibits us from moral arrogance. Recognizing this leads to the restraint necessary for making forgiveness possible.

Our ability to achieve good is dependent on who we are, and who we are is based partly on our own **creativity**. Since humans are not simply driven by instinct and fixed action patterns, in contrast to every other creature on earth we must create ourselves. This creation must take place within the world that exists around us, and must take into account all of the emotions and predispositions that we do carry with us as biological organisms.

Art - and creative activities of all kinds - can provide comparatively healthy outlets for the constructive expression of anger and rage. Creativity cannot, however, always substitute for psychotherapy. Nevertheless, creativity is at the very core of the psychotherapeutic project: The patient is encouraged to become more creative in psychologically restructuring his or her inner world, and then to continue this creative process in the outer world, not only by accepting and adjusting to reality, but, whenever possible, by reshaping it…

“Creativity” can be broadly defined as the constructive utilization of the daimonic. Creativity is called forth from each one of us by the inevitable conflicts and chaos inherent in human existence… (pp. 255-256; Diamond, 1996)

Pursuing this creativity is not easy, however. We live in a world that is rapidly changing. Since May’s death in 1994 change in the world has probably even accelerated. May asked whether we would withdraw in anxiety and panic as our foundations where shaken, or would we actively choose to participate in forming the future (May, 1975). Choosing to live in the future requires leaping into the unknown, going where others have not been, and therefore cannot guide us. It involves what existentialists call the anxiety of nothingness (May, 1975). Making this bold choice requires **courage**. One of the reasons we need to be courageous is that we must fully commit ourselves to pursuing a responsible creation of the future, but at the same time we must recognize that sometimes we will be wrong. Those who claim they are absolutely right can be dangerous, since such an attitude can lead to dogmatism, or worse, fanaticism (May, 1975).

Finally, not only must we accept that we might make bad choices, we must also recognize that our creativity is limited. In The Courage to Create (May, 1975), May described having attended a conference where the introductory speaker declared that there is no limit to the possibilities of the human being. Following this statement, the discussion at the conference was a flop. May realized that if there is no limit to what we can accomplish, then there really aren’t any problems any more, we only need to wait until our potentiality catches up with our situation and the problem solves itself. May offered a rather amusing example to clarify this point:

…it is like putting someone into a canoe and pushing him out into the Atlantic toward England with the cheery comment, “The sky’s the limit.” The canoer is only too aware of the fact that an inescapably real limit is also the bottom of the ocean. (pg. 113, May, 1975)

Another inescapable limit is our death. There is no creative act that can change the fact that we will die someday, and that we cannot know when or how it will happen. May believed, however, that these limits are valuable, that creativity itself needs limits. He proposed that consciousness arises from our awareness of these limits, and from the struggle against these limits. May compared this concept to Adler’s theory that much of what we as individuals, and also society as a whole, are arises from our efforts to compensate for inferiority. Thus, our limits lead to what May called a **passion for form**. In its passion for form, the mind is actively forming and re-forming the world in which we live (May, 1975).

Discussion Question: May believed that creatively taking charge of your life required courage. Have you ever had to make a really difficult decision? Did you take the easy way out, or the safe path, or did you make a bold decision that offered great opportunity?

The Cry for Myth

As a practicing psychoanalyst I find that contemporary therapy is almost entirely concerned, when all is surveyed, with the problems of the individual's search for myths. The fact that Western society has all but lost its myths was the main reason for the birth and development of psychoanalysis in the first place. (pg. 9; May, 1991)

The preceding quote is how May began The Cry for Myth, the last book of his career (May, 1991). According to May, the definition of a **myth**is quite simple: it "is a way of making sense in a senseless world." In addition, myths give substance to our existence. In a healthy society the myths provide relief from neurotic guilt and excessive anxiety, and so a compassionate therapist will not discourage them. In the twentieth century, especially in Western culture, we have lost our myths, and with them we have lost our sense of existence and our direction or purpose in life. The danger in this is that people are then susceptible to cults, drugs, superstition, etc., in a vain effort to replace that purpose (May, 1991).

As we pass through the experiences of our lives, our memory is dependent mainly upon myth. It is well accepted today that human memory is constructive, and influenced by our expectations of memory. As May describes it, the formation of a memory, regardless of whether it is real or fantasy, is molded like clay. We then retain it as a myth, and rely on that myth for future guidance in similar situations. For example, an infant is fed three times a day and put to bed 365 days a year, and yet they remember only one or two of these events from their years of early childhood. For whatever reason, good or bad, these specific events take on mythic proportions and greatly influence the course of our lives. May acknowledges the contribution of Alfred Adler in recognizing the value of these early memories, describing Adler as "a perceptive and humble man, he was gifted with unusual sensitivity for children" (May, 1991). As we have seen, Adler considered the basis for neurosis to be a lack of social interest. In therapy, Adler focused on the "guiding fiction" of a child's life, something May considered to be synonymous with a "myth." Since "memory is the mother of creativity," and memory depends upon myth, May believed that the myths that form the identity of our culture are essential for the formation of our self.

May ends his final book with a chapter entitled The Great Circle of Love. Having covered a variety of famous myths in the book, including Dante's Divine Comedy, Marlow's Faust, Captain Ahab in Moby Dick, and Poe's The Raven, May concludes:

In each of these dramas the liberation of both woman and man is possible only when each achieves a new myth of the other sex, leading to a new significant psychological relationship. They are both then liberated from their previous empty and lonely existence. The woman and the man find their true selves only when they are fully present to each other. They find they both need each other, not only physically but psychologically and spiritually as well. (pg. 288; May, 1991)

Existential Psychotherapy

Existential psychotherapy is not so much a technique as it is an overall approach to understanding the nature of the human being. By asking deep questions about the nature of anxiety, loneliness, isolation, despair, etc., as well as about creativity and love, existential psychotherapists seek to avoid the “common error of distorting human beings in the very effort of trying to help them” (May & Yalom, 1995). May believed that American psychology has had both an affinity for and an aversion to existential psychotherapy. The affinity arises from an historical place in American psychology that was very similar to existentialism: William James’ emphasis on the immediacy of experience, the importance of will, and the unity of thought and action. The aversion arises from the Western tendency to dehumanize people through strict adherence to scientific principles of research, i.e., to reform humans in the image of machines (May, 1983).

An essential aspect of existential psychotherapy is to help individuals realize their own being, their own role in choosing the form that their life will take. This is known as the **“I-Am” experience**. It is all too common for us to associate ourselves with external factors: I am a professor, I am a student, I work at a store, I run a business, etc. We repress our own sense of being. To use an example similar to a case described by May: I am a professor, but that is not really who I am. I am a father and a husband, but that isn’t all that I am. I have a family and a career, but that isn’t quite it either. What is left, or what is common in each of these statements? I am! And as May put it, if I am, I have a right to be (example cited in May & Yalom, 1995). This realization is not the solution to my problems, but it is a necessary precondition to finding the courage to pursue the rest of my life.

Once an individual finds the courage to recreate their life, the existential therapist will address a variety of issues. As discussed above, May placed a great deal of emphasis on anxiety. Guilt is also an important issue to be addressed, since we may feel guilty about poor ethical choices or instances when we failed to be responsible with our actions. As with anxiety, guilt can be normal (after actually doing something bad) or neurotic (when we fantasize some transgression). Both anxiety and guilt affect how we experience Kierkegaard’s concept of being-in-the-world. Our world can be viewed in several different ways, however. There is the **Umwelt** (the world around), the **Mitwelt**(the with-world), and the **Eigenwelt**(the own-world). The Umwelt is the world around us, the natural environment. It encompasses our biological needs, and the unavoidable reality that we will die one day. The Eigenwelt refers to our self-awareness and our ability to relate to our selves, and it is uniquely human (May & Yalom, 1995).

The Mitwelt bears a special relationship to another important concept in existential psychotherapy: **time**. Because we tend to think about ourselves spatially, as objects within our life, we tend to focus on the past. In other words, we focus on what we have become, as opposed to what we might be. Moments when we truly encounter ourselves are rare, but it is only when we grasp the moment that we truly experience life. Those moments can be positive, such as the experience of love, or negative, such as the experience of depression, but they are real nonetheless. The Mitwelt contains the inner meaning of the events that occur in our lives. Individuals who suffer from brain damage often cannot think in terms of abstract possibilities, they become trapped in concrete time. In order to be fully healthy, and something essential to the growth of humans, is our ability to transcend time:

If we are to understand a given person as existing, dynamic, at every moment becoming, we cannot avoid the dimension of transcendence. Existing involves a continual emerging, in the sense of emergent evolution, a transcending of one's past and present in order to reach the future. (pg. 267; May & Yalom, 1995)

Although the content described above might seem very different from the type of psychoanalysis described by Freud, the general process of existential psychotherapy is similar to psychoanalysis. It is accepted that the client experiences anxiety, that some of this anxiety is unconscious, and that the client is relying on defense mechanisms in order to cope with the anxiety. A fundamental difference, however, is the focus of the therapy. Rather than digging into the deep, dark past, the existential psychotherapist strives to understand the meaning of the client's current experiences, the depth of experience in the given moment. For this reason, the therapist-client relationship remains important, but the emphasis is not on transference. Rather, the emphasis is on the relationship itself as fundamentally important (May & Yalom, 1995).

Discussion Question: Have you ever had an "I-Am" experience?

Buddhism and Existentialism: The Completion of a Circle?

Buddhism is by far the oldest theory of psychology that we will cover in this book. Applied existentialism, particularly the work of Rollo May, is one of the more recent developments in psychology. And yet, these two approaches share a great deal in common, a fact readily acknowledged by May:

…The likenesses between these Eastern philosophies and existentialism go much deeper than the chance similarity of words. Both are concerned with ontology, the study of being. Both seek a relation to reality which cuts below the cleavage between subject and object. Both would insist that the Western absorption in conquering and gaining power over nature has resulted not only in the estrangement of man from nature but also indirectly in the estrangement of man from himself. The basic reason for these similarities is that Eastern thought never suffered the radical split between subject and object that has characterized Western thought, and this dichotomy is exactly what existentialism seeks to overcome. (pp. 58-59; May, 1983)

In Japan there is a form of psychotherapy, known as **Morita**, which emphasizes the treatment of anxiety. The treatment consists of acceptance, reattribution, dereflection, and active engagement. The dereflection mentioned is the same technique developed by Viktor Frankl. The active engagement continues this effort at distracting the client from their anxiety, hopefully breaking them out of the circle of anticipatory anxiety and subsequent failure described by Frankl. This procedure has proven both successful and, consequently, influential amongst Japanese psychotherapists. A second Japanese technique, **Naikan**, combines a more traditional Buddhist approach with elements of existential psychology. The client is directed to reflect intensely on their past relationships, and then to consider what they have done for others, what others have done for them, and the difficulties they have caused for others. The goal is to help the client recognize the interdependence of humans, and to appreciate whether or not they, as well as others, have acted responsibly within the relationships. By confronting feelings of guilt and unworthiness, it is hoped that the client will realize that they have been loved and appreciated nonetheless (Walsh, 1995).

Belinda Siew Luan Khong (2003) has examined the role of responsibility in a particular form of existential psychotherapy known as **daseinsanalysis** (developed by the Swiss psychiatrist Medard Boss and grounded in the philosophy of Heidegger) and compared it to Buddhist practice in the Theravadan tradition. She found that daseinsanalysis and Buddhist practices share much in common, and that both have something to offer to each other:

…An integration of these two disciplines will make their ideas and practices more accessible to communities outside their traditional domains. The daseinsanalytic and Buddhist perspectives relating to personal and social responsibility provide us with valuable philosophical and psychological insights into this very important human phenomenon and show us practically how individuals can be assisted in taking responsibility for every moment of their existence, and to develop a sense of respond-ability to different situations. (pg. 158; Khong, 2003)

Stephen Batchelor, a former Buddhist monk turned author and teacher, has presented existentialism as an interesting approach to the primary problem facing Buddhism in America today (Batchelor, 1983). According to Batchelor, Buddhism in the west is split between those who wish to follow a traditional path (emphasizing meditation and practice) and those who insist upon an academic approach to the analysis and understanding of Buddhism. Between the two approaches lies a great chasm. As we have seen, existentialism draws its deepest and most meaningful philosophy from nothingness, be it the distinction between Being and beings (Dasein, according to Heidegger) or the shell separating the pour-soi from the en-soi (as proposed by Sartre). Drawing primarily from his Buddhist training and the philosophy of Heidegger and Tillich (see below), Batchelor contrasts **being-alone** and **being-with**. We are essentially alone at birth and at death, in that we cannot share the experience with others, and this leads to unavoidable anxiety throughout our lives (though not necessarily overwhelming anxiety for most people). As we will see in the next chapter, the first noble truth of Buddhism is that human life is suffering. But just as much as we are alone, we are unavoidably linked to others as well. What matters then, is that we experience authentic **being-with-others**, and the root of authentic being-with is concern for others (as opposed to the inauthentic distortion of self-concern; Batchelor, 1983).

The genuine welfare of man, of both oneself and others, is found in the optimum actualization of the potentialities of his being. To exist in the fullest possible way in our aloneness as well as in our relations with others is the fulfillment of the inner aim of human life…(pg. 88; Batchelor, 1983)

Some Final Thoughts on Existentialism and Existential Psychology

Jean-Paul Sartre’s Being and Nothingness (1943) is considered the defining text of modern existentialism. Sartre was an atheist, so the brief introduction to existentialism in this chapter went in the direction of atheism. However, Frankl and May were not atheists, and one of May’s most influential mentors, as well as a close personal friend, was Paul Tillich. Tillich remains a well-known and respected existential philosopher in the spiritual tradition. May went so far as to say that Tillich’ book The Courage to Bemight be the best and most understandable presentation of existentialism as an approach to life that has been written in English (May, 1983).

It is also interesting to note that both Frankl and May were significantly influenced by Alfred Adler. Frankl worked closely with Adler for a time, and May took a summer course with Adler. Both cite Adler regularly in their writings. Adler's focus on the childhood struggle against one's own inferiority, his emphasis on social interest as a responsible means to superiority, and his recognition of the dangers inherent in seeking superiority at the expense of others, all fit well with the existential perspective on making responsible choices in living one's life. This point emphasizes, once again, the profound influence that Adler has had on psychology, and that he is in all probability the most under-recognized figure in the history of psychology.

In 1897, William James published an essay entitled Is Life Worth Living? (James, 1897/1992). James begins by describing how some people see the value in life, indeed they fully enjoy life, no matter what happens to them or around them. However, for most people this is not the case, and there is no magic way to give everyone such an optimistic point of view. So, James presents a series of arguments that one might use with suicidal people (that is the term he uses) in order to convince them that life is worth living. He relies heavily on religious faith, though not on any particular religion, but also leads into a discussion of existential thought. Approximately a decade before Frankl and May were even born, James wrote the following words:

…Suppose, however thickly evils crowd upon you, that your unconquerable subjectivity proves to be their match, and that you find a more wonderful joy than any passive pleasure can bring in trusting ever in the larger whole. Have you not now made life worth living on these terms?…This life is worth living, we can say, since it is what we make it, from the moral point of view, and we are determined to make it from that point of view, so far as we have anything to do with it, a success…These, then, are my last words to you: Be not afraid of life. Believe that life is worth living, and your belief will help create the fact. (pp. 501-503; James, 1897/1992)

The challenges that we all face in trying to live authentic lives, the challenges of making responsible and ethical choices that are true to who we ourselves are, can be difficult. In a fascinating book entitled Not a Genuine Black Man, Brian Copeland (2006) talks about his family's racial struggles during the civil rights movement and the difficulties he faces today as a Black man who has adopted many so-called "White" cultural values. Copeland insists, however, that we cannot so easily claim that any given value or personal interest belongs only to one group of people:

…When all is said and done, I AM indeed a Genuine Black Man - because I am resilient. That's what being black in America is truly about: resilience…I stayed on my feet through taunts and harassment, through police intimidation and bigoted nuns, through schoolyard bullies and Sylvester, through my mother's death and bouts of sometimes crippling depression. I am still standing.

I am black because, as my friend Mr. Wilkins once told me, people should be called what they want to be called. I have the right and the ability to determine my identity regardless of what other blacks or whites say. I am not an "oreo," nor am I "still a nigger." I am a man. I am a black man.

No one person or group of individuals holds the monopoly on what in this society is the "true" black experience. My world is as "black" as that of Malcolm X, Colin Powell, Snoop

Dogg, Jesse Jackson, Usher, Bill Cosby, or Diddy. As their experiences in America are unique, mine is unique - yet it is the same. It is as valid as that of the poor African American living in “the ‘hood,” the rich black rapper balancing a lifestyle of fame and violence, and the black scholar working to better this world through academic dissertation. It is as authentic as the experiences of those who marched with Dr. King for civil rights and those who defy the black community by arguing the conservative point of view.

It is the “true” black experience because it is my experience… (pp. 243-244; Copeland, 2006)

Discussion Question: Brian Copeland talked about how difficult it can be to live an authentic life when you don’t meet the expectations of others. Have you ever gone against the advice of family or friends? Did it prove to be the right decision, or did it at least help you to feel better about your own confidence in yourself?

Personality Theory in Real Life: The Application

of Frankl’s Theories to the Workplace and Everyday Life

In 1989, Stephen Covey published The 7 Habits of Highly Effective People. Covey’s book became very popular, selling millions of copies on the way to becoming a #1 New York Times bestseller. If you were to read the first chapter of that book now, it would seem very familiar. Covey presents a very existential approach to understanding our lives, particularly with regard to the problems we experience every day. Perhaps it should not be surprising, then, that in the chapters describing the first two of these seven habits he cites and quotes Viktor Frankl numerous times. Indeed, Covey cites Frankl’s first two books as being profoundly influential in his own life, and how impressed Covey was having met Frankl shortly before Frankl’s death (see Covey’s foreword in Pattakos, 2004).

The first two habits, according to Covey, are: 1) be proactive, and 2) begin with the end in mind. He briefly describes Frankl’s experiences in the concentration camps, and refers to Frankl’s most widely quoted saying, that Frankl himself could decide how his experiences would affect him, and that no one could take that freedom away from Frankl! People who choose to develop this level of personal freedom are certainly being proactive, as opposed to responding passively to events that occur around them and to them. It is not necessary, of course, to suffer such tragic circumstances in order to become proactive in one’s own life:

…It is in the ordinary events of every day that we develop the proactive capacity to handle the extraordinary pressures of life. It’s how we make and keep commitments, how we handle a traffic jam, how we respond to an irate customer or a disobedient child. It’s how we view our problems and where we focus our energies. It’s the language we use. (pg. 92; Covey, 1989)

Covey compares his habit of beginning with the end in mind to logotherapy, helping people to recognize the meaning that their life holds. Covey works primarily in business leadership training, so the value of working toward a greater goal than simply keeping a company in business from day to day is clear, especially for those who care about employee morale and quality control (see also Principle-Centered Leadership; Covey, 1990). When employees share a sense of purpose in their work, they are likely to have higher intrinsic motivation. Think about it for a moment. Have you ever had a job you didn’t really understand, and didn’t care about? Have you ever been given that sort of homework in school or college? So, how much effort did you really put into that job or assignment?

Covey's remaining habits are: 3) put first things first, 4) think win/win, 5) seek first to understand, then to be understood, 6) synergize, and 7) sharpen the saw. At first glance these principles seem reasonably straight forward, emphasizing practical and responsible actions. However, what does "sharpen the saw" mean? Sharpening the saw refers to keeping our tools in good working order, and we are our most important tool. Covey considers it essential to regularly and consistently, in wise and balanced ways, to exercise the four dimensions of our nature: physical, mental, social/emotional, and spiritual. By investing in ourselves, we are taking care to live an authentic life.

More recently, Covey has examined his principles beyond the business world. In 1997 he published The 7 Habits of Highly Effective Families, a book in which he applies the same 7 habits to family life. Covey certainly has solid credentials as a family man, as father of 9 and grandfather of 43 children, and he won the 2003 Fatherhood Award from the National Fatherhood Initiative. Drawing in large part on his own extensive, personal experience, Covey uses many stories, anecdotes, and examples of real-life situations to help provide context to the challenges of raising a family and how we might best work with them. But first, he introduces a simple process: have a clear vision of what you want to accomplish, have a plan of how you might accomplish it, and use a compass (your own unique gifts that enable you to be an agent of change in your family). In essence, Covey is recommending that you prepare yourself to develop the seven habits. We all know how difficult it is to establish a new habit or break a bad habit; how is your New Year's resolution going?

Just as families change, so does the world we live in. Recently, Covey addressed this change by proposing an eighth habit (Covey, 2004). He says that this was not simply an important habit he had overlooked before, but one that has risen to new significance as we have fully entered the age of information and technology in the twenty-first century. As communication has become much easier (e.g., email), it has also become less personal and meaningful. Thus the need for the eighth habit: find your voice and inspire others to find theirs. According to Covey, "voice is unique personal significance." Essentially, it is the same as finding meaning in one's life, and then helping others to find meaning in their own lives. It is through finding a mission or a purpose in life that we can move "from effectiveness to greatness" (Covey, 2004).

Whereas Covey presented an approach to personal and professional effectiveness (and later to greatness as well) that parallels the principles set forth by Viktor Frankl, Alex Pattakos very directly applies Frankl's theories to both the workplace and one's everyday life in Prisoners of Our Thoughts: Viktor Frankl's Principles at Work(with a foreword by Stephen Covey; Pattakos, 2004). Frankl himself urged Pattakos to publish his book during a meeting in 1996. Pattakos, like Covey, has been profoundly influenced by Frankl's writings throughout Pattakos' career. According to Pattakos, we are creatures of habit, and we prefer a life that is both predictable and within our comfort zone. As the world is changing in the twenty-first century, so the conditions under which we work are changing. Pattakos believes there is a need for humanizing work. More than just balancing one's personal life and career, humanizing work is an attempt to honor our own individuality and to fully engage our human spirit at work. Simply put, it is an effort to apply Frankl's will-to-meaning in our workplace (Pattakos, 2004).

Like Covey, Pattakos presents seven core principles. They are similar to Covey's seven habits, but in keeping with Pattakos' intentions they are aligned more directly with the

principles of logotherapy and existential psychology described by Frankl. The seven core principles are: 1) exercise the freedom to choose your attitude, 2) realize your will-to-meaning, 3) detect the meaning of life's moments, 4) don't work against yourself, 5) look at yourself from a distance, 6) shift your focus of attention, and 7) extend beyond yourself. These principles include not only the ideas of personal freedom and will-to-meaning, but also dereflection (principles 4 and 6) and the will-to-ultimate-meaning (principle 7). Clearly Pattakos has accomplished his goal of applying logotherapy to the workplace, but how well does this application work in real life?

Pattakos describes the case of a probation officer with the state department of corrections. Rick, as Pattakos identifies him, was raised in foster care and orphanages. However, rather than developing a sense of caring and concern for others who have difficulties in their lives, Rick refers to his clients as "maggots." Rick has become insensitive and unforgiving, he has also become deeply depressed and anxious. Overall, he feels lost, unhappy, and unfulfilled, and he doesn't know what to do about it. According to Pattakos, he has become a prisoner of his own thoughts, and only he has the key to his own freedom. Very simply put, he needs to find a new job or find meaning in the one he has now. One possibility is for Rick to consider his own life circumstances in relationship to his clients:

…Whenever we stop long enough to connect to ourselves, to our environment, to those with whom we work, to the task before us, to the extraordinary interdependence that is always part of our lives, we experience meaning. Meaning is who we are in this world. And it is the world that graces us with meaning. (pg. 157; Pattakos, 2004)

By making a responsible choice to seek meaning in our lives, to not work against ourselves, we can put ourselves on a path we had not seen before:

When we live and work with meaning, we can choose to make meaning, to see meaning, and to share meaning. We can choose our attitudes to life and work; we can choose how to respond to others, how to respond to our jobs, and how to make the very best of difficult circumstances. We can transcend ourselves and be transformed by meaning. We can find connection to meaning at work, in the most unusual places and with the most unexpected people. Meaning is full of surprises. (pg. 159; Pattakos, 2004)

And finally, it does not matter what sort of job we have. It is our choice, our freedom:

No matter what our specific job might be, it is the work we do that represents who we are. When we meet our work with enthusiasm, appreciation, generosity, and integrity, we meet it with meaning. And no matter how mundane a job might seem at the time, we can transform it with meaning. Meaning is life's legacy, and it is as available to us at work as it is available to us in our deepest spiritual quests. We breathe, therefore we are - spiritual. Life is; therefore it is - meaningful. We do, therefore we work.

Viktor Frankl's legacy was one of hope and possibility. He saw the human condition at its worst, and human beings behaving in ways intolerable to the imagination. He also saw human beings rising to heights of compassion and caring in ways that can only be described as miraculous acts of unselfishness and transcendence. There is something in us that can rise above and beyond everything we think possible… (pg. 162; Pattakos, 2004)

Discussion Question: Stephen Covey and Alex Pattakos have applied Frankl's theories to both the workplace and our everyday lives. How well do you think the principles of

existential psychology can address the problems that you face at work, home, school, etc.? Is it ever really as simple as applying one's will and choosing to act responsibly? Do you live an authentic life?

Review of Key Points

• Existentialism focuses on an individual's subjective "truth." The freedom and responsibility that come with personal truth lead to anxiety, but they can also elevate the individual to lead an authentic life.

• Heidegger believed that all creatures are connected, but that only humans can become aware of this connection. Dasein, the realization of this connection, allows us to connect with Being. Awareness of our impending death, however, leads to anxiety, but if we accept that truth we can live an authentic life.

• Sartre believed that humans were unique, something he called en-soi. Awareness of the nothingness that separates the en-soi from the pour-soi is what drives some individuals to make something significant of their lives. For those who cannot, Sartre expressed a need for existential psychoanalysis.

• Viktor Frankl developed his ideas for logotherapy (an existential psychoanalysis) during his impressive early career. He had an extraordinary opportunity to put his ideas to the test while imprisoned in the Nazi concentration camps.

• Recognized as the third Viennese school of psychotherapy, logotherapy focuses on one's will-to-meaning, the desire to find meaning and purpose in one's life.

• People who cannot find meaning experience existential frustration, which can lead to a noogenic neurosis.

• Logotherapy itself relies primarily on the techniques of paradoxical intention and dereflection. These techniques are designed to break the cycle of anticipatory anxiety and failure that plague individuals who suffer from existential crises.

• Going beyond ordinary, everyday life, Frankl proposed a super-meaning to life, and he suggested that there is also a will-to-ultimate-meaning that can be pursued through religion or spirituality. In this light, Frankl referred to logotherapy as "height psychology" (in contrast to depth psychology, another term for psychoanalysis).

• Rollo May believed that anxiety underlies nearly every crisis. He proposed that anxiety must be understood in terms of freedom, and he distinguished between normal anxiety and neurotic anxiety.

• Culture has significant effects on the nature and amount of anxiety that people are likely to experience in their lives. Since anxiety can lead to hostility, these cultural factors are, and have been throughout history, very important issues (e.g., opposition to the civil rights movement in the United States, and the recent dramatic rise in international terrorism).

• A critical factor in life, according to May, is our ability to integrate into our world. One of the challenges to integration is the human dilemma: whether we are the subject or the object in our lives. As self-aware beings we can know that we are both subject and object, and so, in psychological terms, we exist in a world between either behaviorism or humanistic psychology.

• There are different types of love, all of which are very important to our lives. Love can give meaning to our lives, but it must be honest and responsible love.

• Through will and intentionality we can give structure to our lives and meaning to our actions. However, overwhelming anxiety can destroy our ability to participate actively in our own lives.

• The daimonic is any function that can take over the whole person. It can be a source of violence, but also a source of creativity. We can choose how the daimonic takes over, and whether that choice is responsible or not determines whether our actions are violent or creative.

• Being creative requires that we live in the future and actively participate in shaping our lives. Such bold choices require courage, especially in light of the inescapable reality that we will die.

• May felt that myth provides an important cultural framework within which we can form our lives. Unfortunately, the Western world has lost many of its myths, making people susceptible to cults, drugs, superstition, etc.

• According to May, the primary goal of existential psychotherapy is to help the client realize their own being, to have an “I-Am” experience. Time is an important aspect of this procedure. The client must be helped to shift their focus from the past to the future, and even more so, to transcend time altogether.

• Although existential psychology is younger than most other schools of psychology, it has much in common with ancient Eastern philosophies, such as Yoga, Buddhism, and Taoism.

• Terrorism is not the result of Islam. Terrorists are found all over the world, from many different races, religions, and nationalities. Islam opposes violence and murder.

• Terrorism is based on psychological factors (a perception that there are no alternatives, and that terrorism is legitimate), and seeks to cause psychological effects (feelings of terror and helplessness). Thus, psychologists have an important role to play in understanding and eliminating terrorism.

• One can easily find those who believe that terrorism either never works or always works. Some believe that we must respond with understanding to eliminate the root causes of terrorism, whereas others believe we must use force (but not too much force, lest we become terrorists as well). Clearly there are no easy answers for dealing with terrorists themselves or terrorism in general.

CHAPTER 10 / CAC.902

Behavior Therapy and Learning Principles

Behavior therapy begins with a simple but powerful idea: behavior is learned in context, and changing the context can change the behavior. This does not mean that thoughts, feelings, or culture are irrelevant. It means that the counselor starts with observable patterns and the conditions that maintain them. For children and adolescents, behavior therapy is often practical because it can involve caregivers, teachers, and daily routines. It also has a strong evidence base for anxiety, disruptive behavior, and habit problems.

Classical conditioning

Classical conditioning occurs when a neutral stimulus becomes associated with a response. A child who is bitten by a dog may fear all dogs. A student who is humiliated in math class may feel panic when entering the room. The fear is not a choice. It is a learned association. Exposure-based treatment works by creating new learning: the feared stimulus is encountered without the expected catastrophe, and the body learns safety. Avoidance prevents this learning.

Operant conditioning

Operant conditioning focuses on consequences. Behavior that is reinforced tends to increase. Behavior that is not reinforced or is punished tends to decrease. Positive reinforcement adds something valued. Negative reinforcement removes something aversive. Punishment adds something aversive or removes something valued. These terms are often confused. A child who tantrums and then receives a tablet has been positively reinforced. A child who tantrums and escapes a demand has been negatively reinforced. Both can increase tantrums, but the mechanism differs.

Functional assessment

A functional assessment asks what happens before and after a behavior. Antecedents include setting events, triggers, and demands. Consequences include attention, escape, tangibles, and sensory effects. The counselor forms a hypothesis about function. A behavior that looks the same may serve different functions. One child hits to escape work; another hits to gain peer attention. The intervention should match the function. Teaching a replacement behavior is usually more effective than punishment alone.

Caregiver and school application

Behavioral parent training teaches caregivers to give clear instructions, reinforce desired behavior, use consistent consequences, and reduce coercive cycles. A coercive cycle occurs when a parent makes a demand, a child escalates, the parent withdraws the demand, and the child's escalation is reinforced. The parent also learns that giving in stops the noise, so both parties are reinforced. Breaking the cycle requires calm consistency and reinforcement of cooperation.

In school, a behavior plan may include a token system, a break card, or a check-in/check-out routine. The plan should be simple, measurable, and reviewed. If the plan fails, the

counselor asks whether the function was wrong, the reinforcer was weak, the demand was too hard, or the context was unsafe. Failure is data, not proof that the child is bad.

Exposure and anxiety

Exposure is a central behavioral treatment for anxiety. The client faces feared stimuli in a planned, gradual way while reducing safety behaviors. A hierarchy orders fears from easier to harder. The goal is not to eliminate anxiety instantly. The goal is to learn that anxiety can be tolerated and that the feared outcome does not occur or is manageable. With children, exposure is often playful. A child afraid of dogs may start with pictures, then videos, then a calm dog at a distance. The child's consent and pacing matter.

Clinical example

Eight-year-old Noah refuses to sleep alone. Each night he cries, and a parent lies with him until he sleeps. The parent is exhausted. A behavioral formulation notes that crying produces parental presence, which reduces Noah's fear and reinforces crying. The parent's presence also reinforces the parent's escape from distress. The plan includes a bedtime routine, gradual fading of parental presence, and reinforcement for staying in bed. The counselor also assesses for trauma and anxiety. If Noah has nightmares from abuse, exposure alone is not enough. Safety and trauma treatment come first.

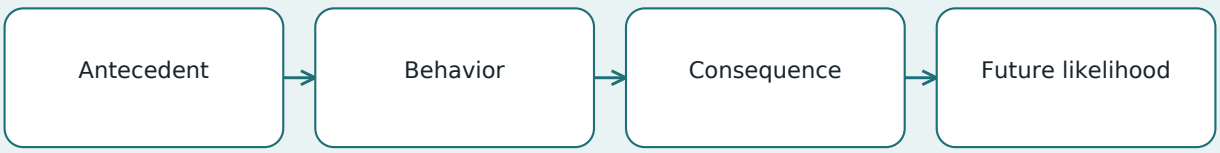
Ethics and limits

Behavior therapy has been criticized for controlling clients. The ethical question is who chooses the goal and whether the goal serves the client's welfare. A plan to make a child silent for adult convenience is not ethical. A plan to help a child communicate needs is ethical. Reinforcement should not be humiliating. Punishment should be avoided when possible. The counselor should consider neurodiversity. Some behaviors, such as stimming, may be self-regulating and should not be extinguished simply because they look unusual.

Reflection

Choose a small behavior you want to change. Identify antecedents, consequences, and function. Design a replacement behavior and a reinforcement plan. Then ask whether the goal is truly yours or imposed by someone else. Behavior therapy works best when the goal is meaningful to the person whose behavior is changing.

The behavior chain



Functional assessment asks what maintains a behavior before choosing an intervention.

Attributed Foundational Reading

Attribution and critical-reading note

Adapted from Personality Theory in a Cultural Context by Mark D. Kelland, OpenStax CNX / Lansing Community College (2015), CC BY 4.0. Selection, organization, and PDF layout modified for this course; new lectures and course diagrams are separate additions.

Read historical and theoretical claims critically. A theory is a lens for inquiry, not a verdict about a child or family. Where source claims are dated or culture-bound, the new lecture supplies cautions and contemporary framing.

Source module m58073 • Personality Theory in a Cultural Context • pinned GitHub snapshot a52283699509. Full repository and license details appear in the attribution register.

While psychodynamic theory was developing in Europe, American psychology was largely under the influence of behaviorism. The American psychologist John B. Watson (of “Little Albert” fame) is considered to be the father of behaviorism. Although he is not known for addressing issues of personality development, he did feel it was important for behaviorists to do so. His approach involved reducing personality to smaller and smaller units of behavior referred to as habit systems, suggesting that personality was very consistent. Nonetheless, through further conditioning the personality of an individual could change, leading Watson to make the bold statement that if he was given a dozen healthy infants he could take any one at random and train him or her for any career, including “doctor, lawyer, artist, merchant-chief and yes, even beggar-man and thief” (cited in Stagner, 1988; see also Lundin, 1979).

As the scientific study of behavior continued, it became common to try determining behavior on the basis of mathematical models. This work led to an era of grand learning theories, which culminated in the highly complex models of behavior proposed by Clark Hull (see Bower & Hilgard, 1981). This research took the behavioral study of personality in a very different direction than psychodynamic theory. The direction in which B. F. Skinner took personality theory, however, was so different that it became known as **“radical behaviorism.”** Skinner rejected anything he could not directly observe, so concepts such as consciousness, thought, reasoning, and the “mind,” were all considered irrelevant to the study of individuals. Only the specific behaviors performed by the individual were open to being examined in Skinner’s form of behaviorism.

In contrast, John Dollard and Neal Miller tried to find some common ground between psychodynamic theory and learning theory. Dollard was a true generalist, with interests in anthropology and sociology in addition to psychology. Miller studied with two renowned learning theorists, Edwin Guthrie and Clark Hull, and was psychoanalyzed by Heinz Hartman in Vienna, while studying in Europe in the 1930s. Together, Dollard and Miller tried to develop a theory that would encompass psychodynamic theory, learning theory, and the influence of sociocultural factors. Their effort to develop what might be called a unified theory of personality stands in stark contrast to the constraints of radical behaviorism. Most importantly, they set the stage for the social learning theorists who followed.

B. F. Skinner and the Behavioral Analysis of Personality Development

Many psychology students find it difficult to apply the strict principles of radical behaviorism to personality development. And yet, psychologists generally consider our discipline to be objective and scientific. Thus, it would seem essential that we acknowledge those psychologists who apply a strict scientific approach to the study of behavior. Skinner represents the extreme conditions under which some psychologists control the study of behavior, and his contributions to understanding the basic underlying principles of reward and punishment, and their consequences, rank him among the most influential psychologists of all time.

A Brief Biography of B. F. Skinner

Burrhus Frederic Skinner was born on March 20, 1904 in Susquehanna, Pennsylvania. He lived there for 18 years, and graduated from the same high school as his mother and father. By his own account he had a happy childhood, though it was somewhat chaotic in the rough and tumble coal town that was Susquehanna. Skinner roamed the hillsides, invented and built all sorts of gadgets, and developed a love for the wide variety of experiences that life has to offer a child living in a “warm and stable” home. He constantly satisfied his curiosity and imagination:

I was always building things. I built roller-skate scooters, steerable wagons, sleds, and rafts…I made seesaws, merry-go-rounds, and slides. I made slingshots, bows and arrows, blow guns and water pistols…and from a discarded water boiler a steam cannon with which I could shoot plugs of potato and carrot over the houses of our neighbors. (Skinner, 1970).

Skinner’s father, a lawyer, bought many books and maintained a large library in their home. Skinner enjoyed school and, under the guidance of an influential teacher named Mary Graves, eventually chose to major in English Literature in college and then to pursue a career in writing. While still at home, Skinner played the piano and the saxophone, and during high school he played in a jazz band. He also became quite interested in spirituality, particularly under the conflicting views of Miss Graves and his grandmother Skinner. Miss Graves was a devout Christian, who had taught Skinner’s Sunday school class, but held fairly liberal views on the Bible. Grandmother Skinner took a more fire-and-brimstone approach, showing Skinner the burning coals in the stove to make sure he understand the dangers of Hell! Ultimately Skinner came to his own perspective, and from that point forward he no longer believed in God (Bjork, 1997; Skinner, 1970, 1976).

Skinner attended Hamilton College, where he majored in English and minored in Romance languages. He felt that he never quite fit in at college, largely because he was no good at sports and because Hamilton College required students to attend daily chapel. By his senior year, he and his friends became involved in some serious pranks, ultimately being threatened with not being allowed to graduate. He did graduate, however, and began a brief attempt at a career as an author. A professor with whom Skinner had taken a summer course introduced him to the renowned poet Robert Frost. Frost offered to review some of Skinner’s stories, and he sent a favorable reply that greatly encouraged Skinner (the letter is reprinted in Skinner, 1976). However, Skinner had only one success as an author. His father had always hoped Skinner would practice law with him, and together they published a private book on legal decisions in the ongoing battles between the coal companies and the unions. Skinner then spent 6 months living a bohemian lifestyle in New York’s Greenwich Village, followed by some time in Paris, France. Ultimately, however, he gave up his career

as an author because he simply “had nothing important to say” (Bjork, 1997; Skinner, 1970, 1976).

As a child, Skinner had always been interested in the behavior of animals and kept many wild pets. In high school he was very interested in philosophy, and in college a professor had introduced him to comparative psychology and Pavlov’s work on classical conditioning. He began reading Pavlov and Watson while living in Greenwich Village, and eventually went to Harvard University to study psychology. At Harvard Skinner developed the rigorous work schedule that was to become one of his personal hallmarks. After leaving Harvard he taught at the University of Minnesota, where, during World War II, he conducted research on using pigeons as the guidance system for missiles. He then moved on for a brief period as the chairman of the psychology department at Indiana University. In 1948 he was asked to return to Harvard, where he worked for the remainder of his career (Bjork, 1997; Skinner, 1970).

Throughout the rest of his career, Skinner attempted to apply aspects of his radical behaviorism to a variety of issues, including child care, education, and the very nature of society itself. His influence has been substantial, particularly with regard for his emphasis on psychology as a science. However, his views on scientific methodology and other fields of psychology have been controversial. For example:

…I suppose it was only my extraordinary luck which kept me from becoming a Gestalt or (so help me) a cognitive psychologist. (pg. 8; Skinner, 1970)

The Freudian mental apparatus doesn’t make much sense to me…I don’t believe that he devised a useful conceptual system… (pp. 5-7; Evans, 1968)

Dreaming…is almost always weak behavior and hence determined by trivia. (pg. 193; Epstein, 1980)

New, deep, real, growth, harmony, understanding potential, unfoldment - an opiate soothing syrup for humanistic psychologists, hashish for the searchers for identity. (an informal review of a new journal for transpersonal psychology; pg. 291; Epstein, 1980)

I’m not at all impressed by the model builders, the information theory analysts, the systems analysts, and so on. They still haven’t shown me that they can do anything important. (pg. 82; Evans, 1968)

In general, scientific methodology is not an accurate reflection of what the scientist really does…it doesn’t reflect the actual behavior of the scientist. Fortunately for science, scientific method and statistics weren’t formulated until the middle of the nineteenth century. (pg. 89; Evans, 1968)

In Skinner’s defense, however, he often felt that his position was misunderstood. One of the most important approaches to the study of behavior that he emphasized was to focus on individuals, not on average measures of behavior that show “none of the characteristic individuality of the organism you’re studying” (pg. 92; Evans, 1968). Despite harsh criticism, Skinner did not take attacks on his scientific perspectives personally. As he described it, he simply reported the facts that arose from his research, and chose not to debate those who disagreed. One exception, however, was Carl Rogers. On several occasions he debated Rogers, whom he described as a friend, about the dignity of man and the control of men (Bjork, 1997; Evans, 1968). He also sent a letter to E. L. Thorndike, who described the Law

of Effect after studying cats escaping from puzzle-boxes well before Skinner was born (published in 1898; see Bower & Hilgard, 1981), apologizing for perhaps having failed to give Thorndike proper recognition for establishing the basic concepts that led to the study of **operant conditioning**. Thorndike replied that he was more honored to have been of service to new scientists than if he were to have received credit for founding a new “school” of psychology (Skinner, 1970).

In 1970, American Psychologist listed Skinner second only to Freud in his influence on twentieth-century psychology, and in 1989 Skinner seemed to express some pride in being cited more often than Freud (Bjork, 1997). Near the end of 1989, however, Skinner was diagnosed with leukemia. He continued to work as best he could, and expressed no anxiety about his approaching death. In August, 1990 he spoke for 20 minutes to a standing-room only audience at the annual meeting of the American Psychological Association. Eight days later he died (Bjork, 1997).

Placing B. F. Skinner in Context: Radical Behaviorism

B. F. Skinner shares at least one thing in common with Sigmund Freud: he provided a target for the behavioral and cognitive theorists who followed him. Freud, of course, was a target for everyone in psychiatry and psychology, including Skinner. Although some might say that Skinner established the experimental analysis of behavior, as far as personality is concerned, Freud also thought that he was being very scientific in his studies of human behavior and the human mind. In order to absolutely define his understanding of behavioral principles, Skinner rejected anything he could not observe. Although this allowed him to claim a level of precision never before possible in the study of behavior, it disallowed the study of the mind, consciousness, thinking, emotion, the very things that most people consider to be the domain of psychology! It also disallowed doing many types of research on humans, and so Skinner, his students, and his colleagues focused much of their effort on studying rats and pigeons.

Like Freud before him, Skinner faced his critics and defended his theories. He certainly had solid scientific data to support his position, but when he went so far as to propose that he could solve the problems of society (in his novel Walden Two), perhaps he was asking for criticism. Nonetheless, in psychology today, behavioral and cognitive approaches to understanding mental illness and conducting psychotherapy are popular and effective. Although behavioral and cognitive theorists incorporate the very things Skinner rejected (emotions, thought, etc.), they have built upon the unquestioned behavioral principles studied first by Skinner. In addition, the application of many of Skinner’s theories, such as reinforcement and punishment, has had a major influence on child rearing and education, or at least in our understanding of those processes.

Although Skinner was not the first behaviorist, that honor goes primarily to John B. Watson (since Pavlov was first and foremost a physiologist), his name is typically the first that comes to mind when recognizing behaviorism as one of three great forces in psychology (the others being psychodynamic and humanistic psychology). Thus, Skinner stands with Freud and with Rogers and Maslow as a giant in the history of psychology.

Scientific Analysis of Behavior and Personality

Late in Skinner’s life (in 1988 to be exact), his former graduate student A. Charles Catania had this to say:

Of all contemporary psychologists, B. F. Skinner is perhaps the most honored and the most maligned, the most widely recognized and the most misrepresented, the most cited and the most misunderstood. (pg. 3; Catania & Harnad, 1988)

Skinner emphasized, above all else, approaching human behavior scientifically. However, he acknowledged that human behavior is complex, and that our familiarity with it makes it difficult for us to be truly objective. In addition, he recognized that many people find it offensive to suggest that human behavior can be understood and predicted in terms of environmental stimuli and their consequences. Still, Skinner took the scientific approach very seriously, and he knew that science is about more than just determining a set of facts or principles. In Science and Human Behavior (Skinner, 1953), Skinner wrote that:

Science is concerned with the general, but the behavior of the individual is necessarily unique. The “case history” has a richness and flavor which are in decided contrast with general principles…A prediction of what the average individual will do is often of little or no value in dealing with a particular individual…The extraordinary complexity of behavior is sometimes held to be an added source of difficulty. Even though behavior may be lawful, it may be too complex to be dealt with in terms of law. (pp. 20-21; Skinner, 1953)

Given this complexity, Skinner focused on **“cause”** and **“effect”** relationships in behavior. In common use, these terms have come to carry a meaning far beyond the original intention. For Skinner, a cause is a change in an independent variable, whereas an effect is a change in a dependent variable. Skinner argued that the terms cause and effect say nothing about how a cause leads to an effect, but rather, only that there is a specific relationship in specific order. If we can discover and analyze the causes, we can predict behavior; if we can manipulate the causes, then we can control behavior (Skinner, 1953). By focusing entirely on observable behavior, Skinner felt that psychologists have an advantage, in that they will not waste time and effort pursuing either inner psychic forces or external social forces that may not even exist. Focusing on actual behavior is simply more direct and practical. Before examining some of the larger implications of this approach, however, let’s review the basic principles of operant conditioning as defined by Skinner.

Discussion Question: Skinner emphasized a scientific approach to the study of behavior, in part, because individual behavior is so unique. Understanding what the average person might do may tell us nothing about a certain individual. However, a science of personality that treats everyone as unique seems to become hopelessly complex, because we must study everyone individually. Does this really seem like a scientific approach, and whether it is or not, can it really help us to understand other people?

Principles of Operant Conditioning

Operant conditioning begins with a response, known as an operant, which has some effect in the organism’s environment. These responses have consequences that determine whether or not the probability of the response will increase or decrease in the future. **Reinforcers** increase the probability of a given response that precedes them, whereas **punishers** decrease the probability of a response that precedes them. In common terms we might say that good consequences increase behaviors, or that the behavior is rewarded. However, Skinner avoids words like reward due to their psychological implications, preferring instead to use the technical term reinforcer (Holland & Skinner, 1961; Skinner, 1953).

Both reinforcement and punishment come in two forms: positive and negative. **Positive reinforcement** involves the application, or administration, of a favorable consequence to a response. For example, when a child cleans their room, they receive some money as an allowance. The response of cleaning the room results in the application of a tangible reinforcer: money. **Negative reinforcement** involves the removal of an **aversive** or **noxious**stimulus. We are commonly told not to scratch itchy bug bites, because we might get them infected. However, an itch is a very noxious stimulus, and it is not easy to ignore them. When we finally give in and scratch, the itching goes away (at least for a while). The response of scratching is negatively reinforced by the removal of the noxious stimulus (no more itching). In both of these examples, the response (the operant of room cleaning or scratching) is followed by a consequence (reinforcement) that increases the likelihood that we will clean our room or scratch our itchy bug bite.

Punishment can also be positive or negative. If a child misbehaves and is spanked, that is a **positive punishment**. In other words, an aversive consequence is applied (the spanking) as a result of the misbehavior. With **negative punishment**, favorable stimuli are withdrawn. For example, a child who misbehaves receives a time-out, thus removing them from toys, playmates, snacks, etc. Other common examples of a negative punishment are being grounded or losing privileges (such as television or video games). Once again, in positive punishment the response (misbehavior) results in the application of an aversive stimulus (a spanking), whereas in negative punishment the response misbehaving results in the removal of favorable consequences (loss of privileges). One of the most common mistakes that psychology students make is to confuse negative reinforcement with punishment. This is understandable, because of the use of the word “negative.” So it is essential to determine first whether a consequence is a reinforcer or a punisher. Then determine whether the reinforcer is positive or negative, or whether the punisher is positive or negative. It is also generally accepted that punishment is not as effective as reinforcement, and it is more difficult to precisely control the cause-effect relationship (Skinner, 1953, 1974, 1987). This is partly due to **discriminative stimuli,** which signal the contingencies that may be in effect at a given time. In other words, the presence or absence of a parent (a discriminative stimulus) may determine whether one will be punished for a given response (if the cat’s away, the mice will play). In addition, the possibility always exists that punishment can cross the line into abuse (physical and/or emotional). As Skinner noted, science is not just about the facts, there is always something more. In theory punishment may seem equivalent to reinforcement, but in practical matters, such as raising children, every situation may require a more detailed analysis.

In order to reliably measure the behavior of animals (typically rats or pigeons) in his laboratory, Skinner built a special piece of equipment commonly known as a **Skinner box** (though its technical name is an **operant conditioning chamber**). This apparatus allowed for the precise measurement of how subjects responded over time under varying conditions, and produced a special measure of behavior known as a **cumulative record**. Although continuous reinforcement is certainly effective for increasing behavior, in most situations we are not reinforced every time we engage in a certain behavior. Skinner identified four basic **schedules of reinforcement**, based on variations in the number of responses necessary for reinforcement, so-called **ratio schedules**, or the time intervals between making reinforcers available, so-called **interval schedules**. Both ratio and interval schedules can be either fixed or variable.

Although the principles of reinforcement may seem relatively straightforward, they can lead to either complex or odd behavior. Complex behavior can be developed with operant conditioning through the process of **shaping**. Shaping involves reinforcing chains of behavior in a specific sequence, with each change being relatively small and, therefore, relatively simple. As a result, complex behavior can be explained in terms of shaping a series of simple changes in behavior. As Skinner describes it:

Operant conditioning shapes behavior as a sculptor shapes a lump of clay. Although at some point the sculptor seems to have produced an entirely novel object, we can always follow the process back to the original undifferentiated lump, and we can make the successive stages by which we return to this condition as small as we wish. At no point does anything emerge which is very different from what preceded it. (pg. 91, Skinner, 1953)

Sometimes, however, this process goes awry. When an individual accidentally associates a consequence with a response, even though no actual relationship existed, **superstitious behavior** can result. For example, if you provide a few seconds of access to food for a hungry pigeon every 20 seconds, regardless of what the pigeon is doing at the time, the pigeon will develop some form of food-getting ritual. Since the food is delivered regardless of what the pigeon does, the ritual that develops is superstitious. The development of superstition in humans is believed to follow the same principles (Skinner, 1953, 1987). For a straightforward description of the principles of operant conditioning, and the prime example of how Skinner believed these principles might be applied to education, see the programmed instruction book entitled The Analysis of Behavior by Holland and Skinner (1961).

Discussion Question: It has become commonly accepted, at least in psychology, that children should never receive positive punishment (e.g., a spanking). Instead, parents should use negative punishment (e.g., a timeout) and then redirect their child’s behavior in positive ways. How does this compare to how you were punished, and do you agree that this is always true?

Personality Development

Based upon the principles of operant conditioning, Skinner proceeded to address the full range of human behavior, including personality development, education, language, mental illness and psychotherapy, and even the nature of society itself.

Skinner believed that the terms “self” and “personality” are simply ways in which we describe the characteristic patterns of behavior engaged in by an individual. Skinner also referred to the self as “a functionally unified system of responses” (Skinner, 1953), or “at best a repertoire of behavior imparted by an organized set of contingencies” (Skinner, 1974). Skinner acknowledges that critics of the science of behavior claim that behaviorists neglect the person or the self. However, Skinner claims that the only thing neglected is a vestige of **animism**, which in its crudest form attributes behavior to spirits. If behavior is disruptive, the spirit is a demon; if behavior is creative, the spirit is a muse or guiding genius (Skinner, 1974). Indeed, Skinner’s arguments describing the self sound quite similar to the Buddhist perspective we will examine later in this book:

When a man jams his hands into his pockets to keep himself from biting his nails, who is controlling whom? When he discovers that a sudden mood must be due to a glimpse of an unpleasant person, who discovers whose mood to be due to whose visual response? Is the self which works to facilitate the recall of a name the same as the self which recalls it? When

a thinker teases out an idea, is it the teaser who also eventually has the idea? (pg. 283; Skinner, 1953)

If the self, or the personality, does not exist, but is instead simply a collection of behavioral attributes and functions, then it is an irrelevant concept that needs to be discarded. Skinner did not discount the value of Freud's explanation of human behavior, since Skinner acknowledged that many sciences take time to develop. But now that behavioral science was advancing, according to Skinner, it became time to discard Freudian concepts of an unconscious mind and mental functioning. Curiously, this is very similar to the way in which Freud addressed religion: as something that had served its purpose in the course of human development, but which should now be discarded in favor of the science of psychoanalysis.

Since no two people have exactly the same experiences (not even identical twins, who do share an identical genetic make-up), each individual is truly unique. When any one of us seems to have an experience of identity, a feeling of self, it always exists within the unique circumstances of our experiential contingencies, the reinforcers, punishers, discriminative stimuli, etc. that have determined our behavioral patterns. Thus, Skinner argues that we do have a unique individuality, but we are not an originating agent, not a self that decides to act a certain way. Instead, we are a **locus**, a point of convergence for genetic and environmental conditions which have come together and that will determine our next act (Skinner, 1974).

Education

Skinner's theories have direct applications to education, particularly with regard to controlling classroom behavior and motivating students to learn. Indeed, when looking at the big picture, the challenges facing educators that Skinner wrote about in the 1970s sound very much like the challenges in education today (Skinner, 1978). Teachers are being asked to do more, to address new and different material in their classrooms, and schools face dwindling budgets and rising costs. A reasonable solution: make education more efficient.

Skinner's approach to increasing the efficiency of teaching was to rely on programmed instruction, either through teaching machines (see, e.g., Skinner, 1959) or specially designed books (e.g., Holland & Skinner, 1961). When I was a teaching assistant at Wayne State University in Detroit, we used The Analysis of Behavior by Holland and Skinner for laboratory sections of the learning course. It proved to be both efficient and effective. Unfortunately, however, programmed instruction is just that, a systematic program, and it takes up time that might otherwise allow for meaningful and stimulating relationships between professors and students. Interestingly, one of the strongest trends in higher education today is to shift from lecture-based classes to learner-centered education. But this is done with the intent of increasing the active participation of students within the classroom, not to isolate them in programmed instruction.

In defense of Skinner's approach, it is true that his simple teaching machines and books were only a start. Today we have access to marvelous educational programs on computer, and most of them are anything but boring. Some of the educational programs available for children are fascinating and fun games, and that may be wonderful for children. But is the same approach appropriate for college-level students? In time, perhaps, technology will bring us yet other innovative approaches that combine the best of programmed instruction and human interaction.

Discussion Question: Skinner proposed that education could be made more efficient and effective through the use of programmed learning and teaching machines. Have you ever experienced either of these approaches? Did you feel that you were getting the most out of your education in these situations?

Language

One of the most controversial areas to which Skinner applied his behavioral theories was that of language. It took Skinner over 20 years to write Verbal Behavior (Skinner, 1957), but in the end he presented an analysis of language in which he argued that even our most complex verbal behavior could be understood in terms of simple behavioral contingencies. Skinner began by considering whether there is any difference between speech and any other behavior. For example, what is the difference between using the word water when asking for a glass of water and using the arm to reach for that glass of water? In looking at the beginnings of verbal behavior in childhood, Skinner emphasized the simplicity of a young child's early use of single words to convey meaning far beyond the particular word. For example, when a 2-year old says "cookie," they are asking for, and expecting to receive, a cookie that they cannot get for themselves. Skinner referred to such simple one word utterances as a **mand**, which he said was short for several related concepts: command, demand, countermand, etc. When the child says "cookie," they will then receive one (reinforcement) or they will not. If it is too close to dinner, or if the child has already been told no, the child may receive a loud "No!" (punishment). To make a long story short, all complex verbal behavior develops from this simple beginning, taking its more complex variations from the process of shaping, just like any other behavior.

Perhaps even more controversial, Skinner assigned "thought" to the role of subaudible speech. In other words, thinking was nothing more than talking to one's self, or behaving in the roles of both the speaker and the listener, but doing so without making any sounds out loud. As strange as it may sound to consider thought as nothing more than another behavior subject to reinforcement or punishment, if one is willing to accept Skinner's theory on verbal behavior in the first place, he then makes a compelling argument:

…speech is only a special case of behavior and subaudible speech a further subdivision. The range of verbal behavior is roughly suggested, in descending order of energy, by shouting, loud talking, quiet talking, whispering, muttering "under one's breath," subaudible speech with detectable muscular action, subaudible speech of unclear dimensions, and perhaps even the "unconscious thinking" sometimes inferred in instances of problem solving. There is no point at which it is profitable to draw a line distinguishing thinking from acting on this continuum. (pg. 438; Skinner, 1957).

There are those, of course, who do not accept Skinner's theory on verbal behavior. The renowned linguist Noam Chomsky published critical reviews of both Verbal Behavior and, later, Beyond Freedom & Dignity (Skinner, 1971). Bower and Hilgard (1981) consider Chomsky's critiques to be perhaps the most effective in challenging Skinner's viewpoint. Chomsky argued that our knowledge of a series of input-output relationships tells us nothing of behavior in general, but rather we should be examining the internal structure, states, and organization of the organism that produced these unique input-output relationships (the very concepts that Skinner rejected). Most importantly, rather than accepting that Skinner had taken an appropriate scientific approach, Chomsky felt that Skinner had placed unnecessary fetters on the scientific process. Chomsky also adopted the cognitive

perspective that addresses whether a stimulus in the environment really exists in isolation from the individual. In other words, is the nature of a stimulus affected by the perception of the individual (e.g., how might a paranoid person react to a friendly greeting)? Attempts at supporting Skinner's view and answering Chomsky's critique have, according to Bower and Hilgard, simply failed to be effective or persuasive. And so, experimental psycholinguistics has remained with the general disciplines of linguistics and cognitive psychology, rather than becoming a branch of behavioral learning theory (Bower & Hilgard, 1981).

Old Age and Walden Two

Although much of our conditioning takes place during the early years of life, Skinner did not neglect the later years. However, he addressed issues of aging in a decidedly unscientific way, mostly by describing ways in which he had personally dealt with the intellectual challenges of aging. Skinner wrote about a variety of techniques he had found useful in dealing with forgetfulness, fatigue, and a lack of motivation (see Skinner, 1987). More importantly, however, was the need to prepare for old age when young. By preparing for old age, we can meet its challenges in the best possible health and frame of mind. In Enjoy Old Age, co-authored with Dr. Margaret Vaughan, one finds the following advice:

Nevertheless, it is probably easier to be happy when you are young...We do not live in order to be old, and for young people to expect that "the best is yet to be" would be a great mistake. But what comes can be enjoyed if we simply take a little extra thought. (pg. 28; Skinner & Vaughan, 1983)

In this relatively brief book, Skinner and Vaughan recommend a series of practical steps that one might take: do something about old age, keep in touch with the world, keep in touch with the past, think clearly, keep busy, have a good day, get along with people, feel better, recognize death as a necessary end, and play the role of old age with dignity. An important part of the latter step is to have a sense of humor. The realities of old age can be frustrating, but when you can laugh at the lighter side of these challenges, then everyone around you has the chance to feel better too (Skinner & Vaughan, 1983).

Having addressed the full range of human life, Skinner also addressed the very nature of society itself. Actually, it was rather early in Skinner's career that he wrote the controversial novel Walden Two (Skinner, 1948). And yes, this book was a novel, not a scientific study, though it certainly addresses Skinner's scientific endeavors. Walden Two is about a utopia, a society based entirely on behavioral principles. Similar to the challenges Skinner faced in his failed attempt at a career as an author, Walden Two was rejected by two publishers, and it was accepted by Macmillan only when Skinner agreed to also write an introductory textbook for them. Few critics were impressed by the book, and it failed to sell for a dozen years. But eventually it did sell, and became a well known, if still controversial, book (Skinner, 1978). Skinner himself has written interesting reflections on Walden Two and its implications, including a fictional conversation between one of the characters and the late George Orwell (author of 1984; Skinner, 1978, 1987).

Perhaps the most interesting aspect of Skinner's behavioral, utopian society is that it has not remained fictional. At least two communities have been established based on the ideas presented in Walden Two. The first, established in 1967, is the Twin Oaks Intentional Community, located in rural Virginia (www.twinoaks.org). The second, established in 1973, is Los Horcones, located in Sonora, Mexico (www.loshorcones.org.mx). Los Horcones has,

among its many interesting programs, developed special education programs for developmentally delayed children, particularly those suffering from autism. Although both communities have been successful, they have found it difficult to expand.

Mental Illness and Behavior Therapy

Although the topics of mental illness and behavior therapy are better left to a course in abnormal psychology, let’s take a brief look at some of the more dramatic applications of Skinner’s theories to this important topic. Today, an important trend in psychology is community mental health, in which it is common for a team of mental health practitioners, including psychologists, psychiatrists, social workers, and mental health nurses, to come together and combine their unique specialties in the treatment of a variety of mental health issues. Following two conferences in 1953 and 1954, on the development and causes of mental disease, Skinner wrote that it is important for psychology to maintain a narrow focus, not an interdisciplinary one.

Specifically, Skinner believed that psychologists should focus on the significant properties of “mental disease.” He describes the organism (or person) as being under the influence of hereditary and environmental influences, and engaging in behaviors. How we define these variables depends on our perspective. We can refer to genetic influences as instincts or, in humans, as traits and abilities. We can refer to environmental variables, both past and present, as memories, needs, emotions, perception, etc. But we do not have to interpret those factors we cannot observe, and Skinner felt it was not useful to do so (Skinner, 1959).

Skinner did not actually reject the possibility of the existence of a mental apparatus, as described by Freud, but he did consider it outside the realm of psychological science. And as with complex verbal behavior, Skinner believed that if we could sufficiently break down the behavioral contingencies that underlie psychotic behavior, then we would be able to describe its significant properties in behavioral terms. This analysis may someday involve a more detailed understanding of what happens in the nervous system (and in the brain), but that analysis may appropriately belong in psychiatry and/or neurology, not in psychology (Skinner, 1959).

Skinner felt that mental illness centered on issues of control, and the development of abnormal contingencies in the control of behavior. Most people fear control, and Skinner posed the somewhat amusing question: How often do psychotics have delusions about benevolent controllers? (pg. 234; Epstein, 1980). When faced with being controlled, under excessive conditions, individuals may attempt to escape, revolt, or resist passively. Given the complexity of human life, these behaviors can take many forms and can result in many emotional by-products, such as fear, anxiety, anger or rage, or depression (Skinner, 1953). When these conditions become maladaptive or dangerous, a need for psychotherapy arises. Skinner viewed psychotherapy as yet another form of control, but one in which the therapist creates a non-punishing situation that allows the patient to address problematic behaviors. The therapist and the patient can then work out programs that reduce occasions of punishment and increase occasions of reinforcement in the patient’s life. As such, Skinner considered psychotherapy to be somewhat the opposite of religion and governmental agencies, both of which tend to rely on punitive measures to control the behavior of people (Skinner, 1953).

Through it all, Skinner was optimistic about the future of humanity, and he felt that behaviorism would help people to achieve their full potential. In this regard, he was similar to Freud, who felt that psychoanalysis was a fully scientific endeavor, which would also help to advance the development of humanity. The difference between these two great scientists of human behavior lies in how this might come about:

An experimental analysis shifts the determination of behavior from autonomous man to the environment - an environment responsible both for the evolution of the species and for the repertoire acquired by each member…but we must remember that it is an environment largely of his own making. The evolution of a culture is a gigantic exercise in self-control…But no theory changes what it is a theory about; man remains what he has always been. And a new theory may change what can be done with its subject matter. A scientific view of man offers exciting possibilities. We have not yet seen what man can make of man. (pp. 214-215; Skinner, 1971)

Connections Across Cultures: Personality within Society

John Dollard was one of three theorists whom we will cover in this book who made significant contributions to studying racial issues and minority groups. However, unlike Erik Erikson (see Chapter 7) and Gordon Allport (see Chapter 13), both of whom addressed these issues later in their careers, Dollard (trained in sociology and anthropology) began these studies before he made his significant contributions to psychology. This fact had an important influence on his later approach to how personality is learned, and the use he and Miller made of cross-cultural examples in the books they published together. Indeed, as something of a prelude to his work with Miller, Dollard ends the book Caste and Class in a Southern Town (Dollard, 1937) with the following passage:

…It is one of the urgent needs of social psychology to see the life-history problem against the background of class structure and to get life records from persons who are also described from the sociological standpoint. (pg. 459; Dollard, 1937)

Dollard made an important distinction between **class** and **caste**, as it applies to Blacks and Whites living in the Southern United States in the 1930s. Class generally refers to a group of people of similar economic and political status, such as lower (or working) class, middle class or upper class. A caste is a group defined by some social and/or hereditary factor, such as being ethnically Black or White. There are lower class Blacks and Whites, and there are middle class Blacks and Whites. However, in our society it has been the Blacks who were most dramatically oppressed, through the institution of slavery. Thus, Blacks have been relegated to a lower caste, regardless of whether their economic success qualifies them for middle class status, or even whether they are wealthy. This has had a detrimental influence on family structure in the Black community, and with it an important influence of personality development:

Personality formation must be intelligible in terms of patterns in the family. The study of the family as a formal unit has been slighted in this research, but some things are known. One is that the lower-class Negro family differs from the middle-class white family and seems by comparison to be “disorganized.” This is undoubtedly a result of the fact that during slavery days it was impossible for Negroes to approximate white family structure. (pp. 413-414; Dollard, 1937)

Dollard discusses a variety of ways in which White plantation owners worked to eliminate all trace of the African family structure and culture among their slaves, not the least of which was the dramatic removal of them from Africa to America. In many ways the slave owners encouraged pleasure-seeking, lack of discipline, and independence from family. These values helped to keep male slaves somewhat satisfied in spite of their conditions, and they encouraged the independence of children so that they might begin working at a young age (Dollard, 1937). These values slowly established a new culture for Blacks in America, often with negative consequences. After the Civil War, some Blacks also began to adopt the values of the White culture, but they were not accepted by many people in America. This conflict is what ultimately led the Supreme Court of the United States, in the 1954 ruling Brown v. Board Education, to declare that separate is not equal.

Dollard also co-authored Children of Bondage, with a social anthropologist named Allison Davis (Davis & Dollard, 1940). Davis was one of the first Black professors to receive tenure at a university that was not an historically Black institution (the University of Chicago). He successfully challenged the racial bias in IQ tests of the past, helping to eliminate their use in the school systems of many cities, and he was recognized by the U. S. government with a postage stamp in a series that included Dr. Martin Luther King, Jr. and Jackie Robinson. In their work together, Davis and Dollard focus on describing the personalities and socialization of eight adolescent Blacks in the deep south. They emphasize the demands placed on these youth by both their particular caste position and the social class into which they were born. Individuals may attempt to rebel against their feelings of inferiority with regard to class, and class mobility is possible, but not so with one's racial identity:

In studying the status controls operating upon an individual one finds that most persons in our society are disposed to conceal, even from themselves, any inferiority in their social rank…Refusal to acknowledge one's inferior status, and the building of defenses to decrease anxiety on this score are especially complex with regard to class status…Color caste, however, is so clearly and so rigidly defined that persons in the lower-caste society (as well as the general American reader of this book) will exhibit much less psychological resistance to the fact of caste status than to that of class position. (pg. 17; Davis & Dollard, 1940)

It was within this context, recognizing caste and class as socializing factors influencing personality development, that Dollard went on to study learning theories related to personality development:

"But what," one may ask, "is the practical use of studying these class patterns of behavior?"…what good is such knowledge to the student of human nature, and to our society?…is it a valuable tool which will help us to predict behavior in any given situation and in the end to change it?

The writers' studies of the class conditioning of Negro children have convinced them…that when properly understood the sanctions of the class position, as enforced by the family, the clique, and the larger class environment, are among the most important controls in the formation of human habits. In order to understand the powerful grip of this class behavior, we must first examine the social environment in which Negro children learn their habits and the specific methods by which this learning is reinforced. (pg. 259; Davis & Dollard, 1940)

John Dollard and Neal Miller: Psychodynamic Learning Theory

Sigmund Freud felt that only his approach to psychodynamic theory and psychoanalysis would allow for an understanding of human behavior. B. F. Skinner felt the same way about radical behaviorism. But very few psychologists have felt so strongly about one, and only one, approach to psychology. John Dollard and Neal Miller attempted to blend psychodynamic theory with learning theory, and the results were quite successful. Their theories on the relationship between frustration and aggression, social learning (developed more fully by Bandura, Rotter, and Mischel, whom we will cover in the next chapter), and conflict are standard topics in introductory psychology textbooks.

Brief Biographies of John Dollard and Neal Miller

John Dollard (1900-1980) and Neal Miller (1909-2002) were born just a few years and a few miles apart in Wisconsin, though Miller's family soon moved to Washington. Dollard was a generalist, with interests in psychology, anthropology, and sociology, who conducted important research on racial discrimination in the American south. Miller was particularly interested in physiological psychology, and his pioneering work on biofeedback is as famous as anything we will discuss in this chapter. Though pursuing very different careers, their paths crossed at Yale University's Institute of Human Relations.

Dollard received his bachelor's degree from the University of Wisconsin in 1922, and then went to the University of Chicago, where he earned his M.A. and Ph.D. degrees in sociology in 1931. His interests at the time were primarily in sociology and anthropology, and he accepted an assistant professorship in anthropology at Yale University. A year later he became an assistant professor of sociology at Yale's newly formed Institute of Human Relations. Dollard remained at Yale throughout his career, earning the status of professor emeritus in 1969. In addition to his work with Miller, Dollard studied the effects of racial segregation and discrimination in the southern United States, resulting in two landmark books. Dollard also traveled to Germany between completing graduate school and beginning his position at Yale. While there, he studied psychoanalysis and was psychoanalyzed at the Berlin Institute.

Miller received his bachelor's degree from the University of Washington in 1931, where he studied with the well-known learning theorist Edwin Guthrie. He received an M.A. from Stanford University, and his Ph.D. from Yale in 1935. While a graduate student at Yale he studied with Clark Hull, one of the most influential learning theorists. Like Dollard, Miller traveled to Europe after graduate school, and was psychoanalyzed at the Vienna Institute for Psychoanalysis (reportedly he could not afford to be analyzed by Freud himself). Upon returning, Miller joined the faculty at Yale's Institute of Human Relations. He remained there from 1936 to 1941, and it was during these years that worked closely with Dollard.

During World War II Miller conducted psychological research for the Army Air Force, while Dollard remained at Yale and studied the effects of combat on fear. Miller returned to Yale as a professor of psychology, and remained there until 1966. He then left Yale to establish the Laboratory of Physiological Psychology at Rockefeller University, where he retired as professor emeritus. Among his many honors, Miller served as president of the American Psychological Association, he received an award for Outstanding Lifetime Contribution to Psychology from APA, and he received a National Medal of Science from President Johnson in 1964.

Placing Dollard and Miller in Context: Learning Theory in Moderation

Dollard and Miller brought important perspectives into the study of learning and personality. Dollard was a sociologist with strong interests in anthropology. Miller was trained as a learning theorist with a future in physiological psychology. Although two such men might seem an unlikely pairing, their combined perspective opened the door for eclectic approaches to psychology. Both Dollard and Miller had also studied psychoanalysis. This combination of psychoanalysis, sociology, and learning led to some of the most famous theories in psychology: the frustration-aggression hypothesis, social learning, and a theoretical basis for understanding behavior in conflict situations. These studies laid the foundation for social learning and cognitive personality theorists.

In addition, Dollard studied cultural effects on personality development, particularly under oppressive conditions. Once again, his work laid the foundation for an appreciation of cross-cultural studies in psychology. However, despite occasional studies by noted theorists, such as Erik Erikson and Gordon Allport, the field of psychology has only recently begun to make a concerted effort to study cross-cultural issues (Sue, 1999). Thus, in some ways, the fulfillment of Dollard and Miller's legacy remains to be realized.

Learning Theory and the Influence of Clark Hull

As mentioned above, Miller was a student of Clark Hull, one of the most influential learning theorists. As an example of just how influential Hull was, five theorists who advanced his ideas (including Miller, Orval Mowrer [a co-author on Dollard and Miller's first book together], and Ernest Hilgard [whose learning theories text, co-authored with Gordon Bower, is cited in this chapter]) went on to the presidency of the American Psychological Association and also received distinguished scientific contribution awards from APA (Stagner, 1988). Hull's theory is not easy to understand, as it is a complex mathematical model of the variables impinging upon an organism's behavior. Unlike Skinner, Hull focused on the organism that exists between the input and output that were the sole focus of radical behaviorists.

According to Hull, the strength of a response (**net response strength**; E), or its probability of occurring, is determined by the strength of an internal **drive** (D) and the strength of relevant **habits** (H), all within the context of **conditioned inhibition** (**extinction**; _sI_r) and the organisms overall level of response inhibition (e.g., fatigue; I_r). Hull expressed this relationship by using a formula:

E = (H x D) - (_sI_r + I_r).

Hull later modified his theory to take into account non-learning factors, such as the effectiveness of an evoking stimulus (V) and the incentive motivation of stimuli (K), resulting in the modified formula:

E = H x D x V x K.

For a more detailed discussion of learning theory see Theories of Learning by Bower and Hilgard (1981). Clearly, Hull's consideration of psychological factors, such as the quality of stimuli, their value as motivators, internal drives and habits, all of which influence the nature of input-output relationships stands in stark contrast to Skinner's theories. It was within this context that Dollard and Miller attempted to blend learning theory with psychodynamic phenomena.

Discussion Question: Clark Hull proposed a mathematical formula for understanding behavior, based in part on habit and the incentive of rewards. Look at the formula E = H x D x V x K, and consider whether you agree that human behavior can be reduced to mathematics.

Dollard and Miller's Psychodynamic Learning Perspective: Frustration-

Aggression; Social Learning and Imitation; Conflict

Dollard and Miller collaborated on three books, in which they attempted to apply Hull's principles of learning theory to Freudian psychoanalysis: Frustration and Aggression(Dollard, Doob, Miller, Mowrer, & Sears, 1939), Social Learning & Imitation (Miller & Dollard, 1941), and Personality and Psychotherapy (Dollard & Miller, 1950). Like Hull before them, Dollard and Miller emphasized drives and habits. They also addressed theoretical differences in the strength of reinforcers and punishers, and they equated Freud's concept of displacement to the behavioral concept of generalization. Rather than considering psychoanalysis and behaviorism as fundamentally opposed, as Skinner had, Dollard and Miller allowed for a synthesis of these two major schools of psychology. Their vision led to influential and popular perspectives (Bower & Hilgard, 1981; Lundin, 1979; Stagner, 1988).

Dollard, Miller, and their colleagues believed that the relationship between **frustration**and **aggression**is absolute. In other words, aggression is always the result of frustration. Equally true, but not always as obvious, is the fact that frustration always leads to some form of aggression. While it may appear that some people very quickly accept situations in which they do not get their own way (i.e., they are frustrated), it is also true that we learn early in life to restrain our aggressive impulses. Although we may appear to be successful in not responding to frustration with aggression, this restraint is most likely only temporary. We may then aggressively respond after some delay, or toward some other target, but there will eventually be an aggressive response as a result of the initial frustration (Dollard et al., 1939). In addition, such aggression does not have to involve active responses, since passive-aggressive behavior is all too common. Indeed, a **passive-aggressive personality disorder** (identified by Theodore Millon as the **negativistic personality disorder** [Millon, 1996; Millon and Grossman, 2005]; see the Appendix) was included in the DSM-IV-TR for further study, although it did not make it into the DSM-V (American Psychiatric Association, 2000, 2013). This is an essential contrast to the view of Skinner. Radical behaviorism does not do a very satisfying job of relating responses and stimuli following long delays, not does it easily address the displacement of aggression. However, by accepting internal psychological processes, more flexible learning theories can incorporate factors such as memory, reasoning, emotion, etc. into the relationships between us and our environments.

Aggression comes in many forms, and Dollard, Miller, and their colleagues addressed a wide variety of social factors, including child and adolescent development, criminality, differences between democracy, fascism, and communism, and the nature of aggression amongst the Ashanti. The Ashanti are a West African tribe of people who were considered powerful and warlike. Their fierceness in intertribal battle was said to strike terror into their enemies. Based on anthropological records (primarily the writings of a Captain Rattray), Dollard, Miller, et al. (1939) described a variety of aspects of the Ashanti culture within the framework of the **frustration-aggression hypothesis**. For example, being captured during war was considered treason, and treason was punishable by death. Thus, it was expected

that a warrior would never surrender, but rather would fight to the death. However, a warrior's primary goal during war was to win, and then return home to the family and way of life they were fighting for. If it became clear to a warrior that he was going to lose, his desire to survive should be foremost in his mind. However, surrendering was punishable by death, so it was not an option. This created profound frustration! The response might then be an aggressive rejection of the culture that demanded his death, and he might go ahead and surrender (as often happened). Another shocking example involved the claiming of infants. In Ashanti culture children were cherished. For the first eight days after birth, an infant was kept in very simple conditions, hidden from any outside observers. Then, after the 8 days, the parents would claim the child in a public ceremony. The child would be named, dressed in fine clothes, given a special sleeping mat, and brought out into the community. If the child died during those first 8 days, while the family was making its preparations for the celebration, the result was a very painful frustration. Consequently, the custom was to violently abuse the body, whipping and mutilating it, and finally burying it in the women's latrine! As shocking as this may seem, it represents an extreme example of the frustration-aggression hypothesis in a culture where aggression plays an important role.

In a study relating Freud's defense mechanism of displacement to the behavioral concept of generalization, Miller conducted a study in which rats were trained to strike each other (see Lundin, 1979). Very simply, a mild electrical shock was applied to the floor of a cage containing two rats, and as soon as the rats bumped into each other the shock was turned off. The rats quickly learned to strike each other as soon as the electrical current was turned on. A white, plastic doll was then added to the cage, and when the electrical current was turned on the rats quickly struck each other. Then, when one of the rats was placed in the cage alone, and the electrical current was turned on, the rat struck the doll. Whether this is displacement or generalization depends simply on whether it is defined by a Freudian or a behaviorist, respectively. Although Dollard, Miller, et al. defined the frustration-aggression hypothesis as we know it today, they credit Freud with "the most systematic and extensive use of the frustration-aggression hypothesis," which led to Freud's proposal of the death instinct and its underlying aggressive energy (Dollard, et al., 1939).

In their second book together, Social Learning & Imitation (Miller & Dollard, 1941), Miller and Dollard addressed the roles of culture and society in the learning process:

…To understand thoroughly any item of human behavior - either in the social group or in the individual life - one must know the psychological principles involved in its learning and the social conditions under which this learning took place. It is not enough to know either principles or conditions of learning; in order to predict behavior both must be known. The field of psychology describes learning principles, while the various social science disciplines describe the conditions. (pg. 1; Miller & Dollard, 1941)

Given Dollard's background in sociology and anthropology, it should not be surprising that they gave such emphasis to the social conditions surrounding learning. Indeed, in the preface to this book they discuss the difficulty they had communicating with one another, since they each brought very different backgrounds into this combined effort. They then defined learning theory as the study of the circumstances under which responses and cue stimuli become connected, and they focused their studies on **imitation** and **copying**.

According to Miller and Dollard (1941), there are four essential factors involved in learning: the cue, the response, drive, and reward. In simple terms, in the presence of an appropriate

signal (the cue), the person responds with a particular behavior, if there is an adequate reward (based on learning). The entire process will not take place, however, if the individual does not want a reward (drive). The normal order, after learning has taken place, would then be: drive → cue → response → reward; when we want something, and we see a signal that it is available, we try to get it, and are rewarded for our actions. Another important aspect of social learning is that both drives and rewards can be acquired. For example, food choices are highly cultural. Would you enjoy a dinner of grubs, whale blubber, and sheep entrails, followed by a few crunchy grasshoppers for dessert? And yet, each of these items is commonly eaten in some cultures, so they must be reinforcers for hungry people there.

As an additional example of the importance of social learning, Dollard and Miller once again turned to the anthropological literature. The Semang people of the Malay Peninsula are quite different than the Ashanti described above. The Semang are known as excessively shy and timid people. When faced with hostility they never respond in kind. Instead, they just retreat farther into the jungle. Dollard and Miller questioned whether this behavior was due to some personality trait or to learning. The Semang often dealt with the more powerful Malay tribe. The Malay often cheated the Semang, stole their land, and enslaved some of their people. When the Semang resisted, they were severely punished by the Malay. As a result, escape behavior among the Semang was rewarded, whereas aggression was severely punished. As this became engrained within the culture of the Semang, it was no longer necessary for each generation to re-learn these contingencies, withdrawal and escape became the cultural norm for the Semang people (Dollard & Miller, 1941). To put it in the terms used above, there is a drive for survival. In the presence of aggressive Malays (a cue), escape (the response) is rewarded with safety (drive → cue → response → reward).

In perhaps their most influential book, Personality and Psychotherapy, Dollard and Miller (1950) advocated an eclectic approach to psychology. They believed that the “ultimate goal is to combine the vitality of psychoanalysis, the rigor of the natural-science laboratory, and the facts of culture” (Dollard & Miller, 1950). They also felt it was important for all psychologists, even experimental psychologists, to understand psychotherapy, since it provides a window to higher mental life. They proposed that psychiatric clinics are full of patients showing three common signs: misery, stupidity (in some ways), and neurotic symptoms. Basically, Dollard and Miller believed that **conflict** produces the misery, that repression produces the stupidity (perhaps we should say “unconsciousness” or “unaware”), and that neurotic symptoms reduce conflict. However, neurotic symptoms do not solve conflicts, they only mitigate the conflict. In order to better understand these conflicts, Dollard and Miller presented a detailed analysis of conflict, which became the best-known portion of their final book together (Dollard & Miller, 1950).

Psychotherapy may take place in a therapist’s office, but misery and conflict can only be relieved in real life. Dollard and Miller give credit to Freud for making this observation, but there is a distinct difference between how Freud viewed conflict, as something primarily intrapsychic (between the id, ego, and superego), and how Dollard and Miller viewed conflict, as a choice between opposing options. Conflict arises when we must make a choice. Some stimuli are rewarding, some are punishing, and some can be both. We tend to approach a rewarding stimulus, and that tendency grows stronger as we get closer to it. Likewise, we tend to move away from a punishing stimulus, and our tendency to move away is stronger if we are close to it. Two other important differences, according to Dollard and Miller (1950), are that the strength of avoidance increases more rapidly as we get closer to

punishing stimuli than does the strength of approaching a rewarding stimulus, and the strength of both varies with their associated drives. These relationships are easier to understand when viewed graphically.

Three types of conflict are most commonly discussed. In **approach-approach conflicts**, the individual must make a choice between two rewarding stimuli, such as having ice cream or cookies for dessert. This type of conflict is easily resolved, since one reward is likely to be more inviting (perhaps you haven't had ice cream for a while). A much more difficult situation is an **approach-avoidance conflict**. In this situation, from a distance the rewarding aspect of the stimulus is stronger, so we approach the stimulus. As we approach, however, the drive to avoid the stimulus grows rapidly, until we move away. For example, when an abusive parent tries to comfort a child (something an abused child cannot trust), there is a drive to move toward the parent for comfort (approach) as well as a drive to avoid further abuse (avoidance). Thus, the child may begin to approach the parent, but as they move closer they become more fearful and anxious, so they back away. This type of conflict can clearly create the sort of misery that Dollard and Miller were writing about. Finally, we have **avoidance-avoidance conflicts**, where both of our choices will result in punishment. For example, if a child has misbehaved, and their parent is going to punish them, the natural response is to run away. With young children this situation can be very easy to observe. However, many parents will then yell: "Don't you dare run away from me!" Now the child must choose between being punished (avoidance) or running away and being punished worse (avoidance). In this situation, the typical response is to freeze, and make no choice at all. The child may still be punished, but they will not have chosen the punishment.

Addressing the nature of an individual patient and their levels of approach and avoidance in conflicting situations is critical. According to Dollard and Miller, neurotic patients generally have high levels of avoidance. The key here is that these people have become patients. Many people suffer, and are urged by their friends to work on improving their situation. When those individuals have relatively low levels of avoidance motivation, they may well be successful in taking care of their problems. It is the ones who cannot overcome their avoidance issues who end up in therapy. Thus, if the therapist attempts to encourage the patient to approach their feared goals, the therapist will only increase the patient's fear and conflict, and the resulting misery will drive the patient out of therapy (Dollard & Miller, 1950). Instead, the therapist must focus on reducing the fears that motivated the patient's avoidance in the first place.

Overall, Dollard and Miller emphasized that psychotherapists must be well-trained, open-minded, stable individuals who put the interests of their patients first.

We have emphasized the precautions important to psychologists who work as psychotherapists. In the same connection we stress that the ability to treat organic disease does not automatically carry with it a skill at psychotherapy. Nor does the possession of any degree such as Ph.D. or M.D. routinely confer such skill. Only the knowledge of theory, the kind of character, and the supervised training discussed here can make a man a psychotherapist. Anyone who undertakes psychotherapy without such training is exposing his patient to real danger and committing a moral, if not yet a legal, fraud. (pg. 422; Dollard & Miller, 1950)

Discussion Question: Dollard and Miller described how the Semang culture seems to lack aggression, as a result of their learning the danger of challenging the Malay. Can you

recognize examples in your own life where a certain group seems to act in a predictable way as the result of how another group acts? If yes, how has one group reinforced or punished the behavior of the other in consistent ways, in keeping with the predictable behavioral outcomes?

Psychoanalysis, Behavior Therapy, and Relational Models

Paul Wachtel has continued studying the relationship between psychoanalysis and behavior therapy, following in the footsteps of Dollard and Miller, and he has also extended this comparison into the “relational world” (Wachtel, 1977, 1997). Wachtel emphasized that psychotherapy should be understood in terms of the theory that guides it, and that the practice of therapy provides the true test of any psychological theory related to personality and/or mental illness. Behavior therapy is appealing in terms of being direct and practical, but Wachtel feels that it can gain in several ways from being blended with psychoanalysis. He questions whether pure behavior therapists can really assess psychological conditions, whether they achieve narrow gains at the expense of broader psychological change, and he addresses the ethical implications of being involved in specifically changing another person’s behavior. He also addresses several problems that behaviorists see for psychoanalysis, such as the focus on internal states that cannot be observed, whether underlying causes are really treated instead of symptoms, and the application of the medical model to psychological processes.

It is my general premise that psychodynamic and behavioral approaches to psychotherapy, and to the understanding of personality, are far more compatible than is generally recognized, and that an integration of the concepts and observations accumulated by these two approaches can greatly enrich our clinical work and our understanding of human behavior…It is my experience that workers guided by either of these two broad frames of reference tend to have only a rather superficial knowledge (and sometimes none at all) of the important regularities observed by those guided by the other viewpoint. (pg. 5; Wachtel, 1977)

In Psychoanalysis, Behavior Therapy, and the Relational World, Wachtel (1997) discusses advances in the recognition of object relations theory in American psychology, and how object relations theory fits into connections between traditional psychodynamic perspectives and learning theory. Although it was not necessarily the intent of object relations theorists, object relations theory has shifted the focus of psychoanalysis from internal psychological conflicts to relationships with other people. Thus, there has been a shift from an internal focus to an external focus, which is more compatible with learning theory (and the input-output contingencies that are essential to learning). Wachtel also considers this shift as favorable for connections between psychoanalysis and family systems approaches to therapy. Surprisingly, Wachtel makes no mention of relational-cultural theory and the work of the Stone Center Group, which seems ideally suited to the integration of approaches proposed by Wachtel. Regardless of the nature and extent to which these various approaches might be integrated, it remains of the utmost importance that the focus of the therapist remains on helping the patient (Wachtel, 1997).

Cultural Effects on Learning

As shown above, Dollard and Miller regularly incorporated interesting cultural examples in their work relating learning theory to psychodynamic processes. Though it may not be

obvious, Skinner's radical behaviorism is as intercultural as any theory. A detailed analysis of environmental influences on behavior, including past contingencies and present cues, must incorporate an examination of the unique cultural factors that are part and parcel of those contingencies in different cultural groups. In addition, of course, some psychologists today continue to study relationships between culture and learning.

Tweed and Lehman (2002) compared Western and Chinese learning styles by using two extraordinary teachers from ancient times as examples of these potentially different styles: Confucius and Socrates. Their article begins with an attempt to address the likely controversy that accompanies such a study. They carefully and thoughtfully point out that one can easily misunderstand any contrast between cultures that are difficult to define. For example, what does "Western" mean? Does it really mean European-Americans, or does that leave out Canadians, Australians, and non-European Westerners? So, Tweed and Lehman prefer the terms **culturally Western** and **culturally Chinese**, openly admitting that another important issue is that some Western people may be culturally Chinese and vice versa. They also point out their study was meant to be descriptive, not judgmental. Having presented such caveats, they proceeded with their study, and yet received some criticism nonetheless (Gurung, 2003; Li, 2003; see also the response to these critiques by Tweed & Lehman, 2003).

Tweed and Lehman offer the following generalizations about culturally Western and culturally Chinese learning: culturally Western learning focuses on overt and private questioning, expressing personal hypotheses, and a desire for self-directed tasks, whereas culturally Chinese learning emphasizes effort-focused learning, pragmatic orientations, and acceptance of behavior reform as an academic goal (Tweed & Lehman, 2002). In keeping with their non-judgmental attitude, they suggest that these disparate approaches both have their place in education, and that the ideal situation for students would be one that is academically bicultural, an environment that offers the opportunity for the strengths of each approach to learning to come out:

…These students would be in a sense academically bicultural and could operate adaptively within environments requiring Confucian or Socratic approaches…Educators…would encourage both thoughtful acquisition (Confucian) and inquiry (Socratic) such that students acquire knowledge and thinking skills that become fully understood, active, and elicited in many domains beyond the academic context. (pg. 97; Tweed & Lehman, 2002)

One of the individuals who commented on Tweed and Lehman's article, Jin Li, has focused more directly on how cultural factors influence learning itself. As essential difference between culturally Western and culturally Chinese learning is that Socrates proposed that the best learners develop and use their minds to inquire into the world, whereas the great Chinese tutor Mencius taught that becoming a better, more virtuous person is the most essential quality for a learner (Li, 2005). This perspective is reminiscent of Wober's study on intelligence amongst the Baganda people in Africa. They consider intelligence to be something closer to what we would call wisdom, and their educational system is focused on an individual's ability to succeed by conforming to the expectations of society, rather than on the ability to solve new and independent problems (Wober, 1974). Since cultural attitudes and beliefs develop early in life (Ferraro, 2006a; Matsumoto & Juang, 2004), a valuable educational objective would be to emphasize the strengths of each approach to learning as early as possible in the school years (or even during the preschool years).

In addition to broad-based effects of culture on learning in general, culture also comes into play for many of the aspects of learning we have examined in this chapter. Henrich et al. (2006) recently demonstrated that a wide variety of societies are willing to participate in costly punishment in order to encourage cooperation among groups. In other words, each party to an agreement is motivated to abide by the agreement because the consequences of breaking it are severe. This study involved fifteen different cultural groups from Africa, North America, South America, Asia, and Oceania, suggesting that willingness to cooperate as a function of severe punishment is universal. Language, which Skinner believed is learned just like any other behavior, appears to be essential for the development of autobiographical memory, which serves primarily social and cultural functions and is intimately related to social and cultural development (Fivush & Nelson, 2004). Presenting an interesting perspective on conflict, Eidelson and Eidelson (2003) have identified five belief domains that propel groups toward conflict. When individuals experience feelings of superiority, injustice, vulnerability, distrust, or helplessness, there is a good chance they will feel frustrated. When their individual-level core beliefs parallel the group-level worldview, the situation may trigger or constrain conflict or, possibly, trigger violent struggle (Eidelson & Eidelson, 2003). In such situations, the worldview of the larger group may be serving as a discriminative stimulus that it is acceptable to act out on one's individual frustration, since the society in which one lives is likely to reinforce any subsequent aggression (since they share the individual's frustration). In the next two chapters, we take a closer look at how the social environment and cognitive processes contribute to the development of our personality.

Personality Theory in Real Life: Positive Reinforcement Keeps Us Behaving!

The presence of positive reinforcement in our lives can be seen in a multitude of ways. We use it to encourage our children and train our pets, it is quite common in education, from the earliest grades through college, and it enters into our lives as adults in many ways. Often the application of reinforcement is not intentional, we give it out of habit, and partly because it leads to reinforcement for us as well. Although learning theorists may disagree about the nature of reinforcement, no one can deny it.

There are many ways in which positive reinforcement is given to children. Infants respond favorably to attention, and as they coo and smile back, that attention is itself reinforced. This helps to create an essential bond between the child and its caregivers. In the case of premature infants, gentle physical contact (massaging or stroking) by the parents is typically reinforced as a result of the babies gaining weight more rapidly and being more active (Field, 1993). In other words, caring for an at-risk infant is reinforced, and thus the caring behavior continues. Infant massage is quite popular in many countries around the world, and it is part of the total approach to therapy recommended by Wilhelm Reich (Field, 2000, 2001).

As children grow a little older, **secondary reinforcers** such as encouraging words ("You can do it!") or clapping are commonly used to reinforce taking first steps or solving a simple puzzle. Of course, **primary reinforcers** (e.g., crackers or juice) remain very popular as well. Such simple examples of reinforcement are certainly not unique to humans. Small pieces of food are often used to train dogs, and the reinforced behavior increases as a result, just like it does with children. While this demonstrates the basic universality of behavioral principles, it does not suggest that dogs have a personality. However, if you were to ask most pet

owners, they will tell you that their animals do have personalities. And junk yard dogs don't become vicious as a result of being treated with loving kindness.

In educational settings, positive reinforcement is used quite intentionally, and can be a lot of fun with young children. Teachers can use a wide variety of stars, smiley face stickers, student of the week awards, and other little treats that children find very rewarding. Sometimes children receive "good citizen" awards when their teachers observe an occasion of **prosocial behavior** (such as helping another child who was hurt on the playground). Each of these reinforcers is intended to increase those behaviors that are viewed favorably in the educational environment (completed assignments, good grades, and good citizenship). As students get older, the reinforcers come less often (as with schedules of reinforcement). Grades are given out at the end of a term or semester (reinforcing, of course, only if they are good grades). Measures such as being Valedictorian, in the top 10 of the class, being on the Dean's List in college, or graduating summa cum laude are the pinnacles of this positive reinforcement hierarchy, yet they depend on long time intervals (some as long as the typical four years of high school or college).

As adults, our reinforcement often becomes more complicated. Based on Hull's formula H x D x V x K, consider the effect of being offered chocolate for dessert. After eating dinner, our drive for food should decrease, and we should not be likely to eat more. However, chocolate is a powerful incentive, so we may go ahead and eat it even if we are no longer hungry. Skinner's radical behaviorism cannot account for the phenomenon of eating something we really like when we have already eaten our fill. And yet many of us have experienced that desire to continue eating something we really like, even after we feel uncomfortably stuffed with food! One might also consider Dollard and Miller's frustration-aggression hypothesis, and the problem many people face when trying to lose weight. It is extremely difficult to stay on a low calorie diet, especially if we cannot eat the foods we like. Very simply, this is frustrating. According to Dollard and Miller, frustration always leads to aggression. How might this aggression be manifested? We return to overeating, even though it is bad for us. In other words, we are harming ourselves and/or rebelling against a culture that seems to demand being thin in order to be attractive. Interestingly, many diet programs have responded to this problem (whether or not they understood it this way) by providing either tantalizing recipes or by offering the food itself in prepackaged form. The hope is that the meals will themselves be reinforcing, thus eliminating the frustration caused by munching endless amounts of celery and rice cakes. Of course, these days you can buy quite an interesting variety of flavored rice cakes, so even they can be reinforcing for some people!

Review of Key Points

• Skinner emphasized the experimental analysis of human behavior, despite its complexity. He addressed causes and effects, but was very precise in describing a cause as a change in an independent variable and an effect as a change in a dependent variable.

• Reinforcers increase the likelihood of behaviors that precede them; punishers decrease the likelihood of behaviors that precede them.

• Positive reinforcement involves giving a reinforcer, negative reinforcement involves removing an aversive or noxious stimulus.

• Positive punishment involves applying a punisher, negative punishment involves removing reinforcers.

• Discriminative stimuli signal the behavioral contingencies in effect at a given time.

• Skinner automated much of his research, inventing the operant conditioning chamber (aka, the Skinner box). The data were typically collected in the form a cumulative record.

• The operant conditioning chamber was designed to control the schedule of reinforcement. These schedules could be either fixed or variable, and based on either ratios (number) or intervals (time).

• Operant conditioning can lead to complex behavior by shaping the behavior in a series of small steps.

• When a subject accidentally forms an inappropriate association, superstitious behavior can result.

• Skinner claimed that our typical idea of the person, or the self, is a vestige of animism, the belief that we are inhabited by spirits. Instead, he believed we are simply a locus for the convergence of genetic and environmental conditions.

• According to Skinner, education can be made more efficient by using programmed instruction and teaching machines.

• Language, according Skinner, begins with the association of simple word elements known as mands.

• Skinner believed that behavioral principles could be applied to improve life in old age as well as to improve society itself.

• In attempting to address mental illness, Skinner did not rule out the possibility of a disordered mind. However, he felt that such a construct served no useful purpose in understanding abnormal behavior. He also believed that mental illness focused on issues of control.

• Clark Hull, a major influence on Dollard and Miller, proposed a mathematical model of learning that included the nature of the mind and experience.

• Dollard and Miller believed that frustration always led to aggression, and that aggression could always be traced back to frustration. They applied their frustration-aggression hypothesis to a variety of cultural groups.

• Dollard and Miller began the field of social learning, with an emphasis on the learning processes of imitation and copying.

• When addressing psychological disorders, Dollard and Miller emphasized conflict. They provided a theoretical basis for understanding approach-approach conflicts, approach-avoidance conflicts, and avoidance-avoidance conflicts.

• By incorporating an appreciation for the contribution of object relations theory, Wachtel has advanced our understanding of how learning theory and psychodynamic theory can be combined in an eclectic approach to psychotherapy.

• Tweed and Lehman caution against regional stereotypes when examining cultural aspects of learning. They propose using the terms “culturally Western” and, for their particular study, “culturally Chinese.”

• Since culture is learned early in life, any attempt to incorporate the best aspects of different cultural approaches to education must begin early.

Attribution and critical-reading note

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Read historical and theoretical claims critically. A theory is a lens for inquiry, not a verdict about a child or family. Where source claims are dated or culture-bound, the new lecture supplies cautions and contemporary framing.

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Psychotherapy: Behavior Therapy

In **psychoanalysis**, therapists help their patients look into their past to uncover repressed feelings. In **behavior therapy**, a therapist employs principles of learning to help clients change undesirable behaviors—rather than digging deeply into one's unconscious. Therapists with this orientation believe that dysfunctional behaviors, like phobias and bedwetting, can be changed by teaching clients new, more constructive behaviors. Behavior therapy employs both classical and operant conditioning techniques to change behavior.

One type of behavior therapy utilizes classical conditioning techniques. Therapists using these techniques believe that dysfunctional behaviors are conditioned responses. Applying the conditioning principles developed by Ivan Pavlov, these therapists seek to recondition their clients and thus change their behavior. Emmie is eight years old, and frequently wets her bed at night. She's been invited to several sleepovers, but she won't go because of her problem. Using a type of conditioning therapy, Emmie begins to sleep on a liquid-sensitive bed pad that is hooked to an alarm. When moisture touches the pad, it sets off the alarm, waking up Emmie. When this process is repeated enough times, Emmie develops an association between urinary relaxation and waking up, and this stops the bedwetting. Emmie has now gone three weeks without wetting her bed and is looking forward to her first sleepover this weekend.

One commonly used classical conditioning therapeutic technique is **counterconditioning**: a client learns a new response to a stimulus that has previously elicited an undesirable behavior. Two counterconditioning techniques are aversive conditioning and exposure therapy. **Aversive conditioning** uses an unpleasant stimulus to stop an undesirable behavior. Therapists apply this technique to eliminate addictive behaviors, such as smoking, nail biting, and drinking. In aversion therapy, clients will typically engage in a specific behavior (such as nail biting) and at the same time are exposed to something unpleasant, such as a mild electric shock or a bad taste. After repeated associations between the unpleasant stimulus and the behavior, the client can learn to stop the unwanted behavior.

Aversion therapy has been used effectively for years in the treatment of alcoholism (Davidson, 1974; Elkins, 1991; Streeton & Whelan, 2001). One common way this occurs is through a chemically based substance known as Antabuse. When a person takes Antabuse and then consumes alcohol, uncomfortable side effects result including nausea, vomiting,

increased heart rate, heart palpitations, severe headache, and shortness of breath. Antabuse is repeatedly paired with alcohol until the client associates alcohol with unpleasant feelings, which decreases the client's desire to consume alcohol. Antabuse creates a conditioned aversion to alcohol because it replaces the original pleasure response with an unpleasant one.

In **exposure therapy**, a therapist seeks to treat clients' fears or anxiety by presenting them with the object or situation that causes their problem, with the idea that they will eventually get used to it. This can be done via reality, imagination, or virtual reality. Exposure therapy was first reported in 1924 by Mary Cover Jones, who is considered the mother of behavior therapy. Jones worked with a boy named Peter who was afraid of rabbits. Her goal was to replace Peter's fear of rabbits with a conditioned response of relaxation, which is a response that is incompatible with fear (). How did she do it? Jones began by placing a caged rabbit on the other side of a room with Peter while he ate his afternoon snack. Over the course of several days, Jones moved the rabbit closer and closer to where Peter was seated with his snack. After two months of being exposed to the rabbit while relaxing with his snack, Peter was able to hold the rabbit and pet it while eating (Jones, 1924).

Source figure caption: Exposure therapy seeks to change the response to a conditioned stimulus (CS). An unconditioned stimulus is presented over and over just after the presentation of the conditioned stimulus. This figure shows conditioning as conducted in Mary Cover Jones' 1924 study.

Thirty years later, Joseph Wolpe (1958) refined Jones's techniques, giving us **systematic desensitization**. Using this method, a person creates a hierarchy of anxiety, ranging from the least-anxiety-producing stimulus to the feared object. These activities are paired with relaxation techniques, which are taught to the client beforehand and used during the graduated exposures. The idea is that a person cannot be nervous and relaxed at the same time. Therefore, if they can learn to relax when they are facing environmental stimuli that make them nervous or fearful, they can eventually eliminate the unwanted fear response. ().

Source figure caption: This person suffers from arachnophobia (fear of spiders). Through exposure therapy they are learning how to face their fear in a controlled, therapeutic setting. (credit: "GollyGforce – Living My Worst Nightmare"/Flickr)

For example, Jayden is terrified of elevators. Nothing bad has ever happened to him on an elevator, but he's so afraid of elevators that he will always take the stairs. That wasn't a problem when Jayden worked on the second floor of an office building, but now he has a new job—on the 29th floor of a skyscraper in downtown Los Angeles. Jayden knows he can't climb 29 flights of stairs in order to get to work each day, so he decided to see a behavior therapist for help. The therapist asks Jayden to first construct a hierarchy of elevator-related situations that elicit fear and anxiety. They range from situations of mild anxiety such as being nervous around the other people in the elevator, to the fear of getting an arm caught in the door, to panic-provoking situations such as getting trapped or the cable snapping. Next, the therapist uses progressive relaxation. They teach Jayden how to relax each of his muscle groups so that he achieves a drowsy, relaxed, and comfortable state of mind. Once he's in this state, the therapist asks Jayden to imagine a mildly anxiety-provoking situation. Jayden is standing in front of the elevator thinking about pressing the call button.

If this scenario causes Jayden anxiety, he lifts his finger. The therapist would then tell Jayden to forget the scene and return to his relaxed state. They repeat this scenario over and over until Jayden can imagine himself pressing the call button without anxiety. Over time the therapist and Jayden use progressive relaxation and imagination to proceed through all of the situations on Jayden's hierarchy until he becomes desensitized to each one. After this, Jayden and the therapist begin to practice what he only previously envisioned in therapy, gradually going from pressing the button to actually riding an elevator. The goal is that Jayden will soon be able to take the elevator all the way up to the 29th floor of his office without feeling any anxiety.

Sometimes, it's too impractical, expensive, or embarrassing to re-create anxiety- producing situations, so a therapist might employ **virtual reality exposure therapy** by using a simulation to help conquer fears. Virtual reality exposure therapy has been used effectively to treat numerous anxiety disorders such as the fear of public speaking, claustrophobia (fear of enclosed spaces), aviophobia (fear of flying), and post-traumatic stress disorder (PTSD), a trauma and stressor-related disorder (Gerardi, Cukor, Difede, Rizzo, & Rothbaum, 2010).

Note: A new virtual reality exposure therapy is being used to treat PTSD in soldiers. Virtual Iraq is a simulation that mimics Middle Eastern cities and desert roads with situations similar to those soldiers experienced while deployed in Iraq. This method of virtual reality exposure therapy has been effective in treating PTSD for combat veterans. Approximately 80% of participants who completed treatment saw clinically significant reduction in their symptoms of PTSD, anxiety, and depression (Rizzo et al., 2010). Watch this Virtual Iraq video that shows soldiers being treated via simulation to learn more.

Some behavior therapies employ operant conditioning. Recall what you learned about operant conditioning: We have a tendency to repeat behaviors that are reinforced. What happens to behaviors that are not reinforced? They become extinguished. These principles, defined by Skinner as operant conditioning, can be applied to help people with a wide range of psychological problems.

One popular operant conditioning intervention is called the **token economy**. This involves a controlled setting where individuals are reinforced for desirable behaviors with tokens, such as a poker chip, that can be exchanged for items or privileges. Token economies are often used in psychiatric hospitals to increase patient cooperation and activity levels. Patients are rewarded with tokens when they engage in positive behaviors (e.g., making their beds, brushing their teeth, coming to the cafeteria on time, and socializing with other patients). They can later exchange the tokens for extra TV time, private rooms, visits to the canteen, and so on (Dickerson, Tenhula, & Green-Paden, 2005).

CHAPTER 11 / CAC.902

Cognitive Therapy and CBT

Cognitive therapy, associated with Aaron Beck, proposes that emotional distress is shaped not only by events but by the meanings people give to events. Cognitive-behavioral therapy, or CBT, combines this cognitive focus with behavioral methods. It is one of the most researched forms of psychotherapy. For child and adolescent counseling, CBT is often adapted to be concrete, active, and developmentally appropriate. A worksheet is not CBT. The core is collaborative empiricism: therapist and client examine thoughts, feelings, behaviors, and evidence together.

The cognitive model

A situation triggers an automatic thought, which influences emotion and behavior. A student receives a low grade and thinks, "I'm stupid." The thought produces shame and avoidance. Another student thinks, "I need a different study plan," and feels disappointment but seeks help. The event is similar; the meaning differs. Cognitive therapy does not say that thoughts cause everything. It says that interpretations are one leverage point. In children, automatic thoughts may be images rather than words. A child may see a picture of being laughed at rather than hear a sentence.

Cognitive distortions and core beliefs

Common distortions include all-or-nothing thinking, catastrophizing, mind reading, overgeneralization, emotional reasoning, and personalization. These are not lies. They are habitual shortcuts that become problematic when they are rigid. Core beliefs are deeper assumptions about self, others, and the world. A child with a core belief "I am unlovable" may interpret a parent's lateness as rejection. The counselor does not argue the child out of the belief. The counselor helps the child notice the belief, test it, and build alternative experiences.

Collaborative empiricism and Socratic questions

The counselor asks questions that help the client evaluate thoughts. "What evidence supports that idea? What evidence does not fit? What would you tell a friend? If the worst happened, how would you cope?" The tone is curious, not cross-examining. With young children, the counselor may use drawings, puppets, or a detective game. The child becomes a scientist of their own mind. The counselor should avoid turning Socratic questioning into a debate the counselor must win. If the client says, "I know it's true," the counselor can explore the feeling and the history rather than push harder.

Behavioral experiments

A behavioral experiment tests a prediction in real life. A socially anxious teen predicts that if they ask a question in class, everyone will laugh. The counselor and teen design a small test: ask one question in a supportive class and record what happens. The experiment is not a trick to prove the counselor right. It is a way to gather data. If peers do laugh, the counselor helps the teen cope and reevaluate the meaning. The goal is flexible learning, not forced positivity.

CBT with children and adolescents

Developmental adaptation is essential. Younger children may need parent involvement, play, and visual aids. Adolescents may need autonomy, privacy, and relevance to their goals. CBT for anxiety often includes exposure. CBT for depression often includes behavioral activation, cognitive work, and problem-solving. CBT for trauma may include trauma narrative and processing. The counselor should match the protocol to the problem and monitor outcome. A generic CBT package is not enough.

Clinical example

Thirteen-year-old Lina has panic attacks at school. She believes a racing heart means she will faint. The counselor teaches the panic cycle, then designs interoceptive exposure: safely running in place to produce a racing heart and learn that it is uncomfortable but not dangerous. Lina also tests the prediction that leaving class prevents fainting. The counselor involves the school so that escape is reduced gradually and safely. Cognitive work and behavioral exposure reinforce each other. Lina learns, "My body can be loud and I can still be safe."

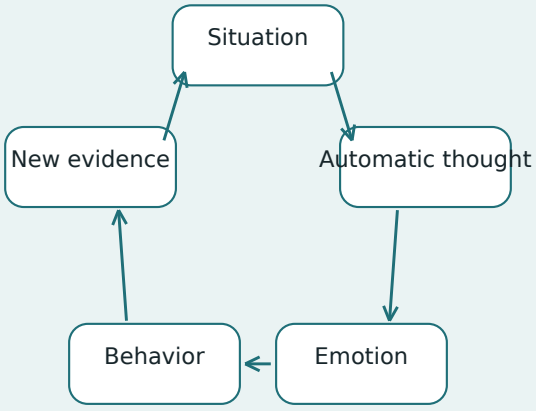
Evidence and limits

CBT has strong evidence for many anxiety and depressive disorders in youth, but effects are not universal. Some clients do not respond, and some protocols are less effective for complex trauma or comorbid conditions. CBT can be misused as a way to tell clients to think positively while ignoring poverty, racism, or abuse. A good CBT counselor validates real threats and targets distorted appraisals only where they exist. The counselor also considers whether the environment must change. Cognitive restructuring cannot make an unsafe home safe.

Reflection

Recall a recent strong emotion. Write the situation, the automatic thought, the emotion, and the behavior. Then ask what evidence supports and challenges the thought. Finally, design a small experiment. Notice whether the exercise feels curious or judgmental. If it feels judgmental, slow down and add compassion. CBT is not about winning an argument with yourself.

The cognitive cycle



CBT tests meanings and actions so that new evidence can update the cycle.

Attributed Foundational Reading

Attribution and critical-reading note

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Read historical and theoretical claims critically. A theory is a lens for inquiry, not a verdict about a child or family. Where source claims are dated or culture-bound, the new lecture supplies cautions and contemporary framing.

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George Kelly's personal construct theory goes beyond the cognitive elements addressed by social learning theorists and provides a full-fledged cognitive theory. Kelly believed that individuals act very much like scientists studying personality: they create constructs, or expectations about the environment and people around them, and then they behave in ways that "test" those beliefs and expectations. For Kelly, the personal constructs are more important than actual reality, since it is the construct that guides cognition and behavior, not the actual situation. His theory was unique, and quite unrelated to others that came before and after. This was, in part, Kelly's very intention:

It is only fair to warn the reader about what may be in store…the term learning, so honorably embedded in most psychological texts, scarcely appears at all. That is wholly intentional; we are for throwing it overboard altogether. There is no ego, no emotion, no motivation, no reinforcement, no drive, no unconscious, no need…all this will make for periods of strange, and perhaps uncomfortable, reading. Yet, inevitably, a different approach calls for a different lexicon... (pg. x; Kelly, 1955a)

Albert Ellis and Aaron Beck are best known for developing therapeutic techniques that are based on a cognitive perspective of personality and behavior. Although they are not known for developing actual theories of personality, their clinical approaches are based on underlying theoretical perspectives, which shed light on how they view the nature of personality. Thus, their influential work is naturally connected to that of Kelly, whose theory of personality was entirely cognitive (as compared to the cognitive social learning theorists Bandura, Rotter, and Mischel). More recently, there have been cognitive approaches to therapy put forth that are connected to much older approaches to human understanding. Acceptance and Commitment Therapy (ACT) takes an experiential approach to changing behavior that shares many similarities to Buddhist approaches (Hayes & Smith, 2005; Hayes, Strosahl, & Wilson, 1999), whereas Radical Acceptance relies directly on Buddhist teachings to encourage people to embrace their own lives (Brach, 2003).

Personal Construct Theory

Kelly's **personal construct theory** departs from cognitive social learning in that he proposes it is not simply enough to know what a person is likely to do in a given situation, even when your predictions are correct. More importantly, we need to know what a person

might have done (Kelly, 1966). Thus, unlike the cognitive social learning theorists who consider cognitive processes as an aspect of the environmental circumstances associated with behavior, Kelly focused on the cognitive constructs first and foremost.

Kelly presented his personal construct theory in a two volume set, which was published in 1955 (Kelly, 1955a,b). In the mid-1960s Kelly was preparing a new book that was to include talks and papers he had presented around the world, many of which were never published. Unfortunately, he did not survive to complete this task, but the project was later completed by Brendan Maher (Maher, 1969), thus furthering the information available on this unique theory.

Brief Biography of George Kelly

Not much is known about George Kelly as a person, particularly regarding his childhood. Later in life he instructed his wife to destroy all of his personal correspondence. Thus, it has been somewhat difficult to piece together a picture of how this fascinating individual became the man he was. He was born in 1905 on a farm near Perth, Kansas. His father had been educated for the Presbyterian ministry, but after getting married his parents moved to the farm in Kansas, where they had their only child. The Kelly family moved around from farm to farm, including a failed farm in Colorado on some of the last free land given to settlers in the West. Kelly’s education was erratic, and he learned what he could when the family would occasionally spend a few weeks in town. He attended four different high schools, and apparently never established any long-term relationships. He was, however, favorably influenced by the exciting stories told by his maternal grandfather, who had been the captain of a sailing ship in the North Atlantic (Fransella, 1995).

Despite his erratic education, Kelly attended college at Friends University and then Park College, where he received a Bachelor’s degree in physics and mathematics. Despite studying math and science, the collegiate debates he experienced had sparked a keen interest in social problems. So, he entered the University of Kansas to earn a Master’s degree in educational sociology, and in 1927 he completed his thesis on the distribution of leisure time activities of workers in Kansas City. He then moved to Minneapolis and supported himself by teaching one night a week in each of three different night schools. He began studying sociology and biometrics at the University of Minnesota, but when the university found out that he couldn’t pay his tuition he was told to leave. He was then hired to teach psychology and speech at Sheldon Junior College in Iowa, where he spent a year and a half. He then returned to the University of Minnesota for a semester of studying sociology, but then went back to Wichita, Kansas and worked as an aeronautical engineer for a few months. He then received an exchange scholar fellowship to study at the University of Edinburgh in Scotland, where he earned a Bachelor’s degree in education in 1930, having written his thesis on predicting teaching success. He then returned to the United States, attended the State University of Iowa, and in 1931 he received a Ph.D. in psychology, with a focus on reading and speech disabilities. Two days later he married Gladys Thompson (Fransella, 1995; Maher, 1969).

As uncharacteristic as it may seem, Kelly finally settled down. He spent the next ten years teaching at Fort Hays Kansas State College. His research and writings during this time focused on the practical aspects of providing clinical psychological services for schools. Much like Alfred Adler had in Austria, he developed traveling clinics to provide training around the state of Kansas, and his model had a dramatic influence on the future of rural

school psychology. With the advent of World War II, however, Kelly entered the Navy as an aviation psychologist. At first he helped to train local civilian pilots, but then he transferred to the Bureau of Medicine and Surgery of the Navy in Washington to help select naval air cadets. After the war he spent a year at the University of Maryland, but in 1946 he was appointed Professor and director of Clinical Psychology at Ohio State University, where he spent the next 20 years (Fransella, 1995; Maher, 1969).

During his first few years at Ohio State he focused on making their clinical psychology training program into one of the best in the country. One of his colleagues in the department was Julian Rotter, also known for his role in shaping the training programs used in psychology today, and one of their students was Walter Mischel (who admired both Rotter and Kelly). Kelly then turned his attention to the theory that made him famous, the two volume work entitled The Psychology of Personal Constructs(Kelly, 1955a,b). Kelly's theory gained immediate recognition as both unique and significant, and he was invited to teach and lecture around the United States and in Europe, the Soviet Union, South America, Asia, and the Caribbean. He was elected president of both the Clinical Division and the Consulting Division of the American Psychological Association, and he served as president of the American Board of Examiners in Professional Psychology. In 1965, Kelly left Ohio State for Brandeis University, where he was appointed to the Riklis Chair of Behavioral Science. He began working on a collection of his papers and lectures from the past decade, but he died unexpectedly in March, 1966. As mentioned above, that collection of works was completed by the man who succeeded Kelly as Riklis Professor of Behavioral Science, and was published under the title Clinical Psychology and Personality: The Selected Papers of George Kelly (Maher, 1969).

Placing Kelly in Context: A Cognitive Theory of Personality

Simply put, Kelly's personal construct theory represents the culmination of the shift from animalistic behaviorism to humanistic cognition. In American psychology, behaviorism was a powerful force, and began with the very traditional approach of theorists such as John B. Watson and B. F. Skinner. Alongside the experimental behaviorists were the learning theorists, such as Clark Hull. As Dollard and Miller tried to find some common ground between psychodynamic theory and traditional learning approaches, they were inevitably led to consider the role of social factors in human learning. Bandura, Rotter, and Mischel built on the legacy of Dollard and Miller, but added to it the active role of cognition in the human species. Finally, Kelly moved to a purely cognitive description of how individuals become who they are.

However, we must address an important caveat. Just as Skinner's radical behaviorism was an extreme position of ignoring cognitive processes, perhaps Kelly's position is equally extreme for attributing significant cognitive processing to all aspects of personality and personality development. Although the man-the-scientist concept may hold a certain curious appeal for some, who doesn't sometimes knowingly try acting in different ways to see what effect it has on others? Likewise, who isn't attracted to a theory that says we desire the ability to both predict and control the events in our lives? However, as Kelly the therapist was keenly aware, many people cannot predict or control the events surrounding them. Is this always the result of failed construct systems, or is it possible that sometimes we just aren't thinking? Regardless of the answer, Kelly is the recognized leader of a significant

development in the field of personality, a development that contributed to the highly regarded cognitive therapies of Ellis and Beck.

It is also interesting to note that the basis for his theory of constructive alternativism questions the reality of the self in a manner similar to Eastern/Buddhist concepts of consciousness and self. Carl Jung was dramatically influenced by the ancient Vedic traditions of India, and Carl Rogers, the founder of humanistic psychology, was influenced by the spiritual traditions of China. And now we have Kelly, whose theory represents the culmination of behavioral-cognitive theories, sharing a fundamental similarity with Buddhist psychology. Clearly, throughout the history of personality theory, there have been important theorists who looked beyond the constraints of their own training and their own culture.

Constructive Alternativism

Kelly begins by questioning the role that psychologists have assigned to themselves. Psychologists consider themselves to be scientists, engaged in the systematic study of human behavior and thought. Kelly questions why we, as psychologists, don't extend the same perspective to all people. According to Kelly, doesn't every person seek to predict and control the course of events in their lives? Doesn't every person have their own theories about situations in life, don't they test their own hypotheses, and weigh the experimental evidence gained through experience?

…it is as though the psychologist were saying to himself, "I, being a psychologist, and therefore a scientist, am performing this experiment in order to improve the prediction and control of certain human phenomena; but my subject, being merely a human organism, is obviously propelled by inexorable drives welling up within him, or else he is in gluttonous pursuit of sustenance and shelter." (pg. 5; Kelly, 1955a)

Since Kelly proposes that each individual is theorizing about and testing their own life circumstances, he suggests the term **man-the-scientist** for understanding how all people (including, of course, women) approach the world around them.

In trying to understand the world around us, using arguments that sound either existential or like Eastern philosophy, Kelly questions the existence of the universe. Of course it exists, he says, but so do the thoughts of each individual, and the correspondence between what people think exists and what does, in fact, exist is constantly changing. Thus, Kelly suggests that it is better to say that the world around us is existing, rather than to say it exists. Likewise, life can only be understood in the context of time, if it is to make sense. However, life is not simply the changes that occur over time. Rather, it is a relationship between living things and their environment. Kelly emphasizes the creative capacity of living things to represent their environment, as opposed to simply reacting to it. These representations are known as **constructs**, patterns that we create in our mind and attempt to fit over the realities of the world. Since our constructs don't always fit with reality, we are constantly modifying them, as well as trying to increase our repertoire of constructs. Over time, we test our constructs for the ability to predict what will happen in our lives. With sufficient time and experience, and if we are willing to learn from our mistakes, we can evaluate all of our interpretations of the world in which we live (Kelly, 1955a).

Kelly believes that all of our present interpretations of the environment are open to revision or replacement; there are always alternative constructs that may help us deal with new or difficult situations. It is this philosophical position that Kelly refers to as **constructive**

alternativism. It is important to keep in mind, of course, that not just any alternative will work in a given situation. Therefore, each potential alternative construct must be evaluated in terms of its specific predictive efficiency, as well as in terms of overall predictive efficiency of the system it would become part of, if that alternative construct were adopted (Kelly, 1955a).

Discussion Question: According to Kelly, it is more important to know what a person might have done, and he believed that people act as scientists, testing their constructs in order to become better at predicting and controlling their lives. Can you think of situations in which someone did what you expected, but you really wanted to know what they had thought about doing as alternatives?

Basic Theory of Personal Constructs

Personal construct theory begins with a **fundamental postulate**, which is then elaborated with eleven corollaries. The fundamental postulate states that “a person’s processes are psychologically channelized by the ways in which he anticipates events” (Kelly, 1955a). The carefully chosen words in this postulate define the nature of personal construct theory. The words person and he emphasize the individuality of this theory, the unique nature of each person’s constructs. Each person is then recognized as a process. The mind does not stop and start, simply reacting to stimuli, but rather it is constantly in motion, constantly experimenting with constructs. These processes operate through a network of pathways, or channels, according to the devices, or ways, a person constructs in order to achieve their goals. Since these processes, ways, and channels have not been identified as specific physiological mechanisms or anatomical structures, Kelly emphasizes that this is a psychologicaltheory. So when we discuss this mechanism we are not necessarily addressing neuroscience on one hand or sociology on the other hand, we are working within the constraints of the field of psychology. We then have anticipation, the “push and pull of the psychology of personal constructs” (Kelly, 1955a). Being man-the-scientist, each of us seeks to predict the future and choose our actions accordingly. Finally, we have real-life events. Kelly was always very practical about both his personality theory and his approach to psychotherapy. Thus, the psychology of personal constructs is not an ethereal theory. Psychological processes, according to Kelly, are tied down to reality, and anticipation is carried forward in order to better represent future reality. Having established the fundamental postulate, Kelly then described eleven corollaries, or propositions, which both follow from the postulate and amplify his system by elaborating on the fundamental idea (Kelly, 1955a).

Construction Corollary: A person anticipates events by construing their replications. Construing refers to placing an interpretation upon an event. Since a new event will not occur exactly as a past event, our anticipation involves interpreting what the new event will be like. Kelly uses the example of a day. Today is not the same as yesterday, tomorrow will not be the same as today, but each day follows something of a similar pattern. Thus, our anticipation of tomorrow involves constructs based on both the similarities and differences between days we have experienced in the past. It is important to note that this process is not the same as cognition, it is not simply thinking about tomorrow. Much of this process is preverbal, or unconscious, and in that sense occurs automatically.

Individuality Corollary: Persons differ from each other in their construction of events. No matter how closely associated two people are, they cannot play exactly the same role in any

situation. Therefore, they will interpret events differently. Although Kelly acknowledges that people often share similar experiences, particularly as they attend to the experiences of others in the same or similar situations, this corollary emphasizes the unique, subjective nature of interpreting and anticipating events.

Organization Corollary: Each person characteristically evolves, for his convenience in anticipating events, a construction system embracing ordinal relationships between constructs. When faced with conflict, there may be solutions that contradict one another. Thus, the constructs we develop may contradict each other. Kelly suggested that we develop our constructs in a systematic and organized way, with some constructs being ranked more highly than others. For example, some constructs may be good vs. bad, or stupid vs. intelligent. A stupid construct might work in a given solution, but an intelligent one would probably be preferred. For instance, suppose you have an electric garage door opener, and the power is out. You could put your car in the garage by driving through the garage door. However, it might be preferable to get out of the car, go into the garage through the house or side door, and then disconnect the garage door from the opener and open it by hand.

Dichotomy Corollary: A person's construction system is composed of a finite number of dichotomous constructs. Every construct has both positive and negative aspects. In the example used above, both ways of putting your car in the garage have advantages and disadvantage. Driving through the garage door is quick and easy in the short term, but results in needing a new garage door. Getting out of the car and using another entrance takes more time and effort, and may be unpleasant in a bad storm, but it protects your property (and saves time and money in the long run). The essential nature of contrast was eloquently described some 2,600 years ago by Lao Tsu:

Under Heaven all can see beauty as beauty only because there is ugliness.

All can know good as good only because there is evil.

Therefore having and not having arise together.

Difficult and easy complement each other.

Long and short contrast each other;

High and low rest upon each other;

Voice and sound harmonize each other;

Front and back follow one another.

Lao Tsu, c600 B.C.

(pg. 4; Lao Tsu, c600 B.C./1989)

Choice Corollary: A person chooses for himself that alternative in a dichotomized construct through which he anticipates the greater possibility for extension and definition of his system. Simply put, since each situation requires us to choose between the options we construct, Kelly believed we choose the alternative that serves us best (at least within our system of constructs, which may be different than the reality of the best choice). But what about situations in which the best choice is not so obvious? Kelly believed that the choice corollary allowed for shades of gray when a decision is not clearly a choice between black and white alternatives. He did not view this as a contradiction, but rather, he proposed that

the choice becomes one between options that are more gray or less gray. Thus, we can maintain the dichotomy of the choice while still also allowing the choice itself.

Range Corollary: A construct is convenient for the anticipation of a finite range of events only. Every personal construct has a range or focus, and few, if any, are relevant to all events. As Kelly points out, the construct tall vs. short may apply well to descriptions of people, trees, and buildings. But what would we mean by tall weather, or short light? Clearly, the construct tall vs. short is limited to certain types of discrete, physical objects.

Experience Corollary: A person's construction system varies as he successively construes the replications of events. As we apply constructs in our efforts to predict what happens in our lives, we sometimes experience unexpected outcomes. As a result, we reconstruct our constructs, and learn from our experiences. In other words, man-the-scientist is by definition a work in process, and that process is ongoing.

Modulation Corollary: The variation in a person's construction system is limited by the permeability of the constructs within whose range of convenience the variants lie. This corollary addresses the ease with which the experience corollary can occur. Although all individuals modify the constructs that guide their anticipation of events, some constructs are modified more easily, and some people are more open to changing their perspectives (and, hence, reconstructing their constructs). This is one area where Kelly re-emphasizes the difference between psychological processes and the process of science. Scientists seek hypotheses, theories, and laws that are not likely to change. Indeed, there is a continuum from hypothesis to law based on how likely it is that a scientific observation is true. People, however, are constantly testing and retesting their constructs, and reconstructing them as necessary and appropriate. Therefore, people may act like scientists, but their psychological processes serve to facilitate the individual's life, not the lives of others (as scientific theories and laws are meant to apply to the whole universe).

Fragmentation Corollary: A person may successively employ a variety of construction subsystems which are inferentially incompatible with each other. This corollary extends from the previous one, but with a twist. As an individual encounters unexpected events, they modify their constructs to the extent that they are able. Thus, their behavior may change slowly, or more quickly, depending on the nature of the constructs that guide their openness to change. The twist comes into play when individuals are either resisting change, or in the process of change, and it involves the dichotomy corollary. If an individual is failing to predict and control events in their life, they may choose an incompatible construct, essentially reversing the course of their behavior. One of the advantages of Kelly's personal construct theory is that these dramatic changes in behavior can now be seen as reasonable progressions in one's ongoing desire for predictability and control.

Commonality Corollary: To the extent that one person employs a construction of experience which is similar to that employed by another, his psychological processes are similar to those of the other person. This corollary is important for interpersonal relations. Even though two people cannot experience the same event in exactly the same way, their ability to share their experiences is facilitated by the similarity of their experiences. This raises important implications for therapists working with clients of different cultures, since they might not share similar constructs based on certain events. It also raises an important distinction between cognitive and behavioral approaches to understanding personality. In behavioral perspectives, simple stimulus-response relationships are the same for everyone

who experiences them. However, in the cognitive perspective, each person necessarily experiences any event in a unique way.

Sociality Corollary: To the extent that one person construes the construction processes of another, he may play a role in a social process involving the other person. Humans are social creatures. Our ability to predict and control our lives is largely based on our ability to predict and either control or work with other people. Thus, it is important for individuals to construe to thoughts and behaviors of others, and in so doing we can each play a role in the lives of others. Kelly suggests that this provides a natural connection between personal construct theory and social psychology, as well as a connection between personal construct theory and cross-cultural psychology.

Dimensions of Transition

Since life is an ongoing process, there are regular transitions in one's personal constructs. According to the organizational and modulation corollaries, individuals have certain preferences amongst their constructs and differences in their ability to reconstruct them. Problems in life arise when individuals find it difficult to transition from an ineffective construct, one that does not allow for predicting or controlling events, to an effective construct. According to Kelly (1955a), the major problems are seen as the psychological phenomena of **threat**, **fear**, **guilt**, and **anxiety**. Kelly defines these terms as follows:

Threat is the awareness of imminent comprehensive change in one's core structures.

Fear is like threat, except that, in this case, it is a new incidental construct, rather than a comprehensive construct, that seems about to take over.

Perception of one's apparent dislodgment from his core role structure constitutes the experience of guilt.

Anxiety is the recognition that the events with which one is confronted lie outside the range of convenience of one's construct system.

In each case, the psychological phenomenon is based on either the failure of one's constructs to provide courses of action or a direct challenge to the system of constructs available to the individual. Given that the individual's personal construct systems define the larger construct of self, these psychological phenomena represent a challenge to the very self experiencing them. In defense of the individual, aggressiveness is seen as the active elaboration of one's perceptual field. In other words, aggressive individuals try to control events in ways that force decisions favorable to the individual. Similarly, hostility is viewed as the continued effort to extort validational evidence in favor of a type of social prediction which has already proved itself a failure. In this case, the individual tries to find confirmation of success following failed constructs, and this can only be done at the expense of others (Kelly, 1955a).

As people live their everyday lives, there are two typical cycles of transition, the **C-P-C Cycle** and the **Creativity Cycle**. The C-P-C Cycle involves circumspection, preemption, and control. Being circumspect refers to being wary, not taking risks. Thus, as we construe events we try to be precise in the development of our constructs. We then preempt these constructs for membership in an exclusive realm, one that best fits the event we are trying to predict and control. Finally, the first two steps have control as their natural consequence. Still, the individual must make the choice of that course of action, so Kelly suggests that the

final C could just as well stand for choice as it does for control. In contrast, the Creativity Cycle begins with loose constructions, and then leads to tightened and validated constructions. What makes the Creativity Cycle meaningful is the individual's ability to quickly experiment with various constructs and then seize upon the most promising, which is then tightened up and tested. Since much of this process is preverbal, the thought processes of creative individuals may not be apparent to others. According to Kelly, although individuals who begin with tight constructions might be productive, they cannot be creative. Creativity requires beginning with loose constructions (Kelly, 1955a). The value of creativity is not simply to be found as a distinction between the types of cycles experienced by individuals in their daily lives. Creativity is an important component of well-being, and a common topic in books on positive psychology and human strengths and virtues (Aspinwall & Staudinger, 2003; Cloninger, 2004; Compton, 2005; Peterson & Seligman, 2004; Snyder & Lopez, 2005). Indeed, Carl Rogers identified creativity as a significant aspect of the personality of a fully functioning person (Rogers, 1961).

Discussion Question: Many psychologists, including Kelly and Carl Rogers, consider creativity to be an essential aspect of healthy psychological development. Have you ever entered into an unfamiliar situation and tried to be creative in how you handled it? Was it difficult to set aside preconceived notions about how to act, or do you find it easy to try different things in new situations?

The Role of the Psychotherapist

Kelly was first and foremost a clinical psychologist, and his writings are full of practical examples related to clinical work. Theories are of little value unless they are rooted in the values of the psychologists using them. Kelly considered this to be particularly true of clinical psychologists, since they are routinely dealing with clients (Kelly, 1955b). Kelly believed that the role of the psychotherapist involves not only the training and perspectives of the therapist, but also a need to understand the client, and essential ethical considerations:

The role of the psychotherapist involves keen alertness to what the client expects from psychotherapy and the initial acceptance of a wide variety of client misperceptions of what psychotherapy is…it involves certain ethical obligations that transcend mere legal status. (pg. 618; Kelly, 1955b)

In his typically practical style, Kelly provides lengthy lists of what psychotherapy means to the client, the client's conceptualization of the therapist, the clinician's conceptualization of their role, and basic approaches to revising the client's constructs. At the beginning of therapy, it is unlikely that the client has a good concept of what therapy is and what it can accomplish. Nonetheless, the client has some construction of what will take place in therapy. The complaint presented by the client says something about what he or she thinks therapy can accomplish, and some see therapy as an end in itself. However, the reality is that therapy is a means to an end, and that end should be generating movement forward on the part of the client. In contrast, some clients are so ready for change that the therapist must be cautious in interpreting the client's state of mind (Kelly, 1955b). Just as the client makes predictions about therapy, they will also have an initial conceptualization of the therapist. They may construe the therapist as a parent, an absolver of guilt, a companion, or even a threat. Hopefully, according to Kelly, the client may construe the therapist as a representative of reality. In this case, the client may feel free to experiment with his or her

constructs without fear of failure. Most importantly, how the client construes the therapist will have a dramatic effect on their relationship:

From the client's conceptualization of psychotherapy comes the role he expects to play and the role he expects the therapist to play…He may be bitterly disappointed in the therapist's enactment of the expected role. He may stretch his perceptions of the therapist in order to construe him in the manner he expected to construe him rather than in the manner the therapist seeks to be construed.…The client may then feel lost and insecure in the psychotherapeutic relationship. (pg. 575; Kelly, 1955b)

As the client is engaging in these processes, the clinician is also conceptualizing their own role. Overall, the goal of any therapist should be to assist in the continuous reconstruction of the client's construct system, and the changes that take place in therapy should set the stage for continued reconstruction after therapy has been discontinued. Initially, the therapist may rely on a variety of techniques to accomplish superficial reconstructions. The therapist needs to be patient, and initially must accept the client's construct systems as they are. The latter point is quite similar to the empathy described by Carl Rogers in humanistic, client-centered therapy, and Kelly does indeed use the word "empathize" in his own writings. As therapy progresses, the therapist needs to help the client select new conceptual elements, accelerate the tempo of the client's experience, and design and implement experiments. Finally, the therapist serves to validate the client's experiments as they attempt to reconstruct their construct systems (Kelly, 1955b).

Psychological Assessment within Personal Construct Theory

Kelly believed that therapy was a joint effort between the therapist and the client, and since the goal was the ongoing reconstruction of the client's psychological systems (even after therapy), the client ultimately needs to become his own therapist. Therefore, the **psychotherapeutic interview** (Note: by "interview," Kelly means what we would commonly call a therapy session) becomes an essential part of therapy. Throughout the process of the interview, the therapist makes decisions regarding the course of the interchange between the therapist and the client. Overall, the decisions made by the therapist are tailored to the specific client, but still the therapist must remain in control of the interview. This requires that the therapist plan for the interview. Those plans include how often to interview the client, how long the interviews should last, the tempo of the interview, and when to terminate the interview. Since the client continues to live their life outside of the interview room, the therapist must also consider whether special circumstances will require special interview plans (Kelly, 1955b, 1958). One of the most practical aspects of the interview is that the client can simply provide information needed by the therapist, to a point:

…there is a useful adage for clinical psychologists to follow on occasion: if you do not know what is wrong with a person, ask him; he may tell you. (pp. 322; Kelly, 1955a)

The **Role Construct Repertory Test**(Rep Test) was developed by Kelly in order to understand how a client's personal constructs influence their personal-social behavior. The client begins with a **Role Title List**, on which they list the names of important people in their lives (see Table 10-1). The names are then grouped three at a time, and the client is asked to describe in what important way two of the three individuals are alike but different than the third person. A more organized form of the Rep Test, particularly useful for research purposes, involves creating the **Repertory Grid**. Once again the client is asked to

identify significant people in their life. The grid provides three-person pairings that address various relational factors (family, intimate friends, conflicted relationships, authority figures, and values), and as before the client provides a construct that associates two of the people yet distinguishes them from the third. The common factor is listed as the **emergent pole**, the distinguishing factor is listed as the **implicit pole**. The Rep Test does not result in specific outcomes, so its interpretation is also subject to different methods. If the Rep Test is interpreted formally, it will provide results on the number and range of constructs present within the client's construct systems. In the hands of an experienced and skilled examiner, information can be gleaned on the equivalence of constructs, thus providing deeper detail on the effective range of the client's construct system. As more information is obtained from the Rep Test, the better able the therapist will be to guide the therapeutic process (Kelly, 1955a).

Fixed-role therapy is a technique derived from personal construct theory. First, the client prepares a **self-characterization sketch**, a technique in which the client is asked to write a character sketch about themselves as if they were the principal character in a play, but written as if by a friend who knows the client well. Using information from the self-characterization sketch, as well as from interviews and perhaps the Rep Test, the therapist then writes a **fixed-role sketch**. The client is asked to act out the fixed-role sketch over a period of weeks. Initially, Kelly and his colleagues emphasized minor changes in the client's construct systems. However, they later found that it is often easier for a client to play out roles that are the opposite of their usual constructs, rather than making only minor changes in their behavior. Over time, it is expected that the client will learn that the new construct systems are more predictive than their old construct systems, and the fixed-role therapy will establish an ongoing process of reconstruction within the client (Kelly, 1955a).

Table 12.1: The Role Title List Used for the Personal Construct Repertory Test

1. A teacher you liked. (Or the teacher of a subject you liked.)2. A teach you disliked. (Or the teacher of a subject you disliked.)3. Your wife or present girl friend.3a. (for women) Your husband or present boy friend.4. An employer, supervisor, or officer under whom you worked or served and whom you found hard to get along with. (Or someone under whom you worked in a situation you did not like.)5. An employer, supervisor, or officer under whom you worked or served and whom you liked. (Or someone under whom you worked in a situation you liked.)6. Your mother. (Or the person who has played the part of a mother in your life.)7. Your father. (Or the person who has played the part of a father in your life.)8. Your brother nearest your age. (Or the person who has been most like a brother.)9. Your sister nearest your age. (Or the person who has been most like a sister.)10. A person with whom you have worked who was easy to get along with.11. A person with whom you have worked who was hard to understand.12. A neighbor with whom you get along well.13. A neighbor whom you find hard to understand.14. A boy you got along well with when you were in high school. (Or when you were 16.)15. A girl you got along well with when you were in high school. (Or when you were 16.)16. A boy you did not like when you were in high school. (Or when you were 16.)17. A girl you did not like when you were in high school. (Or when you were 16.)18. A person of your own sex whom you would enjoy having as a companion on a trip.19. A person of your own sex whom you would dislike having as a companion on a trip.20. A person with whom you have been closely associated recently who appears to dislike you.21. The person whom you would most like to be of help to. (Or whom you feel most sorry for.)22. The most intelligent person whom you know personally.23. The most successful person whom you know personally.24. The most interesting person whom you know personally.

Discussion Question: Kelly's fixed-role therapy requires the client to write a script for how they want to live their life. He found that sometimes it was easier for his clients to act out the opposite of their typical behavior. Would you find it easier to make minor changes in your behavior, or easier to make dramatic changes?

Connections Across Cultures: Understanding Culture's Effects on

Cognitive Style as a Prerequisite for Effective Cognitive-Behavioral Therapy

An essential element of all cognitive therapies is the desire to identify and challenge a client's underlying dysfunctional cognitions, whether they are mistaken beliefs, schemas, automatic thoughts, cognitive distortions, whatever the case may be. To do so, requires that the therapist knows when cognitions are dysfunctional, and to some extent, what would be a reasonable cognition in the client's personal situation. While it may seem obvious that any psychologically healthy person, particularly a trained therapist, would be able to recognize the difference between functional and dysfunctional thoughts, feelings, and behaviors, this assumes that the therapist and the client come from similar environments. This may very well NOT be the case when the client and the therapist come from dramatically different cultures. Furthermore, as G. Morris Carstairs noted regarding psychiatric interviews (1961), it makes a significant difference whether it is the therapist or the client who is outside of their familiar culture. For example, when a psychologist conducts research in a foreign country, particularly in small towns or villages, local people may simply fear and avoid strangers.

Kelly discussed culture at length in The Psychology of Personal Constructs(Kelly, 1955a,b). Both the commonality corollary and the sociality corollary are directly influenced by our understanding of culture. We, and by "we" I mean to include therapists, tend to expect that people from similar cultures have experienced basically similar upbringings and environments. We also tend to believe that people from a given culture share their expectations regarding the behavior of others from that culture. Thus, in order for a therapist to gain access to the personal constructs of their client, it is important for the therapist to learn as much as possible about the client's cultural heritage. Failure to do so may interfere with the therapist's ability to understand some of the client's disruptive anxieties about either therapy itself or their life in general. Indeed, Kelly shares an example in which a White therapist (whom Kelly was supervising) found it difficult to help a Black client, because the Black client was overly anxious about discussing racially charged feelings regarding interracial sexual relationships. Since the client had discussed sexual issues before, the White therapist did not readily recognize the discomfort with which the Black client addressed his attraction to White women (remember, this was in the 1950s!).

Kelly goes on to discuss cultural differences in mannerisms, language, expectations regarding mental illness, the influence of religion, and how a therapist might go about learning more about a client's cultural experiences. He does caution, however, that one should not attribute too much value to the influence of culture:

…It is important that the clinician be aware of cultural variations. Yet, from our theoretical view, we look upon the "influence" of culture in the same way as we look upon other events. The client is not merely the product of his culture, but it has undoubtedly provided him with much evidence of what is "true" and much of the data which his personal construct system has had to keep in systematic order. (pg. 688; Kelly, 1955b)

For example, it is often considered the mark of a sophisticated clinician that he considers all of his clients in terms of the culture groups to which they belong. Yet, in the final analysis, a client who is to be genuinely understood should never be confined to the stereotype of his culture. (pg. 833; Kelly, 1955b)

Albert Ellis and Aaron Beck, originators of the best known cognitive-behavioral therapies (see Beck & Weishaar, 1995; Ellis, 1995), also discussed cultural influences, though not as extensively as Kelly had. Ellis emphasized that each individual develops a belief system which helps them make judgments and evaluate situations. Although each person's belief system is unique, they share many beliefs with other members of their society and/or culture. Perhaps more importantly, different cultures can have very different belief systems. To complicate the situation even further, cultural beliefs can change, either due to gradual evolution of the culture or in a more dramatic fashion when an influential thinker or leader offers a different perspective on life (Ellis, 1977). Beck has discussed how culturally-determined schemas can be so fundamental that they contribute to how and who we both love and hate (Beck, 1988, 1999).

Today, studies on the relationship between culture and cognition continue, both in clinical and non-clinical settings. There are at least two handbooks focusing on cross-cultural and multicultural factors in personality assessment (Dana, 2000; Suzuki, Ponterotto, & Meller, 2001). According to Suzuki, et al. (2001), these handbooks are necessary due to "the growing number of racial and ethnic minorities in the United States and in recognition of the multitude of variables that affect performance on cognitive and personality tests…" As assessment transitions to therapy, it becomes quite a challenge for any therapist to be familiar with the wide variety of cultures in America. Axelson (1999) has identified six basic cultural groups in America: native Americans, Anglo-Americans, European ethnic Americans, African Americans, Hispanic Americans, and Asian Americans. This list obviously does not include the many immigrants living in this country who are not considered to be American. When faced with such cross-cultural challenges, the essential skills for a therapist include careful and active listening, genuine verbal and nonverbal responses that indicate successful communication, being honest about what you do not understand, respecting and caring about the client, and being patient and optimistic (Axelson, 1999).

Additional studies have suggested that cultural knowledge influences the interpretation of stimuli in a dynamic, constructivist fashion (Hong, Morris, Chiu, & Benet-Martinez, 2000), that these processes occur automatically (Bargh & Williams, 2006), and that experiencing a wider variety of cultures in one's education may actually lead to more complex cognitive processing (Antonio, Chang, Hakuta, Kenny, Levin, & Milem, 2004). When considering fundamental cultural differences, what some consider the **core values** that distinguish amongst cultures, most psychology students are familiar with the distinction between individualistic and collectivistic cultures (cultures in which one favors one's own goals as compared to subordinating one's own goals in favor of group goals). However, Laungani (1999) suggests that there are three other common dimensions: free will vs. determinism, materialism vs. spiritualism, and cognitivism vs. emotionalism. According to Laungani, Western cultures tend to be work- and activity-centered. Thus, they operate in a cognitive mode that emphasizes rational, logical, and controlled thought and behavior. Non-Western cultures, in contrast, tend to be relationship-centered, operating in an emotional mode. Public displays of feelings and emotions, both positive and negative, are not frowned upon (Laungani, 1999). These core values carry over into cognitive styles. For example, the cognitive style prevalent in Africa tends toward synthesis, as opposed to analysis. Africans tend to integrate their experiences into an inclusive whole, and they view such tendencies as more natural than the typical Western alternative (Okeke, Draguns, Sheku, & Allen, 1999). Thus, one can imagine a therapeutic situation in which the client resists analyzing their

problems, and the therapist considers that resistance to be a specific problem unique to the client. Any subsequent attempts by the therapist to break down that resistance would be flawed, since the therapist has not understood the underlying cognitive style of the client. The failure of therapists to properly address the significance of cultural factors in therapy, regardless of whether or not their failure was unintentional, has been described as **cultural malpractice** (Iijima Hall, 1997).

Cognitive-Behavioral Therapy and Acceptance Therapy

Albert Ellis and Aaron Beck are not known as personality theorists, but both are well-known therapists and prolific authors. Their unique approaches to therapy are, of course, based on their theoretical perspectives, each of which emphasizes cognitive processes. Thus, we will take a brief look at how they have applied cognitive aspects of personality theory to the treatment of psychological disorders.

Pretzer and Beck (2005) have suggested that cognitive therapies are truly integrative approaches that treat the individual within a **phenomenological** perspective. Beck was trained as psychoanalyst, and began examining his patients' thought processes carefully in an attempt to prove that Freud was right about depression being the result of anger turned inward. However, Beck instead discovered that his patient's thoughts focused more on themes such as despair and defeat, and that their appraisals of situations in life and their consistently negative biases in processing information were better predictors of their mood and behavior. Thus, Beck began to develop a cognitive approach to working with his patients. At the same time Ellis was developing rational-emotive therapy, and the two theories have influenced each other in many ways (Pretzer & Beck, 2005). In addition, cognitive therapy has been influenced by many other developments in the field of psychology, including the work of Freud, Adler, Horney, Rogers, Bandura, and, of course, Kelly. The integration of these various approaches, in order to truly understand the individual, requires the therapist to work actively with the client:

…The idea of "collaborative empiricism" is central to the practice of cognitive therapy. In the course of therapy, the cognitive therapist works with his or her client to collect detailed information regarding the specific thoughts, feelings, and actions that occur in problem situations. These observations are used as a basis for developing an individualized understanding of the client which provides a basis for strategic intervention…For the cognitive therapist to intervene effectively, he or she must endeavor both to understand the individual's subjective experience and to perceive objective reality accurately. (pp. 46-47; Pretzer & Beck, 2005)

More recent developments in cognitive therapy have focused on accepting the circumstances of one's life, doing so not as an excuse, but in order to facilitate moving forward from that point. **Acceptance and Commitment Therapy** (ACT) acknowledges the presence of suffering in human life, and focuses on using mindfulness to re-orient one's relational framework to the circumstances of one's life (Eifert & Forsyth, 2005; Hayes & Smith, 2005; Hayes, Strosahl, & Wilson, 1999). ACT has many elements in common with the traditional practice of Buddhist mindfulness, an approach that is taken directly in Radical Acceptance (Brach, 2003).

Brief Biographies of Albert Ellis and Aaron Beck

Albert Ellis was born in 1913 in Pittsburgh, Pennsylvania. When he was 4 years old his family moved to New York City, and Ellis has remained there ever since. Although Ellis considered his childhood to have had no significant effect on his subsequent career in psychology, there were some rather dramatic factors that influenced the person he became. His father was a traveling salesman who was seldom home, and when he was home he paid little attention to his children. After his parents were divorced, Ellis seldom saw his father again. His mother wasn't much more attentive, doing very little for the children, and often leaving them home alone. Ellis later wrote: "As for my nice Jewish mother, a hell of a lot of help she was!" (cited in Yankura & Dryden, 1994). Ellis was also very sick following tonsillitis and a strep infection. He needed emergency surgery, and then developed nephritis. Over the next 2 years, from 5 to 7 years old, he was hospitalized eight times, once for 10 months. Yet his parents remained uncaring, and he would sometimes go weeks without anyone from the family visiting him in the hospital. The illness kept him from playing sports or other games even when he was home from the hospital. Perhaps as a result of all of these circumstances, or perhaps because of his temperament, Ellis was painfully shy. He dreaded public activities, such as when he won an award for his excellent academic work, and he avoided making social overtures toward any girl he had a crush on (Yankura & Dryden, 1994).

Surprisingly, Ellis grew strong from these experiences. He thrived on his independence and autonomy, and turned his attention toward his schoolwork. He obtained praise from adults other than his parents, and at one point became something of a leader amongst the children in the hospital. Although his shyness plagued him for many years, he developed a strong sense of self-esteem based on his success in academics. Most importantly, he developed a sense of choosing to overcome his adverse childhood. He did not become a strong-willed individual because of his bad childhood, for example he describes his sister as never really being happy, but in spite of it, due largely to being born with an innate capacity for rational thinking (Yankura & Dryden, 1994).

Much like B. F. Skinner, Ellis hoped to become a writer. Having finished high school at the age of 16, he decided to attend business school at the City College of New York so he might make enough money to support his writing career. However, the Great Depression was just beginning, so there was little opportunity for a young man to make money in business. Nonetheless, he wrote a great deal. He wrote a 500,000-word autobiographical novel. By the age of 28 he had written twenty full-length novels, plays, and books of poetry. None of them were published. He also wrote numerous non-fiction works on sex, philosophy, and politics. None of them were published. However, his research on the topics of sex, love, and marriage made him a popular source of advice amongst his friends. And so he decided to pursue professional training in psychology (Yankura & Dryden, 1994).

Ellis began his studies in the psychology program at Columbia University, and then transferred to the clinical psychology program at Teachers College of Columbia University, receiving a Ph.D. in clinical psychology in 1947. That year he also began training as a psychoanalyst at the Karen Horney Institute for Psychoanalysis. His training analyst was Dr. Charles Hulbeck, who had been analyzed by Hermann Rorschach. One of the personal issues Ellis addressed during his training analysis was whether or not to marry the women he had begun dating. He eventually decided not to marry her (he was later married twice, one marriage ended in annulment, the other in divorce). Ellis was successful as a psychoanalyst, but many patients couldn't afford to come as often as was recommended in traditional

psychoanalytic theory. Curiously, Ellis noticed that patients who came less often seemed to fare better in therapy, especially when the constraints of limited time caused Ellis to be more proactive in therapy. He pursued this active-directive approach to therapy, and by the mid-1950s he had developed rational emotive behavior therapy to the point where he published his first articles and began describing the technique at professional conferences (Yankura & Dryden, 1994).

Ellis devoted the rest of his career to establishing rational emotive behavior therapy as a significant force in psychotherapy. In 1959 he established the Institute for Rational Living, and by the 1980s there were similar institutes in Australia, Britain, Canada, Germany, Israel, Italy, Mexico, and the Netherlands. He has written over 75 books, beginning with How to Live with a Neurotic (Ellis, 1957), hundreds of articles, and he has received many distinguished awards. After 60 years as a psychotherapist, marriage and family counselor, and sex therapist, Ellis has been honored as a fellow of five major associations. He was recognized with the Humanist of the Year Award by the American Humanist Association, the Distinguished Psychologist Award of the Academy of Psychologists in Marital and Family Therapy, the Distinguished Practitioner Award of the American Association of Sex Educators, Counselors and Therapists, and the American Psychological Association has recognized him for Distinguished Professional Contributions to Knowledge. In a 1991 survey ranking the “Most Influential Psychotherapist,” Canadian psychologists ranked Ellis #1, whereas American psychologists ranked him second to Carl Rogers, but ahead of third-place Sigmund Freud (Ellis, 1994, 2005; Yankura & Dryden, 1994)!

In most ways, Aaron Beck’s childhood couldn’t have been more different than that of Ellis. Born in 1921, he was the youngest child of loving and supportive parents. His parents were particularly supportive of education: his brother Irving became a physician, and his brother Maurice entered social work after earning a Master’s degree in psychology (the other two children had died in childhood, his only sister dying in the worldwide influenza epidemic of 1919). Like Ellis, however, Beck was extremely ill as a child. When he was seven years old, Beck broke his arm at a playground. An infection set in, which then developed into septicemia (a generalized blood infection). At the time, septicemia was 90 percent fatal, and his brother Irving overheard the doctor tell their mother that Beck would die. Although he obviously survived, he missed so much school that he had to be held back a year. As often happens when young children are held back in school, the effect on his self-esteem was devastating, and Beck came to believe that he was stupid and inept (Weishaar, 1993; for more information on the negative effects of grade retention visit the National Association of School Psychologists’ website at www.nasponline.org).

However, with the help of his brothers, Beck was able to catch up to and eventually surpass his classmates, graduating first in his high school class. Along the way he belonged to the Audubon Society, worked as a camp counselor, became the youngest Eagle Scout in his Boy Scout troop, and was editor of the high school newspaper. He followed his brothers to Brown University, but was unsure of a career path. He majored in English and Political Science, but he took a wide variety of courses, eventually taking the courses necessary to go on to medical school. He graduated magna cum laude, Phi Beta Kappa, and won awards for oratory and essay writing. Despite his dramatic successes, Beck suffered a great deal of anxiety, particularly a blood/injury phobia that most likely resulted from his frightening experiences related to the emergency surgery necessary when he broke his arm. Having been accepted to the Yale School of Medicine, his surgery rotation was very difficult in light

of his fear of blood. However, he worked through his fears cognitively (an obvious foreshadowing of the work that would make him famous), and successfully completed his medical degree (Weishaar, 1993).

Beck never intended to study psychiatry, and thought little of psychoanalysis. However, having graduated in 1946, there were many veterans returning from World War II. In 1949, he began a residency in neurology at a veteran's administration hospital in Massachusetts. Due to a pressing need for psychiatrists, the director of the program began requiring everyone to complete a rotation is psychiatry. The psychiatry program at the hospital was primarily influenced by the Boston Psychoanalytic Institute, and Beck protested that psychoanalytic formulations seemed far-fetched, but he eventually decided to stay in psychiatry and to study psychoanalysis in greater detail. He first studied psychoanalysis at the Austin Riggs Center in Massachusetts, where one of his supervisors was Erik Erikson. After completing his training in psychiatry, he joined the faculty of the University of Pennsylvania Medical School (in 1954), where he has remained ever since. As he began his research career, Beck intended to confirm Freud's hypothesis that depression was the result of hostility turned inward. However, he began to recognize that his patients were greatly influenced by underlying patterns of cognition, the so-called automatic thoughts that are so well-known today. About this same time, Beck learned of Kelly's work on personal constructs (which Beck later referred to as schemas). As a result of these ideas and experiences coming together, Beck's own cognitive theory began to take shape. Then, in 1963, Ellis read an article written by Beck. Ellis sent copies of his own work to Beck, and reprinted Beck's article in the journal Rational Living. Beck then invited Ellis to speak to the psychiatry residents at Penn, and from that point forward the two maintained close contact. Beck has credited Ellis as being an excellent spokesperson for cognitive approaches to psychotherapy (Weishaar, 1993).

During his career, Beck has received many awards, including the Distinguished Scientific Award for the Applications of Psychology from the American Psychological Association. He has also received major awards from the American Psychiatric Association, the American Psychopathological Association, and the American Association of Suicidology. He received an honorary doctorate in medical science from his alma mater, Brown University, in 1987 he was elected a Fellow of the Royal College of Psychiatrists (England), and he is a senior member of the Institute of Medicine. Perhaps the most meaningful tribute, however, is that his daughter, Dr. Judith Beck, has followed in his footsteps. She is currently the Director of the Beck Institute for Cognitive Therapy and Research, and a Clinical Associate Professor at the University of Pennsylvania. She has written a number of books on cognitive therapy and, with her father, developed the Beck Youth Inventories (for more information visit the website for the Beck Institute for Cognitive Therapy and Research at www.beckinstitute.org).

Placing Ellis and Beck in Context: Cognitive Therapy

In one sense, Ellis and Beck do not belong in a book about personality theory. They are not known for their theoretical contributions to our understanding of personality development. In another sense, they are among the most important theorists covered, since the practical application of cognitive theories to psychotherapy has had a dramatic influence on the effectiveness of psychotherapy in treating psychological disorders. Remember that Freud, as well as most of the other well-known psychodynamic theorists, began conducting therapy

first and later developed theoretical perspectives which helped to explain what they saw in their patients and what worked in therapy. Similarly, Ellis and Beck focused on the development of their therapeutic approaches, and to a large extent their theoretical perspective is inferred from the techniques they use in therapy.

In addition, one could argue that the cognitive therapies of Ellis and Beck stand at the pinnacle of the behavioral and cognitive theories of personality that have been so influential in American psychology. As evidence of their significance, Beck received an Albert Lasker Clinical Medical Research Award in 2006. Often called the American Nobel Prize (some seventy recipients have gone on to win a Nobel Prize), in the 60 years that the Lasker prizes have been awarded, Beck is the first psychotherapist to be honored. This award is a testament both to the respect that cognitive psychotherapy has earned in the medical community and to Beck for the honor he has earned amongst many ground-breaking psychotherapists.

Although cognitive therapies may seem highly specialized, both Ellis and Beck drew upon many different areas of psychology and psychiatry, as well as Eastern philosophies, while developing their techniques. Their psychoanalytic training exposed them to the directive approach of Adler and to Horney's emphasis on the (which can be viewed as a neurotic belief or a type of automatic thought). In addition, they cited theorists, authors, and spiritual leaders such as Bandura, Frankl, Rogers, Piaget, the Dalai Lama, D. T. Suzuki, Lao Tsu, Jesus of Nazareth, and many others. The range of ideas that Ellis and Beck synthesized into a cohesive and direct approach to psychotherapy is unparalleled in the fields of psychology and psychiatry.

Rational Emotive Behavior Therapy

In order to understand Ellis' perspective on therapy, one must first understand his perspective on the basis of psychological disturbances, whether they are minor problems with personal adjustment or more serious forms of mental illness. Individuals with problems typically have a long history related to the disorder. Using the same example as Ellis (1957), suppose a woman is chronically depressed because she has been rejected by men she really liked and with whom she wanted to have long-term relationships. She understandably concludes that the activating event of being rejected leads to the consequence of being depressed. However, this conclusion is wrong! According to Ellis, it is not the rejection that causes her depression, it is the belief system that arises within her, particularly irrational beliefs, that cause her depression. For example, if she rationally believed that it was unfortunate that someone she liked rejected her, or that it was frustrating that she was rejected, she might not become depressed. However, if she irrationally believes that it is awfulthat she has been rejected, or that she should have been more beautiful so that he wouldn't have rejected her, then she is quite likely to become depressed. This is what Ellis referred to as the A-B-C's of emotional disturbance or self-defeating behaviors and attitudes. A refers to the **activating event** (being rejected), B refers to the person's beliefs***(this is awful, I'm not pretty), and C is the consequence of the beliefs: depression. The basis for therapy in such situations can be found by extending the A-B-C's to the D-E's: disputing (D) the irrational beliefs, which hopefully leads to the cognitive effect (E - effective new philosophies, emotions, and behaviors) of disrupting the self-defeating patterns of behavior (Ellis, 1957, 1973, 1996). More recently, Ellis has proposed one more letter for his** ABC theory of personality, the letter G for goals

(note: there is no “F”). Goals consist of a person’s purposes, values, standards, and hopes. When these goals are thwarted by an activating event, the person can respond by choosing healthy or unhealthy alternatives, and the nature of that choice is based on one’s beliefs (Ellis, 1994). Ellis considered the ABC theory to be so straightforward that it could prove helpful to anyone.

When an individual has become trapped by unhealthy belief systems and the corresponding self-defeating behavior patterns that accompany them, the potential need for psychotherapy arises. The therapy that Ellis developed has come to be known as **rational emotive behavior therapy**, or **REBT**(the name went through several permutations over Ellis’ career, and he finally settled on REBT). The primary task of the therapist using REBT is to challenge the client’s irrational beliefs and, in so doing, to help the client change their belief systems. In essence, when the client believes that it would be catastrophic for a certain negative outcome to occur, the therapist tries to help them by disputing the irrational belief with questions such as: “Why would a certain outcome be catastrophic?” While trying to dispute the irrational beliefs, the therapist also searches for underlying philosophies that support the irrational belief system, philosophies that can then also be disputed. In addition to the cognitive aspect of REBT, the therapist often encourages the client to act against their irrational fears. If the client is willing, they have the opportunity to experience anxiety-ridden situations without the catastrophic consequences they have feared (although the help of the therapist may prove necessary along the way). Throughout this process, REBT does not intend to ignore the person’s feelings. However, when a client is suffering from unhealthy and self-defeating feelings, such as anxiety, depression, or anger, REBT can help to minimize those unhealthy feelings. In addition, REBT encourages healthy, positive emotions, and recognizes that sometimes a strong negative response, such as sadness or grief, to a tragic activating event may be healthy or constructive. When utilized effectively, REBT offered what Ellis believed was a better, deeper, and more enduring therapy, which could achieve those results in a fairly brief amount of time (Ellis, 1962, 1973, 1994, 1995, 1996). Ellis also believed that REBT was applicable to a wider range of clients than any other psychotherapy:

RET [later known as REBT], on the contrary, seems to be almost the only major kind of psychotherapy (aside, perhaps, from Zen Buddhism, if this is conceptualized as psychotherapy…) that holds that the individual does not need any trait, characteristic, achievement, purpose, or social approval in order to accept himself. In fact, he does not have to rate himself, esteem himself, or have any self-measurement or self-concept whatever. (pg. 65; Ellis, 1973)

First and foremost, Ellis focused on practical applications of psychotherapy, and he considered his approach to be humanistic in its emphasis on the whole person. He acknowledged that REBT shared important elements with the approaches of other classic theorists who had emphasized the value of individuals, including Alfred Adler, Viktor Frankl, Rollo May, Carl Rogers, Abraham Maslow, and Karen Horney (Ellis, 1973, 1995). Ellis’ discussion of the practice of REBT seems to focus on what Horney addressed in her concepts of neurotic needs and the tyranny of the should. Taken together, the desire to focus on practical applications and helping individuals has led to a wide variety of self-help books based on REBT (Yankura & Dryden, 1994). Naturally, this list includes many books by Ellis himself, including titles such as How to Live With a Neurotic (Ellis, 1957), A New Guide to Rational Living (Ellis & Harper, 1975), How to Live With - and Without - Anger (Ellis, 1977),

How to Cope With a Fatal Illness (Ellis & Abrams, 1994), How to Keep People from Pushing Your Buttons (Ellis & Lange, 1994), How to Control Your Anxiety Before It Controls You (Ellis, 1998), and Sex Without Guilt in the 21**st**Century (Ellis, 2003). REBT has also been applied to a wide variety of other problems that have been covered in Ellis' books, such as marriage counseling, personality disorders, depression, and even schizophrenia, and REBT has proved successful in both individual and group settings (Ellis, 1962, 1973, 2001; Ellis & Dryden, 1987).

Discussion Question: Rational emotive behavior therapy is based on the ABC theory of personality. Can you think of situations in which activating events led you to specific consequences, even though your beliefs, if you thought about them enough, were the real reason for the consequence you experienced?

Ellis, as well as other therapists using REBT, has also addressed addictive disorders, including alcoholism (Ellis, 2001; Ellis, McInerney, DiGuiseppe, & Yeager, 1988; Trimpey, Velten, & Dain, 1993; Yeager, Yeager, & Shillingford, 1993). In the early 1990s, **Rational Recovery (RR)**was developed by Jack Trimpey as an alternative to the Alcoholics Anonymous (AA) approach to treating alcohol abuse. Both Ellis and Trimpey challenge the basic principles of AA: that the addict has no control over their alcohol cravings, that they must turn over control to a higher power (such as God), that they can never drink alcohol again, and that only AA works for alcoholics. Objective research simply does not support these assertions, and practitioners of REBT and RR have great faith in the ability of individuals to take control of their own lives (though they may need some help from a therapist to get on the right path). RR also does not include the strong religious overtones of AA, which may be an impediment to recovery for anyone required to attend AA meetings but who does not believe in God:

…The core of these methods is learning to recognize and dispute self-defeating thinking that RR frequently labels "the Beast" or "the addictive voice." In addition to its treatment purposes, RR has a political purpose in advocating for people who are mandated to attend spiritual healing groups but who find that approach useless or offensive. Furthermore, RR attempts to educate professionals about available options and sensitize them to the ethical and possible legal issues involved in overriding client's objections to spiritual healing approaches. (pg. 271; Trimpey, Velten, & Dain, 1993)

Since REBT focuses on the choices that individuals make for themselves, what role is there for religion within such a perspective? Ellis maintains that there is a significant positive correlation between devout religious beliefs and a variety of emotional disturbances. However, he has argued that the problem is not religion per se, but rather highly restrictive, dogmatic religiosity that causes problems for an individual (Ellis, 2004; also see Yankura & Dryden, 1994). Ellis studied the works of Lao Tsu (author of the Tao Te Ching), Gotama Buddha (known by most people simply as the Buddha), and the existential theologian Paul Tillich, finding helpful perspectives for working with his clients and on his own problems (Ellis, 2004). He overcame his early objections to religion and spirituality by recognizing that when one's religious beliefs contribute to good psychological health the belief itself, regardless of whether or not God exists, is helpful. Similarly, Zen Buddhism is not actually a religion, but has significant religious overtones. Nonetheless, Ellis was impressed by the effectiveness of Zen meditation for many clients, so he incorporated various Eastern perspectives into the development of REBT.

Discussion Question: Alcoholics Anonymous has helped many people, but it doesn't help everyone, especially those who are opposed to religiosity or are simply atheists. Nonetheless, the AA approach has become a standard recommendation in many legal jurisdictions when someone commits a crime such as drunk driving. What do you think about Rational Recovery (based on REBT) as an alternative to AA?

Is Self-Esteem a Sickness?

The heading for this section is the title of the first chapter in a fascinating book by Ellis: The Myth of Self-Esteem (Ellis, 2005). Ellis believed that self-esteem is defined by psychologists in a way that requires individuals to rate, or judge, themselves. This may work fine when everything is going well for the person, but people are not perfect. Thus, they will eventually fail at something, perhaps at many things, and they must then judge themselves as bad, or unworthy. If, however, you accept yourself as imperfect, and rate only your behaviors and thoughts, not your self, then you need not suffer from the effects of low self-esteem. In other words, Ellis advocates unconditional self-acceptance, a very old philosophy known to the Greeks, Romans, Buddhists, Daoists, and others (Ellis, 2005). Ellis considers many different approaches to the concepts of self-esteem and self-acceptance, including the perspectives of Solomon, Jesus of Nazareth, the existential philosophers Kierkegaard, Heidegger, and Sartre, Carl Rogers, the Eastern philosophers Lao Tsu, D. T. Suzuki, and the Dalai Lama, and Steven Hayes (founder of Acceptance and Commitment Therapy, see below). Ellis concludes that one must seek not only to unconditionally accept oneself, but also to unconditionally accept others and the nature of life itself. But this is not a pie-in-the-sky philosophy. Ellis recognizes that there are sad and unfortunate events in life (we are fallible, and we are mortal). He strongly recommends that we do not avoid normal sadness and regret, since these emotions provide the motivation for trying to prevent unfortunate events and for seeking new relationships. The choice is yours:

Both self-esteem and self-acceptance, then, can be had definitionally - for the asking, for the choosing. Take one or the other. Choose! Better yet, take no global rating. Choose your goals and values and rate how you experience them - well or badly. Don't rate yourself, being, entity, personality at all. Your totality is too complex and too changing to measure. Repeatedly acknowledge that.

Now stop farting around and get on with your life! (pg. 16; Ellis, 2005)

Discussion Question: Ellis opposes the concept of self-esteem because he believes it requires a person to judge themselves. He advocates instead that we unconditionally accept ourselves, faults included. Does this make sense to you?

Beck's Cognitive Model of Depression

Having begun his research in an attempt to examine Freud's theory on the cause of depression, Beck continued studying depression and suicide throughout his career. The reason for this continued focus was the prevalence of depression in society:

Depression is the most common psychiatric disorder treated in office practice and in outpatient clinics. Some authorities have estimated that at least 12 per cent of the adult population will have an episode of depression of sufficient clinical severity to warrant treatment. (pg. vii; Beck, 1967)

Relying on an interplay between clinical work and research, Beck proposed a cognitive model based on **automatic thoughts**, **schemas**, and **cognitive distortions** (Beck, 1967; Beck & Freeman, 1990; Beck, Rush, Shaw, & Emery, 1979; Beck & Weishaar, 1995; Pretzer & Beck, 2005). Automatic thoughts are an individual's immediate, spontaneous appraisals of a given situation. They shape and elicit a person's emotional and behavioral responses to that situation. Since they are automatic, they are rarely questioned. Even when they are predominantly negative, the individual accepts them as true and can be overwhelmed by constant questions and images that hurt one's self-esteem (questions such as "Why am I such a failure," or seeing oneself as ugly). The reason that even highly negative automatic thoughts are accepted, even when they might be objectively untrue, is that these thoughts do not arise spontaneously. Rather, they are the result of the person's schemas. Every situation is comprised of many stimuli, and when confronted with an unfamiliar situation a person tends to conceptualize it. Although different people will conceptualize the situation differently, each individual will be consistent. These stable cognitive patterns of interpreting situations are known as schemas. An individual's schemas then determine how they are likely to respond, automatically, to many situations.

People are also prone to a variety of cognitive distortions, which can amplify the effects of one's schemas, thus helping to confirm maladaptive schemas even when contradictory evidence is available. Over time, Beck and his colleagues have identified a growing number of such distortions, such as: **dichotomous thinking**, or seeing things as only black or white, without the possibility of shades of gray; **personalization**, the tendency to interpret external events as being related directly to oneself; **overgeneralization**, the application of isolated incidents to either all or at least many other situations; and **catastrophizing**, treating actual or anticipated negative events as intolerable catastrophes, even though they may be relatively minor problems. Overall, these cognitive distortions lead the individual into extreme, judgmental, global interpretations of the situations they experience, which establish general schemas, which lead to automatic thoughts and feelings that support the idiosyncratic experience of the world (Beck, Rush, Shaw, & Emery, 1979; Beck & Weishaar, 1995; Pretzer & Beck, 2005). The goal of cognitive therapy, therefore, is to help the individual break out of this self-supporting, maladaptive pattern of cognition.

Another important aspect of the depressive syndrome is known as the **cognitive triad**, three cognitive patterns that cause the person to view themselves in a negative manner. First, the individual has a negative view of themselves. Primarily, the depressed individual sees themselves as defective in some psychological, moral, or physical way, and because of the presumed defects they are undesirable and worthless. Second, the depressed person has a tendency to interpret their ongoing experiences in negative ways. These negative misinterpretations persist even in the face of incompatible evidence. And finally, they tend to hold a negative view of the future. They anticipate continued difficulty, failure, emotional suffering. As a result, they lack motivation, they become paralyzed by pessimism and hopelessness. According to Beck, suicide can be viewed as an extreme attempt to escape problems that depressed individuals believe cannot be solved and the unbearable suffering that the future holds! These negative cognitive patterns are not something that the depressed person plans or has much control over, since they typically occur in the form of automatic thoughts (Beck, 1967; Beck, Rush, Shaw, & Emery, 1979; Beck & Weishaar, 1995; also see Beck, Resnick, & Lettieri, 1974).

Discussion Question: Beck described a number of common cognitive distortions, including dichotomous thinking, personalization, overgeneralization, and catastrophizing. Think about situations in your own life when you made these distortions. What sort of problems resulted from these cognitive errors, and how often do you make them?

Beck's Cognitive Therapy

Cognitive therapy, according to Beck, "is an active, directive, time-limited, structured approach used to treat a variety of psychiatric disorders" (Beck, Rush, Shaw, & Emery, 1979). With regard to depression, it is most effective after a major depression has lifted somewhat, though it can also be helpful for some patients during depression, particularly if the depression is of the reactive type (as opposed to endogenous depression; Beck, 1967). As mentioned above, the basic procedure is to help the individual break out of the trap of negative schemas, automatic thoughts, and cognitive distortions that support the client's problem. The techniques employed are designed to identify, test the reality of, and correct the cognitive distortions and schemas that lead to dysfunctional automatic thoughts. It involves an active collaboration between the therapist and the client, such that the client learns to reduce their symptoms by thinking and acting more realistically.

Beck referred to the constant interaction between the client and the therapist as **collaborative empiricism**, and contrasted this approach to both psychoanalysis and client-centered therapy. His intention was to provide the client with a series of specific learning experiences that would teach the patient the following skills: (1) monitoring their own negative, automatic thoughts; (2) recognizing the connections between thought, emotion, and behavior; (3) examining evidence for and against their cognitive distortions; (4) substituting reality-based interpretations for their cognitive distortions; and (5) learning to identify and alter the dysfunctional schemas that lead to the cognitive distortions (Beck, Rush, Shaw, & Emery, 1979). The interaction with the client is not superficial, as it involves discussing the very rationale of the therapy to the patient and, ultimately, providing the client with techniques to monitor their dysfunctional thoughts on their own. The therapist teaches the client to recognize the nature of cognition, particularly the client's dysfunctional cognitions, all with the goal of eventually neutralizing the automatic thoughts. Somewhat related to collaborative empiricism is the concept of **guided discovery**. Guided discovery is the process by which the therapist serves as a guide for the client, in order to help them recognize their problematic cognitions and behaviors and also help them design new experiences (behavioral experiments) in which they might acquire new skills and perspectives (Beck & Weishaar, 1995). In addition, the therapeutic relationship provides an opportunity for the client to begin to make progress:

If the patient begins to feel better after the expression of feeling, this may then set up a favorable cycle. Since the depressed patient may have lost hope that he would ever be able to feel better again, this positive experience helps to restore his morale and also his motivation to cooperate in the therapy. Any evidence of feeling better is likely to increase the patient's motivation for therapy and thus contribute to its efficacy. (pp. 43-44; Beck, Rush, Shaw, & Emery, 1979)

Although Beck focused much of his research on depression, cognitive therapy can be used to treat a wide variety of psychiatric and psychological disorders, including anxiety disorders, phobias, substance abuse disorders, anger and violence, and personality disorders (Beck, 1999; Beck & Emery, 1985; Beck & Freeman, 1990; Beck & Weishaar, 1995; Beck, Wright,

Newman, & Liese, 1993; Pretzer & Beck, 2005). In Love is Never Enough (Beck, 1988), Beck extended cognitive therapy to working with couples. He had observed that many of his depressed client's were in troubled relationships, and in other cases his client's depression and/or anxiety had led to relationship problems. As Beck began working with couples, he found that couples were capable of the same cognitive distortions that individuals make, as each party within the relationship began focusing on the negative aspects of the relationship. As conflict grows, the partners blame each other, rather than seeing the conflict as a problem that can be resolved. Just as with individuals, cognitive therapy offers a means for breaking the cycle of conflict and miscommunication.

Discussion Question: Collaborative empiricism and guided discovery both suggest that the client must be an active member of the therapeutic team. In your opinion, is it possible for someone who needs therapy to help in their own recovery? Do you think there is a point at which the therapist must take over in order to ensure that therapy is successful?

Acceptance and Commitment Therapy (ACT) and Radical Acceptance

A recent development in cognitive therapy shares an ancient tradition with Eastern philosophies: mindfulness. Mindfulness training involves learning to accept one's emotional realities, and one of the most significant realities is that much of human life involves suffering. Starting with these basic observations, Steven Hayes and his colleagues have developed **Acceptance and Commitment Therapy** (ACT; Hayes & Smith, 2005; Hayes, Strosahl, & Wilson, 1999; see also Eifert & Forsyth, 2005; Hayes, Follette, & Linehan, 2004). Hayes and his colleagues do not mean acceptance in the sense of resigning oneself to suffering, but rather in the sense of accepting life as it comes. Then, one must commit oneself to moving forward and living a values-based life, regardless of the presence of challenges:

The constant possibility of psychological pain is a challenging burden that we all need to face…This doesn't mean that you must resign yourself to trudging through your life suffering. Pain and suffering are very different. We believe that there is a way to change your relationship to pain and to then live a good life, perhaps a great life, even though you are a human being whose memory and verbal skills keep the possibility of pain just an instant away. (pg. 12; Hayes & Smith, 2005)

Although Hayes and his colleagues make passing reference to mindfulness as a Buddhist teaching, they do not take a spiritual approach with ACT. They do, however, acknowledge that psychologists often make the mistake of ignoring spiritual practices that might prove helpful to their clients (Hayes, Strosahl, & Wilson, 1999). Many people in psychology today, including Ellis and Beck, recognize that cognitive psychology began with the Buddha some 2,500 years ago (Ellis, 2005; Pretzer & Beck, 2005; see also Olendzki, 2005). Dr. Tara Brach, a clinical psychologist and teacher of mindfulness meditation (also known as **vipassana**, which means "to see clearly"), makes no qualms about following a Buddhist approach to therapy. In Radical Acceptance: Embracing Your Life with the Heart of a Buddha (Brach, 2003), Brach talks about living in a trance of unworthiness. Plagued by beliefs of their own inadequacies, some individuals limit their ability to live a full life. Consequently, they cannot trust that they are lovable, and they live with an undercurrent of depression or helplessness. They then embark on a series of strategies designed to protect themselves: they attempt self-improvement projects, they hold back and play it safe, they withdraw from the present moment, they keep busy, they criticize themselves, and they focus on other's faults. **Radical**

Acceptance as a therapeutic approach involves both mindfulness meditation and Buddhist teachings on compassion as a basis for teaching people to accept themselves as they are:

Radical Acceptance reverses our habit of living at war with experiences that are unfamiliar, frightening or intense. It is the necessary antidote to years of neglecting ourselves, years of judging and treating ourselves harshly, years of rejecting this moment's experience. Radical Acceptance is the willingness to experience ourselves and our life as it is. A moment of Radical Acceptance is a moment of genuine freedom. (pg. 4; Brach, 2003)

Mindfulness as a therapeutic technique has also been used by a variety of other therapists: in couples therapy (Christensen, Sevier, Simpson, & Gattis, 2004; Fruzzetti & Iverson, 2004), following traumatic experiences (Follette, Palm, & Rasmussen Hall, 2004), and for the treatment of eating disorders (Wilson, 2004) and substance abuse (Marlatt, et al., 2004). Janet Surrey, one of the founding members of the Stone Center Group, has favorably compared relational psychotherapy to mindfulness (Surrey, 2005), and Trudy Goodman, who studied child development with Jean Piaget, uses mindfulness in therapy with children (Goodman, 2005). Thus, whether in the more structured approach of ACT or Radical Acceptance, or in more informal ways in the hands of therapists familiar with mindfulness meditation, paying attention to the mind in a calm and careful way is becoming an important trend in psychotherapy. According to Steven Hayes (2004), this approach represents a third wave in behavioral-cognitive therapy, following traditional behavior therapy and then the cognitive therapies of Ellis and Beck. This third wave is also the basis for the popular and influential work of Jon Kabat-Zinn (1990, 1994, 2005; see also Germer et al., 2005) and connects behavioral and cognitive theories to the rapidly growing field of **social neuroscience** (the study of the interactive influences between the structure/function of the brain and social behavior; see, e.g., Begley, 2007; Cacioppo et al., 2006; Cozolino, 2002; Harmon-Jones & Winkielman, 2007; Siegel, 1999, 2007).

Personality Theory in Real Life: Beck's Cognitive Therapy

and the Treatment of Personality Disorders

Aaron Beck and a number of his colleagues, as well as others, have attempted to apply cognitive therapy to the treatment of personality disorders. It is widely accepted that personality disorders are highly resistant to treatment, but aside from the problems they present by themselves, there is another important reason to continue trying to address these serious psychological disorders. Personality disorders often co-occur with other psychological conditions (Axis I disorders), and they may be the primary reason why psychotherapy does not work well with certain patients (Pretzer & Beck, 2005). Since cognitive therapy in particular requires that the therapist gain an understanding of what the client is thinking, in order to then help the client recognize their own dysfunctional cognitions so that the client may work toward change, it is necessary for the therapist to have a complete understanding of the client's psychological make-up. As a prelude to treating personality disorders with cognitive therapy, one needs to understand personality disorders in cognitive-behavioral terms.

As with depression, or any other psychological disorder, the cognitive-behavioral perspective suggests that individuals suffering from personality disorders have formed dysfunctional schemas that create an attributional bias, which then causes the person to interpret life's experiences in dysfunctional ways, but in ways that nonetheless support and

maintain the dysfunction of the personality disorder. If this theory is accurate, one should be able to identify typical patterns of dysfunctional schemas that match the characterization of different personality disorder diagnoses. Indeed, Beck and Freeman (1990) have offered those patterns in Cognitive Therapy of Personality Disorders. What distinguishes the negative schemas that characterize personality disorders from schemas that characterize other psychological disorders reflects the basic difference between Axis II and Axis I in the DSM system:

The typical schemas of the personality disorders resemble those that are activated in the symptom syndromes, but they are operative on a more continuous basis in information processing. In dependent personality disorder, the schema “I need help” will be activated whenever a problematic situation arises, whereas in depressed persons it will be prominent only during the depression. In personality disorders, the schemas are part of normal, everyday processing of information. (pg. 32; Beck & Freeman, 1990)

What might the typical schemas associated with other personality disorders be? Beck and his colleagues offer detailed examples for all ten of the personality disorders listed in the DSM, as well as for the passive-aggressive (negativistic) personality disorder (Beck & Freeman, 1990; Pretzer & Beck, 2005). To cite just a few examples, the antisocial individual thinks that “people are there to be taken,” the narcissistic individual thinks “I am special,” and the histrionic individual believes “I need to impress.” As a result of these basic beliefs and attitudes, these individuals adopt corresponding behavioral strategies. The dependent person seeks attachment, the antisocial person attacks, the narcissist engages in self-aggrandizement, and the histrionic person performs dramatically (Beck & Freeman, 1990).

How does a personality disorder arise, according to the cognitive-behavioral perspective? First, there are inherited predispositions that may represent primeval strategies. For example, Beck has suggested that the antisocial personality reflects a predatory strategy, whereas in contrast, the paranoid personality reflects a defensive strategy (see Pretzer & Beck, 2005). Second, the characteristics of personality disorders can result from social learning, especially when the social environment enhances genetic predispositions. A child born with a shy disposition, in a household that seems threatening and/or confusing, may naturally withdraw. That withdrawal, taken to its extreme, is a strategy compatible with the avoidant personality disorder. And finally, there is the possibility of traumatic experiences during development. Personality becomes well established during childhood. If one’s experiences during this important time are dysfunctional and traumatic, the individual is likely to develop a personality that has ingrained dysfunctional schemas, thus affecting the individual’s life from that point forward. In this model, personality disorders are not necessarily any different in form than other psychological conditions, but since they directly involve one’s relationship with others, they become significant, problematic features of one’s daily life:

…The cognitive view of “personality disorder” is that this is simply the term used to refer to individuals with pervasive, self-perpetuating cognitive-interpersonal cycles which are dysfunctional enough to come to the attention of mental health professionals. (pg. 61; Pretzer & Beck, 2005)

The basic approach to treating personality disorders with cognitive therapy is not different than usual, but does require some special attention to detail:

Personality disorders are among the most difficult and least understood problems faced by therapists regardless of the therapist's orientation. The treatment of clients with these disorders can be just as complex and frustrating for cognitive therapists as it is for other therapists…For cognitive therapy to live up to its promise as an approach to understanding and treating personality disorders, it is necessary to tailor the approach to the characteristics of individuals with personality disorders rather than simply using "standard" cognitive therapy without modification. (pp. 44-45; Pretzer & Beck, 2005)

Based on this concern, Pretzer and Beck (2005) have offered a list of twelve key elements that require attention when using cognitive therapy to treat an individual with a personality disorder:

1. Interventions are most effective when based on an individualized conceptualization of the client's problems.

1. It is important for therapist and client to work collaboratively toward clearly identified, shared goals.

1. It is important to focus more than the usual amount of attention on the therapist-client relationship.

1. Consider beginning with interventions which do not require extensive self-disclosure.

1. Interventions that increase the client's sense of self-efficacy often reduce the intensity of the client's symptomatology and facilitate other interventions.

1. Do not rely primarily on verbal interventions.

1. Try to identify and address the client's fears before implementing changes.

1. Help the client deal adaptively with aversive emotions.

1. Anticipate problems with compliance.

1. Do not presume that the client exists in a reasonable environment.

1. Attend to your own emotional reactions during the course of therapy.

1. Be realistic regarding the length of therapy, goals for therapy, and standards for therapist self-evaluation.

Despite these straight-forward steps toward effective cognitive therapy, it seems clear from looking at them that there are going to be challenges when dealing with clients who have a personality disorder. Indeed, the very process of collaborative empiricism can be quite difficult with these clients. Beck & Freeman (1990) have identified nineteen problems associated with establishing an effective collaboration with clients who have a personality disorder:

1. The patient may lack the skill to be collaborative.

1. The therapist may lack the skill to develop collaboration.

1. Environmental stressors may preclude changing or reinforce dysfunctional behavior.

1. Patients' ideas and beliefs regarding their potential failure in therapy may contribute to noncollaboration.

1. Patients' ideas and beliefs regarding effects of the patients' changing on others may preclude compliance.

1. Patients' fears regarding changing and the "new" self may contribute to noncompliance.

1. The patient's and therapist's dysfunctional beliefs may be harmoniously blended.

1. Poor socialization to the model may be a factor in noncompliance.

1. A patient may experience secondary gain from maintaining the dysfunctional pattern.

1. Poor timing of interventions may be a factor in noncompliance.

1. Patients may lack motivation.

1. Patients' rigidity may foil compliance.

1. The patient may have poor impulse control.

1. The goals of therapy may be unrealistic.

1. The goals of therapy may be unstated.

1. The goals of therapy may be vague and amorphous.

1. There may have been no agreement between therapist and patient relative to the treatment goals.

1. The patient or therapist may be frustrated because of a lack of progress in therapy.

1. Issues involving the patient's perception of lowered status and self-esteem may be factors in noncompliance.

Although Beck and his colleagues offer more details and specific clinical examples in their writings (Beck & Freeman, 1990; Pretzer & Beck, 2005), the preceding lengthy list of problems a therapist is like to encounter clearly suggests that working with these clients is difficult at best. So, is cognitive therapy effective in the treatment of personality disorders? Numerous uncontrolled clinical reports suggest that it is, but the small number of controlled studies have offered equivocal results. More important, however, is the reality of "real-life" clinical practice:

In clinical practice, most therapists do not apply a standardized treatment protocol with a homogenous sample of individuals who share a common diagnosis. Instead, clinicians face a variety of clients and take an individualized approach to treatment. A recent study of the effectiveness of cognitive therapy under such "real world" conditions provides important support for the clinical use of cognitive therapy with clients who are diagnosed as having personality disorders… (pg. 102; Pretzer & Beck, 2005)

So what can we conclude from this discussion? There is consensus that personality disorders are prevalent in our society and they are resistant to treatment. Cognitive therapy, and the theory underlying it, has offered a promising avenue for further research. Given the significant impact of personality disorders on both individuals and society as a whole, any promising line of research deserves to be pursued vigorously.

Review of Key Points

• Personal construct theory emphasizes the cognitions that precede behavior, even more than one’s ability to accurately predict behavior.

• Kelly used the term man-the-scientist to describe how each person creates and tests representations of the world, in an effort to predict and control their environment.

• Our representations, or constructs, are open to revision, and there are always alternatives. The process by which we test and modify our constructs is called constructive alternativism.

• Personal construct theory begins with a fundamental postulate, which is then elaborated with eleven corollaries.

• Personal constructs regularly undergo transition, from ineffective constructs to more effective constructs. Problems with these transitions may result in feelings of threat, fear, guilt, or anxiety.

• There are two typical cycles of transition: the C-P-C Cycle and the Creativity Cycle. The first involves circumspection, preemption, and control. The Creativity Cycle requires beginning with loose constructions and then rapidly testing and pursuing new and effective constructs.

• Creativity appears to be an important component of healthy psychological functioning.

• Both the therapist and the client bring their own conceptualizations about therapy into the therapeutic process. The ultimate goal is to help the client generate movement forward, such that the process can continue after therapy has ended.

• In order to facilitate cognitive-behavioral therapy within his theoretical framework, Kelly developed an assessment tool known as the Role Construct Repertory Test and a therapeutic procedure known as fixed-role therapy.

• In diverse settings, it is essential for therapists to be familiar with cross-cultural issues related to therapy in order to understand the nature of a client’s constructs and schemas.

• Ellis proposed an ABC theory of personality: activating events lead to beliefs about a situation, and those beliefs lead to the consequences of the event.

• Rational emotive behavior therapy was designed to dispute the client’s irrational beliefs, thus leading to effective new philosophies, emotions, and behaviors.

• The straightforward nature of rational emotive behavior therapy, and Ellis’ willingness to write for a popular audience, was a major factor in establishing the self-help genera.

• Rational Recovery, based on rational emotive behavior therapy, was developed as an alternative to Alcoholics Anonymous. In particular, it does not include the religious requirements of AA.

• Rational emotive behavior therapy was challenged as being atheistic. Ellis later wrote that he is not opposed to spirituality, but he is opposed to dysfunctional, dogmatic religious beliefs that hinder one’s personal growth.

• Ellis openly challenged the value placed on self-esteem, suggesting instead that what is important is acceptance of ourselves, including our flaws and mistakes.

• Beck developed his cognitive therapy while conducting research on depression.

• Beck's cognitive model is based on automatic thoughts, schemas, and cognitive distortions. Depression in particular results from a cognitive triad: a negative view of past, present, and future.

• Cognitive therapy offers an active, directive, time-limited, and structured approach to psychotherapy.

• Cognitive therapy involves collaborative empiricism and guided discovery. Hopefully, the client is able to learn skills that will allow them to continue improving their life even after therapy has ended.

• Acceptance and Commitment Therapy incorporates mindfulness into a modern therapeutic process, emphasizing the acceptance of one's suffering in life followed by moving forward and living a values-based life. Radical Acceptance incorporates traditional Buddhist mindfulness practice (vipassana) into the therapeutic process.

Attribution and critical-reading note

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Read historical and theoretical claims critically. A theory is a lens for inquiry, not a verdict about a child or family. Where source claims are dated or culture-bound, the new lecture supplies cautions and contemporary framing.

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Psychotherapy: Cognitive Therapy

Cognitive therapy is a form of psychotherapy that focuses on how a person's thoughts lead to feelings of distress. The idea behind cognitive therapy is that how you think determines how you feel and act. Cognitive therapists help their clients change dysfunctional thoughts in order to relieve distress. They help a client see how they misinterpret a situation (cognitive distortion). For example, a client may overgeneralize. Because a person failed one test in Psychology 101, they feel stupid and worthless. These thoughts then cause their mood to worsen. Therapists also help clients recognize when they blow things out of proportion. Because Ray failed his Psychology 101 test, he has concluded that he's going to fail the entire course and probably flunk out of college altogether. These errors in thinking have contributed to Ray's feelings of distress. His therapist will help him challenge these irrational beliefs, focus on their illogical basis, and correct them with more logical and rational thoughts and beliefs.

Cognitive therapy was developed by psychiatrist Aaron **Beck** in the 1960s. His initial focus was on depression and how a client's self-defeating attitude served to maintain a depression despite positive factors in her life (Beck, Rush, Shaw, & Emery, 1979) (). Through questioning, a cognitive therapist can help a client recognize dysfunctional ideas, challenge catastrophizing thoughts about themselves and their situations, and find a more positive way to view things (Beck, 2011).

Source figure caption: Your emotional reactions are the result of your thoughts about the situation rather than the situation itself. For instance, if you consistently interpret events and emotions around the themes of loss and defeat, then you are likely to be depressed. Through therapy, you can learn more logical ways to interpret situations.

Note: View a brief video in which Judith Beck talks about cognitive therapy and conducts a session with a client.

Psychotherapy: Cognitive-Behavioral Therapy

Cognitive-behavioral therapists focus much more on present issues than on a patient's childhood or past, as in other forms of psychotherapy. One of the first forms of cognitive-behavioral therapy was **rational emotive therapy (RET)**, which was founded by Albert Ellis and grew out of his dislike of Freudian psychoanalysis (Daniel, n.d.). Behaviorists such as Joseph Wolpe also influenced Ellis's therapeutic approach (National Association of Cognitive-Behavioral Therapists, 2009).

Cognitive-behavioral therapy (CBT) helps clients examine how their thoughts affect their behavior. It aims to change cognitive distortions and self-defeating behaviors. In essence, this approach is designed to change the way people think as well as how they act. It is similar to cognitive therapy in that CBT attempts to make individuals aware of their irrational and negative thoughts and helps people replace them with new, more positive ways of thinking. It is also similar to behavior therapies in that CBT teaches people how to practice and engage in more positive and healthy approaches to daily situations. In total, hundreds of studies have shown the effectiveness of cognitive-behavioral therapy in the treatment of numerous psychological disorders such as depression, PTSD, anxiety disorders, eating disorders, bipolar disorder, and substance abuse (Beck Institute for Cognitive Behavior Therapy, n.d.). For example, CBT has been found to be effective in decreasing levels of hopelessness and suicidal thoughts in previously suicidal teenagers (Alavi, Sharifi, Ghanizadeh, & Dehbozorgi, 2013). Cognitive-behavioral therapy has also been effective in reducing PTSD in specific populations, such as transit workers (Lowinger & Rombom, 2012).

Cognitive-behavioral therapy aims to change cognitive distortions and self-defeating behaviors using techniques like the ABC model. With this model, there is an Action (sometimes called an activating event), the Belief about the event, and the Consequences of this belief. Let's say Jon and Joe both go to a party. Jon and Joe each have met an interesting person at the party and spend a few hours chatting with them. At the end of the party, Jon and Joe ask to exchange phone numbers with the person they've been talking to, and the request is refused. Both Jon and Joe are surprised, as they thought things were going well. What can Jon and Joe tell themselves about why the person was not interested? Let's say Jon tells himself he is a loser, or is ugly, or "has no game." Jon then gets depressed and decides not to go to another party, which starts a cycle that keeps him depressed. Joe tells himself that he had bad breath, goes out and buys a new toothbrush, goes to another party, and meets someone new.

Jon's belief about what happened results in a consequence of further depression, whereas Joe's belief does not. Jon is internalizing the attribution or reason for the rebuffs, which triggers his depression. On the other hand, Joe is externalizing the cause, so his thinking does not contribute to feelings of depression. Cognitive-behavioral therapy examines specific maladaptive and automatic thoughts and cognitive distortions. Some examples of cognitive

distortions are all-or-nothing thinking, overgeneralization, and jumping to conclusions. In overgeneralization, someone takes a small situation and makes it huge—for example, instead of saying, “This particular person was not interested in me,” the man says, “I am ugly, a loser, and no one is ever going to be interested in me.”

All or nothing thinking, which is a common type of cognitive distortion for people suffering from depression, reflects extremes. In other words, everything is black or white. After being turned down for a date, Jon begins to think, “No woman will ever go out with me. I’m going to be alone forever.” He begins to feel anxious and sad as he contemplates his future.

The third kind of distortion involves jumping to conclusions—assuming that people are thinking negatively about you or reacting negatively to you, even though there is no evidence. Consider the example of Savannah and Hillaire, who recently met at a party. They have a lot in common, and Savannah thinks they could become friends. She calls Hillaire to invite her for coffee. Since Hillaire doesn’t answer, Savannah leaves her a message. Several days go by and Savannah never hears back from her potential new friend. Maybe Hillaire never received the message because she lost her phone or she is too busy to return the phone call. But if Savannah believes that Hillaire didn’t like Savannah or didn’t want to be her friend, she is demonstrating the cognitive distortion of jumping to conclusions.

How effective is CBT? One client said this about his cognitive-behavioral therapy:

I have had many painful episodes of depression in my life, and this has had a negative effect on my career and has put considerable strain on my friends and family. The treatments I have received, such as taking antidepressants and psychodynamic counseling, have helped [me] to cope with the symptoms and to get some insights into the roots of my problems. CBT has been by far the most useful approach I have found in tackling these mood problems. It has raised my awareness of how my thoughts impact on my moods. How the way I think about myself, about others and about the world can lead me into depression. It is a practical approach, which does not dwell so much on childhood experiences, whilst acknowledging that it was then that these patterns were learned. It looks at what is happening now, and gives tools to manage these moods on a daily basis. (Martin, 2007, n.p.)

CHAPTER 12 / CAC.902

Social Cognitive Theory and Self-Efficacy

Albert Bandura's social cognitive theory emphasizes learning through observation, the role of cognition, and the capacity for self-regulation. A central concept is self-efficacy: the belief that one can organize and execute the actions needed to manage a situation. Self-efficacy is not self-esteem. A child may like themselves but doubt their ability to solve math problems. Another child may have low self-worth but high efficacy in sports. Counseling often targets efficacy because it predicts effort, persistence, and recovery from setbacks.

Observational learning

Children learn by watching models. A child who sees a parent handle frustration calmly may learn a regulation strategy. A child who sees aggression rewarded may learn aggression. Modeling is not imitation alone. The observer attends, retains, reproduces, and is motivated. Counselors can use coping models, not just mastery models. A coping model shows struggle and gradual success. A mastery model shows flawless performance. For anxious children, a coping model may be more believable.

Sources of self-efficacy

Bandura described four main sources: mastery experiences, vicarious experience, verbal persuasion, and physiological/affective states. Mastery experiences are the most powerful. A child who succeeds at a small step gains evidence of capability. Vicarious experience comes from seeing similar others succeed. Verbal persuasion can help if it is credible. Physiological states matter because a racing heart may be interpreted as danger or excitement. The counselor can help the client reinterpret arousal and build mastery steps.

Self-regulation and agency

Social cognitive theory views people as agents who set goals, monitor progress, and adjust behavior. Self-regulation includes self-observation, judgment, and self-response. A teen who tracks study time, compares it with a goal, and rewards effort is using self-regulation. The counselor should avoid turning self-regulation into self-blame. If a plan fails, the counselor asks about barriers, skills, and environment. Agency is embedded in social conditions. A child cannot regulate their way out of food insecurity.

Reciprocal determinism

Bandura proposed reciprocal determinism: behavior, personal factors, and environment influence one another. A child's anxiety leads to avoidance; avoidance reduces peer contact; reduced contact increases anxiety. Changing any part can shift the system. The counselor may target behavior through exposure, cognition through reappraisal, or environment through school accommodation. The theory supports multi-level intervention.

Clinical example

Sixteen-year-old Marcus wants to join a robotics club but believes he is not smart enough. The counselor uses social cognitive principles. They identify a coping model: an older student who struggled at first. They break participation into mastery steps: attend one meeting, ask one question, build one small part. Marcus tracks his predictions and outcomes. The counselor also addresses the school environment by asking the club sponsor to welcome newcomers. Marcus's efficacy grows because he accumulates evidence, not because the counselor says, "Believe in yourself."

Media and digital models

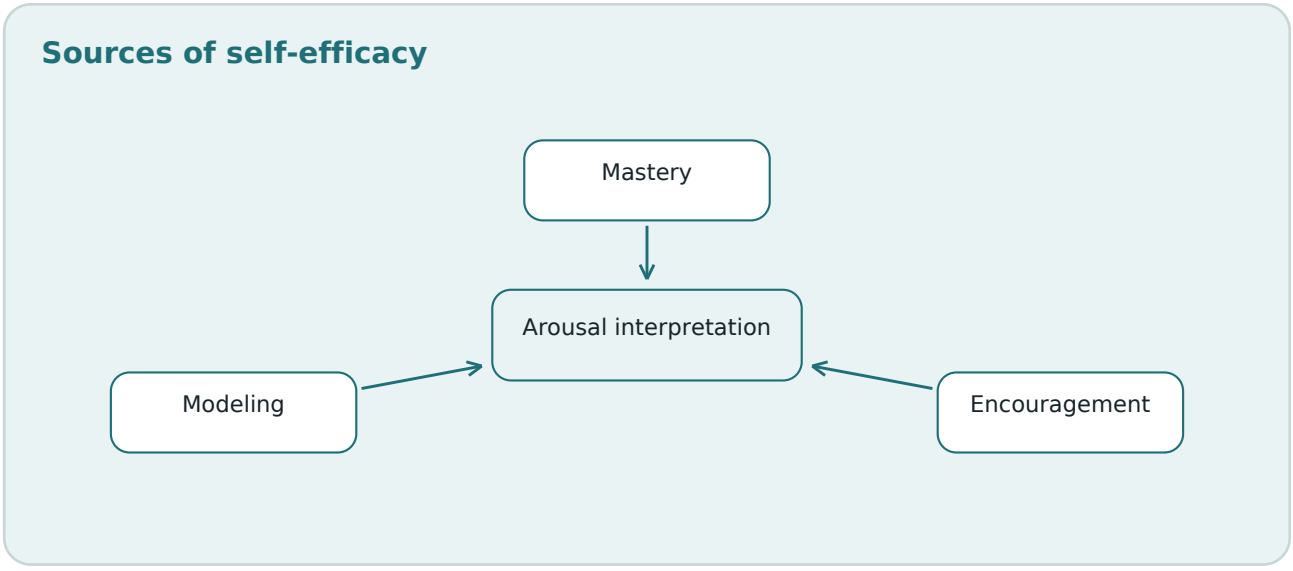
Adolescents learn from online models. Social media can provide supportive communities or harmful comparison. A teen who sees curated images may develop low efficacy about their own body or life. Counselors can teach critical media literacy: Who is the model? What is edited? What is the incentive? The goal is not to ban media but to increase agency in interpreting it.

Evidence and limits

Self-efficacy is a robust predictor in many domains, but it is not a cure-all. High efficacy without skill can lead to failure. Low efficacy may be realistic in an unsafe environment. The counselor should build actual competence and safety, not just confidence. Social cognitive theory integrates well with CBT, behavioral parent training, and school consultation.

Reflection

Choose a goal you have avoided. Rate your efficacy from zero to ten. Identify one mastery step, one model, one source of encouragement, and one way to reinterpret arousal. Then take the step and record what you learn. Efficacy changes through experience.



Self-efficacy grows through experience, observation, credible support, and reinterpretation of arousal.

Attributed Foundational Reading

Attribution and critical-reading note

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Read historical and theoretical claims critically. A theory is a lens for inquiry, not a verdict about a child or family. Where source claims are dated or culture-bound, the new lecture supplies cautions and contemporary framing.

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The social learning theorists observed that the complexity of human behavior cannot easily be explained by traditional behavioral theories. Bandura recognized that people learn a great deal from watching other people and seeing the rewards and/or punishments that other people receive. Social learning theorists do not deny the influence of reinforcement and punishment, but rather, they suggest that it can be experienced through observation and does not require direct, personal experience as Skinner would argue. In addition, observational learning requires cognition, something that radical behaviorists consider outside the realm of psychological research, since cognition cannot be observed. Bandura took a broad theoretical perspective on social learning, whereas Rotter and Mischel focused more closely on specific cognitive aspects of social learning and behavior.

It is also important to point out an artificial distinction that is difficult to avoid in the chapters of this section. Chapters 10, 11, and 12 are roughly set up as chapters on radical behaviorism and formal learning theory, followed by social learning, and then concluding with cognitive theories on personality development. However, as will be evident, the chapters overlap a great deal. For example, Dollard and Miller’s attempt to find a middle ground between Freud and Skinner led to their initial descriptions of social learning, which provided a prelude to this chapter. Bandura, Rotter, and Mischel address a number of aspects of cognition in their theories, but they are not as completely focused on cognition as are Kelly, Beck, and Ellis, hence the separation of this chapter from the following one. In Social Learning Theory, Bandura had this to say:

A valid criticism of extreme behaviorism is that, in a vigorous effort to avoid spurious inner causes, it has neglected determinants of behavior arising from cognitive functioning…Because some of the inner causes invoked by theorists over the years have been ill-founded does not justify excluding all internal determinants from scientific inquiry…such studies reveal that people learn and retain behavior much better by using cognitive aids that they generate than by reinforced repetitive performance…A theory that denies that thoughts can regulate actions does not lend itself readily to the explanation of complex human behavior. (pg. 10; Bandura, 1977).

Albert Bandura and Social Learning Theory

Bandura is the most widely recognized individual in the field of **social learning theory**, despite the facts that Dollard and Miller established the field and Rotter was beginning to examine cognitive social learning a few years before Bandura. Nonetheless, Bandura’s

research has had the most significant impact, and the effects of **modeling** on aggressive behavior continue to be studied today (see “Personality Theory in Real Life” at the end of the chapter). Therefore, we will begin this chapter by examining the basics of Bandura’s social learning perspective.

Brief Biography of Albert Bandura

Albert Bandura was born in 1925, in the small town of Mundare, in northern Alberta, Canada. His parents had emigrated from Eastern Europe (his father from Poland, his mother from the Ukraine), and eventually saved enough money to buy a farm. Farming in northern Canada was not easy. One of Bandura’s sisters died during a flu pandemic, one of his brothers died in a hunting accident, and part of the family farm was lost during the Great Depression. Nonetheless, the Bandura family persevered, and maintained a lively and happy home.

Although Bandura’s parents lacked any formal education, they stressed its value. Despite having only one small school in town, which lacked both teachers and academic resources, the town’s children developed a love of learning and most of them attended universities around the world. Following the encouragement of his parents, Bandura also sought a wide variety of other experiences while he was young. He worked in a furniture manufacturing plant, and performed maintenance on the Trans-Alaska highway. The latter experience, in particular, introduced Bandura to a variety of unusual individuals, and offered a unique perspective on psychopathology in everyday life.

When Bandura went to the University of British Columbia, he intended to major in biology. However, he had joined a carpool with engineering and pre-med students who attended classes early in the morning. Bandura looked for a class to fit this schedule, and happened to notice that an introductory psychology course was offered at that time. Bandura enjoyed the class so much that he changed his major to psychology, receiving his bachelor’s degree in 1949. Bandura then attended graduate school at the University of Iowa, in a psychology department strongly influenced by Kenneth Spence, a former student of Clark Hull. Thus, the psychology program at the University of Iowa was strongly behavioral in its orientation, and they were well versed in the behavioral research conducted in the psychology department at Yale University.

As we saw in the previous chapter, John Dollard and Neal Miller had established the field of social learning at Yale in the 1930s, but they had done so within the conceptual guidelines of Hullian learning theory. Bandura was not particularly interested in Hull’s approach to learning, but he was impressed by Dollard and Miller’s concepts of modeling and imitation. Bandura received his Ph.D. in clinical psychology in 1952, and then began a postdoctoral position at the Wichita Guidance Center. Bandura was attracted to this position, in part, because the psychologist in charge was not heavily immersed in the Freudian psychodynamic approach that was still so prevalent in clinical psychology.

Following his postdoctoral training, Bandura became a member of the faculty at Stanford University, where he spent the rest of his career. The chairman of Bandura’s department had been studying frustration and aggression, and this influenced Bandura to begin his own studies on social learning and aggression. This research revealed the critical role that modeling plays in social learning, and soon resulted in the publication of Adolescent Aggression (co-authored by Richard Walters, Bandura’s first graduate student; Bandura &

Walters, 1959). This line of research also led to the famous “Bobo” doll studies, which helped to demonstrate that even young children can learn aggressive behavior by observing models. Bandura then became interested in self-regulatory behavior in children, and one of the colleagues he collaborated with was Walter Mischel, whose work we will address later in this chapter. During his long and productive career, Bandura became more and more interested in the role played by cognition in social learning, eventually renaming his theory to reflect his social cognitive perspective on human learning. He also examined the role of the individual in influencing the nature of the environment in which they experience life, and how their own expectations of self-efficacy affect their willingness to participate in aspects of that life.

Bandura has received numerous honors during his career. Included among them, he has served as president of the American Psychological Association and received a Distinguished Scientific Contribution Award from APA. He received the William James Awardfrom the American Psychological Society (known today as the Association for Psychological Science), a Guggenheim Fellowship, the Distinguished Contribution Award from the International Society for Research in Aggression, and a Distinguished Scientist Award from the Society of Behavioral Medicine. Bandura has also been elected to the American Academy of Arts and Sciences, to the Institute of Medicine of the National Academy of Sciences, and he has received numerous honorary degrees from universities around the world. The list goes on, not the least of which is his Outstanding Lifetime Contribution to Psychology Award, received from APA in 2004.

Placing Bandura in Context: Social Learning Theory

Establishes Its Independence

Although social learning theory has its foundation in the work of Dollard and Miller, they addressed social learning in the context of Hullian learning theory (complete with mathematical formulae). Bandura shifted the focus of social learning away from traditional behavioral perspectives, and established social learning as a theory on its own. Bandura also freely acknowledged cognition in the learning process, something that earlier behaviorists had actively avoided. By acknowledging both the external processes of reinforcement and punishment and the internal cognitive processes that make humans so complex, Bandura provided a comprehensive theory of personality that has been very influential.

Although Bandura criticized both operant conditioning and Pavlovian conditioning as being too radical, he relied on a procedure that came from Pavlovian conditioning research for one of his most influential concepts: the use of modeling. The modeling procedure was developed by Mary Cover Jones, a student of John B. Watson, in her attempts to counter-condition learned phobias. Subsequent to the infamous “Little Albert” studies conducted by Watson, Jones used models to interact in a pleasant manner with a rabbit that test subjects had been conditioned to fear. After a few sessions, the test subjects were no longer afraid of the rabbit (see Stagner, 1988). This may have been the first use of behavior therapy, and Bandura’s use of the procedure helped to bring together different behavioral disciplines.

Perhaps one of Bandura’s most significant contributions, however, has been the application of his theory to many forms of media. Congressional committees have debated the influence of modeling aggression through violent television programs, movies, and video games. We now have ratings on each of those forms of media, and yet the debate continues because of

the levels of aggression seen in our schools, in particular, and society in general. Bandura's Bobo doll studies are certainly among of the best known studies in psychology, and they are also among the most influential in terms of practical daily applications. The long list of awards that Bandura has received is a testament to both his influence on psychology and the respect that influence has earned for him.

Reciprocal Determinism

One of the most important aspects of Bandura's view on how personality is learned is that each one of us is an agent of change, fully participating in our surroundings and influencing the environmental contingencies that behaviorists believe affect our behavior. These interactions can be viewed three different ways. The first is to consider behavior as a function of the person and the environment. In this view, personal dispositions (or traits) and the consequences of our actions (reinforcement or punishment) combine to cause our behavior. This perspective is closest to the radical behaviorism of Skinner. The second view considers that personal dispositions and the environment interact, and the result of the interaction causes our behavior, a view somewhat closer to that of Dollard and Miller. In each of these perspectives, behavior is caused, or determined, by dispositional and environmental factors, the behavior itself is not a factor in how that behavior comes about. However, according to Bandura, social learning theory emphasizes that behavior, personal factors, and environmental factors are all equal, interlocking determinants of each other. This concept is referred to as **reciprocal determinism** (Bandura, 1973, 1977).

Early theories considered behavior to be a function of the person and their environment, or a function of the interaction between the person and their environment. Bandura believed that behavior itself influences both the person and the environment, each of which in turn affects behavior and each other. The result is a complex interplay of factors known as reciprocal determinism.

Reciprocal determinism can be seen in everyday observations, such as those made by Bandura and others during their studies of aggression. For example, approximately 75 percent of the time, hostile behavior results in unfriendly responses, whereas friendly acts seldom result in such consequences. With little effort, it becomes easy to recognize individuals who create negative social climates (Bandura, 1973). Thus, while it may still be true that changing environmental contingencies changes behavior, it is also true that changing behavior alters the environmental contingencies. This results in a unique perspective on freedom vs. determinism. Usually we think of determinism as something that eliminates or restricts our freedom. However, Bandura believed that individuals can intentionally act as agents of change within their environment, thus altering the factors that determine their behavior. In other words, we have the freedom to influence that which determines our behavior:

…Given the same environmental constraints, individuals who have many behavioral options and are adept at regulating their own behavior will experience greater freedom than will individuals whose personal resources are limited. (pg. 203; Bandura, 1977)

Discussion Question: According to the theory of reciprocal determinism, our behavior interacts with our environment and our personality variables to influence our life. Can you think of situations in which your actions caused a noticeable change in the people or situations around you? Remember that these changes can be either good or bad.

Observational Learning and Aggression

Social learning is also commonly referred to as **observational learning**, because it comes about as a result of observing **models**. Bandura became interested in social aspects of learning at the beginning of his career. Trained as a clinical psychologist, he began working with juvenile delinquents, a somewhat outdated term that is essentially a socio-legal description of adolescents who engage in antisocial behavior. In the 1950s there was already research on the relationships between aggressive boys and their parents, as well as some theoretical perspectives regarding the effects of different child-rearing practices on the behavior and attitudes of adolescent boys (Bandura & Walters, 1959). Much of the research focused, however, on sociological issues involved in the environment of delinquent boys. Choosing a different approach, Bandura decided to study boys who had no obvious sociological disadvantages (such as poverty, language difficulties due to recent immigration, low IQ, etc.). Bandura and Walters restricted their sample to boys of average or above average intelligence, from intact homes, with steadily employed parents, whose families had been settled in America for at least three generations. No children from minority groups were included either. In other words, the boys were from apparently typical, White, middle-class American families. And yet, half of the boys studied were identified through the county probation service or their school guidance center as demonstrating serious, repetitive, antisocial, aggressive behavior (Bandura & Walters, 1959).

Citing the work of Dollard and Miller, as well as others who paved the way for social learning theory, Bandura and Walters began their study on adolescent aggression by examining how the parents of delinquents train their children to be socialized. Working from a general learning perspective, emphasizing cues and consequences, they found significant problems in the development of socialization among the delinquent boys. These boys developed dependency, a necessary step toward socialization, but they were not taught to conform their behavior to the expectations of society. Consequently, they began to demand immediate and unconditional gratification from their surroundings, something that seldom happens. Of course, this failure to learn proper socialization does not necessarily lead to aggression, since it can also lead to lifestyles such as the hobo, the bohemian, or the “beatnik” (Bandura & Walters, 1959). Why then do some boys become so aggressive? To briefly summarize their study, Bandura and Walters found that parents of delinquent boys were more likely to model aggressive behavior and to use coercive punishment (as opposed to reasoning with their children to help them conform to social norms). Although parental modeling of aggressive behavior teaches such behavior to children, these parents tend to be effective at suppressing their children’s aggressive behavior at home. In contrast, however, they provide subtle encouragement for aggression outside the home. As a result, these poorly socialized boys are likely to displace the aggressive impulses that develop in the home, and they are well trained in doing so. If they happen to associate with a delinquent group (such as a gang), they are provided with an opportunity to learn new and more effective ways to engage in antisocial behavior, and they are directly rewarded for engaging in such behaviors (Bandura & Walters, 1959; also see Bandura, 1973).

Having found evidence that parents of aggressive, delinquent boys had modeled aggressive behavior, Bandura and his colleagues embarked on a series of studies on the modeling of aggression (Bandura, Ross, & Ross, 1961, 1963a,b). Initially, children were given the opportunity to play in a room containing a variety of toys, including the 5-foot tall, inflated Bobo doll (a toy clown). As part of the experiment, an adult (the model) was also invited into

the room to join in the game. When the model exhibited clear aggressive behavior toward the Bobo doll, and then the children were allowed to play on their own, they children demonstrated aggressive behavior as well. The children who observed a model who was not aggressive seldom demonstrated aggressive behavior, thus confirming that the aggression in the experimental group resulted from observational learning. In the second study, children who observed the behavior of aggressive models on film also demonstrated a significant increase in aggressive behavior, suggesting that the physical presence of the model is not necessary (providing an important implication for violent aggression on TV and in movies; see “Personality in Theory in Real Life” at the end of the chapter). In addition to confirming the role of observation or social learning in the development of aggressive behavior, these studies also provided a starting point for examining what it is that makes a model influential.

One of the significant findings in this line of research on aggression is the influence of models on behavioral restraint. When children are exposed to models who are not aggressive and who inhibit their own behavior, the children also tend to inhibit their own aggressive responses and to restrict their range of behavior in general (Bandura, Ross, & Ross, 1961). Thus, children can learn from others, in particular their parents, how to regulate their behavior in socially appropriate ways. When the inappropriate behavior of others is punished, the children observing are also vicariously punished, and likely to experience anxiety, if not outright fear, when they consider engaging in similar inappropriate behavior. However, when models behave aggressively and their behavior is rewarded, or even just tolerated, the child’s own tendency to restrict aggressive impulses may be weakened. This weakening of restraint, which can then lead to acting out aggressive impulses, is known as **disinhibition**:

Modeling may produce disinhibitory effects in several ways. When people respond approvingly or even indifferently to the actions of assailants, they convey the impression that aggression is not only acceptable but expected in similar situations. By thus legitimizing aggressive conduct, observers anticipate less risk of reprimand or loss of self-respect for such action. (pg. 129; Bandura, 1973)

Discussion Question: The concept of disinhibition is based on the belief that we all have aggressive tendencies, and our self-control is diminished when we see models rewarded for aggressive behavior. Have you ever found yourself in situations where someone was rewarded for acting aggressively? Did you then adopt an aggressive attitude, or act out on your aggression?

Characteristics of the Modeling Situation

When one person matches the behavior of another, there are several perspectives on why that matching behavior occurs. Theorists who suggest that matching behavior results from simple **imitation**don’t allow for any significant psychological changes. Dollard and Miller discussed imitation in their attempts to combine traditional learning theory with a psychodynamic perspective, but they did not advance the theory very far. A more traditional psychodynamic approach describes matching behavior as the result of **identification**, the concept that an observer connects with a model in some psychological way. However, identification means different things to different theorists, and the term remains somewhat vague. In social learning, as it has been advanced by Bandura, modeling is the term that best

describes and, therefore, is used to characterize the psychological processes that underlie matching behavior (Bandura, 1986).

Observational learning through modeling is not merely an alternative to Pavlovian or operant conditioning:

Learning would be exceedingly laborious, not to mention hazardous, if people had to rely solely on the effects of their own actions to inform them what to do. Fortunately, most human behavior is learned observationally through modeling: from observing others one forms an idea of how new behaviors are performed, and on later occasions this coded information serves as a guide for action. (pg. 22; Bandura, 1977)

Individuals differ in the degree to which they can be influenced by models, and not all models are equally effective. According to Bandura, three factors are most influential in terms of the effectiveness of modeling situations: the characteristics of the model, the attributes of the observers, and the consequences of the model's actions. The most relevant characteristics of an influential model are high status, competence, and power. When observers are unsure about a situation, they rely on cues to indicate what they perceive as evidence of past success by the model. Such cues include general appearance, symbols of socioeconomic success (e.g., a fancy sports car), and signs of expertise (e.g., a doctor's lab coat). Since those models appear to have been successful themselves, it seems logical that observers might want to imitate their behavior. Individuals who are low in self-esteem, dependent, and who lack confidence are not necessarily more likely to be influenced by models. Bandura proposed that when modeling is used to explicitly develop new competencies, the ones who will benefit most from the situation are those who are more talented and more venturesome (Bandura, 1977).

Despite the potential influence of models, the entire process of observational learning in a social learning environment would probably not be successful if not for four important component processes: **attentional processes**, **retention processes**, **production** (or reproduction) **processes**, and **motivational processes** (Bandura, 1977, 1986). The fact that an observer must pay attention to a model might seem obvious, but some models are more likely to attract attention. Individuals are more likely to pay attention to models with whom they associate, even if the association is more cognitive than personal. It is also well-known that people who are admired, such as those who are physically attractive or popular athletes, make for attention-getting models. There are also certain types of media that are very good at getting people's attention, such as television advertisements (Bandura, 1977, 1986). It is a curious cultural phenomenon that the television advertisements presented during the National Football League's Super Bowl have become almost as much of the excitement as the game itself (and even more exciting for those who are not football fans)!

The retention processes involve primarily an observer's memory for the modeled behavior. The most important memory processes, according to Bandura, are visual imagery and verbal coding, with visual imagery being particularly important early in development when verbal skills are limited. Once modeled behavior has been transformed into visual and/or verbal codes, these memories can serve to guide the performance of the behavior at appropriate times. When the modeled behavior is produced by the observer, the so-called production process, the re-enactment can be broken down into the cognitive organization of the responses, their initiation, subsequent monitoring, and finally the refinement of the behavior

based on informative feedback. Producing complex modeled behaviors is not always an easy task:

…A common problem in learning complex skills, such as golf or swimming, is that performers cannot fully observe their responses, and must therefore rely upon vague kinesthetic cues or verbal reports of onlookers. It is difficult to guide actions that are only partially observable or to identify the corrections needed to achieve a close match between representation and performance. (pg. 28; Bandura, 1977)

Finally, motivational processes determine whether the observer is inclined to match the modeled behavior in the first place. Individuals are most likely to model behaviors that result in an outcome they value, and if the behavior seems to be effective for the models who demonstrated the behavior. Given the complexity of the relationships between models, observers, the perceived effectiveness of modeled behavior, and the subjective value of rewards, even using prominent models does not guarantee that they will be able to create similar behavior in observers (Bandura, 1977, 1986).

Producing complex behaviors that cannot be readily observed by the performer can prove quite difficult. John cut behind the defender to drive the ball with his knee over the goalie’s head and into the goal (above). Samuel scores a round kick in a Taekwondo tournament (right).

A common misconception regarding modeling is that it only leads to learning the behaviors that have been modeled. However, modeling can lead to innovative behavior patterns. Observers typically see a given behavior performed by multiple models; even in early childhood one often gets to see both parents model a given behavior. When the behavior is then matched, the observer will typically select elements from the different models, relying on only certain aspects of the behavior performed by each, and then create a unique pattern that accomplishes the final behavior. Thus, partial departures from the originally modeled behavior can be a source of new directions, especially in creative endeavors (such as composing music or creating a sculpture). In contrast, however, when simple routines prove useful, modeling can actually stifle innovation. So, the most innovative individuals appear to be those who have been exposed to innovative models, provided that the models are not so innovative as to create an unreasonably difficult challenge in modeling their creativity and innovation (Bandura, 1977, 1986; Bandura, Ross, & Ross, 1963b).

Discussion Question: Two of the components necessary for modeling to be effective, according to Bandura, are attention and retention. What aspects of commercial advertisements are most likely to catch your attention? What do you tend to remember about advertisements? Can you think of situations in which the way an advertiser gets your attention also helps you to remember the product?

Connections Across Cultures: Global Marketing and Advertising

Although we are constantly surrounded by modeling situations, the most obvious and intentional use of models and modeling is in advertising. As our world becomes increasingly global, the use of advertisements that work well in one place may be entirely inappropriate in a different culture. Marieke de Mooij, the president of a cross-cultural communications consulting firm in the Netherlands, and a visiting professor at universities in the Netherlands, Spain, Finland, and Germany, has undertaken the challenging task of studying

how culture affects consumer behavior and the consequences of those effects for marketing and advertising in different societies around the world (de Mooij, 2000, 2004a,b, 2005).

To some, increasing globalization suggests that markets around the world will become more similar to one another. De Mooij (2000), however, contends that as different cultures become more similar in economic terms, their more personal cultural differences will actually become more significant! Thus, it is essential for global businesses to understand those cultural differences, so that marketing and advertising can be appropriately adjusted. The challenge is in recognizing and dealing with the “global-local paradox.” People in business are taught to think global, but act local. This is because most people throughout the world tend to prefer things that are familiar. They may adopt and enjoy global products, but they remain true to their own culture (de Mooij, 2005). Thus, it is important to understand local culture and consumer behavior in general before beginning an advertising campaign in a foreign country.

In her studies on culture and consumer behavior, de Mooij (2004b, 2005) addresses a wide variety of topics, including several that are covered in this chapter. In terms of the characteristics of models, many countries do not emphasize physical attractiveness and/or fashionable clothes the way we do in America. However, the overall aesthetic appeal of advertising can be more important in many Asian markets, focusing on preferences for values such as nature and harmony. Given that America is generally an individualistic culture and most Asian cultures are collectivistic, it should be no surprise that Americans tend to focus on the appeal of the model whereas Asians tend to focus on the appeal of the overall scene and relationships amongst the various aspects depicted within it. In a similar way, cultures differ in terms of their general perspective on locus of control. In cultures that tend to believe that their lives are determined by external forces, the moral authorities (such as the church and the press) are typically trusted. People in such cultures might not be responsiveness to advertisements that call for individual restraint, such as efforts to reduce cigarette smoking for better health, since they rely on their doctors (external agents) to take care of their health.

There are also significant differences in how people in different cultures think and process information, and cognitive processes underlie all aspects of social learning. Involvement theory suggests differences in how individuals and cultures differ in their approach to purchasing, and how advertising must take those differences into account. For example, amongst American consumers considering a “high involvement” product (such as a car), those who are likely to buy something respond to advertising in which they learn something, develop a favorable attitude toward the product, and then buy it (learn-feel-do). For everyday products, which are considered “low involvement,” consumers respond to advertising in which they learn something, then buy the product, and perhaps afterward they tend to prefer that brand (learn-do-feel). There is now evidence that in typically more collectivist cultures such as Japan, China, and Korea, it is important to first establish a relationship between the company and the consumer. Only then does the consumer purchase the particular product, and then they become more familiar with it (feel-do-learn). Thus, the very purpose of advertising changes from culture to culture (de Mooij, 2004b). Naturally, a number of other approaches to advertising exist, based on concepts such as: persuasion, awareness, emotions, and likeability (de Mooij, 2005). Each of these techniques relies on a different psychological approach, taking into account the observers that the advertiser hopes to influence (the so-called target demographic).

One of the most important aspects of advertising, when it is being carried from one culture to another, is the translation of verbal and written information. Different languages have different symbolic references. They rely on different myths, history, humor, and art, and failing to tap into such differences is likely to result in bland advertising that does not appeal to the local audience (de Mooij, 2004a). Other languages are simply structured differently as well, perhaps requiring meaning in a name, and this sometimes makes direct translation impossible. Thus, an international advertiser must choose a different name for their product. For example, Coca-Cola is marketed in China as the homonym kekou kele, which means “tasty and happy” (de Mooij, 2004a). Visual references also have cultural meanings. A Nokia ad, shown in Finland, used a squirrel in a forest to represent good reception and free movement in a deep forest. A Chinese group, however, understood it as depicting an animal that lives far away from people. They simply did not understand the intent of the commercial. Research on interpersonal verbal communication styles suggests that certain countries can be grouped into preferred styles. Based on a comparison of multiple dimensions of preferred verbal style, some of the countries that share similar styles are: 1) the United States, the United Kingdom, Norway, the Netherlands, Sweden, and Denmark; 2) Austria, Finland, and Germany; 3) India, China, and Singapore; and 4) Italy, Spain, Belgium, France, Argentina, Brazil, and the Arab world (de Mooij, 2004a).

As challenging as it might seem to address these issues (and the many more we have not covered), it is essential for individuals involved in global marketing and advertising:

The cultural variety of countries worldwide as well as in Europe implies that success in one country does not automatically mean success in other countries…finding the most relevant cultural values is difficult, especially because many researchers are based in Western societies that are individualistic and have universalistic values. Western marketers, advertisers, and researchers are inclined to search for similarities, whereas understanding the differences will be more profitable…There are global products and global brands, but there are no global buying motives for such brands because people are not global. Understanding people across cultures is the first and most important step in international marketing. (pg. 314; de Mooij, 2004b)

Finally, none of these approaches to international marketing and advertising is going to be successful if business relationships are not established in the first place. In The Cultural Dimension of International Business, Ferraro (2006b) offers a comprehensive guide to understanding cultural differences. In addition to the importance of being aware of such factors, which the very existence of the books and articles in this section belies, Ferraro emphasizes cultural differences in communication. For individuals working in a foreign country where English is not the first language, good communication becomes a matter of intent:

Because communication is so vitally important for conducting business at home, it should come as no surprise that it is equally important for successful business abroad. The single best way to become an effective communicator as an expatriate is to learn the local language…Besides knowing how to speak another language, expatriate candidates should demonstrate a willingness to use it. For a variety of reasons, some people lack the motivation, confidence, or willingness to throw themselves into conversational situations. [Authors note: see the section on self-efficacy below]…Thus, communication skills must be

assessed in terms of language competency, motivation to learn another language, and willingness to use it in professional and personal situations. (pg. 170; Ferraro, 2006b)

Self-Regulation and Self-Efficacy

Self-regulation and **self-efficacy** are two elements of Bandura's theory that rely heavily on cognitive processes. They represent an individual's ability to control their behavior through internal reward or punishment, in the case of self-regulation, and their beliefs in their ability to achieve desired goals as a result of their own actions, in the case of self-efficacy. Bandura never rejects the influence of external rewards or punishments, but he proposes that including internal, **self-reinforcement** and **self-punishment** expands the potential for learning:

…Theories that explain human behavior as solely the product of external rewards and punishments present a truncated image of people because they possess self-reactive capacities that enable them to exercise some control over their own feelings, thoughts, and actions. Behavior is therefore regulated by the interplay of self-generated and external sources of influence… (pg. 129; Bandura, 1977)

Self-regulation is a general term that includes both self-reinforcement and self-punishment. Self-reinforcement works primarily through its motivational effects. When an individual sets a standard of performance for themselves, they judge their behavior and determine whether or not it meets the self-determined criteria for reward. Since many activities do not have absolute measures of success, the individual often sets their standards in relative ways. For example, a weight-lifter might keep track of how much total weight they lift in each training session, and then monitor their improvement over time or as each competition arrives. Although competitions offer the potential for external reward, the individual might still set a personal standard for success, such as being satisfied only if they win at least one of the individual lifts. The standards that an individual sets for themselves can be learned through modeling. This can create problems when models are highly competent, much more so than the observer is capable of performing (such as learning the standards of a world-class athlete). Children, however, seem to be more inclined to model the standards of low-achieving or moderately competent models, setting standards that are reasonably within their own reach (Bandura, 1977). According to Bandura, the cumulative effect of setting standards and regulating one's own performance in terms of those standards can lead to judgments about one's self. Within a social learning context, negative self-concepts arise when one is prone to devalue oneself, whereas positive self-concepts arise from a tendency to judge oneself favorably (Bandura, 1977). Overall, the complexity of this process makes predicting the behavior of an individual rather difficult, and behavior often deviates from social norms in ways that would not ordinarily be expected. However, this appears to be the case in a variety of cultures, suggesting that it is indeed a natural process for people (Bandura & Walters, 1963).

As noted above, "perceived self-efficacy refers to beliefs in one's capabilities to organize and execute the courses of action required to produce given attainments" (Bandura, 1997). The desire to control our circumstances in life seems to have been with us throughout history. In ancient times, when people knew little about the world, they prayed in the hope that benevolent gods would help them and/or protect them from evil gods. Elaborate rituals were developed in the hope or belief that the gods would respond to their efforts and dedication. As we learned more about our world and how it works, we also learned that we can have a

significant impact on it. Most importantly, we can have a direct effect on our immediate personal environment, especially with regard to personal relationships. What motivates us to try influencing our environment is specific ways is the belief that we can, indeed, make a difference in a direction we want. Thus, research has focused largely on what people think about their efficacy, rather than on their actual ability to achieve their goals (Bandura, 1997).

Self-efficacy has been a popular topic for research, and Bandura's book Self-Efficacy: The Exercise of Control(1997) is some 600 pages long. We will address two key issues on this fascinating topic: the relationships between (1) efficacy beliefs and outcome expectancies and (2) self-efficacy and **self-esteem**. In any situation, one has beliefs about one's ability to influence the situation, and yet those beliefs are typically balanced against realistic expectations that change can occur. Each side of the equation can have both negative and positive qualities. Suppose, as a student, you are concerned about the rising cost of a college education, and you would like to challenge those rising costs. You may believe that there is nothing you can do (negative) and tuition and fees will inevitably increase (negative). This dual negative perspective leads to resignation and apathy, certainly not a favorable situation. But what if you believe you can change the college's direction (positive), and that the college can cut certain costs in order to offset the need for higher tuition (positive). Now you are likely to engage the college community in productive discussions, and this may lead to personal satisfaction (Bandura, 1997). In the first scenario, you are not likely to do anything, in the second scenario you will most likely be highly motivated to act, even energized as you work toward productive changes. Of course, there are two other possible scenarios. You may believe there is nothing you can do (negative), but that change is possible (positive). In this case, you are likely to devalue yourself, perhaps feeling depressed about your own inability to accomplish good. Conversely, you may believe there is something you can do (positive), but that external forces will make change difficult or impossible (negative). This may lead some people to challenge the system in spite of their lack of expected change, resulting in protests and other forms of social activism (Bandura, 1997). Since all of these scenarios are based on beliefs and expectations, not on the unknown eventual outcome that will occur, it becomes clear that what we think about our ability to perform in various situations, as well as our actual expectations of the consequences of those actions, has both complex and profound effects on our motivation to engage in a particular behavior or course of action.

As for self-efficacy and self-esteem, these terms are often used interchangeably, and on the surface that might seem appropriate. Wouldn't we feel good about ourselves if we believed in our abilities to achieve our goals? In fact, self-efficacy and self-esteem are entirely different:

…There is no fixed relationship between beliefs about one's capabilities and whether one likes or dislikes oneself. Individuals may judge themselves hopelessly inefficacious in a given activity without suffering any loss of self-esteem whatsoever, because they do not invest their self-worth in that activity. (pg. 11; Bandura, 1997)

For example, my family was active in the Korean martial art Taekwondo. Taekwondo emphasizes powerful kicks. Because I suffer from degenerative joint disease in both hips, there are certain kicks I simply can't do, and I don't do any of the kicks particularly well. But I accept that, and focus my attention on areas where I am successful, such as forms and

helping to teach the white belt class. Likewise, Bandura notes that his complete inefficacy in ballroom dancing does not lead him into bouts of self-devaluation (Bandura, 1997). So, though it may improve our self-esteem to have realistic feelings of self-efficacy in challenging situations, there is not necessarily any corresponding loss of self-esteem when we acknowledge our weaknesses. And even positive self-efficacy might not lead to higher self-esteem when a task is simple or unpleasant. To cite Bandura's example, someone might be very good at evicting people from their homes when they can't pay their rent or mortgage, but that skill might not lead to positive feelings of self-esteem. This concept was the basis for the classic story A Christmas Carol, featuring the character Ebenezer Scrooge (Charles Dickens, 1843/1994).

The Development of Self-Efficacy

Young children have little understanding of what they can and cannot do, so the development of realistic self-efficacy is a very important process:

…Very young children lack knowledge of their own capabilities and the demands and potential hazards of different courses of action. They would repeatedly get themselves into dangerous predicaments were it not for the guidance of others. They can climb to high places, wander into rivers or deep pools, and wield sharp knives before they develop the necessary skills for managing such situations safely…Adult watchfulness and guidance see young children through this early formative period until they gain sufficient knowledge of what they can do and what different situations require in the way of skills. (pg. 414; Bandura, 1986)

During infancy, the development of perceived causal efficacy, in other words the perception that one has affected the world by one's own actions, appears to be an important aspect of developing a sense of self. As the infant interacts with its environment, the infant is able to cause predictable events, such as the sound that accompanies shaking a rattle. The understanding that one's own actions can influence the environment is something Bandura refers to as **personal agency**, the ability to act as an agent of change in one's own world. The infant also begins to experience that certain events affect models differently than the child. For example, if a model touches a hot stove it does not hurt the infant, so the infant begins to recognize their uniqueness, their actual existence as an individual. During this period, interactions with the physical environment may be more important than social interactions, since the physical environment is more predictable, and therefore easier to learn about (Bandura, 1986, 1997). Quickly, however, social interaction becomes highly influential.

Not only does the child learn a great deal from the family, but as they grow peers become increasingly important. As the child's world expands, peers bring with them a broadening of self-efficacy experiences. This can have both positive and negative consequences. Peers who are most experienced and competent can become important models of behavior. However, if a child perceives themselves as socially inefficacious, but does develop self-efficacy in coercive, aggressive behavior, then that child is likely to become a bully. In the midst of this effort to learn socially acceptable behavior, most children also begin attending school, where the primary focus is on the development of cognitive efficacy. For many children, unfortunately, the academic environment of school is a challenge. Children quickly learn to rank themselves (grades help, both good and bad), and children who do poorly can lose the sense of self-efficacy that is necessary for continued effort at school. According to Bandura,

it is important that educational practices focus not only on the content they provide, but also on what they do to children's beliefs about their abilities (Bandura, 1986, 1997).

As children continue through adolescence toward adulthood, they need to assume responsibility for themselves in all aspects of life. They must master many new skills, and a sense of confidence in working toward the future is dependent on a developing sense of self-efficacy supported by past experiences of mastery. In adulthood, a healthy and realistic sense of self-efficacy provides the motivation necessary to pursue success in one's life. Poorly equipped adults, wracked with self-doubt, often find life stressful and depressing. Even psychologically healthy adults must eventually face the realities of aging, and the inevitable decline in physical status. There is little evidence, however, for significant declines in mental states until very advanced old age. In cultures that admire youth, there may well be a tendency for the aged to lose their sense of self-efficacy and begin an inexorable decline toward death. But in societies that promote self-growth throughout life, and who admire elders for their wisdom and experience, there is potential for aged individuals to continue living productive and self-fulfilling lives (Bandura, 1986, 1997).

Discussion Question: Self-efficacy refers to our beliefs regarding our actual abilities, and self-esteem refers to how we feel about ourselves. What are you good at? Do others agree that you are good at that skill? When you find yourself trying to do something that you are NOT good at, does it disappoint you (i.e., lower your self-esteem)?

Behavior Modification

In Principles of Behavior Modification (Bandura, 1969), Bandura suggests that behavioral approaches to psychological change, whether in clinical settings or elsewhere, have a distinct advantage over many of the other theories that have arisen in psychology. Whereas psychological theories often arise first, become popular as approaches to psychotherapy, but then fail to withstand proper scientific validation, behavioral approaches have a long history of rigorous laboratory testing. Thus, behavioral techniques are often validated first, and then prove to be applicable in clinical settings. Indeed, behavioral and cognitive approaches to psychotherapy are typically well respected amongst psychotherapists (though some might consider their range somewhat limited).

Bandura made several points regarding the application of social learning theory to behaviorally-oriented psychotherapy. For example, Bandura notes that the labeling of psychological disorders, indeed the definition of what constitutes abnormal behavior, is made within a social context. While it has been demonstrated that common categories of mental illness are seen throughout a wide variety of cultures (Murphy, 1976), we still view those with psychological disorders based on sociocultural norms and, in the case of too many observers, with unreasonable prejudice. Bandura also opposed the medical model of categorizing and treating psychopathology, believing that the desire to identify and utilize medications has hindered the advancement of applying appropriate psychotherapies. The application of an appropriate therapy involves issues of ethical concern and goal-setting. Therapy cannot be successful, according to Bandura, if it does not have clear goals characterized in terms of observable behaviors. Choosing goals means that one must make value judgments. In making these decisions it is important that the client and the therapist share similar values (or at least that the therapist work with values appropriate for their client), and that the therapist does not try to impose their own values on the client (Bandura, 1969).

Overall, Bandura presents behavioral approaches to psychotherapy as non-judgmental applications of learning principles to problematic behavior, behavior that is not to be viewed as psychological “illness:”

…From a social-learning perspective, behaviors that may be detrimental to the individual or that depart widely from accepted social and ethical norms are considered not as manifestations of an underlying pathology but as ways, which the person has learned, of coping with environmental and self-imposed demands. (pg. 62; Bandura, 1969)

Cognitive Aspects of Social Learning Theory: The Contributions of Julian Rotter and Walter Mischel

Julian Rotter deserves at least as much credit as Albert Bandura for the establishment of social learning theory. Indeed, his book Social Learning & Clinical Psychology (Rotter, 1954) was published five years before Bandura’s Adolescent Aggression (Bandura & Walters, 1959). In addition, Rotter always focused on cognitive aspects of social learning, something Bandura gave more consideration to only later in his career. But their careers were by no means separated from one another. Walter Mischel was Rotter’s graduate student, and later joined the faculty of Stanford University where he was a colleague of Bandura. Mischel and Bandura collaborated on some of Mischel’s best known research: delayed gratification.

Brief Biographies of Julian Rotter and Walter Mischel

Julian Rotter was born in 1916 in Brooklyn, NY. The son of successful Jewish immigrants, his childhood was quite comfortable. During the Great Depression, however, the family business failed, and for a few years the family struggled (as many people did). This time of struggle instilled in Rotter a profound sense of social justice, as well as an interest in the effects of situational environments.

As a child Rotter was an avid reader, and eventually he read most of the novels in the local library. He then turned to reading books on psychology, taking a particular interest in works by Freud and Adler. During his senior year in high school he was interpreting people’s dreams and he wrote a paper based on Freud’s Psychopathology of Everyday Life (Freud, 1904/1995). He attended Brooklyn College, but chose to major in chemistry instead of psychology, as it seemed more likely to provide a promising career. During college, however, he learned that Adler was teaching at the Long Island College of Medicine. He began attending Adler’s seminars, became Adler’s friend, and was invited to meetings of the Society for Individual Psychology. Another well-known psychology professor who influenced Rotter was Solomon Asch. When he graduated from Brooklyn College, he actually had more credits in psychology than in chemistry.

Rotter attended the University of Iowa, where he earned a Master’s Degree in 1938, and then took a clinical internship at the Worcester State Hospital in Massachusetts. A year later he began working on his Ph.D. at Indiana University, because a professor there, C. M Louttit, had published one of the first books advocating clinical psychology as a career. Rotter received his Ph.D. in Clinical Psychology in 1941. After a short period of time at Norwich State Hospital in Connecticut, Rotter was drafted into the Army. He spent World War II working as a military psychologist. After the war he briefly returned to Norwich, but soon Rotter accepted a position at Ohio State University.

It was during his time at Ohio State University that Rotter developed his ideas on social learning theory. He and George Kelly were the two most prominent members of the psychology department, each of them having a lasting influence in the fields of social and cognitive learning theory. Rotter attracted many excellent graduate students, including Walter Mischel. Rotter was also keenly interested in the training of clinical psychologists, and he helped to outline the training model that became the basis for how doctoral level clinical psychologists are trained today.

As much as he enjoyed his time at Ohio State University, Rotter left in 1963 to direct the rebuilding of the clinical psychology training program at the University of Connecticut. He retired as professor emeritus in 1987. One year later he received an American Psychological Association's Distinguished Scientific Contribution Award, and one year after that he was recognized by the Council of University Directors of Clinical Psychology with their Distinguished Contribution to Clinical Training Award. He has also worked with the Peace Corps. Rotter included a brief autobiography in his self-edited compendium entitled The Development and Applications of Social Learning Theory(Rotter, 1982).

Walter Mischel was born in 1930, into a comfortable home, where he enjoyed a pleasant childhood. They lived in Vienna, a short distance from Sigmund Freud's house. However, when the Nazis invaded Austria at the beginning of World War II, the Mischel family moved to the United States, eventually settling in New York City. In college, Mischel studied to become a social worker. While working as a social worker in the Lower East Side slums, Mischel attended City College of New York and pursued a graduate degree in clinical psychology. He had been taught that Freud's theory offered the best explanation of human behavior, but he did not find this to be true in his work with juvenile delinquents (the same practical conclusion occurred to Carl Rogers in his first clinical position).

He then attended Ohio State University, where he was a graduate student of both Julian Rotter and George Kelly. Rotter and Kelly helped to firmly establish Mischel as a member of the general social learning/cognitive learning camp, and later Mischel became a faculty member at Stanford University, alongside Albert Bandura (from 1962-1983). He then returned to New York as a faculty member at Columbia University, where he continued his work on delayed gratification and the effects of situations on personal behavior. Mischel has been recognized with a Distinguished Scientific Contribution Award by the American Psychological Association.

Placing Rotter and Mischel in Context: Cognitive Social Learning Theory

The distinction between Bandura as a social learning theorist, and Rotter and Mischel as cognitive learning theorists, is not entirely accurate. As Bandura's career progressed, he focused more and more on cognitive factors, and Mischel collaborated with Bandura while both were at Stanford University. What distinguishes Rotter and Mischel is that cognitive factors were always the most important aspect of their learning theories. Although humans are capable of learning simply by watching a model, their expectations regarding the outcome of a situation, and the value the place on the potential reward (or punishment), determines their course of action. According to Mischel, these variables can lead to seemingly inconsistent behavior, but when examined in closer detail individuals demonstrate consistent patterns of variation, a form of consistency in itself.

Rotter and Mischel can also be seen as having encompassed Bandura's career. Although all three men were active during the same general time frame, Rotter's first book on social learning theory preceded Bandura's first book by 5 years. Mischel, a student of Rotter, and then a colleague of Bandura for a while, has continued to modify his most influential theory quite recently, in the 1990s and 2000s.

Rotter and Mischel can also be considered as providing a bridge between the more traditional social learning theory of Bandura and the full-fledged cognitive theory of George Kelly. Kelly was Rotter's colleague at Ohio State University, and Mischel studied under both men while in graduate school. Thus, social learning theory, cognitive social learning theory, and cognitive theories of personality development all occurred in close relationship to one another, and they all offered a dramatic alternative to radical behaviorism, an alternative that helped to fulfill the vision of John Dollard and Neal Miller.

Basic Constructs in Rotter's Social Learning Theory

Rotter's early research focused on the need to understand human behavior and personality so that clinical psychologists might effectively help their patients. In the preface to Social Learning & Clinical Psychology, Rotter wrote:

…the practice of clinical psychology in many instances is unsystematic and confused when viewed from logical or rigorous scientific viewpoints. This confusion, however, is not a necessary condition but the result of the failure of the clinical psychologists' training program to translate and relate the basic knowledge of experimental and theoretical psychology into the practical situations of the clinic, the hospital, and the school… (pg. viii; Rotter, 1954)

Given his emphasis on clinical psychology, Rotter focused on the clinician's ability to predict behavior. According to Rotter, social learning theory assumes that the unit of investigation for the study of personality is the interaction between the individual and their meaningful environment. Although personality has unity, the individual's experiences influence each other. As a result, personality is continuously changing, since each person is always having new experiences. However, personality is also stable in some respects, since previous experiences influence new learning. Given the complexity of each individual, Rotter believed that in order to make reasonable predictions about behavior it was necessary to examine four kinds of variables: **behavior potential**, **expectancy**, **reinforcement value**, and the **psychological situation** (Rotter, 1954, 1964, 1972; Rotter & Hochreich, 1975).

Behavior potential refers to the likelihood of a certain behavior occurring in the context of specific potential reinforcement. For example, in order to earn good grades a student can rely on any number of possible behaviors, such as studying, cheating, skipping class to avoid a bad grade, etc. Each potential behavior can only be described as more or less likely than other potential behaviors, and included as potential behaviors are psychological reactions such as thoughts, emotions, and even defense mechanisms. Expectancy is defined as the probability held by the individual that reinforcement will follow one's chosen behavior. Although Rotter preferred to avoid the concept that expectancy is subjective, he acknowledged that an element of subjectivity is involved. Regardless, it is the individual's point of view, their expectations in a given situation, that are more important for predicting behavior than the realistic probability of a chosen behavior resulting in an expected reinforcement. Reinforcement value, quite simply, refers to the preference for a given

reinforcer. To use Rotter's own example, most people would consistently choose to be paid $10 dollars an hour rather than $1 an hour, if it were simply their choice. Finally, there is the psychological situation. According to Rotter, it is not enough to say that to each individual a given situation might seem different. In order to address the situation in more objective terms, psychologists need to identify a variety of cues within the situation. In an objective sense, consequently, different people can be described as attending to different specific cues in the environment (Rotter, 1954, 1972; Rotter & Hochreich, 1975).

Although Rotter broke new ground in this approach to the study of social learning theory, he did not entirely abandon the use of mathematical formulae similar to those of Dollard and Miller. Rotter proposed the following basic formula for predicting goal-directed behavior:

BP_x,S1Ra = /(E_x,RaS1 & RV_a,S1)

Although this formula appears complicated at first glance, it is relatively straightforward. The potential for behavior x (BPx) to occur in situation 1 with potential reinforcement a (S1Ra) is a function (/) of the expectancy (E) that reinforcement a will follow behavior x in situation 1 (x,RaS1) and the reinforcement value (RV) of reinforcement a in situation 1 (a,S1) (Rotter, 1954; Rotter & Hochreich, 1975). In other words, we are most likely to choose the behavioral option that we realistically expect will result in the most favorable outcome in our current situation.

Discussion Question: Rotter believed that both the expectancy of reward and the perceived value of that reward were essential in determining whether an individual engaged in a particular behavior. Have you ever found yourself doing something even though you did not expect to get anything for your efforts? Have you ever had a job where you felt that you weren't being paid what you deserved? In such situations, how long did you continue your behavior, and how did you feel about it?

Locus of Control

One of the most important generalized expectancies underlying behavior, and perhaps Rotter's best known concept, is referred to as **internal versus external control of reinforcement** (commonly known as **locus of control**):

People are known to differ in their belief that what happens to them is the result of their own behaviors and attributes (internal control) versus the result of luck, fate, chance, or powerful others (external control). Clearly, persons who believe or expect that they can control their own destinies will behave differently, in many situations, than those who expect that their outcomes are controlled by other people or determined by luck. (pg. 105; Rotter & Hochreich, 1975)

Rotter pointed out that almost all psychologists recognize the role that reinforcement or reward plays in determining future behavior, but that this is not a "simple stamping-in process." For beings as complex as humans, the effects of reinforcement depend upon an individual's perception of a causal relationship between their behavior and the potential reward (Rotter & Hochreich, 1975).

A number of scales have been developed to measure locus of control (for an early review see Lefcourt, 1976), including one developed by Rotter himself (Rotter, 1966). Rotter's scale, simply referred to as the **I-E scale** (for internal-external), consists of 29 forced-choice statements. For example:

1.a. Children get into trouble because their parents punish them too much.

1.b. The trouble with most children nowadays is that their parents are too

easy with them.

In each instance, the person taking the test must choose one or the other option. After taking all 29, the person's score is the total number of external choices. Does it seem difficult to determine whether 1.a. or 1.b. is the external choice? Good! Question 1 is actually a filler question, designed to interfere with the test taker's ability to understand what the test is about! So, consider question 2:

2.a. Many of the unhappy things in people's lives are partly due to

bad luck.

2.b. People's misfortunes result from the mistakes they make.

For question 2 it is quite obvious that choice a is the external choice, and if it wasn't clear, the test has choice a marked for you! There are a total of six filler questions, leaving the test itself with 23 choices (Rotter, 1966).

Locus of control appears to arise from two primary sources: the family, and **contingency awareness** (Lefcourt, 1976). The role of the family in the development of locus of control is complex, and appears to be somewhat different based on the behavior of mothers and fathers. The most reliable finding appears to be that individuals with an internal locus of control had mothers who pushed them to achieve independence at an early age. This motherly push, however, must be a careful one. Children need support, guidance and nurturance, but they must not be smothered to the point of being pampered. Lefcourt (1976) cites Adler's concern regarding two extremes in child-rearing, pampering and neglect, neither of which is conducive to the healthy psychological growth of a child. Contingency awareness refers to an understanding of instrumentality, the conception that one's actions are indeed related to certain outcomes. In order for a child to repeat a behavior with purpose, the child must be able to recall that their prior actions resulted in a given outcome, and they must know that their actions were related to the expected outcomes. It would appear that children as young as two months old are capable of this type of social learning, and it tends to result in positive emotional reactions (Lefcourt, 1976).

Early studies on locus of control also focused on some interesting cultural questions. It is generally accepted that social class and ethnic group are important determinants of personality. Battle and Rotter (1963) found that lower class Blacks were significantly more external in their locus of control than were middle class Whites. Interestingly, middle class Blacks were closer to middle class Whites than lower class Whites were to middle class Whites, suggesting that social class may have been the primary factor in these results, rather than the race or ethnicity of the subjects. Furthermore, IQ seems to have exacerbated these results in that the most external individuals were high IQ lower class Blacks (i.e., individuals aware of social injustice in American society) and the most internal individuals were low IQ middle class Whites (who may be blaming themselves for failing to live up to their expected potential; Battle & Rotter, 1963). During the civil rights movement, Gore & Rotter (1963) examined whether locus of control might be a useful measure of social action. They found that students at a southern Black college who expressed interest in attending a civil rights rally or marching on the state capitol scored significantly more internal on the I-E

scale. In other words, those who believed they could personally make a difference were more willing to try making that difference. In a study that followed soon after, Strickland (1965) compared Blacks who were indeed active in the civil rights movement to those who were not (but who were matched for sex, age, education, etc.). As predicted, the individuals who were active in the civil rights movement scored significantly more internal on the I-E scale than those who were not active. Strickland did note, however, that the individuals she studied were pioneers in the civil rights movement, and had become active, in part, because others groups had failed to demonstrate an adequate degree of commitment to the civil rights movement. Strickland's concern seems to contradict earlier results of Mischel (1958a), who found that when individual's make public commitments, they are less likely to change their expectancies (i.e., individuals publicly involved in the civil rights movement should have remained committed to the cause even when faced with initial failure). Still, as Mischel himself noted, one cannot rely entirely on inferences from research when considering the complexities of real-life (and, at the time, dangerous) behavior.

Discussion Question: Do you consider yourself to have an internal or an external locus of control? Do you feel that locus of control is an important influence on personality; might it be good or bad?

Rotter's Emphasis on Clinical Psychology

As noted above, Rotter was actively involved in developing the model that provided the basis for how clinical psychologists are typically trained today. Accordingly, much of Rotter's career was devoted to clinical applications of his work, In addition to writing two books that emphasized clinical psychology (Rotter, 1954, 1964) and developing the I-E scale (Rotter, 1966), Rotter and one of his research assistants published The Rotter Incomplete Sentences Blank: College Form (Rotter and Rafferty, 1950). The book was intended to formalize the **sentence completion method**, particularly for use with college students. The test consists of forty simple statements that require the subject to finish the sentence. For example, one beginning is simply "My father…" The subjects responses are then scored in terms of whether they demonstrate conflict (on a scale of 1-3), are neutral, or whether they are positive (also on a scale of 1-3). The manual offers examples of possible answers for both males and females. For example, conflicted responses for males include breaking promises or being a fool (level 3), or never had much of a chance or is proud (level 1). A neutral response might simply be that the father is a salesman, or is a hard worker. Positive responses for females include that the father is quite a character or is a good man (level 1), or that he has a great sense of humor or is a lot of fun (level 3). Interpreting this test requires a great deal of experience, and an understanding of personality and human nature. Fortunately, Rotter and Rafferty include a number of individual cases as examples of how the **Rotter Incomplete Sentences Blank** can be used to evaluate individuals. Both the Rotter Incomplete Sentences Blank and the I-E scale have proven useful in evaluating patients, as well as normal individuals, in a variety of settings and cultures, including Africa, Sri Lanka, American Indians, Brazil, Black and White college students in America, Ukrainian doctors training in Canada, and amongst military personnel (Janzen, Paterson, Reid, & Everall, 1996; Lefcourt, 1976; Logan & Waehler, 2001; Nagelschmidt & Jakob, 1977; Niles, 1981; Picano, Roland, Rollins, & Williams, 2002; Rossier, Dahourou, & McCrae, 2005; Rotter, 1960, 1966; Trimble & Richardson, 1982). In a particularly interesting study, a unique version of the Sentence Completion Test was developed by Herbert Phillips and provided the basis for a major study on the personality of Thai peasants living in the village

of Bang Chan, Thailand (Phillips, 1965). The Rotter Incomplete Sentences Blank, and other variations of the sentence completion method, remain very popular today (Holaday, Smith, & Sherry, 2000), ranking with the Rorschach Inkblot Test and the Thematic Apperception Test as the most popular projective tests for personality assessment.

Overall, Rotter emphasized the value of training clinical psychologists for just that responsibility, with a particular emphasis on the realities that will face the psychologist in an actual clinical setting (Rotter, 1954, 1964). In 1972, Rotter edited a volume including both original and previously published papers in which social learning theory was applied to psychopathology in general (Phares, 1972) and to such diverse topics as drinking amongst college students, excessively needy individuals, working with mentally retarded children, and electroconvulsive shock therapy (Cromwell, 1972; Dies, 1968; Jessor, Carman, & Grossman, 1968; Jessor, Liverant, & Opochinsky, 1963). A particularly important aspect of therapy also addressed in this volume is the issue of terminating therapy. Strickland & Crowne (1963) found that defensiveness and avoiding self-criticism are common signs in individuals who are likely to end therapy abruptly, whereas Piper, Wogan, & Getter (1972) found that the patient's expectancy regarding improvement, and the value they place on improving, are useful predictors of terminating therapy. Although helping patient's to achieve a level of psychological health that allows terminating therapy should be the goal of every therapist, premature termination might prove even more detrimental to the patient. For Rotter, the proper training of clinical psychologists is not an easy task. In the preface to Clinical Psychology, Rotter wrote:

…Yet psychology itself is a relatively new science and its areas of application are in rapid transition. Neither theory nor "facts" are always agreed upon, and in clinical psychology there is no single set of orthodox, approved skills for which a person can be certified as a trained practitioner…The goal is to gain comprehension without resorting to an oversimplification of the complex nature of man or of the problem of understanding him. (pg. xi; Rotter, 1964).

Behavioral Specificity and Consistency

In 1968, Walter Mischel challenged both state and trait theories of personality. Psychological states typically fall with the domain of psychodynamic theory, whereas trait theories are a perspective unto themselves. According to Mischel (1968), although state and trait theorists use very different language, they tend to approach personality in the same general way: they use responses to infer pervasive, underlying mental structures that exert enduring causal effects on behavior. Thus, both state and trait theorists emphasize consistency in behavior. However, there is a wealth of data that individuals do not act consistently from situation to situation. Instead, Mischel argues, behavior can best be predicted only when one takes into account the specific situation in which the behavior occurs:

Progress in the area of personality psychology and assessment has been hindered by the failure to apply relevant principles about the conditions that produce, maintain, and modify social behavior. The principles that emerge from basic research too often have not been seen as directly relevant to the understanding of the determinants of test responses in the clinic or the assessment project. It is as if we live in two independent worlds: The abstractions and artificial situations of the laboratory and the realities of life. (pg. 1; Mischel, 1968).

In order to support his argument, Mischel examined which aspects of behavior are or are not consistent. Generally, intellect is consistent, including academic ability, achievement, and cognitive style. In contrast, there is little evidence to support consistency of behavior across situations when examining personality variables such as attitudes, moral behavior, sexual identification, dependency, aggression, tolerance, conditionability, etc. (Mischel, 1968). How, then, might we predict behavior? Mischel suggests a dynamic perspective on how persons interact with their situations. If the environment has not changed much, we can expect past behavior to be a reasonable predictor of current behavior (and state and trait theories would seem to hold true as well). However, if the environment changes dramatically, the individual may act in unpredictable ways. In addition, the individual may begin to learn new social conditions, thus allowing for considerable change in behavior over time:

Global traits and states are excessively crude, gross units to encompass adequately the extraordinary complexity and subtlety of the discriminations that people constantly make. Traditional trait-state conceptions of man have depicted him as victimized by his infantile history, as possessed by unchanging rigid trait attributes, and as driven inexorably by unconscious irrational forces…A more adequate conceptualization must take full account of man’s extraordinary adaptiveness and capacities for discrimination, awareness, and self-regulation…and that an understanding of how humans can constructively modify their behavior in systematic ways is the core of a truly dynamic personality psychology. (pg. 301; Mischel, 1968)

Delayed Gratification

Perhaps Mischel’s most famous contribution to psychology is his research on **delayed gratification**. In a series of studies, begun in the late 1950s, Mischel examined the conditions under which children choose immediate gratification or whether they can delay gratification in order to obtain a larger reinforcer at a later time. The ability to delay gratification, according to Mischel, is essential for the development of self-control. From early childhood throughout the lifespan, achieving long-term goals often requires setting aside tempting distractions. Conversely, many personal and social problems result from failures of self-control, such as dropping out of school, poor job performance, and even violent and criminal behavior (Mischel & Mischel, 1980). In an amazing longitudinal study, Mischel and his colleagues offered 4 year-old children the opportunity to grab a marshmallow. But, if the child could wait until the researcher ran an errand, the child could then have two marshmallows! Some children grabbed the marshmallow as soon as the experimenter left, but others were able to wait 15-20 minutes. It was not easy, however. The children who waited demonstrated a variety of behaviors to distract themselves from the marshmallow: they would play, sing, cover their eyes so they didn’t have to look at the marshmallow, etc. The most striking results from this study were actually obtained years later. Mischel and his colleagues tracked down the former 4 year-old subjects as they were graduating from high school. The individuals who had delayed gratification as 4 year-olds were significantly more personally effective and self-assertive, and they were better able to cope with life’s frustrations (Mischel, Shoda, & Rodriguez, 1989; Shoda, Mischel, & Peake, 1990). In addition, the 4 year-old children who had been able to delay gratification were more successful as students in a variety of ways, including eventually earning significantly higher SAT scores (210 points higher, on the combined score), and the ability to delay

gratification proved to be a better predictor of SAT scores than IQ (Peake, cited in Goleman, 1994).

Although the famous marshmallow-grabbing study was conducted at a preschool on the campus of Stanford University, Mischel began this research with very different groups: Black and East Indian children on the islands of Trinidad and Grenada (Mischel, 1958b, 1961). On these relatively poor, Caribbean islands, Mischel not only compared the Black and East Indian children, he also compared the children of Trinidad to the children of Grenada. The main purpose of the second study, however, was to examine the effect of fathers being absent from the home on the preference of children for immediate or delayed gratification. Overall, when fathers are absent from the home, both young boys and young girls (ages 8 to 9 years old) demonstrated a preference for immediate gratification. Mischel suggests that the inability to delay gratification amongst children who lack a father may be related to immaturity or poor psychological adjustment (Mischel, 1961).

While Mischel was at Stanford University, he also collaborated with Bandura. Blending the interests of both men, they examined whether observing models would affect children's choices regarding immediate vs. delayed gratification. They identified two groups of children (both boys and girls) as preferring either immediate gratification (a small candy bar now) or delayed gratification (a larger candy bar later). The children were then exposed to either a live model choosing the alternative strategy, a symbolic model (a description of an adult choosing the alternative), or no model. As expected, exposure to a model choosing the alternative strategy dramatically affected the behavior of the children, and a live model was more effective than the symbolic model. The effects of this modeling appeared to be quite persistent (Bandura & Mischel, 1965). Considering the importance that modeling can play in developing the ability to delay gratification, it is perhaps easy to see why children in families lacking a complete and stable family structure don't develop self-control as well as other children.

Discussion Question: Mischel's most famous contribution is the concept of delayed gratification. How good are you at waiting for gratification? Are some rewards easier to wait for than others? If you know anyone who is significantly different than you, either wanting immediate gratification or being able to delay it without much trouble, does the difference between you create any problems or interesting situations?

The Cognitive-Affective Processing System (CAPS)

More recently, Mischel has turned his attention to solving what has been called the "**personality paradox**:" How do we reconcile our intuition and theories that personality is relatively stable with the overwhelming evidence that personality varies across different situations? Mischel proposes a dynamic personality system that takes into account both: (1) the behavioral consistency that accounts for specific scores on trait tests and indicates what the individual is like in general; and (2) the consistency in how an individual varies across different situations. This consistency of variation is recognized by distinct patterns of if…then… relationships, which are characteristic of the individual's overall personality (Mischel, 2004).

In 1995, Mischel and Shoda first presented this dynamic approach to understanding personality, referring to it then as the cognitive-affective personality system, but now preferring the term **cognitive-affective processing system** (CAPS; Mischel, 2004; Mischel

& Shoda, 1995/2000; Shoda, Leetiernan, & Mischel, 2002). Over a number of years, Mischel, his students, and his colleagues studied children extensively in a residential summer camp. They observed both behaviors and the situations in which they occurred. Over time, they were able to identify patterns of if…then… situation-behavior relations that reflected distinctive and stable characteristics of each child's behavior organization. These observations, therefore, gave rise to **situation-behavior profiles** for each child. It is essential to recognize, however, that the term "situation" in these studies does not refer to simple environmental stimuli, as they might for a behaviorist such as B. F. Skinner. Instead, these situations activate a whole set of internal reactions, including cognitive and emotional elements. They are also not limited to the external world; they can be generated in thought, fantasy, planning, etc. Accordingly, Mischel and Shoda referred to these personality variables as **cognitive-affective units** (or CAUs). These CAUs include encodings, expectancies and beliefs, affects, goals and values, and competencies and self-regulatory plans.

Mischel and Shoda (1995/2000) did not neglect the individual's development in this theory. Our ability to recognize distinct aspects of the environment are influenced by genetic/biological factors, cultural factors, and the interactions between them. These genetic/biological/cultural factors also influence the CAPS, as does our social learning history. In a sense, bringing all of these factors together begins to move us beyond the person-situation debate, since both sides of the debate are correct in the proper context. The future of personality theory may lie in an as yet undetermined synthesis of these perspectives (Fleeson, 2004). For now, according to Mischel, this dynamic approach to understanding personality has at least helped to bring together the major aspects of different schools of personality theory:

The two goals - dispositions and dynamics - that have so long been pursued separately do not require two fields from this perspective. In this theory, dispositions are conceptualized not in semantic terms but as processing structures characterized by stable cognitive affective organizations in the processing system that becomes activated when the individual encounters relevant situational features. (pg. 170; Mischel & Shoda, 1995/2000)

The Impact of Social Learning Theory

It would be difficult to overestimate the impact of social learning theory on psychology, because the human species is so inherently social. Social life seems to come automatically, mediated via mental processes that are largely unconscious (Bargh & Williams, 2006), and our social norms appear to arise from social behavior that is adaptive within local ecologies (Kameda, Takezawa, & Hastie, 2005). It is important to note, however, that social organization is by no means unique to the human species. There are many animal species that live in social groups, some demonstrating a surprising degree of intelligence, suggesting that social living itself may have helped to foster the development of intelligence (Pennisi, 2006). Further evidence for the impact of social learning theory on psychology can be found in the simple name recognition enjoyed by Bandura, certainly one of the most famous psychologists.

There are also interesting lines of research within the field of neuroscience that provide support for Mischel and Shoda's cognitive-affective processing system. Utilizing functional magnetic resonance imaging (fMRI), Decety & Jackson (2006) have found that empathy appears to involve activation of the same brain regions involved in experiencing the situation

about which one is feeling empathy toward another person. For example, there is significant activation of brain regions involved in pain when an individual views pictures of someone else in a clearly painful situation. This would seem to provide neurobiological evidence in support of Mischel and Shoda's cognitive-affective units, the functional components of the cognitive-affective processing system. Similarly, Knoblich & Sebanz (2006) have demonstrated that perceptual, cognitive, and motor processes are enhanced by social interaction, and that such interactions can be measured using event-related potentials that measure brain electrical activity. Some of the data presented in the study by Knoblich & Sebanz are essentially situation-behavior profiles for the individuals in their study.

Finally, let's address the role of expectancy in one of the most challenging social issues facing the world today: diversity. We hear more and more about the value of diversity in higher education and in the workplace, but pursuing diversity is often challenged by prejudice. Expectations of prejudice enhance attention to social cues that threaten one's social identity. In other words, when individuals expect that engaging in diversity will lead to prejudice, and perhaps then to discriminatory behavior, they are more likely to notice evidence of that very outcome (Kaiser, Vick, & Major, 2006). In addition, contact between diverse groups does less to displace feelings of prejudice among members of minority groups than it does among members of the majority group (likely due to the minority group members' recognition of the ongoing effects of prejudice and discrimination; Tropp & Pettigrew, 2005). These represent difficult situation-behavior circumstances, since it can obviously be very difficult for team members to predict the behavioral responses likely to follow the artificial establishment of diversity. In making recommendations to the leaders of organizational teams, Mannix & Neale (2005) suggest clearly defining the team's tasks and goals, providing bridges across diversity, and enhancing the influence of the minority. Perhaps most importantly, there is a need to provide incentives for change. Taken together, these approaches both increase the expectancy of success and raise the reinforcement value of working toward successfully diversifying the team. As such, principles that have arisen from social learning theory can clearly play a positive role is reshaping society.

The Future of Psychology: Bandura's Vision as Compared to Freud

In Toward a Psychology of Human Agency, Bandura (2006) agrees with Freud that religion played a significant role in the advancement of civilization, and that the scientific revolution that began with Darwin's Theory of Evolution shifted our focus toward natural and scientific approaches to understanding and improving human life. However, evolution emphasizes random effects on genetically determined structures and traits, and does not allow for the choices made by individuals, though sociobiologists argue that genetics can exert influences on those choices. Rather than relying on psychoanalysis in order to understand the impulses of the id that drive us to survive, as Freud proposed (1927/1961, 1930/1961), Bandura believes that people are agents of change in their own lives, and that they can choose the direction that change takes.

Being an agent involves intentionally influencing one's functioning and life circumstances, and there are four core properties of human agency. **Intentionality** refers to our ability to form action plans and the strategies necessary for accomplishing them. **Forethought** is the temporal extension of agency, in which we set goals for ourselves and anticipate likely outcomes of our actions to both guide and motivate our behavior. Once having chosen a course of action, of course, we do not simply sit back and wait for the right things to happen.

Agency also involves both **self-reactiveness** and **self-reflectiveness**, processes in which we regulate our behavior, monitor our courses of action, and examine whether we are capable of being successful in our various endeavors (Bandura, 2006). As much as human agency involves our own thoughts, goals, motivation, and expectations, however, we do not exist autonomously. All human behavior occurs within social structures, and there is a reciprocal interplay between personal, behavioral, and environmental determinants (reciprocal determinism). This means that human agency, the exercise of self-influence, is part of the causal structure of our lives. As Bandura points out, this is not “free will,” which would be a throwback to medieval theology, but rather a matter of acting as an agent, the role of an individual in making causal contributions to the course of events in their life (Bandura, 2006).

…Social cognitive theory rejects a duality of human agency and a disembodied social structure. Social systems are the product of human activity, and social systems, in turn, help to organize, guide, and regulate human affairs. However, in the dynamic interplay within the societal rule structures, there is considerable personal variation in the interpretation of, adoption of, enforcement of, circumvention of, and opposition to societal prescriptions and sanctions…freedom is conceived not just passively as the absence of constraints, but also proactively as the exercise of self-influence… (pg. 165; Bandura, 2006)

Personality Theory in Real Life: Media Violence

and Its Effects on Children

The second Bobo doll study conducted by Bandura and his colleagues presented videotapes of the models behaving aggressively toward the Bobo doll (Bandura, Ross, & Ross, 1963a). The observation in this study that children did, indeed, imitate the aggressive behavior of models seen on film, but not in person, has become one the most influential articles in history, both in terms of stimulating further research on the modeling of aggression and in terms of practical applications (e.g., ratings on television programs, video games, and CDs with controversial lyrics). In their introduction to an issue of Psychological Science in the Public Interest (PSPI) on media violence and its effects on children, Ceci and Bjork (2003) describe how the authors of this particular PSPI issue were a collection of experts brought together to prepare a portion of the U.S. Surgeon General’s report on youth violence. However, political concerns led the Surgeon General’s office to radically alter the report. The authors protested, and attempted to satisfy the concerns of the politicians. Ultimately, however, this portion of the report was dropped from the final version. So, the editors of PSPI chose to publish this valuable psychological information, which clearly causes political concerns for some, so that psychologists, psychiatrists, law enforcement officials, teachers and school administrators, government policy makers, parents, indeed anyone who has a vested interest in youth violence, might have the most current information available on the effects of media violence on America’s youth. I would also like to acknowledge an important point made by the authors themselves. This issue of PSPI was a collaborative project by eight experts on the effects of media violence. The authors decided that the fairest way to list their names was alphabetical order. Thus, as I refer to the article as Anderson, et al. (2003), please note that the “et al.” includes widely respected experts, chosen by the National Institute of Mental Health for this project, each of whom contributed significantly to this article.

It would be difficult to do justice to this report by summarizing it here, since the report, which is a review itself, is nearly thirty pages long and cites 245 research studies. Clearly, the attention drawn by this question, the role of media violence in affecting violent behavior among children, provides a testament to the significance of this research. Anderson, et al. (2003) provide an overview of the empirical research, offer theoretical explanations, they address moderating effects, media use and content, and they examine research on interventions.

Empirical research on media violence and aggression has covered of wide variety of media types, including dramatic television and movies, television news violence, music videos and their lyrics, video games, and the internet. The review of this extensive body of research has shown a statistically significant association between exposure to media violence and aggression and violence among youth. Exposure to media violence, and the findings are consistent across the various types of media, increases the likelihood that children have aggressive thoughts and, indeed, engage in aggressive behavior. Most importantly, longitudinal studies have consistently shown that exposure to media violence in childhood, even beginning in late adolescence, are predictors of increased aggression and violent behavior in adulthood. Although the effects are at best small to medium, the authors note that they are as high as other problems that are considered significant public health issues, such as cigarette smoking and exposure to asbestos.

Several theories have been put forth to explain the manner in which exposure to media violence increases aggression in children, not the least of which is Bandura's theory of observational learning. Observational learning appears to be so important to the humans, and other primate species, that we have developed a specific neurological system to learn from watching others: the **mirror-neuron system** (see Rizzolatti & Craighero, 2004). Additional theories suggest that exposure to media violence prepares an individual to engage in aggressive behavior. This can occur either because the media violence primes the individual by activating aggressive thoughts and scripts or by generally arousing the individual, and that arousal then carries over into heightened aggressive behavior in situations that provoke aggressive actions. Finally, there is the widely used, and often misunderstood, term "**desensitization**." Anderson et al. (2003) prefer the term **emotional desensitization** to refer to a reduction in distress-related reactions to observing or thinking about violence. Although emotional desensitization may result in an individual being more willing to engage in aggressive or violent behavior, there is no evidence that it stimulates aggression. Thus, although emotional desensitization may be related to the disinhibition discussed in the chapter (it may even be the same thing), its role remains unclear.

Not all children are affected by media violence in the same way, and not all examples of media violence are equally effective in enhancing aggression. Thus, there must be moderating factors that come into play in the relationship between exposure to media violence and subsequent aggressive behavior. For example, the effects of media violence decline as children grow older, media violence is less likely to affect children who are less aggressive to begin with, and, although there is not a direct effect of intelligence, children of lower intelligence are more likely to watch television and to be at risk for other factors enhancing aggressiveness. The way in which the violence is presented in the media is also important, as is any portrayed justification for the violence as well as its consequences. However, all of these relationships are complex, and have not been studied in great detail. Thus, there is much more work to be done. Finally, one of the most important moderating

factors is parental control. When parents discuss the content of media violence with their children, when they comment regularly on the reality of violence, children are much less likely to demonstrate aggressive tendencies.

This entire discussion would be moot, if not for the prevalence of media in our society and the content of that media. Data reported by Anderson, et al. (2003) from three national surveys provide an amazing view of media availability in America. Virtually all families with children have a television set, most have at least one VCR or DVD player, and about three quarters of those families subscribe to cable or satellite television. Approximately 70 percent of families with children have a video game system and a computer, and most American children have a television in their room (including 30 percent of children age 0 to 3 years old!). Watching television is the third most common activity for children, after sleeping and going to school. A significant number of children watch more than 40 hours of television a week, and children ages 0 to 6 years spend more time engaged in media entertainment than engaged in reading, being read to, and playing outside combined! And what's included in that media content? Plenty of violence: 61 percent of programs contain violence, and only 4 percent of programs contain an anti-violence theme:

…put in another way, 96% of all violent television programs use aggression as a narrative, cinematic device for simply entertaining the audience…Moreover, most aggression on television is glamorized and trivialized…and nearly 75% of all violent scenes feature no immediate punishment or condemnation for violence. (pg. 101; Anderson, et al., 2003)

Similar findings have been reported for violence in video games, both in America and in Japan. Parents seldom recognize the popularity of those violent games, and only about one third of parents even knew the name of their child's favorite video game (Anderson, et al., 2003).

Despite the wealth of information on the relationship between exposure to media violence and aggressive behavior, and a number of potential approaches for intervention, there has been little empirical research on potential interventions. In general, there are three approaches to reducing the influence of violent media: changing attitudes to counter violent messages, encouraging parental monitoring and guidance, and providing education on the content and effects of various forms of media. But once again, none of these potential interventions has been studied in detail. It must also be remembered that this is only one of many factors that contribute to aggressive and violent behavior amongst youth, though it may be the least expensive to address, since it involves little more than making choices about the media children are exposed to:

…However, the troubling truth is that violent media are entering the home and inviting active participation of even very young children - often with little parental supervision…Although additional research to address unresolved questions is needed, it is clear that media violence is a causal risk factor that should be addressed in thoughtful ways…parents can reduce and shape their children's consumption of violent media…from a public-health perspective, today's consumption patterns are far from optimal. And for many youths, they are clearly harmful. (pgs. 105-106; Anderson, et al., 2003)

Research on this important issue certainly did not end with the publication of the PSPI issue in 2003. In a more recent study, Carnagey and Anderson (2005) had college students play one of three versions of a race-car video game: one version rewarded all violence, one

punished all violence, and one was non-violent. Overall, rewarding violent actions in the game increased hostile emotion, aggressive thinking, and aggressive behavior. In contrast, punishing violence still increased hostile emotion, but did not increase aggressive thoughts or behaviors. The authors suggest that an important mechanism through which violent media increases aggressive behavior is to first increase aggressive thoughts and states of mind. Another interesting result in this study was that the non-violent game did not result in as much violence as the game in which violence was rewarded, suggesting that the violent behavior in the game was not simply the result of competitiveness by the players. Of course, not everyone responds aggressively when aggression is modeled. It appears that individuals who score high on the trait of agreeableness (one of the “Big Five” traits, see Chapter 13) are able to short-circuit the effects of aggression-related cues and curb their aggression (Meier, Robinson, & Wilkowski, 2006). Meier, et al. go on to suggest that teaching other people to associate aggression-related cues with prosocial behavior might become a valuable intervention in continued efforts to reduce aggression and violence in our society. First, however, it might prove important to reduce the number of weapons present in American society (or at least our relationship with them). Klinesmith, Kasser, and McAndrew (2006) recently demonstrated that when male college students were exposed to a gun there was an increase in both their testosterone levels and their aggressive behavior. Media violence is clearly not the only aggression-related cue present for most people. However, as noted above, it may well be one area in which a distinct reduction in such cues can easily be accomplished without substantial financial costs (but certainly with substantial benefits) to society.

Review of Key Points

• Reciprocal determinism refers to the concept that behavior, personal factors, and environmental factors are equal, interlocking determinants of each other.

• Observational learning is a specific type of social learning in which observers view the behavior of models.

• Highly aggressive children appear to learn this behavior at home, having experienced their parents modeling aggressive behavior.

• When models are rewarded for aggressive behavior, the result can be the disinhibition of aggression that had previously been restrained.

• Social learning is different than either simple imitation or identification, in that social learning implies underlying psychological processes (cognition).

• In order for social learning to occur, conditions must be met that support the components of this process: attention, retention, production, and motivation.

• Since observers do not copy behavior perfectly, and since they may choose to mix and match the behavior of different models, observational learning can lead to new and different behaviors.

• Self-regulation refers to the processes of self-reinforcement and self-punishment. Self-reinforcement works primarily through its motivational effects.

• Self-efficacy is the belief in one’s capabilities to perform specific behaviors in order to accomplish specific outcomes.

• Self-efficacy and self-esteem are separate concepts. An individual may lack a certain ability and be well aware of it, but if one’s concept of self-worth is not tied to that skill, there will be no corresponding loss of self-esteem.

• Bandura referred to the ability to act as an agent of change in one’s environment as personal agency.

• Bandura believed that behavioral approaches have an advantage over other methods of therapy because of their basis in rigorous, scientific testing.

• Behavioral therapies can only be successful if they focus on goals characterized by clear and observable behaviors.

• Rotter proposed that one must understand four kinds of variables in order to make reasonable predictions about behavior: behavior potential, expectancy, reinforcement value, and the psychological situation.

• Internal versus external control of reinforcement (aka, locus of control) may be the most important generalized expectancy underlying behavior, according to Rotter.

• Rotter developed the I-E scale in order to measure locus of control.

• A key element in locus of control is contingency awareness, the knowledge that one’s behavior is capable of producing specific outcomes.

• Rotter also developed the Rotter Incomplete Sentences Blank, specifically designed to measure the personality and psychological adjustment of college students.

• Delayed gratification refers to the concept of working (or restraining oneself) at the present time for a reward that will be granted only at a later time.

• Working together, Mischel and Bandura showed that modeling can alter the preference of children for delayed or immediate gratification.

• Mischel addressed what is known as the personality paradox, the appearance that behavior is inconsistent, while our intuition suggests that behavior is consistent.

• Mischel and Shoda proposed the Cognitive-Affective Processing System (CAPS) in order to address the personality paradox. By developing situation-behavior profiles, it is possible to identify patterns in the apparent inconsistency of individual behavior.

• fMRI studies have demonstrated specific brain activity that appears to correspond to the cognitive-affective units that underlie the CAPS.

• Situation-behavior characteristics have helped to address some of the problems that arise in situations in which diverse groups do not come together easily.

CHAPTER 13 / CAC.902

Contextual and Mindfulness-Based Approaches

A family of approaches sometimes called the third wave of behavior therapy emphasizes context, acceptance, mindfulness, values, and the relationship to thoughts rather than only the content of thoughts. Acceptance and Commitment Therapy, or ACT, Dialectical Behavior Therapy, or DBT, and mindfulness-based interventions are prominent examples. They do not reject behavior change. They ask a different question: instead of trying to eliminate every difficult thought or feeling, can the client make room for experience while moving toward what matters?

ACT and psychological flexibility

ACT targets psychological flexibility: the ability to contact the present moment and choose behavior aligned with values. Six processes are often described: acceptance, cognitive defusion, contact with the present moment, self-as-context, values, and committed action. Cognitive defusion means noticing thoughts as mental events rather than literal truths. A teen may say, "I'm a failure." Defusion invites, "I am noticing the thought that I'm a failure." The thought may remain, but its grip can loosen.

Acceptance is not resignation. It is willingness to experience what is present when struggle against it causes harm. A child with chronic pain may learn to carry pain while still attending school and friends. Values are chosen directions, not goals to check off. A value of kindness can guide many actions. Committed action translates values into steps.

DBT and dialectics

DBT was developed by Marsha Linehan for borderline personality disorder and chronic suicidality. It combines acceptance and change. Dialectics means holding opposites: I accept myself as I am, and I need to change. DBT includes individual therapy, skills training, phone coaching, and therapist consultation in its standard form. Skills include mindfulness, distress tolerance, emotion regulation, and interpersonal effectiveness. Adaptations exist for adolescents, including family skills. DBT should not be reduced to a few worksheets. Safety planning and fidelity matter.

Mindfulness-based approaches

Mindfulness-Based Stress Reduction and Mindfulness-Based Cognitive Therapy use formal and informal mindfulness practices. The client learns to observe experience with curiosity and reduced reactivity. Mindfulness is not emptying the mind. It is noticing when attention wanders and returning. For trauma survivors, mindfulness can be destabilizing if it forces attention inward without safety. Trauma-sensitive modifications include open eyes, external anchors, choice, and short practices.

Contextual behavioral science

These approaches often draw on functional contextualism: behavior is understood in context, and the truth of an analysis is judged by whether it helps achieve a purpose. The counselor asks what function a behavior serves. Worry may function as avoidance of deeper emotion. Self-criticism may function as an attempt to prevent rejection. The intervention targets the function, not just the form.

Clinical example

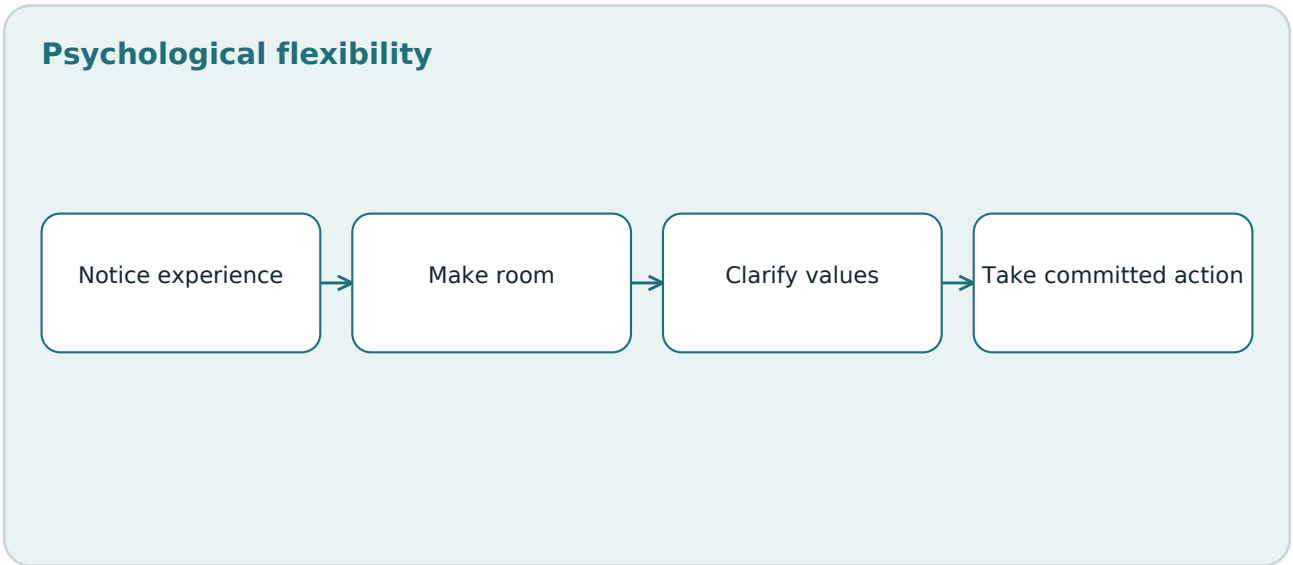
Seventeen-year-old Rina has recurrent self-critical thoughts and avoids applying to college. ACT work helps her notice the thought "I don't belong" as a mental event. She clarifies a value of learning. She commits to one application step while allowing anxiety to be present. The counselor does not promise the thought will disappear. The goal is a larger life, not a quieter mind. If Rina has severe depression, the counselor also assesses safety and may combine ACT with behavioral activation.

Evidence and cautions

ACT and mindfulness-based approaches have growing evidence for various conditions, including anxiety, depression, and chronic pain. DBT has strong evidence for reducing self-harm in borderline personality disorder and is being studied for adolescents. However, these approaches are not replacements for crisis care, trauma treatment, or medication when indicated. Mindfulness can be commercialized and stripped of ethical context. Counselors should consider cultural and religious meanings. Some clients may prefer prayer, movement, or community practices. The counselor should collaborate rather than impose.

Reflection

Write a thought that often controls your behavior. Practice naming it: "I am noticing the thought that..." Then identify a value the thought has blocked. Take one small action in the direction of that value while allowing the thought to be present. Notice the difference between controlling the thought and choosing your action.



ACT targets the relationship to experience rather than only the content of thoughts.

Attributed Foundational Reading

Attribution and critical-reading note

Adapted from Personality Theory in a Cultural Context by Mark D. Kelland, OpenStax CNX / Lansing Community College (2015), CC BY 4.0. Selection, organization, and PDF layout modified for this course; new lectures and course diagrams are separate additions.

Read historical and theoretical claims critically. A theory is a lens for inquiry, not a verdict about a child or family. Where source claims are dated or culture-bound, the new lecture supplies cautions and contemporary framing.

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Historical Description of Buddhism

Siddhattha Gotama

Siddhattha Gotama is recognized as the Buddha, but this is technically incorrect. Anyone can be a Buddha, there were many before Gotama Buddha, many after, and more to come. Indeed, Siddhattha Gotama had lived many lives before he was born into that earthly identity (if, of course, you believe in such things), and this had an important impact on his life. According to legend, Dipankara Buddha foretold that Siddhattha Gotama would be born as a prince in the kingdom of the Shakyas (so he is also referred to as Prince Shakyamuni and as Shakyamuni Buddha), and that in that lifetime he would become a Buddha. Sometime around the fifth or sixth century B.C., Prince Shakyamuni was born. Not wanting his son to leave the kingdom, the king indulged his son with every sensual pleasure known to man. The king also protected his son from knowing the unpleasant realities of life (disease, death, etc.). However, the prince's destiny was set. Prince Shakyamuni decided he wanted to see the kingdom. In order to prevent the prince from seeing the reality of life, the king ordered that everything in the city should be cleaned and decorated and everyone should be on their best behavior. However, four heavenly beings appeared to Prince Shakyamuni: the first as someone suffering the ravages of old age, the second as someone stricken with disease, the third as a corpse, and the fourth as a wandering monk. These visitors made a profound impression on the young prince, who left his wife, child, and home to seek enlightenment.

Living in India, the path to spiritual enlightenment that he followed was to become a yogi. He studied meditation, he became an accomplished ascetic (it is said he lived for a time on one grain of rice a day), but he failed to achieve anything satisfying. So finally he had a nice lunch and sat down under a Bodhi tree, vowing to remain seated until he achieved enlightenment. Finally, he was "awakened," which is the meaning of the word Buddha. In his first sermon, Gotama Buddha revealed the **Four Noble Truths** and the **Middle Way**, the latter also being known as the **Eightfold Path**. Those who have followed his teachings have come to be known as Buddhists. For more on the life of the Buddha, an excellent chapter has been written by Goldstein and Kornfield (2001). The sayings of the Buddha have also been collected, and are readily available (e.g., see Byrom, 1993). In his own words, we can see the relationship between Buddhism and psychology, and how these teachings were meant to guide people toward a healthy and happy life. In the teaching entitled "Choices," the Buddha says:

We are what we think.

All that we are arises with our thoughts.

With our thoughts we make the world.

Speak or act with a pure mind

And happiness will follow you

As your shadow, unshakable. (pgs. 1-2)

Bodhidharma

Bodhidharma (c. 440-528) is recognized as the monk responsible for bringing **Zen** Buddhism from India into China. He was also present during the construction of the Shaolin Temple, and was one of the first monks there. During his time at Shaolin Temple he is most famous for spending nine years in meditation, staring at the wall of a cave. He is also credited with developing kung-fu, the well-known martial arts technique, so that the temple monks could protect themselves from bandits. Although Bodhidharma may have spent a great deal of time in meditation, his Zen teaching was based more on a sword of wisdom (Red Pine, 1987). Some of the strange practices in Zen that we will examine in this chapter can be described as almost surprising people into enlightenment. Of course, many years of practice and discipline are necessary in order to be ready for this enlightenment. Some of Bodhidharma's writings are still available to us today (e.g., Red Pine, 1987), and in his own words (translated, of course) we can get a glimpse of just how strange a Zen understanding of the truth can be:

If you use your mind to study reality, you won't understand either your mind or reality. If you study reality without using your mind, you'll understand both. Those who don't understand, don't understand understanding. And those who understand, understand not understanding. People capable of true vision know that the mind is empty. They transcend both understanding and not understanding. The absence of both understanding and not understanding is true understanding. (pg. 55)

His Holiness the Dalai Lama

Unlike the historical figures Gotama Buddha and Bodhidharma, the Dalai Lama is alive today. Although his home is Tibet, where he was born in 1935, he lives in exile in India. He is believed to be the 14th Dalai Lama, a reincarnation of the previous Dalai Lamas, the first of whom is believed to have been the reincarnation of a boy who lived during the time of Gotama Buddha. That boy was an incarnation of Chenrezig (also known as Avalokiteshvara), the Bodhisattva of Compassion (a Bodhisattva is like a Buddha – see below), and the Dalai Lamas have served for over 650 years as the religious leader of the Tibetan people. Due to political circumstances in Tibet today, it is unclear what may happen to Tibetan culture. The Dalai Lama himself does not know whether he will be the last of the Dalai Lamas, but he hopes that choice will someday be made by a free and democratic Tibetan society (Dalai Lama, 2002).

Placing Buddhism in Context: The First Psychology?

Both Buddhism and Yoga share roots in ancient traditions among the Vedic people. Siddhattha Gotama was a yogi seeking enlightenment, and it was his followers who

established Buddhism as the practice of his new path: the Middle Way. Since most people think of Buddhism and Yoga as separate, it makes things easier to treat them separately.

Buddhism is as old as the Yoga of Patanjali, perhaps even older, and like Yoga had a profound influence on some well-known personality theorists (such as Rogers and Fromm). Since Yoga is usually thought of as a form of exercise in America, because of the popularity of Hatha Yoga, when people think of meditation they often think first of Buddhism. Zen Buddhism emphasizes meditation, and Zen has been the most popular and best-known form of Buddhism in America, largely due to the arrival of D. T. Suzuki in 1897.

There are two major schools of Buddhism in the world today. The Theravada tradition is most popular in southeast Asia. It emphasizes self-discipline and seeking nirvana. The Mahayana tradition is most popular in China, Tibet, Korea, and Japan. The Mahayana tradition emphasizes compassion, and is the school within which Zen developed. Tibetan Buddhism, which also developed within the Mahayana tradition, enjoys something of a celebrity status due to the renown of the Dalai Lama, who is recognized as the spiritual leader of the Tibetan people.

In many ways one can find connections between Buddhism and psychology today. As an interesting example, the well-known author and spiritual leader Jack Kornfield became a Thai Buddhist monk before returning to the United States and becoming a clinical psychologist. Many of the books comparing Eastern philosophy to Western psychology have focused on Zen Buddhism. Zen emphasizes meditation, which in one form or another has become a common element of many types of psychotherapy, particularly in humanistic and cognitive approaches. Since Buddhism shares the same tradition as Yoga, it would make no sense to say that Buddhism has influenced psychology more than Yoga. They are fundamentally the same, and their influence continues to grow.

The Four Noble Truths of Human Life

Following his enlightenment, the Buddha began to teach what he had realized. In his first lesson, he described the Four Noble Truths: 1) suffering is an unavoidable reality in human life; 2) the source of suffering is craving or desire, and the bad karma it creates; 3) the craving that leads to suffering can be destroyed; 4) the Middle Way is the path to eliminate craving and suffering (Suzuki, 1960; World’s Great Religions, 1957; Wilkins, 1967). People often ask why there is so much suffering in the world. When this question is asked, there is usually an unspoken desire to remove this suffering from the world. The Buddha, however, taught us that we cannot escape from reality. Who has never been sick? Who never dies? Who can live without desiring something? The problem is that when our cravings are satisfied, we typically find that we want something else, or something more, we never seem to be really satisfied. And so this cycle of craving, temporary satisfaction, craving again, and so on, continues throughout our life, unless we consciously do something to break the pattern. The Buddha taught us how to do that: by following the Middle Way.

The Middle Way is also known as the Eightfold Path, because there are eight aspects to it: 1) right knowledge, 2) right intention, 3) right speech, 4) right conduct, 5) right livelihood, 6) right effort, 7) right mindfulness, and 8) right concentration. Some believe that each step depends on what goes before it, so that in order to reach higher levels of this discipline one must accomplish the lower levels (Wilkins, 1967). A hierarchical series of steps like this is reminiscent of Maslow’s hierarchy of needs. In Buddhism, we must begin with an

understanding of how things really are, an understanding of the four noble truths, impermanence, and **interbeing** (right knowledge). We must then develop the right intentions, to want to be compassionate, selflessly detached, loving, and non-violent. Once we have developed a conducive state of mind, we can choose to refrain from lying, gossiping, swearing, and other misuse of language (right speech). We can avoid doing things that are immoral, irresponsible, cruel, or illegal (right conduct), and we would not choose a career which required us to do any such things (right livelihood). Then we would be able to focus our will on avoiding any unhealthy states of mind, and on eliminating them quickly should they arise (right effort). Finally we could become more aware of our sensations, feelings, minds, and bodies (right mindfulness), so that we might focus on the discipline necessary to continue our practice of the Middle Way (right concentration).

These principles provide the basis for a practical code of conduct, which all Buddhists must follow, known as the **Five Precepts**. They are: 1) to abstain from killing, 2) to abstain from stealing, 3) to abstain from sexual misconduct, 4) to abstain from lying, and 5) to abstain from mind-altering intoxicants (World's Great Religions, 1957). The first four seem to follow naturally from the Eightfold Path, but the last one is somewhat more interesting. The problem with intoxicating drugs, such as alcohol, is that they cloud the mind. They make it difficult for us to make responsible choices, which is what the Middle Way is all about. Also, if you think back to our discussion of karma, the intentions behind your actions are as important, if not more important, than the actions themselves. When you have made a choice to use intoxicating drugs, and then violate any of the other precepts, it is the consequence of your earlier choice. And who needs bad karma on top of bad karma?

"Calm and compassion are so precious. Make sure not to

lose them through intoxication." – Kornfield, 1994

Characteristics of Existence

Impermanence

The Buddha said that "everything arises and passes away…existence is illusion" (in Byrom, 1993). The idea that nothing is permanent is a central belief in Buddhism. People are born, grow up, grow old, and die. Buildings wear down, cars break down, and enormous trees wither away. Even mountains are eventually worn down by erosion. However, children are born, new cars and buildings are built, new plants grow, and life goes on. The implications for Buddhism are quite interesting. If everything, and everyone, changes, then even someone who is enlightened will change! One cannot be a Buddha, for they will change. We must always continue to grow. Likewise, Buddhism itself will change, so most of their doctrines are not seen as static. They anticipate change over time.

For psychology, this has both good and not so good implications. For people who are depressed or anxious, they might take heart in impermanence, since things should eventually get better. Indeed, studies on the effects of psychotherapy often show that some people get better over time without treatment. However, if things seem to be going great, if you are happy and having lots of fun, those things will change too. But knowing this, we can prepare ourselves for it. An important aspect of coping with life's challenges is a sense of being in control. Although there are a wide variety of variables that contribute to individual resilience, maintaining a positive state of mind can help, and knowledge can help to

maintain that positive state of mind (Bonnano, 2004, 2005; Folkman and Moskowitz, 2000; Ray, 2004).

If we practice **mindfulness** and meditation, we can begin to see the impermanence of our lives. As we let go of our attachments to our self-image, our life will flow by like the pictures of a movie, each one a separate image, which only appears to flow smoothly when viewed at high speed. As we observe these fleeting images, we see how our sensations, thoughts, feelings, every aspect of our lives, change so quickly. We might then embrace the change that is truly our life. This process of letting go can be very difficult, but also very liberating (Goldstein and Kornfield, 2001).

“Do not seek perfection in a changing world.

Instead, perfect your love.” – Kornfield, 1994

Suffering

As we learned with the first of the Four Noble Truths, suffering is an integral part of the human experience. It is easy for us to think of suffering in terms of big pictures: war, famine, natural disasters, and the like. But how often do we think of suffering as an inherent part of our daily lives? Life is difficult, it is a struggle, especially the way most of us live it. A struggle can only lead to suffering. The ultimate outcome of life’s struggle, should we lose the battle, is death. If we could defeat death we would end up alone, and that loneliness might be even worse than the original suffering itself (Suzuki, 1962). Still, we do not even need to look at suffering in terms of a lifetime battle against aging and death, we can see suffering in every moment of the day. Goldstein and Kornfield offer a marvelous description of the daily challenge to be satisfied (2001). It goes something like this. Suppose we woke up on a day when we had no obligations at all. It might be tempting to stay in bed all day, but eventually we become uncomfortable because we have to go to the bathroom. Finally we go, and then crawl back into bed to get warm. But then we get hungry, so finally we get up to get something to eat. Then we get bored, so maybe we watch TV. Then we get uncomfortable, and have to change positions. Even each pleasurable moment is brief, and fails to bring lasting satisfaction. So on, and so forth. We just keep suffering!

The source of this suffering is attachment. We are attached to pleasurable things because we crave them. We are also attached to things that are not pleasant, because they occupy our mind and we cannot be free. The Buddha says, “Free yourself from pleasure and pain. For in craving pleasure or in nursing pain, there is only sorrow” (in Byrom, 1993). It may seem strange that we would be attached to our pain, but the word is used differently here than in most of Western psychology. Traditionally, psychologists think of attachment in a positive way, such as the attachment a child feels toward his or her parents. And yet, some cognitive psychologists do talk about individuals whose automatic thoughts lead them into consistently negative states of mind by disqualifying positive events, catastrophizing events, taking everything too personally, etc. (Pretzer and Beck, 2005). In Buddhism, attachment is neither positive nor negative, it is simply anything that reflects our illusion that the natural world is real. Only when we let go of our attachments to this world can we be one with the universal spirit, and only then can we end our suffering. There is also something hopeful in suffering. Bodhidharma taught that every suffering is a Buddha-seed, because suffering leads us to seek wisdom (in Red Pine, 1987). In this analogy, he describes the body and mind as a field. Suffering is the seed, wisdom the sprout, and Buddhahood the grain.

Discussion Question: Gotama Buddha taught that suffering is the result of craving or desire. Many of us have heard the saying that money is the root of all evil. Is our society excessively focused on buying more and bigger things? Do you ever find yourself obsessed with some material purchase? What problems, if any, have you experienced because people were more concerned with getting things than caring about the people around them?

Selflessness

In keeping with its origins in Yoga, Buddhism teaches that there is no immortal, unchanging soul. All that we are is a temporary collection of attributes, made up of the body, the feelings, the perceptions, the reactions, and the consciousness of the mind (which, coming from the brain, is really part of the body). It is because we confuse our true self (the transcendental self) with this temporary collection of illusory things that we crave satisfaction, and ultimately suffer as a result. Now it may seem illogical to reject everything we are familiar with, including our own physical body, as an illusion, but Buddhists would suggest that there is a danger in choosing intellectual logic over faith. According to D.T. Suzuki (1962), “Faith lives and the intellect kills.” Try the following exercise. Consider your body. Is it real? How much food have you eaten in your life, and where is it now? How many times have you gone to the bathroom, and where did all of that come from? It certainly isn’t the same as when you ate it! Your body has been replaced many, many times. It is being replaced right now. It isn’t real, it is only temporary, ever changing. The same is true with your mind. Even when William James discussed the stream of consciousness, he described a constantly changing awareness, one in which you cannot have the same thought twice. It just isn’t possible. James (1892) realized that we cannot establish a substantial identity continuing from day to day, but concluded that our sense of continuity must reveal a functional identity. Arriving at a very different conclusion, Buddhists consider this to be maya, our inability to see things as they truly are (Suzuki, 1960).

These three characteristics of existence (impermanence, suffering, and selflessness) can be somewhat unsettling. It is not very appealing to believe that we don’t really exist, that we will suffer as long as we believe we do exist, and all of it will just eventually pass away anyway. So, how does one continue in this practice? It is important to keep as our goal a true understanding of the way things are, and the practice of meditation and other aspects of Yoga and Buddhism will help to deepen our realization of these basic truths (Goldstein & Kornfield, 2001). The practice remains challenging, however, because as we deepen our understanding the characteristic most often occupying the center of our greatest realization is that of suffering (Goldstein & Kornfield, 2001; Suzuki, 1962). We must then put aside our intellectualizing, we must slay it and throw it to the dogs, experiencing what Buddhists call the “Great Death” (Suzuki, 1962). Only then will we know the greatest wisdom and compassion. This is the beginning of our transcendence. It is not a separation from others, but a realization that we are all one. In other words, we are all in this together.

Interbeing – A Connection Between All People and All Things

Many people are familiar with the golden rule: do unto others as you would have others do unto you! This Christian saying also has great implications when considered from a Buddhist perspective. Based on the same philosophical/cosmological perspective as Yoga, Buddhists believe that there is one universal spirit. Therefore, we are really all the same, indeed the entire universe of living creatures and even inanimate objects in the physical world come from and return to the same, single source of creation. Thus, we could alter the golden rule

to something like: as you do unto others you are doing unto yourself! This concept is not simply about being nice to other people for your own good, however. Much more importantly, it is about appreciating the relationships between all things. For example, when you drink a refreshing glass of milk, maybe after eating a few chocolate chip cookies, can you taste the grass and feel the falling rain? After all, the cow could not have grown up to give milk if it hadn't eaten grass, and the grass would not have grown if there hadn't been any rain. When you enjoy that milk do you remember to thank the farmer who milked the cow, or the grocer who sold the milk to you? And what about the worms that helped to create and aerate the soil in which the grass grew? Appreciating the concept of interbeing helps us to understand the importance of everyone and everything.

The value of this concept of interbeing is that it can be much more than simply a curious academic topic. The Vietnamese Buddhist monk Thich Nhat Hanh writes very eloquently about interbeing and its potential for promoting healthy relationships, both between people and between societies (Thich Nhat Hanh, 1995):

"Looking deeply" means observing something or someone with so much concentration that the distinction between observer and observed disappears. The result is insight into the true nature of the object. When we look into the heart of a flower, we see clouds, sunshine, minerals, time, the earth, and everything else in the cosmos in it. Without clouds, there could be no rain, and there would be no flower. Without time, the flower could not bloom. In fact, the flower is made entirely of non-flower elements; it has no independent, individual existence. It "inter-is" with everything else in the universe. … When we see the nature of interbeing, barriers between ourselves and others are dissolved, and peace, love, and understanding are possible. Whenever there is understanding, compassion is born. (pg. 10)

Having understood this concept, how might it apply to personality? One of the best known cross-cultural topics in psychology today is the distinction between collectivistic vs. individualistic cultures (Triandis & Suh, 2002; Triandis et al., 1988). It is generally accepted that Western cultures focus on the individual, whereas Eastern cultures focus on society as a collective group. One can easily imagine how people whose religious and cultural philosophy focus on a single, universal spirit (the basis of interbeing) would focus more on their family and societal groups than on the individual. Both individualistic and collectivistic cultures seem to have advantages. People living in individualistic cultures report higher levels of subjective well-being and self-esteem, whereas people in collectivistic cultures have tend to have lower levels of stress and correspondingly lower levels of cardiovascular disease (Triandis & Suh, 2002; Triandis et al., 1988). In collectivistic cultures people tend to view the environment as relatively fixed, and themselves as more flexible, more ready to fit in (Triandis & Suh, 2002). The collectivistic perspective supports the value of social cooperation and social interest (something Alfred Adler would likely appreciate). Still, even within cultures there are individual differences. There are idiocentric persons (those who favor individuality) living in collectivistic cultures, and allocentric persons (those who favor ingroups) living in individualistic cultures. The best relationship between personality and culture may be the "culture fit" model, which suggests that it is best to live in the culture that matches your personal inclinations.

Discussion Question: The concept of interbeing suggests that all things are ultimately connected. Have you ever taken the time to think about all the things that had to happen, and all the people who were involved, in producing anything you hold in your hand? What

about all the things that had to happen, and all the people who were involved, in your creation? And if we are all connected in some way, if we are all interbeing, what have you done to value those relationships?

Connections Across Cultures: The Non-Violent Struggles of Mahatma Gandhi,

Thich Nhat Hanh, Martin Luther King, Jr., and the 14 th Dalai Lama

The four men listed above are famous in a variety of ways, but they are probably best known for their commitment to nonviolence as a way to achieve political and social justice. Most importantly, they vowed non-violence while those around them were committed to terrible violence in order to deny justice to others. The two who are not alive today were both assassinated, and the other two were forced to live in exile. Gandhi was a Hindu who practiced Yoga, Thich Nhat Hanh and the Dalai Lama are Buddhists, and M. L. King, Jr. was a Christian, and it was their spiritual beliefs that so profoundly determined those aspects of their personalities that demanded peace.

Gandhi (1869-1948) is considered the father of modern India. He was born when the British ruled India, and spent much of his life fighting for the independence of his homeland. Twice he was imprisoned by the government, even though he insisted that all protests should be nonviolent. Indeed, he had established a movement of nonviolence known as **Satyagraha**. Ultimately this movement was successful, and India achieved its independence. Gandhi, however, was assassinated less than a year later. As he died, he spoke the name of God: Rama (Easwaran, 1972; Wilkinson, 2005).

Thich Nhat Hanh (1926-present) was born in Vietnam, and saw his country dominated first by the French and then by communists. During those difficult times he helped to develop what he and his friends called “engaged Buddhism.” Rather than sitting in the temple meditating, they went out into the villages and tried to help the poor people of Vietnam. When confronted by soldiers they did their best to remain mindful, and to feel compassion for the soldiers who threatened them. After all, it was clear to Thich Nhat Hanh that many of those young soldiers were frightened themselves, and so their behavior was very hard to predict. Thus, the calm and peace that accompany mindfulness was often essential for protecting everyone in those terrifying encounters. After being exiled from Vietnam in 1966, he established a community called Plum Village in France, where he still resides today (Thich Nhat Hanh, 1966, 2003).

Martin Luther King, Jr. (1929-1968) was a major figure in America’s civil rights movement of the 1950s and 60s. The King children learned at an early age about the realities of racism in America. Coming from an educated and socially active family, both his father and grandfather were ministers, he vowed at an early age to work against racial injustice. According to his sister, he said he would turn the world upside down (Farris, 2003). However, he always insisted on doing so in a nonviolent fashion. For this commitment to nonviolence, in 1964 he became the youngest person to ever receive the Nobel Peace Prize. Despite the peace prize and the passage of both the 1964 Civil Rights Act and the Voting Rights Act in 1965, discrimination continued in America. So did the nonviolent protests led by Dr. King. Then, in 1968, Martin Luther King, Jr. was assassinated (Burns, 2004; Hansen, 2003; Patrick, 1990).

The Dalai Lama (1935-present) lives in exile in India, though he also spends a great deal of time in America. When China invaded Tibet in 1950, he appealed to the United Nations,

other countries, and even tried to reach an agreement with the Chinese leadership. Eventually, however, he was forced to leave Tibet in 1959. Today, nearly 50 years later, he continues to seek a peaceful resolution resulting in freedom for Tibet. He also works to deliberately cultivate feelings of compassion for the Chinese, believing that someday those who have harmed the people of Tibet will have to face the consequences of their actions (Dalai Lama, 2002). The Dalai Lama received the Nobel Peace Prize in 1989.

These men have more in common than simply their shared belief in nonviolence. In addition to M. L. King, Jr. and the Dalai Lama receiving the Nobel Peace Prize, as Nobel Laureates are entitled to do, Dr. King nominated Thich Nhat Hanh for the same award. Dr. King had received a letter from Thich Nhat Hanh asking for help in protesting the Vietnam war, which by the 1960s involved the United States. Dr. King was impressed by the Buddhist monk, and once appeared with him at a press conference in Chicago (Burns, 2004). Dr. King was also familiar with and impressed by the teachings of Gandhi. In 1959 he traveled to India to learn firsthand about Gandhi's Satyagraha, the basis for Gandhi's nonviolent independence movement (King, 2000). In 1966, Dr. King delivered the Gandhi Memorial Lecture at Howard University (Hansen, 2003). Since both the Dalai Lama and Thich Nhat Hanh are alive today, they have met one another and the Dalai Lama has written several forewords for books by Thich Nhat Hanh. If these men from different countries and different cultures can share so much through the simple (though not easy) practice of nonviolence, perhaps there is something special here for everyone to learn more about.

Meditation

Meditation Techniques

Meditation is the means by which we control our mind and guide it in a more virtuous direction (Dalai Lama, 2001). Modern brain imaging techniques have even begun to identify the brain regions involved in these processes (Barinaga, 2003). There are many different meditation techniques in Yoga and Buddhism, and no one technique is necessarily better than another. What is most important is to pick one type of meditation and stick with it. Meditation takes practice. Most of us find it very difficult to relax and clear our mind. Even when we do, it is difficult to stay relaxed and keep our mind clear. We are distracted by constant thoughts, getting uncomfortable, we have itches and sneezes and whatever… But over time we can get better at relaxing. It helps to have a well-described procedure, and it can be very helpful to meditate in a group (especially if they offer classes or lessons on how to meditate). If you try meditation, don't get discouraged the first few times. Keep it up. As with all paths toward self-improvement, it takes time to progress in your ability to meditate.

Some of the writings of Master Dogen (1200-1253), the monk who founded Japanese Soto Zen, have survived during the 800 years since he lived (in Cook, 2002). Master Dogen recommends a very traditional form of seated meditation. Basically, sit straight up on a comfortable cushion with your legs crossed. Place your right hand in your lap, palm up, and your left hand on your right hand in the same manner, so that your thumbs touch slightly. Keep the eyes slightly open, the mouth closed, and breathe softly. Next comes the hard part: "Think about the unthinkable. How do you think about the unthinkable? Non-thinking."

Non-thinking may sound strange, but it is a fascinating experience for those who achieve it. It can actually make a 3- or 6-hour mediation seem to go by more quickly than a shorter meditation in which you never quite clear your mind. If it sounds a little too strange, don't

worry, it isn't the goal of every form of meditation. Some forms of meditation focus on a mantra, or in Christian mediation a short prayer. Trying to focus on God through the celestial eye (in the middle of the forehead) is also a common technique. The Dalai Lama describes several different approaches in one of his books (Dalai Lama, 2001), and Thich Nhat Hanh discusses being reasonable in one's approach to longer meditations (Thich Nhat Hanh, 1991). Once again, there is not a right or wrong method of meditation. Whatever technique you try, whether from a book, a guru, a teacher, or a group, it is whatever works for you on your path to personal development.

Soto Zen and Zazen

Soto Zen is one of the two forms of Zen popular in Japan. Soto Zen emphasizes **zazen**, which translates loosely as "sitting meditation." It is believed that this form of meditation, common to the way most Americans envision meditation, is a tribute to the legend of Zen founder Bodhidharma spending nine years meditating while staring at a wall (Suzuki, 1962). However, there is a problem in the literature on zazen. For those who believe that zazen can help to achieve Buddhahood, the masters have taught that it cannot work. Still, it remains the essential core practice. How can this be? According to Alan Watts (1957), it may be better to consider an alternative translation of zazen, in which it translates as "sitting just to sit." In this view, zazen results in a clear mind, the condition necessary for enlightenment. A wonderful resource for practicing basic meditation of this type has been provided by Jack Kornfield, a clinical psychologist who trained as a Buddhist monk. Meditation for Beginners (Kornfield, 2004) not only briefly describes the history, purpose, and benefits of meditation, but it also includes a CD with guided meditations led by the author.

Rinzai Zen and the Koan

The other form of Zen popular in Japan is Rinzai Zen, founded by the monk Eisai (1141-1215). Buddhists in the Rinzai school practice zazen, but they also emphasize the curious practice of meditating on a koan, a riddle that has no answer. The Soto school of Zen frowns upon the use of the koan, considering it to be superficial and possibly misguided, something of a gimmick. But within the Rinzai school the Zen masters became concerned about what they saw as problems within the communities practicing Zen. One of the chief concerns of the Rinzai masters was the reliance on absolute quietude, pure meditation purely for the sake of meditation. They believed that Zen comes from life, and therefore must grow out of life (Suzuki, 1962). They were also concerned about the intellectualization of Zen, the belief that discerning logic could lead to enlightenment (again, concerned that the pursuit of logical ideas became its own pursuit). And so they developed the **koan**. The purpose of these unsolvable riddles is to put an end to logical thinking, to stop the wandering minds of students and to create a profound sense of doubt in the student's mind (Reps and Senzaki, 1994; Suzuki, 1960, 1962; Watts, 1957). The presentation of the student's views on a koan to the Zen master is an important time, and advanced students may even challenge whether the master appreciates how deeply the student understands Zen. These periodic visits to the Zen master to present an answer to a koan are known as **sanzen** (Suzuki, 1962; Watts, 1957).

Although koan are considered unsolvable, they do have answers. Certainly the most famous koan is the one in which the monk Joshu is asked: Does a dog have Buddha-nature? Joshu replied, "Mu!" Mu is the Chinese symbol for "no thing" or "not." The understanding of this koan is not found in the odd answer, or in Joshu's apparent refusal to provide a straight

answer, “Mu” is Buddha-nature! If you find this particularly difficult to understand, I suggest you back up to the quote I gave you from the writings of Bodhidharma. I doubt it will make it any more clear, but perhaps it will help to illuminate the philosophy of non-attachment: don’t worry too hard about understanding, just understand.

A student of the Rinzai school of Zen will actually study many koan, and the process is kept quite secret. Obviously if the answers to these koan were known, then students could offer the apparent answers. I say apparent answers because a Zen master would clearly recognize the mindless answer of one who really does not understand the koan. Since the process is kept secret, not many koan are publicly known, but there are some available. Not all koan are quite as odd as Joshu’s “Mu;” some are seemingly easier to understand, but challenge our concepts of defining aspects of the natural world. For example, the monk Daie used to carry a short bamboo stick, and he would say, “If you call this a stick, you affirm; if you call it not a stick, you negate. Beyond affirmation and negation, what would you call it?” (Suzuki, 1962) This is clearly a challenge to move beyond logic, beyond the normal way of thinking about things. Enlightenment, of course, does not come from normal ways. Through the use of meditation to clear our minds, and the use of koan to challenge our perspectives, Rinzai Zen seeks to offer a path toward enlightenment that will be successful.

Discussion Question: Have you ever practiced meditation, or perhaps contemplative prayer? If you have, how does it make you feel? If you haven’t, does it sound attractive? Were you surprised to learn that the practice of meditation is so old, and that it has counterparts in all of the major religions?

Mindfulness

Mindfulness is a form of meditation that occurs throughout every moment of the day. Indeed, it is very important to live fully in every moment, and to look deeply into each experience (Thich Nhat Hanh, 1991, 1995). By being mindful, we can enter into awareness of our body and our emotions. Thich Nhat Hanh relates a story in which the Buddha was asked when he and his monks practiced. The Buddha replied that they practiced when they sat, when they walked, and when they ate. When the person questioning the Buddha replied that everyone sits, walks, and eats, the Buddha replied that he and his monks knewthey were sitting, knew they were walking, and knewthey were eating (Thich Nhat Hanh, 1995). Mindfulness can also be applied to acts as simple as breathing. According to Thich Nhat Hanh, conscious breathing is the most basic Buddhist technique for touching peace (Thich Nhat Hanh, 1991, 1995). He suggests silently reciting the following lines while breathing mindfully:

Breathing in, I calm my body.

Breathing out, I smile.

Dwelling in the present moment,

I know this is a wonderful moment!

The concept of mindfulness, viewed in its traditional way, is also being used today in psychotherapy. Two recent books address the use of mindfulness either in combination with cognitive behavioral therapy to treat depression (McQuaid and Carmona, 2004) or as its own approach to the treatment of anxiety (Brantley, 2003). McQuaid and Carmona (2004) discuss how combining cognitive behavioral therapy and mindfulness together can provide a much

stronger approach to treatment than either technique alone. Since the approaches have much in common, they amplify the effectiveness of each, and given their differences, they offer a complete path to moving beyond simple recovery toward more positive self development. Dr. Brantley (2003) moves more completely into the practice of mindfulness, emphasizing that it must become a way of life. It is not simply a clever therapeutic technique or gimmick.

Discussion Question: Mindfulness refers to maintaining a meditative state throughout the day. A similar approach is essential to cognitive/behavioral therapy. Are you aware of what you do during the day, or are you overwhelmed with being too busy? Could you see the practice of mindfulness as a helpful way to deal with your hectic life, and perhaps reduce stress at the same time?

Enlightenment and the Ideal Person

Concepts of Enlightenment

There are a wide variety of different concepts of enlightenment. One concept that is somewhat familiar in America is **nirvana**. Nirvana refers to the extinction of all ideas and concepts, thus resulting in the end of suffering due to the craving that results from being attached to anything in the natural world. In the Theravada tradition this should be the goal of all Buddhists, and a person who achieves nirvana is referred to as an **Arhat**. Other Buddhists, however, seek to avoid nirvana, because it leaves all others behind.

In the Mahayana tradition, an ideal person would be a Bodhisattva, one who vows to forego complete enlightenment until all other beings have been enlightened. According to Thich Nhat Hanh (1995), such individuals touch mindfulness, and as a result of living mindfully they can touch the Buddha and shine their light of awareness on everything they do. The desire to help others achieve enlightenment comes from the deep compassion developed by **Bodhisattvas**. Mindfulness leads to this compassion, as it leads one to develop **Bodhicitta**. Bodhicitta, which means “mind of enlightenment” or “mind of love,” is an inner drive to fully realize oneself and to work for the well-being of all (in other words, it is the driving force of being a Bodhisattva; Dalai Lama, 2001; Thich Nhat Hanh, 1995, 1999).

Curiously, some Buddhists distinguish between “bodhi” as a temporary flash of enlightenment, which can even occur following arduous meditation or by accident, and “nirvana” or true liberation, which can only result from proper knowledge and dedicated practice (Mathew, 2001). This concept is similar to what Maslow described as the difference between a peak experience and a plateau experience. A peak experience is a brief period of fulfillment, usually associated with a particular event. A plateau experience, on the other hand, is a lasting feeling of oneness with the world around us. What is of particular importance for the study of personality is the recognition that individuals with certain personality characteristics, such as being kind-hearted and open-minded, can more easily accomplish long-term fulfillment, whether we call it nirvana or self-actualization.

Compassion and Loving-Kindness

“Just as compassion is the wish that all sentient beings be free of suffering, loving-kindness is the wish that all may enjoy happiness” (Dalai Lama, 2001). With these simple words about Buddhism, His Holiness the Dalai Lama has captured the history of psychology briefly presented in the introductory chapter: that psychology focused for many years on helping to

identify and treat mental illness (hopefully freeing people from suffering), whereas now there is a strong movement toward positive psychology (hoping to improve well-being for all). This recognition of compassion as the strong feeling or wish that others be freed from suffering comes from mindfulness. As one becomes truly aware of the suffering involved in human life, and if one is able to feel genuine empathy for others, then compassion naturally arises (Chappell, 2003; Dalai Lama, 2001; Goldstein & Kornfield, 2001; Thich Nhat Hanh, 1995). Compassion has described as the ideal emotional state (Bankart et al., 2003; Cook, 2002; Dockett & North-Schulte, 2003; Ragsdale, 2003), and Carl Rogers considered genuine empathy to be essential for client-centered therapy to be successful. Aside from Rogers, however, have other psychologists begun to examine the value of compassion and loving-kindness? The answer is an unequivocal “Yes” (Bankart et al., 2003; Batson et al., 2005; Cassell, 2005; Dockett & North-Schulte, 2003; Keyes & Lopez, 2005; Khong, 2003; Ragsdale, 2003; Schulman, 2005; Young-Eisendrath, 2003)!

“Life is so hard, how can we be anything but kind?”

- Kornfield, 1994

Obstacles to Personal Growth: The Three Poisons of Buddhism

Buddhists believe in **three poisons**, the great obstacles to personal development. They are greed, anger, and delusion. These poisons, or realms as they are often called, have no nature of their own, they are created by us and they depend on us. Greed flows from attachment, anger flows from our emotions, and delusion flows from maya. By following the practices of Buddhism, we can free ourselves from these poisons as did the Buddha. According to Bodhidharma, the Buddha made three vows. He vowed to put an end to all evil, by practicing moral prohibitions to counter the poison of greed. He vowed to cultivate virtue by practicing meditation to counter the poison of anger. And he vowed to liberate all beings by practicing wisdom to counter the poison of delusion (in Red Pine, 1987). Likewise, we can devote ourselves to the three pure practices of morality, meditation, and wisdom.

It is interesting to note how well this philosophy fits with the growing field of positive psychology (e.g., see Compton, 2005; Peterson, 2006). Indeed, whole books have been written on the study of virtue in psychology (Fowers, 2005; Peterson & Seligman, 2004). Note, however, that these books are quite recent. Although the seeds of positive psychology, studies on virtue and similar topics have been around since the earliest days of psychology in the Western world, we seem to be just starting to “discover” concepts that have been well established in Eastern philosophy/psychology for thousands of years. As we recognize more similarities between traditional Eastern perspectives and current Western perspectives, it may help to guide these developing areas of psychological research in the Western world.

Zen Buddhism in America

Zen Buddhism is probably the best-known school of Buddhism in America due to the influence of D. T. Suzuki, who first visited America in the year 1897. Suzuki was a renowned Buddhist scholar, who wrote extensively on Zen Buddhism and its relationships to such diverse topics as Christian mysticism (Suzuki, 1957) and psychoanalysis, the latter book being co-authored by Erich Fromm (Suzuki et al., 1960). Zen Buddhism has also made its way into popular literature in the United States. The famous “Beat generation” author Jack Kerouac, who had many discussions with D. T. Suzuki (Suzuki taught for a few years at Columbia University, which happened to be Kerouac’s alma mater), wrote a most entertaining book about his own pursuits on the path of Zen called The Dharma Bums (Kerouac, 1958). And there was the immensely popular classic entitled Zen and the Art of Motorcycle Maintenance, first published in 1974 (Pirsig, 1999), which opened the eyes of a whole new generation of Americans to the philosophy of the Far East.

Buddhism, and Zen Buddhism in particular, is also beginning to have more and more influence on psychotherapy in America today. Although a discussion of therapy techniques is beyond the scope of this chapter, it is becoming easy to find such material. There are books on Zen and psychotherapy (Brazier, 1995; Mruk and Hartzell, 2003), Buddhism and well-being (Brach, 2003), comparisons of psychoanalysis and Buddhism (Epstein, 1995; Suzuki et al., 1960), and a variety of chapters in spiritually oriented handbooks on psychotherapy (e.g., Cooper, 2005; Corbett & Stein, 2005; Crawford, 2005; Finn & Rubin, 2000; Lukoff & Lu, 2005; Roland, 2005; Sharma, 2000; Tan & Johnson, 2005). Not only does this blending of Eastern and Western philosophy promise to expand the horizons and potential effectiveness of psychotherapy, learning more about the different cultural perspectives that led to these different lifestyles will help to prepare psychologists to recognize more quickly and easily the culturally-related issues that are affecting their patients or clients.

Sangha: A Community Practicing Together

The concepts of togetherness, friendship, social support, etc. are certainly well known in the West, despite the fact that Western cultures are generally considered to be individualistic. Adler identified developing friendships as one of three main tasks in life, and the value of social support during times of stress and grief has been well documented. In Yoga and Buddhism these concepts have been central for thousands of years. Buddhists refer to the **Three Jewels** (also known as the Three Gems or the Three Refuges): the Buddha, the **Dharma**, and the **Sangha**. A Buddha is one who is fully enlightened (not just Gotama Buddha), and the Dharma is the way of understanding and love taught by Gotama Buddha. A Sangha is a community of Buddhists who practice the Dharma and seek enlightenment together (Suzuki, 1960; Thich Nhat Hanh, 1995). The Sangha is not, however, just a get-together of companions with similar interests. The Sangha can renew our inspiration and energy, and it can help us to keep practicing when our own motivation wanes (Goldstein & Kornfield, 1987). The energy and motivation we gain from being part of a Sangha can help us to develop Bodhicitta, the altruistic desire to help all people achieve enlightenment. The ceremony to actively generate Bodhicitta within us begins with a series of visualizations in which we imagine Gotama Buddha being with us, surrounded by other Buddhas, great sages, and all sentient beings (Dalai Lama, 2001). Being filled with Bodhicitta makes us a Bodhisattva right away (Thich Nhat Hanh, 1999). This is not simply a belief or devotion,

however. Taking refuge in the Sangha is a practice, one that can only take place in the company of others and with their support (Thich Nhat Hanh, 1995).

The Sangha is by no means unique to Buddhism. In Yoga they refer to Satsanga, associating with the truth or with someone virtuous such as a guru (Feuerstein, 2003; Yogananda, 1946). I remember when a monk, and a monk in training, from the Self Realization Fellowship visited the Yoga retreat center I visit. During the evening they offered Satsanga, a brief lesson followed by a question and answer discussion. In this semi-formal setting we were all able to expand our understanding of Yoga and share our interests and experiences. Indeed, some people practicing traditional Yoga or Buddhism consider the guru (or lama, in Tibetan) to be a fourth jewel in which to seek refuge (Feuerstein, 2003).

Discussion Question: Buddhists strongly support a Sangha, a community of believers. Can you imagine practicing Buddhism alone? What about your own personal groups, whether church groups, clubs, friends, family, etc.? Are they supportive? How important are those groups to the way you live your life, and could you imagine your life without them?

A Final Note

This chapter and the one that follows have very religious overtones. So I would like to say a little more about the importance of covering these topics in a personality textbook. Alfred Adler said that if you want to understand someone, look at their style of life. As stated at the outset of this chapter, I have tried to present this material as guides for one's lifestyle that have developed within different cultures. There are three basic moral codes that influence our lives: community, autonomy, and divinity (see Triandis & Suh, 2002). In collectivistic cultures there is an emphasis on community moral codes, whereas autonomy codes are more influential in individualistic cultures. Both cultures emphasize moral codes related to divinity (as religion or spirituality). If we tried to separate religious culture absolutely from our study of personality, we might very well end up with an academic discipline that misses the richness and wonder of human life. More importantly, what happens to people who ignore spirituality in their own development? Abraham Maslow lamented the defense mechanism of desacralization, the failure of people to consider anything to be truly important and meaningful. Yehudi Menuhin, in his introduction to Light on Yoga by B. K. S. Iyengar (1966), offers a striking impression of those who do not seek harmony with the universe:

What is the alternative? Thwarted, warped people condemning the order of things, cripples criticizing the upright, autocrats slumped in expectant coronary attitudes, the tragic spectacle of people working out their own imbalance and frustration on others. (pg. 12)

I don't know whether Maslow considered desacralization to be this frightening a possibility, but it certainly gives one reason to consider the value of spiritual aspects of human development, especially since spirituality is one of the cultural universals (Ferraro, 2006a; Murdock, 1945). And given that spirituality is universal, these matters are certainly not unique to Eastern culture. In the next chapter we will consider spiritually guided lifestyle recommendations as they apply to cultures influenced by the Abrahamic religions: Judaism, Christianity, and Islam.

Personality Theory in Real Life: Are You Really You?

We ended the first chapter in this book by asking an interesting question: Who are you? In this chapter, we have addressed the possibility that everything you know about yourself is an

illusion, and that even knowing is an illusion. How can this be? The answer may be found, or perhaps not found, in the mystery that is God. The Christian Bible teaches that God's ways are not Man's ways. Paramahansa Yogananda provides a marvelous image of the mystery of the Godhead being so far beyond our comprehension that it defies description (Yogananda, 1946); and Dante's awesome description of the appearance of the divine essence in Paradiso is difficult to envision, even as one reads Dante's words (in Milano, 1947). Perhaps some things are beyond our comprehension.

How then, should we proceed to live our life? Based on the concept of Karma, our past actions will influence our future experiences. Consider things you have done in your life. Have you regretted some of them? Did they seem out of character for you? Try to determine if unfortunate events followed those actions you regret. On the positive side, are there things you have done that make you proud or happy? Have those things involved other people, or were they done for other people? Try to determine whether those good things you have done resulted in favorable consequences for you and for others.

Now, here comes the tricky part. When you have done good things, do they feel more like you than the bad things did? If the answer is yes, it may be that you have begun to touch something special within yourself. You are responsible for both the good things and the bad things you have done in this life. But perhaps the good things feel better, feel more like you, because they begin to connect you with your transcendental self, that spark of the divine within you, which may be called spirit or soul. Thinking this way is a deep and powerful challenge, which requires you to have some faith in yourself. Meditate on this, and see what happens!

Attribution and critical-reading note

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Read historical and theoretical claims critically. A theory is a lens for inquiry, not a verdict about a child or family. Where source claims are dated or culture-bound, the new lecture supplies cautions and contemporary framing.

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Ancient and Modern Approaches to Training the Mind/Body Connection

Wilhelm Reich***was by no means the first person to consider the connection between the body and the mind as something of essential importance to understanding the nature of the human experience. Gotama Buddha had developed just such a system approximately 500 years B.C. (see Chapter 17 for a more detailed discussion of Buddhism). The Buddha did more, however, than simply describe the nature of the human mind. He offered a few ways to begin quieting the mind, so that one could become a more peaceful, aware, and content individual. In the following section, we will consider his four techniques of** mindfulness meditation*.

Buddhism and Mindfulness

Andrew Olendzki (2003, 2005), a scholar of early Buddhist tradition and executive director of the Barre Center for Buddhist Studies in Massachusetts, has done a marvelous job of trying to put the teachings of the Buddha into a perspective understandable to Western psychologists. In very simple terms covering only a small part of what the Buddha taught, when a sense object that we are capable of detecting is, indeed, detected by one of our sensory systems, we become aware of the experience. For example, when a sound is detected by our ear, we become aware of hearing a sound. Consciousness is an emergent phenomenon of each of these individual moments of **contact**, i.e., the moment of contact between the sense object, the sensory organ, and the awareness of the object. Since we are constantly encountering different moments of contact that arise and then fall out of consciousness, from all of our various senses, the Buddhist concept of consciousness is not a continuous one (this is in contrast to the stream of consciousness perspective of America's preeminent psychologist William James). Since consciousness is not continuous, neither is the self. Our sense of self as continuous and real is an illusion, and it is because we cling to that illusion that we inevitably suffer (the first noble truth in Buddhism). In order to alleviate our suffering, and to understand the true nature of our self, the Buddha taught a series of mindfulness meditations to help us see ourselves as we really are.

There are **four mindfulness trainings: mindfulness of body, mindfulness of feeling, mindfulness of mind**, and **mindfulness of mental objects** (Olendzki, 2005; Thanissaro Bhikkhu, 1996). When meditating mindfully on the body, it is common to focus on the breath. This can be done in a variety of positions: sitting, standing, lying down, or walking. One can also become very mindful of the body by performing certain **martial arts** as moving meditation, particularly **Tai Chi Chuan** or **Qigong** (Khor, 1981). When meditating mindfully on feelings, one considers the pleasant or unpleasant quality of each experience. For example, after sitting for a while, pain or discomfort may arise in a knee or hip. There is nothing wrong with this pain, and with practice one can experience it as a sensation without the negative or unpleasant feeling that we describe as pain. This is, of course, not easy. All forms of meditation require time and practice. Still, it is important to remember that if there is a real problem, such sitting on a sharp rock, you may want to move in a slow and mindful manner until comfortable again. When meditating mindfully on the mind itself, one takes notice of the thoughts arising during meditation. One should pay particular attention to whether the thoughts are related to one of the three root causes of suffering: greed, hatred, or delusion.

…In any given moment, the mind is either caught up by one or more of these or it is not, and this is something of which one can learn to be aware. Greed and hatred are the two polarities of desire, the intense wanting or not wanting of an object, while delusion is a strong form of the basic misunderstanding that gives desire its power over us. (pg. 255; Olendzki, 2005)

One does not pass judgment on these thoughts, mindfulness teaches us only to become aware of our thoughts and to recognize their presence and reality. Finally, there is mindfulness of mental objects (or mental qualities), a deep understanding of the content of mental experience that arises as one masters mindfulness meditation (Olendzki, 2005; Thanissaro Bhikkhu, 1996). Mindfulness of mental objects involves focusing on the nature of desires as they arise in relation to the five hindrances: desire, aversion, indolence, restlessness and doubt.

This conservative and traditional understanding of mindfulness may seem rather esoteric, but it is proving to be very influential in psychology today. To be sure, meditation has been described as “now one of the most enduring, widespread, and researched of all psychotherapeutic methods” (Walsh & Shapiro, 2006). A **mindfulness-based stress reduction**program has been developed and popularized by Jon Kabat-Zinn (1990, 1994, 2005), and a similar therapeutic technique, called **Focusing**, had previously been developed in the late 1970s (Gendlin, 1990). Mindfulness has also been incorporated into psychotherapeutic approaches to dealing with anxiety, depression, and feelings of unworthiness and insecurity (Brach, 2003; Brantley, 2003; McQuaid & Carmona, 2004), and it has provided new perspectives on the treatment of addiction and anger issues (Aronson, 2004; Dudley-Grant, 2003). Of particular interest to students, mindfulness has proven to be helpful in alleviating the stress associated with studying psychology in graduate school (Borynski, 2003)! In addition, Janet Surrey, one of the founding members of the Stone Center group, has studied comparisons between mindfulness and relational therapy (Surrey, 2005). Likewise, Trudy Goodman, who studied with Jean Piaget and now also teaches insight meditation, has utilized mindfulness in therapy with children (Goodman, 2005).

This traditional approach to mindfulness is usually associated with Southeast Asia, particularly the Thai forest monks. Jack Kornfield, a former Buddhist monk and currently a clinical psychologist, practiced with the renowned Ajahn Chah. Ajahn Chah’s teachings have been translated into English (Ajahn Chah, 2001), and another of his students has written two books in English (Ajahn Sumedho, 1987; 1995). Thanissaro Bhikku is another interesting individual dedicated to offering the teachings of the Buddha, known as the Dhamma. In conjunction with Dhamma Dana Publications, he has written his own book (Thanissaro Bhikkhu, 1993), translated the works of Buddhist monks and nuns (Ajaan Fuang Jotiko, 2005; Upasika Kee Nanayon, 1995), and translated with commentary some of the Pali Canon, the first written record of the teachings of the Buddha (Thanissaro Bhikkhu, 1996). Dhamma Dana Publications is committed to the free dissemination of these teachings and their books, sending many copies to people in prison who wish to better their lives. This is, of course, an active application of the Buddha’s teachings, and a way to help improve our society.

Discussion Question: The Buddha proposed a method for alleviating the suffering associated with our desires and distresses: the four mindfulness trainings. Have you ever tried meditating, particularly the form of mindfulness meditation taught in the Theravadan tradition? Has it been helpful, or if you haven’t tried it, do you think it might be helpful?

The Neurobiology of Mindfulness

We began this chapter by looking at genetics and biology. We then transitioned into Buddhist mindfulness techniques that are thousands of years old. Today, these two disciplines have come together in some fascinating research. Neurobiologists and psychologists are working together with advanced meditators and respected Buddhist monks (including His Holiness the Dalai Lama) to study the activity of the brain, in real time, during meditation. These studies may also help to advance our understanding of the nature of the mind, but that may still be somewhat beyond our technical abilities. The interest of the field of psychology, and academia in general, is clearly evidenced by articles that have been written about these studies in venues such as the prestigious journal Science (Barinaga, 2003), the popular The Chronicle of Higher Education (Monastersky, 2006), and

the Monitor on Psychology published by the American Psychological Association (Winerman, 2006).

Cognitive neuroscience has taken advantage of many technical advances in brain imaging, such as **functional magnetic resonance imaging (fMRI)**, **single photon emission computed tomography (SPECT)**, and **positron emission tomography (PET)** to study the activity of the brain during mental tasks. Initially, these studies focused on identifying brain regions involved in very specific tasks. More recently, however, some investigators have become interested in using these techniques to study broad questions, such as the nature of the mind. Since we don't know what the nature of the mind is, we don't exactly know what to look for in these brain imaging studies. So, the investigators pursuing this research must creatively examine the brain during meditation (as well as under other conditions). It has been shown that meditation activates neural structures involved in attention and arousal (Lazar et al., 2000, 2005a; Newberg, 2001), alterations in sensory processing and the sense of space (Lazar, 2005a; Newberg, 2001), and a dramatic increase in synchronization of neural activity (Lutz et al., 2004). In perhaps the most striking of these studies, Lazar and her colleagues have demonstrated that long-term meditation practice is associated with increased cortical thickness in brain regions associated with attention and sensory processing (Lazar, 2005a). These effects were most pronounced in the older subjects, suggesting that meditation may have beneficial effects in terms of offsetting age-related declines in cortical thickness. Given these dramatic changes in brain function as a result of meditation, perhaps it should come as no surprise that meditation and mindfulness have proven to be useful adjuncts to therapy for a wide variety of psychological and medical disorders (for reviews see Lazar, 2005b and Newberg & Lee, 2005; see also Cozolino, 2002; Germer et al., 2005; Siegel, 2007).

PET scan images of the author's brain on the anti-Parkinson's Disease drug l-DOPA (for research).

The use of these brain imaging techniques to study the mind during meditation raises the possibility that they may be useful in studying other altered states of consciousness. Indeed, Amir Raz and his colleagues (2005) have utilized fMRI and electrical scalp recording of **event-related potentials** to demonstrate that **hypnotic suggestion** reduces the activity of cortical regions in the brain that have been associated with conflict monitoring. In other words, when hypnosis is used to alter the behavior and cognition of individuals, there are recognizable changes in brain function. When the study of hypnosis is combined with the data obtained on alterations in brain function during meditation and under the influence of mind-altering drugs (see Mathew, 2001), it seems clear that the mind, either in its normal state or in various altered states, is reflected in unique states of neural activity. We may be a long way from fully understanding the details of the relationship between the mind and neural activity, and there may indeed be more to the mind than simply the neural activity itself, but this is certainly a fascinating field of study on the nature of who we are as individuals.

Discussion Question: Cognitive neuroscientists have begun to identify changes in brain activity associated with meditation, and similar changes occur during hypnosis. What do you think this says about the mind?

Martial Arts

When we think of the martial arts, most of us think of East Asia. The different forms are typically associated with the countries where they developed: **Kung Fu** in China, **Taekwondo** in Korea, and **Karate** and **Judo** in Japan. Actually, Karate was developed on the island of Okinawa, and, although it is part of Japan today, its martial arts history has been influenced more by Chinese settlers than by the Japanese (Chesterman, 2003; Hornsey, 2002; Johnson, 2003a; Lewis, 1993; Ribner & Chin, 1978). Today, however, the martial arts are popular worldwide. There are many forms in addition to those listed above, including **Capoeira**, a martial art developed in Brazil by African slaves (Atwood, 1999). Capoeira is a particularly complex martial art, involving play, dance, and music. As some slaves escaped, they banded together to fight Portuguese soldiers and help other slaves to escape. More recently, Capoeira was one of the inspirations for break dancing, an African American dance style that developed in the 1970s and 1980s (Atwood, 1999). Although the Western world certainly has its equivalent forms of armed and unarmed combat, such as wrestling, boxing, and fencing (e.g., see Styers, 1974), they are not typically thought of as belonging to the Asian forms of fighting known as the martial arts.

It is estimated that as many as 18 million people in America alone practice some form of the martial arts (Nathan, 2005), and martial arts films have proven very popular. From Bruce Lee to Jackie Chan, and more recently Chow Yun Fat (star of Crouching Tiger, Hidden Dragon, which won four Academy Awards [Lee, Ling, Schamus, & Jung, 2000]), we have seen examples of the classic good-guy, an honorable individual defending those who are abused by others. The famous American martial artist and movie star Chuck Norris, in cooperation with former President George H. W. Bush, has established a national program called KICKSTART to introduce martial arts to “at risk” middle school students to raise their self-esteem (Nathan, 2005). Although the martial arts are often seen as an opportunity for athletic young men to engage in disciplined and/or ritualized combat, there are also programs for children of all ages, general physical conditioning, and people with disabilities (Chaline, 2003; Johnson, 2003b; McNab, 2003). There is also a rich history of women practicing the martial arts (Atkinson, 1983; Chaline, 2003b). Indeed, Bruce Lee first studied the Wing Chun style of Kung Fu, a style developed some 400 years ago by a Buddhist nun named Ng Mui and her student Yim Wing Chun, who was also a nun. It was later that Lee developed his own technique, known as Jeet Kune Do or “the way of the intercepting fist” (Lee, 1975; see also Johnson, 2003a; Lewis, 1993; Little, 1998; Ribner & Chin, 1978).

What sets the martial arts apart is the balanced approach to both physical exercise and spiritual/mental discipline. Although the martial arts certainly existed farther back in ancient times, it is accepted by many that they were first formalized in the **Shao-Lin temple** by the founder of Zen Buddhism, Bodhidharma. When Bodhidharma first arrived at the Shao-Lin temple in China, after leaving his home in India, he found the monks in very poor physical condition. He developed a series of eighteen exercises that helped the monks to achieve a good level of physical fitness, something necessary for their self-defense as well as for extended periods of sitting in meditation (Johnson, 2003a; Lewis, 1993; Red Pine, 1987; Ribner & Chin, 1978). These exercises established the first formal practice of Kung Fu. It is important to note the role of Bodhidharma, a highly spiritual monk who had left his home to help spread the teachings of the Buddha. Since one of the basic tenets of Buddhism is to not harm any other living being, the martial arts have always emphasized mental discipline and the intention that the fighting skills should only be used in self-defense or in the defense of others who cannot defend themselves. Non-combative forms of the martial arts have

developed around the concept of mindfulness of the body, which can be used as forms of moving meditation. Examples of such forms are Tai Chi Chuan and Qigong (Johnson, 2003a,c; Lewis, 1993; Khor, 1981; Ribner & Chin, 1978). There are also more traditional forms of martial arts, such as **Aikido** and **Hapkido** (Hapkido being “the way of harmony”; Chesterman, 2003), which emphasize the soft style of defending oneself that is advocated in the Tao Te Ching (Lao Tsu, c600 B.C./1989). As with the regular martial arts, these soft, meditative, defensive martial arts originated in the countries with the strongest histories in the more aggressive forms: Tai Chi Chuan and Qigong originated in China, Aikido in Japan, and Hapkido in Korea.

Samuel demonstrating an excellent side kick on the way to earning his black belt in Taekwondo.

When the martial arts are approached properly, as a means to health, strength, and a calm state of mind, we can refer to the practice as the **martial Way**, a means to living one’s life in a virtuous manner (Chu, 2003). Because martial arts training can prepare one to injure others it must be approached with the right attitude:

This concept of power as the cornerstone of personal freedom lies at the bottom of all martial arts philosophy. The recognition that power emanates from physical force and martial capability cuts both ways; it can be channeled toward constructive uses or abused as a means of destruction. This is the reason why martial arts training must always be directed toward the cultivation of the higher ideals of discipline, humility, benevolence and responsibility. (pg. 29; Chu, 2003)

Continuing to emphasize the role that the martial arts can play in helping people to live a more satisfying life, Chu goes on to say:

The demands of work, family, finances, as well as fatigue, neglect and health all distract the martial artist from his best intentions. Even the devoted student may be disappointed if he expects martial arts training to neatly bring his physical and spiritual condition into working order. Nevertheless, regular training can serve as a constant, to discipline him to develop his best self even as the daily routine pulls him in different directions. The strategies underlying training can be effectively applied not just in life threatening situations but to daily life. (pgs. 44-45; Chu, 2003)

In order to help martial artists pursue and maintain this virtuous Way, various codes and tenets have been devised. My family practices Taekwondo, so we have been taught to follow the **five tenets of Taekwondo**: courtesy, integrity, perseverance, self-control, and indomitable spirit. These principles were set forth by General Choi Hong Hi, who re-established the modern forms of Taekwondo when Korea regained its independence after World War II. He believed that if Taekwondo students lived their lives according to these principles they would become better people and help to make the world a better place (Chesterman, 2003; Lewis, 1993). Perhaps the most famous of the martial arts codes is the **Bushido code** of the Japanese **Samurai**. It can sometimes be difficult to translate Asian languages into English, but generally the Bushido code contains seven essential principles: making right decisions, bravery, compassion, taking right actions, honesty, honor, and loyalty. Although these principles seem to include states of mind, or conscious intentions, it is through the physical practice, through the body and the unconscious mind, that Bushido becomes a way of life (Deshimaru, 1982). Only after many years of practice does this become

a natural way of life, without the need for continued attention to one's practice. Then many more years of practice are necessary before one finally becomes a true master. The consciousness, or mindfulness, necessary for this combined practice of body and mind can be found in Zen Buddhism, which is closely intertwined with **Budo**, the Japanese way of the warrior (Deshimaru, 1982). As with the tenets of Taekwondo, the principles of the Bushido code helped warriors to restrain themselves from violent aggression in their daily lives (Chu, 2003). According to Chu (2003), it is the higher ideals of spirituality in codes like Bushido and the tenets of Taekwondo that separates the warrior from the predator.

Despite having emphasized the balance between physical and spiritual aspects of the martial arts, we must still consider that they can play a most important role in self-defense. It is an unfortunate reality that there are many people in this world who don't follow virtuous principles such as the tenets of Taekwondo or the Bushido code. I am fortunate to know a martial artist and special education teacher named David Schied, who wrote a most interesting book combining martial arts, Eastern philosophy, and basic techniques of self-defense in all aspects of one's life (Schied, 1986). Many people live timid lives, some live in outright fear. It has been suggested that as many as 160,000 children miss school every day out of fear that they will be bullied by other students (Nathan, 2005). This fear can seriously disrupt our ability to function in our daily world:

Most of us don't see ourselves as unified human beings (people who can call forth all our resources and use our total capabilities at will). We tend not to give our all to the situation at hand (even when nothing less will do). Instead of giving our best we give "enough" which rarely is enough. Left to our own means most of us respond to life's demands in a fragmented fashion. Instead of reacting to the challenges of everyday life by focusing and directing our energies to the task at hand, we respond haphazardly and incompletely. (pg. iv; Schied, 1986)

By studying the martial arts and other techniques of self-defense and security, and by learning strategies to become aware of and deal with our emotional responses to danger, we can not only resolve our fears in those dangerous situations, but we can also remain calm and in control of other aspects or our lives (Schied, 1986). In addition to simply preparing for danger by learning how to avoid it or how to fight when one can't avoid it, the age-old Eastern techniques of meditation and mindfulness can help to calm one's nerves before, during, and after facing a crisis. As peace of mind becomes your usual emotional state, you become more open to living your own life and enjoying your relationships with others in a loving and compassionate way. Then, when faced with danger from another:

The opening of yourself to life with love will enable you not only to take proper measures for the extension of your life (surviving) in an attack, but also to greet your attacker as everyone else you meet - with an open hand and an open mind. You will begin to see an infinite number of ways to share love with those who mean you bodily harm. By giving to another that love which is so plentiful within you, you will help instead of hurt him. This is the ultimate self-defense. (pg. 208; Schied, 1986)

Discussion Question: The martial Way refers to integrating the physical, spiritual, and mental aspects of the martial arts into your daily life. Have you ever practiced a martial art? If yes, how seriously did you take the spiritual philosophy of your particular martial art or the martial arts in general?

Final Note

This chapter may seem to have an odd assortment of topics, but if we look closely we can better understand the meaningful connections. We have covered: 2,500 year-old Buddhist teachings on mindfulness, 1,500 year-old techniques in the martial arts, 100-year-old Victorian era theories on somatic psychology, recent theories in sociobiology and evolutionary psychology, and cutting-edge brain imaging of real-time changes in mental states! How can all of this possibly fit together? The answer is actually quite simple. For as long as people have been able to wonder, they have wondered about the two things in our lives that cannot be denied: the fact that we have a body, and the fact that we are aware of it. Conscious awareness of our own existence, and the body that is the physical manifestation of that existence, is something that all people, all races and all nationalities, have shared throughout time. These topics are the most thoroughly cross-cultural topics in psychology.

Finally, when Bruce Lee's famous movie Enter the Dragon was first released in the United States, they cut one of the opening scenes in which he is tested by his master on his understanding of fundamental Buddhist/Daoist philosophy as it pertains to the martial arts. In the twenty-fifth anniversary special edition, Warner Brothers studios apparently decided that American audiences would now appreciate this dialogue, so they put the scene back into the movie (Clouse & Allin, 1998).

Personality Theory in Real Life: An Evolutionary Perspective on the Development of Pathological Personality Patterns

In the Appendix, there is a brief presentation of Theodore Millon's alternative classification scheme for personality disorders, as compared to the DSM-IV (Millon, 1996; Millon & Grossman, 2005). This perspective is based on Millon's belief that personality disorders represent patterns of thought and behavior that are adaptive, albeit under abnormal conditions, and therefore have been selected for through the process of evolution. Millon believes that it is necessary for psychology to draw upon related fields of science in order to strengthen the entire discipline:

Much of psychology as a whole remains adrift, divorced from broader spheres of scientific knowledge, isolated from deeper and more fundamental, if not universal, principles…we have failed to draw on the rich possibilities that may be found in both historic and adjacent realms of scholarly pursuit. (pg. 333; Millon & Grossman, 2005)

Millon and Grossman acknowledge the contributions of sociobiology to our understanding of human behavior, and they offer a sociobiological perspective on personality. Personality, they argue, can be thought of as the distinctive style of adaptive functioning that an individual exhibits as they relate to their typical range of environments. Personality development is healthy when the individual encounters average or relatively normal environments and is effective in adapting to them. Personality disorders arise when the individual relies on maladaptive functioning that can be traced to psychic deficiencies, trait imbalances, or internal conflicts that occur when relating to their environment. In other words, when individuals adapt to abnormal environments (e.g., an abusive home), their personality style may then prove to be maladaptive in situations outside of their typical environment.

Millon proposes that every person, indeed every organism, must accomplish four basic goals, each of which has two polarities: they must exist (seek pleasure and avoid pain), they must adapt (respond actively or remain passive), they must reproduce (focus on self or focus on others), and they must deal with unexpected or abstract situations (rely on thinking or react to feelings). These four demands correspond to four neurodevelopmental stages: sensory attachment associated with life enhancement (seeking pleasure) or life preservation (avoiding pain), sensorimotor autonomy associated with modifying the environment (active) or accommodating to the environment (passive), pubertal genital identity associated with propagating oneself (self-oriented) or nurturing children (other-oriented), and finally intracortical integration associated with intellect (thinking-oriented) or emotion (feeling-oriented). It should be clear that these stages cover the range of development from birth to young adulthood. In contrast to theories that focus on critical points in development as key times when psychological problems occur:

…the quality or kind of stimulation the youngster experiences is often of greater importance. The impact of parental harshness or inconsistency, of sibling rivalry or social failure, is more than a matter of stimulus volume and timing. Different dimensions of experience take precedence as the meaning conveyed by the source of stimulation becomes clear to the growing child. (pg. 361; Millon & Grossman, 2005)

While it is difficult to clearly define what constitutes normal vs. abnormal personalities, in simple terms individuals who are relatively normal are able to shift between and balance the demands of each of these polarities as appropriate to the situations they encounter. When examining individuals with pathological personality patterns (a term preferred by Millon & Grossman, since personality disorder implies a medical condition that might be cured), their behavioral constraints arise primarily from within themselves, due to the abnormal conditions in which they developed. The traits associated with these abnormal personality patterns take on an inner momentum and autonomy, so they are expressed regardless of the external situation. In other words, individuals with pathological personality patterns are not able to appropriately adapt their behaviors to different situations in which they find themselves.

The following is a description of one abnormal personality type, the self-defeating (masochistic) personality:

This disorder stems largely from a reversal of the pain-pleasure polarity. These persons interpret events and engage in relationships in a manner that is not only at variance with this deeply rooted polarity but is contrary to the associations these life-promoting emotions usually acquire through learning. To the self-defeating personality, pain may be a preferred experience, tolerantly accepted if not encouraged in intimate relationships. It is often intensified by purposeful self-denial, and blame acceptance may be aggravated by acts that engender difficulties as well as by thoughts that exaggerate past misfortunes and anticipate future ones. (pp. 376-377; Millon & Grossman, 2005)

As strange as this condition seems, how might it arise? How does a person develop an adaptive strategy that seeks pain, and how can such a strategy actually be adaptive? Imagine an abused child, whose only source of love is the parents who abuse them! In such a terrible situation, the best strategy for the child to adapt might be to reverse the pleasure-pain polarity. This is clearly an extreme response, but one that might make the child’s world easier to endure and less likely to create further abuse. However, when the

child grows up and moves on to other relationships, the deeply embedded characteristics of this pathological personality pattern make healthy relationships all but impossible. An important point to make here is that the term adaptive is not always synonymous with our ideas of good psychological health. Adaptation can only be considered within its particular context. And that is exactly the consideration that Millon proposes in his evolutionary perspective on personality development. Each person develops adaptively to their own environment, and as a result they establish persistent characteristics (personality patterns) along the polarities of pain-pleasure, active-passive, self-other, and thinking-feeling. Whether these adaptations are healthy or unhealthy is a matter to be determined after the fact. However, by recognizing how they develop, as psychologists we can attempt to educate others on how to avoid causing these conditions.

CHAPTER 14 / CAC.902

Gestalt and Experiential Therapy

Gestalt therapy, associated with Fritz Perls, Laura Perls, and Paul Goodman, emphasizes present-moment awareness, the whole person, and the contact between organism and environment. It is experiential rather than purely explanatory. The counselor does not only talk about experience. The counselor helps the client experience differently in the room. Emotion-focused therapy and other experiential approaches share an interest in accessing and transforming emotion, though they have different histories and methods.

Awareness and the here and now

Gestalt asks what is happening now: What do you notice in your body? What do you avoid? What do you want? The past is relevant when it is alive in the present. A client may talk about a critical parent while tightening their jaw. The counselor might say, "Notice your jaw as you say that." The goal is not to produce catharsis for its own sake. The goal is to increase awareness so that choice becomes possible.

Contact and boundary disturbances

Contact is the meeting between self and environment. Healthy contact allows connection and separation. Gestalt describes boundary disturbances such as introjection, projection, retroflection, deflection, and confluence. Introjection is swallowing beliefs without chewing. Projection is attributing one's own experience to others. Retroflection is doing to oneself what one wants to do to others. Deflection is avoiding contact through humor or abstraction. Confluence is losing the boundary between self and other. These concepts should be used as hypotheses, not labels.

Experiments and chair work

Gestalt uses experiments to expand awareness. A client may speak to an empty chair representing a parent, then switch chairs and answer. The experiment is not a performance. It is a way to access blocked feelings and perspectives. With children, experiments may be playful: drawing the worry, giving it a voice, or using puppets. The counselor must ensure safety. Intense chair work may be inappropriate for clients with psychosis, severe dissociation, or low stability unless carefully adapted.

Emotion-focused therapy

Emotion-focused therapy, developed by Les Greenberg and others, views emotion as a source of meaning and a target of change. It distinguishes primary adaptive emotions, primary maladaptive emotions, and secondary emotions. A child may feel secondary anger that covers primary sadness. The counselor helps the client access the primary emotion and transform it with a new experience. EFT for individuals and EFT for couples are different approaches; the counselor should not confuse them.

Clinical example

Fourteen-year-old Diego says he does not care that his father left. He smiles while describing the departure. A Gestalt counselor might notice the smile and ask, "What happens if you let your face say what your words do not?" Diego may become angry, then sad. The counselor stays present and does not rush to explain. Over time, Diego learns that sadness does not destroy him. The experiment is not forced emotion. It is an invitation to notice what is already present.

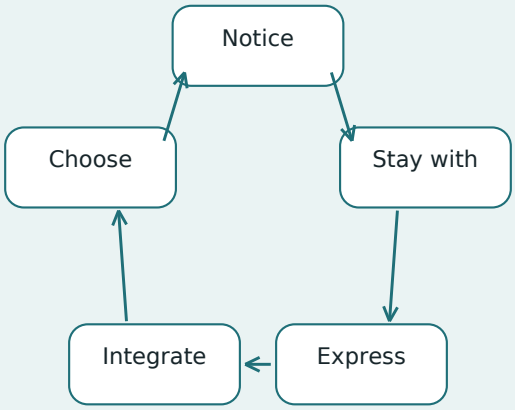
Evidence and limits

Experiential therapies have evidence for some conditions, and emotion-focused therapy has a growing research base. Gestalt's historical style, especially confrontational demonstrations, can be misused. Modern relational Gestalt is more collaborative. Counselors should not use experiential techniques to bypass assessment, safety, or cultural humility. A client from a tradition that values emotional restraint may experience chair work as invasive. The counselor should explain the rationale and obtain consent.

Reflection

Choose a sentence you often say about a difficulty. Say it aloud and notice your body. Then try changing one word or adding a feeling word. Notice what shifts. Experiential work begins with curiosity about lived experience, not with a technique imposed from outside.

Awareness experiment



Experiential work uses present-moment experiments to expand awareness and choice.

CHAPTER 15 / CAC.902

Family Systems and Postmodern Conversations

Family systems theory views the child not as an isolated problem but as a member of a relational system. Symptoms may serve a function in the family, express loyalty, or reflect stress in the system. This does not mean the family is to blame. It means that change in one part affects other parts. Postmodern approaches, including solution-focused and narrative therapy, shift attention from pathology to language, meaning, and preferred stories. They ask how problems are constructed and what exceptions or alternative stories already exist.

Systems concepts

A system has boundaries, roles, rules, and feedback loops. Homeostasis is the tendency to maintain stability. A child's symptom may stabilize a family by drawing attention away from marital conflict. Triangulation occurs when a third person is drawn into a two-person tension. A parent may confide in a child about the other parent, placing the child in a loyalty conflict. The counselor maps patterns rather than blaming individuals.

Structural and strategic family therapy

Structural family therapy, associated with Salvador Minuchin, examines hierarchy, boundaries, and subsystems. A healthy family has clear boundaries and an executive subsystem. Enmeshment means diffuse boundaries; disengagement means rigid boundaries. Strategic family therapy, associated with Jay Haley and others, designs tasks to interrupt problem-maintaining sequences. These approaches can be powerful but require cultural caution. Family structures vary across cultures. A multigenerational household is not automatically enmeshed.

Solution-focused brief therapy

Solution-focused therapy asks about exceptions, preferred futures, and existing resources. The miracle question invites the client to imagine waking up and the problem being solved: What would be different? Scaling questions ask the client to rate progress and identify what keeps the score from being lower. The approach is brief and future-oriented. It does not ignore problems, but it does not assume that deep historical analysis is always necessary.

Narrative therapy

Narrative therapy, associated with Michael White and David Epston, separates the person from the problem. A child is not "an anxious child"; the child is a person whom Anxiety visits. Externalizing conversations explore the problem's effects and the client's influence on the problem. Unique outcomes are moments when the problem did not dominate. Re-authoring builds a preferred story. The counselor asks questions that help the client become the author of their life.

Clinical example

Ten-year-old Emma has stomachaches before school. A systems formulation notes that the parents argue about how to respond. The mother accommodates; the father insists. Emma's symptom draws them into a shared task, temporarily reducing marital tension. The counselor does not tell Emma she is manipulating. The counselor works with the parents to create a consistent plan and addresses the co-parenting conflict. Narrative questions help Emma name Worry and recall times she was brave. Solution-focused scaling tracks small steps.

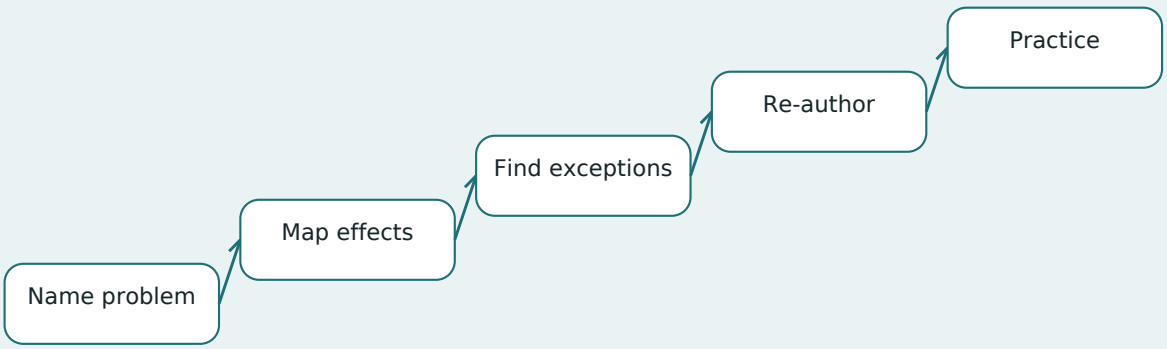
Evidence and limits

Family therapy has evidence for various child and adolescent concerns, especially when caregivers are involved. Solution-focused and narrative approaches have growing evidence but may be less studied for severe disorders. A limitation of systems language is that it can minimize abuse or oppression. A child should not be asked to accommodate violence for the sake of system balance. Safety and protection come first. Cultural humility is essential because family roles and communication styles vary.

Reflection

Draw a simple map of a family or group you know. Mark alliances, conflicts, and roles. Ask what problem might be serving a function. Then ask what would change if the problem improved. Systems thinking is not about blame. It is about seeing the dance.

From problem story to preferred story



Narrative and solution-focused work builds alternative stories from existing strengths.

Attributed Foundational Reading

Attribution and critical-reading note

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Read historical and theoretical claims critically. A theory is a lens for inquiry, not a verdict about a child or family. Where source claims are dated or culture-bound, the new lecture supplies cautions and contemporary framing.

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Family Therapy

Family therapy is a special form of group therapy, consisting of one or more families. Although there are many theoretical orientations in family therapy, one of the most predominant is the systems approach. The family is viewed as an organized system, and each individual within the family is a contributing member who creates and maintains processes within the system that shape behavior (Minuchin, 1985). Each member of the family influences and is influenced by the others. The goal of this approach is to enhance the growth of each family member as well as that of the family as a whole.

Often, dysfunctional patterns of communication that develop between family members can lead to conflict. A family with this dynamic might wish to attend therapy together rather than individually. In many cases, one member of the family has problems that detrimentally affect everyone. For example, a mother’s depression, teen daughter’s eating disorder, or father’s alcohol dependence could affect all members of the family. The therapist would work with all members of the family to help them cope with the issue, and to encourage resolution and growth in the case of the individual family member with the problem.

With family therapy, the nuclear family (i.e., parents and children) or the nuclear family plus whoever lives in the household (e.g., grandparent) come into treatment. Family therapists work with the whole family unit to heal the family. There are several different types of family therapy. In **structural family therapy**, the therapist examines and discusses the boundaries and structure of the family: who makes the rules, who sleeps in the bed with whom, how decisions are made, and what are the boundaries within the family. In some families, the parents do not work together to make rules, or one parent may undermine the other, leading the children to act out. The therapist helps them resolve these issues and learn to communicate more effectively.

Note: Watch this video of a structural family session to learn more.

In **strategic family therapy**, the goal is to address specific problems within the family that can be dealt with in a relatively short amount of time. Typically, the therapist would guide what happens in the therapy session and design a detailed approach to resolving each member’s problem (Madanes, 1991).

CHAPTER 16 / CAC.902

Play, Expressive Work, and Developmental Adaptation

Children do not usually sit on a couch and free associate. They play, draw, build, move, and tell stories. A counselor who works with children must adapt theory to the child's developmental language. Play is not a break from therapy. For many children, play is the therapy. Expressive work can make inner experience visible when words are not yet available. The counselor's task is to provide a safe frame, follow the child's meaning, and use theory to guide attention without taking over the child's expression.

Why play matters

Play allows a child to practice mastery, express conflict, and experiment with roles. A child who has experienced hospitalization may give shots to a doll. A child who has witnessed violence may build and destroy cities. The play is not a literal report. It is a symbolic and emotional process. The counselor should not interpret every action. The counselor tracks themes, affect, and shifts. A child who moves from chaotic destruction to organized repair may be showing a change in capacity.

Nondirective and directive play

Nondirective play therapy, rooted in person-centered and psychodynamic traditions, allows the child to lead. The counselor reflects feeling, sets limits, and returns responsibility. Directive play therapy uses structured activities to target specific goals, such as cognitive-behavioral games for anxiety or trauma narratives. The choice depends on the child, the problem, and the evidence. A child with severe anxiety may need a gradual, structured approach. A child with relational trauma may need nondirective space to test safety. Many counselors integrate both.

Expressive arts

Drawing, music, movement, and sandtray can access experience that is hard to verbalize. The counselor should not act as an art critic. The question is not whether the drawing is good. The question is what the child experiences while making it and what meaning the child gives it. A black scribble may be anger, may be a favorite color, or may be nothing. The counselor asks, "Tell me about this," rather than, "This means you are depressed."

Developmental adaptation of theories

CBT for young children may use puppets to externalize worry. Psychodynamic work may use play sequences rather than free association. Family therapy may use drawing to map relationships. Existential work may use stories about courage and loss. The counselor should match the method to the child's cognitive, language, and emotional development. A seven-year-old may not be able to identify automatic thoughts in adult language. The child may identify the "worry monster" and draw what it says.

Caregiver involvement

Children's therapy usually requires caregiver work. The caregiver may need coaching, support, or their own treatment. A child's play may improve in session but fail at home if the environment is unchanged. The counselor should explain the plan in simple terms and protect the child's privacy. A child's drawing should not be shown to a parent without consideration of safety and consent. The counselor can share themes and progress without exposing every detail.

Clinical example

Six-year-old Amina, introduced earlier, plays a game in which a baby animal is left at a school door. The counselor reflects, "The baby is scared to go in." Amina makes the mother animal stay longer. Over sessions, the mother animal begins to leave for short times and returns. The play mirrors separation and reunion. The counselor coordinates with the parent and teacher to create a brief, predictable goodbye ritual. The play and the real-life plan reinforce each other.

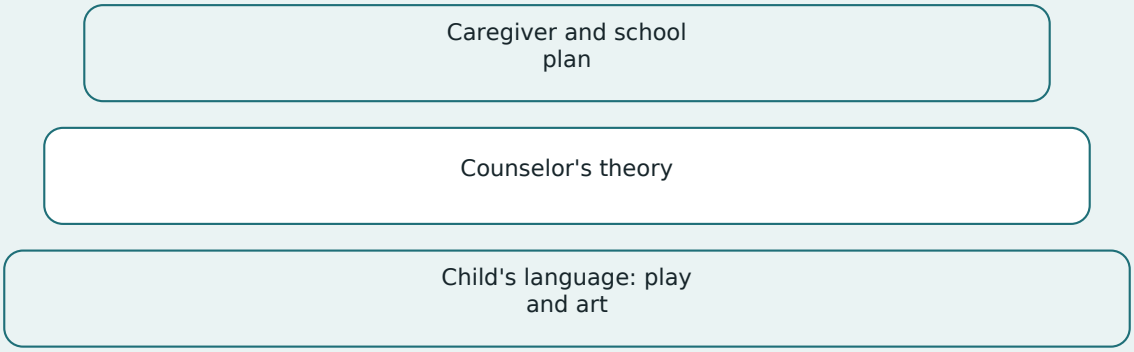
Evidence and ethics

Play therapy has evidence for various emotional and behavioral concerns, though the strength varies by approach and problem. Expressive methods should not replace evidence-based treatment when a specific disorder requires it. Counselors should avoid suggestive questioning, especially in forensic contexts. A child's play is not proof of abuse. The counselor documents observations, not interpretations presented as facts.

Reflection

Observe a child's play from a distance, with permission and without interrupting. Write what you see, not what you think it means. Then write two possible meanings. Notice how the second column is hypothesis. Child counseling requires the discipline of not knowing too quickly.

Adapting theory to development



Child counseling translates theory into the child's developmental language and daily systems.

Attributed Foundational Reading

Attribution and critical-reading note

Adapted from Psychology 2e by Rose M. Spielman, William J. Jenkins, and Marilyn D. Lovett, OpenStax, Rice University (2020), CC BY 4.0. Selection, organization, and PDF layout modified for this course; new lectures and course diagrams are separate additions.

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Psychotherapy: Play Therapy

Play therapy is often used with children since they are not likely to sit on a couch and recall their dreams or engage in traditional talk therapy. This technique uses a therapeutic process of play to “help clients prevent or resolve psychosocial difficulties and achieve optimal growth” (O’Connor, 2000, p. 7). The idea is that children play out their hopes, fantasies, and traumas while using dolls, stuffed animals, and sandbox figurines (). Play therapy can also be used to help a therapist make a diagnosis. The therapist observes how the child interacts with toys (e.g., dolls, animals, and home settings) in an effort to understand the roots of the child’s disturbed behavior. Play therapy can be nondirective or directive. In nondirective play therapy, children are encouraged to work through their problems by playing freely while the therapist observes (LeBlanc & Ritchie, 2001). In directive play therapy, the therapist provides more structure and guidance in the play session by suggesting topics, asking questions, and even playing with the child (Harter, 1977).

Source figure caption: This type of play therapy is known as sandplay or sandtray therapy. Children can set up a three-dimensional world using various figures and objects that correspond to their inner state (Kalff, 1991). (credit: Kristina Walter)

CHAPTER 17 / CAC.902

Culture, African Perspectives, and Feminist Critique

Every theory is born in a culture. The major counseling theories developed largely in Europe and North America. They carry assumptions about self, family, emotion, and health. A counselor who treats these assumptions as universal may misread clients. Cultural humility is not a chapter added at the end. It is a stance that asks whose theory is being used, whose experience is centered, and what has been left out. African perspectives and feminist critiques are not decorations. They are corrections to a field that has often mistaken one cultural standpoint for human nature.

The limits of universal claims

Many theories assume an individual self that seeks autonomy. In many African and other collectivist traditions, the self is relational. A person is a person through other people. Distress may be experienced as a disruption of relationship or community, not only as an internal symptom. A counselor should not force a client into an individualistic frame. The question "What do you want?" may be less useful than "What would help your family?" or "Who should be part of this conversation?"

This does not mean that all African clients are collectivist or that all Western clients are individualist. Cultures are diverse and changing. The counselor asks the client what matters. A Nigerian adolescent in Lagos may value both personal ambition and family obligation. A counselor should not stereotype. Cultural formulation is a conversation, not a checklist.

African perspectives on personality and distress

African psychology emphasizes community, ancestry, spirituality, and the interconnectedness of life. Distress may be understood through social, spiritual, and physical dimensions together. A counselor should respect these meanings without abandoning clinical responsibility. A client may believe that a problem has spiritual causes. The counselor can explore the meaning and collaborate with trusted community or religious supports when appropriate, while still assessing safety and evidence-based treatment needs. The goal is not to replace one dogma with another. The goal is to work within the client's worldview.

Feminist critique

Feminist therapy critiques power, gender roles, and the pathologizing of women's responses to oppression. It argues that the personal is political. A woman's anxiety may be shaped by sexism, violence, or economic dependence. A girl's anger may be labeled oppositional when it is a response to unfairness. Feminist therapy emphasizes collaboration, empowerment, and attention to power in the therapy relationship. The counselor does not pretend to be neutral when power is unequal.

Feminist critique also applies to boys and men. A boy who is shamed for crying may learn to express distress through anger. Gender-diverse youth may face rejection that is mislabeled as pathology. The counselor should ask how gender expectations shape the client's experience. The goal is not to impose a political view but to notice how power operates.

Cultural formulation in practice

A cultural formulation asks: How does the client understand the problem? What language do they use? Who do they trust? What treatments have they tried? What role do family, religion, and community play? What barriers exist? The counselor should ask about discrimination and migration. A child who has moved countries may grieve lost relationships and face language stress. A counselor should not assume that silence means resistance.

Clinical example

Fourteen-year-old Zainab is referred for "defiance" after refusing to attend an after-school program. Her parents want her to focus on study. Zainab wants to care for a younger sibling because the mother works nights. A Western autonomy frame might call the parents controlling. A cultural formulation reveals that Zainab's role is valued in the family and that her refusal is a negotiation of responsibility. The counselor helps the family find a plan that honors Zainab's contribution and her education. The problem is not defiance alone. It is a mismatch of expectations and resources.

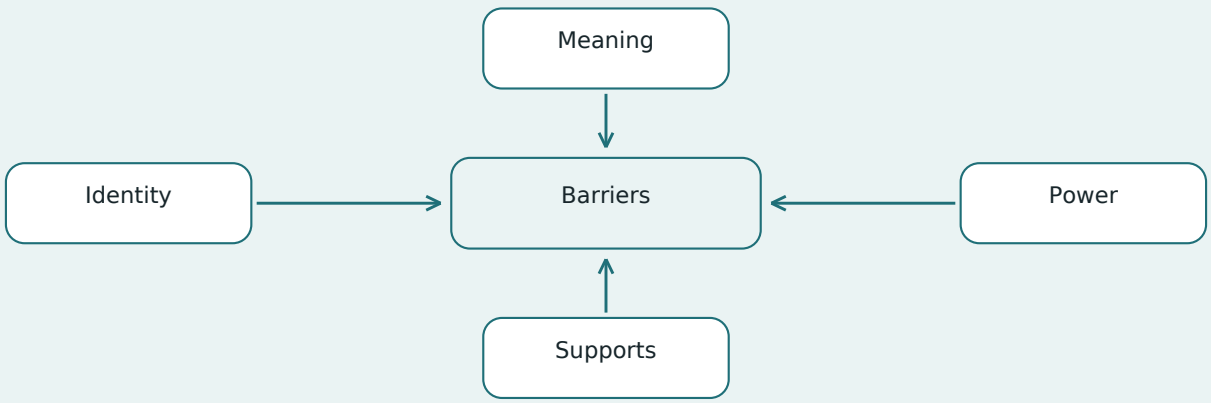
Integrating culture with evidence

Cultural adaptation of evidence-based treatment can improve engagement. The counselor may change language, examples, metaphors, and family roles while keeping core mechanisms. For example, exposure for anxiety may involve a trusted aunt rather than a parent if that is safer. Cognitive work may use proverbs or stories that fit the client's tradition. The counselor should not assume that evidence-based means culture-free. Evidence is gathered in contexts, and contexts matter.

Reflection

Write your own cultural assumptions about a good family, a healthy emotion, and a successful life. Then ask how a client from a different background might answer. Cultural humility begins with the recognition that your normal is not universal.

Cultural formulation questions



Culture is not an add-on. It shapes how distress is understood and help is sought.

Attributed Foundational Reading

Attribution and critical-reading note

Adapted from Personality Theory in a Cultural Context by Mark D. Kelland, OpenStax CNX / Lansing Community College (2015), CC BY 4.0. Selection, organization, and PDF layout modified for this course; new lectures and course diagrams are separate additions.

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In the first chapter we briefly examined the concern of many psychologists that the field of psychology has been slow to embrace the value of cross-cultural research (see Lee et al., 1999; Sue, 1999; Triandis & Suh, 2002). This concern is by no means new. In 1936, Ralph Linton wrote that “different societies seem to show differences in the relative frequency of occurrence of the various psychological types” (pg. 484), and in 1973, Robert LeVine suggested that “this is a moment at which even those who are skeptical about the value of culture and personality study might consider stretching their curiosity in this direction” (pg. ix). Throughout this textbook we will examine a number of theorists who emphasized studying cultural differences as a significant part of their careers and, often, their personality theories as well.

However, it remains true that cross-cultural studies in psychology have only recently moved closer to the mainstream of psychological research and clinical practice. As of 2002, the American Psychological Association has “Guidelines on Multicultural Education, Training, Research, Practice, and Organizational Change for Psychologists” (www.apa.org/pi/multiculturalguidelines/homepage.html). To cite just a few examples of the range of current interest in cross-cultural psychology, we now have a Dictionary of Multicultural Psychology (Hall, 2005) and books on the relationships between culture, mental illness, and counseling (Axelson, 1999; Castillo, 1997), as well as on the relationships between race, class, and the social and personal development of women (Jordan, 1997b; Pack-Brown, Whittington-Clark, & Parker, 1998). There are also major new texts on African American psychology (Belgrave & Allison, 2006) and racism, prejudice, and discrimination in America (Miller & Garran, 2008; Whitley & Kite, 2006).

The fact that studying cross-cultural factors in personality has always been present in the careers and theories of certain individuals, while not becoming a mainstream focus of attention, is more than just an historical curiosity. By emphasizing biological factors (i.e., genetics), Freud’s theory did not allow for cultural differences. Behavioral theorists emphasized environmental factors, a seemingly cultural approach, but they did not allow themselves to address factors beyond immediate scientific control. Thus, they defined with great precision the role of reinforcement, punishment, discriminative stimuli, etc., while not allowing for the richness of cognition and cultural experiences. Likewise, cognitive theorists clung to the scientific approach of the behaviorists, rather than embracing the potential of sociocultural perspectives. In other words, because strict Freudian theorists, as well as behavioral and cognitive theorists, believed that their theories applied to all people equally,

they typically chose not to address differences between people. Thus, those who wished to bring sociocultural perspectives on the development of personality into the field of personality theory faced a degree of direct opposition. And yet, their perseverance is now being fulfilled.

In this chapter, we will briefly examine some of the issues facing personality psychologists who wish to examine personality development in a sociocultural context. The United States, Canada, and Western Europe represent only about one tenth of the world's population. Ralph Linton, a renowned anthropologist with an interest in cultural influences on personality (see Linton, 1945), also edited a book entitled Most of the World: The Peoples of Africa, Latin America, and the East Today (Linton, 1949). Thus, it is essential that we consider the influence of different cultures around the world if we are going to claim that we have really examined human personality in all its variations.

Cultural Studies in the Field of Psychology

Since the 1990s, a number of general books on psychology and culture have been available (e.g., Brislin, 2000; Lonner & Malpass, 1994; Matsumoto, 1994, 1997; Matsumoto & Juang, 2004; Okun, Fried, & Okun, 1999; Price & Crapo, 2002; Segall et al., 1990). Although all of these books address topics such as the "self" and person-perception, and other various aspects of personality, only a few of them devote an actual chapter or section to the topic of personality itself (Matsumoto & Juang, 2004; Price & Crapo, 2002; Segall et al., 1990), and in each case the topics are fairly specific. There is, however, some older literature on the relationships between culture, society, and personality. We will examine that research in the second part of this section. First, let us examine some of the general principles of incorporating cross-cultural perspectives into the study of personality.

The Challenges of Cultural Research

The first problem faced by those who are interested in the study of culture and personality is the question: what exactly is to be studied? At the most basic level, there are two types of research. **Cross-cultural research** typically refers to either parallel studies being conducted in different cultures, or similar concepts being studied in different cultures. In contrast, **intercultural research** is the study of individuals of different cultures interacting with one another (Brislin, 2000; Matsumoto & Juang, 2004; Segall et al., 1990). As you will see in later chapters, some personality theorists consider interpersonal relationships to be the only true domain for studying individual personality. While most of the research done in psychology has been cross-cultural, as the world becomes more and more of a global community the opportunity for, and importance of, intercultural research is rapidly expanding.

Another fundamental problem with the study of culture is our attention to it, or rather, the lack of attention we pay to something that is so deeply ingrained in our daily lives. Richard Brislin suggests the following exercise: write down three answers for someone from a different culture who asks "What should I know about your culture so that we can understand each other better?" (pg. 10; Brislin, 2000). Because we simply take our cultural influences for granted, it proves quite difficult for us to think that they need to be identified or explained. For example, freedom of speech is a cherished right in America. Consequently, we often speak our minds. If I am upset about some new college policy, I might say very negative things about the administration of our college, even about particular

administrators. It does not mean I intend to be disrespectful, or that I dislike those individuals, or that I won't say positive things about them when I agree with the next new policy. It is simply an expression of one of the great freedoms in our society: the right to speak out. However, someone from a different culture, particularly a collectivist culture, might be shocked at my apparent disrespect toward my "superiors."

The next important issue is the difference between **emic** and **etic** tasks or behaviors. Simply put, emic tasks are those that are familiar to the members of a given culture, whereas etic tasks are common to all cultures. In an elegantly simple, yet revealing study, Irwin, Schafer, & Feiden (1974) demonstrated these phenomena in two cultures: American undergraduates and Mano rice farmers (from Liberia). The American college students were consistently better at performing the Wisconsin Card Sort, a well-known psychological test measuring cognitive reasoning skills, which relies on geometric shapes and color. The Mano farmers, however, were consistently better at sorting different categories of rice. Thus, the ability to sort items into categories appears to be an etic task (most likely common to all humans, regardless of culture), whereas the more specific abilities to sort by geometry and color (common to American college students) or type of rice grain (common to Mano farmers in Liberia) is an emic task that requires familiarity. Thus, if we made a judgment about the Mano farmers' cognitive abilities based on the Wisconsin Card Sort, we would clearly be making a mistake in comparing them to Americans, due to the unfamiliarity of the particular task.

Another important aspect of cross-cultural research, which may involve applying our understanding of etics and emics, is the issue of **equivalence**. Is a concept being studied actually equivalent in different cultures? In other words, does a concept mean the same thing in different cultures, is the comparison valid? For example, an etic related to intelligence is the ability to solve problems. So how might we compare different cultural groups? Would the speed with which they solve a problem make sense as a measure of intelligence? Such an answer would be emic, and therefore valid, in America (where we typically value independence and competition). However, among the Baganda of Uganda, slow and careful thought is the emic. Among the Chi-Chewa of Zambia, the emic is responsibility to the community, i.e., solving the problem in order to best get along with other people. Thus, the speed at which people solve problems is conceptually equivalent, since it is the way in which people in each culture identify those individuals who are considered intelligent (Brislin, 2000). However, we cannot compare the actual speed of reporting a solution to others, as this is viewed quite differently in each culture.

One particular type of equivalence that raises a very interesting problem is that of **translation equivalence**. Psychologists often want to use tests developed in their own language with people of a different culture who speak a different language. Translating a test from one language to another can be a difficult task. The best way to assess translation equivalence is through **back translation**. In this procedure, one person translates the test, or survey, into the foreign language, and then a different person translates the foreign language test back into the original language. The original test can then be compared to the back translated test to see how closely they are worded. Ideally they would be identical, but this is seldom the case. To give you a simple example, when I was in graduate school, we had a student from Taiwan join our research group. One day I asked her to translate my last name, Kelland, into a Chinese character. When she had done that, I asked her how she would translate that particular Chinese character into English for someone who was not

Chinese. She translated the character as Kwang. Despite the first letter, I hardly consider Kwang to be a reasonable translation of Kelland, but she didn't seem to think of this as much of a problem (perhaps revealing another cultural difference!). When the process of back translation is used successfully, which may involve working back and forth with the translations, it has the effect of **decentering** the test from the original language. Specifically, that means that the test should be free of any culturally emic references or aspects that interfere with the translation equivalence of the different versions of the test (Brislin, 2000; Matsumoto & Juang, 2004).

While the list of issues pertaining to cross-cultural research goes on, let's consider just two more specific issues: **cultural flexibility** and **cultural response sets**. Cultural flexibility refers to how individuals are willing to change, or adapt, in situations in which they know there are cultural differences. For example, American businesspeople can stand about 15 minutes of small talk before getting down to business. Their Japanese counterparts, in contrast, consider it important to get to know their business partners, and they are comfortable with hours of conversation about a variety of topics. This would, of course, be an important consideration for anyone studying the relationship between individual personality and success in business situations in this intercultural setting. Cultural response sets refer to how a given culture typically responds. If a given culture is more reserved, and they are asked to rate the importance of some value in comparison to how a more open culture rates that value, a difference in the rating may reflect the cultural difference in responding, rather than the degree to which people in each culture value the variable being measured (Brislin, 2000; Matsumoto & Juang, 2004).

Finally, in light of these challenges, it may be particularly important to conduct **cross-cultural validation studies**. Rather than testing hypotheses about specific cultural differences, cross-cultural validation studies are used to examine whether a psychological construct that was identified in one culture is meaningful and equivalent in another culture (Brislin, 2000; Matsumoto & Juang, 2004). For example, as we will see in Chapter 7, Erik Erikson did not feel confident in proposing his eight stages of development (the psychosocial crises) until he had confirmed his observations in two separate Native American tribes. He was able to gain the trust of these groups, and thus able to closely observe their child-rearing practices, thanks to the anthropologists who introduced him to the tribes they had been studying for a long time.

Anthropologists have done much more for psychology than merely introducing some psychologists to cultural issues and unique cultural groups. Some of them have had their own interests in personality. Many anthropologists, as well as some psychologists, have relied on **ethnographies** to report detailed information on the customs, rituals, traditions, beliefs, and the general way of daily life of a given group. They typically immerse themselves in the culture, living for an extended period of time with the group being studied (this helps get past the anxiety of being observed or any lack of cultural flexibility). Comparing the ethnographies of different groups can help guide cross-cultural psychologists in determining the likelihood that their cross-cultural studies are valid (Matsumoto & Juang, 2004; Segall et al., 1990).

Discussion Question: Translating psychological tests into different languages is often a problem for cross-cultural psychologists. Americans have a reputation for only knowing English, whereas people in other countries often speak more than one language. Do you

know a foreign language well enough to actually communicate with someone in another country? How important do you think it is to learn another language as part of understanding their culture?

Placing Cross-Cultural Studies in Context: Blending

Psychology with Anthropology

As the field of psychology entered the twenty-first century, there was a groundswell of interest in cultural factors as they pertain to all areas of psychology. In the field of personality, as well as in other areas, there have always been individuals with an interest in culture and society, but they tended to remain as individuals. Although they were often admired for their unique interests and ideas, the major emphasis in psychology was on the scientific method and data that had been obtained in carefully controlled situations, and then analyzed with similar, exacting precision. Culture, as difficult as it is to define, was left largely to anthropologists and sociologists.

Anthropologists, in particular, were not as shy about addressing the domain of psychology, and a number of anthropologists crossed over into the study of psychology to such an extent that they are often mentioned even in the introductory psychology textbooks. But given that their primary interest was in anthropology, they did not form detailed personality development theories of the type presented in this (or other) personality textbooks. In this chapter, however, we will take a look at some of the ideas presented by the renowned anthropologist Ralph Linton, and his occasional colleague Abram Kardiner, a psychoanalyst with an associate appointment in the same anthropology department as Linton. In addition to their books, students of personality with a strong interest in cultural influences on personality will also find the works of Ruth Benedict and Margaret Mead of great interest.

The Influence of Culture and Society on Personality

Many psychology textbooks mention a few famous anthropologists, such as Ruth Benedict and Margaret Mead, whose research included work on child development and personality. However, less well-known in the field of psychology is the renowned anthropologist Ralph Linton, who paid particular attention to personality development in relation to culture and society. Linton also collaborated with Abram Kardiner, a founding member of the New York Psychoanalytic Institute (and who was analyzed by Sigmund Freud himself in 1921-1922). Linton and Kardiner freely acknowledged the connections between anthropology and psychology, noting the influence of Benedict and Mead, Franz Boas (recognized as the father of American anthropology and mentor to both Benedict and Mead), and the psychoanalysts Anna Freud, Erich Fromm, Karen Horney, and Wilhelm Reich (Kardiner, 1939; Kardiner, Linton, DuBois, & West, 1945; Kardiner & Preble, 1961).

Linton described personality as existing on three levels. First, personality can be described based on either its content or its organization. The organization, furthermore, can be examined in terms of its superficial organization or its central organization. The central organization of personality gives the whole personality its distinctive character, and includes the most invariant aspects of personality, such as the degree of introversion/extraversion, or other aspects of temperament (Linton, 1936, 1945). Although these temperamental attributes are present at birth, they do not comprise personality per se. The superficial organization of personality, however, is based on the goals and interests of the individual, and incorporates the individual’s experiences in life within the context of the central

organization. In this regard, the superficial organization should not be confused with something transient or insignificant. It is “superficial” only in the sense that it is on the surface of the personality, and the goals and interests of the person are based on the content of personality that represents their life experiences as they are organized within the personality. The goals and interests themselves, which incorporate the content of personality, are determined almost entirely by the culture in which the individual is raised. According to Linton (1936), the process of integrating the individual’s experience within the context of one’s temperament (or “constitutional qualities”) forms a “mutually adjusted, functional whole.”

A critical question, of course, is whether cultural experiences can affect the central organization. Linton (1936, 1945, 1955) believed that no matter how an individual receives the cultural characteristics of their society, they are likely to internalize them, a process known as **enculturation**. One of the main reasons that enculturation is so influential in every aspect of the person’s being, is that it pervades every aspect of the society in which the person lives. Thus, even someone who is considered a rebel, most likely exists within a range of rebellion that is possible within that particular culture. This is directly related to the apparent reality that cultures do give rise to certain types of personality. Making the matter even more complicated, or simpler depending on one’s perspective, is the role of status within a culture. Thus, although a given culture or society, or one’s own temperament, may influence personality in one direction, a particular social class might influence personality in a different direction. An individual born into a given class, whose personal constitution does not fit that class, may develop what Linton called a **status personality**, i.e., a persona that fits with societies expectations for the individual in certain settings. For example, someone born into an upper middle class family involved in business, who is personally rather introverted and withdrawn, may present a confident and outgoing personality when working, and only upon returning home do they revert to their natural inclination to be shy and quiet.

One of the most interesting points made by Linton is that individuals with complimentary personalities are also mutually adjusted. The most obvious example is that of the gender roles of men and women. Men are expected, in many cultures and societies, to be the dominant member of the family, as well as the “bread-winner.” Conversely, women are expected to be submissive, and to remain home and care for the household and the children. In this way, the men and women together complete the necessary tasks for family life without entering into conflict (at least in theory!). In some cultures, these gender roles are quite relaxed with regard to the sex of the individual. Amongst the Comanche (a Native American tribe), men whose personalities were not at all suited to being warriors assumed a special role, that of **berdache** (Linton, 1936). The berdache wore women’s clothes, and typically fulfilled a woman’s role, but they were treated with somewhat more respect than women (in keeping with the patriarchal nature of the society). Some were homosexuals (though not all), and even married. This was generally accepted, and any disapproval these relationships received was directed toward the warrior husband, not the berdache!

Abram Kardiner, a psychoanalyst who collaborated with Linton, shared the same general perspective on the relationship between personality and culture, and attempted to put the relationship into psychological terms. He distinguished between the **basic personality**, or **ego structure**, which he considered to be a cultural phenomenon, and the individual’s **character**, which is their unique adaptation to the environment within their cultural setting.

Thus, each individual develops a unique character, but only within the constraints of the culturally-determined range of potential ego structure (Kardiner, 1939). The process of personality development, within a cultural setting, results in what Kardiner called a **security system**. The security system of the individual is the series of adaptations that serve to ensure the individual's acceptance, approval, support, esteem, and status within the group. Thus, for each person within a given cultural group, their basic personality is formed through an ongoing interaction with the very culture in which that person needs to be (and, hopefully, will be) accepted as a member. Both of Kardiner's major books, The Individual and His Society (Kardiner, 1939) and The Psychological Frontiers of Society(Kardiner, et al., 1945), offer extraordinary examples of detailed anthropological studies of a wide variety of cultures followed by psychoanalytic evaluations of the functions served by various aspects of the cultural practices of those people.

Robert LeVine, like Kardiner, was an anthropologist and psychoanalyst with a strong interest in personality (LeVine, 1973, 1974). He begins by asking the question of whether there are differences in personality between different cultural groups. If there are not, then any analysis of the nature or causes of those alleged differences is meaningless. If there are differences, can we then point to specific evidence that the environment can elicit changes in those differences? The answer is yes to both, and as one example LeVine points to the dramatic **acculturation** of rural immigrants from underdeveloped areas of Europe and Asia who emigrated to industrialized countries, such as the United States, and within two or three generations had radically altered not only their basic ways of life, but also their social class (moving from traditional peasantry to the middle-class; LeVine, 1973). LeVine also continued Kardiner's approach of using a psychoanalytic perspective to evaluate and compare the nature of different cultures, and he proposed the term **psychoanalytic ethnography**. In an effort to justify the use of psychoanalytic ethnography, LeVine argues that there are enough common elements in the nature of all people and cultures to provide for valid comparisons of the differences between those same people and cultures (LeVine, 1973).

One of the most striking discussions of the relationship between culture and the potential for personality development was offered by Pitirim Sorokin, the founder of Harvard University's sociology department and a colleague of the trait theorist Gordon Allport (see Chapter 13). Sorokin points out that culture can have a dramatic influence on the biological substrates of personality. For example, through the use of contraception, abortion, etc., many potential individuals are never born. Conversely, if such measures are prohibited, many unwanted children are born. In addition, cultural rules and norms against sexual intercourse and/or marriage between certain age groups, races, social classes, families, religions, etc., directly influence the potential for genetic variation within and across different groups of humans (Sorokin, 1947). Indeed, Sorokin took such a broad view of the role of society and culture in the environmental universe of each individual, that he described trying to understand sociocultural phenomena by locating them in terms of **sociocultural space** and **sociocultural distance**. The concept of sociocultural distance has taken on new meaning since Sorokin proposed it over 50 years ago. Today, anyone can travel around the world in a matter of hours or days, and many people do so regularly. Technology and globalization have dramatically reduced the distance between people, and consequently brought their cultural differences into contact with one another. Efforts to study cultures and societies alter the location of sociocultural phenomena within our own universe of personal development. In

other words, by studying the relationships between society, culture, and personality, we are altering the meaning and influence of those relationships, hopefully for the better.

As a final note, although this section has highlighted the influence of anthropologists and sociologists on cross-cultural research in the study of personality, there has also been an influence from psychology on these investigators. As noted above, both Abram Kardiner and Robert LeVine were psychoanalysts. In addition, Kardiner acknowledges having learned a great deal from a professor named John Dollard. Dollard was a sociologist who had studied psychoanalysis and who collaborated with Neal Miller (a psychologist trained in learning theory) in an effort to apply classical learning theory to psychodynamic theory (see Chapter 10). Dollard contributed a chapter to one of Linton's books, and was cited by both LeVine and Sorokin (who was, again, also a colleague of Allport). Given such an interesting interaction between the fields of psychology, anthropology, and sociology over half a century ago, it seems surprising that psychology is only now emphasizing the value of focusing on cultural influences on personality development.

Discussion Question: Have you ever had an interest in ethnography? When you begin to learn something about another culture, how much does it interest you? How influential do you think your culture has been in your own personal development?

Different Cultural Factors Affecting Personality

Since culture pervades every aspect of our lives, the number of cultural factors that we might examine in the study of personality is quite large. However, there are a few major factors that stand out, and that have been the subject of significant research in the field of psychology. Thus, we will take a brief look at four major factors that will come up repeatedly throughout this book: **religion**, **race**, **gender**, and **age**.

Religion as a Cultural Influence

…religion in its turn exerts the most decisive influence upon all groups and systems of culture, from science and the fine arts to politics and economics. Without knowing the religion of a given culture or group - their systems of ultimate values - one cannot understand their basic traits and social movements. (pg. 228; Sorokin, 1947)

The essential importance of religion was also recognized by Abram Kardiner and Robert LeVine, both of whom, as noted above, studied anthropology and psychoanalysis (see Kardiner, et al., 1945; LeVine, 1973). As we will see in the next chapter, the recognized founder of psychoanalysis, Sigmund Freud, also placed great emphasis on the influence of religion and religious symbolism (though he did not believe in God).

Despite the importance of religion, as perhaps the most significant cultural factor, there is variation in the extent to which formal religious beliefs and practices are a part of the routine life of people in different cultures (see Matsumoto & Juang, 2004). Since most psychologists were not emphasizing cultural factors as an essential aspect of the early development of the field (leaving that to anthropologists and sociologists), and given Freud's powerful and convincing arguments against religion (see Chapter 3), it is not surprising that psychology has not focused on the influence of religion on personality. But that is changing, and despite the role that religion has played in many political battles and outright war (as has been the case in the Middle East for thousands of years!), religion and spirituality are also recognized as potentially favorable aspects of psychological development in general,

and personality development in particular, in the field of positive psychology (Compton, 2005; Peterson, 2006; Peterson & Seligman, 2004; Snyder & Lopez, 2005). Given the importance of religion as a cultural determinant, and the emphasis on culture in this book, we will examine the influence of religion on personality development throughout this textbook.

Religion/spirituality appear to be the most significant cultural factors affecting people's lives and personal development. Islam, Buddhism, Taoism, Yoga, Christianity, and Judaism, taken together, represent the religious or spiritual traditions of some 5 to 6 billion people, most of the world's population. Shown are some of the author's copies of the Holy Bible, Holy Quran, Discourses of the Buddha, Yoga-Sūtra, Bhagavad Gita, and the Tao Te Ching.

The Question of Race and Ethnicity as Cultural Influences

At the very outset we must face three possible alternatives as we consider the concept of race: 1) there is such a thing as race in mankind; 2) there is not such a thing as race in mankind; 3) even if race in mankind exists, it can have no significance save as people think of it and react to their conception of it. (pg. 38; Krogman, 1945)

Although religion may be the most significant cultural factor, the concept of race has probably existed even longer, and it is certainly the most visually obvious factor. But is it really? The fact is that there is no clear answer to the question of what actually constitutes race (Krogman, 1945; Linton, 1936, 1955; Sorokin, 1947). Although most people quickly think of three major races (White, Black, and Asian), and many of us would add a fourth category (Latino), studies have suggested that there may actually be as many as thirty-seven distinct races (see Matsumoto & Juang, 2004). In addition, genetic studies have suggested that there is more inter-group variation than there is between-group variation, further suggesting that race is nothing more than a social construction. As an alternative to race, some people use the term **ethnicity**, which identifies groups according to commonalities such as nationality, culture, or language. This fails to solve our problem, however, since the concept of ethnicity suffers from the same problems as the concept of race (Brislin, 2000; Matsumoto & Juang, 2004; Miller & Garran, 2008; Whitley & Kite, 2006).

Although the terms race and ethnicity are often used interchangeably with culture, they are quite different. The United States, for example, has large populations of people from different races, ethnic groups, religions, and nationalities, but they all contribute to the greater cultural identity of "American." Indeed, the very concept of America as a "melting pot" defies the use of racial or ethnic characterizations of the American people. This argument goes both ways, of course. We cannot simply refer to people who live within the boundaries of the United States as American, and expect that they are similar in every other cultural respect. Although this may seem rather confusing, that is exactly the point. Critical thinking must always be applied to personality theories and their application in broad ways. This does not mean they are not useful, just that we must be careful in our interpretations of people's behavior and personality if they are from another culture.

Although ethnicity and race may be of questionable value as cultural factors, there are two critically important issues that arise from them. A common problem in cross-cultural research is that of **ethnocentrism**, the belief that one's own culture has the right beliefs and practices, whereas other cultures have wrong beliefs and practices (Matsumoto & Juang, 2004; Whitley & Kite, 2006). Such value judgments interfere with the objectivity of

cross-cultural research, and can have negative effects on intercultural communication. The other, very serious problem is that of **racism**. As noted in the quote above, race is very real if people believe in it and act according to their perception of it. We will examine racism later in the textbook. For now, consider the following quote from a recently published book entitled Racism in the United States: Implications for the Helping Professions:

Racism has evolved as a persistent part of the human condition. Its obstinacy and intractability are frustrating and at times baffling. We live in a world in which most nations have signed United Nations declarations of human rights and claim to be democracies, yet racial and ethnic conflict abound. (pg. xvii; Miller & Garran, 2008)

Gender and Culture

Gender has been the subject of a wide range of studies, from pop-psychology books like Men Are From Mars, Women Are From Venus (Gray, 1992) and Self-Made Man: One Woman's Journey into Manhood and Back Again (Vincent, 2006) to such ominous sounding titles in academic psychology as The Longest War: Gender and Culture (Wade and Tavris, 1994). In 2005, the president of Harvard University suggested that one of the reasons there were so few women in math and science fields was that they lacked the intrinsic aptitude. The subsequent uproar led to the end of his presidency at Harvard, and a renewed effort to examine the reasons why few women succeed in math and science careers. An extensive study, led by former APA President Diane Halpern came to no specific conclusions, due to the complex interactions of a variety of factors, but in so doing made it clear that no blame can be placed directly on inherent/genetic ability (Halpern, et al., 2007; see also Barnett, 2007).

Gender is a distinctly cultural term, representing the behaviors or patterns of activity that a given culture or society expects from men and women. It is perhaps most commonly used to address differences between males and females, with an underlying assumption that sex differences lead to gender differences. However, apparent sex differences may actually be cultural gender differences, and cultures and societies exert significant influence on **gender roles** from a very early age (Brislin, 2000; Matsumoto & Juang, 2004; Stewart & McDermott, 2004). Still, some cultural factors may also have a basis in biological reality. For example, males are typically larger and stronger than females, so it makes sense for males to do the hunting and fight the wars. Women become pregnant and then nurse the infants, so it makes sense for them to provide early childcare. How this led to man have greater control and prestige in society, however, remains unclear, especially since that is not universally the case (Wade & Tavris, 1994). In addition, older men often become involved in childcare after their hunting/warrior days are behind them, further complicating the issue.

Among the differences between men and women that seem to be fairly common across cultures, and which may stem from sex differences, are aggression and emphasizing relationships. Men are typically more aggressive, and women seem to focus more on relationships with other people. In accordance with these tendencies, women typically defer to men, particularly in situations that may be confrontational. It also leads to conflict between men and women due to their difficulties communicating, hence the popularity of John Gray's book suggesting that men and women are from completely different planets. Given the status of men, the challenges that these gender differences create for women were not typically given a great deal of attention. However, Karen Horney (see Chapter 8) and more recently the women of the Stone Center Group (see Chapter 9) have made great

strides in changing that situation. Not only have the members of the Stone Center Group provided a number of collected works on the psychology of women (Jordan, 1997b; Jordan, Kaplan, Miller, Stiver, & Surrey, 1991; Jordan, Walker, & Hartling, 2004), there are also textbooks devoted exclusively to the subject (e.g., Matlin, 2004).

Aging within a Cultural Context

Age is used as routinely as sex to divide the people in a society. All societies recognize at least three age groups: child, adult, and old. Childhood is typically further divided into young childhood and adolescence. Each group has different rights, responsibilities, roles, and status (Linton, 1936; Sorokin, 1947). Sometimes, these can come into conflict. For example, among the Comanche, as with most Plains tribes in North America, the adult male was expected to be a warrior, whereas the old man was respected for his wisdom and gentleness. Transitioning from being a warrior to being an old man was very difficult, and Comanche men often hoped to die in battle in order to avoid the transition. Those who were forced to make the transition became very dangerous adversaries for the young men transitioning from childhood to adulthood, and often the old men would kill the young men when they could (out of sheer envy). Moving even beyond old age, into death, there are many societies in which the dead remain in the minds of the community members, and deceased relatives and heroes are even worshipped. In some cultures, the relationship with those who are dead is a very important part of daily life (Linton, 1936).

Throughout history, as societies have changed, so have the ways in which they treated and cared for (or did not care for) aged individuals. Although modern industrialization is correlated with a significantly longer lifespan, such dramatic cultural changes favor the young people who can more readily adapt to the changes. In addition, industrialized societies typically shift some of the responsibility of caring for the aged from the family to the state. Curiously, this removes the responsibility of caring for aged persons from the very family whom those aged individuals had cared for and raised themselves! The one area in which aged members of the community are likely to retain their leadership status is religion, and the rituals associated with it (Holmes, 1983; Johnson & Thane, 1998; Schweitzer, 1983).

David Gutmann, an early gerontologist with an interest in the effects of aging on personality, has focused his career on studying men in four cultures: a typical American population (to the extent that there is such a thing), the Navajo in the United States, both Lowland and Highland Maya in Mexico, and the Druze in Israel (see Gutmann, 1987, 1997). One of the most interesting realities that he begins with is the recognition that the human species is the only one in which aged individuals remain active long past their reproductive prime. What possible evolutionary advantage does this offer our species? Gutmann believes that our elders fill unique roles in society, thus providing essential benefits to the extended family and the community, particularly for the young. Indeed, Gutmann points out that it is uniquely human to favor the ends of the lifespan, both childhood and old age, over the middle of the lifespan, when reproductive fitness is at its biological peak. As we noted above, however, the transition into old age is not always easy, and this leads to some unique changes in personality associated with aging.

The beginning of old age is marked by the maturity of one’s children, such that the adult individual no longer needs to provide care for their children. Thus, both men and women can begin to express those aspects of their personality that were set aside in order to mutually facilitate raising children. Consequently, there is often a relaxing, or even reversal to some

extent, of gender roles. A particularly significant change for men who no longer have the physical strength to be warriors (or to engage in the physical labor of their community) is the manner in which they seek mastery over their lives. Young men have the ability to seek **active mastery**, they strive toward autonomy, competence, and control. Older men must seek **passive mastery**, through adaptation and accommodation. The oldest men must rely on **magical mastery**. The world becomes one of potential providers and potential predators. They rely on primitive defense mechanisms, and wish fulfillment becomes synonymous with reality. Their relationship to the world is marked by feelings of vulnerability (Gutmann, 1987, 1997). It is easy to see how they would rely heavily on religion, and the promise of a supernatural being for protection and eternal reward, thus inclining them toward an involvement in religious practice that would naturally lead to a degree of respect, or at least acknowledgement, as religious leaders. Of course, the degree to which a society provides for its oldest members, such as through retirement benefits, would have a significant effect on this aging process. Nonetheless, Gutmann found evidence for these changes in mastery style amongst men in mainstream America as well as in the Navajo, Maya, and Druze cultures.

Discussion Question: To what extent have religion, race, gender, and age been important factors in your personal development (either currently, or in the past)? Which do you expect will be the most important in your future development?

Addressing the Degree of Cultural Integration

Adding to the complexity of culture's role in shaping our personalities are two important factors. First is the degree to which an individual is integrated into their culture, and vice versa. As Sorokin points out, it is exceedingly rare that an individual is either totally integrated into their culture or not integrated into it at all (Sorokin, 1947; see also Kardiner, et al., 1945; Linton, 1936). Thus, culture provides a framework within which individual variation is possible, but at the same time there will always be some consistent basis for understanding the people within a given culture. This becomes particularly important when considering cross-cultural research, since it may be reasonable to make some general assumptions about an individual from another culture, but we must also be prepared for their own unique variation as a person in that cultural group.

A second important factor is that cultural phenomena do not exist in isolation. Both gender and race/ethnicity, for example, influence how one adapts to the aging process (see, e.g., Arber, Davidson, & Ginn, 2003; Barrow, 1986; Calasanti & Slevin, 2001; Cool & McCabe, 1983; Holmes, 1983). Gender also interacts with race/ethnicity in determining one's reactions to group psychotherapy (Pack-Brown, Whittington-Clark, & Parker, 1998) and/or adapting to life as a minority student on a majority campus (Levey, Blanco, & Jones, 1998). Religion is considered to be such an important factor in the African American community that its role has been the subject of special interest (see, e.g., Belgrave & Allison, 2006; Taylor, Chatters, & Levin, 2004). Obviously many more examples can be found, the point being that as an individual develops, with multiple cultural factors influencing them, and each factor being integrated to a great or lesser degree, the potential for individual personality differences is extraordinary, even when the overall effect of the specific culture, or society, is to guide its members toward certain underlying tendencies that become characteristic of that culture's members.

Discussion Question: Are you, or is anyone you know, distant or unintegrated with your family's culture or that your community? If so, what sort of problems does that create for your identities? If none, does your cultural integration provide a sense of integrity?

Culture and Diversity

The importance of studying culture can be found in the diversity of people both around the world and within our own communities. For example, although many communities may be quite limited in terms of religion and race/ethnicity, nearly all communities have a mixture of gender and age. Although religion, race/ethnicity, gender, and age may be the major factors that have traditionally been studied in the field of psychology, in the instances where culture was studied, it is important to remember two additional points. First, there are other cultural factors that may be very important for certain individuals and/or select groups of people, and second, people can be excitingly (or frustratingly, depending on your point of view) unique in their individuality.

One area of diversity that has been receiving more attention as a cultural factor affecting the lives of many people is that of physical disability. In the past, although it was recognized that individuals with physical disabilities experience basically the same personality development processes as other people, disabilities were considered to be specific conditions that isolated the disabled person from their surroundings (Barker et al., 1953; Pintner et al., 1941). Over time, as more research became available on the psychology of people with disabilities (e.g., Goodley & Lawthorn, 2006; Henderson & Bryan, 1984; Marks, 1999; McDaniel, 1976; Roessler & Bolton, 1978; Stubbins, 1977; Vash, 1981; Wright, 1983), perspectives on how to study these individuals changed as well. In 2004, the Society for Disability Studies adopted preliminary guidelines for developing programs in disability studies. They emphasize challenging the previously held view that disabilities are individual deficits or defects that can or should be fixed by "experts." Rather, they recommend exploring models that examine cultural, social, political, and economic factors which integrate personal and collective responses to difference (the society's website is www.disstudies.org).

There are several chapters in this book where we will address the biological aspects of personality development, including the mind-body connection. Whereas a few academic authors have made passing mention of the value of exercise, self-defense training, and spirituality in coping with physical disabilities (Nardo, 1994; Robinson, 1995; Sobsey, 1994), one particularly interesting area in which culture, physical disability, the mind-body connection, positive psychology, and spirituality all come together is martial arts training (see Kelland, 2009, 2010). A number of notable martial arts experts actively encourage people with disabilities to practice the physical, psychological, and spiritual aspects of these ancient exercises (such as Grandmaster Mark Shuey Sr. of the Cane Masters International Association, Master Jurgen Schmidt of the International Disabled Self-Defense Association, and Grandmaster John Pellegrini of the International Combat Hapkido Federation), and several books are available on this subject (McNab, 2003; Robertson, 1991; Withers, 2007). We will revisit this topic later in the book, but for now consider the diversity of cultures and personal interests that come together when, for example, a disabled American living in the modern world pursues the spiritual and physical development associated with an ancient, Asian practice of self-development.

When considering the life of an individual like Shawn Withers, the son of a Maine fisherman, who suffered a massive stroke at the age of 20, but then went on to earn a black belt in

Kenpo Karate and then developed his own style known as Broken Wing Kenpo (Withers, 2007), broad descriptions of personality theory and cultural perspectives fall short of giving us an understanding of the person. Thus, some researchers, like Dan McAdams (McAdams, 1985, 2006; McAdams et al., 2001), have emphasized the need for studying a narrative framework within which we not only live our lives, but actually create them:

…like stories in literature, the stories we tell ourselves in order to live bring together diverse elements into an integrated whole, organizing the multiple and conflicting facets of our lives within a narrative framework which connects past, present, and an anticipated future and confers upon our lives a sense of sameness and continuity - indeed, an identity. As the story evolves and our identity takes form, we come to live the story as we writeit, assimilating our daily experience to a schema of self that is a product of that experience. (pg. v; McAdams, 1985)

Although this textbook will cover broad personality theories and cultural perspectives, there are also reflective elements and discussion questions included to help you try to address your own narrative stories. In addition, there are biographies at the beginning of each chapter on the major theorists, which although they are not personal narratives, will nonetheless give some insight into the sort of person that theorist was, and hopefully, how their life and their personal experiences helped to shaped the personality theory they developed.

Culture and Mental Illness

Although this book focuses on normal personality development, one cannot escape the fact that most of the famous personality theorists were clinicians who were trying to understand how their patients/clients had developed psychological disorders. So, our understanding of personality development grew hand-in-hand with our understanding of psychological disorders. The Diagnostic and Statistical Manual of Mental Disorders began addressing the importance of culture in the 4th edition, and more recently it has taken a dramatic step forward with the publication of the DSM's 5th edition (American Psychiatric Association, 2000, 2013).

The DSM-V includes a section on Emerging Measures and Models, one chapter of which is called Cultural Formulation. Although the DSM-IV began to present an outline for cultural formulation, the DSM-V includes two valuable sets of questions that have been field-tested to help clinicians assess the cultural identity of a patient/client and how that cultural identity may affect the diagnosis and treatment of any potential psychological disorder. The first set of questions is the basis for the Cultural Formulation Interview, and the second set comprise the Cultural Formulation Interview - Informant Version (which is given to someone who is knowledgeable about the life circumstances and potential clinical problems of the patient/client).

In our increasingly global and multicultural world it is more and more likely that therapists will encounter individuals from different cultural backgrounds than their own. Thus, in order for the therapist to fully understand the individual and the context of their psychological distress, the therapist must be aware of and attentive to possibly significant cultural differences. Failure to do so might result in what Iijima Hall (1997) has described as **cultural malpractice**!

A Final Challenge

As important as it is to keep cultural factors in mind when studying personality, the unfortunate reality is that the major personality theories in psychology, as we recognize psychology today, have arisen within Western intellectual settings. Thus, we do not have corresponding systems of personality theory that arose in other cultures that we might compare to the theories we do have. This somewhat limits our perspective on cross-cultural personality theory to attempts to apply our Western theories to people of other cultures. This limitation should not, however, keep us from considering these issues. It is merely an inconvenience that you should keep in mind as you consider the theories present in this textbook. Should your career lead you into the field of psychology, perhaps you will be one of the people to help develop and advance some theory that moves beyond this limitation.

Another concern has to do with the nature of this textbook, and personality courses in general. Although we have emphasized anthropology and sociology in this chapter, this is a psychology textbook. Nonetheless, culture is an all-encompassing factor in the development and psychology of both individuals and the groups in which they live. Indeed, in Personality and Person Perception Across Cultures, Lee, McCauley, & Draguns (1999) boldly state that “human nature cannot be independent of culture” (pg. vii). Thus, it is essential that we learn as much as possible about culture. As an encouragement for studying other cultures, Ralph Linton had this to say:

The ability to see the culture of one’s own society as a whole, to evaluate its patterns and appreciate their implications, calls for a degree of objectivity which is rarely if ever achieved…Those who know no culture other than their own cannot know their own…Even such a master as Freud frequently posited instincts to account for reactions which we now see as directly referable to cultural conditioning. (pp. 125-126; Linton, 1945).

Personality Theory in Real Life: Examining Your Own Cultural Background

I consider myself to be an American. But what does that actually mean? I know a few tidbits about my ancestors that are quite interesting. One of my ancestors, a great aunt, was on the Titanic when it sank (like most women and children, she was one of the survivors). I am directly descended from John Howland and Elizabeth Tilley, who came to America on the Mayflower, in the year 1620. Actually, John Howland fell overboard in the middle of the Atlantic Ocean during rough seas, but was saved when he grabbed a rope trailing in the water and was then pulled back aboard! Among John Howland and Elizabeth Tilley’s other direct descendants (and, therefore, my distant relatives) are the U. S. Presidents Franklin D. Roosevelt, George H. W. Bush, and George W. Bush, the renowned poets Ralph Waldo Emerson and Henry Wadsworth Longfellow, and the founder of the Mormon church, Joseph Smith. This lineage does not, however, come down through the Kelland name, as the Kellands came to America later. If you add one more generation, John Howland’s brothers include among their descendants U. S. Presidents Richard Nixon and Gerald Ford, as well as British Prime Minister Winston Churchill. The other side of my family was primarily German, and when they first came to America they settled in Kansas and became well-respected wheat farmers.

What do you know about your cultural background? Are you proud of your background in a way that has shaped your life? For example, knowing one of my ancestors was on the Mayflower helped to kindle in me an ongoing interest in history. If you don’t know much about your family’s history, who might you turn to for information? Try it; you may learn something fascinating about yourself.

Review of Key Points

• It is important for the field of psychology to consider both cross-cultural and intercultural research.

• Emic tasks are familiar to the members of a given culture, whereas etic tasks are common to all cultures.

• In cross-cultural research, it is important to determine whether a concept is equivalent in each culture being studied.

• One of the most important considerations for the equivalence of psychological tests is the issue of translation equivalence. This can be addressed, in part, through examining back translations of the test(s) being used.

• Back translation helps to decenter a psychological test from the cultural influence of the original language/culture.

• Cultural flexibility and cultural response sets determine the range within which members of different cultures respond. Thus, one must have some understanding of these factors for a given culture when attempting to interpret cross-cultural or intercultural research.

• Cross-cultural validation studies specifically examine whether a given cross-cultural study makes sense within the context of a research project.

• Ethnographies provide detailed information on the daily lives and habits of the members of a given culture. They are often conducted by anthropologists, and can be of great value to cross-cultural psychologists.

• Anthropologists, such as Ralph Linton, believe that personality develops on multiple levels. Central organization involves the biological aspects of personality (such as temperament), whereas the superficial organization is profoundly influenced by culture.

• The process of enculturation involves internalizing cultural norms, and may be able to influence the central organization of individuals, as well as the superficial organization.

• When an individual’s basic personality contradicts that which their social class expects of them, they may develop a status personality.

• Complimentary personalities, such as those seen in typical gender roles, tend to be mutually adjusted.

• The psychoanalyst Abram Kardiner, who worked with Linton, distinguished between the culturally-determined basic personality, or ego structure, and the individual’s character, which is their unique adaptation to the environment given their inherent tendencies and personal experiences.

• According to Kardiner, personality development within a cultural setting provides a security system.

• Acculturation can lead to changes both in individuals and in entire cultures.

• The anthropologist/psychoanalyst Robert Levine proposed the term psychoanalytic ethnography to describe the work done by researchers like himself and Kardiner.

• The sociologist Pitirim Sorokin described personality development within one’s entire cultural universe, referring to sociocultural phenomena in terms of their sociocultural space and sociocultural distance.

• Religion appears to be the single most significant cultural factor.

• Race and ethnicity are complex, and are hard to consider as cultural factors because they cannot easily be defined. Nonetheless, as individuals think about them in their own ways, they often give rise to ethnocentrism and, potentially, racism and discrimination.

• Although sex is a biological distinction, gender roles are an influential cultural factor that is applied from very early in life. Certain aspects of gender roles likely reflect some of the underlying biological differences between males and females.

• All societies recognize distinct age groups, and treat those age groups differently. However, there is great variation in the status of each age group, which often leads to conflict.

• Old age is of particular interest, since the human species is the only in which individuals remain active long past their reproductive prime.

• One of the common cross-cultural factors facing old men is the transition in how they seek master in their lives. Only young men can expect to be successful seeking active mastery, whereas older men seek passive master and then magical mastery.

• In addition to sometimes dramatic differences between cultures, individuals within a culture also differ in the extent to which they integrate different aspects of their culture into their own lives.

• Sometimes cultural phenomena interact, making for interesting, yet complex, situations. For example, religion has played an important role in the cultural identity of African Americans throughout their history in America.

Attribution and critical-reading note

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Read historical and theoretical claims critically. A theory is a lens for inquiry, not a verdict about a child or family. Where source claims are dated or culture-bound, the new lecture supplies cautions and contemporary framing.

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To suggest that there is such a thing as an African personality may be misleading. Africa is the second largest continent, with just over 1 billion people spread out among over fifty different countries. It has been the target of extensive colonization over the centuries, and the struggle for liberation from European countries has surely left an indelible mark on the nature of the people there. In addition, the Sahara Desert creates a significant natural division of the people in the north from those in the south. The people of **North Africa** are primarily Arab-Berber Muslims, with ready access to southern Europe across the

Mediterranean Sea. This region can rightly be viewed as an extension of Western Asia, in terms of culture, spirituality, and race/ethnicity (Chatterji, 1960; Senghor, 1971). In contrast, the Black Africans live south of the Sahara Desert, and they are the people usually referred to when we think about Africans. Indeed, for the remainder of this section I will use the term African to refer to Blacks living in **Sub-Saharan Africa**. Though many people in Africa identity themselves in terms of their unique ethnicity, history, and geography, this book would be incomplete if noeffort was made to address the people of this continent. Keep in mind, however, that there is a great deal more work to do regarding our understanding of indigenous people around the entire world.

In 1999, James Lassiter wrote a very helpful article covering many of the historical problems that have affected the study of personality in Africa. Unfortunately, many studies sought to identify the nature of personality among Africans in terms of Western ideals, values, and socioeconomic and technological advancement. This biased view created a very negative attitude toward the people of Africa, a negative attitude that the people of Africa often adopted themselves. Thus, the study of personality fell into disrepute, and largely came to a halt. However, a number of professionals from other disciplines, such as sociology and anthropology, continued to examine whether or not there were characteristics common to the people of Africa, a unique and valuable personality distinct from other regions of the world. Though some controversy remains, and the definitions of what personality is from an African perspective are quite different than those we might recognize in traditional Western psychology, this work has led to some interesting insights. Fundamentally, these perspectives are summarized by the following simple proverb:

Umuntu ngumuntu ngabantu (a person is a person through other persons)

• Xhosa proverb (cited in Lassiter, 1999 and Tutu, 1999)

The African Worldview and Spirituality

For many authors, a common African personality derives from a common African **worldview**.

According to Khoapa (1980), an African's existential reality is one of collective being, they seek to understand the world through their intersection with all aspects of the world and other people. This worldview is holistic and humanistic, and it focuses on interdependence, collective survival, harmony, an important role for the aged, the oral tradition, continuity of life, and rhythm. In addition, there is a fundamental belief in a metaphysical connection between all that exists within the universe, through an all-pervasive energy or "**spirit**" that is the essence of all things (Chatterji, 1960; Grills, 2002; Grills & Ajei, 2002; Khoapa, 1980; Mwikamba, 2005; Myers, 1988; Obasi, 2002; Parham et al., 1999; Senghor, 1965, 1971; Sofola, 1973).

At the center of the African worldview is spirit, or life itself, a vital force that animates the universe and that imparts feeling to all things from God down to the smallest grain of sand. Although this spirit pervades all things, there is a distinct hierarchy among the things that make up the universe. At the top of the hierarchy is God, followed by the ancestors (including the founders of the tribes, aka the "god-like ones") and the living. Then come the animals, plants, and minerals. Being in the center, humans hold a privileged position. As living beings, people are able to increase their **being** (using this term in the same context as in existentialism). The source of spirit, and the **spiritness** within each person, is divine, and transcends both the physical universe and time. Thus, it can connect us to any person, place,

or thing. This is part of the basis for African veneration of their ancestors. In order for the ancestors to avoid becoming “completely dead,” they must devote themselves to strengthening the lives of the living. As a result, they can still participate in life. When a person recognizes that through spirit all things become one, and if they adhere to this realization, they lose all sense of individual ego/mind. Instead, they experience the harmony of collective identity and a sense of extended self that includes ancestors, those not yet born, all nature, and their entire community (Busia, 1972; Grills, 2002; Grills & Ajei, 2002; Jahn, 1972; Myers, 1988; Obasi, 2002; Parham, 2002; Parham et al., 1999; Senghor, 1965).

Based on the previous paragraph, it should be clear that religion and spirituality are very important to Africans. We share a biological connection with animals, and an inherent spiritual connection with plants and minerals, but our privileged position at the junction of spirit and nature allows us to participate in a spiritual life that separates us from the animals, plants, and minerals. This is how Africans believe they are able to increase their being. According to Khoapa (1980), we link the universe with God, we awaken it, we speak to it, listen to it, and try to create harmony. This leads to a profound connection with the rhythm of the universe. Senghor (1965) describes rhythm as the “architecture of being…the pure expression of the life-force.” Rhythm has become an important aspect of African life, particularly in art, music, and poetry (also see Busia, 1972; Chatterji, 1960; Jahn, 1972; Mwikamba, 2005; Senghor, 1971; Sofola, 1973).

African music, like sculpture, is rooted in the nourishing earth, it is laden with rhythm, sounds and noises of the earth. This does not mean that it is descriptive or impressionist. It expresses feelings. (pg. 86; Senghor, 1965)

As noted above, the transcendent aspect of spirit leads to connections between past, present, and things that have not yet happened. This has led to a distinct relationship to time, one that differs dramatically from the Western world. Africans believe there is a rhythmic, cyclical pattern to life set in place by God, and God knows what is right. This includes the seasons, the rising and setting of the sun, and stages of life (birth, adolescence, adulthood, old age, and death). Events in the past are typically referred to in terms of reference points, such as a marriage or a birth. As for the future, in most African languages there is no word for the distant future, and plans for the near future are once again typically made around events rather than a specific time on a clock. Accordingly, time is something to be shared with others, there isn’t really any such thing as wasting time. Tribal elders are respected for the wisdom they have accumulated over a lifetime, and the “living” dead are kept alive by the tribe’s oral historian (Jahn, 1972; Parham et al., 1999; Sofola, 1973; Tembo, 1980).

Discussion Question: The African worldview focuses on the universe and all the people within it as an interconnected whole, and seeks harmony and rhythm. Do you see life in a holistic way, do you try to relate to others as if we are all part of one creation? Do you think the world would be a better place if everyone tried to relate to others in this way?

Family and Community

For Africans, the basic unit is the **tribe**, not the individual. Since the tribe seeks collective survival, cooperation is valued over competition and individualism. Since close, personal interconnections are so fundamental, aggression toward others is considered an act of aggression against oneself, and the concept of alienation doesn’t exist. This concern for the

community is reflected in the family structure. For Africans, family includes parents, children, brothers, sisters, cousins, aunts, uncles, etc. All relatives have the responsibility to care for one another, and when parents become old it is the responsibility of their children to care for them (Khoapa, 1980; Kithinji, 2005; Lambo, 1972; Parham et al., 1999).

According to Khoapa (1980), Westerners are surprised when they observe Africans in normal conversation. There is a great deal of spontaneity, laughter, and the conversation goes on and on. They do not wait to be introduced before engaging in conversation. No reason is necessary for someone to drop by and engage in a conversation. Every gathering is an extension of the family, so there is no reason for inhibiting one's behavior. Simply being together is reason enough to engage others. Khoapa suggests that the "deafening silence" observed when traveling in the Western world is very strange and confusing to Africans.

The cultural institution of marriage provides an interesting example of these principles in action. Marriage is a unifying link in the rhythm of life: past, present, and future generations are all represented. Having children is an obligation, and marriage provides the accepted opportunity to fulfill that obligation. Indeed, since the purpose of marriage is to have children, a marriage is not considered complete until children have been born (Khoapa, 1980; Kithinji, 2005; Lambo, 1972; Parham et al., 1999; Wanjohi, 2005). Marriages can also be a profound source of connection between people that goes far beyond the basic family unit (two parents and their children). The spirit that underlies and provides energy for the fulfillment of being experienced in a family unites that family with other families around the world. In a more practical sense, when a man and a woman from different tribes are married, the members of each tribe see themselves as all becoming one extended family through that marriage (Parham, et al., 1999; Samkange & Samkange, 1980).

The belief that we are all interconnected extends beyond one's family and tribe to all people. **Hospitality** is an important characteristic that Africans expect will be extended to all visitors, including strangers. Different than in the West, however, is the expectation that hospitality will precede asking any questions. Thus, when a visitor is met at the door, they will be invited in, offered something to eat and/or drink, and friendly conversation may ensue, all before asking anything about the visit or even who the person is (if they aren't known). Being benevolent to everyone is seen as a sign of good character or good reputation. African myth and folklore often includes stories about gods or spirits who travel in disguise, rewarding people in kind for how the god or spirit is treated. Selfishness does not promote the well being of the tribe, so a selfish person is likely to be held in contempt and stigmatized. The responsibility for becoming caring people begins with the family (Kithinji, 2005; Lambo, 1972; Sofola, 1973).

Every Yoruba, the stranger inclusive, is expected to demonstrate that he was well brought up by his parents whose emblem he carries about by the virtue of his existence and former socialisation. A good home to the Yoruba African is a place where good training and nurturing in character and good behaviour including good mode of addressing people are imparted to the young…The good child is supposed not only to accept and show good character in the home but should show the glory of the home outside through his own good behaviour… (pp. 97-98; Sofola, 1973)

Discussion Question: In African culture, marriage and family are very important. How important are they to you? How has your personal history affected your feelings about marriage and family?

Ubuntu

The traditional African concept of **ubuntu** is one that encompasses the best that the people of Africa have to offer in terms of social harmony. It has come into play several times during difficult periods of nation building as African countries have gained independence and moved toward democracy. Archbishop Desmond Tutu, winner of the Nobel Peace Prize in 1984, served as Chairman of the Truth and Reconciliation Commission as the nation of South Africa transitioned from Apartheid to democracy. Rather than seeking revenge and the punishment of those who had supported Apartheid, or attempting to achieve some sort of national amnesia through blanket amnesty, the South Africans chose a third alternative. Amnesty would be granted only to those who admitted what had been done in the past. While some were concerned that such an option would allow crimes to go unpunished, the deep spirit of humanity that is ubuntu can lead to being magnanimous and forgiving.

Ubuntu…speaks of the very essence of being human. When we want to give high praise to someone we say, “Yu, u nobuntu”; “Hey, so-and-so has ubuntu.” Then you are generous, you are hospitable, you are friendly and caring and compassionate. You share what you have. It is to say, “My humanity is caught up, is inextricably bound up, in yours.” We belong in a bundle of life. (pg. 31, Tutu, 1999)

Samkange and Samkange (1980) discuss how extensively ubuntu (aka, hunhu, depending on the language) is intertwined with life amongst the people of Zimbabwe. It leads to a sense of deep personal relationship with all members of different tribes related by the marriage of two individuals. It has influenced the development of nations as they achieved freedom from colonial governments, and it encourages amicable foreign policies. Ubuntu can help to guide judicial proceedings, division of resources, aid to victims of war and disaster, and the need to support free education for all people. The special characteristic that ubuntu imparts on African people can also be seen among the African **diaspora**, those Africans who have been displaced from their homeland. For example, Black Americans typically have something unique that distinguishes them from White Americans, something called “**soul**.” According to Samkange and Samkange (1980) “soul is long suffering (“Oh Lord, have mercy”); soul is deep emotion (“Help me, Jesus”) and soul is a feeling of oneness with other black people.” As a result of the Black American’s experience with slavery, we now have **soul food**, **soul music**, and **soul brothers**.

Discussion Question: It has been suggested that the essence of personality among African people has given something special to members of the African diaspora known as “soul.” However, this may be a characteristic of all dispossessed people. Have you seen examples of this sort of “soul?” If yes, what was the experience like, and how did it affect your own views of life?

Although ubuntu is uniquely African, the peace and harmony associated with it can be experienced by all people. According to Archbishop Tutu it is the same spirit that leads to worldwide feelings of compassion and the outpouring of generosity following a terrible natural disaster, or to the founding of an institution like the United Nations, and the signing of international charters on the rights of children and woman, or trying to ban torture, racism, or the use of antipersonnel land mines (Tutu, 1999). Though ubuntu itself may belong to Africa, the essence of it is something shared by all dispossessed groups around the world (Mbigi & Maree, 1995). It embodies a group solidarity that is central to the survival of all poor communities, whether they are inner city ghettos in the West, or poor rural

communities in developing countries. According to Mbigi and Maree (1995), the key values of ubuntu are group solidarity, conformity, compassion, respect, human dignity, and collective unity. They believe that African organizations need to harness these ubuntu values as a dynamic transformative force for the development of African nations and the African people. Samkange and Samkange share that view:

…ubuntuism permeates and radiates through all facets of our lives, such as religion, politics, economics, etc…Some aspects of hunhuism or ubuntuism are applicable to the present and future as they were in the past…It is the duty of African scholars to discern and delineate hunhuism or ubuntuism so that it can, when applied, provide African solutions to African problems. (pg. 103; Samkange & Samkange, 1980)

Negritude and Nigrescence

Leopold Senghor (1965) has defined **Negritude** as “the awareness, defence and development of African cultural values…the sum total of the values of the civilization of the African world.” For Senghor this is not a racial phenomenon, but a cultural one, based primarily on cooperation. He distinguished this cooperation from the collectivist idea we typically associate with Asian cultures by focusing more on a communal perspective. In other words, collectivist cultures may be seen as an aggregate of individuals, but in the truly communal society, whether in the family, the village, or the tribe, there is a connection from the center of each person in their heart (see also Grills, 2002; Senghor, 1971). This is what Senghor believes has always been held in honor in Africa, and it ultimately encourages dialogue with others in Africa (the White Africans, the Arab-Berbers in North Africa) and beyond, so that we can assure peace and build the “Civilization of the Universal.”

Negritude, then, is a part of Africanity. It is made of human warmth. It is democracy quickened by the sense of communion and brotherhood between men. More deeply, in works of art, which are a people’s most authentic expression of itself, it is sense of image and rhythm, sense of symbol and beauty. (pg. 97; Senghor, 1965)

Abiola Irele has discussed the history of Negritude as a literary and ideological movement among Black, French-speaking intellectuals in Africa. It was initially a reaction to, and in opposition to, the colonial oppression of the African people. As such, it has been criticized by some as its own form of racism (see, e.g., Irele, 1981, 2001; Tembo, 1980), or as something unique to intellectuals, as opposed to more common people in Africa. However, as noted above, Negritude is about culture, not race per se. In addition, a small but nonetheless interesting study by Tembo (1980) provided evidence that scores on an **African Personality Scale** did not differ based on sex, marital status, having been educated in rural or urban schools, or whether they wished to pursue higher education in Africa or England. Irele compared Senghor’s view of Negritude to that of the existential philosopher Jean-Paul Sartre. Sartre viewed Negritude as a stage in the development of Black consciousness, a stage that would be transcended by the ultimate realization of a human society without racism. In contrast, according to Irele, Senghor’s Negritude is an inner state of Black people. It is a distinctive mode of being, which can be seen in their way of life, and which constitutes their very identity (Irele, 1981). Irele finds value in the concept of Negritude “insofar as it reflects a profound engagement of African minds upon the fundamental question of the African being in history…”

At a time when Africans are trying to experiment with new ideas and institutions, adapt them to their needs in the light of their traditional value systems, there is the need for a sustained belief in oneself, and this belief can be generated and kept alive by an ideology. This has been, and still is, the function of Negritude. (pg. 86; Ghanaian scholar P. A. V. Ansah, cited in Irele, 1981)

Although the concept of Negritude is not without its critics, if one accepts its premise there are important implications for people of the Black diaspora (Irele, 2001). **Nigrescence** has been described as the process of converting from Negro to Black, i.e., rejecting the deracination imposed by Whites and embracing traditional African values and a Black identity (Parham, 2002; Parham et al., 1999; Tembo, 1980). This process of searching for one's identity can be very powerful, leading perhaps to a positive self-identity or, at least, serving as a buffer against racism and oppression (Parham & Parham, 2002). For additional information on the importance of identify formation and the development of negative identity, I refer you back to the discussion of negative personality development among Black Americans in the chapter on Erik Erikson. But what triggers this critical search for one's identity?

For people of African descent in places such as the United States, the process of nigrescence seems to follow four stages: **pre-encounter**, **encounter**, **immersion-emersion**, and **internalization** (Parham, 2002; Parham et al., 1999). In the pre-encounter stage, the indivdiual views the world from a White frame of reference. They think, act, and behave in ways that devalue and/or deny their Black heritage. Then, however, they encounter personal and/or social events that do not fit with their view of society. Muhammad Ali (formerly Cassius Clay) described in vivid and shocking detail how he was refused service at a restaurant because he was Black, after he had won the Olympic gold medal in boxing and been given the key to the city by the mayor of Louisville, Kentucky (Ali & Ali, 2004)! The individual then becomes immersed in Black culture. This can be a psychologically tumultuous time. For some, everything of value must reflect some aspect of Black and/or African heritage. They withdraw from contact with other racial/ethnic groups, and strong anti-White attitudes and feelings can emerge. Eventually, however, the individual internalizes their Black identity and becomes more secure. The tension, emotionality, and defensiveness of the previous stage is replaced with a calm and secure demeanor. The individual becomes more open minded, more ideologically flexible, and although Black values move to and remain at the forefront, there is a general trend toward being more pluralistic and nonracist, and anti-White attitudes and feelings decline (Parham et al., 1999; see also Mbalia, 1995).

Some Issues for Modern Africa

In a fascinating book entitled Education for Self-Reliance, Julius Nyerere (1967) discussed the importance of building the post-colonial educational system in Tanzania. A fundamental premise, according to Nyerere, is that the educational system needed to serve the goals of Tanzania (see also Gichuru, 2005; Khoapa, 1980). Therefore, they had to decide what kind of society they were building. He said their society was based on three principles: equality and respect for human dignity, sharing of resources, and work by everyone and exploitation by none. Interestingly, these principles do not focus on academic content. The successful community life of the village was more important. Social goals, the common good, and cooperation were all emphasized over individual achievement. Nyerere considered it

particularly important to avoid intellectual arrogance, so that those who became well educated would not despise those whose skills were non-academic. “Such arrogance has no place in a society of equal citizens” (pg. 8; Nyerere, 1967).

The aim of education in Tanzania became one in which students were to realize they were being educated by the community in order to become intelligent and active members of the community. Since education is provided at the expense of the community, the community is well within its rights to expect those students to become leaders and innovators, to make significantly greater contributions to the community than if they had not received an education (Bennaars, 2005; Sanyal & Kinunda, 1977). To this end, the training of teachers places ideology ahead of content. Student-teachers are taught: 1) the true of meaning of the Tanzanian concept of ujamaa (familyhood and socialism; a basis for planned, self-contained villages), 2) to be dedicated and capable teachers who understand and care for the children in their charge, and 3) to deepen the students’ general education. Since colonial rulers exploited, humiliated, and ignored the people of Africa for so long, it was believed that teachers should be of sound mind and sound body. Thus, admission into a teacher training program requires a good academic background, sound character, physical fitness, and a good all-around background (Mmari, 1979). Thus, teachers were trained to be good role models for the development of Tanzania and her people (see also Bennaars, 2005; Mbalia, 1995).

Discussion Question: In post-colonial Africa, some countries trained their teachers to educate children in being good citizens, and to be role models for how children should live their lives. Do you agree that teachers should play such an intentional role in helping to raise children? If not, does it seem that this was necessary for a time, given the history of colonization in Africa?

Although most of the work covered in this section has been done by writers, anthropologists, and sociologists, is there a role for more formalized personality testing in Africa? While this may not be the ideal approach for studying personality in African, it would allow us to compare this work with our Western concepts of personality (which constitutes the large majority of this book). There is preliminary evidence that the Five-Factor Model applies well when measuring the personality traits of Africans in Zimbabwe and South Africa (McCrae, 2002; Piedmont et al., 2002). Tembo (1980) developed an African Personality Scale on which Zambian college freshman did indeed demonstrate pro-African personality views (as opposed to anti-African personality views that would have indicated negative effects as the result of colonization; see, however, Mwikamba, 2005). Thomas Parham (2002) has used two personality tests designed to focus more specifically on the concept of an African personality: the **Racial Identity Attitude Scale** (RIAS; which Parham helped develop) and the **African Self-Consciousness Scale**. The RIAS measures the nigrescence construct, whereas the African Self-Consciousness Scale is grounded in Afrocentric theory (closer to the concept of Negritude). However, Parham has come to the conclusion that both of these tests fall short of measuring the core elements of what might be a common African personality, particularly spiritness and the potential biogenetic nature of African people (Parham, 2002). Thus, if this is an appropriate field of study, there certainly needs to be further investigation to determine whether Western concepts of personality assessment apply to the essence of African personality.

A Final Thought

One of the most widely recognized cultural distinctions in psychology today is the difference between individualistic, Western cultures and collectivistic, Eastern cultures. In Western societies, such as the United States of America, the individual not only has the freedom to seek purely personal advancement, it is expected of them. In contrast, the individual in countries such as China is expected to subordinate their own desires and ambitions for the good of the family and their community. With regard to a broad view of the African personality, we find a middle ground. There is significant individual freedom, but individuals are expected to serve their family and community. As a result, the individual also benefits from the overall success of the family and community. Thus, there is an ongoing interplay between the value of the individual and the values of family and community.

When this system works to its best potential, the results are people who flourish and can be proud of themselves. In the words of Dr. J. A. Sofola:

…the philosophy, the world-view, values and thought-patterns that form the ingredients or the building-blocks of the African Personality are live-and-let-live; the emphasis on wholesome human relations; the belief of the universality of man and communality of the people in the community; the historic sense of the unity of the human society as consisting of the ancestors, the living and the future generations yet unborn; spiritual attitude to life and attachment to communal life with communal responsibilities; a keen sense of rhythm; the conception of man as one roaming spirit in the chain of spirits in the universe…This is the personality which in its expression of an inward peace and stillness maintains an external composure and gait, head and chin raised high, and with deliberate, calculated dignified steps proclaims to the world: “Black is beautiful” and “I am black and proud of being so.” (pp. 143-144; Sofola, 1973)

Review of Key Points

• Roughly speaking, African can be divided into two distinct regions: North Africa and sub-Saharan Africa.

• Black Africans are the people of sub-Saharan Africa.

• The African worldview focuses on interdependence, rhythm, harmony, and spirit.

• Religion and spirituality are very important to Africans, and humans hold a privileged position at the junction of spirit and nature.

• Time is part of the cyclical rhythm of life, set by God, who knows what is right.

• The tribe is the basic unit of life, not the individual. The family includes a broad range of relatives, and caring for one’s family, especially the aged, is an important obligation.

• Marriage is an important institution, providing a rhythmic link between past, present, and future. A marriage is only complete when the couple has children.

• Hospitality to all people, including strangers, is expected, and a sign of good character.

• Ubuntu encompasses the social harmony that is valued in African life.

• Among the people of the African diaspora, ubuntu may be seen in action as the soul referred to in soul food, soul music, and soul brothers.

• Negritude represents the active claiming of those best elements of a common African personality by the people of Africa.

• Nigrescence is the process of converting from Negro to Black among the people of the African diaspora. It appears to follow four stages: pre-encounter, encounter, immersion-emersion, and internalization.

• In post-colonial African countries, educational systems were developed based on the ideals of the new nations. Teachers were trained to embody those ideals.

• There is some evidence for the usefulness of traditional approaches to studying personality in Africa with formalized testing. However, significantly more work needs to be done.

• Common elements of personality across Africa seem to represent a middle ground between the individualistic personalities seen in the West, and the collectivistic personalities seen in the East.

Attribution and critical-reading note

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Read historical and theoretical claims critically. A theory is a lens for inquiry, not a verdict about a child or family. Where source claims are dated or culture-bound, the new lecture supplies cautions and contemporary framing.

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Although Karen Horney was the first female psychoanalyst to openly challenge Sigmund Freud’s theories regarding the psychology of women, she abandoned this line of work when she came to the conclusion that culture was a more significant issue than gender in determining the psychology of women. Of course, that decision is based on separating gender from culture, which is not something that everyone would agree with. In the 1970s, the Stone Center was established, as a group of pioneering women began the work that led to a theory on the personality development and psychology of women based on a combination of seeking and forming relationships within a cultural context. This work continues today, and the theory is being expanded to include the personality development of all people, women and men.

However, not all female theorists have separated themselves so clearly from Freud’s basic theories. One of those women holds a special place in the history of psychology, since she was instrumental in helping both Sigmund and Anna Freud escape Austria as the Nazi regime came to power. Her name was Marie Bonaparte. Bonaparte was a princess, a great-grandniece of Emperor Napoleon I of France, a patient and student of Sigmund Freud, and in 1953 she wrote Female Sexuality (Bonaparte, 1953). Her perspective on women closely followed a traditional Freudian view. In contrast, Nancy Chodorow’s feminist perspective has significantly separated her perspective on the psychology women from that of Freud. Still, Chodorow has worked to combine feminist and psychoanalytic perspectives, in a manner similar to the object relations theories put forth by the Neo-Freudians, and she has focused on the unique female experience of mothering.

We will begin this chapter by examining the work of Princess Bonaparte, as an example of a female theorist who remained true to Freud’s own theories. Then, we will examine the

alternative presented by the members of the Stone Center group. Finally, this chapter will conclude with a brief look at Chodorow's efforts to combine the psychoanalytic and feminist perspectives.

Feminine Psychology in the Freudian Tradition

Although Sigmund Freud believed that female psychology was the result of an incomplete and frustrated male development, he also acknowledged that he did not fully understand the psychology of women. A particularly interesting passage can be found in his New Introductory Lectures on Psycho-Analysis:

One might consider characterizing femininity psychologically as giving preference to passive aims. This is not, of course, the same thing as passivity; to achieve a passive aim may call for a large amount of activity…we must beware in this of underestimating the influence of social customs, which similarly force women into passive situations. All this is still far from being cleared up… (pgs. 143-144; Freud, 1933/1965)

So, Freud did suggest the possibility that cultural factors (social customs) play a role in the development of girls and women. Furthermore, he acknowledged that there was much more to learn about these developmental processes. Freud ended his lecture on femininity with the following:

That is all I had to say to you about femininity. It is certainly incomplete and fragmentary and does not always sound friendly. But do not forget that I have only been describing women in so far as their nature is determined by their sexual function. It is true that that influence extends very far; but we do not overlook the fact that an individual woman may be a human being in other respects as well. If you want to know more about femininity, enquire from your own experiences of life, or turn to the poets, or wait until science can give you deeper and more coherent information. (pg. 167; Freud 1933/1965)

Published in 1933, this was one of the last times Freud wrote about femininity and the psychology of women. Always the scientist, Freud suggested that future research will provide a better understanding of this topic. The Stone Center group, whose work we will encounter shortly, is perhaps the most complete effort made toward fulfilling Freud's expectations. First, however, let's consider the interesting work of Princess Marie Bonaparte, as one of the female psychodynamic theorists who adhered closely to Freud's perspective.

Princess Marie Bonaparte

Marie Bonaparte (1882-1962), Her Royal Highness Princess George of Greece, was a patient, student, and dear friend of Sigmund Freud. She was the great-grandniece of Napoleon Bonaparte, Emperor of France, and she married Prince George of Greece in 1907. As a wealthy aristocrat, she was able to help both Freud himself and the financially struggling International Psychoanalytic Publishing House (known more commonly as the Verlag, from its name in German). It was Marie Bonaparte who paid a ransom to the Nazis in Austria in order to secure Freud's release as World War II approached. Earlier, she had used her wealth to help support the Verlag, which Freud had established to provide a means for publishing a variety of works on psychoanalysis (Gay, 1998; Jones, 1957). However, Bonaparte was far more than just a wealthy colleague.

Bonaparte shared Freud's interest in antiquities, and often helped him find the best pieces for his collection (M. Freud, 1983). She also loved dogs, particularly Chows, and Freud came to love that breed as well. As Freud, his wife Martha, and their daughter Anna waited to escape Austria in 1938, Freud and Anna spent some of their time translating books and articles into German. One of those books was entitled Topsy, written by Bonaparte about her favorite dog (M. Freud, 1983; Jones, 1957). But it was not just a simple matter of waiting to leave Austria. In a particularly poignant story, Martin Freud described the time when his sister Anna was arrested by the Gestapo. Bonaparte was with her, and demanded to be arrested as well. However, at that point in time the Nazis were still intimidated by members of the royal houses of Europe, and Anna Freud was taken alone. She was released later, but Freud is said to have paced all day in his house worrying about her (M. Freud, 1983). When the Freud family finally left Austria for England, only Bonaparte was able to safely transfer their gold out of the country. She did so by sending the gold to the king of Greece, who then sent it to the Greek embassy in London (Jones, 1957). Thus, Freud and his family were financially secure upon reaching London, and Freud was able to repay the ransom that Bonaparte had paid for his release.

Bonaparte also served the field of psychoanalysis in an important way other than her own work. Early in Freud's career, during the time when he underwent his own psychoanalysis, he had a very close friend named Wilhelm Fliess. So close were these friends, and at such a critical time in Freud's career, that their correspondence contained a great many intimate details. In addition to personal correspondence, Freud sent many scientific notes about his theory to Fliess. When Fliess died in 1931, his widow asked Freud to return the letters Fliess had written to Freud. However, Freud had destroyed all of them years earlier, and he wanted her to do the same to his letters. However, she chose to sell the letters to a bookseller. The bookseller then sold them to Bonaparte. When Bonaparte told Freud that she had them, he insisted that she destroy them. She refused, however, and those letters eventually became available to the fields of psychology and psychiatry (Gay, 1998; Jones, 1953).

As Bonaparte became involved in psychoanalysis professionally, Freud both admired and supported her work (Gay, 1998; Jones, 1957). In a letter to Bonaparte after Freud had reviewed her paper on psychoanalysis and time, Freud wrote "The work does you honor." (cited in Jones, 1957). She was also active in establishing the growing field as a whole. She had helped to establish a psychoanalytic society in France, and Freud later nominated her to be vice-president of the International Psychoanalytic Association. In nominating her, Freud considered her a worthy candidate:

…not "only because one can show her off to the outside world," but because she "is a person of high intelligence, of masculine capacity for work, has done fine papers, is wholly devoted to the cause, and, as is well known, also in a position to lend material aid. She has now become 50 years old, will probably turn away increasingly from her private interests and steep herself in analytic work. I need not mention that she alone keeps the Fr[ench] group together." (pg. 586; cited in Gay, 1998).

Female Sexuality

Bonaparte first met Freud as a patient seeking help with her **frigidity**. The psychoanalysis does not appear to have been successful, but the experience did provide Bonaparte with a new goal in life (Gay, 1998). Given the nature of her own problems, it should not be

surprising that her writings on psychoanalysis focused on sexuality. Female Sexuality is a wide-ranging book that draws heavily on Freud's work, but also relies on the works of Horney and Klein. In addition, she mentions Adler in a somewhat favorable light, though she concludes that both Freud and Adler failed to fully understand female sexuality (as they themselves acknowledged). Still, she bases most of her work on a paper of the same title written by Freud in 1931, although she attempts to describe the development of girls and women more thoroughly and with more consideration given to potential alternatives.

Bonaparte began by describing three types of women. The so-called "true women" are those who have succeeded in substituting the desire for a penis (penis envy) with a desire to have children (particularly a son); their sexuality is normal, vaginal, and maternal. They are known as **acceptives**. The second type, the **renouncers**, gives up all competition with men, fail to seek external love objects, and live largely unfulfilled lives. The **claimers**, however, deny reality and cling to both psychical and organic male elements present in all women. While it may appear to us today that the claimers are asserting themselves as being proud to be female, Bonaparte considered this position to cause an inability to adapt to one's erotic function. As Freud had described, in order for a girl to develop, she must transfer both her love object (from mother to father) and her erotogenic zone (from clitoris to vagina). According to Bonaparte, claimers who will not transfer their love object will become lesbians, those who do not transfer their erotogenic zone will never achieve fully satisfying sexual relationships as adults. In other words, they will be frigid. Evidence of the psychical nature of the problem of frigidity can be seen in the responsiveness of patients to psychoanalysis. Patients who are totally frigid, those who experience no pleasure in sexual activity, often respond well to psychoanalysis. However, women who are partially frigid, those who have more specifically not transferred their erotogenic zone from the clitoris to the vagina, tend to be very resistant to psychoanalysis. According to Bonaparte, partial frigidity is much more common than total frigidity. Partial frigidity is also much more common than men realize, since many women hide this reality by pretending to enjoy sexual activity.

In agreement with Freud, Bonaparte considers boys and girls to begin their sexual lives equally, in an oral erotic stage focused on the mother's breast. As they transition into the anal stage, there are the beginnings of a contrast between active and passive forces: the expulsion of feces vs. the retention of feces. The important activity of toilet training begins in this stage, and so social conditioning is also coming into play. Although Bonaparte, like Freud, continued to emphasize biological factors in sexual development, the acknowledgement that sociocultural factors related to toilet training come into play lays the foundation for girls being pushed toward the passive role that strict Freudians believe they must play.

In the transition from the anal stage to the phallic stage, the interplay between active and passive forces that were present during the anal stage takes a different direction in boys and girls. Very simply, since the boys penis actively protrudes, and his love object can continue to be his mother (or, later, other women as substitutes), the boy will develop an active relationship with the world around him. Girls, however, ultimately need to transfer their sexuality from the clitoris (which had been related to a small penis until this point) to the vagina, a passive organ with regard to sexuality. Girls must also transfer their love object to their father (or, later, to other men), and accept the physical penetration that is required for sexual intercourse. In this manner, according to Freud, Bonaparte, and others, boys grow

into aggressive men and girls grow into passive women. Provided, of course, that women accept their role.

Adding to the complexity of this process for girls, who need to transfer the libidinal cathexes from both the clitoris and the love object of mother, is the consequence of when the girl first experiences an orgasm. Since this potentially can occur at any time during the dynamic processes of transferring these libidinal cathexes, the first orgasm can have a variety of either positive or negative effects. For boys it is simply easier, since the penis is the one obvious source of sexual pleasure, and the boy never has to transfer his love object away from women (though it should transfer from the mother to another woman). This difference in sexual development is summed up by Bonaparte:

It is on these diverse superimposed courses that the edifice of female sexuality rises. Constitutional factors are its foundation, and life builds thereon. Finally, we see the feminine psychosexual structure in its main varieties, varieties more multiform even than those to which male sexuality is susceptible, centered as it is on the phallus, that highly differentiated organ developed to serve the male erotic function. (pg. 140; Bonaparte, 1953)

As an interesting side note, Bonaparte also discussed some of the research that had been done up to that point in time on female circumcision/mutilation, particularly in primitive cultures. She speculated on how psychoanalytic theories of sexuality might apply to those practices, and how societies today might compare to primitive cultures that have retained such practices. Can we really say that things have changed since Bonaparte wrote the following passage?

It would appear that humans, living in communities, cannot dispense with sexual repression of some kind and that, if it has not succeeded in coming from within, it must go on coming from without. (pg. 157; Bonaparte, 1953)

Before we turn our attention to the Stone Center group, I would like to mention something that may have already entered your mind. This book is about personality, not sexuality. While it may be true that sexuality is an important part of life, it is certainly not the same thing as one’s personality. Unless, of course, you happen to be a strict Freudian theorist, as was Bonaparte. She does tend to equate the psychology of women with their sexuality. The psychologists of the Stone Center group, however, have moved beyond this biased view of the psychology of women.

Discussion Question: In keeping with Freud’s original theory, Bonaparte believed that sexual development is much more difficult for girls than it is boys. Do you agree with that, and if you do, what is it that makes things so much more difficult for girls? Are there any unique challenges that only boys face?

Placing the Psychology of Women in Context: Sexism vs. Feminism

Sigmund Freud developed a theory of female sexuality that helped to explain his observation that most people in psychoanalysis were women. Karen Horney agreed that women suffer more than men, but she placed the blame on men, and the patriarchal culture that maintains special privileges for men only. Despite the fact that Freud and Adler admitted that they did not fully understand women, and that there were many women among the neo-Freudians, it was a long time before a unique perspective on the psychology of women developed.

In contrast to women like Princess Bonaparte and Helene Deutsch, the first leading female member of the Vienna Psychoanalytic Society (the very first female member, Hermine von Hug-Hellmuth, was murdered in 1924; Deutsch, 1973, Sayers, 1991) and the first person to devote an entire book to the psychology of women (a two volume set published in consecutive years; Deutsch, 1944, 1945), Jean Baker Miller and her colleagues at the Stone Center developed a unique theory on the psychological development and personality of women. Although their theory, based on personal relationships and culture, developed in part as a result of increasing interest in feminist studies in the 1960s and 1970s, the work that continues today strives to include the personality development of all people (women and men).

There are some women, however, who believe that a feminist perspective can be combined more readily with Freud's basic ideas (see, e.g., Mitchell, 2000). Nancy Chodorow has worked to combine both psychodynamic and feminist ideas into a comprehensive theory. Although the result is basically an object relations theory, Chodorow's work has been reserved for this chapter due to her inclusion of the feminist side of the perspective.

It is also important to note that the work of the Stone Center group and Nancy Chodorow is much more contemporary than that of Bonaparte, Deutsch, and many of the neo-Freudians discussed in this book (most of whom are no longer alive). Thus, the development of feminist perspectives on the psychology of women continues today.

Human Relations and a Modern Perspective on the Psychology of Women

Despite the valuable contributions of women included among the neo-Freudians, and Horney's suggestion of womb envy as a powerful counterpart to penis envy, theories on the psychology of women remained framed within a psychodynamic perspective. Until, that is, the 1970s, when Jean Baker Miller and a group of women colleagues created a revolution in our potential understanding of the psychology of women.

Jean Baker Miller and the Stone Center Group

In 1974, Wellesley College in Massachusetts established the Center for Research on Women, and in 1981, the Stone Center for Developmental Services and Studies was established. Working in collaboration as the Wellesley Centers for Women (WCW), the Center for Research on Women conducts a variety of interdisciplinary studies on matters related to gender equity, while the Stone Center focuses on psychological well-being and a comprehensive understanding of human development, particularly the psychological development of women. A wide range of information on the WCW can be found on their website (http://www.wcwonline.org).

Jean Baker Miller (1927-present) was a practicing psychoanalyst who had already written one book on the psychoanalysis of women when she published Toward a New Psychology of Women (Miller, 1976). This book has been credited with nothing less than changing the very way in which we study the psychology of women. Since the earliest work of Sigmund Freud, women were seen as inferior, and so-called feminine attributes (e.g., vulnerability, weakness, emotionality, helping others; see Miller, 1976) were seen as psychologically weak. Miller and her colleagues at the Stone Center have worked hard to change that perspective. Typically working in collaboration, publishing collections of writing in books such as Women's Growth in Connection (Jordan, Kaplan, Miller, Stiver, & Surrey, 1991), Women's Growth in Diversity (Jordan, 1997b), and The Complexity of Connection (Jordan,

Walker, & Hartling, 2004), they developed a relational model of human development that focuses on **connections, disconnections, mutuality,** and **empathy**. Examples of how relationships can be damaged when one person seeks connection but the other person seeks to disconnect are all around us. Miller presented an example from a patient she identified as Doris. Doris was trying to share with her husband how upset she was after a day of finding it very difficult to deal with her colleagues at work:

He listened for about ten minutes. That's about his limit. Then he said, "Aw, don't let the bastards upset you." That's just the sort of thing I suspect. It sounds fine and even supportive. But it really means, "Shut up. I've heard enough." (pg. 100; Miller, 1976)

More recently, as members of the Stone Center became increasingly aware of the role of culture in development, the relational model evolved into the **relational-cultural theory** (RCT)****of human development (Jordan & Walker, 2004). The inclusion of culture in the theory should not be underestimated or taken for granted. Psychological theories are not immune from the bias inherent in societies that seek to maintain their hierarchical power structures. Western societies are highly individualistic, and when individuality is favored in our theories the result can be unfortunate:

In a culture that valorizes separation and autonomy, persons with cultural privilege can falsely appear more self-sufficient and so will be judged as healthier, more mature, more worthy of the privilege the society affords. Those who enjoy less cultural privilege (whether by virtue of race, ethnicity, sexual orientation, or economic status) will more likely be viewed as deficient and needy. They are more likely to be subject to systematic disadvantage and culture shaming. (pgs. 4-5; Jordan & Walker, 2004)

Relational-Cultural Theory

Miller established the foundation of RCT by addressing two fundamental differences in status and power that are part of human life: the differences between children and adults, and the differences between boys/men and girls/women. Children lack the privileges of adulthood, but this is temporary, and it is the role of parents and other adults (e.g., teachers) to help children grow up. In most modern cultures, women have typically lacked the privileges of men, and to a large extent that continues today. Since male/female differences are permanent, cultural phenomena usually develop in which men seek to maintain their power and status over women. This is the reason why psychology, a field traditionally dominated by men, has equated feminine attributes with psychological pathology (Miller, 1976). Curiously, not only men participate in this bias. Anna Freud did not challenge her father's views on girls and women, Melanie Klein claimed to be closer to Freud's point of view than even his daughter was, and Marie Bonaparte believed that women who do not accept the role defined for them by men would never be able to experience sexual satisfaction. It is not uncommon for a subordinate group to participate in this adaptive role, according to Miller, and as a result women may have gained their greatest advantage: the responsibility, and with it the privilege, of the intense emotional connection necessary to raise a child (Miller, 1976).

Considering the primary object relation necessary for a child to grow and thrive, the relationship between a mother and her infant, a relationship in which the mother serves the child first, many feminine attributes take on new meaning. Vulnerability, weakness, helplessness, emotionality, participating in the development of others, cooperation, and

creativity are all essential to giving oneself over to others, which is necessary to care for a baby, while at the same time allowing that baby to develop its own sense of mastery over the world and its own sense of individuality. Should it be surprising that women want to relate to other adults in the same way? According to Miller, one of the greatest difficulties men face in relationships with women is that men actually want to reclaim those very same elements of personality that men have delegated to women, and that gave rise to the woman's defined role in society. In accomplishing this task, as women advance their own place within society, men will have to adapt their coping strategies (Miller, 1976).

Now let us consider the essential elements of RCT. In RCT, the concept of object relations is viewed in light of connections vs. disconnections. People seek connections: family, friends, clubs, church groups, neighbors, the list goes on. Very few people live in isolation, and fewer still want it that way. But this raises a question about the meaning of the word "self." If a woman's experience is based on connections, do women develop a sense of self, and what is the nature of that self? Miller (1991) suggests that we don't get caught up in technicalities regarding the words we use to define this construct, but simply accept an open-minded definition of self. It appears to her that boys do develop a more clearly delineated sense of self, whereas girls may develop a more encompassing sense of self. Women do talk more about relationships, but not because they want or need to be either dependent or independent. Women simply want to be in relationships with others, to be connected. Looking more closely at the meaning of being "dependent" in a relationship, Stiver (1991) suggests that women often adopt a role of apparent dependence in relationships with men in order to connect with them in a manner acceptable to the man's gender role perspective. It does not appear to her that women are any more dependent in relationships than men, but when they do seek connection they do so by whatever means necessary. So when it is necessary for forming connections, Stiver considers "to depend" on another as part of an interpersonal dynamic:

I would like to define dependency as: A process of counting on other people to provide help in coping physically and emotionally with the experiences and tasks encountered in the world when one has not sufficient skill, confidence, energy, and/or time. I have defined it as a process to stress that it is not static but changes with opportunities, circumstances, and inner struggles. (pg. 160; Stiver, 1991)

Making successful connections involves two other important processes: mutuality and empathy. These closely related constructs come into play in meaningful relationships. Mutuality refers to both participants (or more, as the case may be) in a relationship being fully engaged in the connection. Each person is interested in and aware of the other, they are willing and able to share their thoughts, feelings, and needs, they do not manipulate each other, they value the connection, and they are open to change. Perhaps most importantly, they also experience empathy with other persons (Jordan, 1991a). Empathy, according to Jordan (1991b), is "an understanding of that aspect of the self that involves we-ness, transcendence of the separate, disconnected self." Jordan acknowledges a connection between her views and those of Kohut, who considered empathy an essential aspect of the mirroring that helps an infant to first see itself through the eyes of another as it plays with its mother (see also Mitchell & Black, 1995; Strozier, 2001). Empathy is a complex cognitive and emotional process necessary for a sense of separateness within connection, and self-empathy is an important therapeutic construct (Jordan, 1991b). Interactions of such intimacy are not new to object relations theory, but are usually only

considered in the context of the earliest relationship between mother and child. RCT considers mutuality and empathy as essential attributes of connections made by adults, particularly connections made by women. An often overlooked consideration is that mutuality and empathy need to be taught and learned. For example, Winnicott's "good enough mother" does not simply appear when a child is born (Surrey, 1991).

Discussion Question: Relational-cultural theory proposes that people seek connections in their lives, such as family, friends, church groups, clubs, etc. What groups do you consider yourself to be a member of, and how important to you is that membership? Do you, or people you know, consider the groups they belong to as more important than themselves?

According to relational-cultural theory (RCT), women seek connections in their lives, and support each other with mutuality and empathy. These attributes have helped women in their role as the primary caregivers for our children. However, as a cultural theory, RCT suggests that these so-called feminine attributes are the result of gender roles, not some inherent difference between males and females.

The source of most suffering in life, according to RCT, is disconnection. An acute disconnection can often be recognized by the loss of energy in the moment. This may be followed by negative emotions, such as sadness, anger, or depression. There may be a heightened sense of self-consciousness and relational awareness may slow down, we may even become immobilized (Jordan, 2004). After repeated disconnections we may become fearful of turning to others for help and support, even when we need it most. This has been referred to as the **central paradox of connection/disconnection**. When we hurt someone we love, or are hurt by someone we love, the conflict often leads to withdrawal and the development of **strategies of disconnection**. As a consequence of this ongoing process, although we all share a desire to connect with others, those who have been hurt by loved ones believe that they can connect with others only if they hold back part of themselves when they try to connect. Within the context of RCT, this desire for connection, while holding back from it, is also known as the **central relational paradox** (Miller et al., 2004).

This may very well be a significant factor in the fact that so many marriages end in divorce. Marriage is probably the most significant connection that adults in Western cultures choose to make, and so divorce would also be the most significant disconnection. There is certainly no easy answer for the high divorce rate, but an interesting possibility has been suggested by Harville Hendrix, who specializes in marital therapy. Hendrix (1988) believes that we choose a mate based on the unconscious recognition of characteristics they have in common with our parents, and that we hope through marriage to solve the psychological and emotional damage we suffered as children. In other words, we think we are connecting with our spouse, but we really want to reconnect with our parents. Unfortunately, this creates a false connection, a connection that cannot easily be resolved, especially given the apparently different communication styles of men and women (Gray, 1997, 1999, 2002, 2004; Vincent, 2006).

So how do we resolve problems of disconnection? Relational therapy is based on the idea that a therapist can provide a relationship in which the patient can experience connection, mutuality, and empathy. In looking at the therapeutic approaches of people like Sigmund Freud, Winnicott, and Kohut, Judith Jordan (1997a) argues that the actual engagement in the therapeutic relationship is aloof and disconnected. She feels that an obvious and overlooked aspect of therapy is that the more engaged the therapist is the more one

enhances the self, the other, and the relationship. Consequently, the therapeutic relationship can enhance one's capacity to be more whole, real, and integrated in other relationships as well. This relational perspective, which provides the basis for relational therapy, is based on three principles:

1. That people grow in, through, and toward relationships.

1. For women in particular, connection with others is central to psychological

well-being.

1. Movement toward relational mutuality can occur throughout life, through

mutual empathy, responsiveness, and contribution to the growth of each

individual and to the relationship. (Jordan, 1997a)

Once again we see the importance of empathy. Empathy involves more than just sharing the other person's feelings, it stresses the capacity to "feel into" the other person's experience (Jordan, 1997a; Mitchell & Black, 1995). Kohut emphasized the importance of empathy, as did the humanistic psychologist Carl Rogers. In contrast to empathy, the emphasis that has existed in therapy on the autonomous self as the "real self" can result in the creation of pathologically isolated individuals, individuals who feel self-sufficient, but who are really disconnected from others. Relational therapy does not focus on the self, but rather on relationships. To accomplish this goal, the therapist must be willing to respond to their patients in an authentic manner (Miller, Jordan, Stiver, Walker, Surrey, & Eldridge, 2004). If the therapist can convey to the patient that they are moved, the patient will be moved, knowing that her/his thoughts and feelings have reached another person, they do matter, and they can be part of a mutual experience. This is connection, and appears to be the key source of change in relational therapy (Miller et al., 2004).

Discussion Question: Relational therapy focuses on providing an environment for the patient to experience connection, empathy, and mutuality. It requires an engaged therapist. What are your ideas about how therapy should be conducted?

Connections Across Cultures: Janet Surrey and

Eastern Perspectives on Human Relations

Janet Surrey is one of the founding members of the Stone Center group, and for over 20 years she has been working to synthesize Buddhist mindfulness with relational-cultural theory and relational therapy. Most people think of mindfulness meditation as a solitary activity, but at its core is a desire to connect with the universal spirit that we all share. So, from the latter perspective, mindfulness meditation fosters a deep connection with others. Connecting with others is also at the core of relational therapy (with disconnection seen as the primary cause of suffering in life). Thus, the practice of mindfulness meditation can enhance the connections sought in relational therapy, and relational therapy can enhance one's attention to the present moment in both relational therapy and in one's everyday relationships.

Of particular value for the therapist, the practice of mindfulness meditation can deepen one's empathic skills. According to Surrey, during a mindfulness-informed therapy session the skilled therapist is attentive to their own sensations, feelings, thoughts, and memories as the patient is describing the same psychological phenomena. This helps the therapist to both

experience the patient and attend to the flow of the relationship. Thus, the therapist can be fully aware of the shifting qualities of the connections and disconnections within the therapeutic relationship. Although therapists typically rely on verbal interaction, the practice of mindfulness offers a unique opportunity to experience a conscious silence. Just such a rare opportunity for silence in one's busy life can be created in the genuine connection that results from a healthy and meaningful relationship (Surrey, 2005).

Surrey is by no means alone in drawing connections between Buddhist mindfulness and either the value of relationships or relational forms of therapy. His Holiness the Dalai Lama has emphasized that human beings cannot live in isolation, our very nature is that we are social animals. Our communities, indeed our entire society, require us to live cooperatively. This cooperation is best accomplished through love and compassion. It is not enough, however, to care only for those who care for us. When we harbor negative emotions toward those whom we do not like, those negative emotions harm ourselves. Thus, the Dalai Lama considers it essential to cultivate equanimity, the ability to care for everyone equally, no matter whom they may be (Dalai Lama, 2001, 2002). Likewise, the widely respected Buddhist monk Thich Nhat Hanh (nominated for a Nobel Peace Prize by Martin Luther King, Jr.) stresses the importance of practicing mindfulness within a supportive group, and then extending the compassion that arises to all others (Thich Nhat Hanh, 1995, 1999).

Within the field of psychology, the well-known therapist/authors Jon Kabat-Zinn, who developed the Mindfulness Based Stress Reduction program, and Steven Hayes, the founder of , have emphasized the importance of relationships with other people and the world around us, as well as how mindfulness can help to enhance those relationships (Hayes, 2004; Hayes et al., 1999; Kabat-Zinn, 1990). In addition, personal relationships appear to be particularly important for a variety of groups in American society, including: African Americans (Belgrave & Allison, 2006; Cook & Wiley, 2000; Taylor et al., 2004), Native Americans (Axelson, 1999; Trujillo, 2000), aged individuals (Belsky, 1999; Hillier & Barrow, 1999), and those who are dying (Kubler-Ross, 1969, 1983). Indeed, the ability to form and maintain healthy relationships has been identified as a vitally important human strength and an important aspect of well-being (Berscheid, 2003; Cantor, 2003; Cloninger, 2004; Sears, 2003). Thus, by examining cross-cultural factors that aid in developing and maintaining healthy relationships and, therefore, a healthy personality, we can continue to move toward a psychology that benefits us all.

Nancy Chodorow's Psychoanalytic Feminism and the Role of Mothering

In 1974, Juliet Mitchell suggested that Freudian psychoanalysis offered an important means for understanding the psychology of women, and that feminism should embrace Freud's theoretical perspective. She did not suggest that Freud was necessarily right about the psychological development of women, but she did emphasize the importance of object relations theory and the interactions between mothers, their children, and families as a whole (Mitchell, 2000). Some 50 years earlier, Helene Deutsch had suggested that women do not seek to become mothers due to penis envy, but rather they want to replace passive femininity with an active role as a woman and mother (Deutsch was analyzed and trained by Sigmund Freud himself; Deutsch, 1944, 1945, 1973, Sayers, 1991). Deutsch (1973) wrote that she had great admiration for Marie Bonaparte as a person and a scholar (Deutsch knew Bonaparte personally), but Deutsch found little of interest in Bonaparte's strict application of Freudian theory to the psychology of women. The person best known today for attempting

to combine elements of Freud's theory with an objective perspective on a psychology of women is Nancy Chodorow (1944-present), a sociologist and psychoanalyst who has focused on the special relationship between mothers and daughters.

In 1978, Chodorow published The Reproduction of Mothering. Twenty years later, she wrote a new preface for the second edition, in which she had the advantage of looking back at both the success of her book and the criticism that it drew from some. Chodorow acknowledged that many feminists felt obliged to choose between a biologically-based psychology of women and mothering (the essential Freudian perspective) versus a view in which the psychology of women and their feelings about mothering were determined by social structure and cultural mandate. Chodorow believed that social structure and culture were important, but she insisted nonetheless that the biological differences between males and females could not be dismissed. Indeed, they lead to an essential difference in the mother-daughter relationship as compared to the mother-son relationship (Chodorow, 1999a).

According to Chodorow, when a woman becomes a mother, the most important aspect of her relationship with any daughter is the recognition that they are alike. Thus, her daughter can also become a mother someday. This special connection is felt by the daughter and incorporated into her psyche, or ego. It is important to remember that much of this is happening at an unconscious level. It is not as if women choose to favor their daughters over their sons, and it is not as if women reject their sons. Chodorow argues that it just simply happens, because of the biological similarity between females. As a consequence of this special relationship, daughters are subtly shaped in ways that lead to what we often think of as feminine attributes: a sense of **self-in-relation**, feeling connected to others, being able to empathize, and being embedded in or dependent on relationships. For Chodorow, the internalization of the mother-daughter relationship, from the daughter's point of view, is the development of a most important object relation. As adults, many women feel a desire to have children, which is often described as a maternal instinct or a biological drive (the feeling that their "biological clock" is ticking). As an alternative, Chodorow suggests that these feelings have instead been shaped by the unconscious fantasies and emotions associated with the woman's internal relationship to her own mother (Chodorow, 1999a).

In contrast to the development of daughters, Chodorow suggests that sons are influenced by the essential feelings of difference conveyed by their mother. Consequently, and in contrast to women, men grow up asserting their independence, and they will be anxious about intimacy if it signals dependence on another. In addition, within the cultural framework of society, men develop a greater concern with being masculine than women are concerned with their femininity (Chodorow, 1999a).

The cultural differences between men and women, as well as the early childhood differences in their relationships with their parents, create problems for the typical family structure. Since men tend to avoid relationships, they are unlikely to fulfill the relational needs that women have. In addition, young girls most likely experience their relationship with their father within the context of their relationship with their mother, whereas young boys have a more direct two-person relationship with their mother (in terms of heterosexual relationships; Chodorow, 1999a). Therefore, in order for a woman to balance the relational triangle she experienced with her mother and father, and the subsequent intrapsychic object-relational structure she developed, she needs to have a child. In other words, by

having children, women can “reimpose intrapsychic relational structure on the social world,” and they can relate to the father of their child in terms of a family structure they were familiar with in childhood. Furthermore, having a child recreates the intimacy a woman shared with her own mother.

One critique of The Reproduction of Mothering that Chodorow agreed with was her emphasis on a universal mother-daughter experience, within a heterosexual nuclear family. In her later writings, Chodorow emphasized individual subjectivity, still in relation to others, but also within a wider range of family structures and individual situations (Chodorow, 1989, 1994, 1999b). She felt that a balance between the principles of psychoanalysis and an understanding of culture was the best overall approach:

A psychoanalysis that begins with the immediacy of unconscious fantasy and feeling found in the clinical encounter illuminates our understanding of individual subjectivity and potentially transforms all sociocultural thought…At the same time, feminist, anthropological, and other cultural theories require that psychoanalysts take seriously the ways in which cultural meanings intertwine with and help to constitute psychic life. (pg. 274; Chodorow, 1999b)

Personality Theory in Real Life: The Experience of Mothering

When Helene Deutsch wrote the first books devoted entirely to the psychology of women, the second volume was devoted entirely to Motherhood (Deutsch, 1945). She described motherhood as providing a wonderful opportunity to directly experience a sense of immortality. She distinguished, however, between **motherhood** and **motherliness**. Motherhood, according to Deutsch, refers to the relationship between mother and child, which varies from individual to individual and from culture to culture. When Deutsch wrote of motherliness, she referred to both a quality of character that pervades a woman’s whole personality and emotional phenomena related to a child’s helplessness and need for care. In a motherly woman, one’s own need for love is transferred from the ego to the child, and this maternal love has the chief characteristic of **tenderness** (Deutsch, 1945). Of course, no two women experience motherhood in exactly the same way. Deutsch recognized two primary types of mothers. The first type is the woman whose world is opened to a new reality by the birth of a child. She feels no loss, and she develops her own personality fully only after having a child. The second type of mother feels restricted and impoverished by her children. Such women, according to Deutsch, have spent their emotionality on other pursuits (such as sexuality, or a career), and they lack sufficient libido to withstand the emotional burden of children:

The woman’s relation to her husband and family, her economic situation, and the position of the child in her existence, give a personal color to each woman’s motherliness. (pg. 55; Deutsch, 1945)

Deutsch had several miscarriages in the early years of her marriage, causing her a great deal of anxiety during the pregnancy that finally gave her a son named Martin. In her autobiography, she speaks both fondly and proudly of her only child, as well as of the wonderful relationship he shared with his father. With regard to being a busy, working woman during her son’s childhood, Deutsch wrote that this could only be worked out on individual basis, and with some necessary compromise (Deutsch, 1973).

Although fathers play a role in parenting, only a woman can really understand what it's like to be a mother. Two entertaining books written by mothers about their relationships with their children are good-enough mother (Syler, 2007) and Mother Shock: Loving Every (Other) Minute of It (Buchanan, 2003). In addition to stories about the joys of raising children, they also discuss the trials and tribulations as well. In accordance with the work of the Stone Center group, they also talk about how important it is for mothers, as women, to have meaningful relationships with friends both in and beyond their families. In a chapter entitled mommy needs a playdate, Syler writes:

…behind every good-enough mother is another good-enough mother with whom to commiserate, shop, or just hang out and have a crab-fest. It's a healthy dose of friendship that fuels us to fight another day. (pg. 189; Syler, 2007)

But what happens when a woman is not a good mother? Christine Lawson has studied the mothering abilities of women who suffer from borderline personality disorder. In Understanding the Borderline Mother (2000), Lawson has identified four types of borderline mother: the waif (characterized by helplessness and hopelessness), the hermit (characterized by perfectionism and worrying), the queen (characterized by demanding attention and feelings of emptiness), and the witch (characterized by a desire for power and the very real threat of being physically abusive). The tragic challenge for the children of borderline mothers, according to Lawson, is that our mother is the first thing any of must understand in our lives, and our survival depends on understanding her. Although borderline personality is highly resistant to treatment in therapy, as with any personality disorder, therapy can help women with borderline personality disorder to avoid passing on the condition to their children.

While it is understandable that mothering would be difficult for women suffering from psychological disorders, it also appears to be true that no such thing as a "maternal instinct" exists. Anthropologist Sarah Hrdy, who has been elected to both the National Academy of Sciences and the American Academy of Arts and Sciences, has studied infanticide in different cultures and other primate species. She found it quite surprising that some mothers (human, ape, and monkey) will contribute to the death of their own child (or children), and in some cases then mate with the male that killed them. While this is a complex issue, covered by Hrdy in a rather lengthy book entitled Mother Nature (Hrdy, 1999), she raises some profound questions. For example, if women instinctively love their children, why do so many women directly or indirectly contribute to their deaths? Since fathers contribute equally to the genetic makeup of the child, why haven't fathers evolved a greater interest in and commitment to caring for their children? And perhaps most interestingly, "just why did these little creatures evolve to be so plump, engaging, and utterly adorable?" (Hrdy, 1999).

Consider your own mother and your relationship with her. Do you consider her to have been a good-enough mother? Whether you are a man or a woman, how has your relationship with your mother affected your relationships with others (especially if you have children)?

Discussion Question: Chodorow proposed that women desire to have children in order to recreate the intimacy they had with their mother when they were a baby, and to balance their relationship with their husband in a manner similar to how they first experienced their father. Does this suggestion seem reasonable, or do you think there may be a more fundamental biological drive toward bearing offspring?

Review of Key Points

• Sigmund Freud acknowledged that he really did not fully understand female psychology.

• Within a strict Freudian paradigm, Marie Bonaparte described three types of women. There are women who accept their proper role in society, those who give up on meaningful relationships, and those who fight society. Bonaparte believed that the women who fight their role in society can never be fulfilled sexually.

• Bonaparte suggested that psychological development in boys is easier than it is for girls, since all children begin by loving a women (their mother), which is appropriate for boys. She also believed the physical structure of the penis made understanding sexuality easier for boys.

• Jean Baker Miller began addressing the differences in power and status that exist between boys/men and girls/women, and the cultural system that develop to maintain those differences.

• Since a woman must make major sacrifices to raise an infant, Miller believed that so-called feminine attributes are essential to accomplish that goal.

• According to Miller, and other theorists at the Stone Center, a woman’s experience is based on connections with others. This led to the formation of relational-cultural theory.

• Empathy and mutuality are essential to forming successful connections.

• Problems arise when people become disconnected. Repeated disconnection can make individuals afraid to form connections, resulting in the central paradox of connection/disconnection.

• Relational therapy seeks to provide an opportunity for patients to experience empathy, mutuality, and connection. It requires therapists to become fully engaged in the therapeutic process.

• According to Chodorow, the mother-daughter relationship is special because the mother experiences her daughter(s) as similar to herself. The daughter incorporates this special relationship as a primary object relation.

• A woman’s development of the special object relation regarding her mother leads to a need to balance her relational world by becoming a mother herself.

• Chodorow believed that since mothers experience their sons as different, boys begin to develop as more independent individuals.

CHAPTER 18 / CAC.902

Assessment and Case Formulation Across Theories

Assessment is the process of gathering information to understand a client and plan care. Case formulation is the organized hypothesis that links assessment to intervention. A diagnosis names a pattern; a formulation explains how the pattern developed and what maintains it. Doctoral counseling requires both. A counselor should be able to write a formulation that is theoretically coherent, culturally responsive, and testable. The formulation is a living document, not a verdict.

Assessment is multimethod

Assessment includes clinical interview, observation, collateral information, standardized measures, and functional analysis. With children, the counselor may use play, drawings, caregiver reports, teacher reports, and school records. No single source is complete. A child may appear fine at school and distressed at home. A parent may minimize problems because of stigma. A teacher may see only the behavior that disrupts class. The counselor triangulates.

The biopsychosocial-spiritual frame

A broad assessment considers biological factors, psychological processes, social context, and spiritual or meaning systems. Sleep, nutrition, medical conditions, medication, and neurodevelopment matter. A child with untreated ADHD may look oppositional. A child with anemia may look depressed. The counselor does not reduce the child to biology, but the counselor does not ignore it. Social factors include poverty, housing, discrimination, school climate, and family stress. Spiritual factors include faith, meaning, and community.

Formulation models

A cognitive-behavioral formulation may use the five-part model: situation, thought, emotion, behavior, physical sensation. A psychodynamic formulation may describe core conflict, defenses, and relational patterns. A systemic formulation may map sequences and functions. An attachment formulation may describe internal working models and caregiving cycles. An integrative formulation may combine these. The key is to state hypotheses clearly and identify what would confirm or disconfirm them.

The four P's

A simple formulation structure is predisposing, precipitating, perpetuating, and protective factors. Predisposing factors include history and vulnerability. Precipitating factors include recent triggers. Perpetuating factors include what keeps the problem going. Protective factors include strengths and supports. For a school-refusing child, predisposing factors may include anxiety temperament and family history. Precipitating factors may include a bullying incident. Perpetuating factors may include avoidance and parental accommodation. Protective factors may include a supportive teacher and a strong peer relationship.

Writing a testable formulation

A good formulation includes a mechanism and a prediction. "Amina's separation fear is maintained by avoidance and parental accommodation. If goodbye rituals are shortened gradually and the child experiences successful reunions, distress should decrease." The prediction can be checked. If distress increases, the counselor revises. Perhaps bullying is the main driver. Perhaps trauma needs treatment first. The formulation is not defended at all costs.

Clinical example

Thirteen-year-old Daniel, introduced in Chapter 3, is assessed with interviews, a mood measure, and a functional analysis of oppositional behavior. A psychodynamic formulation highlights fear of abandonment and testing. A behavioral formulation highlights escape from demands. A systemic formulation highlights the mother's inconsistent limits after the father's departure. The counselor writes an integrative formulation: Daniel's oppositional behavior protects against vulnerability and is reinforced by escape and caregiver guilt. The plan combines relationship repair, consistent limits, and emotion-focused work. The counselor monitors whether the relationship becomes safer and whether behavior improves.

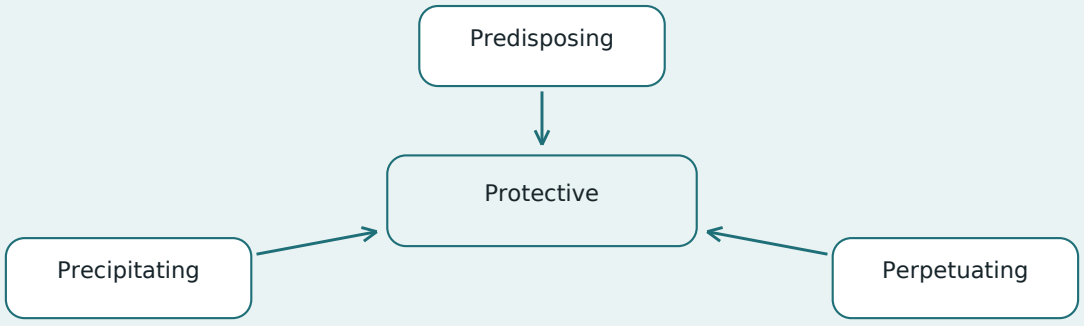
Common errors

Common formulation errors include confirming the favorite theory, ignoring culture, blaming the family, overdiagnosing, and failing to update. A counselor should ask what evidence would change the formulation. The counselor should also ask the client whether the formulation fits. A formulation that the client cannot recognize may be wrong or may need better language.

Reflection

Take a case you know. Write a four P's formulation. Then write one alternative formulation from a different theory. Compare what each makes visible. Choose a first intervention and state what outcome would make you revise the plan.

The four P's formulation



A testable formulation links history, triggers, maintenance, and strengths to a plan.

Attributed Foundational Reading

Attribution and critical-reading note

Adapted from Personality Theory in a Cultural Context by Mark D. Kelland, OpenStax CNX / Lansing Community College (2015), CC BY 4.0. Selection, organization, and PDF layout modified for this course; new lectures and course diagrams are separate additions.

Read historical and theoretical claims critically. A theory is a lens for inquiry, not a verdict about a child or family. Where source claims are dated or culture-bound, the new lecture supplies cautions and contemporary framing.

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When you first think of personality, what comes to mind? When we refer to certain people as being “personalities,” we usually mean they are famous, people like movie stars or your favorite band. When we describe a person as having “lots of personality,” we usually mean they are outgoing and fun-loving, the kind of person we like to spend time with. But does this tell us anything about personality itself? Although we may think we have an understanding of what personality is, professional psychologists always seek to move beyond what people think they know in order to determine what is actually real or at least as close to real as we can come. In the pursuit of truly understanding personality, however, many personality theorists seem to have been focused on a particularly Western cultural approach that owes much of its history to the pioneering work of Sigmund Freud.

Freud trained as a physician with a strong background in biomedical research. He naturally brought his keen sense of observation, a characteristic of any good scientist, into his psychiatric practice. As he worked with his patients, he developed a distinctly medical model: identify a problem, identify the cause of the problem, and treat the patient accordingly. This approach can work quite well, and it has worked wonderfully for medical science, but it has two main weaknesses when applied to the study of personality. First, it fails to address the complexity and uniqueness of individuals, and second, it does not readily lend itself to describing how one chooses to develop a healthy personality.

The diversity that is the human experience can be seen in the faces of the people around us.

Quite soon in the history of personality theory, however, there were influential theorists who began to challenge Freud’s perspective. Alfred Adler, although a colleague of Freud’s for a time, began to focus on social interest and an individual’s style of life. Karen Horney challenged Freud’s perspective on the psychology of women, only to later suggest that the issue was more directly related to the oppression of women as a minority, rather than a fundamental difference based on gender. And there were Carl Jung and Carl Rogers, two men profoundly influenced by Eastern philosophy. Consequently, anyone influenced by Jung or Rogers has also been influenced, in part, by Eastern philosophy. What about the rest of the world? Have we taken into account the possibility that there are other, equally valuable and interesting perspectives on the nature of people? Many fields in psychology have made a concerted effort to address cross-cultural issues. The primary purpose of this textbook is to address some of these different cultural perspectives, and to compare them to, and contrast them with, the traditional Western perspectives. In addition, we will examine the

relationships between the traditional approaches as well. In particular, the final section of this book introduces a number of paths developed throughout history to help people choose how to live their lives. Although each path is intimately identified with a religious perspective, the paths themselves represent more of a style of life. As we examine these perspectives, you will see that they are all quite similar in their essential elements, making it clear that the principles involved transcend religious culture. My hope is that when you have read this book, you will have a broad understanding of the field of personality, and an appreciation for both what we have in common and what makes us unique, as members of our global community.

Definitions and Descriptions of Personality

It would seem to make sense that we should begin our study of personality by defining the term. Unfortunately, there is no single definition that fits the variety of theories that have been developed in the field of personality research. Most psychologists agree that the term personality comes from the Latin word **persona**, a term referring to the masks worn by actors performing ancient Greek plays. Often there were not enough actors available to play all of the roles in a play, so they would wear these masks to let the audience know that they were playing different roles. But are our personalities just masks? Freud certainly considered the unconscious mind to be very important, Cattell considered source traits to be more important than surface traits, and Buddhists consider the natural world (including the self) to be an illusion. Adler believed the best way to examine personality is to look at the person’s style of life, and Rogers felt that the only person who could truly understand you is yourself. What definition could possibly encompass all that?

Still, we need a working definition as a starting point for discussion. Borrowing loosely from Allport’s definition of personality, personality can be viewed as the dynamic organization within an individual of various psychological factors that determines the person’s characteristic thoughts and behaviors. In simpler terms, a variety of factors blend together to create each person, and as a result of those factors the individual is most likely to think and act in somewhat predictable ways. However, given the complexity of human life, those predictions may prove to be elusive. Theodore Millon (1996, 2004; Millon & Grossman, 2005), a renowned clinician and theorist in the field of personality disorders, has sought a definition of personality broad enough to encompass both normal and abnormal personality. Millon describes the modern view of personality as a complex pattern of psychological characteristics that are deeply embedded, largely unconscious, and resistant to change. These intrinsic and pervasive traits arise from a complex matrix of biological dispositions and experiential learning, and express themselves automatically in nearly every aspect of the individual’s unique pattern of perceiving, feeling, thinking, coping, and behaving (e.g., Millon, 1996).

Another challenge we face in defining personality is how we approach the question in the first place. Traditionally, there have been two basic approaches to the study of personality: the **nomothetic** perspective and the **idiographic** perspective. The nomothetic perspective seeks to identify general rules that pertain to personality as a **construct** (a working hypothesis or concept used to identify something we can describe but not see, such as IQ or the self). Thus, it can be rather abstract, and often fails to appreciate the uniqueness of individuals. In contrast, the idiographic perspective focuses specifically on the individuality and uniqueness of each person. Although the idiographic approach often seems more

appealing to students, especially since it enhances their self-esteem by considering them as individually important, it is difficult for any theory of personality to encompass research that treats only one person at a time. Such a theory would naturally suffer from problems of generalizability, and may be useful for therapists working with one patient or client at a time, but it will not be particularly useful for enhancing our overall understanding of personality in general. It is important to note, however, that many early personality theories were based on individual case studies, and this critique is one that we will see several times in this book.

As is often the case in psychology, the best approach may be to attempt blending the nomothetic and idiographic perspectives, seeking the generalizability of the nomothetic perspective's general principles on personality and personality development - while maintaining an appreciation for the idiographic perspective's recognition of the value of an individual's unique character. Millon (1996) suggests an integrative approach to defining personality. Not only would an integrative approach combine the nomothetic and idiographic perspectives, it would also help to bring together the two broad traditions of clinical and applied psychology. Clinical psychologists are compelled by the nature of their work with patients, or clients, to try to understand the individual. Thus, they need to follow a more idiographic approach. In contrast, applied psychologists (e.g., experimental psychologists) are more construct-focused, and find the nomothetic approach more appealing and useful for developing generalizable theories on the nature of various aspects of personality. If personality can be defined in a satisfactory way by an integrative approach, then clinicians may benefit more from applied research, and experimental psychologists may see their work more directly applied in clinical settings where it may help people in our society.

In order to better understand how some of the different disciplines within the field of psychology contribute to our definition of personality, let's take a brief look at some of the widely recognized factors that come into play:

Discussion Question: The nomothetic and idiographic perspectives approach personality in very different ways. Do you believe that your personality can be described in a way that might also be used to describe the personalities of other people (maybe your friends), or do you feel it is necessary to describe each person as an individual?

Psychodynamic Factors

The very word "psychodynamic" suggests that there are ongoing interactions between different elements of the mind. Sigmund Freud not only offered names for these elements (id, ego, and superego), he proposed different levels of consciousness. Since the unconscious mind was very powerful according to Freud, one of the first and most enduring elements of psychodynamic theory is that we are often unaware of why we think and act the way we do. Add to that the belief that our personality is determined in early childhood, and you can quickly see that psychological problems would be very difficult to treat. Perhaps more importantly, since we are not aware of many of our own thoughts and desires, it would difficult or even impossible for us to choose to change our personality no matter how much we might want to.

Most psychodynamic theorists since Freud have expanded the influences that affect us to include more of the outside world. Those theorists who remained loyal to Freud, typically known as neo-Freudians, emphasized the ego. Since the ego functions primarily in the real

world, the individual must take into account the influence of other people involved in their lives. Some theorists who differed significantly from the traditional Freudian perspective, most notably Alfred Adler and Karen Horney, focused much of their theories on cultural influences. Adler believed that social cooperation was essential to the success of each individual (and humanity as a whole), whereas Horney provided an intriguing alternative to Freud's sexist theories regarding women. Although Horney based her theories regarding women on the cultural standing between men and women in the Victorian era, to a large extent her theory remains relevant today.

Learning and Cognitive Factors

As a species, human beings are distinguished by their highly developed brains. Animals with less-developed nervous systems rely primarily on instinctive behavior, but very little on learning. While the study of animals' instinctive behavior is fascinating, and led to a shared Nobel Prize for the ethologists Nikolaas Tinbergen, Konrad Lorenz, and Karl von Frisch, animal behavior remains distinctly limited compared to the complex learning and cognitive tasks that humans can readily perform (Beck, 1978; Gould, 1982). Indeed, the profound value of our abilities to think and learn may be best reflected in the fact that, according to Tinbergen's strict definition of instinct (see Beck, 1978), humans appear not to have any instinctive behavior anymore. Yet we have more than made up for it through our ability to learn, and learning theory and behaviorism became dominant forces in the early years of American psychology.

John B. Watson and B.F. Skinner are among the most famous and influential of American psychologists. Learning about their groundbreaking research on classical and operant conditioning is standard fare in psychology courses. More recently, Albert Bandura has enjoyed similar popularity and respect in the field of social learning theory. Anyone who has children knows full well how eagerly they observe us and mimic our actions and speech. An important aspect of the learning perspective is that our personalities may develop as a result of the rewards and/or punishments we receive from others. Consequently, we should be able to shape an individual's personality in any way we want. Early behaviorists, like Watson, suggested that they could indeed take any child and raise them to be successful in any career they chose for them. Although most parents and teachers try to be a good influence on children, and to set good examples for them, children are often influenced more by their peers. What children find rewarding may not be what parents and teachers think is rewarding. This is why a social-cognitive approach to learning becomes very important in understanding personality development. Social-cognitive theorists, like Bandura, recognize that children interact with their environment, partly determining for themselves what is rewarding or punishing, and then react to the environment in their own unique way.

As suggested by the blend of behaviorism and cognition that Bandura and others proposed, there is a close association between behaviorism and the field of cognitive psychology. Although strict behaviorists rejected the study of unobservable cognitive processes, the cognitive field has actually followed the guidelines of behaviorism with regard to a dispassionate and logical observation of the expression of cognitive processes through an individual's behavior and what they say. Thus, the ability of human beings to think, reason, analyze, anticipate, etc., leads them to act in accordance with their ideas, rather than simply on the basis of traditional behavioral controls: reward, punishment, or associations with unconditional stimuli. The success of the cognitive approach when applied to therapy, such

as the techniques developed by Aaron Beck, has helped to establish cognitive theory as one the most respected areas in the study of personality and abnormal psychology.

Biological Factors

Although humans may not exhibit instinctive behavior, we are still ultimately a product of our biological makeup, our specific DNA pattern. Our individual DNA pattern is unique, unless we happen to be an identical twin, and it not only provides the basis for our learning and cognitive abilities, it also sets the conditions for certain aspects of our character. Perhaps the most **salient** of these characteristics is **temperament**, which can loosely be described as the emotional component of our personality. In addition to temperament, twin studies have shown that all aspects of personality appear to be significantly influenced by our genetic inheritance (Bouchard, 1994; Bouchard & McGue, 1990; Bouchard et al., 1990). Even such complex personality variables as well-being, traditionalism, and religiosity have been found to be highly influenced by our genetic make-up (Tellegen et al., 1988; Waller et al., 1990).

Sociobiologists and evolutionary psychologists also emphasize the role of genetics and adaptation over time. Sociobiologists consider how biological factors influence social behavior. For example, they would suggest that men are inclined to prefer multiple sexual partners because men are biologically capable of fathering many children, whereas women would be inclined to favor one successful and established partner, because a woman must physically invest a year or more in each child (a 9-month pregnancy followed by a period of nursing). Similarly, evolutionary psychologists consider how human behavior has been adaptive for our survival. Humans evolved from plant-eating primates, we are not well suited to defend ourselves against large, meat-eating predators. As a group, however, and using our intellect to fashion weapons from sticks and rocks, we were able to survive and flourish over time. Unfortunately, the same adaptive influences that guide the development of each healthy person can, under adverse conditions, lead to dysfunctional behaviors, and consequently, psychological disorders (Millon, 2004).

Discussion Question: Some research suggests that personality is largely determined by genetics. Do you see similarities in your personality as compared to your parents, grandparents, brothers, sisters, etc.? Do you think that your environment, things like your community, your friends, television, movies, the Internet, etc., are more influential than your biological inheritance from your parents?

Inherent Drives

Freud believed that we are motivated primarily by psychosexual impulses, and secondarily by our tendency toward aggression. Certainly it is necessary to procreate for the species to survive, and elements of aggression are necessary for us to get what we need and to protect ourselves. But this is a particularly dark and somewhat animalistic view of humanity. The humanistic psychologists Carl Rogers and Abraham Maslow believed in a positive view of people, they proposed that each of us contains an inherent drive to be the best that we can be, and to accomplish all that we are capable of accomplishing. Rogers and Maslow called this drive **self-actualization**. Interestingly, this concept is actually thousands of years old, and having spent time in China, Rogers was well aware of Buddhist and Yogic perspectives on the self.

Somewhat related to the humanistic concept of self-actualization, is the existential perspective. Existential theorists, like Rollo May, believe that individuals can be truly happy only when they find some meaning in life. In Eastern philosophical perspectives, coming from Yoga and Buddhism, meaning in life is found by realizing that life is an illusion, that within each of us is the essence of one universal spirit. Indeed, Yoga means “union,” referring to union with God. Thus, we have meaning within us, but the illusion of our life is what distracts us from realizing it.

Discussion Question: Do you feel that you are driven to accomplish something great, or to find some particular meaning in life? Do you believe that there might be pathways to guide you, particularly spiritual or religious pathways?

Sociocultural Influences

Culture can broadly be defined as “everything that people have, think, and do as members of a society” (Ferraro, 2006a), and appears to be as old as the Homo genus itself (the genus of which we, as Homo sapiens, are the current representatives; Haviland et al., 2005). Culture has also been described as the memory of a society (see Triandis & Suh, 2002). Culture is both learned and shared by members of a society, and it is what the makes the behavior of an individual understandable to other members of that culture. Everything we do is influenced by culture, from the food we eat to the nature of our personal relationships, and it varies dramatically from group to group. What makes life understandable and predictable within one group may be incomprehensible to another. Despite differences in detail, however, there are a number of cultural universals, those aspects of culture that have been identified in every cultural group that has been examined historically or ethnographically (Murdock, 1945; see also Ferraro, 2006a). Therefore, if we truly want to understand personality theory, we need to know something about the sociocultural factors that may be the same, or that may differ, between groups.

In 1999, Stanley Sue proposed that psychology has systematically avoided the study of cross-cultural factors in psychological research. This was not because psychologists themselves are biased, but rather, it was due to an inherent bias in the nature of psychological research (for commentaries see also Tebes, 2000; Guyll & Madon, 2000; and Sue, 2000). Although some may disagree with the arguments set forth in Sue’s initial study, it is clear that the vast majority of research has been conducted here in America, primarily by American college professors studying American psychology students. And the history of our country clearly identifies most of those individuals, both the professors and the students, as White, middle- to upper-class men. The same year, Lee et al. (1999) brought together a collection of multicultural perspectives on personality, with the individual chapters written by a very diverse group of authors. In both the preface and their introductory chapter, the editors emphasize that neither human nature nor personality can be separated from culture. And yet, as suggested by Sue (1999), they acknowledge the general lack of cross-cultural or multicultural research in the field of personality. Times have begun to change, however. In 2002, the American Psychological Association (APA) adopted a policy entitled “Guidelines on Multicultural Education, Training, Research, Practice, and Organizational Change for Psychologists (which is available online at www.apa.org/pi/multiculturalguidelines/homepage.html). The year 2002 also saw a chapter in the prestigious Annual Review of Psychology on how culture influences the development of personality (Triandis & Suh, 2002). In a fascinating article on whether psychology actually

matters in our lives, former APA president and renowned social psychologist Philip Zimbardo (2004) identified the work of Kenneth and Mamie Clark on prejudice and discrimination, which was presented to the United States Supreme Court during the Brown vs. Board of Education of Topeka, KS case (which led to the end of school segregation in America) as one of the most significant impacts on American life that psychology has contributed to directly (see also Benjamin & Crouse, 2002; Keppel, 2002; Pickren & Tomes, 2002). Finally, an examination of American Psychologist (the principle journal of APA) and Psychological Science (the principle journal of the American Psychological Society) since the year 2000 reveals studies demonstrating the importance of cross-cultural research in many areas of psychology (see Table 1.1). So, although personality theorists, and the field of psychology in general, have been somewhat slow to address cross-cultural and diversity issues, in more recent years psychologists appear to be rapidly gaining a greater appreciation of the importance of studying human diversity in all its forms.

As mentioned in the opening paragraphs of this chapter, one of the primary goals of this book is to incorporate different cultural perspectives into our study of personality theory, to take more of a global perspective than has traditionally been done. Why is this important? It is actually very easy to point out the answer to that question. The United States of America has less than 300 million people. India has nearly 1 billion people, and China has over 1 billion people. So, two Asian countries alone have nearly 7 times as many people as the United States. How can we claim to be studying personality if we haven’t taken into account the vast majority of people in the world? Of course, we haven’t entirely ignored these two particular countries, because two of the most famous personality theorists spent time in these countries when they were young. Carl Jung spent time in India, and his theories were clearly influenced by ancient Vedic philosophy, and Carl Rogers spent time in China while studying to be a minister. So it is possible to draw connections between Yoga, Buddhism, psychodynamic theory, and humanistic psychology. Sometimes this will involve looking at differences between cultures, and other times we will focus on similarities. At the end of the book I hope you will appreciate not only the diversity of personality and personality theory, but also the connections that tie all of us together.

Discussion Question: Do you notice cultural differences around you every day, or do you live in a small community where everyone is very much the same? What sort of challenges do you face as a result of cultural differences, either because you deal with them daily or because you have little opportunity to experience them?

Some Basic Questions Common to All Areas of Personality Theory

In addition to the broad perspectives described above, there are a number of philosophical questions that help to bring the nature of personality into perspective. Thinking about how these questions are answered by each theory can help us to compare and contrast the different theories.

Is our personality inherited, or are we products of our environment? This is the classic debate on nature vs. nurture. Are we born with a given temperament, with a genetically determined style of interacting with others, certain abilities, with various behavioral patterns that we cannot even control? Or are we shaped by our experiences, by learning, thinking, and relating to others? Many psychologists today find this debate amusing, because no matter what area of psychology you study, the answer is typically both! We are born with a certain range of possibilities determined by our DNA. We can be a certain

height, have a certain IQ, be shy or outgoing, we might be Black, Asian, White or Hispanic, etc. because of who we are genetically. However, the environment can have a profound effect on how our genetic make-up is realized. For example, an abused child may become shy and withdrawn, even though genetically they were inclined to be more outgoing. A child whose mother abused alcohol during the pregnancy may suffer from fetal alcohol syndrome, the leading cause of preventable mental retardation, even though the child was genetically endowed with the possibility of being a genius. So the best perspective may be that our genetic make-up provides a range of possibilities for our life, and the environment in which we grow determines where exactly we fall within that range.

Are we unique, or are there common types of personality? Many students want to believe that they are special and truly unique, and they tend to reject theories that try to categorize individuals. However, if personality theories were unique to each person, we could never possibly cover all of the theories! Also, as unique as you may be, aren't many people, like your friends, similar to you? In order to understand and compare people, personality theorists need to consider that there are common aspects of personality. It is up to each of us to decide whether we are still willing to find what is unique and special about each separate person.

Which is more important, the past, present, or future? Many theorists, particularly psychodynamic theorists, consider personality to be largely determined at an early age. Similarly, those who believe strongly in the genetic determination of personality would consider many factors set even before birth. But what prospects for growth does this allow, can people change or choose a new direction in their life? Cognitive and behavioral theorists focus on specific thoughts, beliefs, and behaviors that are influencing our daily lives, whereas existential theorists search for meaning in our lives. Other theorists, such as the humanists and those who favor the spiritually-oriented perspectives we will examine, consider the future to be primary in our goals and aspirations. Self-actualization is something we can work toward. Indeed, it may be an inherent drive.

Do we have free will, or is our behavior determined? Although this question seems similar to the previous one, it refers more to whether we consciously choose the path we take in life as compared to whether our behavior is specifically determined by factors beyond our control. We already mentioned the possibility of genetic factors above, but there might also be unconscious factors and stimuli in our environment. Certainly humans rely on learning for much of what we do in life, so why not for developing our personalities? Though some students don't want to think of themselves as simply products of reinforcement and punishment (i.e., operant conditioning) or the associations formed during classical conditioning (anyone have a phobia?), what about the richness of observational learning? Still, exercising our will and making sound choices seems far more dignified to many people. Is it possible to develop our will, to help us make better choices and follow through on them? Yes, according to William James, America's foremost psychologist. James considered our will to be of great importance, and he included chapters on the will in two classic books: Psychology: Briefer Course, published in 1892 and Talks to Teachers on Psychology and to Students on Some of Life's Ideals, which was published in 1899. James not only thought about the importance of the will, he recommended exercising it. In Talks to Teachers…, he sets forth the following responsibility for teachers of psychology:

But let us now close in a little more closely on this matter of the education of the will. Your task is to build up a character in your pupils; and a character, as I have so often said, consists in an organized set of habits of reaction. Now of what do such habits of reaction themselves consist? They consist of tendencies to act characteristically when certain ideas possess us, and to refrain characteristically when possessed by other ideas (p. 816).

Table 1.1: Sampling of Cross-Cultural Research in Select Psychology Journals Since the Year 2000	
Attachment	Chao, 2001; Gjerde, 2001; Kondo-Ikemura, 2001; Posada & Jacobs, 2001; Rothbaum et al., 2000; Rothbaum et al., 2001; van Ijzendoorn & Sagi, 2001
Child Development	Callaghan et al., 2005; Goldin-Meadow & Saltzman, 2000; Lal, 2002
Cognitive Dissonance	Kitayama et al., 2004
Cognition and Creativity	Antonio et al., 2004; German & Barrett, 2005; Hong et al., 2000; Leung et al., 2008; Norenzayan & Nisbett, 2000; Tomasello, 2000
Conflict and Perceptions of Safety	Eidelson & Eidelson, 2003; Graham, 2006; Juvonen et al., 2006; Van Vugt et al., 2007
Cooperation	Wong & Hong, 2005
Cultural Research and Cultural Competency in Psychotherapy	Goldston et al., 2008; Heine & Norenzayan, 2006; Leong, 2007; Matsumoto & Yoo, 2006; Smith et al., 2006; Sue, 2003; Vasquez, 2007; Whaley & Davis, 2007
Education	Tucker & Herman, 2002
Emotion	Elfenbein & Ambady, 2003; Frijda & Sundararajan, 2007; Hejmadi et al., 2000; Tsai, 2007
Globalization, Nationality, Race Relations	Arnett, 2002; Heine et al., 2008; Henry & Hardin, 2006; Inglehart et al., 2008; McCrae & Terracciano, 2006; Sue, 2004; Tropp & Pettigrew, 2005
Family Dynamics	Dudley-Grant, 2001; Halpern, 2001; Kameguchi & Murphy-Shigematsu, 2001; Kaslow, 2001
Goal-Seeking Behavior	Elliot et al., 2001; Markus et al., 2006
Intelligence	Daley et al., 2003; Sternberg, 2004
Learning	Gurung, 2003; Li, 2003; Li, 2005; McBride-Chang & Treiman, 2003; Tweed & Lehman, 2002; Tweed & Lehman, 2003
Memory	Cohen & Gunz, 2002; Fivush & Nelson, 2004

Neural Substrates of Attention	Hedden et al., 2008
Perception and Spatial Representation	Bar-Haim et al., 2006; Cohen & Gunz, 2002; Dobel et al., 2007; Feng et al., 2007; Ji et al., 2001; Kitayama et al., 2003; Leung & Cogen, 2007; Maass & Russo, 2003; Miyamoto et al., 2006
Self, Self-esteem, Social Perspective	Perunovic & Heller, 2007; Wang, 2006a,b; Wu & Keysar, 2007; Yamagishi et al., 2008; Yamaguchi et al., 2007
Stress Responses	Taylor et al., 2007

Table 1.2: Brief Comparison of Factors Influencing Personality	
Psychodynamic Factors	Emphasis is on the unconscious mind, interactions between elements of the mind, early childhood experiences, stages of development, defense mechanisms, etc.
Learning and Cognitive Factors	Emphasis is on environmental stimuli and/or thought patterns that predictably influence behavior; focus is on observable behavior or identifiable thoughts.
Biological Factors	Emphasis is on genetic factors, which set ranges within which the individual may develop. This approach does not ignore the environment, but genetic factors (e.g., inborn traits and temperament) may cause different environmental influences to be experienced in similar ways, or conversely, may cause similar environmental influences to be experienced in different ways.
Inherent Drives	Humanistic psychologists focus on self-actualization; existentialists and spiritually-oriented psychologists focus on the search for meaning in one’s life.
Sociocultural Influences	Cross-cultural and multicultural psychologists remind us that all of the above categories must be considered in terms of the rich diversity that is the human experience. Addresses both the differences between and the similarities among groups of people around the world.

Personality as a Discipline within the Field of Psychology

As difficult as it may be to define personality, it is important to know something about it. Personality is probably the most important field in psychology. Understanding who we are as individuals, and why we think certain thoughts and do certain things is the starting point for addressing clinical issues, abnormal psychology, and health psychology, it is the ultimate goal of studying human development, and it is the point from which we begin to address social psychology. Without an appreciation of the individual, without concern for each person, these other areas of psychology become little more than academic subjects.

Personality as a Common Thread in the History of Psychology

Most historians identify the starting point of the modern field of psychology with the experimental psychologists, particularly the establishment of Wilhelm Wundt's laboratory in Leipzig, Germany in 1879. Personality theorists, however, were not far behind. Freud, Adler, and Jung were all beginning to work in the field of psychiatry (they were medical doctors) in the late 1800s as well. If one wants to put an official date on the start of modern personality theory, it would most likely be 1900, the year in which Freud published "The Interpretation of Dreams (Freud, 1900/1995)." Since Freud's theories were based on his work with patients, this date is also the beginning of a relationship between personality theory, abnormal psychology, and psychotherapy, a relationship that continues today. Early psychodynamic theorists were also influential in developmental psychology and school psychology. Freud began his theory of personality with a proposed series of developmental stages: the five psychosexual stages. Erik Erikson, another well known psychodynamic theorist, is probably better known as a developmental psychologist for his eight-stage theory of psychosocial crises. Adler considered the early years of childhood so important that he felt parents and teachers should have access to training and counseling in the schools. With the endorsement of the minister of education, Adler and his colleagues established child guidance centers in many public schools.

As psychodynamic theory dominated the European scene in the early 1900s, America was largely influenced by behaviorism. Behavioral theorists also considered personality within their domain, suggesting that personality is learned. John B. Watson boasted that he could use behavioral principles to direct any child into any given career path. B.F. Skinner constructed an advanced version of his famous "Skinner Box," which was typically used to train rats or pigeons, to raise and care for children during the early years of life. As behaviorism continued to develop, social learning theorists focused on a rich mixture of imitation, observation of the rewards and punishment experienced by others, expectations, and personal assessment of the value of potential rewards and punishers.

Against this backdrop of psychodynamic theory and behaviorism, Rogers and Maslow became the leading advocates of a new and openly positive view of human development, referred to most commonly as humanistic psychology. This new field emphasized self-actualization, though self-actualization itself does not appear to have been a new concept. It closely resembles the enlightenment described by Yoga and Buddhism (each of which is thousands of years old), though Yoga and Buddhism ultimately reject the existence of the self. Is there a relationship between Eastern philosophical perspectives and humanistic psychology? Rogers had traveled throughout Asia, particularly in China, and Maslow had studied with renowned psychodynamic theorists who were fascinated by Buddhism (such as Horney), so both were well acquainted with the basics of Eastern philosophical thought.

Closely related to the behavioral perspectives, cognitive theories of personality are also prominent in psychology. Today, the use of non-invasive brain imaging techniques (e.g., functional magnetic resonance imaging), which function in real time, have made the study of cognitive processes one of the most exciting areas of psychological research. Studying cognition is hardly new, however, since the earliest studies of consciousness can be traced to William James in the late 1800s. Still, what James was able to study over 100 years ago is completely different than what modern cognitive neuroscientists are able to study today. Nonetheless, if we can find something in common between the studies of over a century ago

and the research of today, perhaps we will really begin to understand the complexity and diversity of personality.

Positive Psychology and Spirituality: New Directions in the Field of Personality

Young children enjoy helping even younger children. Positive psychology encourages us all to study and promote such behavior.

In 1998, Martin Seligman, then president of APA and author of What You Can Change & What You Can't (1994), urged psychologists to rediscover the forgotten mission of psychology: to build human strength and nurture genius. Seligman called this new area Positive Psychology (for thorough overviews see Compton, 2005 or Peterson, 2006). In 2000, American Psychologistpublished a special edition on happiness, excellence, and optimal human functioning (American Psychologist, Vol. 55, Number 1, 2000; with an introduction by Seligman & Csikszentmihalyi, 2000). The general goal of positive psychology is to find ways in which psychological research can help people to be happier, and to lead more fulfilling lives. Positive psychology can also serve as a focus for psychologists to become more appreciative of not only human nature, but also of the potential for the field of psychology itself to benefit all people (Sheldon & King, 2001). Table 1.3 offers a sampling of the wide range of interest in positive psychology that exists today.

Closely related to positive psychology is the concept of resilience. Many individuals face difficult or traumatic challenges in life, and yet some manage to maintain stability in their lives in spite of these unfortunate circumstances. How exactly these individuals maintain stability and a positive direction in their lives is not always clear, and there may be a variety of different ways that individuals respond to such extreme stress (Bonanno, 2004, 2005a; Masten, 2001; for commentary on the first article see also Bonanno, 2005b; Kelley, 2005; Linley & Joseph, 2005; Litz, 2005; Maddi, 2005; Roisman, 2005). Among the important factors, particularly for our perspective here, is the ability to maintain positive emotions and to pursue self-enhancement (Bonanno, 2004, 2005; Masten, 2001). Throughout history, a variety of cultures have given rise to spiritual pursuits that help to guide the development of individuals in positive directions. We will cover some of these spiritual paths in the last section of this book, taking just a brief look here at the relationship between spirituality, positive psychology, and personality.

Table 1.3: Selection of Books and Articles Related to Positive Psychology Published Since the Year 2000	
Books on Positive Psychology	Compton, 2005; Snyder & Lopez, 2005
Human Strengths and Virtues	Aspinwall & Staudinger, 2003; Fowers, 2005
Happiness and Well-Being	Cloninger, 2004; Hampson, 2008; Hsee et al., 2008; Huppert, 2009; Inglehart et al., 2008; Molden et al., 2009; Seligman, 2002; Siegel, 2007
Articles on Academic Excellence and Creativity	Lubinski & Benbow, 2000; Simonton, 2000; Winner, 2000
Cognition and Motivation	Baltes & Staudinger, 2000; Lyubomirsky, 2001; Peterson, 2000

Coping	Folkman & Moskowitz; 2000; Vaillant, 2000
Economics	Diener, 2000; Diener & Seligman, 2004; Myers, 2000; Smith et al., 2005
Emotions, Happiness, Well-Being	Dolan & White, 2007; Ekman et al., 2005; Fredrickson, 2001; Fredrickson & Joiner, 2002; Fredrickson & Losada, 2005; Kesebir & Diener, 2008; Kim & Moen, 2001; Napier & Jost, 2008; Oishi et al., 2007; Robinson et al., 2004; Weiss et al., 2008
Enjoying a Good Life	Bauer & McAdams, 2004a,b; Bauer et al., 2005a,b; Schneider, 2001
Evolution	Buss, 2000a; Massimini & Fave, 2000
Physical Health	Cohen & Pressman, 2006; Ray, 2004; Salovey et al., 2000; Taylor et al., 2000
Relationships	Myers, 2000
Self-Determination	Ryan & Deci, 2000; Schwartz, 2000
Therapeutic Intervention	Ahmed & Boisvert, 2006; Joseph & Linley, 2006; Seligman et al., 2005
Youth Development	Larson, 2000

It appears that spirituality is an essential attribute of human nature. It has been recognized for some time that religious ritual is a cultural universal (Murdock, 1945; see also Ferraro, 2006a). More than simply a cultural universal, however, spirituality appears to be a natural consequence of child development. Deborah Kelemen (2004) brought together a number of different theories, and was able to demonstrate that young children, around

the age of 5 years old, have both the ability and the inclination to explain the world around them in terms of an intentional act by a supernatural being. Thus, Kelemen suggests that young children are what she calls “intuitive theists.” Surprisingly, this tendency appears to continue into adulthood, since even college students studying evolution exhibit a tendency to think of evolution as a purposeful agent itself, an agent that guides further evolution according to a thoughtful plan (Kelemen, 2004).

The relationship between psychology and religious/spiritual pursuits has a long and interesting history. One of William James’ most famous books is The Varieties of Religious Experience: A Study in Human Nature (James, 1902/1987), and around the turn of the century in 1900 psychologists of the day actually used religion in the popular press to help engender respect for the new field of psychology (Pickren, 2000). Since the more recent turn of the century there have been a number of books and articles published connecting psychology, spirituality, religion, and psychotherapy (see Table 1.4). Thus, a topic that was viewed as important at the beginning of the field of psychology, but was then pushed aside as unscientific, is once again become an area of interest and importance. Although spirituality is certainly not synonymous with positive psychology, it does appear to be an important factor in positive psychology.

Table 1.4: Selection of Recent Books and Articles Relating Spirituality to Psychology	
Books on psychology seen through the eyes of faith	Myers & Jeeves, 2003
Relationships between meditation, mindfulness, Buddhism and psychology	Brantley, 2003; Dockett et al., 2003; Helminiak, 2005; McQuaid & Carmona, 2004
Spiritual counseling from a variety of perspectives	Brach, 2003; Brazier, 1995; Mruk & Hartzell, 2003; Richards & Bergin, 2000, 2004, 2005; Sperry & Shafranske, 2005; Weiner et al., 2005; Williams et al., 2007
Multicultural clinical assessment	Dana, 2000; Suzuki et al., 2001
Articles on the religious roots of individualism vs. collectivism	Burston, 2001; Lynch, 2001; Lynch Jr., 2001; Margolis, 2001; Sampson, 2000
Childhood Development of Faith	Kelemen, 2004
Shamanism	Krippner, 2002
The relationships between spirituality, religion, and health	Ginges et al., 2009; Hill & Pargament, 2003; Kier & Davenport, 2004; McCormick, 2004; Miller & Thoresen, 2003; Miller & Thoresen, 2004; Powell et al., 2003; Rayburn, 2004; Richmond, 2004; Seeman et al., 2003; Seybold & Hill, 2001; Wallace & Shapiro, 2006

Numerous studies have shown that individuals who are actively spiritual have higher levels of well-being and fewer serious problems in their lives (see Compton, 2005; Myers, 2000; Seligman, 2002; Seligman & Csikszentmihalyi, 2000). The recently published Handbook of Positive Psychology has two chapters devoted specifically to spiritual pursuits and their benefits (Pargament & Mahoney, 2005; Shapiro et al., 2005). Peterson and Seligman (2004) have identified spirituality as one of the twenty-four specific character strengths that have consistently emerged across history and culture. Indeed, they believe that spirituality "is the most human of the character strengths as well as the most sublime…People with this strength have a theory about the ultimate meaning of life that shapes their conduct and provides comfort to them" (pg. 533; Peterson & Seligman, 2004). In the last section of this book we will examine a number of spiritual approaches to life, each of which suggests a path for positive development. Despite being associated with very different religions, which range from 1,400 years old to perhaps more than 5,000 years old, these spiritual paths have much in common. Perhaps this should not be surprising, as it may help to explain the inherent nature of children to be "intuitive theists" and the universality of religious ritual in human culture.

Discussion Question: Do you believe that psychology should work to develop itself as a field that focuses on helping people to develop in positive ways? Can spirituality or religion be helpful, or might they present more problems?

Methods of Studying Personality

In all types of research, we need to consider two closely related concepts: **hypothesis** vs. **theory**. An hypothesis can loosely be defined as an educated guess about some relationship or circumstance that we have observed, and the purpose of the hypothesis is to explain what we have experienced and to provide a starting point for further research. When a set of observations seems to come together, especially as the result of testing our hypotheses, we might then propose a theory to bring those observations together. However, a theory is not necessarily our end point, since the theory itself may generate new hypotheses and more research. In this way, all scientific endeavors continue to develop, expand, clarify, change, whatever the case may be, over time. As a result, we have many different personality theories, since different theorists have viewed the human condition differently, and they have also used different techniques to study personality.

A variety of methods have been used to study personality. Much of the early research was based on clinical observations, which were not done according to strict experimental methods. Today, ethical restrictions on the types of research we can conduct with people limit our ability to re-evaluate many of those classic studies. So we are left with a field that is rich in theory, but somewhat poor in the validation of those theories. Of course some personality theorists have approached personality in a more scientific manner, or at least they have tried, but that has limited the questions they have been able to ask. Since a detailed analysis of experimental psychology and research design is beyond the scope of this textbook, we will only cover this topic briefly (though it may come up again within individual chapters).

Case Studies

Many of the best-known personality theorists relied on case studies to develop their theories. Indeed, it was after seeing a number of patients with seemingly impossible neurological complaints that Freud began to seek an explanation of psychological disorders. Basically, the case study approach relies on a detailed analysis of interesting and unique individuals. Because these individuals are unique, the primary criticism of the case study approach is that its results may not generalize to other people. Of greater concern, is the possibility that early theorists chose to report only those cases that seemed to support their theories, or perhaps they only recognized those elements of a patient’s personality that fit their theory? Another problem, as mentioned above, is that two different theorists might view the same cases in very different ways. For example, since Carl Rogers worked initially with children, he found it difficult to accept Freud’s suggestions that even children were motivated primarily by sexual and aggressive urges. Consequently, Rogers sought a more positive view of personality development, which led to the establishment of the humanistic perspective. Thus, the case study approach can lead to very different conclusions depending on one’s own perspective while conducting research. In other words, it can easily be more subjective than objective, and psychologists who focus on our field as a scientific discipline always strive for more objective research.

Correlational Designs

When conducting correlational research psychologists examine the relationships that exist between variables, but they do not control those variables. The measure that is typically used is the **correlation coefficient**, which can range from –1.0 to 0.0 to +1.0. A value close to zero suggests that there is no relationship between the variables, whereas a value closer

to –1.0 or +1.0 suggests a strong relationship, with the direction of the relationship determining whether the value is positive or negative. It is important to remember that the strength of the correlation is determined by how far the correlation coefficient is from zero, not whether it is positive or negative. For example, we would most likely find a positive correlation between the number of hours you study for a test and the number of correct answers you get (i.e., the more you study, the more questions you get right on the test). On the other hand, the exact same data will give us a negative correlation if we compare the number of hours you study to the number of questions you get wrong (i.e., the more you study, they fewer questions you get wrong). So the way in which you ask the question can determine whether you have a positive or negative correlation, but it should not affect the strength of the relationship.

Since the investigator does not control the variables in correlational research, it is not possible to determine whether or not one variable causes the relationship. In the example used above, it certainly seems that studying more would lead to getting a better grade on a test. But consider another example: can money buy happiness? There is some evidence that wealthy people are happier than the average person, and that people in wealthy countries are happier than those in poorer countries. But does the money affect happiness? Certainly a million dollars in cash wouldn't help much if you were stranded on a desert island, so what can it do for you at home? People with money can live in nicer, safer communities, they have access to better health care (so they may feel better physically), they may have more time to spend with their family and friends, and so in many ways their lives might be different. We can also look at the correlation the other way around; maybe happy people get more money. If you ran a company, and were going to hire or promote someone, wouldn't you want to find someone who is friendly and outgoing? Wouldn't you look for someone who other people will enjoy working with? So, maybe happy people do find it easier to be successful financially. Either way, we simply can't be sure about which variable influences the other, or even if they influence each other at all. In order to do that, we must pursue experimental research.

In these figures, adapted from research conducted by the author (Kelland et al., 1989), we see two correlations reported in an actual study. In the figure on the left, we can see a significant positive correlation between the firing rate of dopamine neurons in the rat brain and the dose of the drug quinpirole needed to inhibit those cells. In the figure on the right, we can see that the correlation is eliminated (the dose of quinpirole needed is not related to the firing rate of the cell) following administration of the drug MDMA (more commonly known as Ecstasy!).

Experimental and Quasi-Experimental Designs

The experimental design is usually preferred within psychology, as with any other science. The goal is to control every aspect of the experiment and then manipulate a single variable, thus allowing us to attribute the results to that single manipulation. As a result, experiments allow us to make cause-and-effect statements about the relationships between the variables.

A simple experiment begins with defining the **independent variable**, the factor that will be manipulated, and the **dependent variable**, the factor that will be measured. Ideally, we then select our subjects in a random fashion, and assign them randomly to a **control group** and an **experimental group**. The experimental group is then exposed to the independent variable, whereas the control group is not. If we have successfully controlled all other variables through random selection of subjects (i.e., all subjects in a specified population

have an equal chance of being selected for the study) and random assignment to the control and experimental groups (so that hopefully each group has an equal representation of gender, races, age, intelligence, personal habits, etc.), we should see a difference in the dependent variable that was caused by the independent variable.

Unlike the natural sciences, however, we can seldom control human behavior in the precise ways that true experimental designs require. For example, if we want to study the effects of prenatal exposure to cocaine on personality development, we certainly cannot ask pregnant women to use cocaine. Unfortunately, there are pregnant women who abuse cocaine and other illegal drugs. Therefore, we can try to identify those women, and subsequently study the development of their children. Since a variety of other factors led these women to abuse illegal drugs, we lose the control that is desired in an experiment. Such studies are called quasi-experimental, because they are set up as if we did an experiment, and can be analyzed in similar ways. The quasi-experimental approach has many applications, and can provide valuable information not available otherwise, so long as the investigators keep in mind the limitations of the technique (for the classic discussion of this design see Campbell & Stanley, 1963).

Table 1.5: Research Designs in the Study of Personality	
Case Studies	Focus is on a detailed examination of unique and interesting cases. May provide a great deal of information, but due to the individual nature of the case that information may not generalize to others.
Correlational Designs	Focus is on how variables change in relation to each other. However, since there is no control over that change, we cannot determine whether one variable affects the other, or vice versa, or even if the change is due to some unidentified outside factor.
Experiments	Experiments are usually the preferred type of research, since the control exerted over the variables involved allows the investigator to make cause-and-effect statements about the results obtained. However, ethical considerations often make experimental research with humans unacceptable.
Quasi-Experiments	When individuals choose to put themselves in situations that psychologists could not ethically create, a situation arises in which the consequences can be studied as if an experiment was done. These are not true experiments, however, since the investigator cannot be sure of all the variables that led the individuals to put themselves in such situations to begin with.

Cross-Cultural andMulticultural Research	In an effort to control the conditions of their research, psychologists try to work with clearly defined groups. Unfortunately, that means their results may not generalize to other groups of people. Cross-cultural and multicultural psychologists remind us that, in a truly global world, we must strive to address both the differences and the similarities that characterize human nature.

Cross-Cultural Approaches to the Study of Personality

Cross-cultural approaches to studying personality do not really represent a different type of research, but rather an approach to research that does not assume all people are influenced equally by the same factors. More importantly, cross-cultural psychologists recognize that seemingly common factors may, in reality, be quite different when viewed by people of very different cultures. The most obvious problem that arises when considering these issues is the potential difference between cross-cultural and multicultural research. Cross-cultural research is based on a comparison of cultures; two well-known categorizations are Eastern vs. Western perspectives and the somewhat related topic of individualistic vs. collectivistic cultures. However, a multicultural approach tells us that we must consider the true complexity of the human race. What is “Eastern,” is it Asia, China, Japan, does it include India, and what about Muslim groups of people? Should Buddhism be viewed as an Eastern perspective or a religious perspective? This book will address a variety of spiritual paths toward positive psychological development, but none of the associated religions are indigenous to Africa, so will our discussions be complete? The list goes on and on, because there are so many different cultures in the world. And finally, is it practical to really try coming up with a theory of personality that can encompass all the different groups of people throughout the world? Only by pursuing an understanding of different cultures can psychology truly be considered a global science, and that pursuit has only just begun. Since we have a long way to go, the future is ripe for new students to pursue careers in psychology and the study of personality.

Discussion Question: Do you consider psychology to be a science? Has psychology successfully applied the scientific method to the study of mind and behavior, particularly the study of personality and personality development?

Application of Personality Theory - Assessing Personality

As in the section above on research methods, an extensive discussion of personality assessment is beyond the scope of this textbook. However, this is such an important issue that we will look at it briefly here, and then will take a closer look in some of the chapters throughout the rest of the book. There are a number of excellent handbooks available on psychological assessment (e.g., Goldstein & Hersen, 1990; Groth-Marnat, 2003), including two that focus on cross-cultural and multicultural assessment (Dana, 2000; Suzuki et al., 2001).

Personality assessment most commonly occurs in a clinical setting, when an individual is seeking help for some problem, whether it is an adjustment disorder or a potential mental illness. Assessing personality goes beyond this singular role, however. Certainly a clinical psychologist would be using personality assessment in order to understand a patient’s symptoms, provide a diagnosis (if appropriate), and recommend a preferred course of therapy. Similarly, school psychologists use assessment to identify any possible learning

disorders and/or adjustment issues as they pertain to the educational environment. But other psychologists use personality assessment for a variety of reasons as well. Industrial/organizational psychologists use personality assessment to identify preferred candidates for particular jobs, career counselors use these assessments in order to provide valid recommendations regarding the choice of a career path, and research psychologists use assessment in their ongoing efforts to correlate certain personality types to observable behavior or other measures. Thus, the assessment tools used to describe and/or understand personality have a wide range of potential applications.

Reliability, Validity, and Standardization

A particular personality assessment is of little value if it has no reliability or validity and if it is not presented in a standardized format. **Reliability** refers to the likelihood that a test will give essentially the same result on different occasions, or that two versions of the same test will give similar results. **Validity** refers to whether a test actually measures what it purports to measure. **Standardization** refers to the manner in which a test is given, which must be the same for every person receiving the test if there is to be any value in comparing the results among different people.

Determining the reliability and validity of a test can be a long and complicated process, involving a variety of statistical methods to confirm the results. During this process the psychologist(s) developing the test will also typically establish norms. **Norms** are consistent ways in which particular groups score on a test. For example, on measures of aggressiveness the "normal" level for men may be quite different than the "normal" level for women. Standardization is quite a bit simpler to establish, since the test can include precise instructions dictating the manner in which it is to be given.

Assessing Personality with Objective Tests

The most famous self-report inventory is the **Minnesota Multiphasic Personality Inventory** (or **MMPI**). The MMPI is also probably the most widely used psychological test in the world, and it has stood the test of time (it is currently in its second version, a 1989 revision of the 1943 original). The current version consists of 567 true-false questions, which address not only normal personality traits, but psychopathology and the accuracy of the test-taker as well. The test has several built in "lie" scales, in case a person were trying to fake a mental illness (e.g., if they were trying to fake an insanity defense to avoid responsibility for a crime) or minimize any symptoms they may actually be experiencing. The questions themselves range from rather simple (e.g., I enjoy drama.) to rather strange (e.g., I am a prophet of God.), but when put all together they provide a highly valid assessment that can easily be scored by computer (hence the popularity of the test, for both reasons). NOTE: Those are not actual questions from the MMPI, but they are based on real questions. The MMPI is an empirically based instrument. That is, interpretations are based on the pattern of responding obtained by various psychiatric samples. Since the standard MMPI was developed for adults and is rather lengthy, an abbreviated version was developed for use with adolescents: the MMPI-A.

A number of alternatives to the MMPI have been developed. The California Psychological Inventory has been available almost as long as the MMPI and, more recently, the Personality Assessment Inventory has become popular. Another important test is Millon’s Clinical Multiaxial Inventory (the MCMI), which was developed in accordance with Millon’s own

theories on personality development and personality disorders (see Appendix A). The MCMI was designed with certain advantages in mind, including being relatively short compared to the MMPI and being connected with a specific clinical theory. However, since the test was designed specifically to distinguish amongst psychiatric populations, it is not as useful when assessing “normal” individuals (Keller et al., 1990; Groth-Marnat, 2003).

Behavioral assessment and thought sampling are techniques designed to gain an appreciation of what an individual actually does and/or thinks on a day-to-day or moment-to-moment basis. In each case, observers are trained to make precise observations of an individual at precise times. This provides a statistical sample of the individual's actual behavior and/or thoughts over time. Naturally the only person who can record an individual's thoughts is that person himself or herself, but as long as they are carefully informed of the procedure and are fully cooperating, the technique works fine. When applied correctly, the great value of these techniques is that they are truly objective, in other words, they record actualbehaviors and actualthoughts.

Assessing Personality with Projective Tests

The two most famous projective tests are the **Rorschach Inkblot Technique** and the **Thematic Apperception Test** (or **TAT**). Both tests involve the presentation of ambiguous stimuli in an attempt to draw out responses from a patient, responses reflecting impulses and/or thoughts that the patient may not even be aware of (i.e., the patient projects their own thoughts and feelings onto the ambiguous stimuli, even if those thoughts and feelings are subconscious).

The Rorschach Inkblots are just that, inkblots on a piece of paper that can look like most anything. An individual being tested is first asked to say what each inkblot looks like, and then they are asked to explain how they saw what they identified. The answer to a single inkblot is not particularly informative, since any one inkblot may remind the person of some particular thing. However, as the patient goes through all 10 inkblots, trends should become apparent to the psychologist that reflect the dominant issues affecting the personality of the patient (again, even if those issues are subconscious and not available to the conscious awareness of the patient). Initially, the Rorschach was reviewed unfavorably and then ignored. Rorschach became depressed, and died only 9 months after the test was published. Eventually, however, the test became more and more popular, and today is certainly one of the most widely recognized psychological tests. However, studies comparing the Rorschach and the MMPI have shown the latter to be far more valid. In an effort to improve both the reliability and validity of the Rorschach technique, there is now a standardized scoring system.

The TAT is similar to the Rorschach, except that it involves actual pictures of people (although they are still very ambiguous drawings) and the patient is asked to tell a story about the people in the picture. There is no objective scoring system for the TAT, so reliability and validity remain arguable, and the test is more famous than popular as an assessment tool. However, it has been shown to have high validity for certain specific research studies, such as studies on the need for achievement, and continues to serve a function in clinical formulations.

Clinical Interviews

As valuable and informative as the well-established psychological tests are, there is certain vital information that simply cannot be addressed with most tests, such as: a person's appearance, their attitude, facial expressions, ability to communicate with another person, etc. In addition, tests often lead to further questions, or the need for clarification or explanation. In order to address such issues, both in general and in greater detail, clinical interviews are an essential part of the overall personality assessment. Although the results of an interview are somewhat subjective, when viewed in the context of the psychologist's clinical experience, along with results of an assessment tool, they provide psychologists with a much more complete understanding of the person whom they are evaluating.

Discussion Question: Have you or anyone you know ever had psychological testing (don't forget standardized tests of knowledge and intelligence in school!)? If you are at all familiar with psychological testing, for any reason, what effect did it have on you (or someone you know)?

Critical Thinking in Psychology

Critical thinking is always important in psychology, but given the complexity of individual personalities, the many different theories, and the variety of approaches for studying and assessing personality, it is particularly important for our consideration here. Although we often think of the word critical as something negative, when we talk about critical thinking in psychology we are actually talking about being open-minded to many possible answers, but arriving at a most likely answer in a reasoned and logical fashion. Critical thinking is a skill, but unfortunately one that all too often isn't taught (Halpern, 1996, 2007; Sternberg, 2007).

A typical approach to teaching critical thinking is to use examples of false claims and systematically deconstruct the manner in which they are made to appear true, while at the same time discussing the psychological processes involved in decision making (see, e.g., Halpern, 1996; Ruscio, 2006). John Ruscio has done a nice job of organizing his discussion around four areas pertaining to the tactics of pseudoscientists who would intentionally mislead us: 1) deception, the methods they use to deceive us; 2) self-deception, the types of evidence that lead us toward unwittingly deceiving ourselves; 3) psychological tricks, a variety of tricks that create and sustain unwarranted beliefs; and 4) the decision-making process and the ethical concerns of pseudoscientific practices. Ruscio (2006) has also provided a handy list of the characteristics of pseudoscience:

1. Outward Appearance of Science

1. Absence of Skeptical Peer Review

1. Reliance on Personal Experience

1. Evasion of Risky Tests

1. Retreats to the Supernatural

1. The Mantra of Holism

1. Tolerance of Inconsistencies

1. Appeals to Authority

1. Promising the Impossible

1. Stagnation

While it may seem tempting for you to take for granted that you do not need to apply critical thinking to the theories presented in this book, that could present something of a problem for you. Many of these theories disagree with one another. Although the major theories have all been proposed by famous and respected theorists, some critics claim they were not developed scientifically, and the spiritual paths that will be discussed in the last section of the book have many skeptics. As you consider each theory, there are some critical thinking skills you can keep in mind. What is your goal as you evaluate a theory? What do you know and how are you drawing conclusions? If your class is having a debate or a discussion what is being said, how is it being said, and how are the arguments being analyzed? Are certain conclusions probable; are you, or others, overconfident in your conclusions? Have you considered alternatives? Practicing these, and other, skills can help to develop your critical thinking abilities (Halpern, 2007). Finally, consider this “simple” definition of critical thinking offered by Diane Halpern:

Critical thinking is the use of those cognitive skills or strategies that increase the probability of a desirable outcome. It is used to describe thinking that is purposeful, reasoned, and goal directed - the kind of thinking involved in solving problems, formulating inferences, calculating likelihoods, and making decisions, when the thinker is using skills that are thoughtful and effective for the particular context and type of thinking task. (pg. 6; Halpern, 2007)

Discussion Question: When you hear someone make a claim, whether it is something scientific or a commercial advertisement, do you tend to believe it, or do you apply critical thinking to evaluate whether the claim is likely to be true?

Personality Theory in Real Life: Making the Connection Between

Your Life and Personality Theory

In this chapter we do not have a particular theory or perspective within which to consider your own life. So, let’s try considering your life in any way you want. I do want you to consider one basic question, though. Who are you? You might also ask yourself what makes you the person you think you are. Try writing down some of your thoughts. Writing the ideas down helps to force you to really pay attention to your thoughts, rather than just casually thinking about the questions without going into any detail. When you are done, take a look at what you have written. Ask yourself again, “Is that really me?” You may want to write down your new thoughts after evaluating what you have written.

Then try something that may be very interesting, but possibly a little unnerving. Ask a friend or relative, someone you think really knows you well, and have them write down some ideas on who you are. Don’t bother them, or distract them, while they are doing this. Let them have the time they need to do it. Then look at what they have written, and once again ask, “Is that really me?” Finally, compare what you wrote and what they wrote. Is there a difference, and if so, is it a big difference?

Whether the different descriptions of who you are or, in other words, the descriptions of your personality are the same or different, how do you feel about that? Some may find comfort in learning that others see them as they see themselves. Some may be confused if others see them quite differently than they see themselves. There are no right or wrong

answers here, it is just an exercise to help you begin thinking about how psychologists study personality. As we move through the various theories and perspectives presented in this textbook, it will provide a starting point from which you can hopefully learn something interesting about yourself and about the people you interact with every day.

Review of Key Points

• There is no simply definition of personality. The nomothetic approach focuses on personality as a construct, while the idiographic approach emphasizes the uniqueness of individuals.

• A wide variety of theoretical perspectives influence how psychologists view personality, including psychodynamic factors, learning/cognitive factors, biological factors, inherent drives, and sociocultural influences.

• The various personality theories also address questions related to nature vs. nurture, whether individuals are unique or whether there are types of personality common to all people, the relative importance of the past, present, and future, and the significance of free will.

• An important trend in psychology today is the emphasis on positive psychology, and the potential for the field of psychology to contribute in positive ways to society.

• A variety of research designs have been used by personality theorists. Historically, many famous theorists relied on case studies. When possible, however, many psychologists prefer the experimental design, since only true experiments allow psychologists to make case-and-effect statements. More recently, some psychologists have begun to focus on cross-cultural and multicultural approaches to studying personality.

• A wide variety of personality tests have been developed, both objective and projective tests. Since it often proves difficult to establish the reliability and validity of some personality tests, a clinical interview is an essential step in forming an opinion regarding someone’s personality (especially if there is a question of mental illness).

• Critical thinking is purposeful, reasoned, goal-directed thinking aimed at evaluating claims that are made as being true.

CHAPTER 19 / CAC.902

Choosing, Integrating, and Evaluating Treatment

Integration is not a random mixture of techniques. It is a reasoned approach to combining theories and methods in the service of the client. Some counselors use technical eclecticism, choosing techniques based on evidence without necessarily combining theories. Others use theoretical integration, building a coherent framework. Others use common-factor approaches, emphasizing relationship and shared mechanisms. The doctoral counselor should be able to explain why a method is chosen, how it fits the formulation, and how outcome will be evaluated.

Why integrate

No single theory explains every problem. A child with anxiety and a coercive family cycle may need exposure and parent training. An adolescent with depression and identity conflict may need behavioral activation and existential dialogue. Integration allows the counselor to match treatment to complexity. However, integration can become confusion if the counselor lacks a rationale. The counselor should not switch theories whenever the client resists. Resistance may be data about the alliance, the formulation, or the environment.

A decision framework

A practical framework asks: What is the target problem? What does the evidence support for this problem and population? What does the formulation suggest maintains the problem? What does the client prefer? What is feasible in the context? What are the risks? The counselor then chooses a primary approach and supplements as needed. The primary approach provides coherence. Supplements address gaps.

Measurement-based care

Evaluation is part of treatment. The counselor selects measures that match the goals and reviews them regularly. A child's anxiety may be tracked with a brief weekly rating. A parent's coercive cycle may be tracked with a daily log. The counselor should not wait until termination to discover that treatment is not working. If progress stalls, the counselor checks alliance, formulation, fidelity, and context. Measurement is a conversation, not a punishment.

Fidelity and flexibility

Evidence-based protocols should be delivered with fidelity to core mechanisms, but flexibility is needed for culture, development, and comorbidity. The counselor should know which elements are essential and which can be adapted. For exposure, the essential element is new learning through approach. The form can be playful. For CBT, the essential element is collaborative testing of meanings. The worksheet can be replaced with a game. Fidelity without flexibility is rigid; flexibility without fidelity is drift.

Clinical example

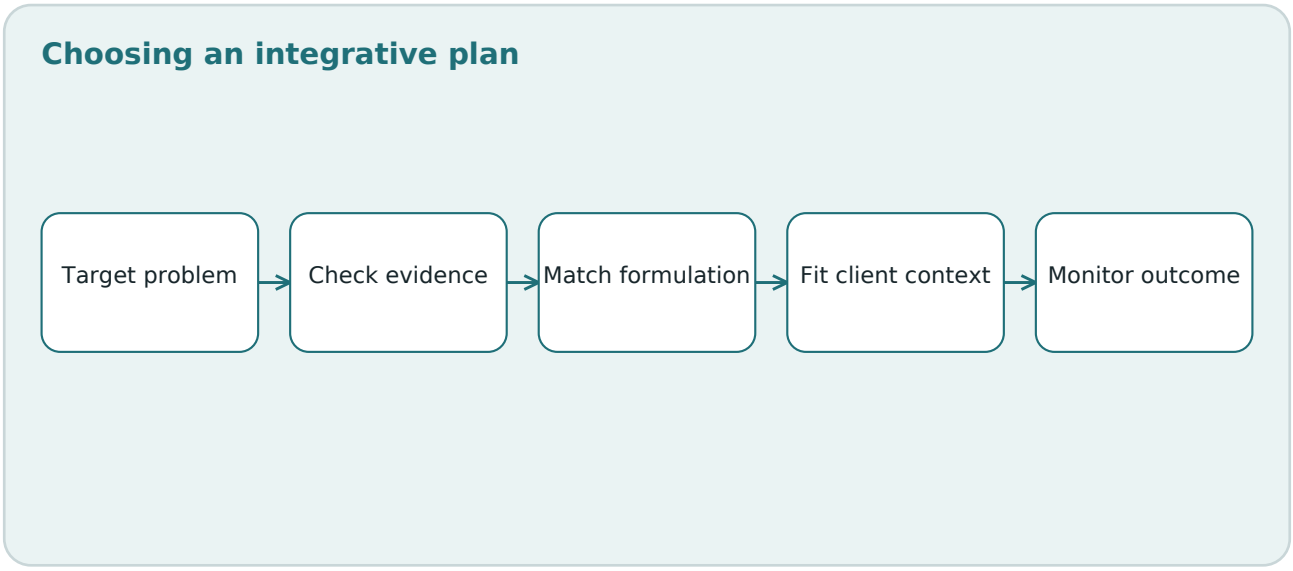
A seventeen-year-old with depression and social anxiety is treated with behavioral activation as the primary approach because evidence supports it for depression. The counselor adds exposure for social anxiety once activation improves energy. The client prefers a values-based frame, so ACT language is used to connect actions to values. The counselor tracks mood and avoidance weekly. When progress stalls, the counselor discovers that the client is being bullied online. The plan adds safety and school intervention. The theory did not fail; the context was incomplete.

Ethics of integration

Integration requires competence. A counselor should not use a technique without training. The counselor should obtain informed consent for the approach and explain the rationale. When combining methods, the counselor should consider interactions. Intense experiential work may destabilize a client who needs stabilization first. Trauma processing may require preparation. The counselor should document the rationale and supervise complex cases.

Reflection

Choose a case and write a treatment plan with a primary theory, two supplements, and a measurement plan. State the criteria for changing the plan. Integration is not a license to do everything. It is a discipline of doing what fits.



Integration is a reasoned sequence, not a random mixture of techniques.

Attributed Foundational Reading

Attribution and critical-reading note

Adapted from Psychology 2e by Rose M. Spielman, William J. Jenkins, and Marilyn D. Lovett, OpenStax, Rice University (2020), CC BY 4.0. Selection, organization, and PDF layout modified for this course; new lectures and course diagrams are separate additions.

Read historical and theoretical claims critically. A theory is a lens for inquiry, not a verdict about a child or family. Where source claims are dated or culture-bound, the new lecture supplies cautions and contemporary framing.

Source module m82285 • Psychology 2e • pinned GitHub snapshot f5c9cf506867. Full repository and license details appear in the attribution register.

The sociocultural perspective looks at you, your behaviors, and your symptoms in the context of your **culture** and background. For example, José is an 18-year-old Hispanic male from a traditional family. José comes to treatment because of depression. During the intake session, he reveals that he is gay and is nervous about telling his family. He also discloses that he is concerned because his religious background has taught him that being gay is wrong. How does his religious and cultural background affect him? How might his cultural background affect how his family reacts if José were to tell them he is gay?

Mental health professionals must develop **cultural competence** (), which means they must understand and address issues of race, culture, and ethnicity. They must also develop strategies to effectively address the needs of various populations for which Eurocentric therapies have limited application (Sue, 2004). For example, a counselor whose treatment focuses on individual decision making may be ineffective at helping a Chinese client with a collectivist approach to problem solving (Sue, 2004).

Multicultural counseling and therapy aims to offer both a helping role and process that uses modalities and defines goals consistent with the life experiences and cultural values of clients. It strives to recognize client identities to include individual, group, and universal dimensions, advocate the use of universal and culture-specific strategies and roles in the healing process, and balances the importance of individualism and collectivism in the assessment, diagnosis, and treatment of client and client systems (Sue, 2001).

This therapeutic perspective integrates the impact of cultural and social norms, starting at the beginning of treatment. Therapists who use this perspective work with clients to obtain and integrate information about their cultural patterns into a unique treatment approach based on their particular situation (Stewart, Simmons, & Habibpour, 2012). Sociocultural therapy can include individual, group, family, and couples treatment modalities.

Source figure caption: How do your cultural and religious beliefs affect your attitude toward mental health treatment? (credit “top-left”: modification of work by Staffan Scherz; credit “top-left-middle”: modification of work by Alejandra Quintero Sinisterra; credit “top-right-middle”: modification of work by Pedro Ribeiro Simões; credit “top-right”: modification of work by Agustin Ruiz; credit “bottom-left”: modification of work by Czech Provincial Reconstruction Team; credit “bottom-left-middle”: modification of work by Arian Zwegers; credit “bottom-right-middle”: modification of work by “Wonderlane”/Flickr; credit “bottom-right”: modification of work by Shiraz Chanawala)

Note: Watch this short video about cultural competence and sociocultural treatments to learn more.

Barriers to Treatment

Statistically, ethnic minorities tend to utilize mental health services less frequently than White, middle-class Americans (Alegría et al., 2008; Richman, Kohn-Wood, & Williams, 2007). Why is this so? Perhaps the reason has to do with access and availability of mental health services. Ethnic minorities and individuals of low socioeconomic status (SES) report that barriers to services include lack of insurance, transportation, and time (Thomas & Snowden, 2002). However, researchers have found that even when income levels and insurance variables are taken into account, ethnic minorities are far less likely to seek out and utilize mental health services. And when access to mental health services is comparable across ethnic and racial groups, differences in service utilization remain (Richman et al., 2007).

In a study involving thousands of women, it was found that the prevalence rate of anorexia was similar across different races, but that bulimia nervosa was more prevalent among Hispanic and African American women when compared with non-Hispanic White people (Marques et al., 2011). Although they have similar or higher rates of eating disorders, Hispanic and African American women with these disorders tend to seek and engage in treatment far less than White women. These findings suggest ethnic disparities in access to care, as well as clinical and referral practices that may prevent Hispanic and African American women from receiving care, which could include lack of bilingual treatment, stigma, fear of not being understood, family privacy, and lack of education about eating disorders.

Perceptions and attitudes toward mental health services may also contribute to this imbalance. A recent study at King’s College, London, found many complex reasons why people do not seek treatment: self-sufficiency and not seeing the need for help, not seeing therapy as effective, concerns about confidentiality, and the many effects of stigma and shame (Clement et al., 2014). And in another study, African Americans exhibiting depression were less willing to seek treatment due to fear of possible psychiatric hospitalization as well as fear of the treatment itself (Sussman, Robins, & Earls, 1987). Instead of mental health treatment, many African Americans prefer to be self-reliant or use spiritual practices (Snowden, 2001; Belgrave & Allison, 2010). For example, it has been found that the Black church plays a significant role as an alternative to mental health services by providing prevention and treatment-type programs designed to enhance the psychological and physical well-being of its members (Blank, Mahmood, Fox, & Guterbock, 2002).

Additionally, people belonging to ethnic groups that already report concerns about prejudice and discrimination are less likely to seek services for a mental illness because they view it as an additional stigma (Gary, 2005; Townes, Cunningham, & Chavez-Korell, 2009; Scott, McCoy, Munson, Snowden, & McMillen, 2011). For example, in one recent study of 462 older Korean Americans (over the age of 60) many participants reported suffering from depressive symptoms. However, 71% indicated they thought depression was a sign of personal weakness, and 14% reported that having a mentally ill family member would bring shame to the family (Jang, Chiriboga, & Okazaki, 2009).

Language differences are a further barrier to treatment. In the previous study on Korean Americans' attitudes toward mental health services, it was found that there were no Korean-speaking mental health professionals where the study was conducted (Orlando and Tampa, Florida) (Jang et al., 2009). Because of the growing number of people from ethnically diverse backgrounds, there is a need for therapists and psychologists to develop knowledge and skills to become culturally competent (Ahmed, Wilson, Henriksen, & Jones, 2011). Those providing therapy must approach the process from the context of the unique culture of each client (Sue & Sue, 2007).

Note: In the United States, about one in six children and one in five adults experiences a mental health disorder, but fewer than half of these people receive professional support for their disorder (Whitney & Peterson, 2019). Access to qualified mental health professionals is not universal or equitable, but it has improved to the point that more people could receive help if they sought it. Why then, do so many people go without support, therapy, or treatment? It seems that the public has a negative perception of people with mental health disorders. According to researchers from Indiana University, the University of Virginia, and Columbia University, interviews with over 1,300 U.S. adults show that they believe children with depression are prone to violence and that if a child receives treatment for a psychological disorder, then that child is more likely to be rejected by peers at school. Bernice Pescosolido, author of the study, asserts that this is a misconception. And it is not limited to perceptions of mental health issues in children: adults living with mental health issues may face even more scrutiny when sharing their condition or seeking support. Stigmatization of psychological disorders is one of the main reasons why people do not get the help they need when they are having difficulties. Pescosolido and her colleagues caution that this stigma surrounding mental illness, based on misconceptions rather than facts, can be devastating to emotional and social well-being. Fortunately, we are starting to see discussions related to the destigmatization of mental illness and an increase in public education and awareness. Dozens of leaders have contributed to the conversation, including athletes like Naomi Osaka, Simone Biles, Michael Phelps, Kevin Love, and Dak Prescott, as well as artists such as Adele, Bruce Springsteen, Ariana Grande, Big Sean, and Bebe Rexha. Mental health awareness is stronger within workplaces, educational settings, and communities overall. However, stigma remains, particularly regarding mental health issues that are frequently misunderstood. I've become accustomed to the reaction when I tell people where I'm from. Eleven months later, it's as consistent as it was back in January. Just yesterday, inquiring as to the availability of a rental house this holiday season, the gentleman taking my information paused to ask, "Newtown, CT? Isn't that where that...that thing happened? A recent encounter in the Massachusetts Berkshires, however, took me by surprise. It was in a small, charming art gallery. The proprietor, a woman who looked to be in her 60s, asked where we were from. My response usually depends on my present mood and readiness for the inevitable dialogue. Sometimes it's simply, Connecticut. This time, I replied, Newtown, CT. The woman's demeanor abruptly shifted from one of amiable graciousness to one of visible agitation. "Oh my god," she said wide eyed and open mouthed. "Did you know her?" . . . . "Her?" I inquired That woman," she replied with disdain, "that woman that raised that monster." "That woman's" name was Nancy Lanza. Her son, Adam, killed her with a rifle blast to the head before heading out to kill 20 children and six educators at Sandy Hook Elementary School in Newtown, CT last December 14th. When Nelba Marquez Greene, whose beautiful 6-year-old daughter, Ana, was killed by Adam Lanza, was recently asked how she felt about "that woman," this was her reply: "She's a

victim herself. And it's time in America that we start looking at mental illness with compassion, and helping people who need it. “This was a family that needed help, an individual that needed help and didn't get it. And what better can come of this, of this time in America, than if we can get help to people who really need it?” (pars. 1–7, 10–15) Join the effort by encouraging and supporting those around you to seek help if they need it. To learn more, visit the National Alliance on Mental Illness (NAMI) website (http://www.nami.org/).

Summary

The sociocultural perspective looks at you, your behaviors, and your symptoms in the context of your culture and background. Clinicians using this approach integrate cultural and religious beliefs into the therapeutic process. Research has shown that ethnic minorities are less likely to access mental health services than their White middle-class American counterparts. Barriers to treatment include lack of insurance, transportation, and time; cultural views that mental illness is a stigma; fears about treatment; and language barriers. Supporting mental health treatment involves speaking and listening openly about mental health, avoiding assumptions, being conscious about language, and encouraging others to get help when needed.

Review Questions

The sociocultural perspective looks at you, your behaviors, and your symptoms in the context of your _____.

1. education

1. socioeconomic status

1. culture and background

1. age

Which of the following was not listed as a barrier to mental health treatment?

1. fears about treatment

1. language

1. transportation

1. being a member of the ethnic majority

Critical Thinking Question

Lashawn is a 24-year-old African American female. For years she has been struggling with bulimia. She knows she has a problem, but she is not willing to seek mental health services. What are some reasons why she may be hesitant to get help?

Personal Application Question

What is your attitude toward mental health treatment? Would you seek treatment if you were experiencing symptoms or having trouble functioning in your life? Why or why not? In what ways do you think your cultural and/or religious beliefs influence your attitude toward psychological intervention?

Attribution and critical-reading note

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Read historical and theoretical claims critically. A theory is a lens for inquiry, not a verdict about a child or family. Where source claims are dated or culture-bound, the new lecture supplies cautions and contemporary framing.

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Evaluating Various Forms of Psychotherapy

How can we assess the effectiveness of psychotherapy? Is one technique more effective than another? For anyone considering therapy, these are important questions. According to the American Psychological Association, three factors work together to produce successful treatment. The first is the use of evidence-based treatment that is deemed appropriate for your particular issue. The second important factor is the clinical expertise of the psychologist or therapist. The third factor is your own characteristics, values, preferences, and culture. Many people begin psychotherapy feeling like their problem will never be resolved; however, psychotherapy helps people see that they can do things to make their situation better. Psychotherapy can help reduce a person’s anxiety, depression, and maladaptive behaviors. Through psychotherapy, individuals can learn to engage in healthy behaviors designed to help them better express emotions, improve relationships, think more positively, and perform more effectively at work or school.

Many studies have explored the effectiveness of psychotherapy. For example, one large-scale study that examined 16 meta-analyses of CBT reported that it was equally effective or more effective than other therapies in treating PTSD, generalized anxiety disorder, depression, and social phobia (Butlera, Chapmanb, Formanc, & Becka, 2006). Another study found that CBT was as effective at treating depression (43% success rate) as prescription medication (50% success rate) compared to the placebo rate of 25% (DeRubeis et al., 2005). Another meta-analysis found that psychodynamic therapy was also as effective at treating these types of psychological issues as CBT (Shedler, 2010). However, no studies have found one psychotherapeutic approach more effective than another (Abbass, Kisely, & Kroenke, 2006; Chorpita et al., 2011), nor have they shown any relationship between a client’s treatment outcome and the level of the clinician’s training or experience (Wampold, 2007). Regardless of which type of psychotherapy an individual chooses, one critical factor that determines the success of treatment is the person’s relationship with the psychologist or therapist.

CHAPTER 20 / CAC.902

Doctoral Capstone: Ethics, Supervision, and Research

The final chapter turns from theory to the professional container that makes theory safe. Doctoral counseling is not only the accumulation of knowledge. It is the formation of a practitioner who can think critically, act ethically, use supervision, and contribute to research. Theories are powerful. They can help, and they can harm when used rigidly, culturally insensitively, or without evidence. The capstone task is to hold theory lightly enough to learn and firmly enough to act.

Ethics as theory in action

Ethical principles include beneficence, nonmaleficence, autonomy, justice, and fidelity. In child work, autonomy is developing and consent is shared with caregivers. The counselor must balance the child's privacy with the parent's right to know. Confidentiality limits should be explained early. A counselor should not promise secrecy that cannot be kept. When safety is at risk, the counselor acts to protect while preserving as much trust as possible.

Justice requires attention to access and bias. A counselor should not offer only expensive, long-term therapy when brief, evidence-based options exist. A counselor should not pathologize cultural differences. A counselor should consider language, disability, and economic barriers. Ethical practice is not a code memorized for an exam. It is a habit of asking who benefits, who is harmed, and whose voice is missing.

Supervision and the use of self

Supervision is a structured relationship in which the counselor reviews cases, receives feedback, and develops competence. The counselor should bring not only successes but also confusion, countertransference, and mistakes. A supervisee who hides difficulty cannot be helped. Supervision is also a place to examine cultural assumptions and power. A supervisor should not impose a single theory as the only valid lens. The supervisee should learn to justify choices and remain open to correction.

The use of self means that the counselor's own history, values, and reactions are part of the work. The counselor does not erase the self. The counselor becomes aware of it. A counselor who was parentified as a child may overidentify with a child caregiver. A counselor who values achievement may push a client too hard. Supervision helps the counselor separate the client's needs from the counselor's unfinished business.

Research and the scholar-practitioner

Doctoral work includes research literacy. The counselor should be able to read a study, evaluate its design, and apply findings cautiously. Randomized trials, meta-analyses, and qualitative studies each answer different questions. A treatment that works in a university clinic may not work in a rural school without adaptation. The counselor should contribute to knowledge by documenting outcomes, participating in research, and staying current. Research is not separate from practice. It is the discipline of checking whether practice helps.

A capstone formulation project

A capstone project might integrate the book's theories around one case. The student writes a multimethod assessment, compares at least three theoretical formulations, chooses an integrative plan, and defines evaluation criteria. The student also writes a cultural formulation and an ethical analysis. The project is not about proving one theory correct. It is about demonstrating reasoned, responsive, and accountable practice.

Continuing the journey

Theories will change. New evidence will emerge. Some ideas in this book will be revised. The counselor's task is not to memorize a final answer. The task is to remain curious, humble, and rigorous. A child who enters the room is not a theory. The child is a person with a future. The counselor's privilege is to help that future become more possible.

Reflection

Write your own statement of theoretical orientation. Include what you believe about change, what you consider essential in the relationship, how you use evidence, and how you handle uncertainty. Then write what you still need to learn. A doctoral orientation is not a finished identity. It is a commitment to ongoing formation.

The scholar-practitioner container



Doctoral practice is held by ethics, supervision, and evidence, not by theory alone.

Glossary

Glossary

Acceptance: Willingness to experience thoughts, feelings, and sensations without unnecessary struggle, especially when struggle blocks valued action.

Alliance: The working relationship in therapy, including bond, agreement about goals, and agreement about tasks.

Archetype: In Jungian theory, a proposed universal pattern of image or behavior; used clinically as metaphor rather than settled science.

Attachment: An enduring emotional bond that shapes expectations about safety, comfort, and availability of care.

Automatic thought: A quick, often unnoticed interpretation of a situation that influences emotion and behavior.

Basic anxiety: Horney's term for the feeling of being isolated and helpless in a potentially hostile world.

Behavioral activation: A treatment that increases contact with rewarding and meaningful activities to reduce depression.

Case formulation: A testable hypothesis that links assessment information to a treatment plan.

Cognitive defusion: Noticing thoughts as mental events rather than literal truths that must control action.

Cognitive restructuring: Examining and revising unhelpful interpretations through evidence and alternative perspectives.

Common factors: Elements shared across therapies, such as alliance, empathy, hope, and goal agreement.

Congruence: Genuineness or transparency in the therapist, within professional limits.

Countertransference: The therapist's emotional reactions to the client, used as information rather than acted out.

Defense mechanism: A psychological strategy that manages anxiety or conflict, often outside awareness.

Dereflection: A logotherapy technique that shifts attention from excessive self-focus toward meaningful action.

Dialectics: Holding opposites together, especially acceptance and change, as in DBT.

Empathy: Accurate understanding of the client's inner frame of reference, communicated and checked.

Exposure: Planned approach to feared stimuli so that new safety learning can occur.

Externalization: Separating the person from the problem, as in narrative therapy.

Functional assessment: Analysis of antecedents, behavior, and consequences to identify what maintains a behavior.

Gestalt: An experiential approach emphasizing present awareness, contact, and the whole person.

Holding environment: A reliable relational frame that supports development and repair, from Winnicott.

Individuation: In Jungian theory, the process of becoming a more whole and integrated person.

Internal working model: An expectation about self and others formed in early caregiving relationships.

Logotherapy: Frankl's approach centered on the will to meaning.

Mentalization: The capacity to understand behavior in terms of mental states such as feelings, beliefs, and intentions.

Mindfulness: Paying attention to present experience with curiosity and reduced reactivity.

Negative reinforcement: Increasing a behavior because it removes an aversive condition.

Object relations: Psychodynamic theories emphasizing internalized relationship patterns.

Parataxic distortion: Sullivan's term for distorted interpersonal perception based on earlier patterns.

Persona: In Jungian theory, the social mask presented to the world.

Positive reinforcement: Increasing a behavior because it adds a valued consequence.

Projective identification: A psychodynamic process in which a person induces feelings in another that match disowned experience.

Psychological flexibility: The ability to stay present and choose values-guided action even when difficult experiences are present.

Rupture and repair: A strain in the therapeutic relationship followed by acknowledgment, listening, and adjustment.

Self-efficacy: Belief in one's capability to organize and execute actions needed for a situation.

Shadow: In Jungian theory, qualities excluded from the conscious self-image.

Social interest: Adler's term for connection and contribution to the common good.

Splitting: Experiencing self or others as all-good or all-bad rather than integrated.

Technical eclecticism: Choosing techniques from different approaches based on evidence and fit, without necessarily merging theories.

Theoretical integration: Combining theories into a coherent framework.

Transference: Redirecting feelings and expectations from earlier relationships onto the therapist.

Transitional object: An object that supports a child's move from dependence toward separateness.

Unconditional positive regard: Valuing the client without making worth conditional on behavior.

Working through: Repeatedly examining and experiencing a pattern until it becomes more flexible.

Zone of proximal development: The gap between what a learner can do alone and what they can do with support; useful for developmental adaptation.

Applied Workbook

Applied Workbook: Theory-to-Practice Exercises

This workbook turns each chapter into a small professional task. Use fictional or de-identified cases. Do not practice intense techniques on real clients without training, consent, and supervision. The goal is not to prove a theory right. The goal is to practice reasoning, humility, and evidence-informed choice.

Exercise 1: Theory as a map

Choose one child behavior, such as school refusal. Write three hypotheses from three different theories. For each, list one observation that would support it and one observation that would challenge it. End with a question you would ask the child to check your map.

Exercise 2: Alliance audit

Recall a helping conversation. Rate bond, goal agreement, and task agreement from zero to ten. Identify the weakest part. Write one repair sentence you could use, such as, "I think I moved too fast. What did I miss?"

Exercise 3: Defense and meaning

Write a behavior that puzzles you. Describe it without interpretation. Then write a possible defense or protective function. Finally, write a gentle, tentative statement that invites the client to confirm or correct the hypothesis.

Exercise 4: Attachment cycle map

Draw a cycle: child distress, caregiver response, child expectation, future behavior. Mark where a new response could interrupt the cycle. Write one caregiver coaching sentence that validates the child's feeling while keeping a limit.

Exercise 5: Belonging strategy

Identify a child's apparent goal: attention, power, revenge, avoidance, or contribution. Write one encouragement statement that is specific and genuine. Avoid empty praise. Explain how the statement supports social interest.

Exercise 6: Protective moves

Write a case where a client moves toward, against, or away from people. Identify the anxiety underneath. Write an intervention that increases safety so the client can choose more flexibly.

Exercise 7: Symbolic work with consent

Choose a client image or metaphor. Write three questions that follow the client's meaning rather than imposing an interpretation. Include one question that asks what the image protects and one that asks what it invites.

Exercise 8: Core conditions script

Write a short dialogue in which a counselor demonstrates congruence, positive regard, and empathy with a child who failed a test. Include a check for accuracy. Then write what the counselor should not say.

Exercise 9: Meaning in a constrained life

Write a case where a client cannot change a major circumstance. Identify one response that remains possible. Write a values question that does not minimize the injustice. Explain how you would assess safety.

Exercise 10: Functional behavior plan

Choose a behavior. Write antecedents, behavior, consequences, and a hypothesis about function. Design a replacement behavior and a reinforcement plan. State how you would know the plan is working.

Exercise 11: Thought record and experiment

Write a situation, automatic thought, emotion, and behavior. Generate one alternative interpretation. Design a small behavioral experiment. State what result would make you revise the formulation.

Exercise 12: Self-efficacy ladder

Choose a goal a teen avoids. Build a mastery ladder with five steps. Identify a coping model, a source of encouragement, and a way to reinterpret arousal. Write the first step the teen could take this week.

Exercise 13: Values and defusion

Write a controlling thought. Practice naming it as a thought. Identify a value it blocks. Write one committed action that can be taken while the thought is present. Explain why the goal is not thought elimination.

Exercise 14: Awareness experiment

Write a sentence a client says while showing a body cue. Design a gentle present-moment experiment. Include a consent check and a stop rule. Explain how you would ground the client afterward.

Exercise 15: Externalizing conversation

Take a problem label, such as anxiety, and externalize it. Write five questions: naming, effects, exceptions, influence, and preferred story. Avoid blaming the child.

Exercise 16: Developmental adaptation

Choose an adult technique, such as cognitive restructuring. Adapt it for a seven-year-old using play or art. Write the materials, the steps, and the caregiver role. State what would make you simplify further.

Exercise 17: Cultural formulation

Write five questions for a family from a different cultural background. Include meaning, identity, supports, power, and barriers. Write one assumption you must not make.

Exercise 18: Four P's formulation

Write predisposing, precipitating, perpetuating, and protective factors for a case. Then write one testable prediction. State what would make you revise the formulation.

Exercise 19: Integration plan

Choose a primary theory and two supplements for a case. Justify each choice with evidence, formulation, and client preference. Write a measurement plan and a criterion for changing course.

Exercise 20: Capstone orientation statement

Write a one-page statement of your theoretical orientation. Include your view of change, the relationship, evidence, culture, and uncertainty. End with three learning goals for supervision.

Portfolio guidance

Collect your best exercises into a portfolio. For each, include the task, your response, feedback received, and a revision. A doctoral portfolio should show growth in reasoning, not just polished final answers. Keep client information de-identified and follow your program's confidentiality rules.

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